THE NCE OWNERSHIP METHOD

A Complete Study Guide for the National Counselor Examination · Second Edition

Every wrong answer belongs to someone. Learn who.

PassReady Prep · GA Integrated Therapeutic Perspectives LLC

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Not clinical or legal advice. This is a test-preparation book. It does not replace supervision, consultation, legal counsel, the current *ACA Code of Ethics*, the *NBCC Code of Ethics*, your state's laws, or the *DSM-5-TR*. Passing a practice test does not guarantee passing the NCE; the author's own first-attempt pass is one person's experience, not a promise of results.

A note on abuse. No amount of abuse is acceptable — not physical, not emotional, not sexual, not financial, not verbal. Abuse is not a communication problem. If you or someone you know needs help, call the National Domestic Violence Hotline at 1-800-799-7233 (thehotline.org) or text HOME to 741741 (Crisis Text Line).

[KE — add cover designer and interior layout credits at layout.]

A note on safety. No amount of abuse is acceptable — not physical, not emotional, not sexual, not financial, not verbal. Abuse is not a communication problem. If you or someone you know is being hurt, contact the National Domestic Violence Hotline at 1-800-799-7233 (thehotline.org) or the Crisis Text Line (text HOME to 741741).

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About the Author

KekeTheLPC (MA, LPC, NCC, BC-TMH, CPCS) is a Licensed Professional Counselor, National Certified Counselor, Board Certified–TeleMental Health Provider, and Certified Professional Counselor Supervisor in Georgia. She is the founder of GA Integrated Therapeutic Perspectives LLC and of PassReady Prep, an exam-preparation platform for counselors.

She passed the NCE on her first attempt in 2014 using the two habits this book is built on: pairing every term with the theory that owns it, and listening to review audio on repeat until the pairs were automatic. As a supervisor she now watches pre-licensed counselors prepare for the same exam, and she wrote this book to give them the method she wishes someone had handed her on day one.

She is also the author of *The Complete NCMHCE Study Guide*.

[KE — add years in practice / clinical focus / city if you want them public.]

How I Passed

I have always been a nervous tester. So it surprised no one, least of all me, that I put off the NCE after graduation. I put it off for two years.

The job market at the time meant I had to work outside the field to keep the bills paid on time. Working outside the field cost me my counseling tribe, the people who would have talked shop with me and kept the material alive. Losing them made the idea of testing scarier, not easier, and I got very good at bellyaching my excuses.

Then I caught one of my best friends in a bad mood on the right day. He said, “I’m sick of you.” A few days later we went to lunch, and he secretly left me the money for the testing fee. I did not want to disappoint someone who believed in me that much, so I registered for the exam and set the date six months out.

I bought every prep resource I could find: the ones written by real practitioners, and one from an overpriced testing mill. Then I studied. Two hours a day or more, five days a week. The more I studied, the dumber I felt. Remember, I had been out of the field for over two years. So I quit, and I started trying to figure out how to pay my friend back.

Then, out of nowhere, the text came: *How’s the studying going?*

I hadn’t found the money. I believe in karma. So I picked that purple bible back up and started again. But this time, the Lord — or the universe, whichever you can accept — said that memorizing wasn’t going to get me what I needed. That is how this process was formed.

I stopped trying to memorize everything and started pairing each term with the theory that owns it. Empty chair: Gestalt. Miracle question: solution-focused. Differenti-

ation of self: Bowen. On the exam, the wrong answers are rarely nonsense. They are real counseling terms that belong to a different theory. If you know who owns what, you can see the trap before you step in it. I put the pairs on audio and played them on repeat during every drive, until I didn't have to think about them anymore. They were just there.

I took the NCE in 2014 and passed on my first attempt.

That's the method this book is built on. Learn who owns each term. Hear it until it's automatic. Then practice spotting the owner of every wrong answer.

This book should also keep you from buying everything you can get your hands on, or dusting off all the textbooks you have in storage. I tried to make it as comprehensive and affordable as possible, and still liftable. If you are a nervous tester who has been putting this off, I wrote this for you.

— KekeTheLPC

How to Use This Book

The idea in one sentence

NCE distractors — the wrong answer options — are almost never made-up words. They are real terms that belong to a different theory, theorist, test, ethical standard, or diagnosis. **If you can name the owner of every option, the right answer is the only one left standing.** This book calls that the Ownership Method.

The four-part loop

Chapters 2–23 are built for the same loop. Run it once per chapter, in order.

Step	What you do	Where it lives
1. Read	Learn the owners and the teaching. Notice the "What the exam rewards / punishes" box.	Chapter body
2. Drill	Cover the owner column of the swap-pair table and say each owner out loud.	Swap-Pair Drill, end of each chapter
3. Listen	Play the chapter's audio track during any drive, walk, or chore. Answer during the pauses.	Audio Companion (PassReady Prep)
4. Test	Answer the quick checks. For every option, right or wrong, name its owner before you read the rationale.	Quick Checks, end of each chapter

Then log every miss in the error log (Workbook): *"Who did I think owned it? Who actually owns it?"* The error log is your personal study guide for the final two weeks.

How each chapter is laid out

1. **2027 task map**: which NBCC 2027 work tasks the chapter covers, so you know why it's here.
2. **Ownership tables**: term → owner, with the look-alike it gets confused with.
3. **Teaching**: the concepts, written for single-answer recall and application.
4. **What the exam rewards / punishes**: the answer patterns that score and the traps that don't.
5. **Swap-pair drill**: the pairs most likely to appear as each other's distractors.
6. **Quick checks**: 10 or more items (Chapter 1 has two). Every rationale names the owner of every wrong option, not just why the key is right.
7. **Audio**: the track number(s) and target length for that chapter.

Four chapters also carry a section written for the 2027 specifications: Frameworks and Tools the 2027 Specifications Name (Chapter 5), Practice Management and Compliance (Chapter 17), Continuity of Care (Chapter 21), and Counselor Self-Awareness and Professional Development (Chapter 22). Each has its own swap-pair drill and quick checks. Chapter 23, Indirect Client Care, is new in this edition.

A note on options. The chapter quick checks keep four options, A–D. They are drills: one more distractor means one more owner to name, and that is the skill you're building. The practice exams and the real NCE use 3 options per item.

Where to start

- **Eight weeks or more:** follow the 8-week plan in the Workbook. It is weighted the way the exam is weighted and schedules the three practice exams in weeks 5, 7, and 8.

- **Four weeks or less:** read Chapters 1 and 2, then Chapters 4–6, 17, and 23. That covers the two largest domains, Provision of Counseling Interventions and Legal & Ethical Compliance, plus Indirect Client Care. Take Practice Exam 1 at the end of week 1, and let its domain scores choose your next chapters.
- **Retaking:** start with your NBCC score report. Go straight to the chapters for your two weakest domains (The NCE at a Glance maps each domain to its chapters), then take a practice exam.

Chapter 2 is the spine. If you read nothing else before a practice test, read Chapter 2 and drill its swap-pair table until you can do it without looking.

How to use the practice exams

Each practice exam has 170 items with 3 options each, built to the 2027 domain weights. Take each one in a single sitting of 3 hours, 45 minutes, at the pacing in Chapter 1. You get an optional 15-minute break at the halfway point, just as on test day. Review the first half before you start the second, then don't go back to it. Then grade it with the **Ownership Map** answer key: for each item, it gives the correct answer and its owner, each of the other options and its owner, and the pair to learn. The score-by-domain table at the end of each key tells you where your next hours should go.

A practice-exam score is a direction, not a prediction. NBCC reports a scaled score from 100–500, and a scaled score is not a percentage. No percentage here maps onto a pass or fail.

How to use the audio

The audio companion is call-and-response: the narrator gives a term, pauses, and you say the owner before the answer plays. Say it out loud. Hearing yourself answer is what moves a pair from "I've seen that" to "I know that."

Replay tracks on repeat; repetition is the point, not a sign you're behind.

Track numbers match chapter numbers (Track 10.1 goes with Chapter 10). Chapter 1 has no track. Chapter 23 and the four 2027 sections listed above have no track either. For those, read the swap-pair drill out loud and answer before you uncover the owner.

The NCE at a Glance

This edition covers the NCE given on or after July 1, 2027.

Sponsor	National Board for Certified Counselors (NBCC)
Used for	National Certified Counselor (NCC) credential and licensure in many states
Items	170 multiple-choice: 140 scored, 30 field-test items, unmarked and mixed in randomly
Options	3 per item; one best answer
Item format	Standalone single items (no case sequences)
Time	3 hours, 45 minutes (about 79 seconds per item), with an optional 15-minute break at the halfway point
Review	You can review the items in each half before you submit it. Once a half is submitted and the break begins, you can’t go back to it.
Scoring	Scaled score from 100–500; passing score 360. The passing point is set by Angoff standard setting and equated across forms. A scaled score is not a percentage.
Score report	Unofficial score report with a preliminary pass/fail and feedback by domain
Spanish forms	Spanish-translated forms are available, with Spanish counseling terms and an English pop-up for each item
Wrong-answer penalty	None. Never leave an item blank.

Domain	Scored items	Share	Where it's taught
1. Professional Development & Counselor Self-Awareness	21	15%	Ch 22 (incl. Counselor Self-Awareness and Professional Development), 17 (Part 5), 18, 3
2. Intake & Assessment	25	18%	Ch 19, 20, 6
3. Treatment Planning & Continuity of Care	21	15%	Ch 21 (incl. Continuity of Care); Ch 10–16 for presenting concerns
4. Provision of Counseling Interventions	28	20%	Ch 2, 4–9, 22
5. Indirect Client Care	17	12%	Ch 23; Ch 21 (Part 4)
6. Legal & Ethical Compliance	28	20%	Ch 17 (incl. Practice Management and Compliance), 6
Total	**140**	**100%**	

The two 20% domains, Provision of Counseling Interventions and Legal & Ethical Compliance, account for 56 of the 140 scored items: 40% of everything that's scored. The rest is spread across four domains, and none of them is small enough to skip. Those 140 scored items are mixed in among 170 total questions, and the 30 field-test items look exactly like the rest. You can't tell which ones count, so answer every question as if it does, and never skip one because you suspect it's a practice item.

Testing before July 1, 2027? You'll take the current NCE: 200 items, four options each, 160 scored, built on NBCC's 2023 content outline (NBCC, 2023). The owners and the content in this book apply to both exams. For the current exam's format, use the first edition's format chapter or nbcc.org.

Source: NBCC (2026), *National Counselor Examination (NCE): Examination Specifications, effective July 1, 2027.* Confirm current details at nbcc.org.

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Presenting Concerns 10. Mood, Anxiety, Obsessive, and Trauma Presentations 11. Substance Use and Process Addictions 12. Life Span and Development 13. Relationships and Family Life 14. Career and Occupation 15. Culture, Identity, and Meaning 16. Body and Mind

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PART ONE

The Exam and the Method

Chapter 1 — How the NCE Works and How to Take It

This chapter describes the NCE given on or after July 1, 2027.

> **Testing before July 1, 2027?** You'll take the current NCE: 200 items, four options each, 160 scored, built on NBCC's 2023 content outline (NBCC, 2023). The owners and the content in this book apply to both exams. For the current exam's format, use the first edition's format chapter or nbcc.org.

What the NCE Is

The National Counselor Examination (NCE) measures whether an entry-level counselor can recall and apply the counseling knowledge needed to practice safely. It is used for two purposes: state licensure in many states, and the National Certified Counselor (NCC) credential (NBCC, 2026).

The 2027 exam is built from a national work analysis NBCC ran in 2024–2025 with more than 14,000 counselors. That analysis produced **101 work tasks**, grouped into **six domains**. Three domains are split into two subdomains each (1A and 1B, 2A and 2B, 3A and 3B). In this book, a code like 4.10 means Domain 4, task 10, and 3A.7 means subdomain 3A, task 7. You'll see these codes in each chapter's 2027 task map.

The exam is written for the **minimally qualified candidate**: an entry-level counselor who is a graduate, or a well-advanced student, of a CACREP program or a program housed in an institutionally accredited college or university. The specifications still list the eight CACREP core

content areas you studied in graduate school. In practice, this means three things:

1. The **score** is built from the six domains.
2. The **content** inside those domains still draws on everything CACREP covers: theories, development, career, groups, assessment, research, ethics, and culture.
3. **Presenting concerns** (depression, grief, divorce, job loss, and the rest) are no longer a domain of their own. NBCC lists them, along with theoretical frameworks and tools, as **supplemental content guidance**: material you may benefit from reviewing. They show up inside the six domains, mostly as the client in the stem.

The Format

Feature	NCE
Total items	170 multiple-choice, 3 options each
Scored items	140
Field-test (unscored) items	30, mixed in randomly, same format
Time	3 hours, 45 minutes (225 minutes)
Break	an optional 15-minute break at the halfway point
Review	Review the items in each half before you submit it; once a half is submitted and the break begins, you can’t return to it
Item type	Standalone single items (no case sequences)
Language	English; Spanish-translated forms are available, with Spanish counseling terms and an English pop-up for each item

Source: NBCC (2026).

The 30 field-test items look exactly like the scored ones. You will never know which is which. This matters psychologically: when you hit an item that seems strange, poorly worded, or about something you've never heard of, there is a real chance it doesn't count. Answer it, flag it if you want, and move on. Do not let one odd item cost you three good ones.

The Six Domains

NBCC #	Domain	% of items	Scored items
1	Professional Development & Counselor Self-Awareness	15%	21
2	Intake & Assessment	18%	25
3	Treatment Planning & Continuity of Care	15%	21
4	Provision of Counseling Interventions	20%	28
5	Indirect Client Care	12%	17
6	Legal & Ethical Compliance	20%	28
	Total	**100%**	**140**

Source: NBCC (2026).

Read that table twice. **The two 20% domains, Provision of Counseling Interventions and Legal & Ethical Compliance, account for 56 of the 140 scored items: 40% of everything that's scored.** The other four domains split the rest, and none is small enough to skip. Those 140 scored items are mixed in among 170 total questions, and the 30 field-test items look exactly like the rest. You can't tell which ones count, so answer every question as if it does, and never skip one because you suspect it's a practice item.

If your study plan spends as much time on Indirect Client Care (17 items) as on Provision of Counseling Interventions (28 items), it is misallocated.

How It's Scored

There is no fixed passing percentage. NBCC sets the passing point with an **Angoff** standard-setting study: subject-matter experts estimate how a minimally qualified candidate would perform on each item. Forms differ slightly in difficulty, so NBCC uses **statistical equating** across forms. A pass on one form means the same as a pass on another.

Your raw score is the number of scored items you answer correctly. NBCC converts it to a **scaled score** from 100–500. The passing score is **360**. A scaled score is not a raw score and not a percentage, so don't try to convert it.

What this means for you:

- **Ignore any website that tells you "you need 70%" or "you need 84%."** Those numbers are not published by NBCC.
- **Every item you answer correctly counts the same.** There is no penalty for a wrong answer beyond not earning the point. **Never leave an item blank.**
- You get an unofficial score report with a preliminary pass/fail and feedback by domain. Use that feedback if you need to retake.

Pacing

You have 225 minutes for 170 items: **about 79 seconds per item.** Aim for 70, which banks about 27 minutes for review.

Checkpoint	Target time elapsed	Hard limit
Item 43	0:50	0:57
Item 85 (end of first half)	1:39	1:52
Item 128	2:29	2:49
Item 170	3:18	3:45

The halfway submit. The exam comes in two halves of 85 items. When you reach the end of the first half, go back through your flagged items **before** you submit it.

Once you submit and the break begins, you cannot return to the first half. Use half your banked review time here, not all of it at the end. Then take the break if you want it: you get an optional 15-minute break at the halfway point.

Most NCE items are short. Recall items ("Which theorist…") should take 20–30 seconds. Applied items ("What should the counselor do next…") take longer. The time you save on recall pays for the applied items.

Flagging rule: flag an item only if you can name what you'd check on a second look. "I'm torn between B and C because I'm not sure whether this is Bowen or Minuchin" is a good flag. "I don't feel good about this" is not. Pick your best answer before you flag, every time.

The Four-Step Item Routine

Use this on every item until it is automatic.

1. Read the last line first. The final sentence tells you what's being asked: a theorist, a technique, a next step, an ethical action, a definition. Knowing the question type before you read the details tells you which details matter.

2. Name the owner. Ask: *which theory, domain, or framework is this item living in?* A stem (the question part of the item) that mentions "irrational beliefs" lives in REBT. A stem about "the group's second session, members testing the leader" lives in group stage theory. Naming the owner narrows the right answer before you see the options.

3. Predict, then look. Form your answer before reading the choices. If your prediction is among the options, it's probably right. If it isn't, read the options carefully. You may have named the wrong owner.

4. Eliminate by owner. For each wrong-looking option, ask who owns it. On the NCE, distractors are usually real terms from a *different* theory, stage model, or framework. When you can name the other owner, you can cross the option out with confidence instead of guessing. The math

is on your side: with 3 options, a blind guess is about a 33% chance. Eliminate one option by owner and it becomes 50%.

Example (original item)

> A counselor asks a client, “Suppose you went to sleep tonight and, while you slept, the problem was solved. What would be different tomorrow?” This intervention is most associated with which approach?
> A. Narrative therapy B. Solution-focused brief therapy C. Reality therapy D. Gestalt therapy

Last line: which approach owns the technique. **Owner:** the miracle question belongs to solution-focused brief therapy (de Shazer & Berg). **Eliminate:** narrative owns externalizing and re-authoring; reality therapy owns WDEP and choice theory; Gestalt owns the empty chair and here-and-now awareness. **Answer: B.**

Notice that every wrong answer is a real, well-known approach a counselor might plausibly use. That’s how NCE distractors work: they are plausible because they are real.

The chapter drills keep four options, like this one, so you name one more owner per item. The real exam gives you 3.

Rules for Applied Items

When the item asks what a counselor *should do*, these priorities decide most of them:

1. **Safety first.** Any sign of risk to self or others comes before every other goal.
2. **Relationship before technique.** Early in counseling, the alliance-building response (empathy, reflection) beats the clever intervention.
3. **Assess before you intervene.** Don’t treat what you haven’t assessed.

4. **Follow the code, not your instincts.** When ethics are involved, the ACA Code of Ethics decides, and consultation is almost always part of the right answer.
5. **Stay in scope.** When a need falls outside your competence, refer or consult rather than improvise.
6. **Client autonomy.** Between two otherwise equal options, choose the one that involves the client in the decision.

Allocating Your Study Time

Study hours should follow scored items. For an 8-week plan of about 10 hours per week:

Domain	Scored items	Hours (of 80)
Professional Development & Counselor Self-Awareness	21	12
Intake & Assessment	25	14
Treatment Planning & Continuity of Care	21	12
Provision of Counseling Interventions	28	16
Indirect Client Care	17	10
Legal & Ethical Compliance	28	16

The Theory Ownership work in Chapters 2 and 3 is not a separate block. It runs through Provision of Counseling Interventions (theory-based interventions), the presenting concerns (developmental and career theorists), and Professional Development & Counselor Self-Awareness (research and evidence owners). Do 15 minutes of ownership drills every day, and put the audio companion on during any drive, walk, or chore. That's where the repetition comes from.

Test Day

- Arrive early. Bring the ID your registration requires.
- Use the scratch materials the testing center provides to write the pacing checkpoints before you start.

- If you blank on an item, use step 4 anyway. Eliminating one option by owner turns a 33% guess into a 50% guess.
- At item 85, stop. Review your flags in the first half and check that no item is blank. Then submit the half and take the break if you want it. You can't come back.
- At the end, check that no item in the second half is blank before you submit.

Quick Checks

1. Approximately how many NCE items contribute to your score? A. 100 B. 140 C. 150 D. 170

2. You encounter an item on a topic you've never studied. Best response? A. Skip it and leave it blank to save time B. Spend extra time, since it is probably scored C. Choose your best answer, flag if useful, and move on D. Answer randomly without reading

Answers: 1-B. Of 170 items, 140 are scored; 30 are field-test items. 2-C. The item may be unscored, a blank earns nothing, and time spent here costs you items you know.

References

National Board for Certified Counselors. (2023). *Content outline: The National Counselor Examination (NCE).* https://www.nbcc.org/exams/nce

National Board for Certified Counselors. (2026). *National Counselor Examination (NCE): Examination specifications, effective July 1, 2027.* https://nbcc.org/assets/exam/NCE_exam_spec_2027.pdf

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Chapter 2 — The Theory Ownership Matrix

2027 task map: 4.6 Implement empirically supported interventions · 4.11 Provide psychoeducation · 4.7 Implement interventions aligned with service modality · CACREP: Counseling and Helping Relationships; Human Growth and Development (family systems content)

Why This Chapter Comes First

Most NCE theory items are not hard because the content is obscure. They are hard because **every option is real.** A question about Gestalt will offer you other genuine techniques, each owned by a different approach. If you know a technique only vaguely ("that's one of the experiential ones"), every option looks possible. If you know exactly who owns each one, the wrong ones disappear.

So this chapter teaches theory the way the exam tests it: as **ownership.** For every major approach you'll learn six things:

Field	The question it answers
Owner	Who founded or is most associated with it?
Key terms	What vocabulary signals this theory in a stem?
Signature techniques	What does the counselor actually do?
View of the problem	Where does distress come from?
Goal	What counts as success?
Counselor role	Expert, collaborator, coach, observer, director?

You don't need to memorize the tables in one pass. Read through once, then use the three-pass drill at the end of the chapter.

Family 1 — Psychodynamic

The psychodynamic family shares one idea: **what you can't see is driving what you do.** Early experience and unconscious material shape present behavior, and insight is the route to change. Stems signal this family with words like *unconscious, early childhood, insight, defense, transference,* and *interpretation.*

Theory	Owner	Key terms	Signature techniques	View of problem
Psychoanalysis	Sigmund Freud	Id, ego, superego; psychosexual stages (oral, anal, phallic, latency, genital); unconscious; anxiety (reality, neurotic, moral); defense mechanisms	Free association; dream analysis (manifest vs. latent content); interpretation; analysis of transference and resistance	Unresolve unconsci conflict, c from earl developm
Individual Psychology	Alfred Adler	Social interest (*Gemeinschaftsgefühl*); inferiority feelings and striving for superiority; lifestyle; birth order; family constellation; fictional finalism; basic mistakes	Early recollections; lifestyle assessment; encouragement; "acting as if"; "spitting in the client's soup"; push-button technique; catching oneself; the Question ("What would be different if you were well?")	Discourag and mista beliefs ab how to be
Analytical Psychology	Carl Jung	Collective unconscious; archetypes; persona; shadow; anima/ animus; self; introversion/ extraversion; individuation	Dream and symbol work; active imagination	One-sided of person unintegra parts of t psyche

Theory	Owner	Key terms	Signature techniques	View of problem
Object Relations / Attachment	Melanie Klein, Donald Winnicott, Margaret Mahler, John Bowlby	Internal objects; splitting; "good-enough mother" and holding environment (Winnicott); separation–individuation (Mahler); secure base, internal working models (Bowlby)	Exploring relational patterns as they appear in the therapeutic relationship	Early rela patterns internaliz repeated

Defense mechanisms belong to Freud (later systematized by Anna Freud). If a stem asks the counselor to help a client recognize repression, projection, or displacement, you are in the psychodynamic family, and the answer involves insight, not skills training.

Defense	What it looks like
Repression	Pushing threatening material out of awareness
Denial	Refusing to acknowledge an external reality
Projection	Attributing one's own unacceptable feelings to someone else
Displacement	Redirecting feelings to a safer target
Reaction formation	Expressing the opposite of what one feels
Rationalization	Creating a logical-sounding excuse
Regression	Returning to an earlier developmental level
Sublimation	Channeling impulses into acceptable outlets (considered mature)
Intellectualization	Retreating into abstract thinking to avoid feeling

Family 2 — Humanistic and Existential

This family shares a belief that **people have an inherent drive toward growth and meaning,** and that distress comes from being blocked from it. The relationship and present awareness do the healing. Signal words: *here and now, self-actualization, authenticity, meaning, freedom, responsibility, awareness.*

Theory	Owner	Key terms	Signature techniques	View of problem	Goal
Person-Centered	Carl Rogers	Actualizing tendency; conditions of worth; incongruence between self-concept and experience; core conditions (congruence/genuineness, unconditional positive regard, accurate empathic understanding)	Reflection of feeling; empathic responding; the core conditions themselves *are* the technique; nondirective stance	Incongruence created by conditions of worth	Cong beco fully funct pers
Gestalt	Fritz Perls (with Laura Perls, Paul Goodman)	Here and now; awareness; figure/ground; unfinished business; top dog/underdog; contact-boundary disturbances (introjection, projection, retroflection, confluence, deflection); "what" and "how" over "why"	Empty chair; two-chair dialogue; exaggeration; staying with the feeling; "I take responsibility for…"; making the rounds (group); dream work as parts of self	Blocked awareness; unfinished business; fragmented self	Awar and integ owni expe

Theory	Owner	Key terms	Signature techniques	View of problem	Goal
Existential	Viktor Frankl; Rollo May; Irvin Yalom	Meaning; freedom and responsibility; existential anxiety (normal) vs. neurotic anxiety; four givens: death, freedom, isolation, meaninglessness (Yalom); logotherapy (Frankl)	Paradoxical intention and dereflection (Frankl); Socratic dialogue about meaning; confronting the givens	Avoiding or being overwhelmed by the givens of existence	Auth livin findi mea acce resp
Self-actualization (needs theory)	Abraham Maslow	Hierarchy of needs: physiological → safety → love/ belonging → esteem → self-actualization; peak experiences	(Theory, not a therapy)	Unmet lower needs block growth	Self- actu

Watch the trap: Rogers's *actualizing tendency* and Maslow's *self-actualization* have different owners. Rogers wrote a therapy; Maslow wrote a motivational theory.

Family 3 — Behavioral and Cognitive-Behavioral

This family focuses on **what can be observed, measured, and changed.** Behavior is learned; thoughts drive feelings; change happens through practice, homework, and testing beliefs. Signal words: *reinforcement, homework, irrational, automatic thoughts, distortions, measurable, skills.*

Behavioral

Theory	Owner	Key terms	Signature techniques	View of problem
Classical conditioning	Ivan Pavlov; John B. Watson ("Little Albert")	Unconditioned/ conditioned stimulus and response; generalization; extinction	Exposure; counterconditioning	Learned associations
Systematic desensitization	Joseph Wolpe	Reciprocal inhibition; anxiety hierarchy; SUDS (subjective units of distress)	Relaxation training + graded imagined exposure up the hierarchy	Conditioned anxiety
Operant conditioning	B. F. Skinner	Positive/ negative reinforcement; positive/ negative punishment; extinction; shaping; schedules of reinforcement	Token economy; shaping by successive approximations; contingency management	Behavior maintained by its consequences
Social learning / social cognitive	Albert Bandura	Modeling; observational learning; self-efficacy; reciprocal determinism	Modeling; behavioral rehearsal; mastery experiences	Learned by observation; low self-efficacy

Other behavioral terms with owners: **Premack principle** (David Premack: a preferred activity reinforces a less-preferred one), **implosive therapy** (Thomas Stampfl: imagined flooding), **token economy** (Ayllon & Azrin).

Negative reinforcement is not punishment. Negative reinforcement *removes* something unpleasant to *increase* a behavior (seatbelt chime stops when you buckle). Punishment *decreases* a behavior. This distinction is easy to test and easy to miss.

Cognitive and Cognitive-Behavioral

Theory	Owner	Key terms	Signature techniques	View of problem
Rational Emotive Behavior Therapy (REBT)	Albert Ellis	A-B-C(-D-E): Activating event, Belief, Consequence, Disputing, Effect; irrational beliefs; "musts," "shoulds"; "musturbation"; awfulizing; low frustration tolerance; unconditional self-acceptance	Disputing irrational beliefs (often forcefully); shame-attacking exercises; rational-emotive imagery; homework	Irrational beliefs, not events, create disturbance
Cognitive Therapy (CT)	Aaron Beck	Automatic thoughts; cognitive distortions; schemas/core beliefs; cognitive triad of depression (negative view of self, world, future); collaborative empiricism	Socratic questioning; thought records; behavioral experiments; testing evidence; activity scheduling	Distorted information processing
Cognitive Behavior Modification	Donald Meichenbaum	Self-talk; self-instructional training; stress inoculation training (conceptualization → skill acquisition → application)	Self-instructional training; stress inoculation	Maladaptive internal dialogue

Theory	Owner	Key terms	Signature techniques	View of problem
Reality Therapy / Choice Theory	William Glasser (WDEP: Robert Wubbolding)	Choice theory; five basic needs (survival, love/ belonging, power, freedom, fun); quality world; total behavior (acting, thinking, feeling, physiology); no excuses; no punishment; responsibility	WDEP: Wants, Doing, Evaluation, Planning; "Is what you're doing getting you what you want?"	Unsatisfying relationships and ineffective choices to meet needs
Dialectical Behavior Therapy (DBT)	Marsha Linehan	Dialectics (acceptance *and* change); biosocial theory; validation; wise mind	Skills modules: mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness; diary cards; chain analysis	Emotional vulnerability + invalidating environment
Acceptance and Commitment Therapy (ACT)	Steven Hayes	Psychological flexibility; cognitive fusion/ defusion; experiential avoidance; values; committed action; self-as-context	Defusion exercises; values clarification; mindfulness; willingness	Fusion with thoughts and avoidance of experience
Multimodal Therapy	Arnold Lazarus	BASIC I.D. (Behavior, Affect, Sensation, Imagery, Cognition, Interpersonal, Drugs/biology); technical eclecticism	Assessment across all seven modalities; bridging; tracking	Problems across multiple modalities

Change-Process Models (often confused with theories)

Model	Owner	Key terms	What it's for
Motivational Interviewing (MI)	William Miller & Stephen Rollnick	Spirit of MI (partnership, acceptance, compassion, evocation); OARS (Open questions, Affirmations, Reflections, Summaries); change talk; sustain talk; developing discrepancy; rolling with resistance (called "responding to sustain talk and discord" in the 2013 edition)	A *method* for resolving ambivalence
Transtheoretical Model (Stages of Change)	James Prochaska & Carlo DiClemente	Precontemplation, contemplation, preparation, action, maintenance (and relapse/ termination)	A *description* of readiness, not a therapy
Transactional Analysis (TA)	Eric Berne	Ego states (Parent, Adult, Child); transactions (complementary, crossed, ulterior); strokes; games; life scripts; life positions ("I'm OK, you're OK")	Analyzing communication and life scripts

Watch the trap: "Contemplation" is a *stage of change* (Prochaska & DiClemente). It is never a *group stage*. It makes a tempting distractor in group-stage items.

Family 4 — Postmodern and Constructivist

This family assumes **reality is constructed through language and stories,** and that clients are the experts on their own lives. Focus is on strengths, solutions, and alternative narratives, not on pathology or history. Signal words: *exceptions, solutions, strengths, story, externalize, preferred future, client as expert.*

Theory	Owner	Key terms	Signature techniques	View of problem	Goal	Coun role
Solution-Focused Brief Therapy (SFBT)	Steve de Shazer & Insoo Kim Berg	Exceptions; preferred future; small changes; client as expert; "if it works, do more of it"	Miracle question; exception questions; scaling questions (0–10); coping questions; compliments; first-session formula task	Clients get stuck doing more of what doesn't work	Construct solutions; do more of what works	Collab curiou future focuse
Narrative Therapy	Michael White & David Epston	Problem-saturated story; dominant discourse; unique outcomes; thin vs. thick descriptions; "the person is not the problem; the problem is the problem"	Externalizing the problem; mapping the problem's influence; identifying unique outcomes; re-authoring; therapeutic letters; definitional ceremonies/ outsider witnesses	Oppressive dominant stories	Re-author a preferred story	Co-au curiou expert

Watch the trap: Both use questions, and both are future/strength-oriented. Tell them apart by the signature move. *Exceptions and miracles* belong to SFBT. *Externalizing and unique outcomes* belong to narrative. "Unique outcome" and "exception" mean similar things; the word in the stem decides the owner.

Family 5 — Family Systems

In this family, **the client is the system,** not the individual. Symptoms serve a function in the family, and change in one member changes the whole. Signal words: *system, homeostasis, identified patient (IP), boundaries, triangles, hierarchy, circular causality, family of origin.*

Theory	Owner	Key terms	Signature techniques	View of problem
Bowenian / Multigenerational	Murray Bowen	Differentiation of self; triangles/ triangulation; emotional cutoff; multigenerational transmission process; nuclear family emotional process; family projection process; sibling position; societal regression	Genogram; detriangulation; coaching the client to differentiate in the family of origin; calm, process questions	Low differenti anxiety p across generatio
Structural	Salvador Minuchin	Structure; subsystems (spousal, parental, sibling); boundaries (rigid, clear, diffuse); enmeshment; disengagement; hierarchy; coalitions	Joining; accommodating; enactment; family mapping; boundary making; unbalancing; reframing	Dysfuncti structure unclear boundari inverted hierarchy
Strategic	Jay Haley; Cloé Madanes	Hierarchy; power; symptom as communication	Directives (straightforward and paradoxical); prescribing the symptom; ordeal therapy (Haley); pretend techniques (Madanes)	Symptom maintain dysfuncti hierarchy

Theory	Owner	Key terms	Signature techniques	View of problem
MRI / Brief Strategic	Mental Research Institute: Paul Watzlawick, John Weakland, Don Jackson, Richard Fisch (from Gregory Bateson's work)	Double bind (Bateson); homeostasis; first-order vs. second-order change; attempted solutions become the problem; "you cannot not communicate"	Reframing; paradoxical interventions; interrupting attempted solutions	The famil attempte solution maintains problem
Milan Systemic	Mara Selvini Palazzoli, Luigi Boscolo, Gianfranco Cecchin, Giuliana Prata	Hypothesizing, circularity, neutrality; family games	Circular questioning; positive connotation; invariant prescription; rituals; team behind mirror	Family "g maintains symptom
Experiential / Human Validation	Virginia Satir	Self-esteem; communication stances: placater, blamer, super-reasonable (computer), irrelevant (distracter), congruent; family rules	Family sculpting; family reconstruction; role-play; "parts party"	Low self- esteem a incongru communi
Symbolic-Experiential	Carl Whitaker	Absurdity; spontaneity; "the battle for structure, the battle for initiative"	Co-therapy; playfulness; provoking with absurdity	Emotiona deadness suppresse impulses

Theory	Owner	Key terms	Signature techniques	View of problem
Emotionally Focused Therapy (EFT)	Sue Johnson (with Les Greenberg)	Attachment; negative interaction cycle (pursue–withdraw); primary vs. secondary emotions; softening	De-escalating the cycle; accessing primary emotion; enactments; restructuring bonds	Insecure attachme expressed through a negative
Gottman Method	John & Julie Gottman	Four Horsemen (criticism, contempt, defensiveness, stonewalling); Sound Relationship House; repair attempts; flooding; 5:1 ratio	Assessment; softened startup; repair; love maps; building fondness	Negative interactio patterns the friend

Watch the traps: - **Bowen vs. Minuchin:** *Triangles, differentiation, genogram, cutoff* → Bowen. *Boundaries, enmeshment, enactment, joining, subsystems* → Minuchin. - **Minuchin vs. Haley:** both care about hierarchy. *Boundaries and structure* → Minuchin. *Power, directives, ordeals* → Haley. - **Haley vs. Frankl:** both use paradox. *Prescribing the symptom in a family* → strategic. *Paradoxical intention with an individual* → Frankl (existential/logotherapy). - **Satir vs. Bowen:** both look at family of origin. *Communication stances, sculpting, self-esteem* → Satir. *Differentiation and multigenerational transmission* → Bowen.

Family 6 — Feminist and Multicultural

Theory	Owner	Key terms	Signature techniques	Goal	C ro
Feminist Therapy	Jean Baker Miller; Carolyn Zerbe Enns; Laura Brown; others (collective founding)	"The personal is political"; egalitarian relationship; gender-role socialization; power; social change; relational-cultural theory (Jean Baker Miller)	Gender-role analysis; power analysis; assertiveness training; reframing symptoms as responses to oppression; bibliotherapy; social action	Empowerment; social as well as individual change	Eg pa de co
Multicultural Counseling Competencies	Derald Wing Sue and colleagues; Multicultural and Social Justice Counseling Competencies (MSJCC; Ratts et al.)	Counselor self-awareness, knowledge of client worldview, culturally appropriate skills; action; cultural humility; privilege and oppression	Broaching; culturally adapted interventions; advocacy	Culturally responsive, just practice	Av hu ac

The Swap-Pair Table

These are the pairs most likely to appear as each other's distractors. Learn them as pairs.

If you see…	It belongs to…	Not to… (because)
Disputing irrational beliefs; "musts"	Ellis (REBT)	Beck: Beck *tests* automatic thoughts collaboratively; Ellis *disputes* beliefs directly

If you see…	It belongs to…	Not to… (because)
Cognitive triad; collaborative empiricism; Socratic questioning	Beck (CT)	Ellis
Social interest; birth order; lifestyle	Adler	Freud: Freud is drives and psychosexual stages
Archetypes; collective unconscious; persona	Jung	Adler or Freud
Empty chair; unfinished business; "what" and "how"	Perls (Gestalt)	Rogers: Rogers is nondirective and reflective
Unconditional positive regard; nondirective	Rogers	Gestalt
Meaning; logotherapy; paradoxical intention	Frankl	Haley's paradoxical *directives*
Four givens (death, freedom, isolation, meaninglessness)	Yalom	Frankl, who focuses on meaning
Systematic desensitization; reciprocal inhibition; SUDS	Wolpe	Skinner: Skinner is operant (consequences)
Shaping; token economy; schedules of reinforcement	Skinner / operant	Wolpe
Modeling; self-efficacy	Bandura	Skinner
WDEP; quality world; five needs	Glasser / Wubbolding	Ellis
Stress inoculation; self-instructional training	Meichenbaum	Beck
BASIC I.D.	Lazarus	Beck
Ego states (Parent, Adult, Child); strokes; games	Berne (TA)	Freud's id/ego/superego

If you see…	It belongs to…	Not to… (because)
OARS; change talk; rolling with resistance	Miller & Rollnick (MI)	Prochaska & DiClemente
Precontemplation → maintenance	Prochaska & DiClemente	MI or group stages
Miracle question; exceptions; scaling	de Shazer & Berg (SFBT)	Narrative
Externalizing; unique outcomes; re-authoring	White & Epston (narrative)	SFBT
Differentiation; triangles; genogram; cutoff	Bowen	Minuchin
Boundaries; enmeshment; enactment; joining	Minuchin	Bowen or Haley
Directives; ordeal; prescribing the symptom	Haley (strategic)	Minuchin
Circular questioning; positive connotation; invariant prescription	Milan	MRI
Double bind	Bateson / MRI	Milan
Placater, blamer, super-reasonable, irrelevant; sculpting	Satir	Bowen
Four Horsemen	Gottman	Sue Johnson (EFT)
Pursue–withdraw cycle; softening; attachment in couples	Sue Johnson (EFT)	Gottman
Good-enough mother; holding environment	Winnicott	Bowlby
Separation–individuation	Mahler	Erikson
Hierarchy of needs	Maslow	Rogers

The Three-Pass Drill

Repeat these passes until each takes under five minutes per family.

Pass 1 — Term → Owner. Cover the owner column of the swap-pair table. Read each term and say the owner out loud. Mark every miss.

Pass 2 — Owner → Terms. Name a theorist. List three key terms and one signature technique without looking. Example: “Minuchin — boundaries, enmeshment, subsystems; technique: enactment.”

Pass 3 — Distractor autopsy. After every practice item, right or wrong, name the owner of each wrong option. If you can’t, that’s your next study target. Write it in the error log: *“Option C — I thought it was Bowen. Actually Satir.”*

Pass 3 is the habit that pays off on test day. It trains you to see an owner behind every option, not one right answer.

Daily dose: 15 minutes of drills at a desk, plus the matching audio track during any drive or walk.

What the Exam Rewards / What It Punishes

Rewards: - Matching a technique to its theory precisely - Matching the counselor’s *role* to the theory (directive vs. nondirective is a frequent discriminator) - Knowing that a theory’s goal shapes the intervention (insight vs. behavior change vs. new story)

Punishes: - Picking the option that “sounds therapeutic” rather than the one that fits the named theory - Confusing stage models (Prochaska’s stages, Tuckman’s group stages, Erikson’s stages) - Confusing paired theorists (Ellis/Beck, Bowen/Minuchin, SFBT/narrative)

Quick Checks

Every rationale names the owner of every option. That is the point.

1. A counselor asks a client to rate her anxiety from 0 to 10 and then asks what would move her one point lower. This counselor is most likely practicing: A. Cognitive therapy B. Solution-focused brief therapy C. Systematic desensitization D. Reality therapy

2. Which concept is central to Bowen family systems theory? A. Enmeshment B. Differentiation of self C. Ordeal D. Family sculpting

3. A client says, "I absolutely must get this promotion, or I'm a failure." A counselor using REBT would most likely: A. Reflect the client's feelings without directing B. Dispute the belief that the client *must* get the promotion C. Ask what the client is doing to get what he wants D. Explore early recollections

4. A counselor helps a family see that the parents' subsystem has a diffuse boundary with the oldest child. This counselor's orientation is: A. Structural B. Bowenian C. Strategic D. Experiential

5. "Is what you're doing getting you what you want?" belongs to: A. Ellis B. Glasser/Wubbolding C. Adler D. Meichenbaum

6. Removing a chore when a child completes homework, in order to increase homework completion, is an example of: A. Positive punishment B. Negative punishment C. Negative reinforcement D. Extinction

7. A counselor says to a client describing depression, "How has Depression been trying to convince you to stay home?" This is: A. Externalizing (narrative) B. An exception question (SFBT) C. Reframing (MRI) D. Disputing (REBT)

8. Which counselor stance is most associated with person-centered therapy? A. Directive and didactic B. Neutral

and strategic C. Nondirective, genuine, and accepting D. Confrontive and experimental

9. Sue Johnson's emotionally focused couples therapy primarily targets: A. The Four Horsemen B. The negative interaction cycle and attachment needs C. Communication stances D. Differentiation of self

10. A counselor tells a client with insomnia to try as hard as possible to stay awake. This is most consistent with: A. Frankl's paradoxical intention B. Wolpe's reciprocal inhibition C. Beck's behavioral experiment D. Minuchin's enactment

Answers and Owner Rationales

1 — B. Scaling questions belong to SFBT (de Shazer & Berg). A: Beck uses thought records and Socratic questioning. C: Wolpe uses SUDS ratings, but to build a fear hierarchy, not to ask what would make things better. D: Glasser/Wubbolding use WDEP.

2 — B. Differentiation of self is Bowen's central concept. A: enmeshment is Minuchin (structural). C: ordeal therapy is Haley (strategic). D: family sculpting is Satir (experiential).

3 — B. Disputing "musts" is Ellis's REBT. A: nondirective reflection is Rogers. C: WDEP evaluation is Glasser/Wubbolding. D: early recollections are Adler.

4 — A. Subsystems and boundaries are Minuchin's structural vocabulary. B: Bowen would speak of triangles and differentiation. C: strategic (Haley) speaks of hierarchy and power and uses directives. D: experiential (Satir) speaks of communication stances and self-esteem.

5 — B. The evaluation step of WDEP (Wubbolding's operationalization of Glasser's reality therapy). A: Ellis disputes beliefs. C: Adler's "the Question" asks what would be different if the client were well. D: Meichenbaum works on self-talk.

6 — C. Removing something unpleasant (a chore) to increase a behavior (homework) is negative reinforcement (Skinner/operant). A: positive punishment *adds* something unpleasant to *decrease* behavior. B: negative punishment *removes* something pleasant to *decrease* behavior. D: extinction withholds reinforcement so a behavior fades.

7 — A. Speaking of Depression as a separate entity acting on the client is externalizing (White & Epston). B: an exception question (SFBT) would ask about times depression was absent or weaker. C: reframing (MRI/strategic) gives behavior a new meaning. D: disputing (Ellis) challenges an irrational belief.

8 — C. Rogers's core conditions: genuineness, unconditional positive regard, empathy, delivered nondirectively. A: directive and didactic is Ellis. B: neutral and strategic fits Milan or Haley. D: confrontive and experimental is Perls.

9 — B. EFT (Johnson) de-escalates the negative cycle and restructures attachment bonds. A: the Four Horsemen are Gottman. C: communication stances are Satir. D: differentiation is Bowen.

10 — A. Asking an individual to intend the feared symptom is Frankl's paradoxical intention (logotherapy). B: reciprocal inhibition (Wolpe) pairs relaxation with anxiety stimuli. C: a behavioral experiment (Beck) tests a specific belief with evidence. D: enactment (Minuchin) has family members interact in session.

Audio Companion

This chapter's audio is split into four tracks, grouped by theory family.

Track	Covers	Target length
2.1	Psychodynamic + Humanistic/Existential	~12 min
2.2	Behavioral + Cognitive-Behavioral + change models	~10 min

Track	Covers	Target length
2.3	Postmodern + Family Systems	~10 min
2.4	Swap Pairs Rapid Fire (all families)	~8 min

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Chapter 3 — Ownership Beyond Theory

2027 task map: 2B.11 Select, administer, and/or interpret appropriate instruments/assessments that align with contextual, cultural, and developmental considerations · 4.2 Explore the impact of the client's culture, context, values, and worldview · 4.5 Foster the group therapeutic process · CACREP: Human Growth and Development, Career Development, Group Counseling, Assessment and Testing, Research and Program Evaluation · 2027 supplemental content guidance: presenting concerns

The Same Trap, Six More Times

Chapter 2 showed that NCE theory items are built from real terms owned by different theorists. The same construction runs through the rest of the CACREP recall content. A development item offers real stage names from different models. A career item offers real theorists who all wrote about choosing work. A statistics item offers real tests that all compare something.

This chapter gives you the ownership tables for six areas:

1. Human growth and development
2. Identity development models
3. Career development
4. Group work
5. Assessment and testing
6. Research and statistics

Each area ends with its swap pairs. The deeper clinical teaching for these topics comes later (Chapters 8, 12, 14, 15, 18, and 20). This chapter is the recall layer: the owners, the terms, and the order of the stages.

One rule for every stage model: learn the stages *in order*, with one anchor word each. NCE items frequently ask which stage comes next, or which stage a described person is in.

Area 1 — Human Growth and Development

The Stage Theorists

Owner	Theory	Stages / key terms (in order)	Anchor idea
Erik Erikson	Psychosocial development	1. Trust vs. mistrust (infancy) · 2. Autonomy vs. shame and doubt (toddler) · 3. Initiative vs. guilt (preschool) · 4. Industry vs. inferiority (school age) · 5. Identity vs. role confusion (adolescence) · 6. Intimacy vs. isolation (young adult) · 7. Generativity vs. stagnation (middle adult) · 8. Integrity vs. despair (late adult)	A psychosocial crisis at each stage across the *whole life span*
Sigmund Freud	Psychosexual development	Oral · anal · phallic (Oedipus/Electra) · latency · genital	Libido focused on body zones; fixation

Owner	Theory	Stages / key terms (in order)	Anchor idea
Jean Piaget	Cognitive development	1. Sensorimotor, 0–2 (object permanence) · 2. Preoperational, 2–7 (egocentrism, animism, centration, no conservation) · 3. Concrete operational, 7–11 (conservation, reversibility, classification) · 4. Formal operational, 11+ (abstract, hypothetical reasoning) · Processes: schema, assimilation, accommodation, equilibration	Children actively construct knowledge in stages
Lev Vygotsky	Sociocultural development	Zone of proximal development (ZPD); scaffolding; more knowledgeable other; private speech	Learning happens socially, just beyond current ability
Lawrence Kohlberg	Moral development	**Preconventional:** 1. Obedience and punishment · 2. Self-interest/ instrumental exchange · **Conventional:** 3. Interpersonal conformity (“good boy/good girl”) · 4. Law and order · **Postconventional:** 5. Social contract · 6. Universal ethical principles · Heinz dilemma	Moral *reasoning*, judged by justice

Owner	Theory	Stages / key terms (in order)	Anchor idea
Carol Gilligan	Moral development (ethic of care)	Orientation to individual survival → goodness as self-sacrifice → morality of nonviolence (care for self and others); *In a Different Voice*	Critique of Kohlberg; care and relationships, not only justice
Margaret Mahler	Separation–individuation	Normal autism → symbiosis → separation–individuation subphases: differentiation, practicing, rapprochement, object constancy	Infant psychologically "hatches" from caregiver
Daniel Levinson	Adult development (*The Seasons of a Man's Life*)	Early adult transition; age-30 transition; midlife transition; "the Dream"; mentor	Alternating stable and transitional periods
Robert Havighurst	Developmental tasks	Tasks at each life stage that must be mastered for later success	Mastery → happiness and success with later tasks
Arnold Gesell	Maturational theory	Development unfolds by biological timetable; developmental norms	Nature over nurture
David Elkind	Adolescent egocentrism	Imaginary audience; personal fable	Adolescents feel watched and invulnerable/ unique

Owner	Theory	Stages / key terms (in order)	Anchor idea
James Marcia	Identity statuses (built on Erikson)	Diffusion (no crisis, no commitment) · Foreclosure (commitment, no crisis) · Moratorium (crisis, no commitment) · Achievement (crisis, then commitment)	Crisis (exploration) × commitment

Attachment, Parenting, and Temperament

Owner	Key terms
John Bowlby	Attachment theory; secure base; internal working models; separation distress
Mary Ainsworth	Strange Situation; secure, anxious-avoidant, anxious-resistant (ambivalent)
Mary Main & Judith Solomon	Added the **disorganized** attachment classification
Harry Harlow	Rhesus monkeys; contact comfort beats food (wire vs. cloth mother)
Diana Baumrind	Parenting styles: authoritative (warm + firm, best outcomes), authoritarian (firm, not warm), permissive (warm, not firm)
Maccoby & Martin	Added the fourth style: neglectful/uninvolved
Thomas & Chess	Temperament: easy, difficult, slow-to-warm-up
Konrad Lorenz	Imprinting; critical period (geese)

Grief, Stress, and Loss

Owner	Key terms
Elisabeth Kübler-Ross	Five stages of dying: denial, anger, bargaining, depression, acceptance (described for terminally ill patients; not a required sequence)
J. William Worden	Four tasks of mourning: accept the reality of the loss · process the pain · adjust to a world without the deceased · find an enduring connection while embarking on a new life
Hans Selye	General Adaptation Syndrome: alarm → resistance → exhaustion; eustress vs. distress
Holmes & Rahe	Social Readjustment Rating Scale (life change units)
Nancy Schlossberg	Transition theory: the 4 S's (situation, self, support, strategies); anticipated, unanticipated, and nonevent transitions

Swap Pairs — Development

If you see…	Owner	Not…
Industry vs. inferiority	Erikson	Piaget (stages named by cognition, not crisis)
Conservation; object permanence	Piaget	Erikson or Vygotsky
Zone of proximal development; scaffolding	Vygotsky	Piaget
Law and order; social contract	Kohlberg	Gilligan
Ethic of care	Gilligan	Kohlberg
Rapprochement	Mahler	Bowlby
Strange Situation	Ainsworth	Bowlby (theory) or Harlow (monkeys)

If you see…	Owner	Not…
Foreclosure; moratorium	Marcia	Erikson (Marcia built on Erikson's identity stage)
Imaginary audience; personal fable	Elkind	Piaget
Authoritative vs. authoritarian	Baumrind	Thomas & Chess (temperament, not parenting)
Alarm, resistance, exhaustion	Selye	Kübler-Ross
Tasks of mourning	Worden	Kübler-Ross (stages of dying)
4 S's of transition	Schlossberg	Super (career stages)

Watch the age trap. Erikson's stage 4 (industry vs. inferiority) and Piaget's concrete operational stage cover about the same school-age years. If the stem describes a child comparing her work to classmates' and feeling incompetent, that's Erikson. If it describes a child who now understands that a tall glass and a wide glass hold the same water, that's Piaget.

Area 2 — Identity Development Models

These models describe how people come to understand a cultural, racial, or sexual identity. They appear as presenting concerns in the 2027 supplemental content guidance and in the cultural tasks of Provision of Counseling Interventions (4.2, 4.8).

Owner	Model	Stages (in order)
William Cross	Nigrescence (Black racial identity)	Pre-encounter → encounter → immersion–emersion → internalization → internalization–commitment

Owner	Model	Stages (in order)
Janet Helms	White racial identity	Contact → disintegration → reintegration → pseudo-independence → immersion/ emersion → autonomy
Atkinson, Morten, & Sue	Racial/Cultural Identity Development (R/CID)	Conformity → dissonance → resistance and immersion → introspection → integrative awareness
Vivienne Cass	Homosexual identity formation	Identity confusion → comparison → tolerance → acceptance → pride → synthesis
John Berry	Acculturation strategies	Integration (keep heritage + join host) · assimilation (drop heritage, join host) · separation (keep heritage, avoid host) · marginalization (neither)

Swap Pairs — Identity

If you see…	Owner	Not…
Encounter; immersion–emersion	Cross	Helms
Contact; disintegration; reintegration	Helms	Cross
Conformity → integrative awareness	Atkinson, Morten, & Sue	Cross
Identity confusion → synthesis	Cass	Marcia (identity statuses, not sexual identity)
Integration vs. assimilation	Berry	Piaget’s assimilation (a cognitive process)

Watch the word trap: “assimilation” belongs to *Piaget* when it means fitting new information into an existing schema, and to *Berry* when it means adopting the host culture. The stem’s context decides.

Area 3 — Career Development

Owner	Theory	Key terms	Anchor idea
Frank Parsons	Trait-factor	*Choosing a Vocation* (1909); know yourself, know the world of work, use “true reasoning” to match them	Father of vocational guidance
E. G. Williamson	Trait-factor (Minnesota point of view)	Directive counseling; analysis, synthesis, diagnosis, prognosis, counseling, follow-up	Systematized Parsons’s matching
John Holland	Theory of vocational personalities and work environments	RIASEC: Realistic, Investigative, Artistic, Social, Enterprising, Conventional; hexagon; congruence, consistency, differentiation, identity; Self-Directed Search (SDS)	Match personality type to environment type

Owner	Theory	Key terms	Anchor idea
Donald Super	Life-span, life-space	Vocational self-concept; Life-Career Rainbow; life roles; stages: growth (0–14), exploration (15–24), establishment (25–44), maintenance (45–64), disengagement (65+); career maturity; recycling	Career is implementing self-concept across a lifetime
Ginzberg, Ginsburg, Axelrad, & Herma	Developmental	Fantasy (to ~11) → tentative (11–17) → realistic (17+: exploration, crystallization, specification); compromise	Career choice is a developmental process
Anne Roe	Personality/ needs-based	Early parent–child relationships shape orientation toward or away from people; fields and levels of occupations; influenced by Maslow	Childhood needs → occupational choice
John Krumboltz	Social learning theory of career decision making; happenstance learning theory	Genetic endowment, environmental conditions, learning experiences, task-approach skills; planned happenstance	Career choice is learned; unplanned events are opportunities

Owner	Theory	Key terms	Anchor idea
Linda Gottfredson	Circumscription and compromise	Stages: orientation to size and power → sex roles → social valuation → internal, unique self; zone of acceptable alternatives	Children rule out options early by gender and prestige
Tiedeman & O'Hara	Decision-making	Anticipation (exploration, crystallization, choice, clarification) and implementation (induction, reformation, integration)	Career identity forms through repeated decisions
Lent, Brown, & Hackett	Social Cognitive Career Theory (SCCT)	Self-efficacy, outcome expectations, personal goals (built on Bandura)	Beliefs about ability shape interests and choices
Mark Savickas	Career construction theory; life design	Career adaptability: concern, control, curiosity, confidence; career stories	Clients construct careers through narrative
Dawis & Lofquist	Theory of Work Adjustment	Satisfaction (worker) and satisfactoriness (employer); correspondence	Fit between worker and work environment over time
Peterson, Sampson, & Reardon	Cognitive Information Processing (CIP)	Pyramid of information processing; CASVE cycle (Communication, Analysis, Synthesis, Valuing, Execution)	Career decisions as problem-solving

Owner	Theory	Key terms	Anchor idea
Robert Hoppock	Job satisfaction	Needs-based theory of occupational choice and satisfaction	Job satisfaction reflects how well needs are met

Career instruments and resources: Self-Directed Search (Holland), Strong Interest Inventory (Holland codes), Kuder Occupational Interest Survey, Career Maturity Inventory (Crites), Myers-Briggs Type Indicator (based on Jung's types), O*NET (Department of Labor database that replaced the *Dictionary of Occupational Titles*), *Occupational Outlook Handbook*.

Swap Pairs — Career

If you see…	Owner	Not…
RIASEC; hexagon; congruence	Holland	Super
Life-Career Rainbow; self-concept; maintenance stage	Super	Holland or Ginzberg
Fantasy, tentative, realistic	Ginzberg et al.	Super
Planned happenstance; learning experiences	Krumboltz	Savickas
Circumscription and compromise	Gottfredson	Ginzberg (who also used "compromise")
Parent–child relationship shapes career	Roe	Super
Self-efficacy + outcome expectations in career	Lent, Brown, & Hackett (SCCT)	Krumboltz
Career adaptability; career stories	Savickas	Super (who used "career maturity")
CASVE cycle	Peterson, Sampson, & Reardon	Tiedeman & O'Hara

If you see…	Owner	Not…
Know yourself + know the work world + match	Parsons	Williamson (who systematized it)

Area 4 — Group Work

Stage Models

Owner	Stages (in order)	Anchor
Bruce Tuckman (with Mary Ann Jensen, 1977)	Forming → storming → norming → performing → adjourning	Rhyming stages; adjourning added in 1977
Gerald Corey	Pregroup (formation) → initial (orientation, exploration, trust) → transition (anxiety, resistance, conflict, struggle for control) → working (cohesion, productivity, genuine self-disclosure) → final (consolidation, termination) → postgroup (follow-up)	The stage model most counseling texts use

Where the NCE tests this: behaviors matched to stages. Members testing the leader, conflict, and defensiveness: *transition* (Corey) or *storming* (Tuckman). Members expressing genuine thoughts and feelings, taking risks, giving direct feedback: *working* (Corey) or *norming/performing* (Tuckman). Members tentative, polite, and unsure of norms: *initial* or *forming*.

Other Group Owners

Owner	Key terms
Irvin Yalom	11 therapeutic factors: instillation of hope, universality, imparting information, altruism, corrective recapitulation of the primary family group, development of socializing techniques, imitative behavior, interpersonal learning, group cohesiveness, catharsis, existential factors; here-and-now focus; the group as a social microcosm
Kurt Lewin	Field theory; group dynamics; T-groups (National Training Laboratories); leadership styles with Lippitt & White: autocratic, democratic, laissez-faire
J. L. Moreno	Psychodrama (protagonist, director, auxiliary egos, audience, stage); doubling; role reversal; mirror technique; sociometry; credited with popularizing the term “group psychotherapy”
Joseph Pratt	First recognized therapeutic group (1905, tuberculosis patients; “class method”)
Wilfred Bion	Work group vs. basic assumption group: dependency, fight–flight, pairing
S. R. Slavson	Activity group therapy (children)
Luft & Ingham	Johari Window: open, blind, hidden, unknown
ASGW (Association for Specialists in Group Work)	Four group types: task/work, psychoeducational, counseling, psychotherapy; Best Practice Guidelines

Swap Pairs — Group

If you see…	Owner	Not…
Storming; norming	Tuckman	Corey

If you see…	Owner	Not…
Transition stage; working stage	Corey	Tuckman
Contemplation	Prochaska & DiClemente (stages of change)	Corey's group stages (contemplation is never a group stage)
Universality; instillation of hope	Yalom	Lewin
Autocratic, democratic, laissez-faire	Lewin, Lippitt, & White	Yalom
Protagonist; auxiliary ego; role reversal	Moreno (psychodrama)	Perls (Gestalt also uses dramatic techniques, but these terms are Moreno's)
Fight–flight; pairing	Bion	Tuckman
Blind and hidden areas	Luft & Ingham (Johari Window)	Yalom

Area 5 — Assessment and Testing

Intelligence Owners

Owner	Contribution
Francis Galton	Individual differences; early mental measurement; often called the father of psychometrics
Alfred Binet (with Théodore Simon)	First practical intelligence test (1905); mental age
Lewis Terman	Stanford-Binet; popularized the ratio IQ (mental age ÷ chronological age × 100)
David Wechsler	WAIS, WISC, WPPSI; deviation IQ (mean 100, SD 15)
Charles Spearman	*g* (general intelligence); two-factor theory; Spearman–Brown formula; Spearman rho
Louis Thurstone	Primary mental abilities; Thurstone attitude scale

Owner	Contribution
Raymond Cattell	Fluid vs. crystallized intelligence (with John Horn); 16PF personality inventory
Howard Gardner	Multiple intelligences
Robert Sternberg	Triarchic theory: analytical, creative, practical
J. P. Guilford	Structure of intellect; convergent vs. divergent thinking

Personality and Projective Owners

Instrument	Owner
MMPI	Hathaway & McKinley
Myers-Briggs Type Indicator	Isabel Briggs Myers & Katharine Cook Briggs (based on Jung)
16PF	Raymond Cattell
Rorschach Inkblot Test	Hermann Rorschach
Thematic Apperception Test (TAT)	Henry Murray (with Christiana Morgan); needs and press
Beck Depression Inventory	Aaron Beck
Likert scale	Rensis Likert
Semantic differential	Charles Osgood
Guttman scale	Louis Guttman
Social distance scale	Emory Bogardus

Reliability and Validity (concepts, not owners, but swapped constantly)

Term	Asks	Method / example
Test-retest reliability	Is it stable over time?	Same test, same people, two times
Alternate (parallel) forms	Are two versions equivalent?	Two forms, same people
Split-half reliability	Are the two halves consistent?	Correlate halves; correct with Spearman–Brown

Term	Asks	Method / example
Internal consistency	Do the items hang together?	Cronbach's alpha; KR-20 for right/ wrong items
Interrater reliability	Do scorers agree?	Two raters, same responses
Content validity	Does it sample the whole domain?	Expert review against the blueprint
Criterion validity: concurrent	Does it match a current criterion?	Correlate with an established measure now
Criterion validity: predictive	Does it predict a future criterion?	Correlate with later performance
Construct validity	Does it measure the theoretical trait?	Convergent (correlates with similar) and discriminant (doesn't correlate with unrelated); multitrait–multimethod matrix (Campbell & Fiske)
Face validity	Does it *look* like it measures the trait?	Not a technical form of validity

Rule: a test can be reliable without being valid, but it cannot be valid without being reliable. Reliability is necessary but not sufficient.

Scores

Score	Mean	Standard deviation
z-score	0	1
T-score	50	10
Deviation IQ (Wechsler)	100	15
Stanine	5	about 2 (range 1–9)

Normal curve: about 68% of scores fall within ±1 SD, 95% within ±2 SD, 99.7% within ±3 SD. **Standard error of measurement (SEM)** builds a confidence band around one person's score; higher reliability means a smaller SEM.

Norm-referenced compares a person to others. **Criterion-referenced** compares a person to a fixed standard (pass the driving test). **Ipsative** compares a person to themselves (strengths relative to the person's other traits).

Swap Pairs — Assessment

If you see…	Owner / answer	Not…
Deviation IQ	Wechsler	Terman (ratio IQ)
Ratio IQ (MA/CA × 100)	Terman / Stanford-Binet	Wechsler
g factor	Spearman	Thurstone (primary mental abilities)
Fluid vs. crystallized	Cattell (and Horn)	Gardner
Needs and press	Murray (TAT)	Maslow
Items hang together	Internal consistency	Test-retest
Predicts later performance	Predictive validity	Concurrent validity
Correlates with similar measures	Convergent validity	Discriminant validity
Mean 50, SD 10	T-score	Stanine or z

Area 6 — Research and Statistics

Scales of Measurement (S. S. Stevens) — learn as NOIR

Scale	Property	Example
Nominal	Categories only	Diagnosis, gender, jersey numbers
Ordinal	Rank order	Class rank, Likert ratings (usually treated as ordinal)
Interval	Equal intervals, no true zero	Temperature in °F, IQ scores
Ratio	Equal intervals and a true zero	Weight, age, number of sessions

Which Test Owns Which Question

Question	Test
Do two group means differ?	t-test (William Gosset, “Student”)
Do three or more group means differ?	ANOVA (Ronald Fisher; F statistic)
Do group means differ after controlling for a covariate?	ANCOVA
Do groups differ on two or more dependent variables at once?	MANOVA
Do observed frequencies differ from expected (nominal data)?	Chi-square (Karl Pearson)
How strongly do two interval/ratio variables relate?	Pearson r
How strongly do two ranked variables relate?	Spearman rho
Nonparametric version of the two-group t-test	Mann–Whitney U
Nonparametric version of one-way ANOVA	Kruskal–Wallis

Errors and Significance

Term	Meaning
Type I error (alpha)	Rejecting a true null hypothesis: a false positive. “Finding” an effect that isn’t there
Type II error (beta)	Failing to reject a false null hypothesis: a false negative. Missing a real effect
Power	$1 - \beta$: the probability of detecting a real effect. Increased by larger samples
Effect size	How large the difference is (e.g., Cohen’s d), independent of significance
Skew	Positive skew: tail to the right; mean > median > mode. Negative skew: tail to the left; mean < median < mode

Design Owners

Owner	Contribution
Campbell & Stanley	Threats to internal validity: history, maturation, testing, instrumentation, statistical regression, selection, experimental mortality (attrition), selection–maturation interaction; experimental vs. quasi-experimental designs
Richard Solomon	Solomon four-group design (controls for pretest sensitization)
Elton Mayo (Western Electric studies)	Hawthorne effect: behavior changes because participants know they're being studied
Robert Rosenthal	Experimenter expectancy effect (Pygmalion/Rosenthal effect)
Glaser & Strauss	Grounded theory (qualitative)
Lincoln & Guba	Qualitative trustworthiness: credibility, transferability, dependability, confirmability

Design anchors: a *true experiment* requires random assignment. A *quasi-experiment* lacks it. *Single-subject* designs use baselines (A) and interventions (B): AB, ABA, ABAB. *Formative* evaluation improves a program while it runs; *summative* evaluation judges it at the end.

Swap Pairs — Research

If you see…	Answer	Not…
Two groups, compare means	t-test	ANOVA
Nominal frequencies	Chi-square	Pearson r
Rejecting a true null	Type I	Type II
Random assignment present	True experiment	Quasi-experiment
Participants improve because they're observed	Hawthorne effect	Placebo effect

If you see…	Answer	Not…
Extreme scorers drift toward the mean on retest	Statistical regression	Maturation
Participants drop out unevenly	Mortality/attrition	Selection
Improve a program in progress	Formative evaluation	Summative evaluation

The Master "Who Owns It?" Drill

At the end of each week, run this mixed list from Chapters 2 and 3. Cover the right column and answer out loud.

Term	Owner
Social interest	Adler
Zone of proximal development	Vygotsky
RIASEC	Holland
Storming	Tuckman
Universality	Yalom
Differentiation of self	Bowen
Deviation IQ	Wechsler
Ethic of care	Gilligan
Planned happenstance	Krumboltz
Psychodrama	Moreno
Generativity vs. stagnation	Erikson
Life-Career Rainbow	Super
Pre-encounter	Cross
Disorganized attachment	Main & Solomon
Circumscription and compromise	Gottfredson
Threats to internal validity	Campbell & Stanley
Tasks of mourning	Worden
Fight–flight (group)	Bion
Fluid intelligence	Cattell

Term	Owner
Integration (acculturation)	Berry

What the Exam Rewards / What It Punishes

Rewards: - Stage order and the anchor behavior for each stage - Picking the precise owner when two theorists wrote about the same topic - Choosing the correct statistical test from the type of data and number of groups

Punishes: - Mixing stage models (stages of change, group stages, Erikson, Super) - Treating a near-synonym as a match (“exception” vs. “unique outcome”; “assimilation” in Piaget vs. Berry) - Confusing reliability with validity

Quick Checks

1. A 9-year-old becomes discouraged and says he’s “bad at everything” after comparing his schoolwork to classmates’. This reflects which Eriksonian stage? A. Initiative vs. guilt B. Industry vs. inferiority C. Identity vs. role confusion D. Concrete operational

2. A client who adopts a career identity chosen by her parents without exploring alternatives is in which identity status? A. Diffusion B. Moratorium C. Foreclosure D. Achievement

3. A counselor helps a client see that an unexpected layoff could open new career possibilities. This most reflects: A. Holland’s congruence B. Super’s maintenance stage C. Krumboltz’s planned happenstance D. Gottfredson’s circumscription

4. Group members begin challenging the leader and each other, and some express doubt about the group’s value. According to Corey, the group is most likely in the: A. Initial stage B. Transition stage C. Working stage D. Contemplation stage

5. A new member says, “I thought I was the only one who felt this way.” Yalom would call this: A. Catharsis B. Altruism C. Universality D. Imitative behavior

6. A researcher wants to compare depression scores across four treatment groups. The most appropriate test is: A. Independent t-test B. Chi-square C. Pearson r D. One-way ANOVA

7. A test produces nearly identical scores when the same clients take it two weeks apart. This demonstrates: A. Test-retest reliability B. Predictive validity C. Internal consistency D. Content validity

8. A T-score of 60 is how many standard deviations above the mean? A. 0.5 B. 1 C. 1.5 D. 2

9. An immigrant client maintains strong ties to her heritage culture while also participating fully in her new community. Berry would describe this as: A. Assimilation B. Separation C. Integration D. Marginalization

10. A researcher concludes a treatment works when, in reality, it does not. This is a: A. Type I error B. Type II error C. Lack of power D. Regression effect

11. Which theorist proposed the Life-Career Rainbow? A. Holland B. Super C. Ginzberg D. Roe

12. “Self-efficacy, outcome expectations, and personal goals” are core constructs of: A. Social Cognitive Career Theory B. Trait-factor theory C. Theory of Work Adjustment D. Career construction theory

Answers and Owner Rationales

1 — B. Erikson’s stage 4, industry vs. inferiority, covers school-age competence. A: initiative vs. guilt is Erikson’s preschool stage. C: identity vs. role confusion is Erikson’s adolescent stage. D: concrete operational is Piaget’s cognitive stage, not a psychosocial crisis.

2 — C. Commitment without exploration is Marcia’s foreclosure. A: diffusion has neither exploration nor commitment. B: moratorium is active exploration without com-

mitment. D: achievement is commitment after exploration.

3 — C. Turning unplanned events into opportunities is Krumboltz's planned happenstance. A: congruence is Holland's person–environment fit. B: maintenance is Super's stage of holding one's position in midcareer. D: circumscription is Gottfredson's process of ruling out options in childhood.

4 — B. Conflict, resistance, and challenges to the leader define Corey's transition stage (Tuckman's storming). A: the initial stage is tentative and polite. C: the working stage is cohesive and productive. D: contemplation is a stage of change (Prochaska & DiClemente), not a group stage.

5 — C. Realizing one is not alone is Yalom's universality. A: catharsis is emotional release. B: altruism is benefiting from helping others. D: imitative behavior is learning by watching members or the leader.

6 — D. Three or more group means: one-way ANOVA (Fisher). A: a t-test compares two means. B: chi-square compares frequencies of nominal data. C: Pearson r measures the relationship between two continuous variables.

7 — A. Stability over time is test-retest reliability. B: predictive validity is about forecasting a future criterion. C: internal consistency is about items agreeing with one another in one administration. D: content validity is about covering the domain.

8 — B. T-scores have a mean of 50 and an SD of 10; 60 is one SD above the mean. A, C, and D would be T-scores of 55, 65, and 70.

9 — C. Keeping heritage while joining the host culture is Berry's integration. A: assimilation gives up heritage. B: separation keeps heritage and avoids the host culture. D: marginalization connects with neither.

10 — A. A false positive, rejecting a true null, is a Type I error. B: a Type II error misses a real effect. C: low power

raises the risk of Type II, not Type I, error. D: regression is a threat to internal validity (Campbell & Stanley), not an error type.

11 — B. The Life-Career Rainbow is Super's. A: Holland owns RIASEC. C: Ginzberg owns fantasy–tentative–realistic. D: Roe owns parent–child relationships as career influences.

12 — A. SCCT (Lent, Brown, & Hackett) is built on Bandura's self-efficacy. B: trait-factor (Parsons, Williamson) matches traits to jobs. C: the Theory of Work Adjustment (Dawis & Lofquist) is about satisfaction and satisfactoriness. D: career construction (Savickas) is about adaptability and life stories.

Audio Companion

Track	Covers	Target length
3.1	Development + identity models	~5 min
3.2	Career theorists	~3 min
3.3	Group stages and group owners	~2 min
3.4	Assessment and research rapid fire	~4 min

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PART TWO

The Content

Chapter 4 — The Alliance and the Microskills

2027 task map: 4.14 Use core counseling skills to facilitate treatment · 4.15 Monitor and resolve therapeutic ruptures · CACREP: Counseling and Helping Relationships

Why This Chapter Matters

Provision of Counseling Interventions is tied for the largest domain on the NCE: 28 of the 140 scored questions. A large share of those questions ask one thing in different clothes: **what should the counselor say or do next?**

The answer choices are usually all real skills. One is a reflection, one is a question, one is advice or a confrontation. The skill you need is the same one Chapter 2 taught for theories: **name what each option is.** Once you can label every option ("that's a paraphrase," "that's premature confrontation," "that's advice-giving"), the right answer is the one that fits the moment in the session.

This chapter teaches you to label options, and then to pick the one the moment calls for.

Who Owns the Skills Vocabulary

The skills have owners too. NCE items may name these owners outright.

Owner	Contribution	Key terms
Carl Rogers	Core conditions	Congruence (genuineness), unconditional positive regard, accurate empathic understanding

Owner	Contribution	Key terms
Edward Bordin	Working alliance	Three parts: agreement on **goals**, agreement on **tasks**, and the **bond**
Allen Ivey (with Mary Bradford Ivey & Carlos Zalaquett)	Microskills hierarchy; intentional interviewing	Attending behavior → basic listening sequence → influencing skills; the five-stage interview
Gerard Egan	*The Skilled Helper*	**SOLER** attending posture; three-stage model (current picture → preferred picture → the way forward)
Robert Carkhuff	Empathy rating scale	Five levels: 1–2 subtract from the client's message, **3 is interchangeable** (minimally facilitative), 4–5 add to it
Clara Hill	Helping skills model	Three stages: **exploration → insight → action**
Safran & Muran	Alliance ruptures and repair	Withdrawal ruptures and confrontation ruptures; repair by addressing the rupture openly

SOLER (Egan)

Letter	Behavior
S	Face the client **Squarely**
O	**Open** posture (arms and legs uncrossed)
L	**Lean** slightly toward the client
E	Appropriate **Eye** contact
R	Stay **Relaxed**

Cultural caution: SOLER describes a Western norm. Sustained eye contact, direct facing, and close seating

can read as disrespectful or aggressive in some cultures. When an item mentions a client's cultural background and nonverbal behavior, the correct answer usually adapts attending to the client rather than applying SOLER rigidly.

The Microskills Hierarchy: Attend → Listen → Influence

Ivey's hierarchy is a sequence, not a menu. You earn the right to influence by attending and listening first.

Level	Skills	What they do
Attending	Eye contact, body language, vocal tone, verbal tracking (staying on the client's topic), comfortable silence	Shows the client you are present
Listening (the basic listening sequence)	Open and closed questions, minimal encouragers, paraphrasing, reflection of feeling, summarizing	Shows the client you understand
Influencing	Reflection of meaning, confrontation, interpretation/ reframe, self-disclosure, immediacy, feedback, information-giving, directives	Helps the client see or do something new

The NCE's cardinal rule: validate before you challenge. When a client expresses distress, the best first response reflects the feeling. When a client resists, the best first response acknowledges the resistance rather than arguing with it. When a client disagrees with the counselor, the best first response explores the client's view rather than defending the counselor's.

Labeling the Options: A Skill-by-Skill Guide

Use one client statement to see every skill side by side.

> **Client:** "My sister keeps calling to tell me how to raise my kids. I just let it go to voicemail now. I don't even want to deal with her anymore."

Skill	What it does	Example response
Minimal encourager	Signals attention, keeps the client talking	"Mm-hmm… go on."
Open question	Invites elaboration; usually begins with *what* or *how*	"What happens for you when you see her name on your phone?"
Closed question	Gathers a specific fact; answerable in a word	"How often does she call?"
Paraphrase	Restates the *content* in fresh words	"Your sister's been calling with parenting advice, and you've started letting it go to voicemail."
Reflection of feeling	Names the *emotion* under the content	"You sound worn out and pretty frustrated with her."
Reflection of meaning	Names the value or belief underneath	"Being trusted to raise your own kids matters a lot to you."
Summary	Ties together several statements or themes	"So far you've talked about your sister's calls, feeling second-guessed as a parent, and wanting some distance."
Clarification	Checks an unclear message	"When you say you don't want to deal with her, do you mean right now, or for good?"

Skill	What it does	Example response
Reframe	Offers a new meaning for the same facts	"It sounds like she's worried about the kids, even if the way she shows it feels like criticism."
Confrontation	Points out a discrepancy, gently	"You say you don't care what she thinks, and you've also told me her calls ruin your whole day."
Interpretation	Proposes a connection outside current awareness	"I wonder if her advice echoes how your mom used to correct you."
Self-disclosure	Shares something about the counselor	"I've had family members weigh in on my choices too, and it can sting."
Immediacy	Addresses what is happening *between counselor and client* right now	"I notice you looked away when I asked about your sister. What's happening for you right now, here with me?"
Advice	Tells the client what to do	"You should tell her to stop calling."

Advice is almost never the right answer on the NCE. It shifts responsibility from client to counselor and skips exploration. The exception is psychoeducation or information the client needs for safety or an informed decision. That is information-giving, not advice.

The Most-Confused Pairs

Pair	Tell them apart by...
Paraphrase vs. reflection of feeling	Paraphrase restates **facts**. Reflection names an **emotion**.
Reflection of feeling vs. reflection of meaning	Feeling names **what they feel**. Meaning names **why it matters** to them.

Pair	Tell them apart by…
Paraphrase vs. summary	Paraphrase covers **one statement**. Summary ties together **several**.
Reframe vs. interpretation	A reframe offers a **new angle on the same facts**. An interpretation proposes a **connection the client hasn't made** (often to the past).
Confrontation vs. immediacy	Confrontation points out a **discrepancy**. Immediacy talks about the **counselor–client relationship right now**.
Immediacy vs. here-and-now (Gestalt)	Immediacy is a **skill** about the relationship. Here-and-now is a **theoretical focus** (Perls) on present experience generally.
Self-disclosure vs. immediacy	Self-disclosure shares the **counselor's life or experience**. Immediacy shares the counselor's **reaction to this client, in this moment**.
Clarification vs. paraphrase	Clarification **asks** because the meaning is unclear. Paraphrase **states** a meaning the counselor already understood.

Empathy, Measured: Carkhuff's Five Levels

NCE items sometimes ask which response is "most empathic" or "interchangeable." Carkhuff's scale gives you the yardstick.

> **Client:** "I got passed over for the promotion again. I've worked there eleven years."

Level	Description	Example
1	Ignores or hurts; irrelevant	"Have you updated your résumé lately?"
2	Subtracts; misses the feeling or gets it partly wrong	"Promotions can be political."

Level	Description	Example
3	**Interchangeable**: captures the same content and feeling the client expressed	"After eleven years, being passed over again feels really discouraging."
4	Adds: reaches a feeling or meaning just below the surface	"After eleven years, being passed over again sounds discouraging, and maybe like you're starting to wonder whether your work is seen at all."
5	Significantly adds, and points toward action or deeper understanding	Level 4 plus an invitation to explore what the client wants next

Level 3 is the minimum facilitative response. On the NCE, a level-3 or level-4 reflection usually beats a question, advice, or reassurance, especially early in counseling or right after an emotional disclosure.

When Influencing Skills Are Right

Skill	Use it when…	Don't use it when…
Confrontation	The alliance is established; a real discrepancy exists (words vs. words, words vs. actions, words vs. nonverbals); the discrepancy is keeping the client stuck	It's the first session; the client is in acute distress; you'd be confronting to win a point
Interpretation	You've explored enough to have evidence; the client seems ready; you offer it tentatively ("I wonder if…")	Early in counseling; as a pronouncement

Skill	Use it when…	Don't use it when…
Self-disclosure	It's brief, relevant, for the client's benefit, and you return focus to the client right away	It shifts attention to the counselor, is lengthy, or meets the counselor's needs
Immediacy	Something in the counseling relationship matters more right now than the topic: the client is repeating a relational pattern with you, withdrawing, or there's tension between you	Nothing is happening in the relationship worth naming
Reframe	The client is stuck in one meaning that keeps the problem going, and an alternative meaning is believable to them	The reframe would minimize real harm (never reframe abuse as care)

Self-disclosure test: after the disclosure, whose experience is the session about? If the answer is the counselor's, it failed.

Questions: Open, Closed, and the "Why" Problem

Open questions are the default. They invite the client to lead.

Closed questions are the right tool for specific facts, and they are *required* in risk assessment. "Are you thinking about killing yourself?" is a closed question, and asking it directly is correct practice. Asking about suicide does not plant the idea.

Avoid "why" questions. "Why did you do that?" asks the client to justify, which invites defensiveness. Convert it to *what* or *how*: "What was going on for you right before that?"

Avoid question strings. Asking three questions in a row without letting the client answer turns a session into an interrogation.

The Alliance: Building, Monitoring, Repairing

Bordin's working alliance has three parts, and an NCE item about a struggling alliance is usually asking which part broke:

Part	Breakdown looks like…	Repair
Goals	Client and counselor want different outcomes ("I'm here because my wife made me come")	Explore the client's own goals; find shared ground
Tasks	Client doesn't see the point of the work ("These worksheets are useless")	Explain the rationale, collaborate on methods, adjust
Bond	Distrust, distance, conflict with the counselor	Empathy, reliability, naming and repairing ruptures

Ruptures (Safran & Muran)

- **Withdrawal rupture:** the client goes quiet, agrees too easily, changes the subject, or misses sessions.
- **Confrontation rupture:** the client openly complains about the counselor or counseling.

The NCE answer for a rupture is almost always to name it non-defensively and explore it ("I noticed you got quiet after I suggested homework. I'd like to hear how that landed for you."). Wrong answers defend the counselor, ignore the rupture, refer the client out, or press on with the plan.

A repaired rupture often *strengthens* the alliance.

Resistance and Reluctance

Treat resistance as information, not opposition. The scored response acknowledges it and explores it ("It sounds like part of you isn't sure counseling will help"). This mirrors motivational interviewing's "rolling with resistance" (Miller & Rollnick): don't argue, don't push harder.

Mandated clients: acknowledge the circumstances openly, clarify what will and won't be shared with the referral source (informed consent), and look for a goal the client actually wants.

Containing Intense Feelings (4.14)

When a client becomes overwhelmed in session (sobbing, panic, rage, dissociation):

1. **Stay present and calm.** Your regulation helps theirs.
2. **Slow down.** Lower your voice, slow your pace, allow silence.
3. **Ground if needed.** Feet on the floor, breathing, naming things in the room, orienting to the present.
4. **Reflect without amplifying.** Name the feeling simply; don't push for more detail in the moment.
5. **Assess safety** if the intensity includes any sign of risk.
6. **Close the session with the client stabilized.** Don't open new painful material in the last minutes; leave time to contain and plan.

What the exam punishes: reassurance that dismisses the feeling ("It's going to be fine"), changing the subject, ending the session abruptly, or pushing for trauma detail while the client is flooded.

Silence

Silence after a significant disclosure gives the client room. Filling it with reassurance or a new question often reflects the counselor's discomfort. Comfort with silence

varies across cultures, so calibrate it rather than applying a rule.

The Stages of the Counseling Relationship

Stage	Main task	Skills the exam expects
Beginning	Build rapport and trust; informed consent; explore the concern	Attending, open questions, reflections; no confrontation
Middle (working)	Deepen understanding; work toward goals	Listening plus influencing skills; immediacy; confrontation once the alliance is strong
Ending	Consolidate gains; prepare for termination	Summarizing progress; relapse prevention; processing feelings about ending; referral if needed

Termination is introduced *before* the final session and discussed openly, including the client's feelings about ending.

What the Exam Rewards / What It Punishes

Rewards: - Reflecting feeling after an emotional disclosure - Open, *what*/*how* questions over *why* - Naming and exploring ruptures and resistance - Brief, client-focused self-disclosure, if any - Adapting attending behavior to culture - Direct, closed questions about suicide when risk is present

Punishes: - Advice, reassurance, and premature problem-solving - Confrontation or interpretation before the alliance is established - Defending the counselor when the client is upset with counseling - Self-disclosure that shifts the focus to the counselor - Pushing for detail when a client is flooded

Swap-Pair Drill

Cover the right column and label each response.

Counselor response	Skill
“You’ve been juggling work, your dad’s appointments, and the kids’ schedules, and you said you haven’t slept well in weeks.”	Summary
“You sound exhausted.”	Reflection of feeling
“Help me understand what you mean by ‘done.’”	Clarification
“You said the move doesn’t bother you, and you’ve cried twice talking about it today.”	Confrontation
“Right now, as you tell me this, I notice you’re apologizing to me a lot.”	Immediacy
“Maybe her checking in so often is her way of showing she’s scared of losing you.”	Reframe
“I wonder if being ignored by your boss stirs up how invisible you felt as a kid.”	Interpretation
“So your manager moved the deadline up without asking you.”	Paraphrase
“Being treated fairly is really important to you.”	Reflection of meaning
“You should just tell him how you feel.”	Advice

Quick Checks

Rationales label every option.

1. In a first session, a client says, “I don’t even know why I’m here. Nothing’s going to change.” The best response is: A. “Counseling has a high success rate, so try to stay positive.” B. “It sounds like you’re feeling pretty hopeless that anything can be different.” C. “Why did you make the

appointment, then?” D. “You’re saying nothing will change, but you came today anyway.”

2. Which response best represents a level-3 (interchangeable) empathic response to: “My dog died last week, and the house is so quiet now”? A. “Have you thought about getting another dog?” B. “Losing a pet is hard for everyone.” C. “The house feels empty without him, and you’re really missing him.” D. “Grief has five stages, and you’re probably in denial.”

3. A client who has been engaged for six sessions arrives late, answers in single words, and says, “Whatever you think is fine” to every suggestion. The best counselor response is: A. Continue with the planned intervention B. Refer the client to a different counselor C. Note the change and explore it: “Today feels different between us. I’m wondering what’s going on for you.” D. Remind the client that being late violates the agreement

4. A counselor says, “Right now, I’m noticing that every time I ask about your marriage, you make a joke.” This is: A. Interpretation B. Immediacy C. Reframe D. Reflection of meaning

5. Which self-disclosure is most appropriate? A. A detailed account of the counselor’s own divorce B. “I’ve been through something similar, and I remember how lonely it felt. What’s it been like for you?” C. Sharing the counselor’s current stress about work to show the client they aren’t alone D. Telling the client about another client with the same problem

6. According to Bordin, a client who says, “I don’t see how talking about my childhood will help my anxiety” is signaling a problem with which part of the alliance? A. Bond B. Goals C. Tasks D. Transference

7. A client becomes tearful and begins breathing rapidly while describing a car accident. The counselor’s best first action is to: A. Ask the client to describe the accident in more detail B. Help the client slow down and ground in

the present C. End the session early D. Interpret the reaction as unresolved trauma

8. Which question is least likely to produce defensiveness? A. “Why didn’t you take your medication?” B. “Why do you keep going back to him?” C. “What gets in the way of taking your medication?” D. “Don’t you think you should leave him?”

9. “SOLER” is associated with: A. Ivey B. Egan C. Carkhuff D. Hill

10. A counselor tells a client, “It sounds like your son’s arguing might also be his way of practicing independence.” This is: A. Confrontation B. Reframe C. Paraphrase D. Advice

11. Hill’s helping skills model proceeds in which order? A. Insight → exploration → action B. Exploration → action → insight C. Exploration → insight → action D. Action → insight → exploration

12. A mandated client says, “The judge sent me. I don’t have a problem.” The best response is: A. “If you don’t participate, I’ll have to report that you’re noncompliant.” B. “Everybody who comes here says that at first.” C. “Being required to come here when you don’t think you need it sounds frustrating. What would make this time worth something to you?” D. “Let’s start with your drinking history.”

Answers and Rationales

1 — B. A reflection of feeling meets hopelessness with understanding, which is the right first move in a first session. A: reassurance that dismisses the feeling. C: a “why” question that invites defensiveness. D: a confrontation, premature in a first session.

2 — C. It captures both the content (the empty house) and the feeling (missing him) without adding or subtracting: interchangeable. A: problem-solving advice (level 1). B: a generalization that loses the client’s specific loss (level 2). D: a misused stage model and an interpretation,

which is hurtful (level 1). Kübler-Ross described dying patients, not a required sequence for grief.

3 — C. A withdrawal rupture (Safran & Muran) is best named and explored non-defensively, using immediacy. A: ignores the rupture. B: referral abandons the alliance instead of repairing it. D: defends the counselor's rules and escalates the rupture.

4 — B. Commenting on what is happening between counselor and client in the moment is immediacy. A: interpretation would propose *why* (e.g., a link to the client's past). C: a reframe would offer a new meaning. D: reflection of meaning would name an underlying value.

5 — B. Brief, relevant, and immediately returns focus to the client. A: lengthy and counselor-centered. C: meets the counselor's needs and burdens the client. D: a confidentiality violation, not self-disclosure.

6 — C. Disagreement about the *methods* of counseling is a tasks problem. A: bond is the emotional connection and trust. B: goals are the desired outcomes (the client still wants less anxiety). D: transference is a psychodynamic concept, not part of Bordin's model.

7 — B. Containment and grounding come first when a client is flooded. A: pushing for detail increases dysregulation. C: ending abruptly leaves the client unstabilized. D: interpreting in the moment skips containment.

8 — C. A "what" question invites exploration. A and B: "why" questions invite justification. D: a leading question that is really advice.

9 — B. SOLER is Egan's (*The Skilled Helper*). A: Ivey owns the microskills hierarchy. C: Carkhuff owns the five-level empathy scale. D: Hill owns the exploration–insight–action model.

10 — B. A new, plausible meaning for the same behavior is a reframe. A: confrontation would point out a discrepancy. C: a paraphrase would restate the content without new meaning. D: advice would tell the client what to do.

11 — C. Hill's model moves from exploration to insight to action. The other orders scramble the sequence.

12 — C. Acknowledges the mandate and the feeling, then looks for a goal the client owns. A: a threat that damages the alliance (and the reporting terms belong in informed consent, not a threat). B: dismisses the client's experience. D: jumps to assessment content before engagement.

Audio Companion

Track	Covers	Target length
4.1	Name That Skill: label the counselor's response	~10 min

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Chapter 5 — Theory into Intervention

2027 task map: 4.6 Implement empirically supported interventions · 4.7 Implement interventions aligned with service modality · 4.3 Facilitate client's skill development · 4.11 Provide psychoeducation · 4.4 Facilitate clients' ability to navigate interpersonal relationships · CACREP: Counseling and Helping Relationships; Human Growth and Development

From "Who Owns It" to "When Do I Use It"

Chapter 2 taught you to name the theory behind a technique. This chapter asks the next question the NCE asks: **given this client, which intervention fits?**

Most applied intervention items turn on one of four matches:

1. **Problem → approach.** Which intervention has the best support for this presenting concern?
2. **Development → approach.** Is the intervention right for this client's age and cognitive level?
3. **Modality → approach.** Is this individual, couple, family, or group work, and does the intervention fit that format?
4. **Readiness → approach.** Is the client ready for action, or still deciding whether to change?

When two options are both real interventions, the one that matches all four usually wins.

Match 1 — Problem → Approach

The NCE rewards knowing which approaches have the strongest research support for common concerns. Each one has an owner, so the ownership habit still applies.

Presenting concern	Strongly supported approaches (owner)	Key mechanism
Specific phobia	Exposure therapy; systematic desensitization (Wolpe)	Graded contact with the feared stimulus until anxiety drops
Panic	CBT for panic with interoceptive exposure	Exposure to feared body sensations; correcting catastrophic misinterpretation
Obsessive-compulsive symptoms	Exposure and response prevention (ERP)	Face the obsession *without* doing the compulsion
Generalized worry	CBT; worry exposure; applied relaxation	Test worry beliefs; tolerate uncertainty
Trauma (adults)	Prolonged exposure (Edna Foa); cognitive processing therapy (Patricia Resick); EMDR (Francine Shapiro)	Process the memory; change trauma-related beliefs
Trauma (children/ adolescents)	Trauma-focused CBT (Cohen, Mannarino, & Deblinger)	Includes caregiver; graduated trauma narrative; the PRACTICE components
Depression	CBT (Beck); behavioral activation (Lewinsohn; Neil Jacobson); interpersonal therapy (Klerman & Weissman)	Change thoughts, increase rewarding activity, or address role disputes, transitions, grief, interpersonal deficits
Chronic self-harm, emotion dysregulation	Dialectical behavior therapy (Linehan)	Skills: mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness

Presenting concern	Strongly supported approaches (owner)	Key mechanism
Substance use	Motivational interviewing (Miller & Rollnick); CBT/ relapse prevention (Marlatt); contingency management; 12-step facilitation	Resolve ambivalence; cope with triggers; reinforce abstinence
Insomnia	CBT for insomnia (CBT-I)	Stimulus control, sleep restriction, sleep hygiene, cognitive work
Adolescent anorexia	Family-based treatment ("Maudsley approach"; Lock & Le Grange)	Parents temporarily take charge of refeeding
Young child behavior problems	Parent management training; Parent–Child Interaction Therapy (PCIT; Sheila Eyberg)	Coach parents in real time; consistent consequences
Couple distress	Emotionally focused therapy (Sue Johnson); Gottman Method	Attachment and the negative cycle; friendship and conflict management
Grief (uncomplicated)	Supportive counseling; meaning-making; Worden's tasks	Normalize, don't pathologize

The exam's trap: an option that is a *real* therapy but the *wrong match*. Psychoanalysis for a specific phobia. Insight work alone for OCD. Talk therapy without exposure for panic. When a strongly supported approach is among the options for one of the concerns above, it usually wins.

Exposure, Compared

Technique	Owner / family	How it works
Systematic desensitization	Wolpe	Relaxation + *imagined*, gradual steps up a hierarchy
Graded (in vivo) exposure	Behavioral	*Real-life* contact, step by step
Flooding	Behavioral	Immediate, intense, prolonged exposure (real life)
Implosive therapy	Stampfl	Intense *imagined* exposure, often exaggerated
ERP	Behavioral (OCD)	Exposure *plus* blocking the compulsion
Interoceptive exposure	CBT for panic	Bringing on feared body sensations (spinning, breathing through a straw)

Match 2 — Development → Approach (4.6)

The same goal needs a different intervention at different ages.

Children

Children communicate through play and concrete experience more than through talk.

Approach	Owner	Key idea
Child-centered play therapy	Virginia Axline; Garry Landreth	Rogers's core conditions applied to play; "toys are the child's words and play is the child's language" (Landreth's principle)
Filial therapy	Bernard & Louise Guerney	Parents are trained to conduct child-centered play sessions at home

Approach	Owner	Key idea
Sand tray / sandplay	Margaret Lowenfeld (world technique); Dora Kalff (Jungian sandplay)	Child builds scenes in sand with miniatures
Bibliotherapy	(general)	Stories that mirror the child's situation
Art and expressive therapies	(general)	Drawing, puppets, and play as communication
Behavioral charts, token systems	Skinner/operant	Concrete, immediate reinforcement

Rules for children: use concrete language, short sessions, and activity-based methods. Involve parents or caregivers. Remember Piaget: a preoperational child cannot do abstract cognitive restructuring, and a concrete-operational child does better with concrete examples than hypotheticals.

Adolescents

- Respect the drive for autonomy (Erikson: identity vs. role confusion).
- Clarify confidentiality and its limits with both teen and parent at the start.
- Collaborative, less directive approaches tend to engage better than lectures.
- Peer influence matters; group work is often effective.

Older Adults

Approach	Owner	Key idea
Life review	Robert Butler	Structured review of one's life to resolve conflicts and find meaning
Reminiscence therapy	(general)	Recalling positive memories to support mood and connection
Integrity vs. despair	Erikson	Developmental task framing late-life work

Rules for older adults: screen for cognitive changes and medical contributors; adjust pace, print size, and hearing accommodations; don't assume decline or that change isn't possible.

Intellectual Disability and Cognitive Limits

Use simpler language, concrete and visual materials, repetition, behavioral approaches, and involvement of support people. Assess capacity for informed consent (2A.1).

Match 3 — Modality → Approach (4.7)

Modality	Who is the client?	What changes
Individual	One person	Intrapersonal focus; confidentiality with one person
Couple	The relationship	Both partners present; secrets policy must be set at the start; neutrality between partners
Family	The family system	Symptoms seen as serving a function; circular rather than linear causality; the identified patient (IP) is not "the problem"
Group	Each member, *and* the group as a whole	Confidentiality can't be guaranteed; group dynamics and therapeutic factors become tools (Chapter 8)

Key modality rules: - **Couples work is contraindicated when there is active intimate partner violence** with coercive control. Assess safety individually first. - In family work, the counselor treats the system, so blaming one member (the IP) is a wrong answer. - A **no-secrets policy** in couple and family work should be explained during informed consent, before it's needed.

Match 4 — Readiness → Approach (4.6)

Motivation is the match with the most moving parts. Two owners run this area. Keep them separate:

- **Prochaska & DiClemente** (Transtheoretical Model) describe *where the client is*.
- **Miller & Rollnick** (motivational interviewing) describe *how to talk with the client* about change.

Stage-Matched Interventions

Stage	Client sounds like…	Best counselor move	Wrong move
Precontemplation	"I don't have a problem."	Build rapport; raise awareness gently; provide information; explore what others are concerned about	Action plans; confrontation; arguing
Contemplation	"Maybe I drink too much, but it helps me relax."	Explore ambivalence; decisional balance (pros and cons of changing and not changing); develop discrepancy	Pushing for a quit date
Preparation	"I'm going to cut back starting next month."	Build a specific plan; set goals; strengthen commitment	Re-exploring whether to change
Action	"I haven't had a drink in three weeks."	Support new behaviors; skills; reinforcement; coping with triggers	Ignoring relapse risk

Stage	Client sounds like…	Best counselor move	Wrong move
Maintenance	"It's been eight months."	Relapse prevention; lifestyle change; identify high-risk situations	Ending support abruptly

Relapse is a normal part of change, not a failure. Treat it as information and help the client re-enter the cycle.

Motivational Interviewing in Brief

Element	Content
Spirit	Partnership, acceptance, compassion, evocation
Four processes	Engaging → focusing → evoking → planning
Core skills (OARS)	Open questions, Affirmations, Reflections, Summaries
Change talk (DARN-CAT)	Preparatory: **D**esire, **A**bility, **R**easons, **N**eed. Mobilizing: **C**ommitment, **A**ctivation, **T**aking steps
Sustain talk	The client's arguments *for* staying the same, met with reflection, not argument
Developing discrepancy	Highlighting the gap between current behavior and the client's own values or goals
Righting reflex	The counselor's urge to fix or persuade, which MI teaches you to resist
Rulers	"On a scale of 0 to 10, how important is this change to you? How confident are you that you could do it?"

The MI move the NCE rewards: when a client is ambivalent, reflect both sides. "Part of you wants to keep drinking because it helps you relax, and part of you is worried about what it's doing to your marriage."

Relapse Prevention (Marlatt)

- Identify **high-risk situations** (people, places, emotions).
- Build **coping responses** for each.
- Watch for **seemingly irrelevant decisions** that set up a lapse.
- Address the **abstinence violation effect**: after one slip, the belief "I've blown it, so I might as well keep going." Reframe a lapse as a learning event, not a collapse.

Skills Training and Psychoeducation (4.3, 4.4, 4.11)

Psychoeducation is teaching: giving clients information about their condition, treatment, or coping strategies. Skills training goes further: the client practices.

Skill area	Core content	Owner (where there is one)
Assertiveness	Passive vs. aggressive vs. assertive vs. passive-aggressive; "I" statements; broken record; saying no; behavior rehearsal	Assertiveness training grew from behavioral work (Wolpe, Salter, Lazarus)
Communication	Active listening; I-messages instead of you-messages; speaker–listener technique	Thomas Gordon (Parent Effectiveness Training: I-messages, active listening)
Conflict resolution	Separate people from problems; focus on interests, not positions; generate options; agree on criteria; time-outs when flooded	Interest-based negotiation (Fisher & Ury); Gottman (softened startup, repair attempts)

Skill area	Core content	Owner (where there is one)
Stress management	Progressive muscle relaxation; diaphragmatic breathing; mindfulness; time management; cognitive reappraisal	Progressive relaxation (Edmund Jacobson); relaxation response (Herbert Benson); Mindfulness-Based Stress Reduction (Jon Kabat-Zinn)
Problem-solving	Define the problem → generate alternatives → decide → implement → evaluate	D'Zurilla & Goldfried
Anger management	Identify cues; time-outs; cognitive restructuring; relaxation; assertive alternatives	(CBT tradition; Novaco)
Divorce adjustment	Grief for the relationship; co-parenting; children's adjustment; practical changes	(psychoeducational groups)

Assertive, Passive, Aggressive

Style	Example ("A coworker keeps taking credit for your work")
Passive	Says nothing; resents it privately
Aggressive	"You're a liar and a thief."
Passive-aggressive	"Must be nice to have someone else do your job."
Assertive	"When my work is presented as yours, I feel frustrated. I'd like us to credit each person's part going forward."

The "I" statement formula: *When* [specific behavior], *I feel* [feeling], *because* [impact]. *I'd like* [request].

Homework

Between-session tasks are a core part of CBT, REBT, reality therapy, SFBT (the first-session formula task), and skills training. The exam rewards homework that is **collaboratively set, specific, and reviewed at the next session.** Assigning homework and never asking about it teaches the client that it doesn't matter.

Educating Clients About Transference and Defenses (4.11)

One 2027 task (4.11, psychoeducation) asks you to *teach* these psychodynamic concepts in plain language, not just recognize them.

Term	Meaning
Transference	The client redirects feelings and expectations from important past relationships onto the counselor
Countertransference	The counselor's own emotional reactions to the client, often from the counselor's history
Defense mechanism	An unconscious strategy that protects against anxiety (see Chapter 2)

How to educate: use plain language, connect it to something the client has noticed, and offer it tentatively. ("Sometimes, when someone reminds us of an important person from our past, we react to them the way we reacted back then. I wonder if some of what you feel toward me might fit that.")

Countertransference is the counselor's responsibility. When a counselor notices strong personal reactions to a client, the correct answer is self-reflection and **supervision or consultation**, not disclosing the reaction to the client, ignoring it, or immediately referring the client out.

Implementing the Plan (4.7)

Every intervention should trace back to a treatment goal. If an item describes a counselor introducing a technique unrelated to the client's stated goals, that's the wrong answer even if the technique is valid. When progress stalls, the next step is to **review the plan with the client**, not to keep repeating the same intervention or to switch approaches without discussing it (Chapter 21).

Frameworks and Tools the 2027 Specifications Name

Why it's here: NBCC's 2027 examination specifications list these frameworks and tools in their additional content guidance (NBCC, 2026).

Know the Owner, the Idea, and the Look-Alike

The 2027 specifications name frameworks and tools that candidates may benefit from reviewing. You do not need to practice them. You need to recognize each one from a short description, name its owner, and tell it apart from the approach it gets confused with.

Each entry gives the **owner**, the **core idea**, the **signature terms**, **when it fits**, and the **look-alike**. Items usually hand you a signature term ("exiles," "pendulation," "cultural opportunities") and ask for the model, or describe a client and ask which approach fits.

Part 1 — Who Owns It

Owner	What it owns
Richard Schwartz (1995)	**Internal Family Systems (IFS)**: Self; managers, firefighters, exiles; unburdening
Owen et al. (2011); **Davis et al.** (2018)	**Multicultural Orientation (MCO)**: cultural humility, opportunities, comfort

Owner	What it owns
Stanley, Brown, et al. (2009)	**CBT for suicide prevention (CBT-SP)**
Stanley & Brown (2012)	**Safety Planning Intervention**
Rudd & Bryan (Rudd et al., 2015)	**Brief CBT for suicide prevention (BCBT)**
David Jobes (2016)	**CAMS**; the Suicide Status Form
Segal, Williams, & Teasdale (2013)	**Mindfulness-Based Cognitive Therapy (MBCT)**
Jon Kabat-Zinn (1990)	**Mindfulness-Based Stress Reduction (MBSR)**
Marsha Linehan (1993)	**DBT** mindfulness skills; wise mind
John Krumboltz	**Social learning theory of career decision making**; **planned happenstance**; **happenstance learning theory**
Peter Levine (1997)	**Somatic Experiencing (SE)**: titration, pendulation
Pat Ogden (Ogden et al., 2006)	**Sensorimotor Psychotherapy**
BCIA	Voluntary **biofeedback** and **neurofeedback** certification
Cohen, Mannarino, & Deblinger (2017)	**Trauma-Focused CBT (TF-CBT)**: PRACTICE
Patricia Resick (Resick et al., 2017)	**Cognitive Processing Therapy (CPT)**
Edna Foa (Foa et al., 2007)	**Prolonged Exposure (PE)**
NIMH (Horowitz et al., 2012)	**Ask Suicide-Screening Questions (ASQ)**
Posner et al. (2011)	**Columbia-Suicide Severity Rating Scale (C-SSRS)**
Kroenke, Spitzer, & Williams (2001)	**PHQ-9**, including item 9
Myers & Briggs, from **Jung** (1921)	**Myers-Briggs Type Indicator (MBTI)**
E. K. Strong Jr.	**Strong Interest Inventory**
John Holland (1997)	**RIASEC**; **Self-Directed Search (SDS)**

Owner	What it owns
Kretzmann & McKnight (1993)	**Asset-based community development**; asset mapping
American Psychological Association (2020)	**Publication Manual, 7th ed.**; bias-free language

Part 2 — Frameworks

Internal Family Systems (Schwartz)

Owner: Richard Schwartz (1995; Schwartz & Sweezy, 2020), a family therapist who applied systems thinking to the inner world.

Core idea: The mind is naturally made of **parts**, and everyone has an undamaged core **Self** that can lead. Symptoms come from parts carrying **burdens** (extreme beliefs and feelings from painful experience) and from protectors working to keep those burdens buried.

Signature terms: - **Exiles**: young, wounded parts carrying pain, shame, or fear. - **Managers**: **proactive** protectors that prevent triggering through control, perfectionism, criticism, or caretaking. - **Firefighters**: **reactive** protectors that douse an exile's pain fast, through bingeing, substances, dissociation, or rage. - **Self**, marked by the **8 C's**: calmness, curiosity, clarity, compassion, confidence, courage, creativity, connectedness. - **Unblending** (a part steps back so Self can relate to it) and **unburdening** (after being witnessed, an exile releases its burden).

When it fits: clients who speak in parts ("part of me wants to quit"), trauma survivors with harsh inner critics, and self-destructive behavior that serves a protective function. No part is treated as bad.

Look-alike: Gestalt **two-chair** work (Perls) voices inner conflict without the Self/protector/exile map. **Bowen** family systems works with the actual family, not inner parts.

Multicultural Orientation (Owen et al.)

Owner: Jesse Owen and colleagues (Owen et al., 2011), reviewed by **Davis et al. (2018)**.

Core idea: Multicultural competence is a **way of doing** (awareness, knowledge, skills). Multicultural orientation is a **way of being**: how the counselor engages culture moment to moment in session.

Three pillars: - **Cultural humility**: an other-oriented stance that does not assume expertise in the client's identity. - **Cultural opportunities**: in-session moments when the client's cultural identity could be explored. Does the counselor notice and take them? Missed opportunities have been linked to weaker outcomes. - **Cultural comfort**: the counselor's ease and openness when cultural topics arise.

When it fits: any client, especially when identity, faith, race, gender, or migration surfaces and the counselor must decide whether to explore it now.

Look-alike: the **Multicultural Counseling Competencies** (Sue, Arredondo, & McDavis, 1992) and the **MSJCC** (Ratts et al., 2016) are **competency** frameworks. MCO is **process**. "Noticed the moment and invited the client to say more" → MCO.

CBT-SP and Its Neighbors

Owner: Barbara Stanley, Gregory Brown, and colleagues (Stanley et al., 2009). Developed for adolescents after a suicide attempt, within the NIMH-funded **Treatment of Adolescent Suicide Attempters (TASA)** study, drawing on CBT, DBT, and Brown et al.'s (2005) cognitive therapy for adult suicide attempters.

Core idea: Suicidal behavior is the **direct target** of treatment, not a symptom expected to fade with depression.

Signature components: a **chain analysis** of the recent suicidal crisis; early **safety planning**; **skills** (cognitive restructuring, problem solving, emotion regulation, family work); and a **relapse-prevention task**, in which the client imagines a crisis in detail and rehearses using the new skills.

Neighbors: - **Safety Planning Intervention** (**Stanley & Brown, 2012**): a **brief stand-alone intervention**, not a full therapy. A written, prioritized plan: warning signs, internal coping, people and places for distraction, people to ask for help, professionals and crisis lines, and **making the environment safe**. It is **not** a no-suicide contract. - **Brief CBT for suicide prevention** (**Rudd** and **Bryan**; Rudd et al., 2015): a short structured protocol tested with active-duty soldiers, using a crisis response plan. - **CAMS** (**Jobes, 2016**): a **framework**, not a single therapy. Counselor and client complete the **Suicide Status Form** side by side and target the client's suicide **drivers**.

Mindfulness-Based Cognitive Therapy (Segal, Williams, & Teasdale)

Owner: Zindel Segal, Mark Williams, and John Teasdale (2013).

Core idea: Relapse prevention for recurrent depression. A low mood can reactivate old depressive thinking. MBCT teaches people in recovery to notice that shift early and step back from it.

Signature terms: decentering (thoughts as mental events, not facts); **doing mode vs. being mode**; an **8-week group** with home practice; the **3-minute breathing space**. MBCT adapts MBSR and adds cognitive therapy elements.

When it fits: a client **in remission** with **several prior episodes**. Benefit appears clearest with three or more. It is not designed as first-step treatment for an acute severe episode. Some national guidelines, including the UK's

NICE, have listed MBCT for relapse prevention; verify current wording before citing.

Look-alikes: MBSR (**Kabat-Zinn, 1990**) is the parent program, built for **stress, pain, and illness**. **DBT mindfulness** (**Linehan, 1993**) is one skills module: "what" and "how" skills and **wise mind**, for emotion dysregulation. **ACT** (**Hayes**) uses mindfulness for acceptance, defusion, and values-based action.

Krumboltz's Career Theories

Owner: John Krumboltz, who applied Bandura's social learning theory to career choice (Mitchell & Krumboltz, 1996).

Core idea: Career preferences are **learned**, and people can keep learning. Four factors shape decisions: 1. **Genetic endowment and special abilities** 2. **Environmental conditions and events** (economy, family, opportunity) 3. **Learning experiences**: **instrumental** (consequences of one's own actions) and **associative** (pairings and observation) 4. **Task-approach skills** (work habits, problem solving, goal setting)

These produce **self-observation** and **worldview generalizations**. Faulty ones block decisions; the **Career Beliefs Inventory** surfaces them.

Planned happenstance (Mitchell, Levin, & Krumboltz, 1999) and **happenstance learning theory** (Krumboltz, 2009): unplanned events are normal and useful. Counselors help clients **create and use chance** through **curiosity, persistence, flexibility, optimism, and risk taking**. Indecision becomes **open-mindedness**.

When it fits: a client who "doesn't know their passion," was derailed by a layoff, or holds limiting beliefs from narrow experience.

Look-alikes: SCCT (Lent, Brown, & Hackett, 1994) also builds on Bandura but centers **self-efficacy, outcome**

expectations, and goals. **Holland** matches types to environments. **Super** maps life stages and roles.

Somatic Therapies (Levine; Ogden)

Core idea: Trauma is held in the body as well as in memory and belief. **Bottom-up** work starts with sensation, movement, and arousal. **Top-down** work (such as cognitive therapy) starts with thought and meaning.

Somatic Experiencing (**Peter Levine**, 1997): **titration** (approaching activation in small doses); **pendulation** (moving attention between activation and a resource so the system learns it can return to calm); **resourcing**; completing **thwarted defensive responses**.

Sensorimotor Psychotherapy (**Pat Ogden**; Ogden, Minton, & Pain, 2006): integrates body-based work with cognitive and emotional processing; tracks posture and movement in the moment; keeps arousal within the **window of tolerance** (Siegel, 1999).

When it fits: survivors who cannot tolerate talking about the trauma, who report body-based symptoms, or who flood or shut down in talk therapy.

Evidence: still developing. Early studies, including a randomized trial of SE for PTSD (Brom et al., 2017), are encouraging, but these are not first-line PTSD treatments in major guidelines.

Look-alikes: EMDR (Shapiro) uses bilateral stimulation in a set protocol. **Polyvagal theory** (Porges) is a **theory** many somatic clinicians draw on, not a therapy.

Biofeedback

Core idea: Sensors measure a body process and feed it back in real time so the client learns to change it.

Modality	Measures
EMG	Muscle tension

Modality	Measures
Thermal	Skin temperature
EDA / GSR	Skin conductance
HRV	Beat-to-beat heart rate variation; paced “resonance” breathing
Neurofeedback (EEG)	Brain electrical activity; evidence varies by condition

Who provides it: clinicians from many licensed disciplines. The **Biofeedback Certification International Alliance (BCIA)** offers **voluntary** certification. Certification is a training credential, not a license, and does not expand legal scope of practice. Whether biofeedback is within a counselor’s scope is set by state law; check your state.

Counselor’s role: obtain education, training, and supervised experience before using it (**ACA C.2.a** Boundaries of Competence; **C.2.b** New Specialty Areas of Practice), or refer. Equipment-free self-regulation skills such as paced breathing remain available.

Look-alikes: relaxation training and **mindfulness** teach self-regulation **without instrumented feedback**. Neurofeedback is a **subtype** of biofeedback.

TF-CBT vs. CPT vs. PE

Owner: Judith Cohen, Anthony Mannarino, and Esther Deblinger (2017). A short-term, components-based treatment for **children and adolescents (about ages 3–18)** with a **nonoffending caregiver** in parallel and joint sessions.

PRACTICE: Psychoeducation and **P**arenting skills · **R**elaxation · **A**ffective modulation · **C**ognitive coping · **T**rauma narrative and processing · **I**n vivo mastery · **C**onjoint child-caregiver sessions · **E**nhancing safety. Skills come first; the **trauma narrative** follows and is later shared with the caregiver.

Feature	TF-CBT	CPT (Resick)	PE (Foa)
Population	Children and teens, with caregiver	Primarily adults	Primarily adults
Mechanism	Skills, then narrative exposure	**Cognitive**: challenge **stuck points**	**Exposure**: emotional processing
Signature terms	PRACTICE; trauma narrative	**Impact statement**; safety, trust, power/control, esteem, intimacy	**Imaginal** and **in vivo** exposure

Part 3 — Tools and Assessments

Ask Suicide-Screening Questions (ASQ)

Owner: NIMH; validated with youth in pediatric emergency departments (Horowitz et al., 2012).

What it is: a brief **screen** for **medical settings**, not a risk assessment. Validated with **youth ages 10–21** and, in later NIMH-led research, with **adult medical patients** (verify current age guidance in the NIMH toolkit).

Structure: four yes/no questions (recent wish to be dead, belief that family would be better off, recent thoughts of killing oneself, any past attempt). Any yes → the **acuity question**: thoughts of killing yourself **right now**? - **Yes to acuity → acute positive**: urgent safety and full mental health evaluation; the patient cannot leave until evaluated for safety. - **Yes to any of the four, no to acuity → non-acute positive**: a **brief suicide safety assessment** to decide whether a full mental health evaluation is needed.

The screen sorts. A clinician still assesses.

Look-alikes: the **C-SSRS** (**Posner et al., 2011**) grades **ideation severity** from wish to be dead to active ideation with plan and intent, and separately asks about **behavior**.

PHQ-9 item 9 (**Kroenke, Spitzer, & Williams, 2001**) is one item on a **depression** measure; a positive answer requires follow-up, but it is not a risk assessment.

MBTI, the Strong, and the SDS

MBTI (**Isabel Briggs Myers and Katharine Cook Briggs**, from **Jung**, 1921): four **dichotomies** (E–I, S–N, T–F, J–P) combine into **16 types**. Widely used for self-understanding, career exploration, and team building. **Limits:** it forces continuous traits into either/or categories, and many people get a different type on retest (Pittenger, 2005). Its publisher's own guidance discourages using it for hiring or selection.

Strong Interest Inventory (**E. K. Strong Jr.**): measures **interests**, compared with people **satisfied in specific occupations**. Reports Holland's six General Occupational Themes, Basic Interest Scales, Occupational Scales, and Personal Style Scales. Publisher-scored; professionally interpreted.

Self-Directed Search (**John Holland**, 1997): built on **RIASEC** (Realistic, Investigative, Artistic, Social, Enterprising, Conventional). **Self-administered, self-scored, self-interpreted.** Yields a **three-letter Holland code**.

Choose instruments appropriate to the client and purpose (**ACA E.6.a** Appropriateness of Instruments) and within your training (**ACA E.2.a** Limits of Competence).

Community Needs and Strengths Assessment

Owner of the strengths side: John Kretzmann and John McKnight (1993), **asset-based community development**.

Core idea: A **needs assessment** asks what a community lacks; used alone, it defines a community by its problems. An **asset-based** approach starts with what the community **already has**. Good assessments do both. **Asset mapping** inventories the gifts of **individuals**, **associ-**

ations (congregations, clubs, mutual-aid groups), and **institutions** (schools, libraries, clinics, businesses).

Needs-assessment methods: key informants (people who know the community well); **community forums** (residents describe needs in their own words); **surveys** of residents; **social indicators** (existing data such as poverty or unemployment rates); and **service utilization** (who already uses which services).

When it fits: planning a program, starting outreach, or community-level advocacy. Ask the community first.

Look-alike: program evaluation judges an existing program. A needs and strengths assessment comes **before** the program.

The APA Publication Manual (7th Edition)

Owner: the **American Psychological Association** (2020). Counselors use it for research, journal articles, grants, and many professional reports.

Bias-free language: describe people at the **appropriate level of specificity** and only when relevant; use the labels people use for themselves. **Person-first** ("a person with schizophrenia") and **identity-first** ("an autistic person") are **both acceptable**; follow the person's or group's preference. "Older adults," not "the elderly." Singular **"they"** is accepted.

Look-alikes: the **Publication Manual** sets writing style; the **ACA Code of Ethics** sets conduct. The **DSM** comes from a different APA, the **American Psychiatric Association**.

What the Exam Rewards / What It Punishes

Rewards: - Naming the owner from one signature term. - Treating a **positive screen** as a step toward **assessment**. - MBCT for **relapse prevention** in remitted recurrent depression; TF-CBT for a **child with a care-**

giver. - Building **competence before** offering biofeedback, or referring. - MBTI for **self-exploration**, with its limits named. - Starting community work with **assets and residents' voices**.

Punishes: - Calling a safety plan a no-suicide contract. - Using PHQ-9 item 9 alone as a suicide risk assessment. - Using the MBTI to hire. - MBCT as first-step treatment for an acute severe episode. - Treating somatic therapies as the best-supported PTSD treatment. - Assuming BCIA certification changes legal scope. - Treating an IFS protector as the enemy.

Swap-Pair Drill

If you see…	It's…	Not…
Managers, firefighters, exiles; 8 C's	IFS → Schwartz	Gestalt two-chair (Perls)
Cultural opportunities; way of being	MCO → Owen et al.	MCC (Sue et al.)
Teen attempt; chain analysis; relapse-prevention task	CBT-SP → Stanley, Brown, et al.	Safety Planning Intervention
Brief written plan ending in means safety	Safety Planning Intervention → Stanley & Brown	No-suicide contract
Suicide Status Form; drivers	CAMS → Jobes	CBT-SP
Decentering; 3-minute breathing space	MBCT → Segal, Williams, & Teasdale	MBSR (Kabat-Zinn)
Wise mind; "what" and "how" skills	DBT → Linehan	MBCT
Four factors; instrumental and associative learning	Krumboltz	SCCT (Lent, Brown, & Hackett)
Titration, pendulation	Somatic Experiencing → Levine	EMDR (Shapiro)

If you see…	It's…	Not…
Posture and movement; window of tolerance	Sensorimotor → Ogden	Polyvagal theory (Porges)
Voluntary biofeedback credential	BCIA	State license
PRACTICE; caregiver; trauma narrative	TF-CBT → Cohen, Mannarino, & Deblinger	CPT (Resick)
Stuck points; impact statement	CPT → Resick	PE (Foa)
Four questions plus acuity; medical setting	ASQ → NIMH	C-SSRS (Posner et al.)
16 types; four dichotomies	MBTI → Myers & Briggs	Strong
Self-scored three-letter code	SDS → Holland	Strong
Individuals, associations, institutions	Asset mapping → Kretzmann & McKnight	Program evaluation
Person-first vs. identity-first	APA Publication Manual	ACA Code of Ethics

Quick Checks

1. Whenever a client feels the old shame of being left behind, "something takes over" and he drinks until numb. In IFS terms, the drinking is: A. A manager B. A firefighter C. An exile D. The Self

2. A client in remission after four depressive episodes is referred to an 8-week group that teaches the 3-minute breathing space. The approach is: A. Mindfulness-Based Stress Reduction B. DBT mindfulness skills C. Acceptance and commitment therapy D. Mindfulness-Based Cognitive Therapy

3. Discussing a job loss, Tomás briefly mentions that his family expects him to support relatives abroad. The counselor notices and invites him to say more. In the multicul-

tural orientation framework, the counselor has: A. Engaged a cultural opportunity B. Demonstrated a multicultural competency in knowledge C. Shown cultural comfort only D. Completed a social justice advocacy intervention

4. A 9-year-old with PTSD symptoms learns relaxation and cognitive coping, then writes a trauma narrative he later shares with his foster parent. This is: A. Cognitive Processing Therapy B. Prolonged Exposure C. Trauma-Focused CBT D. Somatic Experiencing

5. In a pediatric clinic, a 16-year-old answers yes to one ASQ question and no to the acuity question. The next step is: A. Discharge with crisis line information B. A brief suicide safety assessment C. Immediate emergency hospitalization D. Repeating the PHQ-9 to confirm

6. Priya says she has no idea what she wants and feels behind her friends. The counselor reframes indecision as open-mindedness and helps her plan ways to meet people in fields she is curious about. The theory is: A. Krumboltz's happenstance learning theory B. Social cognitive career theory C. Holland's theory of vocational personalities D. Super's life-span, life-space theory

7. A therapist asks a client to notice a small tightness in his chest, shift attention to his feet on the floor, then return to the chest. The technique is: A. Imaginal exposure B. Unburdening C. Decentering D. Pendulation

8. A hiring manager asks a counselor to use the MBTI to choose among applicants. The best response reflects that the MBTI: A. Is a validated selection tool when publisher-scored B. Compares interests with those of satisfied workers C. Is designed for self-understanding and is not appropriate for hiring D. Produces a three-letter RIASEC code

9. Planning a youth program, a counselor first lists the neighborhood's congregations, sports leagues, library, and residents with useful skills. This reflects: A. Asset-based community development B. Summative program evaluation C. A social indicators analysis D. Planned happenstance

10. A counselor with no biofeedback training buys an HRV device to use with clients next week. The most accurate statement is: A. BCIA certification is federally required before any use B. The counselor should first obtain education, training, and supervised experience, or refer C. Biofeedback is outside every counselor's scope in every state D. HRV measures skin temperature, so no training is needed

Answers and Rationales

1 — B. Owner: Schwartz (IFS): firefighters are reactive protectors that douse an exile's pain fast. A: Schwartz (IFS): managers are proactive protectors that prevent triggering. C: Schwartz (IFS): the exile is the shamed part, not the drinking. D: Schwartz (IFS): the Self leads with the 8 C's; it does not numb.

2 — D. Owner: Segal, Williams, & Teasdale (MBCT): 8-week group, breathing space, relapse prevention. A: Kabat-Zinn (MBSR): parent program for stress, pain, and illness. B: Linehan (DBT): one skills module for emotion dysregulation. C: Hayes (ACT): acceptance and values work, not this protocol.

3 — A. Owner: Owen et al. (MCO): noticing and exploring an identity moment is engaging a cultural opportunity. B: Sue, Arredondo, & McDavis (MCC): a competency area, not in-session process. C: Owen et al. (MCO): comfort is the counselor's ease; the key act is taking the opportunity. D: Ratts et al. (MSJCC): advocacy is action beyond the session.

4 — C. Owner: Cohen, Mannarino, & Deblinger (TF-CBT): child, caregiver, skills, then a shared trauma narrative. A: Resick (CPT): adult cognitive work on stuck points. B: Foa (PE): adult imaginal and in vivo exposure. D: Levine (SE): titration and pendulation, no narrative protocol.

5 — B. Owner: NIMH (ASQ): a non-acute positive requires a brief suicide safety assessment to decide whether a full evaluation is needed. A: NIMH (ASQ): a positive

screen is not closed with a hotline number. C: NIMH (ASQ): emergency evaluation is for an acute positive. D: Kroenke, Spitzer, & Williams (PHQ-9): a depression measure; it does not replace the assessment.

6 — A. Owner: Krumboltz (happenstance learning theory): indecision as open-mindedness; create and use chance. B: Lent, Brown, & Hackett (SCCT): self-efficacy, outcome expectations, goals. C: Holland: matching RIASEC types to environments. D: Super: career stages and life roles.

7 — D. Owner: Levine (SE): pendulation moves attention between activation and a resource. A: Foa (PE): imaginal exposure retells the trauma memory. B: Schwartz (IFS): an exile releasing its burden. C: Segal, Williams, & Teasdale (MBCT): thoughts as mental events.

8 — C. Owner: Myers & Briggs (MBTI): a self-understanding tool with weak type stability on retest (Pittenger, 2005); not for selection. A: Myers & Briggs (MBTI): publisher scoring does not make it a hiring tool. B: Strong (Strong Interest Inventory): compares interests with satisfied workers. D: Holland (SDS): produces the three-letter code.

9 — A. Owner: Kretzmann & McKnight: asset mapping of individuals, associations, and institutions. B: Program evaluation: judges a program after it runs. C: Needs-assessment methods: social indicators use existing statistics. D: Krumboltz: planned happenstance is a career theory.

10 — B. Owner: ACA Code (2014) C.2.a and C.2.b: new specialty areas require education, training, and supervised experience. A: BCIA: certification is voluntary. C: State law: scope varies; check your state. D: Biofeedback modalities: HRV measures heart rate variation; thermal measures skin temperature.

What the Exam Rewards / What It Punishes

Rewards: - Matching strongly supported approaches to the presenting concern - Play-based and concrete methods for young children; involving caregivers - Stage-matched responses to motivation - Reflecting ambivalence instead of arguing with it - Collaborative, specific homework that gets reviewed - Supervision for countertransference

Punishes: - Action plans with a precontemplative client - Abstract cognitive work with young children - Conjoint couples work when violence is present - Advice disguised as psychoeducation - Treating a relapse as treatment failure

Swap-Pair Drill

If you see…	It's…	Not…
Face the obsession, block the compulsion	ERP	Systematic desensitization
Relaxation + imagined hierarchy	Systematic desensitization (Wolpe)	Flooding
Intense imagined exposure	Implosive therapy (Stampfl)	Flooding (real life)
Parents run play sessions at home	Filial therapy (Guerney)	Child-centered play therapy (Axline/ Landreth)
Structured review of one's whole life	Life review (Butler)	Reminiscence therapy
Parents take charge of eating	Family-based treatment (Maudsley)	Structural family therapy
Coaching parents live through an earpiece	PCIT (Eyberg)	Filial therapy
Desire, Ability, Reasons, Need	Change talk (MI)	Stages of change
"I've blown it, so I might as well keep going"	Abstinence violation effect (Marlatt)	Contemplation

If you see…	It's…	Not…
I-messages	Thomas Gordon	Ellis
Relaxation response	Herbert Benson	Edmund Jacobson (progressive relaxation)

Quick Checks

1. A client with contamination obsessions washes her hands 40 times a day. The most strongly supported intervention is: A. Psychoanalytic exploration of early toilet training B. Exposure and response prevention C. Person-centered reflection of feelings D. Life review

2. A 5-year-old has become withdrawn since his parents' separation. The most developmentally appropriate approach is: A. Cognitive restructuring of his automatic thoughts B. Child-centered play therapy C. Decisional balance exercise D. Assertiveness training

3. A client says, "My wife thinks I drink too much, but I don't see a problem." He is in which stage of change? A. Precontemplation B. Contemplation C. Preparation D. Action

4. For the client in question 3, the best counselor response is: A. "Let's set a quit date." B. "You're an alcoholic, and denial is part of the disease." C. "Your wife's concerns don't seem to match how you see it. What do you think she's noticing?" D. "Let's talk about relapse prevention."

5. A client says, "Part of me wants to stop gambling, but it's the only time I feel excited." In MI, the best response is: A. "Gambling is ruining your finances; you need to stop." B. "On one hand, gambling gives you a rush you don't get elsewhere, and on the other, part of you wants to stop." C. "Why do you keep gambling if you want to stop?" D. "Let's make a list of Gamblers Anonymous meetings."

6. After 90 days sober, a client drinks at a wedding and says, “I’ve ruined everything, so there’s no point stopping now.” This reflects: A. Precontemplation B. The abstinence violation effect C. Transference D. Sustain talk

7. A counselor finds herself feeling unusually irritated by a client who reminds her of her ex-husband. The best action is: A. Tell the client about the resemblance B. Refer the client immediately C. Seek supervision or consultation about the countertransference D. Ignore the reaction and continue

8. A counselor teaches parents to hold weekly child-centered play sessions at home. This is: A. PCIT B. Filial therapy C. Sandplay D. Structural family therapy

9. Which is an assertive response to a roommate who leaves dishes in the sink? A. Washing them silently and feeling resentful B. “You’re such a slob.” C. “When dishes stay in the sink overnight, I feel frustrated because the kitchen gets crowded. Could we agree to wash them by bedtime?” D. “Wow, nice of you to leave these for me again.”

10. A 78-year-old client is struggling to find meaning after retirement. Which approach is best matched to her developmental stage? A. Life review B. PCIT C. Exposure therapy D. Filial therapy

11. A couple seeks counseling. During intake, one partner discloses being afraid of the other, who controls the finances and monitors her phone. The best next step is: A. Begin conjoint sessions focused on communication B. Assess safety individually before deciding whether couples counseling is appropriate C. Teach both partners “I” statements D. Use enactment to observe the couple’s conflict

12. Which intervention for panic disorder deliberately produces the physical sensations the client fears? A. Systematic desensitization B. Interoceptive exposure C. Flooding D. Implosive therapy

Answers and Rationales

1 — B. ERP has the strongest support for OCD symptoms. A: psychoanalysis (Freud) is a real therapy but a poor match. C: person-centered work (Rogers) doesn't address the compulsion cycle. D: life review (Butler) is a late-life approach.

2 — B. Play is a young child's language (Axline, Landreth). A: abstract restructuring is beyond a preoperational child (Piaget). C: decisional balance is an MI/TTM tool for adults weighing change. D: assertiveness training is a skills intervention for adolescents and adults.

3 — A. No acknowledgment of a problem is precontemplation (Prochaska & DiClemente). B: contemplation includes some acknowledgment and ambivalence. C: preparation includes intent and a plan. D: action involves changed behavior.

4 — C. Raising awareness gently, without arguing, fits precontemplation. A: action-stage move. B: labeling and confrontation increase resistance. D: maintenance-stage move.

5 — B. A double-sided reflection of ambivalence is a core MI skill (Miller & Rollnick). A: the righting reflex. C: a "why" question invites defensiveness. D: an action-stage referral before the client has resolved ambivalence.

6 — B. The all-or-nothing reaction to a lapse is Marlatt's abstinence violation effect. A: precontemplation is not recognizing a problem. C: transference involves feelings toward the counselor. D: sustain talk is MI's term for any argument against changing; this item describes a specific reaction to a lapse.

7 — C. Countertransference is managed through supervision or consultation. A: shifts the burden to the client. B: premature; the counselor's reaction can usually be managed. D: unmanaged countertransference can harm the client.

8 — B. Training parents to run play sessions is filial therapy (Guerney). A: PCIT (Eyberg) coaches parents live during interaction, with a behavioral focus. C: sandplay (Kalff) is a therapist-run sand tray method. D: structural therapy (Minuchin) works on boundaries and hierarchy.

9 — C. Specific behavior, feeling, impact, request: an assertive "I" statement. A: passive. B: aggressive. D: passive-aggressive.

10 — A. Life review (Butler) fits Erikson's integrity vs. despair. B and D: child-focused parent interventions. C: exposure targets fear and avoidance.

11 — B. Signs of coercive control make conjoint work potentially dangerous; assess safety individually first. A, C, D: all proceed with couples work before safety is assessed. D is also Minuchin's technique, which could escalate risk.

12 — B. Interoceptive exposure targets feared body sensations. A: systematic desensitization (Wolpe) uses imagined scenes with relaxation. C: flooding is intense real-life exposure to external situations. D: implosive therapy (Stampfl) is intense imagined exposure.

Audio Companion

Track	Covers	Target length
5.1	Match It: problem, age, and stage → intervention	~10 min

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Chapter 6 — Crisis, Safety, and Risk

2027 task map: 4.10 Provide crisis intervention and de-escalation · 2B.2 Assess risk to self and others · 6.2 Adhere to internal policies or standard operating procedures (SOPs) relevant to your clinical practice (e.g., informed consent, record retention, release of information, mandated reporting protocols) · 2A.1 Discuss and obtain informed consent/assent from client or guardian · 5.2 Collaborate with non-clinical stakeholders (e.g., legal counselor, community resources) · 3B.1 Refer clients and/or family members to community resources or to specialists · CACREP: Counseling and Helping Relationships; Professional Counseling Orientation and Ethical Practice

Safety Outranks Everything

Chapter 1 gave you six priorities for applied items. The first one was safety. This chapter is where that rule does most of its work.

When any answer option addresses a risk to life, it usually beats every option that doesn't, no matter how good the others are clinically. Building rapport, exploring feelings, and staying on the treatment plan all wait until safety is addressed.

The NCE tests risk in five forms:

1. **Crisis** — a person overwhelmed and unable to cope
2. **Suicide risk** — assessment, level of care, and safety planning
3. **Harm to others** — threats, homicide risk, and the duty to protect
4. **Abuse** — children, older adults, and dependent adults; mandated reporting

5. **Intimate partner violence** — assessment and safety planning with the person at risk

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Part 1 — Crisis Theory and Intervention

Who Owns Crisis Theory

Owner	Contribution
Erich Lindemann	Studied survivors' grief after the 1942 Cocoanut Grove nightclub fire; described acute grief reactions; founded the field of crisis intervention
Gerald Caplan	Preventive psychiatry; crisis as a time-limited state (often described as about 4–6 weeks) when usual coping fails; primary, secondary, and tertiary prevention
Albert Roberts	Seven-stage crisis intervention model
James & Gilliland	Six-step crisis intervention model; a person in crisis experiences both danger and opportunity
Kenneth Doka	Disenfranchised grief: loss that isn't socially recognized (e.g., death of an ex-partner, pregnancy loss, a pet)

What a Crisis Is

A crisis is not the event. It is a person's response when an event overwhelms their usual ways of coping. The same event can be a crisis for one person and a manageable stressor for another.

Key features: - **Time-limited.** Crises resolve, for better or worse, within weeks. - **Danger and opportunity.** A crisis can lead to lower functioning or to growth. - **Heightened openness.** People in crisis are often more receptive to help than at other times.

Type	Example
Developmental	Predictable life transitions: birth of a child, retirement, adolescence
Situational	Sudden, unexpected events: accident, job loss, assault, sudden death
Existential	Inner conflicts about meaning, purpose, or identity
Ecosystemic	Disasters affecting whole communities: hurricane, pandemic, mass violence

Roberts's Seven-Stage Model

1. Plan and conduct a thorough **biopsychosocial and lethality/imminent danger assessment**
2. **Make psychological contact** and rapidly establish the relationship
3. Identify the **major problems**, including crisis precipitants
4. Encourage exploration of **feelings and emotions** (active listening, validation)
5. Generate and explore **alternatives and new coping strategies**
6. Restore functioning through an **action plan**
7. Plan **follow-up** and booster sessions

James & Gilliland's Six-Step Model

Listening steps: 1. Define the problem 2. Ensure client safety 3. Provide support

Acting steps: 4. Examine alternatives 5. Make plans 6. Obtain commitment

Safety assessment runs *throughout* all six steps, not just at step 2.

How Crisis Counseling Differs

Ongoing counseling	Crisis intervention
Explores history and patterns	Focuses on the here-and-now and the immediate problem
Counselor is often less directive	Counselor is **more directive** as needed
Long-term goals	Goal: **restore pre-crisis functioning** and safety
Weekly sessions	Brief, immediate, often a single contact
Insight is a goal	Stabilization comes first; insight can come later

The NCE's crisis rule: in an acute crisis, a more directive response is correct. An option that explores childhood patterns or begins long-term goal setting during an active crisis is a wrong answer.

Psychological First Aid (PFA)

PFA is an evidence-informed approach for the hours and days after a disaster or traumatic event (National Child Traumatic Stress Network and National Center for PTSD). Core actions:

- Contact and engagement
- Safety and comfort
- Stabilization (if needed)
- Information gathering on current needs
- Practical assistance
- Connection with social supports
- Information on coping
- Linkage with collaborative services

PFA is not debriefing. Forcing survivors to describe the event in detail soon after it happens (single-session psychological debriefing) is not recommended and may increase distress for some people. PFA does not push people to retell the trauma.

Part 2 — Suicide Risk

Myths the Exam Tests

Myth	Fact
Asking about suicide puts the idea in someone's head	Asking directly does not increase risk and is the standard of care
People who talk about suicide won't do it	Many people who die by suicide told someone first
A sudden improvement in mood means the danger has passed	A sudden calm can follow a decision to die; reassess
A no-suicide contract keeps a client safe	No-suicide contracts have no evidence of preventing suicide and can give the counselor false reassurance; use a safety plan
Suicidal people want to die	Most are ambivalent; part of them wants to live

Who Owns Suicide Theory

Owner	Contribution
Émile Durkheim	Sociological types of suicide: **egoistic** (too little social integration), **altruistic** (too much integration; self-sacrifice for the group), **anomic** (sudden disruption of social norms or regulation, e.g., economic collapse), **fatalistic** (excessive regulation and oppression)
Edwin Shneidman	Founder of suicidology; **psychache** (intolerable psychological pain); psychological autopsy
Thomas Joiner	Interpersonal theory of suicide: **thwarted belongingness + perceived burdensomeness** produce the desire for death; **acquired capability** (reduced fear of death, increased pain tolerance) makes an attempt possible

Owner	Contribution
Aaron Beck	Hopelessness as a stronger predictor of suicide than depression itself; Beck Hopelessness Scale; Beck Scale for Suicide Ideation
Kelly Posner and colleagues	Columbia–Suicide Severity Rating Scale (C-SSRS)
Barbara Stanley & Gregory Brown	Safety Planning Intervention
David Jobes	CAMS (Collaborative Assessment and Management of Suicidality)
Patterson, Dohn, Bird, & Patterson	SAD PERSONS mnemonic (a teaching tool; not validated for predicting risk on its own)

Assessing Risk

Ask directly, calmly, and in plain language: **"Are you having thoughts of killing yourself?"** Avoid euphemisms like "hurting yourself," which a client may not interpret as suicide.

Then assess, starting with ideation and escalating through plan, intent, and means:

Element	Questions
Ideation	Passive ("I wish I wouldn't wake up") or active ("I want to kill myself")? How often, how long, how intense?
Plan	Is there a plan? How specific (method, time, place)?
Intent	Does the client intend to act?
Means	Does the client have access to the method? Firearms in the home?
Preparatory behavior	Rehearsing, giving away possessions, writing a note, saying goodbyes, acquiring means

Element	Questions
History	Prior attempts (the strongest single predictor of a future attempt); family history of suicide
Current state	Hopelessness, intoxication, agitation, insomnia, recent loss, isolation
Protective factors	Reasons for living, children, faith, social support, engagement in treatment, future plans

Warning signs (the American Association of Suicidology's IS PATH WARM mnemonic): Ideation, **S**ubstance use, **P**urposelessness, **A**nxiety, **T**rapped, **H**opelessness, **W**ithdrawal, **A**nger, **R**ecklessness, **M**ood change.

Demographic patterns the NCE may test: men die by suicide at higher rates; women make more nonfatal attempts. Firearms account for a large share of suicide deaths in the United States, which is why means restriction matters. Check current CDC data before quoting specific rates; they change.

Matching Level of Care to Risk

Risk	Typical picture	Typical response
Low	Passive ideation; no plan or intent; protective factors present	Collaborative safety plan; monitor at every session; address contributing problems
Moderate	Active ideation; vague plan; no clear intent; some risk factors	Safety plan; means restriction; increase session frequency; involve supports (with consent); consider a higher level of care
High	Specific plan, intent, access to means, recent attempt or preparation, intoxication	Do not leave the client alone; arrange immediate evaluation for hospitalization; voluntary admission if possible

Risk	Typical picture	Typical response
Imminent	States intent to act now and has means	Emergency services; stay with the client; involuntary hold if the client won't accept voluntary care

Least restrictive appropriate care: the NCE doesn't reward hospitalizing every client who mentions suicide. It rewards matching the response to the risk. Under-responding to high risk is wrong. Over-responding to low risk (hospitalization for passive ideation with strong protective factors) is also wrong.

The Safety Plan (Stanley & Brown)

A safety plan is written **with** the client, in the client's words, and kept where the client can reach it. The six steps:

1. **Warning signs** — thoughts, moods, situations, or behaviors that signal a crisis is starting
2. **Internal coping strategies** — things the client can do alone to take their mind off the crisis
3. **People and social settings that provide distraction** — contacts and places that help, without needing to discuss the crisis
4. **People the client can ask for help** — family or friends who can be told about the crisis
5. **Professionals and agencies to contact** — counselor, after-hours line, local emergency services, and the **988 Suicide & Crisis Lifeline** (call or text 988 in the United States)
6. **Making the environment safe** — reducing access to lethal means

Many counselors also add the client's **reasons for living** to the plan. Review and update the plan at later sessions.

Means restriction counseling (also called lethal means counseling) is a strongly supported prevention strategy: helping the client and family limit access to firearms,

stockpiled medications, and other lethal means during periods of risk. Many suicidal crises are brief, and putting time and distance between a person and a lethal method saves lives.

Self-Injury Without Suicidal Intent

Non-suicidal self-injury (NSSI) is deliberate self-harm *without* the intent to die, often used to regulate overwhelming emotion. It still requires a suicide risk assessment: NSSI is associated with higher later suicide risk, and the two can coexist. Respond calmly and without judgment, assess medical needs and suicide risk, and work on alternative ways of coping (DBT skills have strong support). Don't demand that the client stop immediately or make continued counseling conditional on stopping.

Part 3 — Harm to Others and the Duty to Protect

Tarasoff

Case	Holding
***Tarasoff v. Regents of the University of California* (1974)** — "Tarasoff I"	Therapists have a **duty to warn** an identifiable victim of a client's serious threat
***Tarasoff v. Regents of the University of California* (1976)** — "Tarasoff II"	Broadened to a **duty to protect**: take reasonable steps to protect the intended victim, which may include warning, notifying police, or arranging hospitalization
***Jablonski v. United States* (1983)**	Duty can extend to a foreseeable victim based on the client's history of violence, even without an explicit threat
***Ewing v. Goldstein* (2004)**	A credible threat reported to the therapist by a client's family member can trigger the duty (California)

State law varies. Some states *require* counselors to act (mandatory duty), some *permit* disclosure without requiring it, and a few have no statute. The NCE tests the principle, not your state's law.

The general triggers: - A **serious threat of physical violence** - Against a **reasonably identifiable victim** - That the counselor believes, or should believe, is credible

What the counselor does: assess the threat (plan, means, history of violence, intent); consult with a supervisor or colleague and document; take protective action required or permitted by law; continue to treat the client if safe to do so. Disclose only the information needed to protect the person.

Assessing Risk of Violence

Ask about: intent, plan, access to weapons, identifiable target, history of violence (the strongest predictor of future violence), substance use, impulsivity, recent losses or humiliations, and protective factors.

Part 4 — Abuse and Mandated Reporting

Child Abuse and Neglect

- Counselors are **mandated reporters** in every U.S. state.
- The standard is **reasonable suspicion**, not proof. **Don't investigate.** That's the job of child protective services and law enforcement.
- Report **promptly**, within the time your state requires.
- Mandated reporters who report in **good faith** are protected from liability.
- Reporting is an **exception to confidentiality**. Explain this limit during informed consent, before it comes up.
- **Neglect** is the most commonly reported form of child maltreatment.

Type	Possible signs
Physical abuse	Unexplained or patterned injuries; explanations that don't fit the injury; fear of going home
Sexual abuse	Sexual knowledge or behavior beyond developmental level; genital pain; sudden behavior change
Emotional abuse	Extreme withdrawal or aggression; persistent belittling by caregiver
Neglect	Chronic hunger; poor hygiene; untreated medical needs; frequent absences; lack of supervision

When a child discloses: stay calm, listen, believe them, don't ask leading questions, don't promise to keep it secret, tell the child what happens next in age-appropriate words, document the child's words, and report.

Older Adults and Dependent Adults

Most states require or permit reporting suspected abuse, neglect, or exploitation of older or dependent adults to adult protective services. Types include physical, sexual, emotional, financial exploitation, neglect by a caregiver, and **self-neglect**. Requirements vary by state more than child abuse laws do; know your state's.

What Is *Not* Usually Reportable

- **Past abuse disclosed by an adult client about their own childhood**, when no child is currently at risk, is generally not a mandated report (check state law; in some states the answer changes if the abuser has current access to children).
- **Intimate partner violence between competent adults** is generally not a mandated report to protective services (some states require health care providers to report certain injuries). The counselor's role is assessment and safety planning with the client.

Part 5 — Intimate Partner Violence (IPV)

Owner	Contribution
Lenore Walker	Cycle of violence: tension-building → acute battering incident → honeymoon (reconciliation/calm)
Domestic Abuse Intervention Programs (Duluth, MN)	Power and Control Wheel
Jacquelyn Campbell	Danger Assessment instrument for lethality risk in IPV
Evan Stark	Coercive control

The Exam's IPV Rules

1. **Screen privately.** Ask about safety only when the partner is not in the room.
2. **Couples counseling is contraindicated** while there is ongoing violence or coercive control. It can increase danger and prevent the victim from speaking freely.
3. **Respect the client's autonomy.** Leaving is often the most dangerous time. The counselor helps the client assess risk and plan, without pressuring the client to leave on the counselor's timeline.
4. **Assess lethality.** High-risk indicators include prior strangulation, threats to kill, access to a firearm, escalating frequency or severity, extreme jealousy, and the victim's own sense that the partner could kill them.
5. **Safety planning with the victim:** emergency contacts, a safe place to go, important documents and money ready, a code word with trusted people, the National Domestic Violence Hotline, and local shelter resources.
6. **Batterer intervention programs**, not anger management alone, are the standard referral for those who use violence.

Part 6 — Documentation and Consultation

In every risk situation: - **Consult** with a supervisor or colleague when possible. - **Document** the assessment, the reasoning behind your decision, the actions taken, and the consultation. - **Follow up.** Check the safety plan, confirm the client reached the referral, and reassess at the next contact.

“If it isn’t documented, it didn’t happen” is a guiding principle in risk management.

What the Exam Rewards / What It Punishes

Rewards: - Asking about suicide directly - Safety plans and means restriction over no-suicide contracts - Matching the level of care to the level of risk - A more directive stance during an acute crisis - Reporting on reasonable suspicion, promptly, without investigating - Assessing IPV privately and declining conjoint work when violence is present - Consultation and documentation

Punishes: - Avoiding the word “suicide” or relying on euphemisms - Hospitalizing every client who mentions suicide, or none of them - Exploring childhood history during an active crisis - Promising a child to keep abuse secret - Waiting for proof before reporting abuse - Pressuring an IPV victim to leave immediately - Forcing trauma retelling right after a disaster

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Swap-Pair Drill

If you see…	It belongs to…	Not…
Cocoanut Grove fire; acute grief	Lindemann	Caplan
Preventive psychiatry; crisis lasts about 4–6 weeks	Caplan	Lindemann
Seven-stage crisis intervention model	Roberts	James & Gilliland

If you see…	It belongs to…	Not…
Six-step crisis model: listening, then acting	James & Gilliland	Roberts
Anomic suicide	Durkheim	Shneidman
Psychache	Shneidman	Joiner
Thwarted belongingness; perceived burdensomeness	Joiner	Durkheim
Hopelessness predicts suicide	Beck	Joiner
Safety Planning Intervention	Stanley & Brown	Jobes (CAMS)
Duty to warn (1974)	Tarasoff I	Tarasoff II (duty to protect, 1976)
Tension → battering → honeymoon	Walker	Duluth (Power and Control Wheel)
Danger Assessment	Campbell	Posner (C-SSRS, suicide)
Coercive control	Stark	Walker
Grief not socially recognized	Doka (disenfranchised grief)	Worden

Quick Checks

1. A client says, “Sometimes I think everyone would be better off without me.” The counselor’s best next response is: A. “You have so much to live for.” B. “Are you having thoughts of killing yourself?” C. “Let’s talk about your childhood.” D. “Will you promise not to hurt yourself before our next session?”

2. Which is the strongest single predictor of a future suicide attempt? A. Female gender B. A previous suicide attempt C. Being married D. Anxiety

3. A client with passive ideation, no plan, no intent, strong family support, and no prior attempts is best served by: A. Involuntary hospitalization B. A collaborat-

ive safety plan and continued outpatient counseling with ongoing monitoring C. A no-suicide contract D. Referral to a psychiatrist and ending counseling

4. A depressed client who has been withdrawn for weeks arrives calm and cheerful, and mentions he gave his guitar to his brother. The counselor should: A. Celebrate the improvement and reduce session frequency B. Reassess suicide risk directly C. Begin termination planning D. Explore his relationship with his brother

5. Which theorist proposed that thwarted belongingness and perceived burdensomeness create the desire for suicide? A. Durkheim B. Shneidman C. Joiner D. Beck

6. A client says he's going to "make sure" his ex-girlfriend's new partner "pays," and that he knows where the man works and owns a gun. The counselor's best action is to: A. Keep the information confidential because the client hasn't acted B. Assess the threat, consult, document, and take protective action consistent with state law C. Terminate counseling immediately D. Confront the client about his jealousy

7. A 9-year-old tells her school counselor that her stepfather hits her with a belt and asks her not to tell anyone. The counselor should: A. Promise to keep it secret to protect the relationship B. Interview the stepfather to confirm the story C. Report to child protective services and explain to the child, in simple terms, what will happen D. Wait until there is physical evidence

8. A woman in individual counseling discloses that her husband has choked her twice and recently threatened to kill her. He has a gun. The counselor's priority is to: A. Recommend couples counseling B. Assess lethality and develop a safety plan with her C. Insist she leave the house tonight D. Report the husband to child protective services

9. A client calls in crisis after a house fire. Using James & Gilliland's model, which step comes first? A. Examine al-

ternatives B. Obtain commitment C. Define the problem D. Make plans

10. Which is the most appropriate approach in the first days after a community disaster? A. Mandatory group debriefing in which every survivor describes the event in detail B. Psychological first aid C. Long-term psychodynamic therapy D. Exposure therapy

11. A client cuts her arms to "feel something" when overwhelmed but says she doesn't want to die. The counselor should: A. Tell her counseling will stop if she cuts again B. Accept her statement and skip suicide assessment C. Assess suicide risk and medical needs, and work on alternative coping skills D. Hospitalize her immediately

12. The 1976 Tarasoff decision established that therapists have a duty to: A. Keep all threats confidential B. Protect an intended victim of a client's serious threat C. Report all clients with violent histories to police D. Warn the client's family about any threat

Answers and Rationales

1 — B. Indirect statements of being a burden call for a direct question about suicide. A: reassurance that dismisses the feeling. C: exploring history before assessing safety. D: a no-suicide contract instead of an assessment.

2 — B. A prior attempt is the strongest single predictor. A: women attempt more often, but gender alone isn't the strongest predictor, and men die by suicide at higher rates. C: being married is generally protective. D: anxiety can raise risk but is not the strongest predictor.

3 — B. Low risk matched to a low-intensity, collaborative response. A: overly restrictive for low risk. C: no-suicide contracts lack evidence and can create false reassurance. D: abandons the client.

4 — B. Sudden calm plus giving away a possession are warning signs; reassess directly. A and C: treat a warning sign as improvement. D: explores before assessing safety.

5 — C. Joiner's interpersonal theory. A: Durkheim's types are sociological (egoistic, altruistic, anomic, fatalistic). B: Shneidman owns psychache. D: Beck owns hopelessness as a predictor.

6 — B. A serious threat, an identifiable victim, and access to a weapon can trigger the duty to protect; assess, consult, document, and act per state law. A: confidentiality has limits. C: abandonment doesn't protect anyone. D: misses the safety issue.

7 — C. Reasonable suspicion triggers a mandated report; tell the child honestly what comes next. A: counselors can't promise secrecy about abuse. B: investigation is CPS's role and could put the child at greater risk. D: proof isn't required.

8 — B. Strangulation, threats to kill, and a firearm are high-lethality indicators (Campbell); assess and plan for safety. A: conjoint work is contraindicated. C: pressuring her to leave on the counselor's timeline ignores her autonomy and the danger of leaving without a plan. D: CPS is for children; nothing here indicates a child at risk.

9 — C. Defining the problem is step 1 of James & Gilliland's model. A: step 4; D: step 5; B: step 6 (the acting steps).

10 — B. PFA is the recommended early response. A: forced detailed retelling soon after trauma isn't recommended and may increase distress for some. C and D: not first-response approaches in the immediate aftermath.

11 — C. NSSI still requires suicide and medical assessment, plus skill-building (DBT). A: an ultimatum harms the alliance and doesn't build skills. B: NSSI and suicide risk can coexist. D: over-response without assessment.

12 — B. Tarasoff II (1976) established the duty to protect. A: the opposite. C: a history of violence alone doesn't create a duty to report. D: the duty runs to the intended victim, not the client's family.

Audio Companion

Track	Covers	Target length
6.1	Safety First: crisis models, suicide assessment, duty to protect, reporting	~8 min

Note to readers: If you or someone you know is in crisis, call or text 988 (Suicide & Crisis Lifeline, United States) or call 911.

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Chapter 7 — Couples and Family Systems in Session

2027 task map: 4.1 Engage in couples/family dynamic counseling · 4.4 Facilitate clients' ability to navigate interpersonal relationships · 2B.8 Use formal (e.g., assessment results) and informal (e.g., body language) observations · 4.7 Implement interventions aligned with service modality · CACREP: Counseling and Helping Relationships; Human Growth and Development

A Different Way of Seeing

Chapter 2 gave you the owners of the family therapy models: Bowen, Minuchin, Haley, MRI, Milan, Satir, Whitaker, Johnson, and Gottman. This chapter teaches what they share and how the NCE tests it in session.

The shift is this: **in systems work, the client is the relationship or the family, not one person.** A teenager's behavior problem is not seen as a defect inside the teenager. It is seen as part of a pattern that the whole family keeps going, often without meaning to. The NCE rewards answers that look at the pattern and punishes answers that blame one member.

Systems Concepts and Their Owners

Concept	Owner	Meaning
General systems theory	Ludwig von Bertalanffy	A system is more than the sum of its parts; parts are understood through their relationships
Cybernetics	Norbert Wiener	The study of feedback and self-regulation in systems

Concept	Owner	Meaning
Double bind	Gregory Bateson (with Jackson, Haley, Weakland)	Contradictory messages at different levels, with no way to comment on or leave the contradiction
Homeostasis	Don Jackson (applying Cannon's term)	Families resist change and work to restore their usual balance
Second-order cybernetics	Heinz von Foerster	The observer (counselor) is part of the system being observed

Core Vocabulary

Term	Meaning	Exam example
Wholeness	The family is more than the sum of its members	Change one member, and the whole family is affected
Circular causality	Behaviors cause and are caused by each other in loops	"She nags because he withdraws; he withdraws because she nags"
Linear causality	A causes B (the view systems thinking replaces)	"He withdraws because she nags"
Negative feedback	Reduces deviation; keeps the system the same (morphostasis)	Parents tighten rules when a teen pushes limits
Positive feedback	Amplifies deviation; pushes the system toward change (morphogenesis)	An argument that escalates with each reply
Morphostasis vs. morphogenesis	Stability vs. change	Healthy families need both
Equifinality	Different starting points can lead to the same outcome	Two very different families can end up with the same problem
Multifinality	The same starting point can lead to different outcomes	Two siblings respond to the same divorce differently

Term	Meaning	Exam example
Identified patient (IP)	The member whose symptoms bring the family to counseling	The symptom often serves a function for the family
First-order change	Change within the system's rules; the pattern stays	Grounding a teen for a week instead of a day
Second-order change	Change to the system's rules themselves	The parents start agreeing on rules together
Open vs. closed systems	Exchange with the outside world vs. isolation	Closed families resist outside help and new information

"Negative" and "positive" feedback describe direction, not value. Negative feedback is not bad; it keeps things stable. Positive feedback is not good; it can escalate a conflict. This is the same trap as negative reinforcement in Chapter 2.

Who Is the Client? Ethics in Couple and Family Work

Systems work changes several ethical defaults (ACA Code of Ethics, 2014: A.8, Multiple Clients, and B.4.b, Couples and Family Counseling).

Issue	Rule
Identifying the client	At the start, clarify who the clients are and the counselor's relationship with each person
Informed consent	Every participating member consents; minors assent, and parents or guardians consent
Secrets policy (B.4.b)	Decide and explain at the start: a **no-secrets** policy (individually shared information may be brought into joint sessions) or a **limited-secrets** policy. Explain it *before* anyone shares a secret

Issue	Rule
Confidentiality	Each person's right to confidentiality is discussed; the counselor can't promise what other members will keep private
Conflicting roles	If the counselor may be asked to serve in conflicting roles (e.g., testifying in a custody dispute), clarify, adjust, or withdraw
Neutrality	The counselor doesn't take sides or form alliances with one member (Bowen: stay out of the triangle; Milan: neutrality)
Safety	Couples work is contraindicated when there is ongoing violence or coercive control (Chapter 6)

When a partner discloses an affair in an individual session and the counselor has a no-secrets policy, the scored answer is usually to encourage and support that partner in disclosing it in a joint session, and to follow the policy already explained. The wrong answers: promising to keep the secret despite a no-secrets policy, revealing it in a joint session without warning the partner, or ending counseling.

Assessing the Family

Tool	Owner	What it shows
Genogram	Bowen (concept); standardized by Monica McGoldrick & Randy Gerson	Three or more generations: relationships, patterns, cutoffs, illnesses, substance use, conflict
Ecomap	Ann Hartman	The family's connections to outside systems: school, work, church, extended family, agencies, friends

Tool	Owner	What it shows
Family sculpting	Satir (also Duhl, Kantor, & Duhl)	Members physically arrange each other to show closeness, distance, and power
Family mapping	Minuchin	Diagrams of boundaries, subsystems, coalitions
Circumplex Model; FACES	David Olson	Two dimensions: **cohesion** (disengaged → enmeshed) and **flexibility** (rigid → chaotic); balanced families are in the middle range of both
McMaster Model of Family Functioning	Nathan Epstein and colleagues	Six dimensions: problem solving, communication, roles, affective responsiveness, affective involvement, behavior control
Beavers Systems Model	W. Robert Beavers	Family competence and style

Genogram vs. ecomap: the genogram looks **across generations** inside the family. The ecomap looks **outward** at the family's connections to the community and support network (4.4).

Common Genogram Conventions

Symbol	Meaning
Square	Male
Circle	Female
X inside a symbol	Death
Horizontal line between partners	Marriage
One slash across the line	Separation
Two slashes across the line	Divorce

Symbol	Meaning
Dashed line	Cohabiting or committed relationship without marriage
Double or triple lines between people	Close or fused relationship
Zigzag line	Conflict
Broken line with gap	Cutoff
Double outline around a symbol	The identified patient (index person)

The Family Life Cycle

Families move through predictable stages, and symptoms often appear when a family struggles to make a transition (Carter & McGoldrick; McGoldrick, Garcia Preto, & Carter).

Stage	Key emotional process	Second-order changes required
1. Leaving home: single young adults	Accepting emotional and financial responsibility for self	Differentiation from family of origin; intimate peer relationships; work and financial independence
2. Joining of families through marriage/ union	Commitment to a new system	Forming the couple system; realigning relationships with extended families and friends
3. Families with young children	Accepting new members into the system	Making space for children; taking on parenting roles; realigning with grandparents
4. Families with adolescents	Increasing flexibility of boundaries to allow adolescents' independence	Shifting parent–child relationships; refocus on midlife couple and career issues; beginning care for older generation

Stage	Key emotional process	Second-order changes required
5. Launching children and moving on	Accepting many exits from and entries into the family	Renegotiating the couple as a dyad; adult-to-adult relationships with grown children; in-laws and grandchildren; coping with parents' disability and death
6. Families in later life	Accepting the shifting of generational roles	Maintaining functioning in the face of decline; supporting a more central role for the middle generation; dealing with loss of spouse, siblings, peers

This is the classic six-stage version of the model (Carter & McGoldrick). Newer editions (McGoldrick, Garcia Preto, & Carter, 2016) divide the later stages more finely, but the exam-level point is the same: symptoms cluster at transitions. Divorce and remarriage add their own stages and tasks (Chapter 13).

Exam use: when a family's symptoms begin right around a transition (a first child, a teen's push for independence, the last child leaving), the systemic answer frames the problem as difficulty making the transition, not as an individual disorder.

Running a Family or Couple Session

Haley's Structured First Interview

Haley described stages for a first family session:

1. **Social stage** — greet each member; help everyone feel comfortable; observe seating and interactions
2. **Problem stage** — ask *each* member for their view of the problem

3. **Interaction stage** — have family members talk to each other about the problem while the counselor observes
4. **Goal-setting stage** — define a clear, solvable goal
5. **Task-setting stage** — give a directive (homework) to move toward the goal

Joining Comes First

Every family model starts by **joining**: connecting with each member, accommodating the family's style, and earning trust. Minuchin named it, but all systemic approaches require it.

- Greet and engage **every** member, including young children and quiet members.
- Don't side with the person who called for the appointment.
- Respect the family's hierarchy and culture at first, even if you will later help them change it.

Systemic Interventions by Purpose

Purpose	Intervention	Owner
See the pattern live	**Enactment** — family members interact in session	Minuchin
Follow the sequence	**Tracking** — ask who does what, then what happens next	Structural/strategic
Show relationships and positions	**Sculpting**	Satir
Reveal multigenerational patterns	**Genogram**, process questions	Bowen
Draw out how members see each other	**Circular questions** ("When Dad gets quiet, what does Mom do?")	Milan
Change meaning	**Reframing** ("Her nagging may be her way of trying to stay close to you")	MRI, structural, strategic

Purpose	Intervention	Owner
Change structure	**Boundary making**, **unbalancing**	Minuchin
Interrupt a stuck pattern	**Directives**, **prescribing the symptom**	Haley; MRI
Lower reactivity	**Detriangulation**; calm "I" positions	Bowen
Change the problem story	**Externalizing** the problem	White & Epston
De-escalate a couple's cycle	Identify the pursue–withdraw cycle; access primary emotions	Johnson (EFT)
Strengthen friendship and repair	Softened startup; repair attempts; love maps	Gottman

Circular Questions vs. Linear Questions

Linear (individual focus)	Circular (systemic focus)
"Why is Jamal acting out?"	"When Jamal acts out, who gets most upset? What does Dad do then?"
"How do you feel about your husband's drinking?"	"When your husband drinks, who in the family notices first? Who tries to fix it?"
"What's wrong with your daughter?"	"If your daughter's problem disappeared tomorrow, how would things change between the two of you?"

Circular questions put the pattern, not the person, in the center.

Family of Origin and Extended Family (4.1)

- **Family of origin work** explores how patterns from the families partners grew up in shape the current relationship (Bowen's multigenerational transmission; Satir's family reconstruction).

- **Extended family** may be a source of support or stress: in-law conflict, grandparent involvement, triangulation of a child into a grandparent's conflict.
- **Culture shapes what "healthy" looks like.** In many cultures, close involvement of extended family, shared living, and strong parental authority are normal and protective. Labeling them "enmeshment" without considering culture is a scored error.

Triangles in Practice

When two people in conflict draw in a third to reduce tension, that's a triangle (Bowen). Examples: a parent who confides in a child about the other parent; a couple who only talk about the child's problems; a mother-in-law pulled into every argument.

The counselor's job: notice the triangle, stay out of it, and help the original pair deal directly with each other.

Couples Work

Communication and Conflict Patterns

Pattern	Owner	Description
Four Horsemen	Gottman	Criticism, contempt, defensiveness, stonewalling. **Contempt** is identified as the strongest predictor of divorce
Antidotes	Gottman	Gentle startup (for criticism); building a culture of appreciation (for contempt); taking responsibility (for defensiveness); physiological self-soothing (for stonewalling)

Pattern	Owner	Description
Pursue–withdraw (demand–withdraw)	Johnson (EFT); also Christensen	One partner pursues connection or change; the other withdraws; each reaction intensifies the other
Communication stances	Satir	Placater, blamer, super-reasonable, irrelevant; the goal is congruent communication
Speaker–listener technique	PREP (Markman, Stanley, & Blumberg)	One speaks briefly; the other paraphrases before replying

Integrative Behavioral Couple Therapy (IBCT)

Andrew Christensen and Neil Jacobson. Combines change strategies (communication and problem-solving training) with **acceptance** strategies (empathic joining around the problem; unified detachment; tolerance building). Some differences between partners won't change; acceptance of them is a goal.

The Exam's Couples Rules

1. **Assess safety privately** before conjoint work (Chapter 6).
2. **Stay neutral.** Don't side with the partner who seems more reasonable.
3. **Focus on the pattern**, not on who's right.
4. **Set the secrets policy** at the start.
5. **Have partners talk to each other**, not just to the counselor (enactment).

Culture and Family Composition (4.1)

- Families come in many forms: single-parent, blended, multigenerational, same-sex parents, foster and adoptive, grandparents raising grandchildren, chosen family. Don't assume a two-parent nuclear model.

- **Acculturation gaps** (Chapter 3, Berry) between immigrant parents and children raised in the host culture are a common source of conflict. Frame them as a difference in acculturation, not as defiance or rigidity.
- Ask about the family's own beliefs about roles, hierarchy, decision-making, and help-seeking before applying a model built on one culture's norms.
- **Broaching**: openly acknowledging cultural differences between counselor and family can strengthen trust.

What the Exam Rewards / What It Punishes

Rewards: - Seeing the problem as a pattern; using circular questions - Joining with every member before attempting change - Clarifying who the client is and the secrets policy at the start - Staying neutral and out of triangles - Using the genogram for generational patterns and the ecomap for outside supports - Framing symptoms around life cycle transitions - Considering culture before labeling closeness as enmeshment

Punishes: - Treating the identified patient as "the problem" - Siding with one partner or parent - Keeping secrets that contradict the stated policy - Starting conjoint work without screening for violence - Linear "why" questions about one member

Swap-Pair Drill

If you see…	It's…	Not…
General systems theory	von Bertalanffy	Wiener (cybernetics)
Observer is part of the system	Second-order cybernetics (von Foerster)	First-order
"She nags because he withdraws because she nags"	Circular causality	Linear causality

If you see…	It's…	Not…
Feedback that keeps things the same	Negative feedback (morphostasis)	Positive feedback
Same start, different outcomes	Multifinality	Equifinality
Change in the rules themselves	Second-order change	First-order change
Family's links to school, church, agencies	Ecomap (Hartman)	Genogram
Cohesion and flexibility	Olson's Circumplex Model	McMaster Model
Six dimensions incl. roles and behavior control	McMaster Model (Epstein)	Circumplex
Social, problem, interaction, goal-setting, task-setting	Haley's first interview	Roberts's crisis model
Contempt predicts divorce	Gottman	Sue Johnson (EFT)
Acceptance plus change in couples	IBCT (Christensen & Jacobson)	EFT
Speaker–listener technique	PREP (Markman et al.)	Gottman

Quick Checks

1. A family brings in their 14-year-old for skipping school. A systems-oriented counselor would most likely: A. Assess the teen individually for a conduct disorder B. Explore how the teen's behavior fits within patterns in the whole family C. Refer the teen for individual counseling D. Advise the parents to set stricter consequences

2. "When your mother gets upset, what does your father do?" is an example of: A. A linear question B. A circular question C. Enactment D. Reframing

3. At the start of couples counseling, the counselor explains that information shared in individual sessions may be brought into joint sessions. This is: A. A breach of con-

fidentiality B. A no-secrets policy C. Triangulation D. Paradoxical intervention

4. Which tool best shows a family's connections to outside resources such as school, church, and social services? A. Genogram B. Ecomap C. FACES D. Family sculpting

5. A couple's conflict escalates each time one partner raises his voice and the other raises hers in response. This is an example of: A. Negative feedback B. Positive feedback C. Homeostasis D. Multifinality

6. Olson's Circumplex Model assesses families on: A. Problem solving and roles B. Cohesion and flexibility C. Differentiation and triangles D. Boundaries and hierarchy

7. In Haley's first interview, which stage asks each member for their view of the problem? A. Social stage B. Problem stage C. Interaction stage D. Task-setting stage

8. A mother regularly complains to her 12-year-old son about his father's spending. A Bowenian counselor would describe this as: A. Enmeshment B. Triangulation C. Disengagement D. Coalition building

9. A family from a culture where three generations share a home and grandparents help make parenting decisions comes to counseling. The counselor's best first approach is to: A. Identify the grandparents' involvement as enmeshment B. Coach the parents to set firmer boundaries with the grandparents C. Explore the family's own views of roles and decision-making before applying a model D. See only the parents, since they are the legal guardians

10. According to Gottman, the strongest predictor of divorce among the Four Horsemen is: A. Criticism B. Contempt C. Defensiveness D. Stonewalling

11. Symptoms appear in a family soon after their oldest child begins high school and pushes for more independence. The systemic view is that the family is: A. Dysfunctional and needs restructuring B. Struggling with a life cycle transition that requires more flexible boundaries C.

Showing signs of the adolescent's oppositional defiant disorder D. In homeostasis and needs no change

12. After another curfew violation, the parents extend their teen's grounding from one week to two, while each parent keeps enforcing rules separately. This is an example of: A. Second-order change B. First-order change C. Morphogenesis D. Equifinality

Answers and Rationales

1 — B. Systems thinking looks at the pattern, not one member. A: an individual, linear focus on the IP. C: removes the problem from the family context. D: advice that assumes a linear cause.

2 — B. Asking about one member's response to another's behavior is a circular question (Milan). A: a linear question would ask why one person does something. C: enactment (Minuchin) has members interact in session. D: a reframe (MRI) offers a new meaning.

3 — B. A no-secrets policy explained during informed consent (ACA A.8; B.4.b). A: it isn't a breach because clients agreed in advance. C: triangulation (Bowen) pulls a third person into a conflict. D: paradox (Haley, MRI) prescribes the symptom.

4 — B. The ecomap (Hartman) maps outside connections. A: the genogram (McGoldrick & Gerson) maps generations within the family. C: FACES (Olson) measures cohesion and flexibility. D: sculpting (Satir) shows relationships through physical positions.

5 — B. Escalation that amplifies change is positive feedback. A: negative feedback dampens change. C: homeostasis is a return to balance. D: multifinality is about different outcomes from the same start.

6 — B. Cohesion and flexibility are Olson's dimensions. A: problem solving and roles are McMaster dimensions (Epstein). C: differentiation and triangles are Bowen's concepts. D: boundaries and hierarchy are Minuchin's.

7 — B. The problem stage gathers each person's view. A: the social stage is greeting and engagement. C: the interaction stage has members talk to each other. D: the task-setting stage gives a directive.

8 — B. Pulling a child into the parents' conflict is triangulation (Bowen). A: enmeshment (Minuchin) is diffuse boundaries generally. C: disengagement (Minuchin) is rigid, distant boundaries. D: coalition is a structural term for an alliance against a third member, but this item asks for the Bowenian term.

9 — C. Cultural humility before applying a model. A: labeling culturally normal closeness as pathology. B: imposes one culture's norms. D: excludes members who share family decision-making.

10 — B. Gottman identified contempt as the strongest predictor. A, C, D: the other three Horsemen.

11 — B. The adolescent stage of the family life cycle (Carter & McGoldrick) requires more flexible boundaries. A: pathologizes a normal transition. C: individual diagnosis of the IP. D: homeostasis is the family's resistance to the needed change, not a sign that no change is needed.

12 — B. Doing more of the same within the existing rules is first-order change (MRI). A: second-order change alters the rules. C: morphogenesis is change in the system's structure. D: equifinality is about different paths to the same outcome.

Audio Companion

Track	Covers	Target length
7.1	Think in Systems: concepts, tools, life cycle, couples	~7 min

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Chapter 8 — Group Work

2027 task map: 4.5 Foster the group therapeutic process · 2A.6 Obtain informed consent appropriate to clientele (e.g., group, family) · CACREP: Group Counseling and Group Work

Why Group Gets Its Own Chapter

The 2027 specifications give group its own task in Provision of Counseling Interventions (4.5, foster the group therapeutic process), plus group informed consent in Intake & Assessment (2A.6). Group also shows up in ethics and treatment planning items. Group work is one of the most consistently tested topics on the NCE.

Chapter 3 gave you the group owners: Tuckman, Corey, Yalom, Lewin, Moreno, Bion, and Luft & Ingham (the Johari Window). This chapter teaches what a group leader actually **does**, and how the NCE tests it.

The central shift: **in group, the group itself is a tool.** A good leader doesn't do individual counseling with an audience. The leader gets members talking to each other, because members learn from each other in ways they can't learn from the leader alone.

Types of Groups (ASGW)

The Association for Specialists in Group Work (ASGW) describes four types.

Type	Purpose	Example	Leader role
Task/work group	Accomplish a goal or product	Committee, team, planning group	Facilitator of process and productivity

Type	Purpose	Example	Leader role
Psychoeducational	Teach information and skills; prevention	Anger management, parenting skills, divorce adjustment for children	Teacher; structured
Counseling	Address normal life problems and growth through interpersonal support	Grief support, career transition, adjustment	Facilitator; less structured
Psychotherapy	Treat more serious or longer-standing psychological problems	Group for depression, personality patterns, trauma	Clinician; deeper work

Self-help and support groups (e.g., 12-step groups) are usually peer-led and not run by a counselor.

Structure Choices

Choice	Options	Trade-offs
Open vs. closed	Open: members join and leave over time. Closed: same members from start to end	Open groups have more turnover and lower cohesion but are available when needed. Closed groups build more cohesion and trust
Homogeneous vs. heterogeneous	Members share a key characteristic, or differ	Homogeneous groups bond faster and suit focused, short-term goals. Heterogeneous groups offer more perspectives and suit interpersonal learning

Choice	Options	Trade-offs
Size	Varies by age and type	Smaller for children and intensive work; psychoeducational groups can be larger. Interpersonal process groups often run about 7–8 members (Yalom)
Duration and frequency	Time-limited or ongoing	Children need shorter sessions; adults commonly meet for 60–120 minutes
Co-leadership	One leader or two	Co-leaders share the load, model healthy interaction, and offer two perspectives. Co-leaders must meet before and after sessions to plan and process; open disagreement in front of members without repair can harm the group

Before the Group: Screening and Preparation

Screening (ACA A.9.a)

Counselors screen prospective members to select people whose needs and goals fit the group, who won't impede the group process, and whose well-being won't be jeopardized by it.

Members who are often **not** a good fit for a general interpersonal group: - Acutely suicidal clients - Clients in active psychosis or severe crisis - Clients who are actively violent or extremely hostile - Clients whose needs require individual attention first

The pregroup interview is also the member's chance to decide whether the group is right for them.

Protecting Members (ACA A.9.b)

Leaders take reasonable precautions to protect members from physical, emotional, and psychological harm.

Informed Consent and Group Rules (2A.6)

Explain at the start: - Purpose, format, and goals of the group - **Confidentiality and its limits**, including that the leader **cannot guarantee** members will keep what's shared private (ACA B.4.a) - Attendance expectations and how to leave the group - Rules about contact between members outside group - Fees, records, and the leader's qualifications - Termination criteria

Confidentiality in group: the leader keeps confidentiality and asks members to do the same, but must tell members plainly that it can't be guaranteed. A wrong NCE answer promises members that everything said in group will stay private.

Stages and Stage-Matched Interventions (4.5)

This table is Corey's model, with Tuckman's equivalent names. Learn what members do *and* what the leader should do at each stage.

Corey stage (Tuckman)	What members do	What the leader does
Pregroup	(Not yet a group)	Screen, select, prepare members; set goals and structure
Initial (forming)	Polite, tentative, testing safety; look to the leader; superficial talk; identify themes	Provide structure; explain process and rules; model openness; build trust; help members set personal goals; use **linking** to show common ground; teach members to give feedback

Corey stage (Tuckman)	What members do	What the leader does
Transition (storming)	Anxiety, defensiveness, resistance, conflict, challenges to the leader, struggle for control	Normalize conflict; respond to challenges **non-defensively**; encourage members to express reactions directly; work with resistance rather than against it; model direct, respectful conflict
Working (norming/ performing)	Cohesion, trust, genuine self-disclosure, risk-taking, direct feedback, focus on here-and-now, change	Step back and let members lead more; reinforce risk-taking; encourage translating insight into action; process themes; use **here-and-now** work
Final (adjourning)	Sadness and anxiety about ending; consolidating learning; may pull back	Address feelings about ending; review and consolidate learning; plan how to apply changes outside; deal with unfinished business; give and receive final feedback; discuss referrals
Postgroup	—	Follow-up session or contact; evaluate the group

Exam pattern: match the described behavior to the stage, then pick the leader response that fits that stage. A leader who pushes deep disclosure in session one, or who responds defensively when challenged in the transition stage, is a wrong answer.

Transition-stage challenge example: > A member says in session four, “I don’t see the point of this. You just sit there and don’t tell us anything.”

The best response explores the reaction non-defensively (“It sounds like you’re frustrated with how I’m leading. I’d

like to hear more, and I'm curious whether others feel something similar."). Wrong answers defend the leader's approach, ignore the challenge, or ask the member to leave.

Leader Skills (4.5)

Skill	What it does	Example
Linking	Connects one member's concern to another's; builds universality and member-to-member interaction	"Maria, what you said sounds a lot like what Devon shared last week. Devon, does that fit?"
Blocking	Stops harmful or unproductive behavior **while protecting the member**	"Let me stop you there, Jim. I'd like to hear how you're feeling, rather than more about what happened."
Cutting off	Interrupts a member who goes on too long or off topic	"I want to pause here so others have room to respond."
Drawing out	Invites quieter members in	"Tasha, you've been nodding. What's coming up for you?"
Modeling	Demonstrates the behavior the leader wants members to learn	Giving feedback that's specific, honest, and kind
Facilitating	Encourages members to talk to each other, not just to the leader	"Instead of telling me, could you tell Ray directly?"
Process observation	Comments on *how* the group is interacting, not just *what* it's discussing	"I notice that every time conflict comes up, someone makes a joke."
Universalizing	Highlights shared experience	"Several of you have mentioned feeling invisible at home."
Protecting	Keeps members safe from attacks or pressure	Stepping in when a member is being ganged up on

Skill	What it does	Example
Supporting	Encourages a member who's taking a risk	"That took courage to say."
Summarizing	Ties together themes at the end of a segment or session	"Today's theme seemed to be trust."
Goal setting	Helps members define specific, workable goals	"What would you like to be different by the end of the group?"
Evaluating	Assesses the group's progress	A check-in on whether the group is meeting its goals
Terminating	Prepares the group and members for ending	Reviewing growth; planning for after the group

Content vs. Process

- **Content** is *what* members talk about (a member's divorce).
- **Process** is *how* members interact while talking about it (who speaks, who's silent, who rescues, how the group reacts).

The NCE rewards attention to process, especially in the working stage. A leader who only discusses content is running individual counseling in a group setting.

Here-and-Now Focus (Yalom)

Yalom describes two tiers: the group first **experiences** the here-and-now (members react to each other in the room), then **illuminates** it (reflects on what just happened). The group is a **social microcosm**: members eventually show the same interpersonal patterns in group that cause problems outside it, which makes those patterns available to work on directly.

Feedback (4.5)

Effective feedback is: - **Specific** and about behavior, not character - **Timely** - **Owned** by the giver ("I felt…" rather

than “Everyone thinks…”) - **Balanced**, with strengths as well as concerns - Offered in a way the receiver can **use**

The leader models both giving and receiving feedback, including receiving criticism without defensiveness.

Structured Activities (4.5)

Structured exercises (icebreakers, rounds, role-plays, sentence completions, art activities, psychodrama techniques) are useful especially in the initial stage, in psychoeducational groups, and with children. They lower anxiety and focus the group.

The caution: exercises should serve a purpose and be processed afterward. Overusing them can make the group leader-dependent and keep members from the less structured interaction that builds cohesion. Members can decline to participate in an exercise.

Psychoeducation in Group (4.5)

In psychoeducational groups, the leader teaches content, but process still matters. Leave time for members to react to and personalize the material, and link it to their experiences.

Yalom’s Therapeutic Factors in Action (4.5)

Factor	What it looks like in group
Instillation of hope	Seeing other members improve
Universality	“I’m not the only one”
Imparting information	Psychoeducation from the leader or advice from members
Altruism	Helping another member raises one’s own sense of worth
Corrective recapitulation of the primary family group	Re-experiencing family-like dynamics in the group and responding differently
Development of socializing techniques	Learning social skills through feedback

Factor	What it looks like in group
Imitative behavior	Learning by watching the leader or other members
Interpersonal learning	Learning how one affects others and how others affect oneself (input and output)
Group cohesiveness	Belonging and acceptance; the group equivalent of the therapeutic alliance
Catharsis	Emotional release, most helpful when paired with understanding
Existential factors	Accepting responsibility, mortality, and the limits of what others can provide

Cohesiveness is often described as the group equivalent of the alliance in individual counseling, and a precondition for other factors to work.

Challenging Harmful Member Behaviors (4.5)

Behavior	What it looks like	Leader response
Monopolizing	One member dominates the time	Cut off gently; invite others; explore what monopolizing does for the member and how others feel
Silence/withdrawal	A member rarely speaks	Draw out gently without forcing; respect cultural differences in participation; explore what silence means for the member
Storytelling	Long narratives about past or outside events	Redirect to feelings and the here-and-now: “How are you feeling as you tell us this?”

Behavior	What it looks like	Leader response
Questioning/ interrogating	Firing questions at another member	Ask the questioner to share the statement behind the question
Advice-giving	Rushing to tell others what to do	Encourage sharing personal experience or feelings instead; explore the impulse to fix
Rescuing ("band-aiding")	Smoothing over another member's pain too quickly	Block gently; let the member stay with the feeling; explore the rescuer's discomfort
Hostility/attacking	Sarcasm, put-downs, aggression	**Block** to protect the target; ask for direct, owned feedback; explore the anger
Scapegoating	The group targets one member	**Block and protect**; redirect members to their own feelings; explore what the group is avoiding
Intellectualizing	Talking about feelings in abstract terms	Invite the member to speak personally and in the present
Dependency	Constantly seeking the leader's approval or answers	Redirect to members; encourage self-reliance
Socializing outside group	Subgroups, relationships, or secrets formed outside sessions	See below

The leader's priority when a member is attacked: protect. Blocking, not ignoring, is the scored answer.

Contact Outside the Group (4.5)

Contact between members outside sessions can form **subgroups** that pull energy away from the group, create secrets, and threaten cohesion and safety.

- Set expectations about outside contact **during informed consent**.

- Expectations vary by group type. Outside contact is often discouraged in psychotherapy groups; it may be acceptable or even encouraged in some support and task groups.
- When outside contact happens, the expected norm is that members **bring it back into the group** to discuss, rather than keeping it secret.

Transitions in Membership (4.5)

Situation	Leader response
A new member joins an open group	Prepare the new member beforehand; prepare existing members; introduce and orient the new member; expect a temporary dip in cohesion
A member wants to leave	Encourage the member to tell the group and say goodbye rather than disappear; explore the decision, without coercion (members have the right to leave)
A member stops attending	Follow up; address the absence with the group
The group ends	Allow time to process endings; review progress; plan maintenance; offer referrals; follow up

Multicultural Group Work

ASGW's *Multicultural and Social Justice Competence Principles for Group Workers* (2012) call for leaders to: - Be aware of their own cultural values and biases - Recognize that norms about self-disclosure, eye contact, conflict, and emotional expression vary across cultures - Avoid treating one culture's style of participation as the only "healthy" one - Address microaggressions and power dynamics within the group - Adapt structure, language, and goals to members' cultural contexts

A quiet member from a culture that values restraint in front of strangers is not automatically “resistant.”

Group Ethics Summary

Topic	Rule
Screening	Required (ACA A.9.a)
Protecting members	Required (ACA A.9.b)
Confidentiality	Leader explains it **can’t be guaranteed** (ACA B.4.a)
Voluntary participation	Members can decline exercises and can leave the group
Leader competence	Leaders need group-specific training and supervision
Dual relationships	Same standards as individual counseling
Minors in groups	Parent/guardian consent; age-appropriate explanation of confidentiality limits
Records	Individual records for each member, with care not to include other members’ identifying information

What the Exam Rewards / What It Punishes

Rewards: - Screening before the group begins - Telling members confidentiality can’t be guaranteed - Stage-matched leader responses - Responding to challenges in the transition stage without defensiveness - Linking members and promoting member-to-member interaction - Blocking to protect a member who is attacked or scapegoated - Attending to process, not just content - Bringing outside contact back into the group

Punishes: - Doing individual counseling in front of the group - Promising confidentiality - Pushing deep disclosure in the initial stage - Defending yourself when members challenge the leader - Ignoring scapegoating or at-

tacks - Forcing silent members to speak - Using “contemplation” as a group stage (it’s a stage of change)

Swap-Pair Drill

If you see…	It’s…	Not…
Connecting two members’ concerns	Linking	Universalizing (the leader names a shared theme for the whole group)
Stopping harmful behavior to protect a member	Blocking	Cutting off (managing time or focus)
Inviting a quiet member in	Drawing out	Linking
Smoothing over pain too fast	Rescuing / band-aiding	Supporting
Commenting on how the group interacts	Process observation	Summarizing content
Experiencing, then reflecting on, what just happened in the room	Here-and-now (Yalom)	Catharsis
Group equivalent of the alliance	Cohesiveness	Universality
Learning how one affects others	Interpersonal learning	Imitative behavior
Members can join anytime	Open group	Closed group
Teaching skills, prevention focus	Psychoeducational group	Counseling group
Members test the leader and conflict appears	Transition (Corey) / storming (Tuckman)	Initial / forming

Quick Checks

1. During the first session of a counseling group, the leader explains the group’s purpose and rules, including that

confidentiality can't be guaranteed. Which stage is this? A. Transition B. Working C. Initial D. Final

2. A member says, "Everything said here stays here, right?" The best leader response is: A. "Yes, everything is completely confidential." B. "I'll keep what you share confidential except where the law requires otherwise, and I'll ask everyone to do the same, but I can't guarantee what other members will do." C. "Confidentiality doesn't apply in groups." D. "That's up to each member to decide."

3. In the fourth session, a member says, "This group is pointless, and you don't know what you're doing." The best leader response is: A. Explain the leader's training and qualifications B. Ask the member to leave if he's not committed C. Acknowledge the frustration non-defensively and invite the group to share reactions D. Move on to the planned activity

4. A member begins crying after describing her father's death. Another member immediately says, "Don't cry, he's in a better place now." The leader should: A. Agree, to comfort the grieving member B. Gently block the rescue and give the grieving member room to stay with her feelings C. Change the topic to reduce distress D. Ask the grieving member to leave until she's composed

5. Several members repeatedly criticize one member for "always complaining." The leader's priority is to: A. Let the group handle it on its own B. Agree that the member complains too much C. Block the scapegoating, protect the member, and explore what the group is reacting to D. Ask the targeted member to change

6. "James, when Lena talked about feeling unseen at work, you nodded a lot. Does that connect to anything for you?" This is: A. Blocking B. Linking C. Cutting off D. Summarizing

7. A leader comments, "I notice that each time someone gets close to talking about anger, someone else changes the subject." This is: A. Process observation B. Content focus C. Drawing out D. Imparting information

8. Two members admit they've been meeting for coffee after group and talking about other members. The best leader response is to: A. Ignore it because it happened outside the group B. Remove both members immediately C. Bring it into the group to discuss its impact, consistent with the rules set at the start D. Meet privately with each member and keep it confidential

9. Which of Yalom's factors is the group equivalent of the therapeutic alliance? A. Universality B. Catharsis C. Group cohesiveness D. Altruism

10. A prospective member is acutely suicidal and was discharged from the hospital yesterday. Based on screening guidelines, the best decision is to: A. Add her to the group immediately B. Defer group placement and provide individual services until she's more stable C. Add her but tell no one about her history D. Refuse services entirely

11. Which group type is most appropriate for teaching coping skills to middle school students whose parents are divorcing? A. Psychotherapy group B. Task group C. Psychoeducational group D. Self-help group

12. A new member is joining an ongoing open group. The best leader action is to: A. Add the member without telling the group, so things stay normal B. Prepare the new member and the existing members, then help integrate the new member C. Ask the new member to stay silent for the first few sessions D. Restart the group from the initial stage

Answers and Rationales

1 — C. Orientation, rules, and structure belong to Corey's initial stage (Tuckman's forming). A: transition involves conflict and resistance. B: working involves cohesion and risk-taking. D: final involves consolidation and ending.

2 — B. Accurate informed consent: the leader's commitment, its legal limits, and the fact that members' confidentiality can't be guaranteed (ACA B.4.a). A: a promise

the leader can't keep. C: confidentiality does apply to the leader. D: abdicates the leader's role in setting norms.

3 — C. Transition-stage challenges are met non-defensively and opened to the group. A: defensive. B: punitive and threatening to the group's safety. D: ignores what's happening in the group.

4 — B. Blocking the band-aid lets the member experience her grief and lets the group learn. A: joins the rescue. C: avoids the feeling. D: shames emotion and harms safety.

5 — C. Protecting a scapegoated member is the leader's priority (ACA A.9.b). A: leaves the member unprotected. B: joins the attack. D: blames the target.

6 — B. Connecting one member's experience to another's is linking. A: blocking stops harmful behavior. C: cutting off manages time or focus. D: summarizing ties together themes.

7 — A. Commenting on how the group interacts is process observation. B: content would be the subject itself (anger at someone). C: drawing out invites a quiet member. D: imparting information is one of Yalom's factors, teaching content.

8 — C. Outside contact is brought back into the group, following norms set at the start. A: ignores a threat to cohesion and trust. B: punitive without exploration. D: creates more secrets.

9 — C. Cohesiveness is the group's version of the alliance. A: universality is "I'm not alone." B: catharsis is emotional release. D: altruism is benefiting from helping others.

10 — B. Screening excludes members whose well-being could be jeopardized or who need individual attention first (ACA A.9.a). A: unsafe. C: deceptive and unsafe. D: abandons her; she still needs services.

11 — C. Teaching coping skills with a prevention focus is a psychoeducational group. A: psychotherapy groups

treat more serious problems. B: task groups accomplish a product. D: self-help groups are peer-led.

12 — B. Preparing both the new member and the group supports integration (4.5). A: blindsides the group. C: excludes the new member. D: unnecessary; the group can integrate the member while expecting a temporary dip in cohesion.

Audio Companion

Track	Covers	Target length
8.1	Lead the Group: stages, skills, harmful behaviors, ethics	~7 min

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Chapter 9 — Delivery Formats and Special Populations

2027 task map: 4.8 Integrate contextual and cultural influences into counselor–client relationship · 4.6 Implement empirically supported interventions · 4.2 Explore the impact of the client's culture, context, values, and worldview · 6.4 Adhere to telemental health best practice guidelines · 6.20 Market your counseling business appropriately (e.g., proper utilization of social media, proper protection of client information) · 6.14 Provide adequate accommodations for clients with disabilities · CACREP: Social and Cultural Diversity; Counseling and Helping Relationships

One Question Runs Through This Chapter

Every section of this chapter asks the same question: **does this intervention fit this client?** Not just the client's problem, which Chapter 5 covered, but the client's culture, faith, identity, life circumstances, and the format through which counseling is delivered.

The NCE's wrong answers in this area tend to share one flaw: they apply a single standard to everyone. The right answers adapt.

Part 1 — Multicultural Counseling

Owners and Frameworks

Owner	Contribution
C. Gilbert Wrenn (1962)	**Cultural encapsulation**: counselors who substitute their own cultural assumptions for reality and ignore cultural variation

Owner	Contribution
Sue, Arredondo, & McDavis (1992)	Multicultural Counseling Competencies: counselor **awareness** of own values and biases, **knowledge** of the client's worldview, and culturally appropriate intervention **skills**
Ratts, Singh, Nassar-McMillan, Butler, & McCullough (2016)	Multicultural and Social Justice Counseling Competencies (MSJCC): four quadrants based on privilege and marginalization of counselor and client; developmental domains include self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions
Pamela Hays	**ADDRESSING** framework for cultural influences
Derald Wing Sue	Racial microaggressions; worldview model based on locus of control and locus of responsibility
Norma Day-Vines	**Broaching**: openly raising race, ethnicity, and culture in counseling
Joshua Hook and colleagues	**Cultural humility**: an other-oriented, curious stance that recognizes the limits of one's cultural knowledge
Edward T. Hall	High-context vs. low-context communication
Geert Hofstede	Cultural dimensions, including individualism vs. collectivism and power distance
Lewis, Arnold, House, & Toporek (2002); updated by Toporek & Daniels (2018)	ACA Advocacy Competencies

ADDRESSING (Hays)

Letter	Influence
A	Age and generational influences
D	Developmental disabilities

Letter	Influence
D	Disabilities acquired later in life
R	Religion and spiritual orientation
E	Ethnic and racial identity
S	Socioeconomic status
S	Sexual orientation
I	Indigenous heritage
N	National origin
G	Gender

Key Concepts

Term	Meaning
Etic	A universal approach: the same concepts apply across cultures
Emic	A culture-specific approach: understand the client from within their own culture
Individualism vs. collectivism	Priority on the individual's goals and independence vs. the family or group's goals and interdependence
High-context vs. low-context	Meaning carried in context, relationship, and nonverbals vs. meaning carried in explicit words
Worldview (Sue)	Two dimensions: **locus of control** (internal vs. external: who controls what happens to me?) and **locus of responsibility** (internal vs. external: who is responsible for my situation, me or the system?)
Acculturation	Psychological and cultural change after contact with another culture (Berry's four strategies, Chapter 3)
Acculturative stress	Stress from the process of acculturation

Term	Meaning
Privilege	Unearned advantages based on group membership
Intersectionality (Kimberlé Crenshaw)	Overlapping identities create unique experiences of privilege and oppression
Cultural humility	Ongoing self-reflection and openness, rather than claiming "competence" in a culture

Microaggressions (Sue)

Type	Description	Example
Microassault	Explicit, often conscious, derogatory action	A slur; deliberately avoiding someone
Microinsult	Subtle rudeness or insensitivity that demeans identity	"You're so articulate" to a Black client, implying surprise
Microinvalidation	Negates the thoughts, feelings, or experiences of a person	"I don't see color"; "Where are you *really* from?"

When a counselor commits a microaggression and the client reacts, the scored response is to acknowledge it, take responsibility, explore its impact non-defensively, and repair. Wrong answers deny it, explain the counselor's intent, or move on.

Broaching

Broaching means the counselor raises cultural factors directly rather than waiting for the client to bring them up: "I'm a White counselor, and you've mentioned experiences of racism at work. I'm wondering how it feels to talk about that with me." Done well, broaching builds trust and signals that culture is welcome in the room. The NCE generally rewards respectful broaching over avoidance.

Interpreters

- Use a **trained professional interpreter**.

- **Don't use family members, especially children**, as interpreters. It compromises accuracy, confidentiality, and family roles.
- Speak directly to the client, not to the interpreter.
- Explain the interpreter's confidentiality obligations to the client.

The Cultural Formulation Interview (CFI)

The DSM-5-TR includes a 16-question Cultural Formulation Interview covering four areas: the cultural definition of the problem; cultural perceptions of cause, context, and support; cultural factors affecting self-coping and past help-seeking; and cultural factors affecting current help-seeking. Use it at intake to understand the problem in the client's own terms (Chapter 19).

Advocacy (ACA Advocacy Competencies)

The competencies describe advocacy at three levels, each with acting **with** the client (collaboration, empowerment) or acting **on behalf of** the client:

Level	Acting *with*	Acting *on behalf of*
Client/student	Empowerment: help clients advocate for themselves	Client advocacy: negotiate services for the client
School/community	Community collaboration	Systems advocacy
Public arena	Collective action	Social/political advocacy

Exam principle: where possible, empower clients to advocate for themselves before acting on their behalf.

Part 2 — Religious and Spiritual Values (4.2)

ASERVIC Competencies

The Association for Spiritual, Ethical, and Religious Values in Counseling (ASERVIC) competencies (2009) call on

counselors to: - Explore their own religious and spiritual beliefs and how they might affect counseling - Understand diverse religious and spiritual beliefs, including the absence of belief - Respond to clients' spiritual expressions with acceptance and sensitivity - Recognize when spiritual beliefs are a resource and when they are part of the client's distress - Use spiritual practices in counseling only when they fit the client's beliefs and goals, and with the client's consent

Spiritual Assessment

Ask open questions about whether faith or spirituality matters to the client, how it helps, whether it creates conflict, and whether the client wants it included in counseling. Spiritual and religious resources (community, prayer, meaning) can be strong protective factors.

Fowler's Stages of Faith

James Fowler described six stages of faith development (plus a pre-stage of "primal faith" in infancy): intuitive–projective, mythic–literal, synthetic–conventional, individuative–reflective, conjunctive, and universalizing. This is another owner to know for development items.

Values Imposition and Referral

Standard	Rule
ACA A.4.b	Counselors are aware of their own values and **avoid imposing** them on clients, especially when the counselor's values are inconsistent with the client's goals or discriminatory in nature
ACA A.11.b	Counselors **refrain from referring** clients **based solely on the counselor's personally held values**, attitudes, beliefs, and behaviors

Standard	Rule
ACA C.5	Counselors don't discriminate based on age, culture, disability, ethnicity, race, religion/ spirituality, gender, gender identity, sexual orientation, marital/partnership status, language, socioeconomic status, immigration status, or any basis proscribed by law

The exam pattern: a counselor's personal or religious values conflict with a client's identity or choice (e.g., a gay client seeking relationship help; a client considering divorce). The correct answer is to **seek supervision or consultation, examine one's own values, and continue serving the client competently.** Referring the client away because of the counselor's beliefs is a values-based referral and is the wrong answer. Two well-known court cases involved counseling students who sought to refer LGBTQ+ clients for values reasons: *Ward v. Wilbanks* and *Keeton v. Anderson-Wiley*.

Referral is appropriate when the issue is competence, not values: a client needs a specialized treatment the counselor isn't trained to provide, and supervision or training can't bridge the gap in time.

Part 3 — Addiction (4.6)

Models of Addiction

Model	Core idea	Owner / association
Moral model	Addiction is a personal failing or choice	Historical; now rejected by the field
Disease model	Addiction is a chronic, progressive disease	E. M. Jellinek (phases of alcoholism); AA
Biopsychosocial model	Biological, psychological, and social factors interact	Current mainstream view

Model	Core idea	Owner / association
Learning/ behavioral model	Use is learned and maintained by reinforcement	Skinner's principles applied
Harm reduction	Reduce the harms of use, whether or not the person is ready to stop	Public health approach (e.g., naloxone access, safer use)

Key Terms

Term	Meaning
Tolerance	Needing more of a substance to get the same effect, or getting less effect from the same amount
Withdrawal	A substance-specific syndrome when use stops or decreases
Cross-tolerance	Tolerance to one substance produces tolerance to another in the same class
Synergism (potentiation)	Two substances together produce a stronger effect than either alone, as with alcohol and benzodiazepines; this can be deadly
Enabling	Others' behavior that shields the person from consequences of use
Codependency	A family member's over-focus on the user, often at their own expense

Alcohol and benzodiazepine withdrawal can be medically dangerous (seizures; delirium tremens with alcohol). A client in significant withdrawal needs medical evaluation; medically managed withdrawal comes before outpatient counseling.

Screening and Assessment

Tool	Use
CAGE	Four alcohol questions: Cut down, Annoyed, Guilty, Eye-opener

Tool	Use
AUDIT (World Health Organization)	10-item alcohol screen
CRAFFT	Adolescent substance use screen (Car, Relax, Alone, Forget, Friends, Trouble)
DAST	Drug Abuse Screening Test
SASSI	Substance Abuse Subtle Screening Inventory
ASI	Addiction Severity Index
SBIRT	Screening, Brief Intervention, and Referral to Treatment: a public health process for identifying and responding to risky use

ASAM Criteria

The American Society of Addiction Medicine (ASAM) Criteria use six dimensions to determine level of care (third-edition names shown; most exam prep materials still use them):

1. Acute intoxication and/or withdrawal potential
2. Biomedical conditions and complications
3. Emotional, behavioral, or cognitive conditions and complications
4. Readiness to change
5. Relapse, continued use, or continued problem potential
6. Recovery/living environment

Levels range from early intervention and outpatient services through intensive outpatient, partial hospitalization, residential, and medically managed intensive inpatient care. The fourth edition (2023) revised some dimension names and level numbers, so check the current edition before quoting exact levels. **The exam principle: match the level of care to the dimensions, and use the least restrictive level that's safe.**

Treatment Approaches

Approach	Notes
Motivational interviewing	Resolve ambivalence (Chapter 5)
CBT and relapse prevention	Coping skills, high-risk situations, abstinence violation effect (Marlatt)
Contingency management	Tangible rewards for verified abstinence
Community Reinforcement Approach (CRA)	Make a sober lifestyle more rewarding than use
CRAFT (Robert Meyers)	Community Reinforcement and Family Training: teaches family members to encourage a loved one into treatment; generally more effective at engaging loved ones than confrontational "Johnson-style" interventions
12-step facilitation	Encourages active involvement in AA/NA; sponsor, meetings, steps
Medications for addiction treatment	For opioid use: methadone, buprenorphine, naltrexone. For alcohol use: naltrexone, acamprosate, disulfiram. Counselors **don't prescribe**; they support, coordinate, and refer
Integrated treatment	For co-occurring mental health and substance use disorders, treat both at the same time, ideally by the same team

Mutual Support

Group	For
Alcoholics Anonymous (AA), Narcotics Anonymous (NA)	People with addiction; 12-step
SMART Recovery	People with addiction; CBT-based, not 12-step
Al-Anon	Family and friends of people with alcohol problems
Alateen	Teens affected by someone's drinking

Group	For
Adult Children of Alcoholics (ACoA)	Adults who grew up with alcoholic or dysfunctional caregivers

Family Roles (Wegscheider-Cruse)

Sharon Wegscheider-Cruse described roles in families affected by addiction: the **dependent** (the user), the **enabler** (chief enabler), the **hero**, the **scapegoat**, the **lost child**, and the **mascot**.

Confidentiality: 42 CFR Part 2

Federal regulations (42 CFR Part 2) give extra confidentiality protection to records from federally assisted substance use disorder programs. A 2024 final rule aligned Part 2 more closely with HIPAA, but its consent requirements remain stricter than those for general health records in several respects. On the exam: **substance use treatment records generally require specific written consent to disclose.**

Process Addictions

Gambling disorder is the only behavioral addiction included as a diagnosis in the DSM-5-TR's Substance-Related and Addictive Disorders chapter. **Internet gaming disorder** is listed in Section III as a condition for further study. Other compulsive behaviors (sex, pornography, shopping, internet use) are addressed clinically but aren't separate DSM diagnoses.

Part 4 — Distance Counseling and Technology (6.4, 6.20)

The ACA Code of Ethics (2014) **Section H** covers distance counseling, technology, and social media.

Standard	Rule
H.1.a — Knowledge and competency	Counselors who use technology develop knowledge and skills in its technical, ethical, and legal aspects
H.1.b — Laws and statutes	Counselors follow the laws of both the counselor's location and the **client's place of residence**
H.2.a — Informed consent	Covers distance-specific issues: credentials and physical location, risks and benefits, possibility of technology failure and backup plans, response time, **emergency procedures when the counselor isn't available**, time zones, cultural and language differences, and possible denial of insurance
H.2.b–d — Confidentiality and security	Inform clients of the limits of confidentiality with technology; use encryption and security measures that meet legal requirements
H.3 — Client verification	Verify the client's identity at the start and throughout (e.g., code words, numbers, graphics)
H.4.c–d — Technology-assisted services; effectiveness	Determine whether distance counseling is appropriate for this client (H.4.c); when it's ineffective, consider face-to-face services (H.4.d)
H.4.e–f	Access to services and accessibility (H.4.e); effects of lacking visual and audio cues (H. 4.f)
H.5 — Records	Maintain electronic records according to law; inform clients about how records are stored
H.6.a — Virtual professional presence	Keep **separate professional and personal** social media accounts
H.6.c — Client privacy on social media	Counselors respect clients' privacy on social media and **don't view clients' social media without consent**

Standard	Rule
H.6.d	Counselors take precautions not to disclose confidential information through social media

Practical Telehealth Rules

- **Licensure follows the client.** Counselors generally must be licensed (or otherwise authorized) in the state where the client is physically located during the session. The interstate **Counseling Compact** is creating a privilege-to-practice system across member states; check its current implementation status before relying on it.
- At the start of each session, **confirm the client's physical location** (for emergencies) and that they're in a private space.
- Have an **emergency plan** in place: the client's local emergency resources and an emergency contact.
- Use **HIPAA-compliant platforms** and a business associate agreement (BAA) with the vendor.
- **Not every client is a good fit.** Acute risk, severe dissociation, or lack of privacy at home may call for in-person care.

Social Media and Electronic Communication

- Don't "friend" or follow current clients on personal accounts (a boundary issue).
- Don't search clients online without consent and a clinical reason.
- Explain in informed consent how email and text will (and won't) be used, response times, and that they're not for emergencies.
- Avoid discussing clinical details by unencrypted email or text.

Artificial Intelligence in Counseling

AI tools (note-writing assistants, chatbots, screening apps, scheduling and billing systems) are spreading

quickly in counseling practice. The 2014 ACA Code of Ethics doesn't mention AI by name, but the 2027 NCE specifications do: training in the ethical and legal use of AI (1A.7), technology-assisted tools such as AI note takers (6.11), and using AI within counseling practice (6.19). Chapter 17's Practice Management and Compliance section and Chapter 22's self-awareness section cover those tasks. The existing standards for technology, confidentiality, competence, and informed consent apply, and the American Counseling Association's AI Work Group has published recommendations for counselors:

Principle	What it means in practice
Informed consent and transparency	Tell clients when and how AI tools are used (for example, an AI note-taker in session), explain the purpose, benefits, and risks, and get their consent. Clients can decline
Clinical judgment stays human	Don't use AI as the **sole** basis for diagnosis, assessment, or risk decisions. The counselor reviews and is responsible for any AI-generated output, including notes
Privacy and security	Use tools that meet HIPAA and other legal requirements, with a business associate agreement where required. Don't enter identifiable client information into consumer AI tools that aren't built for protected health information
Bias	AI can carry bias from the data it was trained on, with greater risk of harm to marginalized groups. Check output critically
Competence	Understand a tool's capabilities and limits before using it; practice within the boundaries of your competence (ACA C.2.a)
The relationship comes first	AI can support counseling but shouldn't replace the human therapeutic relationship

Principle	What it means in practice
Clients' own AI use	Ask about it. Some clients use AI chatbots for emotional support; explore the benefits and risks with them, including privacy, accuracy, and the absence of crisis response

The exam-style principle: when a new technology appears in a question, apply the standards you already know: informed consent, confidentiality and security, competence, the counselor's responsibility for clinical decisions, and the client's welfare.

Part 5 — Specific Populations (4.8)

Population	What the exam expects
Veterans and military	Understand military culture (mission, unit cohesion, chain of command, stigma about help-seeking); screen for PTSD, traumatic brain injury (TBI), military sexual trauma (MST), substance use, and suicide risk; **moral injury** (distress after acts that violate one's moral code); know VA and Vet Center resources; involve families, who experience deployment too
LGBTQ+ clients	**Affirmative** counseling; use the client's name and pronouns; don't assume sexual orientation is the presenting problem; understand minority stress (Ilan Meyer); conversion/reparative therapy is harmful and contrary to professional standards; ALGBTIC (now SAIGE) competencies

Population	What the exam expects
People with disabilities	Ask the client's preference for person-first ("person with a disability") or identity-first ("Deaf person," "autistic person") language; know the **medical model** (disability as an individual deficit) vs. the **social model** (disability created by environmental and social barriers); provide reasonable accommodations under the **Americans with Disabilities Act (ADA)**; don't assume the disability is the focus of counseling
Deaf clients	Deaf culture (capital D) as a cultural and linguistic identity; use a qualified **ASL interpreter**; ASL is a full language, not signed English
Older adults	See Chapters 5 and 12; screen for cognitive and medical contributors; ageism; elder abuse reporting (Chapter 6)
Immigrants and refugees	Acculturative stress; pre-migration trauma; language access; immigration-related fears; collectivist family structures; practical needs first
People living in poverty	Maslow: basic needs before insight work; reduce barriers (sliding scale, scheduling, transportation, telehealth); case management and advocacy
Rural clients	Fewer resources; stigma; unavoidable overlapping relationships (manage boundaries carefully rather than refusing all contact); telehealth expands access
Justice-involved clients	Mandated treatment (Chapter 4); clarify limits of confidentiality with the court or probation officer during informed consent

Population	What the exam expects
Racial and ethnic minority clients	Racial trauma and race-based stress; mistrust grounded in historical and current experience; broaching; avoid cultural encapsulation

What the Exam Rewards / What It Punishes

Rewards: - Self-awareness and cultural humility - Broaching culture respectfully - Acknowledging and repairing a microaggression - Professional interpreters, not family members - Exploring spiritual values with the client's consent - Supervision when the counselor's values conflict with a client's - Matching addiction treatment level to the ASAM dimensions; medical evaluation for dangerous withdrawal - Telehealth informed consent with emergency plans and location checks - Following the law where the client is located - Informed consent for AI tools; keeping clinical judgment with the counselor

Punishes: - Cultural encapsulation: one standard for everyone - Values-based referrals - Imposing the counselor's beliefs - Using a child as an interpreter - Confrontational interventions and moral judgment in addiction work - "Friending" clients or viewing their social media without consent - Telehealth across state lines without authorization

Swap-Pair Drill

If you see…	It's…	Not…
Counselor imposes own culture as reality	Cultural encapsulation (Wrenn)	Microaggression
Awareness, knowledge, skills	Sue, Arredondo, & McDavis	MSJCC (Ratts et al.)

If you see…	It's…	Not…
Privileged/ marginalized counselor and client quadrants	MSJCC (Ratts et al.)	Sue et al. (1992)
"I don't see color"	Microinvalidation	Microassault
Universal (culture-general) approach	Etic	Emic
Locus of control and locus of responsibility	Sue's worldview model	Rotter's locus of control alone
Raising culture directly in session	Broaching (Day-Vines)	Cultural humility
Faith develops through six stages	Fowler	Kohlberg
Referring a client because of the counselor's beliefs	Values-based referral (prohibited, ACA A. 11.b)	Competence-based referral
Training family to encourage a loved one into treatment	CRAFT (Meyers)	Johnson intervention
CBT-based, non-12-step mutual support	SMART Recovery	AA
Hero, scapegoat, lost child, mascot	Wegscheider-Cruse	Satir's stances
Disability from environmental barriers	Social model	Medical model
Licensure for telehealth	Where the **client** is located	Where the counselor is located only

Quick Checks

1. A counselor says to a Latina client, "Your English is so good!" The client looks uncomfortable. This is an example of: A. A microassault B. A microinsult C. Broaching D. Cultural humility

2. In the situation above, the counselor's best next step is to: A. Explain that it was meant as a compliment B. Move on so the client doesn't feel awkward C. Acknowledge the

comment, take responsibility, and invite the client to share how it landed D. Refer the client to a Latina counselor

3. A counselor with strong religious beliefs is uncomfortable working with a client in a same-sex relationship who wants help with communication. The counselor should: A. Refer the client to another counselor B. Seek supervision, examine personal values, and continue providing competent services C. Tell the client about the counselor's beliefs so the client can decide D. Focus counseling on the client's sexual orientation

4. A family arrives for an intake; the parents speak limited English. The best choice for interpretation is: A. Their 11-year-old son, who is fluent B. A trained professional interpreter C. An English-speaking neighbor D. Gestures and simplified English

5. Which DSM-5-TR tool helps counselors understand a problem in the client's cultural terms? A. MMPI-3 B. Cultural Formulation Interview C. CAGE D. ASI

6. A client reports drinking heavily every day for years and says he plans to stop "cold turkey" tomorrow. The counselor's best action is to: A. Praise the decision and schedule a session next week B. Recommend he wait until he feels more motivated C. Arrange a medical evaluation because alcohol withdrawal can be dangerous D. Refer him to AA only

7. A mother wants help getting her adult son, who uses opioids, into treatment. The approach with the strongest support for engaging him is: A. A surprise confrontational intervention B. CRAFT C. Cutting off all contact until he agrees D. Waiting until he hits rock bottom

8. Before beginning telehealth with a new client, the counselor's informed consent should include: A. A promise that technology is completely secure B. Emergency procedures, risks of technology failure, and backup plans C. The counselor's personal social media handles D. A statement that location doesn't matter

9. A counselor licensed only in Georgia wants to continue video sessions with a client who has permanently moved to Oregon. The counselor should first: A. Continue, since the counselor is licensed in Georgia B. Determine whether Oregon law permits the counselor to practice with the client there, and arrange a transition if not C. Switch to phone sessions, which aren't regulated D. Ask the client to give a Georgia address

10. A counselor looks up a new client's Instagram account to "understand her better." Under the ACA Code, this is: A. Recommended to build rapport B. Acceptable if the account is public C. Inconsistent with respecting client privacy on social media without consent D. Required as part of intake

11. Which framework lists Age, Developmental and acquired Disabilities, Religion, Ethnicity, Socioeconomic status, Sexual orientation, Indigenous heritage, National origin, and Gender? A. MSJCC B. ADDRESSING C. ASERVIC D. CFI

12. A veteran describes intense guilt over actions he took during combat that violated his own moral beliefs. This is best described as: A. Military sexual trauma B. Moral injury C. Traumatic brain injury D. Survivor stress

13. A client mentions she's been talking to an AI chatbot "every night" about her anxiety and finds it helpful. The counselor's best response is to: A. Tell her to stop, since chatbots are unethical B. Ignore it, since it happens outside of counseling C. Ask about her experience with it and explore the benefits and risks, including privacy and the lack of crisis response D. Recommend she replace counseling with the chatbot

Answers and Rationales

1 — B. A comment that conveys surprise about a person's ability based on ethnicity is a microinsult (Sue). A: a microassault is explicit and deliberate. C: broaching is raising culture respectfully on purpose. D: cultural humility is a stance of openness, not a remark.

2 — C. Acknowledge, take responsibility, and explore impact. A: defends intent over impact. B: avoids repair. D: unnecessary; a rupture can be repaired.

3 — B. ACA A.4.b and A.11.b: avoid imposing values and don't refer based solely on personal values; seek supervision. A: a values-based referral. C: burdens the client with the counselor's beliefs. D: the client's orientation isn't the presenting concern.

4 — B. A professional interpreter protects accuracy, confidentiality, and family roles. A: using a child reverses family roles and exposes the child to adult material. C: confidentiality and accuracy concerns. D: inadequate communication.

5 — B. The CFI is the DSM-5-TR's cultural assessment. A: the MMPI-3 is a personality inventory. C: CAGE is an alcohol screen. D: the ASI measures addiction severity.

6 — C. Abrupt cessation after heavy, long-term drinking can cause seizures or delirium tremens; medical evaluation comes first (ASAM Dimension 1). A: ignores medical risk. B: discourages change. D: AA can be part of recovery but doesn't address withdrawal risk.

7 — B. CRAFT (Meyers) has strong support for engaging treatment-refusing loved ones. A: confrontational interventions are generally less effective at engagement. C and D: withdraw support without evidence of benefit.

8 — B. ACA H.2.a requires distance-specific informed consent, including emergency and technology-failure plans. A: no system is completely secure. C: separate professional and personal presence (H.6.a). D: location matters for law and emergencies.

9 — B. Counselors must be authorized where the client is located, and H.1.b makes them subject to the laws of both their own location and the client's place of residence. A: Georgia licensure alone doesn't authorize practice in Oregon. C: phone sessions are still counseling. D: dishonest and unethical.

10 — C. ACA H.6.c: respect clients' privacy on social media unless given consent. A, B, D: not supported by the Code.

11 — B. ADDRESSING (Hays). A: MSJCC (Ratts et al.) is built on privilege and marginalization quadrants. C: ASERVIC covers spiritual competencies. D: the CFI is the DSM-5-TR interview.

12 — B. Distress after acts that violate one's own moral code is moral injury. A: MST is sexual assault or harassment during military service. C: TBI is a physical brain injury. D: not a standard term for this experience.

13 — C. The ACA AI Work Group recommends asking about clients' AI use and exploring benefits and risks; this also respects autonomy. A: imposes the counselor's view without exploring. B: misses information relevant to her care and safety. D: AI shouldn't replace the human counseling relationship.

Audio Companion

Track	Covers	Target length
9.1	Fit the Client: culture, faith, addiction, telehealth, populations	~7 min

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Chapter 10 — Mood, Anxiety, Obsessive, and Trauma Presentations

2027 task map: 2B.1 Assess for trauma · 4.3 Facilitate client's skill development · 2B.12 Use all available data to select the appropriate diagnosis from the most recent DSM/ICD code · 2B.6 Determine co-occurring diagnoses, differential diagnoses, and other comorbidities · CACREP: Counseling and Helping Relationships; Assessment and Testing; Human Growth and Development · 2027 supplemental content guidance: presenting concerns

How the NCE Tests Presenting Concerns

Presenting concerns are no longer a domain of their own. The 2027 specifications list them as supplemental content guidance, and they show up inside every domain, most often as the client in the stem. Unlike the NCMHCE, the NCE doesn't hand you a diagnosis and walk you through a case. It asks single questions about **presenting concerns**: what a symptom pattern suggests, how long symptoms must last, which screening tool fits, which theory explains the problem, and which treatment approach has the best support.

This chapter covers the presentations that bring the most clients to counseling: depression, mood swings, worry, panic, fear, obsessions, trauma, and emotional dysregulation. For each one you'll learn four things:

1. **What it looks like** and the key time frames
2. **Who owns the theory** that explains it
3. **How to screen** for it
4. **What helps**, and where the counselor's scope ends

Time frames are high-yield. Many NCE diagnostic items turn on duration alone. They're collected in one table at the end of the chapter.

Part 1 — Depression and Hopelessness

Presentations

Presentation	Key features	Time frame
Major depressive episode	At least **5 of 9** symptoms, including **depressed mood or loss of interest/ pleasure (anhedonia)**; clinically significant distress or impairment	At least **2 weeks**
Persistent depressive disorder (dysthymia)	Depressed mood most of the day, more days than not, plus at least 2 other symptoms	At least **2 years** in adults (**1 year** in children and adolescents, and mood can be irritable)
Premenstrual dysphoric disorder	Mood symptoms in the final week before menses that improve after onset	Most cycles in the past year
Disruptive mood dysregulation disorder (DMDD)	Severe recurrent temper outbursts (3 or more per week) with persistently irritable mood between them	At least **12 months**; not diagnosed before age 6 or after age 18; onset before age 10
Adjustment disorder with depressed mood	Emotional or behavioral symptoms in response to an identifiable stressor, not meeting criteria for another disorder	Begins within **3 months** of the stressor; resolves within **6 months** after the stressor ends

The nine symptoms of a major depressive episode (a common memory aid is **SIG E CAPS** plus depressed mood): **S**leep change, loss of **I**nterest, **G**uilt or worthless-

ness, low **E**nergy, poor **C**oncentration, **A**ppetite or weight change, **P**sychomotor agitation or slowing, **S**uicidal thoughts, and depressed mood.

Useful specifiers: - **With peripartum onset**: onset during pregnancy or within 4 weeks after delivery - **With seasonal pattern**: a regular relationship between episodes and a particular time of year - **With anxious distress**, **with melancholic features**, **with psychotic features**

Grief and depression: the DSM-5 removed the "bereavement exclusion," so major depression can be diagnosed during bereavement if criteria are met. Normal grief tends to come in waves tied to reminders of the loss, with self-esteem usually preserved; depression is more pervasive, with worthlessness and self-loathing. Prolonged grief disorder is covered in Chapter 12.

Physical Signs of Depression

Sleep changes, appetite and weight changes, fatigue, psychomotor slowing or agitation, unexplained aches and pain, low libido, and slowed thinking. **Medical conditions can cause or mimic depression**, especially hypothyroidism, anemia, vitamin deficiencies, and some medications. A counselor who sees new depressive symptoms should make sure the client has had a recent medical evaluation.

Who Owns the Theories of Depression

Owner	Theory	Core idea
Aaron Beck	Cognitive theory	Cognitive triad (negative views of self, world, future); distortions; schemas
Martin Seligman	**Learned helplessness**	Repeated uncontrollable bad events teach that action is useless

Owner	Theory	Core idea
Abramson, Seligman, & Teasdale	Reformulated helplessness (attributional) theory	Depression follows attributing bad events to **internal, stable, global** causes
Abramson, Metalsky, & Alloy	**Hopelessness theory**	Expecting bad outcomes with no way to change them leads to a hopelessness subtype of depression
Peter Lewinsohn	Behavioral theory	Too little **response-contingent positive reinforcement**; basis for behavioral activation
Susan Nolen-Hoeksema	**Response styles theory**	**Rumination** (repetitively focusing on distress and its causes) prolongs and deepens depression; distraction and problem-solving shorten it
Klerman & Weissman	Interpersonal therapy (IPT)	Depression linked to four problem areas: grief, role disputes, role transitions, interpersonal deficits
John Bowlby	Attachment	Early loss and insecure attachment increase vulnerability

Rumination is repetitive, passive dwelling on one's distress. It's the core target in rumination-focused CBT and mindfulness-based cognitive therapy (MBCT; Segal, Williams, & Teasdale), which was developed to prevent depressive relapse.

Screening Tools

Tool	Notes
PHQ-9 (Kroenke, Spitzer, & Williams)	9 items matching the DSM symptoms. **Item 9 asks about thoughts of death or self-harm; a positive answer requires a suicide risk assessment**
PHQ-2	First two PHQ-9 items as a quick screen
BDI-II (Beck)	21-item self-report
Beck Hopelessness Scale	Hopelessness as a suicide risk indicator
Hamilton Depression Rating Scale (HAM-D)	Clinician-rated
Edinburgh Postnatal Depression Scale (EPDS)	Perinatal depression
Geriatric Depression Scale (GDS)	Older adults
Children's Depression Inventory (CDI)	Children and adolescents

What Helps

- **CBT**, **behavioral activation**, and **IPT** have strong support.
- **MBCT** for preventing relapse after recovery.
- **Combined psychotherapy and medication** often works better than either alone for moderate to severe depression.
- **Always assess suicide risk** (Chapter 6).
- Counselors don't prescribe. Refer for a medication evaluation when symptoms are severe, don't respond to counseling, or include psychotic features. Know that antidepressants carry an FDA boxed warning about increased suicidal thoughts and behaviors in children, adolescents, and young adults under 25, so monitoring early in treatment matters.

Part 2 — Mood Swings: Bipolar Spectrum

Presentation	Key features	Time frame
Manic episode	Abnormally elevated, expansive, or irritable mood **and** increased energy or activity, plus at least 3 symptoms (4 if mood is only irritable); marked impairment, hospitalization, or psychotic features	At least **1 week** (any duration if hospitalized)
Hypomanic episode	Same symptom pattern, but **not** severe enough for marked impairment or hospitalization, and **no psychotic features**	At least **4 consecutive days**
Bipolar I	At least one manic episode (depression common but not required)	—
Bipolar II	At least one hypomanic episode **and** at least one major depressive episode; **never** a full manic episode	—
Cyclothymic disorder	Many periods of hypomanic and depressive symptoms that don't meet full episode criteria	At least **2 years** in adults (**1 year** in children and adolescents)

Manic symptoms (a common memory aid is **DIG FAST**): **D**istractibility, **I**mpulsivity or risky activities, **G**randiosity, **F**light of ideas or racing thoughts, increased goal-directed **A**ctivity or agitation, decreased need for **S**leep, **T**alkativeness or pressured speech.

Why it matters for counselors: - Ask about past manic or hypomanic periods before treating depression. Antidepressants alone can trigger mania in people with bipolar

disorder. - Bipolar disorder needs a medication evaluation. Mood stabilizers (such as lithium) are a mainstay; counseling supports adherence, sleep regularity, early warning signs, and family psychoeducation. - **Interpersonal and social rhythm therapy** (IPSRT; Ellen Frank) stabilizes daily routines and sleep. - Screening: **Mood Disorder Questionnaire (MDQ)**.

Part 3 — Worry, Fear, and Panic

Fear vs. Anxiety

- **Fear** is the response to a real or perceived **immediate** threat.
- **Anxiety** is anticipation of a **future** threat.
- **Worry** is the thinking part of anxiety: repetitive "what if" thoughts about possible bad outcomes.

Presentations

Presentation	Key features	Time frame
Generalized anxiety disorder	Excessive, hard-to-control worry about many things, plus at least 3 of 6 symptoms (restlessness, fatigue, poor concentration, irritability, muscle tension, sleep problems); 1 symptom needed in children	At least **6 months**
Panic disorder	**Recurrent unexpected** panic attacks, followed by persistent worry about more attacks or a change in behavior to avoid them	At least **1 month** of worry or behavior change after an attack

Presentation	Key features	Time frame
Panic attack (specifier, not a disorder)	Abrupt surge of intense fear peaking within minutes, with at least 4 of 13 symptoms	Peaks within minutes
Agoraphobia	Fear or avoidance of at least **2 of 5** situations (public transportation, open spaces, enclosed spaces, lines or crowds, being outside home alone) because escape or help might be hard	Typically **6 months** or more
Specific phobia	Marked fear of a specific object or situation (animal, natural environment, blood-injection-injury, situational, other)	Typically **6 months** or more
Social anxiety disorder	Fear of social situations where one might be scrutinized or judged negatively; **performance only** specifier	Typically **6 months** or more
Separation anxiety disorder	Excessive fear about separation from attachment figures	At least **4 weeks** in children and adolescents; typically **6 months** or more in adults
Selective mutism	Consistently not speaking in specific situations (such as school) despite speaking in others	At least **1 month** (not just the first month of school)

Blood-injection-injury phobia is the one phobia that often involves a **vasovagal response**: heart rate and blood pressure rise briefly, then drop, and the person may faint. Treatment can include **applied tension** (Öst), tensing muscles to keep blood pressure up during exposure.

Physical Signs of Anxiety

Racing or pounding heart, shortness of breath, chest tightness, dizziness, trembling, sweating, nausea and GI upset, muscle tension, headaches, and sleep problems. **Medical rule-outs matter**: hyperthyroidism, cardiac conditions, asthma, low blood sugar, caffeine, stimulants, and substance withdrawal can all cause anxiety symptoms. A first panic attack, especially with chest pain, warrants medical evaluation.

Who Owns the Theories of Anxiety

Owner	Theory	Core idea
O. Hobart Mowrer	**Two-factor theory**	Fear is **acquired** through classical conditioning and **maintained** through operant conditioning (avoidance is negatively reinforced because it reduces anxiety)
David Clark	Cognitive model of panic	**Catastrophic misinterpretation** of normal body sensations ("My heart is racing, so I'm having a heart attack")
David Barlow	Panic control treatment; Unified Protocol	Interoceptive exposure; transdiagnostic treatment for emotional disorders
Thomas Borkovec	Avoidance theory of worry	Worry is a mostly verbal activity that dampens emotional and physical arousal, so it's negatively reinforced and kept going
Michel Dugas	Intolerance of uncertainty model	People with GAD find uncertainty especially hard to tolerate

Owner	Theory	Core idea
Adrian Wells	Metacognitive model	Beliefs *about* worry ("Worrying keeps me prepared"; "My worry is uncontrollable") drive GAD
Clark & Wells	Cognitive model of social anxiety	Self-focused attention and safety behaviors maintain social anxiety
Martin Seligman	**Preparedness**	People are biologically prepared to fear some things (snakes, heights) more easily than others
Yerkes & Dodson	**Yerkes–Dodson law**	Performance improves with arousal up to a point, then declines

Screening Tools

Tool	Notes
GAD-7 (Spitzer et al.)	7-item anxiety screen
BAI (Beck)	21-item self-report focused on physical symptoms
Hamilton Anxiety Rating Scale (HAM-A)	Clinician-rated
State-Trait Anxiety Inventory (STAI) (Spielberger)	Separates **state** anxiety (right now) from **trait** anxiety (general tendency)
Liebowitz Social Anxiety Scale	Social anxiety
SCARED	Child anxiety

What Helps

- **Exposure** is central for phobias, panic, and social anxiety (Chapter 5): graded exposure, interoceptive exposure for panic, and in vivo exposure for agoraphobia.

- **CBT** for GAD and panic: cognitive restructuring, worry exposure, problem-solving, relaxation, and tolerance of uncertainty.
- **Reduce safety behaviors and avoidance**, which maintain the anxiety cycle.
- **Benzodiazepines** relieve anxiety quickly but carry risks of dependence and withdrawal and can interfere with exposure-based learning. This is a medication-prescriber decision; the counselor's role is to coordinate care and educate.

Part 4 — Obsessions and Intrusive Thoughts

Presentations

Presentation	Key features
Obsessive-compulsive disorder	**Obsessions** (intrusive, unwanted thoughts, urges, or images that cause anxiety) and/or **compulsions** (repetitive behaviors or mental acts done to reduce anxiety or prevent a feared outcome); time-consuming (often over 1 hour a day) or impairing. Insight specifiers: **good or fair**, **poor**, **absent/ delusional**. **Tic-related** specifier
Body dysmorphic disorder	Preoccupation with perceived flaws in appearance that others don't see or see as slight; repetitive checking, grooming, or comparing
Hoarding disorder	Persistent difficulty discarding possessions, regardless of value, leading to cluttered living areas
Trichotillomania	Recurrent hair pulling with attempts to stop
Excoriation disorder	Recurrent skin picking with attempts to stop

Common obsession themes: contamination, harm (fear of hurting others), symmetry and exactness, unacceptable sexual thoughts, religious or moral obsessions (scrupulosity), and doubt.

Obsessions vs. Similar Experiences

Experience	How it differs from an obsession
GAD worry	Worry is about real-life concerns (money, health, family) and feels reasonable to the person; obsessions are often intrusive, unwanted, and feel alien (**ego-dystonic**)
Depressive rumination	Rumination dwells on past losses and one's own distress; it isn't usually experienced as intrusive and doesn't drive compulsions
Obsessive-compulsive personality disorder (OCPD)	A pervasive pattern of perfectionism, orderliness, and control that the person usually sees as right (**ego-syntonic**); no true obsessions or compulsions
Delusions	Fixed false beliefs held with full conviction; OCD obsessions are usually recognized as excessive (except with absent insight)
Intrusive trauma memories	Tied to a specific traumatic event (PTSD)

Who Owns the Theories of OCD

Owner	Contribution
Stanley Rachman	Cognitive theory: everyone has intrusive thoughts; OCD develops when the person interprets them as meaningful or dangerous
Paul Salkovskis	**Inflated responsibility**: belief that one is responsible for preventing harm drives neutralizing and compulsions

Owner	Contribution
Mowrer	Two-factor theory applies: compulsions are negatively reinforced because they reduce anxiety
Victor Meyer	Early development of exposure and response prevention
Edna Foa	Developed and studied ERP extensively

Screening and Treatment

- **Y-BOCS** (Yale-Brown Obsessive Compulsive Scale; Goodman et al.); **CY-BOCS** for children.
- **ERP** has the strongest support (Chapter 5).
- **SSRIs** (often at higher doses) are a common medication approach; this is a prescriber decision.
- **Reassurance is a compulsion.** Repeatedly reassuring a client with OCD ("No, you didn't hit anyone with your car") reduces anxiety short-term but strengthens the cycle. The scored answer helps the client tolerate uncertainty instead.

Part 5 — Trauma (2B.1)

Presentations

Presentation	Key features	Time frame
Acute stress disorder	Trauma symptoms right after exposure	**3 days to 1 month** after the trauma
Posttraumatic stress disorder	Exposure to actual or threatened death, serious injury, or sexual violence, followed by symptoms in **four clusters**	More than **1 month**
Adjustment disorder	Distress after a stressor that need not be traumatic	Begins within **3 months**; resolves within **6 months** after the stressor ends

Presentation	Key features	Time frame
Reactive attachment disorder	Inhibited, emotionally withdrawn behavior toward caregivers after severe neglect	Evident before age 5; developmental age of at least 9 months
Disinhibited social engagement disorder	Overly familiar behavior with unfamiliar adults after severe neglect	Developmental age of at least 9 months

PTSD's Four Symptom Clusters

Cluster	Examples
Intrusion	Intrusive memories, nightmares, flashbacks, distress at reminders, physical reactions to reminders
Avoidance	Avoiding memories, thoughts, feelings, people, places, or situations linked to the trauma
Negative alterations in cognition and mood	Amnesia for parts of the event, persistent negative beliefs, distorted blame, persistent negative emotions, loss of interest, detachment, inability to feel positive emotions
Alterations in arousal and reactivity	Irritability or anger, reckless behavior, hypervigilance, exaggerated startle, poor concentration, sleep problems

Specifiers: **with dissociative symptoms** (depersonalization or derealization) and **with delayed expression** (full criteria not met until at least 6 months after the event). There are separate criteria for children 6 and younger.

Exposure can be direct or indirect: experiencing the event, witnessing it, learning it happened to a close family member or friend (if violent or accidental), or repeated or extreme exposure to aversive details (as for first responders). Exposure through media doesn't count unless it's work-related.

Who Owns Trauma Theory

Owner	Contribution
Judith Herman	*Trauma and Recovery*; **complex trauma** from prolonged, repeated trauma; three stages of recovery: **(1) safety and stabilization, (2) remembrance and mourning, (3) reconnection**
Bessel van der Kolk	Trauma's effects on the body and brain; *The Body Keeps the Score*
Daniel Siegel	**Window of tolerance**: the zone of arousal where a person can function; trauma narrows it, causing swings into **hyperarousal** (anxiety, panic, anger) or **hypoarousal** (numbness, shutdown, dissociation)
Felitti & Anda	The **Adverse Childhood Experiences (ACE) Study** (1998): more childhood adversity is linked to higher risk of later physical and mental health problems
Ronnie Janoff-Bulman	**Shattered assumptions**: trauma breaks basic beliefs that the world is benevolent and meaningful and that the self is worthy
Tedeschi & Calhoun	**Posttraumatic growth**: positive change that can follow struggle with trauma
George Bonanno	**Resilience** is the most common response to potential trauma
Stephen Porges	Polyvagal theory

Physical and Emotional Effects of Trauma (2B.1)

Hyperarousal, sleep disturbance, chronic pain, headaches, GI problems, fatigue, startle, numbing, shame, guilt, anger, difficulty trusting, and dissociation. The ACE research links childhood adversity to later heart disease, substance use, depression, and other health problems.

Trauma-Informed Care (SAMHSA)

SAMHSA describes six principles: **safety**; **trustworthiness and transparency**; **peer support**; **collaboration and mutuality**; **empowerment, voice, and choice**; and **cultural, historical, and gender issues**. Its four "Rs": a trauma-informed program **realizes** the impact of trauma, **recognizes** its signs, **responds** by building that knowledge into practice, and **resists re-traumatization**.

Screening and Treatment

Tool	Notes
PCL-5	PTSD Checklist for DSM-5; 20-item self-report
CAPS-5	Clinician-Administered PTSD Scale; structured interview, the standard for diagnosis in research
PC-PTSD-5	Brief primary care screen
Life Events Checklist (LEC-5)	Identifies trauma exposure
ACE questionnaire	Childhood adversity history

Treatments with strong support: prolonged exposure (Foa), cognitive processing therapy (Resick), EMDR (Shapiro), and trauma-focused CBT for children and adolescents (Cohen, Mannarino, & Deblinger). See Chapter 5.

The phase-oriented rule: stabilize before processing. A client who is unsafe, using substances heavily, or frequently dissociating needs safety and coping skills first (Herman's stage 1). Pushing for a detailed trauma narrative before the client can stay within the window of tolerance is a scored error.

Part 6 — Emotional Dysregulation (4.3)

Emotional dysregulation is difficulty managing the intensity and duration of emotions: quick, intense reactions; slow return to baseline; and impulsive behavior to escape the feeling. It appears across many presentations, includ-

ing borderline personality disorder, PTSD and complex trauma, bipolar disorder, ADHD, and DMDD in children.

Owner	Contribution
Marsha Linehan	**Biosocial theory**: emotional vulnerability (biological sensitivity) plus an **invalidating environment** produces pervasive emotion dysregulation; DBT
James Gross	**Process model of emotion regulation**: situation selection, situation modification, attentional deployment, cognitive change (reappraisal), response modulation (e.g., suppression)
Daniel Siegel	Window of tolerance; "name it to tame it" (labeling emotions helps regulate them)

What helps: DBT skills (mindfulness, distress tolerance, emotion regulation, interpersonal effectiveness), validation, grounding, the window of tolerance as a shared language, and attention to safety when dysregulation includes self-harm or suicidal behavior (Chapter 6).

Time Frames at a Glance

Presentation	Time frame
Major depressive episode	2 weeks
Persistent depressive disorder	2 years adults; 1 year children/adolescents
Manic episode	1 week (any duration if hospitalized)
Hypomanic episode	4 days
Cyclothymic disorder	2 years adults; 1 year children/adolescents
DMDD	12 months; diagnosed ages 6–18; onset before 10
GAD	6 months

Presentation	Time frame
Panic disorder	1 month of worry/behavior change after attacks
Specific phobia, social anxiety, agoraphobia	Typically 6 months
Separation anxiety	4 weeks (children/adolescents); typically 6 months (adults)
Selective mutism	1 month (not the first month of school)
Acute stress disorder	3 days to 1 month after trauma
PTSD	More than 1 month; delayed expression if criteria met 6+ months later
Adjustment disorder	Onset within 3 months; resolves within 6 months after stressor ends

What the Exam Rewards / What It Punishes

Rewards: - Knowing the time frames - Asking about past mania before treating depression - Medical rule-outs for new depression or anxiety symptoms - Following up any positive PHQ-9 item 9 with a suicide risk assessment - Exposure for phobias, panic, and OCD; reducing avoidance and safety behaviors - Stabilizing before trauma processing - Validation and skills for emotional dysregulation

Punishes: - Reassuring a client with OCD repeatedly - Pushing trauma detail before safety and stabilization - Assuming grief rules out depression - Treating anxiety only with relaxation when exposure is indicated - Recommending or adjusting medications (outside scope)

Swap-Pair Drill

If you see...	It belongs to...	Not...
Learned helplessness	Seligman	Beck
Internal, stable, global attributions	Abramson, Seligman, & Teasdale	Lewinsohn

If you see…	It belongs to…	Not…
Too little response-contingent reinforcement	Lewinsohn	Seligman
Rumination prolongs depression	Nolen-Hoeksema	Beck
Fear acquired by classical, maintained by operant	Mowrer's two-factor theory	Wolpe
Catastrophic misinterpretation of body sensations	Clark	Borkovec
Worry as avoidance of emotional arousal	Borkovec	Clark
Intolerance of uncertainty	Dugas	Wells
Beliefs about worry	Wells (metacognitive)	Dugas
Inflated responsibility in OCD	Salkovskis	Rachman
Safety, remembrance and mourning, reconnection	Herman	Kübler-Ross
Window of tolerance	Siegel	Porges
ACE Study	Felitti & Anda	van der Kolk
Shattered assumptions	Janoff-Bulman	Tedeschi & Calhoun
Posttraumatic growth	Tedeschi & Calhoun	Bonanno
Biosocial theory of dysregulation	Linehan	Gross
Process model of emotion regulation	Gross	Linehan
State vs. trait anxiety	Spielberger (STAI)	Beck (BAI)
Ego-dystonic obsessions	OCD	OCPD (ego-syntonic)

Quick Checks

1. A client reports 3 weeks of depressed mood, insomnia, poor appetite, fatigue, trouble concentrating, and feelings

of worthlessness. This best fits: A. Persistent depressive disorder B. A major depressive episode C. Cyclothymic disorder D. Adjustment disorder

2. A client seeking help for depression says that last spring she slept 3 hours a night for 5 days, felt “on top of the world,” and started three businesses, but kept working normally. The counselor should first: A. Begin CBT for depression B. Explore this history further because it suggests possible hypomania C. Recommend an antidepressant D. Diagnose persistent depressive disorder

3. The theory that depression results from attributing failures to internal, stable, and global causes belongs to: A. Lewinsohn B. Nolen-Hoeksema C. Abramson, Seligman, and Teasdale D. Klerman and Weissman

4. A client has had sudden episodes of racing heart, shortness of breath, and fear of dying, “out of the blue,” and now avoids driving in case one happens. The most likely presentation is: A. GAD B. Panic disorder C. Specific phobia D. Social anxiety disorder

5. According to Mowrer’s two-factor theory, avoidance maintains fear because it is: A. Positively reinforced B. Negatively reinforced C. Punished D. Extinguished

6. A client with OCD asks the counselor several times per session whether she might have accidentally poisoned her family. The best counselor response is to: A. Reassure her each time that she didn’t B. Help her notice the reassurance-seeking and tolerate the uncertainty C. Refer her for a psychotic disorder evaluation D. Explore the childhood origins of her fear

7. Symptoms began 10 days after a car accident and include nightmares, avoidance of driving, and hypervigilance. The most fitting presentation is: A. PTSD B. Acute stress disorder C. Adjustment disorder D. Specific phobia

8. A client with complex trauma, frequent dissociation, and recent self-harm asks to “talk through everything that happened” in the first session. The best approach is to: A.

Begin a detailed trauma narrative right away B. Focus first on safety, stabilization, and coping skills C. Refer the client to inpatient care D. Tell the client she isn't ready for counseling

9. A positive response on PHQ-9 item 9 requires the counselor to: A. Score the remaining items first B. Conduct a suicide risk assessment C. Refer immediately for hospitalization D. Repeat the PHQ-9 next week

10. Daniel Siegel's window of tolerance describes: A. The range of arousal in which a person can function and process experience B. Stages of grief C. The period after trauma before PTSD can be diagnosed D. Tolerance to a substance

11. A client believes, "If I stop worrying, something bad will happen to my kids." This belief is central to which model? A. Clark's cognitive model of panic B. Wells's metacognitive model C. Seligman's preparedness D. Linehan's biosocial theory

12. A new client describes sudden anxiety, weight loss, heat intolerance, and a racing heart. The counselor's most important step is to: A. Teach diaphragmatic breathing B. Ensure she gets a medical evaluation C. Start exposure therapy D. Diagnose GAD

Answers and Rationales

1 — B. Five or more symptoms including depressed mood for at least 2 weeks. A: PDD requires 2 years. C: cyclothymia requires 2 years of fluctuating hypomanic and depressive symptoms. D: adjustment disorder requires an identifiable stressor and not meeting criteria for another disorder.

2 — B. Four or more days of decreased need for sleep, elevated mood, and increased goal-directed activity without marked impairment suggests hypomania; exploring this is essential before treating depression. A: risks missing bipolar II. C: outside scope, and antidepressants alone can trigger mania. D: ignores the hypomanic history.

3 — C. The reformulated helplessness (attributional) theory. A: Lewinsohn emphasizes low reinforcement. B: Nolen-Hoeksema emphasizes rumination. D: Klerman and Weissman developed IPT.

4 — B. Recurrent unexpected attacks followed by behavior change. A: GAD involves ongoing worry, not discrete unexpected attacks. C: a phobia is triggered by a specific object or situation. D: social anxiety centers on fear of judgment.

5 — B. Avoidance removes anxiety, which negatively reinforces it. A: nothing pleasant is added. C: punishment would decrease avoidance. D: extinction would weaken fear, which avoidance prevents.

6 — B. Reassurance is a compulsion; tolerating uncertainty is the therapeutic goal. A: strengthens the cycle. C: she recognizes the fear as excessive, which doesn't suggest psychosis. D: misses the maintaining cycle.

7 — B. Symptoms between 3 days and 1 month after trauma. A: PTSD requires more than 1 month. C: adjustment disorder applies when criteria for a trauma disorder aren't met. D: a phobia doesn't explain the nightmares and hypervigilance.

8 — B. Herman's stage 1: safety and stabilization first. A: risks overwhelming the client and pushing her outside her window of tolerance. C: not indicated without imminent risk. D: abandons the client.

9 — B. Item 9 screens for thoughts of death or self-harm and requires follow-up assessment. A: delays assessing safety. C: over-response before assessment. D: ignores a possible risk.

10 — A. The optimal arousal zone (Siegel). B: stage models of loss belong to Kübler-Ross (dying) or Bowlby (mourning), not Siegel. C: the 1-month PTSD threshold is a DSM time frame. D: tolerance is a substance term.

11 — B. Positive beliefs about worry are central to Wells's metacognitive model. A: Clark's model concerns

catastrophic misreading of body sensations. C: preparedness concerns biologically easy-to-learn fears. D: Linehan's theory concerns emotion dysregulation.

12 — B. These symptoms suggest a possible medical cause (such as hyperthyroidism); rule it out first. A and C: treat before ruling out a medical cause. D: diagnoses without ruling out medical conditions and without 6 months of worry.

Audio Companion

Track	Covers	Target length
10.1	Time Frames and Owners: mood, anxiety, OCD, trauma	~8 min

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Chapter 11 — Substance Use and Process Addictions

2027 task map: 2B.3 Assess substance use · 2B.6 Determine co-occurring diagnoses, differential diagnoses, and other comorbidities · 2B.7 Determine the appropriate level of care · CACREP: Counseling and Helping Relationships; Assessment and Testing; Human Growth and Development · 2027 supplemental content guidance: presenting concerns

Where This Chapter Fits

Chapter 9 covered how counselors **treat** addiction: models, screening tools, the ASAM dimensions, motivational and behavioral approaches, mutual support, and confidentiality under 42 CFR Part 2. This chapter covers how addiction **presents**: the diagnostic criteria, severity levels, what intoxication and withdrawal look like for each substance class, and behavioral addictions such as gambling.

NCE items here usually ask one of four things: 1. How many criteria, and what severity? 2. What do intoxication or withdrawal from this substance look like? 3. Is this withdrawal dangerous? 4. Is this a substance-induced problem or an independent disorder?

Part 1 — Substance Use Disorder Criteria

The DSM-5-TR uses one set of **11 criteria** for substance use disorders, grouped into four clusters. Symptoms must cause clinically significant impairment or distress within a **12-month period**.

Cluster	Criteria
Impaired control	1. Using more, or for longer, than intended · 2. Persistent desire or unsuccessful efforts to cut down · 3. A lot of time spent getting, using, or recovering · 4. **Craving**
Social impairment	5. Failure to fulfill major role obligations · 6. Continued use despite social or interpersonal problems · 7. Giving up important activities
Risky use	8. Recurrent use in physically hazardous situations · 9. Continued use despite knowing it causes or worsens a physical or psychological problem
Pharmacological	10. **Tolerance** · 11. **Withdrawal**

Severity

Severity	Criteria met
Mild	2–3
Moderate	4–5
Severe	6 or more

Important exceptions: tolerance and withdrawal **don't count** toward the diagnosis when they occur only with medications taken as prescribed under medical supervision (opioid pain medications, sedatives, stimulants). A patient taking prescribed opioids who develops tolerance doesn't have an opioid use disorder on that basis alone.

Remission Specifiers

Specifier	Meaning
Early remission	At least 3 months but less than 12 months with no criteria met, **except craving**
Sustained remission	12 months or more with no criteria met, **except craving**
In a controlled environment	Access to substances is restricted (e.g., a locked facility)

Specifier	Meaning
On maintenance therapy	For opioid and tobacco use: taking an agonist or antagonist medication (e.g., methadone, buprenorphine, nicotine replacement)

DSM History the NCE May Test

The DSM-IV had two separate diagnoses, **substance abuse** and **substance dependence**. The DSM-5 combined them into a single **substance use disorder** on a severity continuum, dropped the "legal problems" criterion, and added **craving**.

Part 2 — Substance Classes: Intoxication and Withdrawal

The DSM-5-TR recognizes ten classes: alcohol; caffeine; cannabis; hallucinogens (phencyclidine and other hallucinogens); inhalants; opioids; sedatives, hypnotics, and anxiolytics; stimulants; tobacco; and other or unknown substances.

Class	Intoxication	Withdrawal	Withdrawal danger
Alcohol	Slurred speech, poor coordination, unsteady gait, nystagmus, impaired attention or memory, stupor or coma	Tremor, sweating, rapid pulse, insomnia, nausea, anxiety, agitation, transient hallucinations, **seizures**; **delirium tremens** (confusion, hallucinations, autonomic instability) typically 2–4 days after stopping	**Potentially life-threatening**

Class	Intoxication	Withdrawal	Withdrawal danger
Sedatives, hypnotics, anxiolytics (benzodiazepines, barbiturates)	Like alcohol: slurred speech, poor coordination, memory problems	Like alcohol: anxiety, tremor, insomnia, **seizures**, delirium	**Potentially life-threatening**; taper under medical care
Opioids (heroin, fentanyl, oxycodone)	**Pinpoint (constricted) pupils**, drowsiness, slurred speech, **slowed breathing**; overdose can be fatal	**Flu-like**: dilated pupils, runny nose, tearing, yawning, goosebumps, sweating, muscle aches, nausea, vomiting, diarrhea, insomnia	Very uncomfortable but **rarely life-threatening** in otherwise healthy adults; the big danger is **overdose after tolerance drops**
Stimulants (cocaine, methamphetamine, amphetamines)	Euphoria, **dilated pupils**, rapid heart rate, high blood pressure, sweating, agitation, paranoia, possible psychosis	**"Crash"**: depressed mood, fatigue, vivid unpleasant dreams, increased sleep and appetite	Not usually medically dangerous, but **depression and suicide risk** can be high
Cannabis	Euphoria or anxiety, red eyes, increased appetite, dry mouth, rapid heart rate, impaired coordination and time sense	Irritability, anxiety, sleep difficulty, decreased appetite, restlessness, depressed mood	Not medically dangerous

Class	Intoxication	Withdrawal	Withdrawal danger
Tobacco/nicotine	(No intoxication diagnosis)	Irritability, anxiety, difficulty concentrating, increased appetite, restlessness, depressed mood, insomnia	Not medically dangerous
Caffeine	Restlessness, nervousness, insomnia, flushed face, GI upset, rapid heart rate	Headache, fatigue, low mood, difficulty concentrating	Not medically dangerous
Hallucinogens (LSD, psilocybin, MDMA)	Perceptual changes, synesthesia, dilated pupils, anxiety or paranoia	**No withdrawal diagnosis**	—
Phencyclidine (PCP)	Agitation, violence, numbness, nystagmus, high blood pressure	**No withdrawal diagnosis**	—
Inhalants (glue, solvents, aerosols)	Dizziness, slurred speech, incoordination, euphoria; most common among younger adolescents; risk of sudden death	**No withdrawal diagnosis**	—

High-yield distinctions: - **Pinpoint pupils → opioids. Dilated pupils → stimulants** (and hallucinogens; opioid *withdrawal*). - **Alcohol and sedative withdrawal can kill.** Refer for medical evaluation and medically managed withdrawal. - **Opioid overdose → naloxone** (Narcan) can

reverse it. Fentanyl's potency has made accidental overdose much more common. - **Mixing respiratory depressants** (alcohol + benzodiazepines, or opioids + benzodiazepines) multiplies the risk of fatal respiratory depression. - After a period of abstinence (detox, incarceration, treatment), **tolerance drops**. Returning to a previous dose can be fatal. This is a key relapse-prevention teaching point. - **Hallucinogen persisting perception disorder**: flashbacks of perceptual changes after stopping hallucinogens.

Part 3 — Substance-Induced vs. Independent Disorders (2B.6)

Many people who use substances also have depression, anxiety, psychosis, or sleep problems. The question is whether the substance is causing the symptoms.

Points toward substance-induced	Points toward an independent disorder
Symptoms began during or soon after intoxication or withdrawal	Symptoms **began before** substance use
Symptoms improve with sustained abstinence	Symptoms **persist for a substantial period** (about **1 month**) after acute withdrawal or severe intoxication ends
The substance is known to cause these symptoms	There's a history of the disorder during periods without substance use

Co-occurring disorders are common. Integrated treatment that addresses both at once, ideally by the same team, is the standard (Chapter 9). Don't wait for sobriety before treating a co-occurring mental health problem, and don't ignore the substance use while treating the mental health problem.

Part 4 — Who Owns Addiction Theory

Owner	Contribution
E. M. Jellinek	Disease concept of alcoholism; described **phases**: prealcoholic, prodromal, crucial, chronic
Alan Marlatt	Relapse prevention; abstinence violation effect; harm reduction advocate
Miller & Rollnick	Motivational interviewing
Prochaska & DiClemente	Stages of change
Sharon Wegscheider-Cruse	Family roles in addiction (hero, scapegoat, lost child, mascot, enabler)
Claudia Black	"Don't talk, don't trust, don't feel" rules in alcoholic families
Hunt & Azrin	Community reinforcement approach (CRA)
Robert Meyers	CRAFT (Community Reinforcement and Family Training), developed from CRA
Bill W. and Dr. Bob	Founded Alcoholics Anonymous (1935)
José Szapocznik	Brief Strategic Family Therapy (BSFT) for adolescent substance use
Howard Liddle	Multidimensional Family Therapy (MDFT) for adolescent substance use

Risk Factors

Family history and genetics, early age of first use, childhood adversity and trauma (ACEs; Chapter 10), co-occurring mental health conditions, peer use, easy access, and stressful life circumstances. Protective factors include strong family bonds, parental monitoring, school connection, and coping skills.

The Brain

Substances act on the brain's **reward pathway** (the mesolimbic dopamine system). Repeated use changes the

brain in ways that strengthen craving and habit and weaken self-control. This is the basis for viewing addiction as a chronic, relapsing condition rather than a moral failing.

Part 5 — Special Populations

Population	What the exam expects
Adolescents	Screen with **CRAFFT**; involve family; family-based approaches (BSFT, MDFT, adolescent CRA) have strong support; confidentiality rules for minors vary by state
Older adults	Alcohol and prescription misuse are often missed; lower tolerance; interactions with medications; falls and cognitive effects
Pregnant clients	Prioritize engagement and prenatal care; medications for opioid use disorder (methadone, buprenorphine) are recommended during pregnancy rather than medically supervised withdrawal
People with co-occurring disorders	Integrated treatment
Family members	Al-Anon, Alateen, CRAFT; address enabling and codependency

Part 6 — Process (Behavioral) Addictions

Gambling Disorder

The only behavioral addiction in the DSM-5-TR's Substance-Related and Addictive Disorders chapter.

- **At least 4 of 9 criteria in a 12-month period**: needing to gamble with increasing amounts to get excited (tolerance); restless or irritable when cutting down (withdrawal); repeated unsuccessful efforts to

stop; preoccupation; gambling when distressed; **"chasing" losses** (returning to win back money); lying to hide gambling; jeopardizing a relationship, job, or opportunity; relying on others for money to relieve a desperate financial situation.

- **Severity:** mild 4–5, moderate 6–7, severe 8–9.
- **Specifiers:** episodic or persistent; early or sustained remission.
- **Not better explained by a manic episode.**

"Chasing" losses is the classic gambling criterion.

Suicide risk is elevated in gambling disorder, especially with mounting debt. Always assess.

Treatment and support: CBT (addressing cognitive distortions like the "gambler's fallacy" and illusion of control), motivational interviewing, **Gamblers Anonymous**, **Gam-Anon** for families, financial counseling, and self-exclusion programs at casinos and online platforms.

Other Behavioral Problems

Problem	DSM-5-TR status
Internet gaming disorder	Section III: a **condition for further study**, not an official diagnosis
Compulsive sexual behavior / pornography use	Not a DSM-5-TR diagnosis; **ICD-11** includes **compulsive sexual behaviour disorder** as an impulse control disorder
Gaming disorder	**ICD-11** includes it as a disorder due to addictive behaviors
Compulsive buying, internet use, exercise	Not DSM diagnoses; addressed clinically when they cause distress or impairment

Exam principle: a counselor can address a behavior that is causing distress or impairment even when it isn't a formal diagnosis. Assess the function the behavior serves, its consequences, the client's own values, co-occurring problems (depression, anxiety, ADHD, trauma), and readi-

ness to change. Avoid imposing the counselor's moral views (Chapter 9, ACA A.4.b).

What the Exam Rewards / What It Punishes

Rewards: - Counting criteria and assigning severity correctly - Recognizing dangerous withdrawal (alcohol and sedatives) and arranging medical care - Knowing pupil signs (pinpoint = opioids; dilated = stimulants) - Teaching overdose risk after lost tolerance; naloxone - Deciding whether symptoms are substance-induced or independent - Integrated treatment for co-occurring disorders - Assessing suicide risk in gambling disorder and stimulant withdrawal

Punishes: - Counting prescribed-medication tolerance or withdrawal toward a diagnosis - Treating alcohol withdrawal as a counseling-only issue - Requiring sobriety before addressing a co-occurring disorder - Calling internet gaming disorder an official DSM diagnosis - Moralizing about behavioral addictions

Swap-Pair Drill

If you see…	It's…	Not…
SUD severity: 2–3 criteria	Mild	Moderate
SUD severity: 4–5 criteria	Moderate	Severe
SUD severity: 6 or more criteria	Severe	Moderate
SUD remission: 3–12 months without criteria (except craving)	Early remission	Sustained remission
SUD remission: 12+ months without criteria (except craving)	Sustained remission	Early remission

If you see…	It's…	Not…
Pinpoint pupils, slow breathing	Opioid intoxication	Stimulant intoxication
Runny nose, yawning, goosebumps, diarrhea	Opioid withdrawal	Alcohol withdrawal
Tremor, sweating, seizures, hallucinations	Alcohol or sedative withdrawal	Opioid withdrawal
Crash: fatigue, hypersomnia, hunger, low mood	Stimulant withdrawal	Cannabis withdrawal
Substances with no DSM withdrawal diagnosis	Hallucinogens, PCP, inhalants	Cannabis (which has one)
Chasing losses	Gambling disorder	Substance use disorder
Gambling disorder severity: 6–7 criteria	Moderate	Mild
Gaming addiction: DSM condition for further study	Internet gaming disorder	Gambling disorder
Prealcoholic, prodromal, crucial, chronic	Jellinek	Prochaska
Adolescent family therapy for substance use	BSFT (Szapocznik), MDFT (Liddle)	Filial therapy

Quick Checks

1. A client meets 5 substance use disorder criteria for alcohol in the past year. The severity is: A. Mild B. Moderate C. Severe D. Cannot be determined

2. A patient takes prescribed opioid pain medication as directed and has developed tolerance and mild withdrawal when doses are late. She meets no other criteria. She: A. Has mild opioid use disorder B. Doesn't meet criteria, because tolerance and withdrawal from prescribed use

don’t count C. Has moderate opioid use disorder D. Has opioid-induced depressive disorder

3. Which withdrawal syndrome is most likely to be medically life-threatening? A. Opioid B. Cannabis C. Alcohol D. Caffeine

4. A client arrives with pinpoint pupils, slurred speech, and very slow breathing. The most likely cause is: A. Cocaine intoxication B. Opioid intoxication C. Cannabis withdrawal D. Hallucinogen intoxication

5. After 6 weeks in jail, a client with opioid use disorder is released and plans to “celebrate just once.” The most important teaching point is: A. Opioid withdrawal is life-threatening B. His tolerance has likely dropped, so his previous dose could be fatal; carry naloxone and avoid using alone C. He should switch to stimulants D. He’s in sustained remission

6. Four months after stopping heavy drinking, a client has no use disorder symptoms except occasional cravings. The specifier is: A. Sustained remission B. Early remission C. In a controlled environment D. On maintenance therapy

7. A client’s depressive symptoms began during heavy cocaine use and resolved 2 weeks after he stopped. This best suggests: A. An independent major depressive disorder B. A substance-induced depressive disorder C. Bipolar disorder D. Persistent depressive disorder

8. “Chasing losses” is a criterion of: A. Stimulant use disorder B. Gambling disorder C. Internet gaming disorder D. Compulsive buying

9. In the DSM-5-TR, internet gaming disorder is: A. An official diagnosis in the addictions chapter B. A condition for further study in Section III C. A type of impulse control disorder D. Listed under gambling disorder

10. The DSM-5 changed substance diagnoses by: A. Adding a “legal problems” criterion B. Combining abuse and dependence into one use disorder and adding craving

C. Removing tolerance as a criterion D. Creating separate diagnoses for each severity level

11. Which substance class has no withdrawal diagnosis in the DSM-5-TR? A. Cannabis B. Tobacco C. Inhalants D. Caffeine

12. A client with severe gambling disorder has lost his savings and says his family "would be better off" with his life insurance. The counselor's first priority is to: A. Refer him to Gamblers Anonymous B. Conduct a suicide risk assessment C. Develop a budget D. Explore cognitive distortions about gambling

Answers and Rationales

1 — B. 4–5 criteria is moderate. A: mild is 2–3. C: severe is 6 or more. D: the count determines severity.

2 — B. Tolerance and withdrawal from medically supervised, prescribed use aren't counted. A and C: incorrectly count those criteria. D: no depressive symptoms are described.

3 — C. Alcohol (and sedative) withdrawal can cause seizures and delirium tremens. A: opioid withdrawal is miserable but rarely life-threatening in healthy adults. B and D: not medically dangerous.

4 — B. Pinpoint pupils and respiratory depression are classic opioid signs. A: cocaine dilates pupils and speeds the heart. C: cannabis withdrawal involves irritability and sleep problems. D: hallucinogens dilate pupils.

5 — B. Lost tolerance after abstinence makes overdose a serious risk; naloxone and not using alone are harm reduction steps. A: the danger is overdose, not withdrawal. C: harmful and not counseling. D: 6 weeks is too short for any remission specifier; early remission requires at least 3 months.

6 — B. At least 3 but less than 12 months with no criteria except craving. A: sustained requires 12 months or more.

C: he isn't in a restricted environment. D: no maintenance medication is mentioned.

7 — B. Onset during use and resolution with abstinence point to a substance-induced disorder. A: independent depression would typically predate use or persist a month or more after abstinence. C and D: not supported by the history.

8 — B. Chasing losses is a gambling disorder criterion. A: stimulant use disorder uses the 11 substance criteria. C: internet gaming disorder is a condition for further study. D: compulsive buying isn't a DSM diagnosis.

9 — B. It's in Section III as a condition for further study. A: only gambling disorder is in the chapter. C and D: incorrect placement.

10 — B. DSM-5 merged abuse and dependence into one use disorder and added craving. A: the legal problems criterion was removed. C: tolerance remains. D: severity is a specifier of one diagnosis.

11 — C. Inhalants (along with hallucinogens and PCP) have no withdrawal diagnosis. A, B, D: cannabis, tobacco, and caffeine all have withdrawal diagnoses.

12 — B. A statement suggesting he'd be better off dead, plus financial devastation, requires a suicide risk assessment first (Chapter 6). A, C, D: useful later, but safety comes first.

Audio Companion

Track	Covers	Target length
11.1	Count It and Spot It: criteria, severity, intoxication, withdrawal, gambling	~5 min

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Chapter 12 — Life Span and Development

2027 task map: 2B.11 Select, administer, and/or interpret appropriate instruments/assessments that align with contextual, cultural, and developmental considerations · 4.6 Implement empirically supported interventions · CACREP: Human Growth and Development · 2027 supplemental content guidance: presenting concerns

From Owners to Clients

Chapter 3 gave you the developmental theorists and their stages: Erikson, Piaget, Vygotsky, Kohlberg, Gilligan, Mahler, Marcia, Bowlby, Ainsworth, Baumrind, Kübler-Ross, Worden, Selye, and Schlossberg. This chapter puts those owners to work on the concerns clients actually bring:

1. Children's development and neurodevelopmental differences
2. Separation from caregivers
3. Adolescence and adulthood
4. Aging and retirement
5. Grief and loss
6. Terminal illness and the end of life

The theme that runs through the chapter: **normal developmental struggle is not pathology.** The NCE rewards answers that frame a problem as a developmental task under strain, while still screening for the disorders that can come with it.

Part 1 — Child Development and Neurodevelopmental Presentations (2B.11)

Developmental Domains

Child development is usually tracked across five domains: **physical/motor**, **cognitive**, **language**, **social-emotional**, and **adaptive** (self-care). When a child is behind in one or more domains, the counselor's role is to notice, screen, and refer for a **developmental or educational evaluation**, not to diagnose by observation alone.

The Child in Context: Bronfenbrenner

Urie Bronfenbrenner's ecological systems theory (Bronfenbrenner, 1979) places the person at the center of nested systems. The **microsystem** is direct contact: family, classroom, peers. The **mesosystem** is the links between microsystems, such as a parent talking with a teacher. The **exosystem** shapes the person without including them, such as a parent's workplace or the school board. The **macrosystem** is culture, values, laws, and economics. The **chronosystem**, added later (Bronfenbrenner, 1986), is time: life transitions and historical change, such as a divorce or a pandemic. Use it when an item asks *where* in a child's world a problem or a support sits. Don't confuse it with Hays's **ADDRESSING** framework (Chapter 9). ADDRESSING organizes **cultural identity factors** (age, disability, religion, ethnicity, and the rest). Bronfenbrenner organizes **levels of environment**.

Neurodevelopmental Presentations

Presentation	Key features	Key details
Intellectual developmental disorder (intellectual disability)	Deficits in **intellectual functioning** (reasoning, problem-solving, learning) **and** in **adaptive functioning** (conceptual, social, practical), with onset in the developmental period	**Severity (mild, moderate, severe, profound) is based on adaptive functioning, not IQ score alone.** Intellectual functioning roughly two standard deviations below the mean (an IQ around 70, allowing for measurement error)
Global developmental delay	Delays in several areas in children under 5 who can't yet be reliably assessed	Reassess later
Autism spectrum disorder	Persistent deficits in **social communication and interaction**, plus **restricted, repetitive behaviors, interests, or activities** (including sensory differences); present from early development	Severity described by **level of support needed**: Level 1 (requiring support), Level 2 (substantial support), Level 3 (very substantial support)
ADHD	Persistent inattention and/or hyperactivity-impulsivity	**6 or more** symptoms (5 or more for age 17+) for at least **6 months**; several symptoms present **before age 12**; symptoms in **two or more settings** (home, school, work). Presentations: combined, predominantly inattentive, predominantly hyperactive-impulsive

Presentation	Key features	Key details
Specific learning disorder	Persistent difficulty in reading, written expression, or math well below age expectations, despite targeted help	At least **6 months** despite intervention
Tourette's disorder	**Multiple motor tics and at least one vocal tic**	Tics for more than **1 year**; onset **before age 18**
Communication disorders	Language, speech sound, childhood-onset fluency (stuttering), social (pragmatic) communication	Speech-language evaluation

Borderline intellectual functioning is not a disorder; it's a condition that may be a focus of clinical attention (coded R41.83).

Who Owns Child Development Assessment

Instrument	Use
Wechsler Intelligence Scale for Children (WISC)	Intellectual functioning, ages 6–16
Wechsler Preschool and Primary Scale of Intelligence (WPPSI)	Young children
Vineland Adaptive Behavior Scales	**Adaptive functioning** (required for intellectual disability diagnosis)
ADOS-2 (Autism Diagnostic Observation Schedule)	Autism assessment
M-CHAT	Autism screening for toddlers
Conners Rating Scales; **Vanderbilt**	ADHD rating scales (parent and teacher forms)
Ages and Stages Questionnaires (ASQ)	Developmental screening

School Supports

Law or plan	What it provides
IDEA (Individuals with Disabilities Education Act)	Special education for eligible students; an **Individualized Education Program (IEP)** with goals and services; ages 3–21; early intervention (Part C) for birth to 3
Section 504 (Rehabilitation Act of 1973)	A **504 plan** with accommodations for students with a disability that substantially limits a major life activity, even if they don't need special education
FERPA	Privacy of student education records; parents' rights to access them (these rights transfer to the student at 18 or in postsecondary school)

Counseling Approaches

- Use **concrete, developmentally matched** methods (play, art, activities; Chapter 5).
- Involve parents and teachers; **parent training** has strong support for ADHD and behavior problems.
- For autism, **applied behavior analysis (ABA)** has broad research support for skill-building; social skills groups help many children and adolescents.
- Address the whole family: siblings and parents of children with disabilities have their own stresses.

Part 2 — Separation from Primary Caregivers

Separation happens through divorce, military deployment, incarceration of a parent, foster care placement, immigration and family separation, hospitalization, and death.

Owners

Owner	Contribution
John Bowlby	Attachment theory; children's responses to separation: **protest → despair → detachment** (with James Robertson's observations of hospitalized children)
Mary Ainsworth	Strange Situation; secure, avoidant, and resistant/ ambivalent attachment
Main & Solomon	Disorganized attachment
Margaret Mahler	Separation–individuation; rapprochement
Pauline Boss	**Ambiguous loss**: a loss without clarity or closure, either physically absent but psychologically present (a missing or deployed parent) or physically present but psychologically absent (a parent with dementia or addiction)

Related Presentations

Presentation	Key features
Separation anxiety disorder	Excessive fear about separation from attachment figures; at least 4 weeks in children (Chapter 10)
Reactive attachment disorder	Emotionally withdrawn, rarely seeks or responds to comfort, after **severe neglect**; evident before age 5
Disinhibited social engagement disorder	Overly familiar with unfamiliar adults, little hesitation to go off with strangers, after severe neglect
Adjustment disorder	Distress after a stressor such as divorce or a move

What Helps

- **Predictability and honest, age-appropriate information** about the separation.

- **Maintaining connection** with the absent caregiver when safe (letters, calls, visits, video).
- **Stable routines** and a consistent caregiver.
- **Caregiver involvement**: interventions that strengthen the caregiver–child relationship (e.g., Parent–Child Interaction Therapy, Child–Parent Psychotherapy, Attachment and Biobehavioral Catch-up for children in foster care).
- **Children of deployed service members**: emotional cycle of deployment; school counselors and military family life resources.

Part 3 — Adolescence and Adulthood (2B. 11)

Adolescence

Concept	Owner
Identity vs. role confusion	Erikson
Identity statuses (diffusion, foreclosure, moratorium, achievement)	Marcia
Imaginary audience and personal fable	Elkind
Formal operational thinking	Piaget
Emerging adulthood (roughly 18–29): identity exploration, instability, self-focus, feeling in-between, possibilities	**Jeffrey Arnett**

Counseling adolescents: respect growing autonomy, clarify confidentiality and its limits with both teen and parents at the start, and expect some risk-taking as normal while screening for serious risk (substances, self-harm, unsafe relationships).

Adulthood

Concept	Owner
Intimacy vs. isolation; generativity vs. stagnation	Erikson
Life structure; transitions (e.g., age-30 transition, midlife transition); the Dream; mentor	Levinson
Developmental tasks of each life stage	Havighurst
Transitions: situation, self, support, strategies (the 4 S's)	Schlossberg
Sandwich generation: midlife adults caring for both children and aging parents	(general term; Dorothy Miller is often credited with coining it)

Midlife: "midlife crisis" is a popular idea, but research suggests most adults don't experience a dramatic crisis. Midlife is better understood as a period of reassessment.

Part 4 — Aging and Retirement

Theories of Aging

Theory	Owner	Core idea
Disengagement theory	Cumming & Henry (1961)	Older adults and society mutually withdraw; seen as natural (largely criticized now)
Activity theory	Havighurst and colleagues	Staying active and involved leads to greater satisfaction
Continuity theory	Robert Atchley	People adapt by keeping patterns, roles, and identity consistent with earlier life

Theory	Owner	Core idea
Successful aging	Rowe & Kahn (1997)	Low risk of disease and disability, high mental and physical functioning, and active engagement with life
Selective optimization with compensation (SOC)	Paul & Margret Baltes	Choose what matters most (selection), work to maintain it (optimization), and use strategies or aids to make up for losses (compensation)
Socioemotional selectivity theory	Laura Carstensen	As time horizons shorten, people focus on emotionally meaningful goals and relationships and prune peripheral ones
Integrity vs. despair	Erikson	Accepting one's life as it was
Gerotranscendence	Lars Tornstam	A shift toward a more cosmic, less materialistic perspective in late life
Ageism	Coined by **Robert Butler** (1969)	Stereotyping and discrimination based on age

Cognitive Changes: Delirium, Dementia, and Depression

Feature	Delirium	Major neurocognitive disorder (dementia)	Depression
Onset	Sudden (hours to days)	Gradual (months to years), though some types (e.g., vascular) can begin more abruptly	Weeks to months

Feature	Delirium	Major neurocognitive disorder (dementia)	Depression
Course	Fluctuates over the day	Usually progressive	Often worse in the morning; tied to mood
Attention	**Markedly impaired**	Usually intact early	May be poor
Cause	Medical: infection (e.g., urinary tract infection), medications, dehydration, surgery	Alzheimer's, vascular, Lewy body, frontotemporal, others	Mood disorder
Response	**Medical emergency**; usually reversible if treated	Support, planning, caregiver help	Treatable; cognition often improves with treatment

Mild vs. major neurocognitive disorder: both involve decline in **one or more** cognitive domains. In **major**, the decline interferes with **independence** in everyday activities; in **mild**, independence is preserved (perhaps with more effort or compensation).

Screening tools: Montreal Cognitive Assessment (MoCA), **Mini-Mental State Examination (MMSE)**, **Mini-Cog**, **Geriatric Depression Scale (GDS)**.

Exam rule: sudden confusion in an older adult is a **medical** problem until proven otherwise. Refer for immediate medical evaluation.

Other Late-Life Concerns

- **Elder abuse**: physical, sexual, emotional, financial exploitation, neglect, and self-neglect; report per state law (Chapter 6).
- **Suicide risk**: older men have among the highest suicide rates of any group; screen even when the client doesn't raise it.

- **Loneliness and isolation** (Chapter 16).
- **Caregiver strain** for spouses and adult children (Chapter 16).
- **Medication and alcohol interactions**.

Retirement

Phase (Atchley)	What happens
Pre-retirement	Planning and anticipating
Honeymoon	Enjoying freedom; travel and new activities
Disenchantment	Letdown when retirement doesn't match expectations
Reorientation	Developing a more realistic, satisfying routine
Stability	A settled, predictable lifestyle
Termination	Retirement role ends, often due to illness or disability

Not everyone goes through every phase, or in order.

Counseling retirement: identity beyond work (Super's disengagement stage, Chapters 3 and 14), structure and meaning, relationships (the couple now home together), finances, health, and **bridge employment** (part-time or new work after retiring). Involuntary retirement is harder than chosen retirement.

Part 5 — Grief and Loss

Owners

Owner	Contribution
Erich Lindemann	Acute grief reactions; also described **anticipatory grief**
Elisabeth Kübler-Ross	Five stages of **dying**: denial, anger, bargaining, depression, acceptance (from work with terminally ill patients; **not a required sequence**)

Owner	Contribution
John Bowlby	Phases of mourning: **numbness → yearning and searching → disorganization and despair → reorganization**
J. William Worden	Four **tasks** of mourning: accept the reality of the loss; process the pain of grief; adjust to a world without the deceased; find an enduring connection with the deceased while embarking on a new life
Therese Rando	Six "R" processes: **R**ecognize the loss, **R**eact to the separation, **R**ecollect and reexperience, **R**elinquish old attachments, **R**eadjust, **R**einvest
Stroebe & Schut	**Dual process model**: healthy grieving **oscillates** between **loss-oriented** coping (facing the grief) and **restoration-oriented** coping (attending to life changes, taking breaks from grief)
Klass, Silverman, & Nickman	**Continuing bonds**: maintaining a connection to the deceased can be healthy, not a failure to "let go"
Kenneth Doka	**Disenfranchised grief**: loss not socially recognized (a pet, an ex-partner, a miscarriage, a death by overdose, a loss in a stigmatized relationship)
Pauline Boss	**Ambiguous loss**
Robert Neimeyer	**Meaning reconstruction** after loss

Stages vs. tasks: Kübler-Ross (writing about dying patients) and Bowlby describe what people **go through**. Worden and Rando describe what grievers **do**. Worden's task language makes grief active, which is why many counselors prefer it.

Prolonged Grief Disorder (DSM-5-TR)

Feature	Requirement
Time since death	At least **12 months** for adults; at least **6 months** for children and adolescents
Core symptom	Intense **yearning** for, or **preoccupation** with, the deceased, nearly every day for at least the last month
Additional symptoms	At least **3 of 8**: identity disruption; marked disbelief about the death; avoidance of reminders; intense emotional pain; difficulty reintegrating into life; emotional numbness; feeling life is meaningless; intense loneliness
Exceeds norms	Duration and severity clearly exceed social, cultural, or religious norms

Grief is not a disorder. Most people grieve without needing a diagnosis. Prolonged grief disorder applies only when grief remains intense and impairing well beyond expected norms. Uncomplicated bereavement is a Z code, not a mental disorder.

Children and Grief

Children's understanding of death develops with cognitive growth (Piaget). Understanding of the core concepts (**irreversibility**: death is permanent; **universality**: all living things die; **nonfunctionality**: the body stops working; **causality**: why death happens) usually develops between about **ages 5 and 7**.

- Use **direct words**: "died," not "went to sleep" or "we lost him." Euphemisms confuse young children and can cause fear (of sleep, of getting lost).
- Answer questions honestly at the child's level.
- Children often grieve in **bursts**, playing one minute and sad the next. This is normal.

- Let children choose whether to take part in rituals such as funerals, after preparing them for what they'll see.
- Expect grief to be revisited at new developmental stages.

What Helps

- **Normalize** grief; avoid pathologizing.
- **Don't rush** or impose a sequence.
- Support both facing the loss and taking breaks from it (dual process).
- Honor **continuing bonds** and cultural rituals.
- **Screen** for depression, prolonged grief, substance use, and suicide risk, especially after sudden or violent deaths and death by suicide.
- For prolonged grief disorder, **complicated grief treatment** (Katherine Shear) has strong support.

Part 6 — Terminal Illness and End of Life

Care Models

Term	Meaning
Palliative care	Care focused on comfort and quality of life for anyone with serious illness, at any stage, alongside curative treatment
Hospice	Comfort care for people expected to live about **6 months or less** who are no longer seeking curative treatment; modern hospice movement founded by **Dame Cicely Saunders**
Advance directive	A legal document stating a person's wishes for medical care if they can't speak for themselves
Living will	A type of advance directive specifying treatments the person wants or refuses

Term	Meaning
Health care power of attorney (health care proxy)	Names a person to make medical decisions if the patient can't
Medical aid in dying	Legal in some U.S. states for terminally ill, mentally capable adults; laws vary

Counseling the Dying

- **Follow the client's lead.** Some want to talk about death; some want to talk about living.
- Address **anticipatory grief** for both the client and family.
- Support **life review** and legacy work (Butler; Chapter 5).
- Help with practical and relational unfinished business.
- Respect cultural and religious beliefs about death, dying, and rituals.
- Coordinate with the hospice or palliative team.
- **Kübler-Ross's stages** describe common reactions, not a required order.

Ethics: ACA B.2.b

Counselors who provide services to **terminally ill clients who are considering hastening their own deaths** have the **option to maintain confidentiality**, depending on applicable laws and the specific circumstances, **after seeking consultation or supervision** from appropriate professional and legal parties.

This is different from the usual duty to protect a suicidal client (Chapter 6). The exam pattern: consultation first, knowledge of state law, and attention to the client's capacity, depression, and coercion. A terminally ill client's wish to die still deserves careful assessment for treatable depression and unmanaged pain.

What the Exam Rewards / What It Punishes

Rewards: - Framing struggles as developmental tasks while still screening for disorders - Basing intellectual disability severity on adaptive functioning - Referring for developmental, educational, or medical evaluation - Treating sudden confusion in an older adult as a medical issue - Using direct language about death with children - Normalizing grief and honoring continuing bonds - Consultation in end-of-life confidentiality decisions

Punishes: - Treating Kübler-Ross's stages as a required sequence - Pathologizing normal grief - Using euphemisms for death with young children - Assuming older adults can't change, or that confusion is "just aging" - Diagnosing autism or ADHD from one observation in one setting

Swap-Pair Drill

If you see...	It's...	Not...
Severity based on adaptive functioning	Intellectual developmental disorder	ADHD
Severity rated by levels of support needed	Autism spectrum disorder	Intellectual disability
Symptoms before age 12, two or more settings	ADHD	Specific learning disorder
Motor and vocal tics, over 1 year, onset before 18	Tourette's disorder	Persistent tic disorder
Micro-, meso-, exo-, macro-, chronosystem	Bronfenbrenner's ecological systems theory	Hays's ADDRESSING
Special education with an IEP	IDEA	Section 504
Accommodations without special education	Section 504	IDEA

If you see…	It's…	Not…
Protest → despair → detachment	Bowlby (with Robertson)	Kübler-Ross
Physically present, psychologically absent	Ambiguous loss (Boss)	Disenfranchised grief (Doka)
Emerging adulthood	Arnett	Levinson
Mutual withdrawal of elders and society	Disengagement theory (Cumming & Henry)	Activity theory
Selection, optimization, compensation	Baltes & Baltes	Carstensen
Focus on meaningful relationships as time shortens	Socioemotional selectivity (Carstensen)	Continuity theory
Coined "ageism"	Robert Butler	Rowe & Kahn
Sudden, fluctuating confusion with poor attention	Delirium	Dementia
Honeymoon, disenchantment, reorientation	Atchley's retirement phases	Kübler-Ross
Oscillating between loss and restoration	Dual process model (Stroebe & Schut)	Worden's tasks
Ongoing connection to the deceased is healthy	Continuing bonds (Klass et al.)	Disenfranchised grief
Six R processes	Rando	Worden
Modern hospice movement	Cicely Saunders	Kübler-Ross
Prolonged grief disorder in adults	12 months	6 months (children and adolescents)

Quick Checks

1. A child's IQ is measured at 68, but she manages self-care, friendships, and schoolwork with typical independence. The best conclusion is: A. She has severe intellectual disability B. She meets criteria for intellectual disability

based on IQ alone C. Intellectual disability requires deficits in adaptive functioning as well, which aren't present here D. She has autism spectrum disorder

2. A student with ADHD needs extended test time but doesn't need special education services. The most appropriate support is: A. An IEP under IDEA B. A Section 504 plan C. FERPA protection D. Early intervention under Part C

3. A 4-year-old whose father has deployed first cries and clings, then seems withdrawn and sad, then appears indifferent when the father calls. This sequence matches: A. Kübler-Ross's stages B. Bowlby's protest, despair, and detachment C. Marcia's identity statuses D. Worden's tasks

4. A woman's husband has advanced Alzheimer's disease. She says, "He's here, but he's gone." This is best described as: A. Disenfranchised grief B. Ambiguous loss C. Anticipatory grief only D. Prolonged grief disorder

5. An 82-year-old client becomes suddenly confused over 2 days, with attention that comes and goes. The counselor should first: A. Begin reminiscence therapy B. Screen with the MoCA and diagnose dementia C. Arrange an immediate medical evaluation for possible delirium D. Treat for depression

6. Which theory says older adults narrow their social circles to focus on emotionally meaningful relationships as their time horizons shorten? A. Disengagement theory B. Socioemotional selectivity theory C. Activity theory D. Continuity theory

7. A recently retired client says retirement has been "a big letdown" after an exciting first few months of travel. According to Atchley, he's most likely in which phase? A. Honeymoon B. Disenchantment C. Stability D. Termination

8. A widow keeps her husband's photos up, talks to him at his grave, and says it comforts her, while she's also reengaging with friends. The best interpretation is: A. Pro-

longed grief disorder; she needs to let go B. Healthy continuing bonds C. Denial, the first stage of grief D. Disenfranchised grief

9. Seven months after his mother's death, an adult client has intense yearning, loneliness, and difficulty reengaging in life. Can prolonged grief disorder be diagnosed? A. Yes, because symptoms are present for more than 6 months B. No, because adults require at least 12 months since the death C. Yes, because grief is always a disorder after 6 months D. No, because prolonged grief disorder doesn't exist in the DSM-5-TR

10. A 5-year-old asks where her grandfather is. The most helpful response is: A. "Grandpa went to sleep and won't wake up." B. "We lost Grandpa." C. "Grandpa died. His body stopped working, and he can't come back. It's OK to feel sad." D. "Grandpa went on a long trip."

11. Which model of grief describes healthy coping as moving back and forth between facing the loss and attending to life changes? A. Kübler-Ross's stages B. Worden's tasks C. Stroebe and Schut's dual process model D. Rando's six Rs

12. A terminally ill client in a state where medical aid in dying is legal tells her counselor she plans to pursue it. Under ACA B.2.b, the counselor should: A. Immediately break confidentiality to prevent a suicide B. Consult with appropriate professional and legal parties, then decide whether to maintain confidentiality, considering applicable law and circumstances C. Refuse to discuss the topic D. Encourage the client to proceed

Answers and Rationales

1 — C. The diagnosis requires deficits in both intellectual and adaptive functioning, and severity is based on adaptive functioning. A: severity isn't set by IQ alone, and her adaptive functioning is typical. B: IQ alone isn't sufficient. D: nothing suggests social communication deficits or restricted behaviors.

2 — B. Section 504 provides accommodations without special education. A: an IEP is for students needing special education. C: FERPA protects records. D: Part C serves infants and toddlers.

3 — B. Bowlby (with Robertson) described protest, despair, and detachment in separated young children. A: Kübler-Ross described reactions to dying. C: Marcia's statuses are adolescent identity. D: Worden's tasks are for mourning a death.

4 — B. Physically present but psychologically absent is ambiguous loss (Boss). A: disenfranchised grief is unrecognized loss. C: anticipatory grief may also be present, but "here but gone" is the hallmark of ambiguous loss. D: he hasn't died.

5 — C. Sudden onset with fluctuating attention suggests delirium, a medical emergency. A: delays medical care. B: dementia typically develops gradually. D: misses a likely medical cause.

6 — B. Carstensen's socioemotional selectivity theory. A: disengagement theory describes mutual withdrawal from society. C: activity theory says staying active brings satisfaction. D: continuity theory emphasizes keeping consistent patterns.

7 — B. A letdown after the honeymoon phase is disenchantment. A: honeymoon is the early excitement. C: stability is a settled routine. D: termination is the end of the retirement role.

8 — B. Maintaining a comforting connection while reengaging with life is healthy continuing bonds (Klass et al.). A: she's reengaging and functioning, and the time frame isn't given. C: Kübler-Ross's stages described dying patients and aren't a required sequence. D: her loss is socially recognized.

9 — B. Adults require at least 12 months since the death. A and C: 6 months applies to children and adolescents. D: prolonged grief disorder was added in the DSM-5-TR.

10 — C. Direct, honest, age-appropriate language that names death and permits feelings. A: can create fear of sleep. B and D: euphemisms suggest the person can be found or will return.

11 — C. Oscillation between loss and restoration is the dual process model. A: Kübler-Ross describes stages of dying. B: Worden describes tasks. D: Rando describes six processes.

12 — B. B.2.b allows counselors to maintain confidentiality for terminally ill clients considering hastening death, depending on law and circumstances, after consultation. A: ignores B.2.b and the legal context. C: abandons the client. D: imposes the counselor's view rather than supporting the client's own decision-making.

Audio Companion

Track	Covers	Target length
12.1	Across the Life Span: neurodevelopment, separation, aging, grief, end of life	~6 min

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Chapter 13 — Relationships and Family Life

2027 task map: 4.1 Engage in couples/family dynamic counseling · 6.2 Adhere to internal policies or standard operating procedures (SOPs) relevant to your clinical practice (e.g., informed consent, record retention, release of information, mandated reporting protocols) · 4.4 Facilitate clients' ability to navigate interpersonal relationships · 2B.2 Assess risk to self and others · CACREP: Human Growth and Development; Counseling and Helping Relationships · 2027 supplemental content guidance: presenting concerns

Where This Chapter Fits

Chapter 7 taught **how** to work with couples and families: systems concepts, assessment tools, joining, and systemic interventions. Chapter 6 covered the **safety** side: mandated reporting and intimate partner violence safety planning. This chapter covers **what** couples and families bring to counseling: love and dating, communication, divorce, co-parenting, remarriage and stepfamilies, adoption, parenting, and abuse.

As in earlier chapters, the NCE tends to test three things here: the owners, the research findings that overturn common assumptions, and the counselor's ethical role.

Part 1 — Love, Dating, and Relationship Problems (4.4, 4.1)

Who Owns Love

Owner	Theory	Key terms
Robert Sternberg	**Triangular theory of love**	Three components: **intimacy**, **passion**, **commitment**. All three together = **consummate love**. Intimacy + passion = romantic love; intimacy + commitment = companionate love; passion + commitment = fatuous love; commitment alone = empty love; passion alone = infatuation; intimacy alone = liking
John Alan Lee	**Love styles**	Eros (passionate), ludus (game-playing), storge (friendship), pragma (practical), mania (possessive), agape (selfless)
Hazan & Shaver	**Adult attachment**	Romantic love as an attachment process; secure, anxious, and avoidant adult attachment styles
Caryl Rusbult	**Investment model**	Commitment depends on **satisfaction**, the **quality of alternatives**, and the size of **investments**

Owner	Theory	Key terms
John Gottman	Research on couple interaction	Four Horsemen; 5:1 positive-to-negative ratio in stable couples; repair attempts; most couple conflicts are **perpetual** (about lasting differences) rather than solvable
Gary Chapman	Five love languages (popular, not research-based)	Words of affirmation, quality time, gifts, acts of service, physical touch

Exam note on Rusbult: people stay in unsatisfying (even harmful) relationships when alternatives seem poor and investments are high. This helps explain why "Why doesn't she just leave?" is the wrong question in IPV (Chapter 6).

Communication Problems (4.1)

Chapter 7 covered Gottman, Satir's stances, EFT's pursue–withdraw cycle, and PREP's speaker–listener technique. The core counseling moves:

- Slow the conversation down and **have partners talk to each other** in session.
- Teach **"I" statements** and **active listening** (reflect before responding).
- Replace criticism with a **softened startup** (Gottman).
- Help partners recognize **flooding** and take breaks (at least 20 minutes) before returning to the conversation.
- Distinguish **solvable** problems from **perpetual** ones; for perpetual problems, the goal is dialogue and acceptance, not a final solution (IBCT, Chapter 7).

Infidelity and Recommitment

Recovery from an affair is commonly described in three phases: **(1) dealing with the impact** (crisis, disclosure, safety, stabilizing), **(2) understanding the context** (how the relationship and each partner contributed, without ex-

cusing the affair), and **(3) moving forward** (forgiveness, recommitment, or separation) (Baucom, Snyder, & Gordon).

The counselor stays neutral, attends to both partners' pain, and follows the secrets policy set at intake (Chapter 7).

Part 2 — Divorce

Bohannan's Six Stations of Divorce

Paul Bohannan described six overlapping experiences ("stations") of divorce:

Station	What it involves
Emotional	The relationship deteriorates
Legal	The court process
Economic	Dividing property and finances
Coparental	Custody, visitation, and co-parenting arrangements
Community	Changes in friends and social networks
Psychic	Regaining individual identity and autonomy

Children and Divorce: What the Research Shows

Researcher	Finding
E. Mavis Hetherington	**Most children of divorce adjust well** within about 2 years; roughly 75–80% show no lasting serious problems. Children do best with competent parenting and low conflict
Judith Wallerstein	A more pessimistic view from a smaller clinical sample; described a **"sleeper effect"** in which some effects appear in adulthood, especially around intimate relationships

Researcher	Finding
Paul Amato	**Parental conflict** is a stronger predictor of children's problems than divorce itself; children in high-conflict marriages may do better after divorce, while children whose low-conflict parents divorce may struggle more

The exam principle: conflict, not divorce itself, is the bigger risk to children. Interventions that reduce children's exposure to conflict (not using children as messengers, not criticizing the other parent in front of them, not making them choose) are the priority.

Children's Reactions by Age

Age	Common reactions
Preschool	Regression, fear of abandonment, self-blame ("Daddy left because I was bad"), confusion
School age	Sadness, loyalty conflicts, reconciliation fantasies, anger, school problems
Adolescents	Anger, worry about their own future relationships, taking on adult roles, distancing, acting out

What helps children: tell them together if possible, in age-appropriate words; reassure them the divorce isn't their fault and both parents still love them; keep routines; let them love both parents; protect them from conflict and adult details.

Co-Parenting Styles

Style	Description
Cooperative	Parents communicate, coordinate, and support each other's parenting

Style	Description
Parallel	Parents disengage from each other, each parenting independently with minimal contact; often recommended for **high-conflict** families to reduce children's exposure to conflict
Conflicted	Ongoing hostility; children are caught in the middle

Parenting coordinators and **co-parenting apps** can help high-conflict parents communicate.

Ethics in Divorce-Related Work

Issue	Rule
Consent to treat a minor	Check **legal custody** and any court orders. In general, a parent with legal custody can consent, but orders vary. Get a copy of the custody documents
The other parent's rights	A noncustodial parent may still have rights to records, depending on state law and the order
Role conflict	Counselors **don't serve as forensic evaluators** (custody evaluators) for **their own current or former clients** (ACA E.13.c). A child's or parent's counselor who is asked to recommend custody should explain the role conflict
Court testimony	A treating counselor may be called as a fact witness; that role is different from being a custody evaluator
Neutrality	Avoid being pulled into one parent's side (triangulation)

Part 3 — Remarriage and Stepfamilies (4.1)

Owners

Owner	Contribution
Emily & John Visher	Early stepfamily research and support; identified stepfamily **myths**: **instant love** (stepparent and stepchildren will love each other quickly), the **wicked stepmother**, and that a stepfamily is just like a first-marriage family
Patricia Papernow	**Stepfamily Cycle**, seven stages in three phases: **Early** (fantasy, immersion, awareness), **Middle** (mobilization, action), **Later** (contact, resolution). Integration often takes **several years** (commonly cited as about 4–7 or more)
McGoldrick & Carter	Remarriage stages added to the family life cycle (Chapter 7)

Stepfamily Challenges

- **Insider/outsider positions**: the biological parent and children share history; the stepparent starts as an outsider.
- **Loyalty binds**: children may feel that liking a stepparent betrays the other biological parent.
- **Discipline**: early on, the **biological parent** should handle discipline while the **stepparent builds a relationship** (more like a mentor, coach, or friendly adult). Stepparents who take on discipline too early often face resentment.
- **Different family cultures** merging: routines, rules, holidays, money.
- **Grief**: children (and adults) may still be grieving the previous family.
- **Ex-partners** remain part of the system.

Exam principle: slow integration does not mean a step-family is "failing." Normalize the time it takes, and correct unrealistic expectations (the instant-love myth).

Part 4 — Adoption

Key Terms

Term	Meaning
Adoption triad (or **adoption constellation**)	The **birth parents**, the **adoptive parents**, and the **adoptee**. "Constellation" includes extended family and others affected
Closed adoption	No contact or identifying information shared
Open adoption	Some ongoing contact or information sharing between birth and adoptive families
Transracial adoption	Adoptive parents and child are of different races
International (intercountry) adoption	Child adopted from another country
Kinship adoption	Adoption by relatives

Seven Core Issues in Adoption (Silverstein & Kaplan)

Deborah Silverstein and Sharon Kaplan identified issues that can affect **all members** of the triad across the life span: **loss, rejection, guilt and shame, grief, identity, intimacy, and mastery/control.**

Counseling Principles

- **Tell children about their adoption early** and in age-appropriate ways, as an ongoing conversation rather than a single disclosure. Secrecy tends to harm trust when the truth comes out later.
- **Adolescence** often brings identity questions and interest in birth family; this is normal, not a sign of rejecting the adoptive family.

- **Transracial and international adoptees** benefit from connection to their racial, ethnic, and cultural heritage and from adoptive parents who prepare them to face racism.
- **Children adopted after early adversity** (neglect, institutional care, foster care) may have attachment, trauma, or developmental needs (Chapter 12).
- Don't assume every problem an adopted person has is about adoption, or that adoption is never relevant. Explore its meaning with the client.
- **Birth parents** experience grief and loss, sometimes disenfranchised (Chapter 12).

Part 5 — Parenting and Co-Parenting (4.1)

Who Owns Parenting

Owner	Contribution
Diana Baumrind	Parenting styles: **authoritative** (warm and firm; best outcomes), **authoritarian** (firm, not warm), **permissive** (warm, not firm); Maccoby & Martin added **neglectful/uninvolved**
Rudolf Dreikurs	Adlerian parenting; four **goals of misbehavior**: **attention, power, revenge, and display of inadequacy**; **natural and logical consequences**; family meetings; encouragement
Dinkmeyer & McKay	**STEP** (Systematic Training for Effective Parenting), based on Adlerian principles
Thomas Gordon	**PET** (Parent Effectiveness Training): active listening, I-messages, "who owns the problem," no-lose conflict resolution
Haim Ginott	Communicating with children: acknowledge feelings, set limits on behavior

Owner	Contribution
Gerald Patterson	**Coercion theory**: parents and children reinforce each other's escalating behavior (the child escalates until the parent gives in, which rewards the child's escalation; the parent's giving in is itself rewarded when the child's behavior stops); basis for **parent management training**
Carolyn Webster-Stratton	**Incredible Years** parenting program
Matthew Sanders	**Triple P** (Positive Parenting Program)
Sheila Eyberg	**PCIT** (Chapter 5)
Jane Nelsen	**Positive Discipline** (Adlerian)

Natural vs. Logical Consequences (Dreikurs)

Type	Meaning	Example
Natural consequence	Happens on its own, without adult action	A child who refuses a coat feels cold
Logical consequence	Set by the adult and **related** to the behavior	A child who rides a bike without a helmet loses bike time

A logical consequence should be **related, respectful, and reasonable**. A punishment unrelated to the behavior is not a logical consequence.

Dreikurs's Goals of Misbehavior

Goal	Child's mistaken belief	Adult's typical feeling
Attention	"I count only when I'm noticed"	Annoyed
Power	"I count only when I'm in control"	Angry, challenged
Revenge	"I count only when I hurt others as I feel hurt"	Hurt

Goal	Child's mistaken belief	Adult's typical feeling
Display of inadequacy	"I can't do anything right, so leave me alone"	Hopeless, helpless

The adult's own feeling is a clue to the child's goal.

Co-Parenting Conflict

Whether parents are together or apart, children do better when parents present a reasonably united front, don't undermine each other, and keep children out of adult conflict. In systems terms (Chapter 7), a strong **parental subsystem** with clear boundaries is the target (Minuchin).

Part 6 — Abuse and Violence in Families (6.2, 2B.2)

Chapter 6 covered mandated reporting, how to respond to a child's disclosure, IPV lethality assessment, and safety planning. This section adds concepts the NCE may test about the effects and patterns of family violence.

Effects of Child Sexual Abuse: Traumagenic Dynamics (Finkelhor & Browne)

David Finkelhor and Angela Browne described four **traumagenic dynamics** that explain much of the harm of child sexual abuse:

Dynamic	Effect
Traumatic sexualization	Sexuality is shaped in developmentally inappropriate and harmful ways
Betrayal	Trust is broken by someone the child depended on, or by caregivers who didn't protect
Powerlessness	The child's will and sense of control are repeatedly overridden

Dynamic	Effect
Stigmatization	Shame, guilt, and a sense of being "damaged" become part of the child's self-image

Types of Intimate Partner Violence (Michael P. Johnson)

Type	Description
Intimate terrorism (coercive controlling violence)	Violence used as part of a pattern of **coercive control**; usually more frequent, severe, and escalating
Situational couple violence	Violence arising from specific conflicts that escalate, **without** a broader pattern of control; the most common type in community samples
Violent resistance	Violence used in response to a partner's intimate terrorism, often in self-defense
Mutual violent control	Both partners are controlling and violent (rare)

Why it matters: the type of violence affects safety and treatment decisions. Conjoint counseling is contraindicated while there is ongoing violence or coercive control (Chapter 6). Distinguishing types requires careful, **separate** assessment, never in front of the partner.

Children Exposed to IPV

Witnessing violence between caregivers is itself harmful to children and is associated with anxiety, depression, trauma symptoms, behavior problems, and later relationship difficulties. Some states treat exposure to domestic violence as a form of child maltreatment; know your state's reporting rules (Chapter 6).

Other Family Abuse

- **Sibling abuse** is often overlooked and dismissed as "normal fighting." Look for a **power imbalance**, repeated victimization of the same child, and fear.

- **Elder abuse** (Chapter 6, Chapter 12).
- **Emotional abuse**: persistent belittling, threats, rejection, isolation. It does lasting harm even without physical injury.
- **Teen dating violence**: screen adolescents; healthy relationship education is a prevention strategy.

What the Exam Rewards / What It Punishes

Rewards: - Knowing conflict, not divorce itself, is the bigger risk to children - Keeping children out of adult conflict; parallel parenting in high conflict - Checking custody documents before treating a minor of divorced parents - Declining to act as custody evaluator for one's own client - Normalizing slow stepfamily integration; biological parent disciplines early - Early, ongoing, age-appropriate adoption disclosure - Related, respectful, reasonable logical consequences - Assessing IPV type separately before any conjoint work

Punishes: - Assuming children of divorce will be damaged - Expecting "instant love" in stepfamilies - Stepparent taking over discipline early - Keeping adoption secret - Recommending custody for one's own client - Treating all IPV as the same - Dismissing sibling abuse as normal rivalry

Swap-Pair Drill

If you see…	It belongs to…	Not…
Intimacy, passion, commitment	Sternberg	Lee
Eros, ludus, storge, pragma, mania, agape	Lee	Sternberg
Satisfaction, alternatives, investments	Rusbult	Gottman

If you see…	It belongs to…	Not…
Adult romantic attachment styles	Hazan & Shaver	Ainsworth (infants)
Six stations of divorce	Bohannan	Kübler-Ross
Most children of divorce adjust well	Hetherington	Wallerstein
Sleeper effect	Wallerstein	Hetherington
Conflict matters more than divorce	Amato	Wallerstein
Fantasy, immersion, awareness…	Papernow	Visher & Visher
Instant love myth; wicked stepmother	Visher & Visher	Papernow
Loss, rejection, guilt, grief, identity, intimacy, mastery	Silverstein & Kaplan	Worden
Attention, power, revenge, inadequacy	Dreikurs	Baumrind
STEP	Dinkmeyer & McKay	Gordon (PET)
Who owns the problem; I-messages	Gordon (PET)	Dreikurs
Coercion theory	Patterson	Baumrind
Incredible Years	Webster-Stratton	Sanders (Triple P)
Traumagenic dynamics	Finkelhor & Browne	Walker
Intimate terrorism vs. situational couple violence	Michael P. Johnson	Walker’s cycle
Counselor can’t evaluate own client for court	ACA E.13.c	ACA A.8

Quick Checks

1. According to Sternberg, a relationship with intimacy and commitment but little passion is: A. Consummate love B. Companionate love C. Fatuous love D. Infatuation

2. A client stays in an unhappy relationship, saying, “We have a house, kids, and fifteen years together, and I don’t think I’d find anyone better.” This is best explained by: A. Sternberg’s triangular theory B. Rusbult’s investment model C. Lee’s love styles D. Bohannan’s stations

3. A parent asks whether divorce will “ruin” her children. Research most strongly supports telling her that: A. Most children of divorce have serious lasting problems B. Children’s adjustment depends heavily on how much conflict they’re exposed to and the quality of parenting C. Staying together is always better for children D. Children are unaffected by divorce

4. Two divorced parents can’t speak without arguing in front of their children. Which co-parenting arrangement is most likely to help the children? A. Cooperative co-parenting with joint family outings B. Parallel parenting with minimal direct contact between parents C. Having the children carry messages between homes D. Requiring weekly joint parent meetings with the children present

5. A divorced father brings his 10-year-old for counseling and says the mother doesn’t need to know. The counselor’s first step is to: A. Begin counseling, since the father brought the child B. Review the custody order to determine who has legal authority to consent C. Contact the mother to get her permission before treating the child D. Refuse to see the child

6. The father’s attorney later asks the child’s counselor to recommend which parent should have primary custody. The counselor should: A. Provide a recommendation, since the counselor knows the child best B. Explain that serving as a custody evaluator for one’s own client is a conflict of roles C. Recommend the parent who pays for counseling D. Ask the child which parent to recommend

7. In a new stepfamily, the stepfather starts enforcing strict rules on his stepchildren, who become angry and resentful. The best guidance is: A. Support the stepfather’s authority so the children learn respect B. Have the

biological parent take the lead on discipline while the stepfather builds a relationship with the children C. Have the children spend less time at home D. Recommend the couple separate

8. A newly remarried couple worries that, after a year, their blended family “still doesn’t feel like a family.” The counselor should: A. Assess for a family disorder B. Normalize that stepfamily integration often takes several years and explore expectations C. Recommend individual counseling for each child D. Suggest the couple was too hasty in marrying

9. Adoptive parents ask when to tell their 3-year-old she’s adopted. The best guidance is: A. Wait until she’s 18 B. Wait until she asks C. Begin now with simple, age-appropriate language, and keep it an ongoing conversation D. Never tell her, to protect her

10. A child who refuses to do homework, argues constantly, and leaves the parent feeling angry and challenged is most likely pursuing which goal of misbehavior (Dreikurs)? A. Attention B. Power C. Revenge D. Display of inadequacy

11. A teen who stays out past curfew loses use of the family car the next weekend. This is an example of: A. A natural consequence B. A logical consequence C. Negative reinforcement D. Extinction

12. A wife reports that her husband occasionally pushes her during heated arguments but doesn’t otherwise control her money, contacts, or movements. According to Johnson’s typology, this pattern is most consistent with: A. Intimate terrorism B. Situational couple violence C. Violent resistance D. Mutual violent control

Answers and Rationales

1 — B. Intimacy plus commitment is companionate love. A: consummate love has all three components. C: fatuous love is passion plus commitment. D: infatuation is passion alone.

2 — B. Investments and poor perceived alternatives keep people committed (Rusbult). A: Sternberg describes love's components. C: Lee describes love styles. D: Bohannan describes divorce.

3 — B. Research (Hetherington, Amato) emphasizes conflict exposure and parenting quality. A: most children adjust well. C: high-conflict marriages can be worse for children than divorce. D: overstates; children are affected, but most adjust.

4 — B. Parallel parenting reduces children's exposure to conflict. A and D: put the children in the conflict. C: using children as messengers is harmful.

5 — B. Legal authority to consent depends on the custody order. A: may proceed without proper consent. C: misstates the issue. D: premature; the child may be eligible for services with proper consent.

6 — B. Treating counselors don't serve as forensic evaluators for their own clients (ACA E.13.c). A: role conflict. C: payment doesn't determine recommendations. D: inappropriate and burdens the child.

7 — B. Early on, the biological parent handles discipline while the stepparent builds a relationship (Visher & Visher; Papernow). A: escalates resentment. C and D: unwarranted.

8 — B. Integration often takes several years; the "instant family" expectation is a myth. A: pathologizes a normal process. C: misses the family-level issue. D: unsupportive and unsupported.

9 — C. Early, age-appropriate, ongoing disclosure supports trust. A, B, D: secrecy and delay tend to harm trust and identity.

10 — B. Power struggles leave adults feeling angry and challenged. A: attention-seeking leaves adults annoyed. C: revenge leaves adults hurt. D: inadequacy leaves adults feeling hopeless.

11 — B. An adult-imposed consequence related to the behavior is logical. A: a natural consequence would happen without adult action. C: negative reinforcement increases behavior by removing something unpleasant. D: extinction withholds reinforcement.

12 — B. Violence arising in specific conflicts without a broader pattern of control fits situational couple violence. A: intimate terrorism involves coercive control. C: violent resistance responds to a partner's intimate terrorism. D: mutual violent control requires both partners to be controlling and violent. **Assessment must still be done separately and carefully; situational violence is still violence and requires safety assessment.**

Audio Companion

Track	Covers	Target length
13.1	Families in Transition: love, divorce, stepfamilies, adoption, parenting, violence	~6 min

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Chapter 14 — Career and Occupation

2027 task map: 3A.5 Integrate assessment results into treatment plans · 2B.11 Select, administer, and/or interpret appropriate instruments/assessments that align with contextual, cultural, and developmental considerations · 4.11 Provide psychoeducation · CACREP: Career Development · 2027 supplemental content guidance: presenting concerns

Career Is Counseling

The NCE treats career development as one of the eight CACREP core areas, and career concerns show up across the domains: as a presenting concern in the 2027 supplemental content guidance, in assessment (2B.11, 3A.5), and in life-span development. Chapter 3 gave you the career theorists. This chapter covers the work itself: the career counseling process, the instruments and information sources, work-related distress, and career concerns for specific groups.

One principle anchors this chapter: **career and personal concerns are intertwined.** Job loss can trigger depression; anxiety can freeze decision-making; family expectations shape choices. The NCE rewards counselors who address the whole person rather than treating career counseling as test-and-tell.

Part 1 — Theory Review in One Table

Owner	Theory	Use it when the stem mentions…
Parsons	Trait-factor	Know yourself, know the world of work, match them (“true reasoning”)
Williamson	Trait-factor (Minnesota)	Directive, six-step counseling: analysis, synthesis, diagnosis, prognosis, counseling, follow-up
Holland	Personality types and environments	RIASEC, hexagon, congruence, Self-Directed Search
Super	Life-span, life-space	Self-concept, life roles, Life-Career Rainbow, stages, career maturity
Ginzberg et al.	Developmental	Fantasy, tentative, realistic periods
Roe	Needs/personality	Early parent–child relationships; 8 occupational groups and 6 levels
Gottfredson	Circumscription and compromise	Children rule out options by gender and prestige
Krumboltz	Social learning; happenstance	Learning experiences, task-approach skills, planned happenstance
Lent, Brown, & Hackett	SCCT	Self-efficacy, outcome expectations, goals; barriers and supports
Savickas	Career construction	Career adaptability (concern, control, curiosity, confidence); life stories

Owner	Theory	Use it when the stem mentions…
Dawis & Lofquist	Theory of Work Adjustment	Satisfaction (worker) and satisfactoriness (employer)
Peterson, Sampson, & Reardon	Cognitive Information Processing	Pyramid; CASVE cycle; negative career thoughts
Tiedeman & O'Hara	Decision-making	Anticipation and implementation phases
Hoppock	Needs-based satisfaction	Job satisfaction depends on how well needs are met

Holland's Hexagon in Practice

Realistic — **I**nvestigative — **A**rtistic — **S**ocial — **E**nterprising — **C**onventional, arranged in a hexagon in that order.

Concept	Meaning	Example
Congruence	Fit between a person's type and the work environment's type	A Social person working as a teacher
Consistency	How close a person's top types are on the hexagon; **adjacent** types (e.g., RI, SE) are consistent; **opposite** types (e.g., RS, AC, IE) are inconsistent	Code "SEA" is fairly consistent
Differentiation	How clearly one type stands out from the others; a flat profile is **undifferentiated**	A profile with one high score and five low scores is well differentiated
Identity	Clarity and stability of one's goals, interests, and talents	Measured by *My Vocational Situation*

Opposite pairs on the hexagon: R–S, I–E, A–C.

Super's Stages and Tasks

Stage	Ages (approx.)	Key tasks
Growth	0–14	Developing self-concept, attitudes, and needs; learning about work
Exploration	15–24	**Crystallizing** (forming a preference), **specifying** (narrowing to a choice), **implementing** (acting on it)
Establishment	25–44	**Stabilizing**, **consolidating**, and **advancing**
Maintenance	45–64	**Holding**, **updating**, and **innovating**
Disengagement	65+	**Decelerating**, **retirement planning**, **retirement living**

Recycling: people can reenter earlier stages (for example, reexploring after a layoff in midlife). For adults, Super later proposed **career adaptability** in place of **career maturity**; Savickas built career construction theory around adaptability.

Life-Career Rainbow: life **roles** (child, student, leisurite, citizen, worker, spouse/partner, homemaker, parent, pensioner) played in **theaters** (home, community, school, workplace) across the life span.

Part 2 — The Career Counseling Process

Most models share these steps:

1. **Establish the relationship** and clarify the client's goals and expectations. Many clients expect "a test that tells me what to do."

2. **Assess** interests, values, skills, personality, and barriers, using both formal instruments and conversation.
3. **Explore** occupational information and options.
4. **Decide**, weighing options against values and constraints.
5. **Act**: plan, implement (training, job search), and follow up.

The exam's career rules: - **Assessments inform decisions; they don't make them.** The counselor interprets results **with** the client and connects them to the client's goals (3A.5). - **Interest isn't ability.** A high Artistic score says the client likes artistic activities, not that the client is talented at them. - Explore **barriers** (financial, family, discrimination, disability, low self-efficacy) and **supports** (SCCT). - Address **mental health** issues that affect decisions: anxiety, depression, perfectionism.

Indecision vs. Indecisiveness

Term	Meaning	Approach
Career indecision	A developmental, often temporary lack of a career decision, usually due to limited information or experience	Information, exploration, experience
Indecisiveness	A **chronic**, generalized difficulty making decisions across life areas, often linked to anxiety	Address the underlying anxiety and decision-making patterns

Decision-Making Styles (Harren)

Style	Description
Rational	Systematic, logical, considers future consequences
Intuitive	Relies on feelings and fantasy; accepts responsibility

Style	Description
Dependent	Relies on others; denies personal responsibility

Part 3 — Career Assessment Instruments

Instrument	Measures	Owner / notes
Strong Interest Inventory (SII)	Interests; General Occupational Themes (Holland's RIASEC), Basic Interest Scales, Occupational Scales, Personal Style Scales	E. K. Strong; compares the client's interests with those of people satisfied in their occupations
Self-Directed Search (SDS)	RIASEC code; **self-administered, self-scored**	Holland
Kuder (Career Search, Occupational Interest Survey)	Interests	Frederic Kuder
O*NET Interest Profiler	RIASEC interests; free	U.S. Department of Labor
Myers-Briggs Type Indicator (MBTI)	Personality type (E/I, S/N, T/F, J/P)	Myers & Briggs, based on Jung; used for self-understanding, not for selecting a career
Career Maturity Inventory (CMI)	Career readiness attitudes and competencies	John Crites
Career Development Inventory (CDI)	Career maturity	Super and colleagues
Career Decision Scale	Career indecision	Samuel Osipow
My Vocational Situation	Vocational identity, need for information, barriers	Holland
Career Beliefs Inventory	Beliefs that block career progress	Krumboltz
Career Thoughts Inventory	Negative career thoughts	Sampson et al. (CIP)

Instrument	Measures	Owner / notes
Minnesota Importance Questionnaire (MIQ)	Work **values** and needs	Dawis & Lofquist (TWA)
ASVAB	Aptitudes; used for military enlistment and career exploration	Department of Defense
GATB	General aptitudes	U.S. Employment Service
Card sorts	Interests or values, qualitatively	Tyler (Tyler Vocational Card Sort)

Interest vs. aptitude vs. achievement vs. values: - **Interest**: what you like - **Aptitude**: what you could learn to do (potential) - **Achievement**: what you've already learned - **Values**: what matters to you in work

Occupational Information Sources

Source	What it is
O*NET (Occupational Information Network)	U.S. Department of Labor database of occupations; **replaced the Dictionary of Occupational Titles (DOT)**
Occupational Outlook Handbook (OOH)	Bureau of Labor Statistics guide to job duties, education, pay, and outlook; updated regularly
Standard Occupational Classification (SOC)	Federal system for classifying occupations
Informational interviewing, job shadowing, internships	Firsthand information

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Part 4 — Work, Unemployment, and Distress

Job Loss (Jahoda's Latent Deprivation Model)

Marie Jahoda argued that work provides **manifest** benefits (income) and **latent** benefits: **time structure, social contact, collective purpose, status and identity, and**

regular activity. Unemployment harms well-being partly because people lose these latent benefits, not just income.

Counseling after job loss: acknowledge the grief and identity loss; screen for depression, substance use, and suicide risk; rebuild structure and activity; address practical needs (finances, benefits, job search); and reframe where possible (Krumboltz's planned happenstance).

Burnout (Christina Maslach)

Three dimensions, measured by the **Maslach Burnout Inventory (MBI)**: 1. **Emotional exhaustion** 2. **Depersonalization** (cynicism, detachment from clients or work) 3. **Reduced personal accomplishment** (inefficacy)

Burnout vs. compassion fatigue vs. vicarious trauma:

Term	Meaning
Burnout	Exhaustion and cynicism from chronic workplace stress
Compassion fatigue (Charles Figley)	Emotional and physical exhaustion from caring for people who are suffering; includes secondary traumatic stress
Vicarious trauma (McCann & Pearlman)	Lasting changes in the helper's beliefs and worldview from repeated exposure to clients' trauma

These apply to counselors too: self-care and impairment are ethical issues (ACA C.2.g; Chapter 17).

Other Workplace Concerns

- **Work–family conflict**: competing demands between roles (Super's Life-Career Rainbow is a useful frame).
- **Underemployment**: working below one's skills or desired hours.
- **Workplace discrimination and harassment**: validate, help the client document incidents, and explore options (HR, legal resources, advocacy).

- **Glass ceiling**: invisible barriers to advancement for women and people of color.
- **Job satisfaction**: Hoppock; Dawis & Lofquist's satisfaction and satisfactoriness.
- **Employee Assistance Programs (EAPs)**: short-term, employer-sponsored counseling and referral.

Part 5 — Career Concerns for Specific Groups

Group	Key issues
Women	Gender-role socialization narrows options early (Gottfredson); lower self-efficacy in some fields (Betz & Hackett); work–family role conflict; pay gaps
Racial and ethnic minority clients	Discrimination; occupational stereotyping; family and community expectations; collectivist decision-making (involve family when the client wants that)
LGBTQ+ clients	Disclosure decisions at work; workplace climate; discrimination
People with disabilities	Accommodations under the ADA; **vocational rehabilitation** services; matching abilities to work; avoiding assumptions about limits
Veterans	Translating military skills to civilian jobs; identity shift; VA vocational services
Older workers	Age discrimination; bridge employment; retirement planning (Chapter 12)
Clients with criminal records	Barriers to hiring; "ban the box" policies; reentry programs
First-generation college students	Less family knowledge of career paths; financial pressure

Career self-efficacy (Betz & Hackett, building on Bandura) helps explain why capable clients avoid fields: low confidence, not low ability. Building self-efficacy through mastery experiences, modeling, encouragement, and managing anxiety opens options.

Part 6 — Ethics in Career Counseling

- The **National Career Development Association (NCDA)** has its own Code of Ethics for career professionals.
- Use instruments you're **trained** to administer and interpret (ACA E.2).
- Choose instruments **normed** on groups similar to the client, and interpret cautiously when they aren't (ACA E.8).
- Online and computer-based assessments still require the counselor to make sure the instrument is valid and that the client receives a sound interpretation of the results (ACA E.7, H).
- Don't let a single test score determine a client's choice.

What the Exam Rewards / What It Punishes

Rewards: - Integrating career and personal concerns - Interpreting assessments **with** the client and linking results to goals - Distinguishing interest from aptitude - Exploring barriers, supports, and self-efficacy - Treating job loss as a loss: screening mood and suicide risk - Using O*NET and the OOH for occupational information

Punishes: - "Test-and-tell": letting a score choose the career - Treating a high interest score as proof of ability - Ignoring cultural and family context in decisions - Using the MBTI to select a career - Ignoring depression or anxiety in a career client

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Swap-Pair Drill

If you see…	It's…	Not…
Opposite on the hexagon	R–S, I–E, A–C	Adjacent types
Adjacent top codes	Consistency	Differentiation
One type clearly highest	Differentiation	Congruence
Person–environment fit	Congruence	Consistency
Self-administered, self-scored RIASEC	SDS (Holland)	Strong Interest Inventory
Compares interests with satisfied workers	Strong Interest Inventory	SDS
Work values	MIQ (Dawis & Lofquist)	Strong
Career indecision	Career Decision Scale (Osipow)	Career Maturity Inventory (Crites)
Blocking career beliefs	Career Beliefs Inventory (Krumboltz)	Career Thoughts Inventory
Replaced the DOT	O*NET	OOH
Crystallizing, specifying, implementing	Super's exploration stage	Ginzberg
Time structure, social contact, purpose, status, activity	Jahoda	Maslach
Exhaustion, depersonalization, reduced accomplishment	Maslach (burnout)	Figley (compassion fatigue)
Career decision-making styles: rational, intuitive, dependent	Harren	Tiedeman
Chronic difficulty deciding anything	Indecisiveness	Career indecision

Quick Checks

1. A client's Holland code is "RSE." The R and S types are: A. Adjacent and consistent B. Opposite and inconsistent C. Undifferentiated D. Congruent

2. A client's SDS shows high scores on all six RIASEC types. This profile is: A. Highly differentiated B. Undifferentiated C. Highly consistent D. Congruent

3. A client asks, "Which career does the test say I should choose?" The best response is to: A. Give the client the top-matched occupation B. Explain that results are one source of information and explore them together in light of the client's goals and values C. Recommend the client take more tests D. Refuse to discuss the results

4. A 52-year-old client laid off after 25 years says he feels "useless" and hasn't left the house in weeks. The counselor's first priority is to: A. Administer the Strong Interest Inventory B. Assess mood and suicide risk C. Update his résumé D. Explain Super's recycling concept

5. According to Jahoda, unemployment harms well-being partly because people lose: A. Only income B. Latent benefits such as time structure, social contact, and purpose C. Career maturity D. Congruence

6. A client with strong math ability avoids engineering because she "isn't a math person." Which theory best explains this and suggests building her confidence? A. Holland's congruence B. SCCT / career self-efficacy C. Roe's needs theory D. Ginzberg's developmental theory

7. Which source replaced the Dictionary of Occupational Titles? A. Occupational Outlook Handbook B. O*NET C. SOC D. ASVAB

8. A college junior has narrowed her interests to two related fields and is choosing between them. In Super's model, she is: A. Crystallizing B. Specifying C. Stabilizing D. Holding

9. A nurse feels emotionally drained, has become cynical about patients, and feels she's no longer effective. This best matches: A. Vicarious trauma B. Burnout C. Indecisiveness D. Underemployment

10. A client struggles to make decisions about everything, from careers to meals, and feels anxious about any choice. This is best described as: A. Career indecision B. Indecisiveness C. Low differentiation D. Low congruence

11. Which instrument measures work values rather than interests? A. Strong Interest Inventory B. Self-Directed Search C. Minnesota Importance Questionnaire D. Kuder

12. A counselor uses the MBTI to tell a client that, as an "INFP," she should become a writer. This practice is: A. Appropriate, because MBTI types predict career success B. Inappropriate, because the MBTI is for self-understanding and shouldn't be used to choose a career C. Required by NCDA ethics D. Appropriate if the client agrees

Answers and Rationales

1 — B. R and S are opposite on the hexagon, so the code is inconsistent. A: adjacent types would be, for example, R and I. C: differentiation concerns how much one type stands out. D: congruence concerns person–environment fit.

2 — B. A flat profile is undifferentiated. A: differentiation means one or two types stand out. C: consistency concerns hexagon position of top types. D: congruence is environment fit.

3 — B. Assessments inform; the client decides, with the counselor helping interpret (3A.5). A: test-and-tell. C: more tests don't address the misunderstanding. D: unhelpful.

4 — B. Withdrawal and feeling useless after job loss call for screening mood and suicide risk first. A, C, D: useful later.

5 — B. Jahoda's latent deprivation model. A: income is the manifest benefit. C and D: Super and Holland concepts.

6 — B. Low self-efficacy despite ability is central to SCCT (Lent, Brown, & Hackett; Betz & Hackett). A: Holland's fit. C: Roe's childhood needs. D: Ginzberg's developmental periods.

7 — B. O*NET replaced the DOT. A: the OOH is a separate BLS publication. C: SOC is a classification system. D: the ASVAB is an aptitude test.

8 — B. Narrowing to a specific choice is specifying. A: crystallizing forms a general preference. C: stabilizing is in establishment. D: holding is in maintenance.

9 — B. Maslach's three burnout dimensions. A: vicarious trauma involves changes in beliefs from exposure to trauma. C: indecisiveness is chronic difficulty deciding. D: underemployment is working below one's capacity.

10 — B. Chronic, generalized difficulty deciding is indecisiveness. A: career indecision is specific and often developmental. C and D: Holland concepts.

11 — C. The MIQ measures work values and needs (TWA). A, B, D: interest inventories.

12 — B. The MBTI supports self-understanding; it isn't designed to dictate career choice. A: overstates its predictive power. C: not an NCDA requirement. D: client agreement doesn't fix a misuse of the instrument.

Audio Companion

Track	Covers	Target length
14.1	Work It: Holland, Super, instruments, burnout, job loss	~4 min

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Chapter 15 — Culture, Identity, and Meaning

2027 task map: 4.2 Explore the impact of the client's culture, context, values, and worldview · 4.8 Integrate contextual and cultural influences into counselor–client relationship · CACREP: Social and Cultural Diversity; Human Growth and Development

Where This Chapter Fits

Chapter 9 covered the **counselor's** side of culture: multicultural competencies, ADDRESSING, microaggressions, broaching, interpreters, ASERVIC, and the ban on values-based referral. Chapter 3 gave you the identity development models (Cross, Helms, Atkinson/Morten/Sue, Cass, Berry). This chapter covers the **client's** concerns: adjusting to a new culture, developing a gender identity, living with racism and oppression, conflicts over religious values, and questions of meaning.

The through-line here: **the client defines their identity and the meaning of their experience.** The counselor's job is to understand, validate, and support, not to diagnose identity or decide what a client's faith or culture should mean to them.

Part 1 — Cultural Adjustment (4.2)

Culture Shock

Kalervo Oberg described **culture shock** as the anxiety that comes from losing familiar cultural cues. A commonly taught sequence:

Phase	Experience
Honeymoon	Excitement and fascination with the new culture

Phase	Experience
Crisis (frustration)	Irritation, homesickness, confusion, anxiety as differences become stressful
Recovery (adjustment)	Learning the new culture's rules; growing competence
Adaptation	Functioning comfortably in the new culture

The **U-curve** (Lysgaard) describes adjustment that starts high, dips, then recovers. The **W-curve** (Gullahorn & Gullahorn) adds a second dip on returning home: **reverse culture shock**.

Acculturation

Review Berry's four strategies from Chapter 3: **integration**, **assimilation**, **separation**, **marginalization**. Integration is generally associated with the best adjustment, and marginalization with the most distress, though fit depends on context.

Acculturative stress is the stress that comes from adapting to a new culture: language barriers, discrimination, loss of status, separation from family, and changing roles.

Acculturation gaps within families: children often acculturate faster than parents, which can lead to conflict and to role reversal (children acting as interpreters and brokers; Chapter 9 explains why counselors shouldn't use them that way).

Immigrants and Refugees

Phase	Possible stressors
Pre-migration	War, persecution, violence, poverty, loss
Migration	Dangerous journeys, detention, family separation, camps

Phase	Possible stressors
Post-migration	Acculturative stress, discrimination, legal uncertainty, unemployment or underemployment, isolation, grief

Counseling principles: - Address **safety and basic needs** first (Maslow). - Use **professional interpreters** (Chapter 9). - Assess trauma with care and **cultural humility**; distress may be expressed through physical complaints or culturally specific idioms. - Use the **Cultural Formulation Interview** (Chapter 9). - Build on **cultural strengths**: community, faith, family, and resilience. - Be aware that fears about legal status may affect disclosure and trust; clarify confidentiality.

Cultural Concepts of Distress (DSM-5-TR)

The DSM-5-TR describes **cultural concepts of distress**: ways cultures experience, understand, and communicate suffering. Examples include ***ataque de nervios*** (Latin American and Caribbean; episodes of intense emotional upset, shouting, crying, trembling), ***susto*** (“fright”; illness attributed to a frightening event that causes the soul to leave the body), ***taijin kyofusho*** (Japan; fear that one’s body or behavior offends others), ***khyâl cap*** (Cambodian “wind attacks” resembling panic), and ***kufungisisa*** (“thinking too much,” Zimbabwe).

Exam principle: understand a client’s distress in the client’s own cultural terms before applying a Western diagnosis.

Part 2 — Gender Identity Development (4.2)

Terms

Term	Meaning
Sex assigned at birth	The sex recorded at birth, usually based on external anatomy

Term	Meaning
Gender identity	One's internal sense of one's own gender
Gender expression	How one outwardly presents gender (clothing, voice, behavior)
Sexual orientation	Whom one is romantically or sexually attracted to; **distinct from gender identity**
Transgender	Gender identity differs from sex assigned at birth
Cisgender	Gender identity matches sex assigned at birth
Nonbinary	Gender identity outside the woman/man binary
Intersex (differences of sex development)	Variations in sex characteristics

Being transgender is not a mental disorder. The DSM-5-TR diagnosis of **gender dysphoria** describes clinically significant **distress** related to incongruence between experienced gender and assigned gender. Criteria require a duration of at least **6 months**, with separate criteria for children and for adolescents and adults.

Who Owns Identity Development Models

Owner	Model
Arlene Istar Lev	**Transgender emergence**: awareness → seeking information and reaching out → disclosure to significant others → exploring identity and self-labeling → exploring transition issues and possible body modification → integration (acceptance and post-transition issues)
Vivienne Cass	Homosexual identity formation (Chapter 3): confusion → comparison → tolerance → acceptance → pride → synthesis

Owner	Model
Richard Troiden	Sensitization → identity confusion → identity assumption → commitment
Anthony D'Augelli	Life-span model of lesbian, gay, and bisexual identity development; identity unfolds through six interrelated processes rather than fixed stages
Ilan Meyer	**Minority stress model** (below)
Caitlin Ryan	**Family Acceptance Project**: family rejection of LGBTQ youth is linked to much higher risk of depression, suicide attempts, and substance use; family acceptance is protective

Minority Stress (Meyer)

Type	Examples
Distal stressors (external)	Discrimination, harassment, violence, rejection
Proximal stressors (internal)	**Expectations of rejection**, **concealment** of identity, **internalized stigma**

Minority stress helps explain higher rates of depression, anxiety, and suicidality among sexual and gender minority people. The problem is the stress, not the identity.

Counseling Principles

- Use the client's **name and pronouns**.
- The client controls **coming out**: when, how, and to whom. Never disclose a client's identity without consent.
- Don't assume identity is the presenting concern, or that it's irrelevant.
- **Affirming** care; **conversion therapy is harmful** and contrary to professional standards (Chapter 9).
- Support families toward acceptance (Ryan).

- Know that the **World Professional Association for Transgender Health (WPATH)** publishes Standards of Care (Version 8, 2022) for transgender and gender diverse people. You may be asked to write letters supporting gender-affirming medical care; do so only within your competence and current standards.

Part 3 — Racism, Discrimination, and Oppression (4.2)

Who Owns the Concepts

Owner	Concept
Robert T. Carter	**Race-based traumatic stress**: racism can produce trauma-like reactions
Sherman James	**John Henryism**: high-effort coping with chronic stressors (including discrimination), linked to health problems in Black Americans with limited resources
Claude Steele & Joshua Aronson	**Stereotype threat**: concern about confirming a negative stereotype about one's group lowers performance
Pauline Clance & Suzanne Imes	**Impostor phenomenon**: high achievers feeling like frauds; often heightened for people in groups underrepresented in their fields
Greenwald, McGhee, & Schwartz	**Implicit Association Test (IAT)**: measures implicit bias
Derald Wing Sue	Microaggressions (Chapter 9)
Iris Marion Young	**Five faces of oppression**: exploitation, marginalization, powerlessness, cultural imperialism, violence
Paulo Freire	**Critical consciousness** (conscientização): recognizing and acting against oppressive conditions
Ignacio Martín-Baró	**Liberation psychology**

Owner	Concept
Kimberlé Crenshaw	**Intersectionality** (Chapter 9)
Peggy McIntosh	"White Privilege: Unpacking the Invisible Knapsack"

Levels of Racism

Level	Description
Individual (interpersonal)	Prejudice and discrimination between people
Institutional	Policies and practices within organizations that produce unequal outcomes
Structural (systemic)	The combined effects of institutions and history that sustain inequality across society
Internalized	A member of a targeted group comes to accept negative messages about their own group

Colorism: discrimination based on skin tone, often within as well as across racial groups.

Counseling Principles

- **Validate** experiences of discrimination. Questioning whether racism "really" happened is a microinvalidation and damages the alliance.
- **Assess the impact**: race-based stress can look like anxiety, depression, hypervigilance, anger, or trauma symptoms.
- Recognize **cultural mistrust** as a reasonable response to historical and current experiences, not paranoia.
- Explore **coping and strengths**: racial and ethnic identity, community, faith, activism.
- Consider **advocacy** at the client, community, or public level (ACA Advocacy Competencies, Chapter 9).
- Counselors examine their **own** biases and privilege (Chapter 9).

Part 4 — Religious Values Conflict (4.2)

Concepts and Owners

Owner	Concept
Gordon Allport	**Intrinsic religiosity** (faith as an end in itself, lived as the center of life) vs. **extrinsic religiosity** (faith used as a means, e.g., for social standing or comfort)
Kenneth Pargament	**Religious coping**: **positive** (seeking spiritual support, benevolent religious reappraisal) and **negative** (feeling punished or abandoned by God; spiritual struggle). Negative religious coping is linked to worse outcomes
James Fowler	Stages of faith (Chapter 9)
ASERVIC	Spiritual competencies (Chapter 9)

Common Religious Values Conflicts

- **Faith and identity**: a client whose sexual orientation or gender identity conflicts with their religious community's teachings
- **Faith transitions**: questioning, leaving, changing, or deepening faith
- **Interfaith relationships and family conflict** over religion
- **Religious scrupulosity**: obsessive fear of sin or offending God (an OCD presentation, Chapter 10)
- **Spiritual struggle** after loss or trauma ("Why did God let this happen?")
- **Religious trauma or harm** from coercive religious environments

Counseling Principles

- **The client leads.** Explore what the client values and wants, not what the counselor believes is right.

- **Don’t impose values** (ACA A.4.b) in either direction: neither pushing a client to leave a faith nor to stay.
- Help clients **hold tension** and explore options; resolution may be integration, change, or acceptance of ambiguity.
- With the client’s consent, faith leaders or spiritual practices can be resources.
- **Moral, Religious, or Spiritual Problem (Z65.8)** is a DSM-5-TR condition that may be a focus of clinical attention, not a mental disorder. It was called “Religious or Spiritual Problem” until the American Psychiatric Association’s September 2025 update, which kept the code and added **moral problems**: experiences that disrupt one’s understanding of right and wrong, or one’s sense of the goodness of oneself, others, or institutions (this includes moral injury; Chapter 9). Religious or spiritual problems include loss or questioning of faith, problems with conversion to a new faith, and questioning spiritual values, whether or not they involve an organized religion.

Part 5 — Spiritual and Existential Concerns (4.2)

Owners

Owner	Concept
Irvin Yalom	Four ultimate concerns: **death, freedom, isolation, meaninglessness** (Chapter 2)
Viktor Frankl	**Will to meaning**; meaning through creative work, love and experience, and the attitude one takes toward unavoidable suffering
Rollo May	Normal vs. neurotic anxiety
Ernest Becker	*The Denial of Death*

Owner	Concept
Greenberg, Solomon, & Pyszczynski	**Terror management theory**: awareness of death motivates people to hold onto cultural worldviews and self-esteem as buffers
John Welwood	**Spiritual bypassing**: using spiritual ideas or practices to avoid dealing with emotional pain or unresolved issues
Abraham Maslow	Peak experiences; self-transcendence (later added above self-actualization)
Stanislav & Christina Grof	**Spiritual emergency**: intense spiritual experiences that can resemble psychosis

Existential Concerns in Counseling

Clients may present with questions like “What’s the point?”, fear of death, loneliness that persists even in the company of others (**existential isolation**), anxiety about choices and responsibility, or a loss of meaning after retirement, illness, or loss.

Counseling principles: - Treat these as real human questions, not only as symptoms. - Screen for **depression and suicide risk** when meaninglessness is prominent (Chapter 6, Chapter 10). - Support **meaning-making** (Frankl, Neimeyer) and **authentic choice**. - Distinguish **spiritual experiences** from psychosis with care: consider the client’s culture, whether the experience is shared within the community, its effect on functioning, and the presence of other psychotic symptoms (Chapter 16).

What the Exam Rewards / What It Punishes

Rewards: - Understanding distress in the client’s cultural terms - Normalizing culture shock and acculturation stress - Using the client’s name and pronouns; client-controlled disclosure - Framing minority stress, not identity,

as the source of distress - Validating experiences of discrimination - Client-led exploration of religious conflicts without imposing values - Taking existential questions seriously while screening for risk

Punishes: - Pathologizing gender identity or sexual orientation - Outing a client - Questioning whether discrimination “really” happened - Labeling cultural mistrust as paranoia - Pushing a client toward or away from a faith - Treating every spiritual experience as psychosis, or none of them

Swap-Pair Drill

If you see…	It’s…	Not…
Honeymoon, crisis, recovery, adaptation	Oberg’s culture shock	Kübler-Ross
Second dip on returning home	W-curve; reverse culture shock	U-curve
Ataque de nervios, susto	Cultural concepts of distress	Panic disorder
Gender identity vs. sexual orientation	Distinct constructs	The same thing
Distress about gender incongruence	Gender dysphoria	Being transgender
Transgender emergence	Lev	Cass
Expected rejection, concealment, internalized stigma	Proximal minority stressors (Meyer)	Distal stressors
Family acceptance protects LGBTQ youth	Caitlin Ryan	Meyer
Race-based traumatic stress	Robert Carter	Sue
High-effort coping and health	John Henryism (Sherman James)	Stereotype threat
Fear of confirming a stereotype lowers performance	Stereotype threat (Steele & Aronson)	Impostor phenomenon

If you see…	It's…	Not…
Feeling like a fraud	Impostor phenomenon (Clance & Imes)	Stereotype threat
Five faces of oppression	Iris Marion Young	Freire
Critical consciousness	Freire	Martín-Baró
Intrinsic vs. extrinsic religiosity	Allport	Pargament
Positive vs. negative religious coping	Pargament	Allport
Fear of death drives worldview defense	Terror management theory	Yalom
Using spirituality to avoid pain	Spiritual bypassing (Welwood)	Spiritual emergency (Grof)
Moral, religious, or spiritual problem	Z65.8 (focus of attention, not a disorder)	A mental disorder

Quick Checks

1. Four months after moving to the U.S. for graduate school, an international student who was initially excited now feels irritable, homesick, and confused. She is most likely in which phase of culture shock? A. Honeymoon B. Crisis C. Recovery D. Adaptation

2. A client from El Salvador describes episodes of uncontrollable shouting, crying, and trembling after family conflicts, which she calls *ataque de nervios*. The counselor's best first step is to: A. Diagnose panic disorder B. Explore what the experience means to her and her family, using the Cultural Formulation Interview C. Refer her for a psychotic disorder evaluation D. Tell her it's a cultural myth

3. Which statement is accurate? A. Being transgender is a mental disorder in the DSM-5-TR B. Gender dysphoria refers to clinically significant distress related to gender incongruence C. Gender identity and sexual orientation

are the same thing D. Gender dysphoria requires a duration of 2 years

4. A gay client hasn't told his coworkers about his partner and constantly monitors what he says. According to Meyer, this is an example of: A. A distal stressor B. A proximal stressor (concealment) C. Internalized racism D. Stereotype threat

5. A 16-year-old client comes out as transgender to her counselor but hasn't told her parents. The parents call to ask what their daughter discusses in counseling. The counselor should: A. Tell the parents about her gender identity B. Protect the client's disclosure within the limits of confidentiality and state law, and explore with the client whether, when, and how she wants to tell them C. Refer the client to a specialist without explanation D. Encourage her to keep it from her parents permanently

6. A Black client describes repeated discrimination at work and asks, "Am I being too sensitive?" The best response is: A. "Maybe they didn't mean it that way." B. Validate the experience and explore its impact on him C. Tell him to report it to HR immediately D. Change the subject to his treatment goals

7. A high-performing Latina engineer says, "Any day now, they'll realize I don't belong here." This best reflects: A. Stereotype threat B. Impostor phenomenon C. John Henryism D. Acculturative stress

8. A client says that after his child's death, he feels God has abandoned and is punishing him. Pargament would call this: A. Positive religious coping B. Negative religious coping C. Extrinsic religiosity D. Spiritual bypassing

9. A client who is a devout member of a conservative faith is struggling to reconcile her faith with her bisexual identity. The counselor should: A. Encourage her to leave her faith community B. Encourage her to change her orientation to fit her faith C. Help her explore her own values and options without imposing the counselor's views D. Refer her to her pastor

10. A client meditates for hours daily and insists "everything happens for a reason" whenever she talks about her abusive childhood, refusing to discuss her feelings. This may reflect: A. Spiritual emergency B. Spiritual bypassing C. Positive religious coping D. Terror management

11. "Moral, religious, or spiritual problem" (Z65.8) in the DSM-5-TR is: A. A mental disorder B. A condition that may be a focus of clinical attention C. A cultural concept of distress D. A personality disorder

12. Yalom's four ultimate concerns are: A. Death, freedom, isolation, meaninglessness B. Exploitation, marginalization, powerlessness, violence C. Loss, rejection, guilt, grief D. Safety, remembrance, mourning, reconnection

Answers and Rationales

1 — B. The shift from excitement to frustration and homesickness is the crisis phase (Oberg). A: honeymoon is the excited early period. C and D: recovery and adaptation bring growing comfort.

2 — B. Understand cultural concepts of distress in the client's terms first. A and C: premature Western diagnosis. D: dismissive.

3 — B. Gender dysphoria is distress related to incongruence. A: being transgender isn't a disorder. C: they're distinct. D: the duration is 6 months.

4 — B. Concealment is a proximal minority stressor. A: distal stressors are external events like discrimination. C: internalized racism concerns race. D: stereotype threat concerns performance.

5 — B. Disclosure belongs to the client; protect it within confidentiality law and support her decision-making. A: outing her could harm her. C: unexplained referral is abandonment. D: imposes the counselor's view.

6 — B. Validation and exploring impact. A: a microinvalidation. C: premature direction before understanding. D: avoids the client's concern.

7 — B. Feeling like a fraud despite success is the impostor phenomenon (Clance & Imes). A: stereotype threat is performance anxiety about confirming a stereotype in a specific situation. C: John Henryism is effortful coping linked to health. D: acculturative stress relates to adapting to a new culture.

8 — B. Feeling punished or abandoned by God is negative religious coping (Pargament). A: positive coping seeks spiritual support. C: extrinsic religiosity is Allport's concept of faith used as a means. D: bypassing uses spirituality to avoid feelings.

9 — C. Client-led exploration without imposing values (ACA A.4.b). A and B: impose a direction. D: referral away from counseling based on the topic isn't warranted; involving clergy would happen only at her request and with her consent.

10 — B. Using spiritual ideas to avoid emotional pain is spiritual bypassing (Welwood). A: spiritual emergency is an intense, destabilizing spiritual experience. C: positive coping uses faith for support while still facing distress. D: terror management concerns death awareness.

11 — B. Z65.8 is a condition that may be a focus of clinical attention. A: not a disorder. C: cultural concepts are culture-specific idioms of distress. D: not a personality disorder.

12 — A. Yalom's ultimate concerns. B: Young's faces of oppression (minus cultural imperialism). C: part of Silverstein and Kaplan's core adoption issues. D: Herman's stages of trauma recovery.

Audio Companion

Track	Covers	Target length
15.1	Identity and Meaning: culture shock, gender identity, minority stress, racism, faith, existential concerns	~4 min

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Chapter 16 — Body and Mind

2027 task map: 2B.6 Determine co-occurring diagnoses, differential diagnoses, and other comorbidities · 3A.11 Develop behavioral management plans · 4.11 Provide psychoeducation · CACREP: Counseling and Helping Relationships; Human Growth and Development · 2027 supplemental content guidance: presenting concerns

The Last Presenting-Concerns Chapter

This chapter gathers the remaining presenting concerns from the 2027 supplemental content guidance. They're grouped loosely around the connection between body and mind: illness, sleep, eating, sex, stress, and attention, plus behavior problems, bullying, caregiving, money, loneliness, and hallucinations.

Two rules organize this chapter: 1. **Rule out the medical first.** Many of these concerns have medical causes or medical risks. The counselor's job is to make sure a medical evaluation happens, then to work within scope. 2. **Function over label.** Ask what the problem does in the client's life and what it costs, not just what to call it.

Part 1 — Adjustment to Illness, Injury, and Disability (2B.6)

Owners

Owner	Contribution
George Engel (1977)	**Biopsychosocial model**: illness involves biological, psychological, and social factors

Owner	Contribution
Irwin Rosenstock and colleagues	**Health belief model**: health behavior depends on perceived **susceptibility**, perceived **severity**, perceived **benefits**, perceived **barriers**, **cues to action**, and (later) **self-efficacy**
Moos & Holahan	Adaptive tasks of coping with illness: dealing with symptoms, treatment, and providers; preserving emotional balance, self-image, and relationships; preparing for an uncertain future
Beatrice Wright	Disability and rehabilitation psychology; acceptance of disability; the "spread" effect (assuming one limitation means global limitation)

What Adjustment Looks Like

Reactions to illness, injury, amputation, chronic pain, or acquired disability vary widely: shock, anxiety, grief, anger, denial, depression, and gradual adaptation. **Adjustment isn't linear and doesn't follow a fixed sequence.** Grief over lost abilities is normal; persistent hopelessness, withdrawal, or suicidal thinking needs assessment.

Related Presentations (DSM-5-TR)

Presentation	Key features
Somatic symptom disorder	One or more distressing physical symptoms plus **excessive thoughts, feelings, or behaviors** about them; at least **6 months** of being symptomatic. The symptoms may or may not have a medical explanation
Illness anxiety disorder	Preoccupation with having or getting a serious illness; **few or no actual somatic symptoms**; at least 6 months

Presentation	Key features
Functional neurological symptom disorder (conversion disorder)	Neurological symptoms (weakness, seizures, numbness) inconsistent with recognized neurological conditions
Psychological factors affecting other medical conditions	Psychological or behavioral factors that worsen a medical condition (e.g., anxiety worsening asthma)
Adjustment disorder	Distress in response to a medical diagnosis or other stressor

Exam principle: never tell a client their symptoms are "all in your head." Validate the suffering, coordinate with medical providers, and focus on coping and functioning.

What Helps

- **CBT** and **ACT** for chronic pain and illness-related distress
- Coping skills, pacing, and problem-solving
- **Integrated care** with medical providers (with a release of information)
- Support groups
- Attention to **meaning**, identity, and relationships
- Accommodations and advocacy for clients with disabilities (ADA; Chapter 9)

Part 2 — Behavioral Problems (3A.11)

Presentations

Presentation	Key features	Time frame
Oppositional defiant disorder	Angry/irritable mood, argumentative/ defiant behavior, or vindictiveness; at least **4 symptoms**, shown with at least one person who isn't a sibling	At least **6 months**

Presentation	Key features	Time frame
Conduct disorder	Violating others' rights or major norms: aggression to people and animals, destruction of property, deceitfulness or theft, serious rule violations; at least **3 of 15** criteria	In the past **12 months**, with at least one in the past 6 months. **Childhood-onset** (at least one symptom before age 10) vs. **adolescent-onset**. Specifier: **with limited prosocial emotions**
Intermittent explosive disorder	Recurrent outbursts of verbal or physical aggression out of proportion to the trigger	Age at least 6
Antisocial personality disorder	Pervasive disregard for others' rights	Age at least 18, with evidence of conduct disorder before age 15

DMDD vs. ODD: if criteria for both disruptive mood dysregulation disorder (DMDD) and ODD are met, diagnose only DMDD (Chapter 10).

Owners and Treatments

Owner	Approach
Gerald Patterson	Coercion theory; **parent management training** (Chapter 13)
Scott Henggeler	**Multisystemic therapy (MST)**: intensive home- and community-based treatment for serious antisocial behavior, targeting family, peers, school, and neighborhood
James Alexander & Bruce Parsons	**Functional family therapy (FFT)**
Alan Kazdin	**Problem-solving skills training (PSST)**
Sheila Eyberg	PCIT for young children (Chapter 5)

Functional Behavior Assessment

A **functional behavior assessment (FBA)** identifies what a behavior **does** for the person using the **ABC** model: - **A**ntecedent: what happens right before - **B**ehavior: what the person does - **C**onsequence: what happens right after, which may be reinforcing the behavior

Common functions: getting **attention**, **escaping** a demand, getting something **tangible**, or **sensory** stimulation. Interventions then teach a replacement behavior that serves the same function.

Part 3 — Bullying

Dan Olweus defined bullying by three features: 1. **Intentional** harm 2. **Repeated** over time 3. A **power imbalance** between the person bullying and the target

Type	Examples
Physical	Hitting, pushing, damaging belongings
Verbal	Name-calling, threats, teasing
Relational (social)	Exclusion, rumors, damaging relationships
Cyberbullying	Harassment through texts, social media, or online platforms; can be constant and public

Roles: the person bullying, the target, **bully-victims** (who both bully and are bullied, and often have the most problems), and **bystanders**, who can either reinforce bullying or interrupt it.

Counseling Principles

- **Screen targets** for depression, anxiety, school avoidance, and **suicide risk**.
- **Peer mediation is not recommended for bullying.** Mediation assumes two equal parties in a conflict; bullying involves a power imbalance, and mediation can revictimize the target.

- **Whole-school approaches** (such as the Olweus Bullying Prevention Program) work better than isolated interventions.
- Teach **bystanders** to intervene safely and to report.
- Work with the person who bullies as well: empathy, consequences, and underlying problems.
- Know school policies and state anti-bullying laws.

Part 4 — Caregiving and Financial Stress

Caregiving

Caring for a family member with illness, disability, or dementia can bring meaning and closeness, along with **caregiver burden**: physical exhaustion, isolation, financial strain, role changes, grief (including anticipatory and ambiguous loss; Chapter 12), and higher rates of depression and anxiety.

- Assess burden (e.g., the **Zarit Burden Interview**).
- Encourage **respite care**, support groups, and help from others.
- Address guilt about limits and about decisions such as placement in residential care.
- Watch for caregiver depression and for signs of elder abuse or neglect (Chapter 6).
- The **sandwich generation** cares for children and aging parents at once (Chapter 12).

Financial Issues

Financial stress is linked to anxiety, depression, relationship conflict, and suicide risk (especially with sudden losses or unmanageable debt).

- Address basic needs and stress first (Maslow).
- Explore **beliefs and emotions about money** (often shaped by family of origin).
- Couples often fight about money; it can reflect deeper issues of power, trust, and values.

- **Stay within scope:** counselors address the emotional and relational side and **refer** to financial professionals (credit counselors, financial planners, legal aid) for specific financial advice.
- Screen for **gambling** and compulsive buying (Chapter 11).

Part 5 — Loneliness and Attachment

Term	Meaning
Social isolation	**Objective**: few social contacts
Loneliness	**Subjective**: the painful gap between desired and actual connection. A person can be lonely in a crowd or content alone
Emotional loneliness (Robert Weiss)	Missing a close, intimate attachment figure
Social loneliness (Robert Weiss)	Missing a broader social network

Research highlights: - **John Cacioppo** showed that loneliness changes social thinking: lonely people become more alert to social threat, which can make reaching out harder. - **Julianne Holt-Lunstad's** research links social isolation and loneliness to higher mortality risk. - The **U.S. Surgeon General's 2023 advisory** described an "epidemic of loneliness and isolation." - A meta-analysis by **Masi and colleagues (2011)** found that interventions targeting **maladaptive social cognition** (negative expectations and interpretations about others) were more effective than those that only increased social contact.

Counseling principles: assess attachment history (Chapter 12), social skills, and negative social expectations; use CBT to address those expectations; encourage graded social engagement; consider groups. Screen for depression and suicide risk, especially in isolated older adults.

Part 6 — Hyper/Hypo Mental Focus

Hypo-focus (difficulty sustaining attention) and **hyper-focus** (intense absorption that's hard to shift) both show up in counseling.

Possible source	Pattern
ADHD	Inattention, distractibility, and often **hyper-focus** on highly interesting tasks (Chapter 12)
Depression	Poor concentration, slowed thinking
Anxiety	Attention captured by worry or threat
Mania/hypomania	Distractibility, racing thoughts (Chapter 10)
Trauma	Hypervigilance; dissociation
Substances or sleep deprivation	Impaired attention
Neurocognitive disorder or delirium	Attention and memory problems (Chapter 12)
Obsessive preoccupation	Attention locked on obsessions (Chapter 10)

Dissociative symptoms include **depersonalization** (feeling detached from oneself, as if watching from outside) and **derealization** (the world feels unreal or dream-like). Dissociation often follows trauma and can disrupt focus and memory.

Exam principle: attention problems have many causes. Assess before assuming ADHD.

Part 7 — Sleep

Sleep Basics

- Sleep cycles through **NREM** stages (N1, N2, N3; N3 is deep, slow-wave sleep) and **REM** sleep in cycles of roughly **90 minutes**.

- **REM** sleep is when most vivid dreaming occurs; the body's major muscles are normally paralyzed.
- The **circadian rhythm** is the roughly 24-hour internal clock regulated by light.

Presentations

Presentation	Key features
Insomnia disorder	Difficulty falling asleep, staying asleep, or waking too early, causing distress or impairment; at least **3 nights per week** for at least **3 months**, despite adequate opportunity to sleep
Hypersomnolence disorder	Excessive sleepiness despite at least 7 hours of sleep
Narcolepsy	Recurrent irresistible sleep episodes; often **cataplexy** (sudden loss of muscle tone, often triggered by emotion)
Obstructive sleep apnea (breathing-related sleep disorder)	Snoring, breathing pauses, daytime sleepiness; requires **medical evaluation** (sleep study)
Circadian rhythm sleep-wake disorders	Mismatch between internal clock and schedule (delayed sleep phase common in adolescents; shift work type)
NREM sleep arousal disorders	**Sleepwalking** and **sleep terrors**; occur in deep NREM sleep, usually early in the night; little or no memory of the episode
Nightmare disorder	Repeated, well-remembered disturbing dreams; occurs in **REM** sleep, usually later in the night; person becomes alert on waking
REM sleep behavior disorder	Acting out dreams because muscle paralysis during REM is absent
Restless legs syndrome	Urge to move the legs, especially in the evening, relieved by movement

Sleep terrors vs. nightmares: sleep terrors happen in **NREM**, early in the night, with **little memory** and hard-to-wake confusion. Nightmares happen in **REM**, later in the night, are **remembered**, and the person wakes fully alert.

What Helps

CBT for insomnia (CBT-I) is the first-line treatment for chronic insomnia. Components:

Component	Owner / idea
Stimulus control	**Richard Bootzin**: use the bed only for sleep (and sex); go to bed only when sleepy; get up if not asleep in about 20 minutes; same wake time daily
Sleep restriction	**Arthur Spielman**: limit time in bed to actual sleep time to build sleep drive, then gradually extend
Sleep hygiene	Consistent schedule; limit caffeine, alcohol, nicotine, and screens near bedtime; comfortable environment (sleep hygiene alone is usually not enough for chronic insomnia)
Cognitive restructuring	Address worries about sleep ("If I don't sleep 8 hours, tomorrow will be a disaster")
Relaxation	Reduce physiological arousal

Imagery rehearsal therapy (rewriting a nightmare's script while awake and rehearsing it) helps nightmare disorder, including trauma-related nightmares.

Refer for **medical evaluation** when sleep apnea, narcolepsy, or another medical cause is suspected.

Part 8 — Maladaptive Eating

Presentations

Presentation	Key features
Anorexia nervosa	Restriction leading to **significantly low body weight**; intense fear of weight gain or behavior that interferes with weight gain; disturbed experience of body weight or shape. Types: **restricting** or **binge-eating/purging**. Severity for adults based on BMI (mild, moderate, severe, extreme)
Bulimia nervosa	Recurrent **binge eating** and **compensatory behaviors** (vomiting, laxatives, fasting, excessive exercise), both at least **once a week for 3 months**; self-worth overly tied to shape and weight; weight is often in the normal range
Binge-eating disorder	Recurrent binge eating with loss of control and marked distress, at least **once a week for 3 months**, **without** regular compensatory behaviors
ARFID (avoidant/restrictive food intake disorder)	Restricted eating due to sensory aversion, lack of interest, or fear of consequences (e.g., choking), **without** body-image concerns
Pica	Eating nonfood substances for at least **1 month**, inappropriate for developmental level
Rumination disorder	Repeated regurgitation of food for at least 1 month

Medical risk is serious. Anorexia has one of the highest mortality rates of any mental disorder. Purging can cause dangerous electrolyte imbalances and heart problems. **Refeeding syndrome** (dangerous shifts in electrolytes when nutrition is restored too quickly) requires medical management.

Screening and Treatment

Tool	Use
SCOFF	Five-question screen
EDE-Q (Fairburn & Beglin)	Eating Disorder Examination Questionnaire
EDE (Fairburn)	Clinician interview

Treatment	Notes
Family-based treatment (Maudsley; Lock & Le Grange)	First-line for **adolescent anorexia** (Chapter 5)
Enhanced CBT (CBT-E) (Christopher Fairburn)	Strong support for **bulimia** and **binge-eating disorder**, and used for anorexia in adults
DBT	When emotion dysregulation drives binge eating
Multidisciplinary team	Medical provider, dietitian, therapist, and psychiatric care as needed

Exam principles: coordinate with medical providers from the start; level of care depends on medical stability; avoid commenting on clients' weight or appearance; screen for suicide risk and co-occurring conditions.

Part 9 — Sexual Functioning

Models of Sexual Response

Owner	Model
Masters & Johnson	Four phases: **excitement → plateau → orgasm → resolution**; developed **sensate focus**
Helen Singer Kaplan	Triphasic model: **desire → excitement → orgasm** (added desire as a phase)
Rosemary Basson	**Circular model** of women's sexual response: desire can be **responsive** (arising after arousal begins) rather than spontaneous; emotional intimacy is central

Owner	Model
Jack Annon	**PLISSIT** model of sex counseling

PLISSIT (Annon)

Level	Counselor action
Permission	Give permission to talk about sex and normalize concerns
Limited **I**nformation	Provide basic, accurate information
Specific **S**uggestions	Offer specific strategies (e.g., sensate focus)
Intensive **T**herapy	Refer for or provide specialized sex therapy

Most clients' needs are met at the first two or three levels. **Intensive therapy requires specialized training.**

Sexual Dysfunctions (DSM-5-TR)

Include erectile disorder, delayed ejaculation, premature (early) ejaculation, female orgasmic disorder, female sexual interest/arousal disorder, male hypoactive sexual desire disorder, and genito-pelvic pain/penetration disorder. Most require symptoms for about **6 months** and clinically significant distress.

Exam principles: - **Rule out medical causes and medication effects first** (e.g., vascular disease, diabetes, hormonal changes; **SSRIs** commonly affect desire and orgasm). - Address relationship factors, anxiety (including performance anxiety), trauma history, and cultural and religious beliefs. - **Sensate focus** (Masters & Johnson) reduces performance pressure through structured, nondemand touch. - Stay within competence; refer for specialized sex therapy when needed.

Part 10 — Stress Management (4.11)

Owners

Owner	Contribution
Hans Selye	General Adaptation Syndrome: alarm → resistance → exhaustion; eustress vs. distress (Chapter 3)
Holmes & Rahe	Social Readjustment Rating Scale: life changes, positive or negative, add stress
Richard Lazarus & Susan Folkman	**Transactional model**: stress depends on **appraisal**. **Primary appraisal**: Is this a threat, harm, or challenge? **Secondary appraisal**: Do I have the resources to cope? Coping is **problem-focused** (change the situation) or **emotion-focused** (manage the feelings)
Suzanne Kobasa	**Hardiness**: the "three Cs" of **commitment**, **control**, and **challenge** buffer stress
Friedman & Rosenman	**Type A** behavior (competitive, impatient, hostile) vs. Type B; later research pointed to **hostility** as the harmful component for heart disease
Herbert Benson	Relaxation response
Edmund Jacobson	Progressive muscle relaxation
Jon Kabat-Zinn	Mindfulness-Based Stress Reduction (MBSR)
Donald Meichenbaum	Stress inoculation training (Chapter 2)

Matching coping to the stressor: problem-focused coping works best when the situation **can** be changed; emotion-focused coping works best when it **can't** (e.g., a loved one's terminal diagnosis).

Part 11 — Hallucinations and Psychotic Experiences

Presentations

Presentation	Key features	Time frame
Schizophrenia	At least **2 of 5** symptoms (delusions, hallucinations, disorganized speech, grossly disorganized or catatonic behavior, negative symptoms), at least one of the first three; decline in functioning	Active symptoms for at least **1 month**; continuous signs for at least **6 months**
Schizophreniform disorder	Same symptoms as schizophrenia	At least **1 month** but less than **6 months**
Brief psychotic disorder	Sudden psychotic symptoms with full return to functioning	At least **1 day** but less than **1 month**
Schizoaffective disorder	A major mood episode concurrent with schizophrenia symptoms, **plus** delusions or hallucinations for at least **2 weeks** without a mood episode	—
Delusional disorder	One or more delusions without other prominent psychotic symptoms; functioning often relatively intact	At least **1 month**
Mood disorder with psychotic features	Psychosis occurs **only** during mood episodes	—

Positive symptoms add something (hallucinations, delusions, disorganization). **Negative symptoms** take something away: **avolition** (low motivation), **alogia** (reduced speech), **anhedonia**, **asociality**, and **flat or blunted affect**.

Hallucinations Without a Psychotic Disorder

Not every hallucination means psychosis: - **Hypnagogic** (falling asleep) and **hypnopompic** (waking) hallucinations are common - **Bereavement**: sensing, seeing, or hearing a deceased loved one is common and often comforting - **Substances**, withdrawal, and **delirium** (Chapter 11, Chapter 12) - **Trauma**: flashbacks and dissociative experiences - **Cultural and religious experiences** shared and accepted within the client's community - **Medical and neurological conditions**

Owners

Owner	Contribution
Emil Kraepelin	Described "dementia praecox"
Eugen Bleuler	Coined "schizophrenia"; the **four As**: loosened **associations**, blunted **affect**, **ambivalence**, **autism** (withdrawal into one's inner world)
George Brown; **Vaughn & Leff**	**Expressed emotion (EE)**: families high in criticism, hostility, and emotional over-involvement are linked to higher **relapse** risk; family psychoeducation lowers it

Counseling Principles

- **Don't argue with or agree with** delusions or hallucinations. Acknowledge the client's experience and feelings ("That sounds frightening") without confirming the content.
- **Assess risk**, especially **command hallucinations** telling the client to harm self or others (risk assessment, Chapter 6).
- **Refer for psychiatric evaluation**; antipsychotic medication is a mainstay of treatment for psychotic disorders.
- **Coordinated specialty care** for **first-episode psychosis** (team-based care including medication,

therapy, family education, and supported employment and education) improves outcomes.

- **CBT for psychosis (CBTp)**, **family psychoeducation**, **social skills training**, and **supported employment** have research support.
- **Consider culture** before labeling an experience psychotic.

What the Exam Rewards / What It Punishes

Rewards: - Medical evaluation first for physical symptoms, sleep apnea, eating disorders, sexual dysfunction, and sudden changes - Validating physical suffering without saying “it’s all in your head” - Functional behavior assessment (ABC) - Rejecting peer mediation for bullying - Referring for specialized financial or sex therapy help when beyond scope - CBT-I for chronic insomnia - Family-based treatment for adolescent anorexia; CBT-E for bulimia and binge eating - Matching coping style to whether the stressor can be changed - Neither arguing with nor confirming hallucinations; assessing command hallucinations

Punishes: - Treating eating disorders without medical coordination - Assuming every attention problem is ADHD - Peer mediation between a bully and a target - Treating a bereaved client’s sense of the deceased’s presence as psychosis - Giving specific financial or medical advice outside scope

Swap-Pair Drill

If you see…	It’s…	Not…
Biopsychosocial model	Engel	Rosenstock
Susceptibility, severity, benefits, barriers, cues	Health belief model	Biopsychosocial model

If you see…	It's…	Not…
Excessive thoughts about real physical symptoms	Somatic symptom disorder	Illness anxiety disorder
Fear of illness with few or no symptoms	Illness anxiety disorder	Somatic symptom disorder
At least 4 symptoms, 6 months, angry and defiant	ODD	Conduct disorder
3 of 15 criteria, 12 months, violating rights	Conduct disorder	ODD
Home- and community-based treatment for antisocial youth	MST (Henggeler)	FFT
Antecedent, behavior, consequence	Functional behavior assessment	Conflict resolution
Intentional, repeated, power imbalance	Olweus's definition of bullying	Conflict
Missing an intimate attachment	Emotional loneliness (Weiss)	Social loneliness
Insomnia: 3 nights a week for 3 months	Insomnia disorder	Hypersomnolence
NREM, early night, little memory	Sleep terrors	Nightmares
REM, later night, remembered	Nightmares	Sleep terrors
Bed only for sleep; get up if awake	Stimulus control (Bootzin)	Sleep restriction (Spielman)
Binge + compensatory behavior	Bulimia nervosa	Binge-eating disorder
Restriction without body-image concern	ARFID	Anorexia
Excitement, plateau, orgasm, resolution	Masters & Johnson	Kaplan
Desire, excitement, orgasm	Kaplan	Masters & Johnson
Responsive desire	Basson	Kaplan

If you see…	It's…	Not…
Permission, limited information, specific suggestions, intensive therapy	PLISSIT (Annon)	Sensate focus
Primary and secondary appraisal	Lazarus & Folkman	Selye
Commitment, control, challenge	Kobasa (hardiness)	Lazarus
Psychotic symptoms 1 day to under 1 month, full recovery	Brief psychotic disorder	Schizophreniform
Schizophrenia symptoms 1 to under 6 months	Schizophreniform	Schizophrenia
Associations, affect, ambivalence, autism	Bleuler	Kraepelin
Family criticism and over-involvement predict relapse	Expressed emotion (Brown; Vaughn & Leff)	Double bind

Quick Checks

1. A client with chronic back pain spends hours a day researching her symptoms, is convinced every twinge signals a serious disease, and has limited her life significantly for 8 months. This best fits: A. Illness anxiety disorder B. Somatic symptom disorder C. Functional neurological symptom disorder D. Malingering

2. A 12-year-old has, for the past year, bullied other students, stolen from stores, and repeatedly stayed out all night. This pattern best fits: A. ODD B. Conduct disorder C. ADHD D. Intermittent explosive disorder

3. A school counselor learns that a student is being repeatedly targeted by a larger, more popular peer. The counselor should NOT: A. Assess the target for depression and suicide risk B. Bring the two students together for peer mediation C. Involve school administrators per policy D. Work with bystanders

4. A client reports trouble falling asleep 4–5 nights a week for the past 5 months and has started napping and spending long hours in bed. The first-line treatment is: A. Sleep hygiene education alone B. CBT for insomnia C. Long-term sleeping medication D. Imagery rehearsal therapy

5. A parent describes her 5-year-old sitting up screaming about an hour after falling asleep, unresponsive to comfort, and remembering nothing the next morning. This best fits: A. Nightmare disorder B. Sleep terrors C. REM sleep behavior disorder D. Narcolepsy

6. A client binges several times a week, then fasts and exercises excessively to compensate; her weight is in the normal range. This best fits: A. Anorexia nervosa, binge/purge type B. Bulimia nervosa C. Binge-eating disorder D. ARFID

7. A client reports low sexual desire since starting an SSRI 2 months ago. The counselor's first step is to: A. Begin sensate focus exercises B. Encourage the client to talk with the prescriber about possible medication side effects C. Diagnose female sexual interest/arousal disorder D. Explore childhood sexual history

8. In the PLISSIT model, explaining that it's common for desire to fluctuate in long-term relationships is an example of: A. Permission B. Limited information C. Specific suggestions D. Intensive therapy

9. A client's spouse has a terminal diagnosis. According to Lazarus and Folkman, which type of coping is likely to help most with the diagnosis itself? A. Problem-focused coping B. Emotion-focused coping C. Avoidance D. Hardiness

10. Three weeks after her husband's death, a widow reports she sometimes hears his voice saying her name and finds it comforting. She has no other unusual symptoms. The counselor should: A. Refer her urgently for a psychotic disorder evaluation B. Normalize this as a common bereavement experience while continuing to monitor C. Dia-

gnose brief psychotic disorder D. Tell her the voice isn't real

11. A client with schizophrenia says the TV is sending him messages. The best counselor response is: A. "That's impossible; TVs can't send messages." B. "Yes, the messages are real." C. "That sounds really unsettling. What are the messages telling you?" D. Ignore the comment and change the subject

12. Family members of a client with schizophrenia are highly critical and emotionally over-involved. Research on expressed emotion suggests this is linked to: A. Lower relapse risk B. Higher relapse risk C. No change in outcomes D. Faster recovery

Answers and Rationales

1 — B. Distressing physical symptoms plus excessive thoughts and behaviors for over 6 months. A: illness anxiety involves few or no somatic symptoms. C: functional neurological symptoms are neurological (weakness, seizures). D: malingering is intentional faking for external gain.

2 — B. Violating others' rights and major rules across 12 months meets conduct disorder patterns. A: ODD is anger, defiance, and vindictiveness without serious rule violations. C: ADHD involves inattention and hyperactivity. D: IED involves explosive outbursts.

3 — B. Peer mediation isn't appropriate because of the power imbalance. A, C, D: all appropriate.

4 — B. CBT-I is first-line for chronic insomnia. A: sleep hygiene alone is usually not enough. C: medication is a prescriber decision and not first-line long-term. D: imagery rehearsal targets nightmares.

5 — B. Early-night, unresponsive, no memory: NREM sleep terrors. A: nightmares are remembered and occur in REM later in the night. C: REM sleep behavior disorder involves acting out dreams during REM, usually in older adults. D: narcolepsy involves irresistible daytime sleep.

6 — B. Bingeing plus compensatory behavior at normal weight. A: anorexia requires significantly low weight. C: binge-eating disorder lacks regular compensation. D: ARFID lacks body-image concerns and bingeing.

7 — B. Rule out medication effects first; SSRIs commonly affect desire. A: treats before ruling out a medical cause. C: premature diagnosis. D: not the first step given the clear timing.

8 — B. Basic, accurate information is the LI level. A: permission normalizes talking about sex. C: specific suggestions are concrete strategies. D: intensive therapy is specialized treatment.

9 — B. When the stressor can't be changed, emotion-focused coping fits. A: problem-focused coping suits changeable situations. C: avoidance tends to worsen outcomes. D: hardiness is Kobasa's trait, not a coping type in this model.

10 — B. Sensing a deceased loved one is common in bereavement. A and C: pathologize a normal experience. D: dismissive.

11 — C. Acknowledge the feeling without confirming or arguing, and assess the content (which can reveal risk). A: arguing damages trust. B: colludes with the delusion. D: misses important information.

12 — B. High expressed emotion is linked to higher relapse risk. A, C, D: contradict the research.

Audio Companion

Track	Covers	Target length
16.1	Body and Mind: illness, behavior, bullying, sleep, eating, sex, stress, psychosis	~7 min

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Chapter 17 — Ethics and Law

2027 task map: 1B.5 Assess your suitability to work with a specific client based on their needs and sociocultural considerations · 6.5 Adhere to the counseling codes of ethics · 4.9 Orient client to the counseling process · 2A.5 Review policies and procedures with client · 2A.1 Discuss and obtain informed consent/assent from client or guardian · 6.7 Engage in billing, third-party reimbursement and other insurance panel processes · 6.20 Market your counseling business appropriately (e.g., proper utilization of social media, proper protection of client information) · 6.4 Adhere to telemental health best practice guidelines · 4.15 Monitor and resolve therapeutic ruptures · 5.6 Review clinical documentation before client sessions · 6.14 Provide adequate accommodations for clients with disabilities · 5.2 Collaborate with non-clinical stakeholders (e.g., legal counselor, community resources) · 3B.1 Refer clients and/or family members to community resources or to specialists · 1A.5 Engage in advocacy for the client and the counseling profession · 1A.2 Seek supervision from a counseling supervisor · 5.8 Write clinical documentation during or after client sessions · 1B.10 Practice self-care · CACREP: Professional Counseling Orientation and Ethical Practice

How the NCE Tests Ethics

Legal & Ethical Compliance is tied for the largest domain on the NCE: 28 of the 140 scored items. Ethics also shows up inside other domains: informed consent in group work, confidentiality in couples work, the duty to protect in crisis items. Every ethics item has the same structure: a situation, plausible actions, and one that best follows the codes and the law.

Three habits answer most of them:

1. **Know the standard.** The ACA Code of Ethics (2014) is the main source. The NBCC Code of Ethics (2023) applies to National Certified Counselors and largely agrees with ACA.
2. **Consult.** When an option includes consultation or supervision, it is very often part of the right answer.
3. **Least harm, most client welfare.** Between two options, choose the one that protects the client and preserves autonomy, and document your reasoning.

Part 1 — Foundations

Ethical Principles

The ACA Code's preamble names six fundamental principles. Karen Kitchener (1984) described the first five as the foundation of ethical decision-making.

Principle	Meaning
Autonomy	Fostering the client's right to control the direction of their own life
Nonmaleficence	Avoiding actions that cause harm
Beneficence	Working for the good of the individual and society by promoting mental health and well-being
Justice	Treating individuals equitably and fostering fairness and equality
Fidelity	Honoring commitments and keeping promises, including fulfilling one's responsibilities of trust
Veracity	Dealing truthfully with individuals

Exam patterns: keeping a promised appointment time is **fidelity**. Being honest about a diagnosis is **veracity**. Letting a client make a choice you disagree with is

autonomy. Sliding-scale fees and equal access are **justice**.

Ethics vs. Law

Term	Meaning
Ethics	Standards set by the profession (ACA, NBCC). Violations can lead to loss of membership or certification
Law	Rules set by government (statutes, regulations, court decisions). Violations can lead to loss of license, lawsuits, or criminal penalties
Mandatory ethics	Following the minimum rules
Aspirational ethics	Striving for the highest standards of practice

When ethics and law conflict (ACA I.1.c): counselors make known their commitment to the ACA Code and take steps to resolve the conflict. If it can't be resolved that way, counselors may follow the law, regulations, or other governing legal authority.

Confidentiality, Privilege, and Privacy

Term	What it is	Who owns it
Privacy	A constitutional and legal right to decide what personal information is shared	The person
Confidentiality	An **ethical** (and often legal) duty of the counselor not to disclose client information	The counselor's duty to the client
Privileged communication	A **legal** right that protects client communications from being disclosed **in court**	**The client** holds the privilege and can waive it

***Jaffee v. Redmond* (1996):** the U.S. Supreme Court recognized psychotherapist–patient privilege in federal

courts, including for licensed clinical social workers. States set their own privilege laws for counselors.

Credentials

Term	Meaning
Licensure	State permission to practice (e.g., LPC, LMHC); the strongest form of credential; protects title and practice
Certification	A credential from a professional body (e.g., NCC from NBCC); voluntary at the national level
Accreditation	Approval of **programs** (e.g., CACREP accredits counseling programs), not individuals
Registry	A listing of practitioners who meet basic criteria

Part 2 — Informed Consent (2A.5, 6.7, 2A.1)

Informed consent is an **ongoing process**, not a one-time form (ACA A.2.a). It's provided in writing and verbally, in language the client can understand (A.2.c), and revisited as counseling changes.

What Clients Need to Know (ACA A.2.b)

- Purposes, goals, techniques, procedures, limitations, potential risks, and benefits of services
- The counselor's qualifications, credentials, and relevant experience
- Continuation of services if the counselor becomes incapacitated or dies
- The role of technology (Chapter 9)
- Implications of diagnosis and the intended use of tests and reports
- Fees and billing arrangements, including procedures for nonpayment
- **Confidentiality and its limits**

- Clients' rights to obtain information about their records and to participate in ongoing counseling plans
- The right to **refuse** any services or modality change and to be advised of the consequences of refusal

Special Situations

Situation	Rule
Minors	Parents or guardians generally consent; the minor gives **assent**. State laws vary on when minors can consent on their own (e.g., for substance use or some mental health care). Counselors respect parents' legal rights while protecting the minor's confidentiality as much as possible (ACA B.5.b)
Clients unable to give consent (A.2.d)	Seek the client's assent; include those legally able to consent on the client's behalf, such as guardians
Mandated clients (A.2.e)	Explain what information will be shared with the referral source (court, probation, employer) **before** counseling begins; clients can refuse services, with the consequences explained
Assessing capacity (2A.1)	Consider age, cognitive ability, intoxication, psychosis, and acute distress
Disabilities (6.14)	Provide reasonable accommodations (ADA), such as interpreters, accessible materials, and accessible space
Language	Use qualified interpreters (Chapter 9)

Part 3 — Confidentiality and Its Limits (2A. 1, 6.4, 5.2)

Exceptions (ACA B.2)

Exception	Standard
Serious and foreseeable harm (B.2.a)	Disclosure is permitted when needed to protect clients from serious and foreseeable harm, or when legal requirements demand it (duty to protect, Chapter 6)
Abuse reporting	Mandated by state law (Chapter 6)
End-of-life decisions (B.2.b)	Option to maintain confidentiality for terminally ill clients considering hastening death, after consultation (Chapter 12)
Contagious, life-threatening diseases (B.2.c)	Counselors **may** be justified in disclosing to an identifiable third party at serious and foreseeable risk of contracting the disease. **Before** disclosing, they assess whether the client intends to inform the third party or to engage in behavior that may harm that person, and they follow state law, which varies. (The 2005 Code also required confirming the diagnosis; the 2014 Code dropped that step)
Court-ordered disclosure (B. 2.d)	Seek the client's written consent or take steps to limit disclosure; disclose as little as possible
Minimal disclosure (B.2.e)	When disclosure is required, share only essential information
Client consent	The client can authorize disclosure in writing
Client's lawsuit	A client who sues the counselor generally waives confidentiality for the issues in the suit

Subpoenas and Court Orders

Document	What to do
Subpoena (often issued by an attorney)	**Don't ignore it, and don't simply hand over records.** Contact the client and/or the client's attorney, consult your own attorney or licensing board guidance, and **assert privilege** on the client's behalf if the client hasn't waived it
Court order (signed by a judge)	Comply, but disclose only what is required, and ask the court to limit disclosure if needed

Sharing Information with Others (ACA B.3)

- **Subordinates** (assistants, students, billing staff): counselors make sure they protect confidentiality.
- **Interdisciplinary teams**: inform the client of the team's existence and composition and what information is shared.
- **Third-party payers**: disclose only with client authorization.
- **Deceased clients**: protect confidentiality consistent with law and agency policy.
- **Consultation** (B.7): don't reveal identifying information unless the client consents or disclosure is unavoidable; share only what's needed.

Groups and Families (ACA B.4)

Counselors explain that confidentiality **can't be guaranteed** in groups (Chapter 8). In couple and family work, counselors clarify who the client is and the secrets policy (Chapter 7).

Federal Privacy Laws

Law	What it covers
HIPAA (Health Insurance Portability and Accountability Act, 1996)	Privacy and security of protected health information (PHI) for covered entities. Clients have a right to access and request amendments to their records (with limited exceptions). **Psychotherapy notes** (a counselor's separate process notes) receive extra protection and generally require specific authorization to disclose. **Minimum necessary** rule for most uses and disclosures (it doesn't apply to disclosures for treatment or those the client authorizes)
HITECH Act (2009)	Strengthened HIPAA enforcement; breach notification requirements for electronic records
42 CFR Part 2	Extra protection for federally assisted substance use disorder program records (Chapter 9)
FERPA (Family Educational Rights and Privacy Act)	Privacy of student education records; parents' access rights (Chapter 12)

Psychotherapy notes vs. the clinical record: the clinical record (dates, diagnosis, treatment plan, progress summaries, medications, test results) is part of the designated record set. Psychotherapy notes are kept **separate** and include the counselor's analysis of session content.

Part 4 — Boundaries and Relationships (4.9, 4.15)

Sexual and Romantic Relationships (ACA A.5)

Relationship	Rule
Current clients, their romantic partners, or their family members (A.5.a)	**Prohibited**, in person and electronically
Previous romantic or sexual partners (A.5.b)	Counselors don't take them on as clients
Former clients, their partners, or family members (A.5.c)	**Prohibited for 5 years** after the last professional contact. After 5 years, counselors must still show forethought and document that the relationship isn't exploitative; if exploitation is possible, they avoid it
Friends or family members (A. 5.d)	Counselors don't counsel them when they can't be objective
Personal virtual relationships with current clients (A.5.e)	**Prohibited** (e.g., social media friendships)
Supervisees and students (F. 3.b, F.10.a)	Sexual or romantic relationships are prohibited

Boundary Extensions (ACA A.6)

Not every nonprofessional contact is harmful. Attending a client's graduation, a hospital visit during illness, or purchasing a service from a client's business might be **boundary extensions**. The counselor:

- Considers the risks and benefits
- Documents the rationale **before** the interaction when feasible (A.6.b)
- Gets client consent where appropriate
- Takes steps to repair harm if it occurs

Boundary crossing vs. boundary violation: a **crossing** departs from the usual role and may be harmless or even helpful (a brief condolence card). A **violation** harms or exploits the client (a business partnership, a sexual relationship). The NCE rewards careful, documented judg-

ment, not rigid avoidance of all contact. In small or rural communities, some overlap is unavoidable; it's managed, not prohibited.

Gifts and Bartering

Topic	Rule
Bartering (A.10.e)	Acceptable **only** if the relationship isn't exploitative or harmful, doesn't give the counselor an unfair advantage, **the client requests it**, and it's an accepted practice in the community. The counselor considers cultural implications and documents the agreement in a written contract
Gifts (A.10.f)	Counselors consider the therapeutic relationship, the **monetary value**, the client's **motivation**, and the counselor's motivation. Small gifts may be accepted, especially where culturally meaningful

Fees and Business Practices (ACA A.10)

- **Self-referral** (A.10.a): counselors in agencies don't refer clients to their own private practices unless the agency allows it and clients are informed of other options.
- **No kickbacks** (A.10.b): no fees for referrals.
- **Establishing fees** (A.10.c): consider clients' financial status and locality; help find comparable, affordable services if fees are a hardship.
- **Nonpayment** (A.10.d): inform clients in advance if collection agencies or legal measures will be used, and give clients a chance to pay first.
- **Contributing to the public good** (C.6.e): counselors make a reasonable effort to provide services for little or no financial return (pro bono).

Part 5 — Competence, Supervision, and Self-Care (1B.5, 1A.2, 1B.10)

Competence (ACA C.2)

- Practice only within the boundaries of competence, based on education, training, supervised experience, credentials, and experience (C.2.a).
- New specialty areas require education, training, and supervised experience; take steps to ensure competence and protect clients while developing skills (C.2.b).
- Pursue continuing education (C.2.f).
- **Assess your competency for a specific client (1B.5)**: when a client's needs exceed your competence and supervision or training can't close the gap in time, **refer** (A.11.a). Don't refer for **values** reasons (A.11.b; Chapter 9).

Impairment (ACA C.2.g)

Counselors monitor themselves for signs of impairment from their own physical, mental, or emotional problems and **refrain from offering services** when impaired. They seek help, and if necessary limit, suspend, or terminate their professional responsibilities until it's safe to resume. Colleagues who notice impairment in another counselor consult with and assist that colleague (C.2.g, I.2).

Self-Care (1B.10)

Burnout, compassion fatigue, and vicarious trauma (Chapter 14) threaten competence. Self-care is an ethical responsibility: supervision, consultation, personal counseling, boundaries on workload, and attention to physical health and relationships.

Supervision and Consultation (1A.2)

Term	Meaning
Supervision	An ongoing, **evaluative**, hierarchical relationship; the supervisor shares responsibility for client care
Consultation	A **voluntary**, nonhierarchical request for advice; the consulting counselor keeps full responsibility
Vicarious liability (*respondeat superior*)	Supervisors can be held legally responsible for supervisees' actions
Direct liability	Supervisors are responsible for their own negligent supervision

Supervision model owners: Bernard's **discrimination model** (supervisor roles: teacher, counselor, consultant; foci: intervention, conceptualization, personalization); Stoltenberg and Delworth's **Integrated Developmental Model** (IDM; supervisees develop through levels of self- and other-awareness, motivation, and autonomy).

Part 6 — Records and Documentation (5.6, 5.8)

- Counselors create, safeguard, and maintain **records** needed to provide services (A.1.b), consistent with law and agency policy.
- Records are **accurate, timely, and sufficiently detailed**; they document services, risk assessments, consultations, and the rationale for clinical decisions. "If it isn't documented, it didn't happen."
- **Client access** (B.6.e): counselors provide reasonable access to records unless there is compelling evidence that access would cause harm; they help clients understand the records and document any limits on access.

- **Retention and disposal** (B.6.h): keep records for the period required by law (state laws vary, often around 7 years for adults, longer for minors), then dispose of them in a way that protects confidentiality.
- **Plan for incapacity or death** (C.2.h, B.6.i): counselors have a plan for transferring records, often through a designated records custodian or professional will.
- **Electronic records**: secure, encrypted, backed up (Chapter 9).
- **Correcting errors**: add a dated correction; never alter or destroy an original entry.

Part 7 — Termination, Referral, and Abandonment (3B.1)

Concept	Rule
Appropriate termination (A.11.c)	When services are no longer needed, the client isn't benefiting, the client is being harmed, or the counselor is in danger from the client or someone in the client's life. Counselors provide **pretermination counseling** and recommend other providers when needed
Termination for nonpayment	Permitted when the fee arrangement was clearly explained, but with pretermination counseling and referrals; never in the middle of a crisis
Abandonment (A.12)	Counselors **don't abandon or neglect clients**. They arrange for continuation of care during absences, illness, and after termination

Concept	Rule
Referral (A.11.a)	When lacking competence, refer to appropriate resources. If the client declines the referral, the counselor discontinues the relationship
Appropriate transfer (A.11.d)	Ensure appropriate clinical and administrative processes and open communication with the client and new provider

Part 8 — Advocacy and Professional Identity (1A.5)

- Counselors **advocate** at individual, group, institutional, and societal levels to address barriers clients face (A. 7.a), getting client consent before advocating on a specific client's behalf (A.7.b).
- **Advertising** (C.3): accurate, not misleading; counselors who use testimonials don't **solicit** them from current clients, former clients, or anyone else vulnerable to undue influence (C.3.b).
- **Credentials** (C.4): claim only credentials actually earned; use "Dr." only with a doctorate in counseling or a closely related field (C.4.d).
- **Nondiscrimination** (C.5) and **sexual harassment** prohibition (C.6.a).

Part 9 — Ethical Decision-Making

Forester-Miller and Davis (1996) — Seven Steps

1. Identify the problem
2. Apply the ACA Code of Ethics
3. Determine the nature and dimensions of the dilemma (principles involved, relevant literature, consultation)
4. Generate potential courses of action
5. Consider the potential consequences of all options and choose a course of action

6. Evaluate the selected course of action
7. Implement the course of action

Three Tests for the Chosen Action (Stadler)

Test	Question
Justice	Would I treat others in this situation the same way?
Publicity	Would I want my behavior reported in the press?
Universality	Would I recommend this course of action to another counselor in the same situation?

Resolving Ethical Violations by Colleagues (ACA I. 2)

1. If an informal resolution is appropriate, **first try to resolve it informally with the colleague** (I.2.a), as long as that doesn't violate confidentiality.
2. If informal resolution isn't appropriate or doesn't work, or the violation has caused or is likely to cause **substantial harm**, take further action: report to the state licensing board, ACA, or NBCC as appropriate (I. 2.b).
3. **Don't file unwarranted complaints** (I.2.e) intended to harm a counselor rather than protect clients.

Part 10 — Malpractice and Risk Management

A **malpractice** claim generally requires four elements, often remembered as the **four Ds**:

1. **Duty**: a professional relationship existed
2. **Dereliction** (breach): the counselor failed to meet the standard of care
3. **Direct causation**: the breach caused the harm
4. **Damages**: the client suffered harm

Common risk areas: sexual misconduct, failure to assess or respond to suicide risk, breaches of confidentiality, practicing outside competence, abandonment, and poor documentation.

Risk management: informed consent, documentation, consultation, practicing within competence, professional liability insurance, and following the code and law.

Part 11 — Artificial Intelligence and Emerging Technology

The 2014 ACA Code predates widespread AI, but its standards on technology (Section H), confidentiality (Section B), competence (C.2), and informed consent (A.2) apply directly. The ACA AI Work Group's recommendations (see Chapter 9) translate them into practice:

- **Informed consent:** tell clients when AI is used (e.g., an AI note-taker or scheduling assistant), what it does, and the risks; respect a client's refusal.
- **Responsibility:** the counselor is responsible for every clinical decision and every note, including AI-drafted ones. AI shouldn't be the sole basis for diagnosis, assessment, or risk decisions.
- **Confidentiality:** use only tools that meet HIPAA requirements (with a business associate agreement where required); never enter identifiable client information into consumer AI tools.
- **Bias and competence:** understand the tool's limits; watch for biased output.
- **Clients' AI use:** ask about it, and explore benefits and risks, including the lack of crisis response from chatbots.

On the exam, treat AI like any new technology: informed consent, confidentiality, competence, and the counselor's responsibility for clinical judgment.

Practice Management and Compliance

2027 task map: 6.1 Activate (i.e., follow) an emergency preparedness plan for your counseling practice in response to a relevant incident · 6.2 Adhere to internal policies or standard operating procedures (SOPs) relevant to your clinical practice (e.g., informed consent, record retention, release of information, mandated reporting protocols) · 6.3 Adhere to relevant state and federal laws and regulations that apply to your clinical practice · 6.4 Adhere to telemental health best practice guidelines · 6.5 Adhere to the counseling codes of ethics · 6.6 Engage in activities to maintain licensure status · 6.7 Engage in billing, third-party reimbursement and other insurance panel processes · 6.8 Engage in compliance-related training (e.g., insurance, credentialing, billing, payment) · 6.9 Ensure telemental health platforms are utilized in compliance with state and federal laws · 6.10 Ensure that reimbursement requests are supported by proper clinical documentation · 6.11 Implement technology-assisted clinical tools (e.g., online therapy platform, AI note takers, virtual reality tools, calming apps, wearable devices to monitor heart rate and breathing) · 6.12 Maintain professional liability insurance · 6.13 Practice in accordance with license portability (e.g., Counseling Compact) · 6.14 Provide adequate accommodations for clients with disabilities · 6.15 Review and update an emergency preparedness plan for your counseling practice · 6.16 Securely maintain case notes, records, and files · 6.17 Solicit best practices for understanding and adhering to obligations of court proceedings · 6.18 Use an electronic health record (EHR) or electronic medical record (EMR) system · 6.19 Use artificial intelligence (AI) within counseling practice · 6.20 Market your counseling business appropriately (e.g., proper utilization of social media, proper protection of client information)

The Practice Is Part of the Care

A claim billed under the wrong code, a chart left on a desk, a video link anyone can open, or a fire alarm treated

as a nuisance can hurt clients as surely as a poor clinical choice. The 2027 outlines list twenty tasks about running a lawful, safe practice.

These items share one test: **which rule owns this situation, and does the answer follow it rather than convenience, comfort, or a myth?** Wrong answers fall into five patterns:

1. **Convenience over safeguards.** A personal phone, a free app, a shared login, or an emailed draft.
2. **Consent used as a cure-all.** A client's signature cannot waive licensure law, turn a consumer app into a compliant one, or make a forensic opinion about one's own client appropriate.
3. **One rule mistaken for another.** A subpoena treated as a court order, an authorization treated as the privacy notice, or a building plan treated as the practice plan.
4. **Payment shaping the record.** A diagnosis chosen for coverage, a code chosen for the fee, or a note written after the claim.
5. **Improvising instead of following the plan.** The practice wrote a procedure for this incident, and the counselor ignores it.

This section builds on the ethics material and adds the practice-business rules the 2027 tasks name. For the professional will, see the continuity-of-care section.

Part 1 — Who Owns It

Owner	What it owns
Practice emergency preparedness plan	Incident procedures, client notification, continuity of care
Building evacuation plan	Exits and alarms; **not** clients, records, or continuity
HIPAA contingency plan, 45 CFR 164.308(a)(7)	Data backup, disaster recovery, emergency mode operation

Owner	What it owns
HIPAA Privacy Rule	Uses and disclosures of protected health information (PHI); clients' rights
45 CFR 164.506	**Treatment, payment, and health care operations (TPO)** without authorization
45 CFR 164.508	**Authorization** for uses and disclosures outside TPO; **164.508(a)(2)** psychotherapy notes; **164.508(a)(3)** marketing
45 CFR 164.502(b), 164.514(d)	**Minimum necessary**; role-based access
45 CFR 164.502(e), 164.504(e)	**Business associate agreement (BAA)**
45 CFR 164.514(b)	**De-identification** (safe harbor or expert determination)
45 CFR 164.520	**Notice of Privacy Practices (NPP)**; **164.520(c)** how it is provided and posted
45 CFR 164.522(a)	Right to request **restrictions**; **(a)(1)(vi)** mandatory restriction when the client pays in full out of pocket
45 CFR 164.522(b)	Right to request **confidential communications**
45 CFR 164.524	**Right of access** to the designated record set
45 CFR 164.526	**Right to amend**
45 CFR 164.528	**Accounting of disclosures**
45 CFR 164.530(c)	**Safeguards** for PHI in any form, including disposal
45 CFR 164.512(e)	Disclosures for **judicial and administrative proceedings** (court orders, subpoenas)
HIPAA Security Rule	Electronic PHI: **164.310** physical safeguards, **164.312(a)(1)** access control, **164.312(a)(2)(i)** unique user ID, **164.312(b)** audit controls, **164.312(e)** transmission security; training (164.308(a)(5))

Owner	What it owns
HIPAA Breach Notification Rule, 45 CFR 164.402, 164.404, 164.410	Presumption of breach (**2013 Omnibus Rule**, replacing "risk of harm"); client notice within 60 days; business associate's duty to notify the practice
HHS Office for Civil Rights (OCR) telehealth notice	Pandemic leniency for consumer video apps; **expired August 9, 2023**
FERPA	School education records; excluded from HIPAA
42 CFR Part 2	Federally assisted substance use disorder program records
State law	Licensure, retention, recording consent, reporting, privilege, stricter privacy rules
ACA Code of Ethics (ACA, 2014)	**A.1.b** records; **A.2.a–A.2.d** consent; **A.10.b–A.10.c** referral fees, fees; **A.11.c, A.12** termination, abandonment; **B. 1.c, B.2.d, B.2.e, B.3.e, B.6.b–B.6.h, B.7.b** confidentiality, court orders, records; **C.1, C.2.f, C.2.h** standards, CE, incapacitation plan; **C.3.a–C.4.b** advertising, credentials; **C.5** nondiscrimination; **C.6.b, C.6.e** third-party reports, pro bono; **D. 1.g** employer policies; **E.5.a, E. 13.c** diagnosis, no forensic evaluation of clients; **H.1.b–H. 6.d** distance counseling and social media; **I.1.a, I.1.c** ethics and law
NBCC Code of Ethics (NBCC, 2023)	Ethics for National Certified Counselors
ACA AI Work Group recommendations (ACA, n.d.)	AI practice guidance; the 2014 Code does **not** name AI
State licensure board	Renewal, CE, audits, reporting deadlines, supervision, complaints
NBCC national certification (NCC)	A **voluntary** credential, separate from the license

Owner	What it owns
Counseling Compact	**Privilege to practice** in member states based on an unencumbered home-state license
NPI Type 1 / Type 2; NPPES	Identifiers for individuals (Type 1) and organizations (Type 2), issued through NPPES
CAQH ProView	Shared payer credentialing profile
National Practitioner Data Bank (NPDB)	Malpractice payments and adverse actions
DEA registration	Controlled substances; not a counseling identifier
Medicare MHC enrollment (Consolidated Appropriations Act, 2023)	Direct Medicare billing by mental health counselors from January 1, 2024
Medicare rules	Mandatory claim submission, opt-out, “incident to,” locum tenens, Advance Beneficiary Notice (ABN), telehealth payment flexibilities
No Surprises Act, 45 CFR 149.610	**Good faith estimate** for uninsured and self-pay clients
CPT psychotherapy codes	90832, 90834, 90837 (time), 90839/90840 (crisis), 90846/90847 (family), 90785 (interactive complexity)
Fraud laws; OIG General Compliance Program Guidance (OIG, 2023)	False claims, copay waivers; compliance program elements; HIPAA training (164.530(b))
Professional liability policy types	Occurrence, claims-made, tail, prior acts; versus general, umbrella, property, cyber
Respondeat superior	Employer liability for an employee’s acts; the employee can still be named
ADA Title III, 28 CFR 36.301(c), 36.302, 36.303	No surcharges; reasonable modifications; auxiliary aids and companion-interpreter limits (36.303(c))
Beck (2021)	CBT homework: rehearsed, linked to targets, reviewed

Owner	What it owns
Automation bias (Parasuraman & Manzey, 2010)	Over-trusting machine output; the counselor stays accountable

Part 2 — Emergency Preparedness (6.1, 6.15)

Three Plans, Three Owners

The exam separates three plans that candidates blur together.

Plan	Owner	What it covers	What it does not cover
Practice emergency preparedness plan	The practice's written policy	Each incident type, roles, staying with the client, client notification, alternate site, continuity of care	Building systems
Building evacuation plan	Landlord or building manager	Exits, alarms, assembly points for the whole building	Clients, records, continuity of care
HIPAA contingency plan	45 CFR 164.308(a)(7)	**Data backup**, **disaster recovery**, and **emergency mode operation** so electronic records survive and stay protected	People's physical safety

A building plan never replaces the practice plan, and **insurance never replaces the contingency plan**; policies pay claims but restore no records.

Activating the Plan (6.1)

The incident activates the plan. The counselor's private judgment about the incident does not. An alarm, a warning, or an announcement is enough. Waiting for smoke, a funnel cloud, or proof of an intruder is the error.

Incident	Plan procedure	Wrong-procedure trap
Fire alarm	**Evacuate**: leave with the client by the posted route to the meeting point; document the disruption afterward	Finishing the session; securing charts first; sending the client out alone
Tornado or severe-weather warning	**Shelter in place**: move with the client to the designated shelter area; check in afterward	Sending the client to drive home; continuing because "most warnings pass"
Lockdown (reported armed intruder)	**Lock down**: lock the door, lights off, stay out of sight with the client, away from doors and windows, until the all-clear	Evacuating to the parking lot; stepping into the hall to "check"
Medical emergency (new chest pain spreading to the left arm, collapse, seizure)	**Call 911**, stay with the client, have staff meet responders	Grounding exercises; "see your doctor after session"; letting the client drive
Power or EHR outage	Confirm safety, then the plan's **downtime procedures** (secured paper forms, or reschedule with a set time)	Typing PHI into a personal phone app; no return date; leaving the client alone in the dark
Office closed (overnight fire, flood)	**Notify clients** through listed contact methods; continue **securely** from the backup site or compliant telehealth; document	Canceling indefinitely; waiting for clients to call; a free consumer app "because it's an emergency"

Error patterns: **normalcy bias** (a real warning treated as nothing), **verification delay** (personal confirmation before following an announced plan), and **diagnostic overshadowing** (new physical symptoms explained away by a known diagnosis; new chest pain in a client with panic or somatic symptoms is a medical emergency until proven otherwise).

Two rules settle most items: **people before paper**, and **stay with the client** unless the plan says otherwise. Emergencies do not suspend privacy law, so care after a disaster continues **securely**.

Reviewing and Updating the Plan (6.15)

A plan is reviewed **on a schedule and whenever conditions change.**

Triggers for review: an office move, a drill or incident that exposed a gap, new staff, a new EHR or service such as telehealth, new hazards, or changed contacts.

What a complete plan contains: - An **all-hazards** risk assessment and a procedure for each incident type, with roles named - A **communication plan**, with a client contact list kept securely and reachable off site - An **alternate site** and secure telehealth option - **Encrypted offsite backups** and a records-access procedure (the contingency plan) - A **professional will** naming a colleague or records custodian (**ACA C.2.h**). Next of kin gain no right to open client files (**B.1.c**). A consent-form line saying services may end is disclosure (**A.2.b**), not a plan. - Training and drills, so staff do not improvise

A plan that closes the practice until things are rebuilt is not a plan. Stopping services without arranging care risks **abandonment (A.12)**.

Part 3 — Standard Operating Procedures and Codes of Ethics (6.2, 6.5)

Why SOPs Matter

When an item says "the practice has a written protocol," the right answer almost always **follows it and documents each step.** Accepting a job implies agreement with the employer's general policies; counselors work to change poor policies rather than ignore them (**ACA D. 1.g**).

SOP	What it requires	Common wrong answer
Informed consent	Before a new procedure, review purpose, techniques, expected distress, alternatives, and the right to refuse; document consent (**A.2.a–A.2.b**). A competent adult consents for himself.	**Retroactive consent** ("once it's working"); the **evidence-base exemption myth** ("it's evidence-based, so no consent needed"); a partner signing for a capable adult
Record retention	Keep records for the period state law and policy require, then destroy them securely. When a client asks for destruction, explain the policy and safeguards.	Shredding on request (the **record-ownership myth**); rewriting notes to suit the client; keeping "fuller" notes at home
Release of information (ROI)	A written release naming the recipient, the information, the purpose, and an expiration (**B.6.g**; HIPAA **164.508** where an authorization is required); send only what is needed (**B. 2.e**)	Acting on a verbal okay; sending the whole chart; refusing to coordinate with medical providers at all

SOP	What it requires	Common wrong answer
Mandated reporting protocol	Who reports, how fast, to which agency, how it is documented, and when to consult. State law sets the duty and threshold.	Delaying while the client "processes"; promising confidentiality the counselor cannot keep
At-risk no-show protocol	Timely outreach to a client with recent suicidal ideation through **consented** contact methods, escalating per the protocol's steps; document each step	Waiting for the client to call ("respecting autonomy"); calling the workplace; jumping straight to a welfare check
Between-session contact policy	Response limits and secure channels. Repeated reassurance-seeking messages are reviewed against the policy and brought into session as clinical material.	Detailed reassurance replies, which feed the anxiety cycle; going silent without explanation; giving out a personal cell number
Recording policy	Written, informed permission before any recording, covering purpose, who will hear it, storage, and the right to decline (**B. 6.c**); state law may add consent rules	Covert recording "just for consultation"; a verbal okay where policy requires written consent; emailing files to a consultant

Codes of Ethics (6.5)

Counselors are bound by the **ACA Code of Ethics (ACA, 2014)**. National Certified Counselors are also bound by the **NBCC Code of Ethics (NBCC, 2023)**. Most state boards adopt a code by rule, so an ethics violation can become a licensure violation.

- **C.1:** counselors read, understand, and follow the ACA Code and applicable law.

- **I.1.a:** not knowing or misunderstanding an ethical responsibility is **not a defense**.
- **I.1.c:** when ethics and law conflict, counselors make their commitment to the Code known and try to resolve the conflict. If they cannot, they may follow the law.

Part 4 — Federal and State Privacy Law (6.3)

HIPAA in One Table

HIPAA applies to **covered entities**, including providers who send standard electronic transactions such as claims. A cash-only counselor may fall outside HIPAA but is still bound by state law and ethics; exam items generally assume HIPAA applies.

Rule	What it governs	Key sections
Privacy Rule	PHI in **any form**; client rights	164.502–164.528, 164.530
Security Rule	**Electronic** PHI: administrative, physical, and technical safeguards	164.308, 164.310, 164.312
Breach Notification Rule	What counts as a breach; who must be notified, and when	164.400–164.414 (164.402, 164.404, 164.410)

HIPAA is a **floor**; a stricter state privacy law still applies.

Uses, Disclosures, and the Special Protection for Psychotherapy Notes

Concept	Rule
TPO (164.506)	Treatment, payment, and health care operations need no authorization. Ethics and many state laws still expect client consent, so practices usually get a release anyway.

Concept	Rule
Authorization (164.508)	Required for uses and disclosures **outside** TPO and outside other permitted purposes. An authorization is **not** the privacy notice, and it is not the form for choosing how the client is contacted.
Minimum necessary (164.502(b), 164.514(d))	Use, disclose, and request only what the purpose needs; staff access records only as their role requires. It does **not** apply to disclosures for treatment, to the client, or under an authorization.
Psychotherapy notes (164.508(a)(2))	The counselor's separately kept notes analyzing session content. They need a **specific authorization** for most disclosures, **even to a health plan for payment**. A plan may **not** condition payment or eligibility on the client signing it.

What psychotherapy notes are not: medications, session start and stop times, modalities and frequency, test results, and summaries of diagnosis, functional status, plan, symptoms, prognosis, and progress. Those belong to the regular record. Minimum necessary narrows a permitted disclosure; **it never substitutes for the authorization psychotherapy notes require**, and neither does a third party's confidentiality pledge.

Client Rights Under HIPAA

Right	Section	What the counselor does	Trap
Notice of Privacy Practices	164.520	Give the NPP no later than the first service; make a **good-faith effort** to obtain written acknowledgment; also post it in the office and on any practice website. If one was missed, give it now and document it.	Relying on the waiting-room posting (**164.520(c)** requires both); treating an authorization as the NPP; **withholding care** until the acknowledgment is signed
Access	164.524	Provide access to the **designated record set** in a timely way, **currently within 30 days with one 30-day extension**, or sooner if state law requires. Separately kept **psychotherapy notes are excluded.** A summary may replace the record **only if the client agrees in advance.**	Requiring a subpoena; requiring a reason; including psychotherapy notes; substituting a summary on the counselor's own initiative

Right	Section	What the counselor does	Trap
Amendment	164.526	Respond **in writing**, generally within **60 days**. Amendment of **accurate and complete** information may be denied, with an explanation and the client's right to file a **statement of disagreement**.	Deleting an accurate diagnosis; ignoring the request; referring the client to the licensing board, which handles complaints, not amendments
Accounting of disclosures	164.528	List certain disclosures to others, generally within **60 days**. The list **excludes** TPO disclosures, disclosures to the client, and disclosures the client authorized. The **first accounting in any 12 months is free.**	Treating it as an access request; refusing because logs are "internal"; charging for the first one
Restrictions	164.522(a)	Clients may ask to limit uses and disclosures; the provider usually need not agree. **Exception (164.522(a)(1)(vi)):** the provider **must** agree not to disclose to a health plan for services the client **paid for in full out of pocket.**	Billing the plan anyway (the **mandatory-submission myth**); agreeing to self-pay and then submitting claims

Right	Section	What the counselor does	Trap
Confidential communications	164.522(b)	**Providers** must accommodate reasonable requests to be contacted by other means or at other locations **without asking why. Health plans** must accommodate such a request when the client says disclosure could **endanger** them.	Requiring a reason from the client; applying the plan's endangerment standard to a provider; stopping all electronic contact

The audit log a system keeps for security (**164.312(b)**) is not the accounting of disclosures. Each is a different duty.

Business Associates and De-Identification

A **business associate** creates, receives, stores, or transmits PHI for the practice: a telehealth platform, EHR vendor, billing service, cloud backup, or AI note-taker. It needs a **business associate agreement (164.502(e), 164.504(e))** before handling PHI. A BAA sets the vendor's duties. **It does not configure the practice's settings, and it does not replace client consent** to recording or AI processing.

De-identified information is no longer PHI. HIPAA recognizes two methods (**164.514(b)**): the **safe harbor** method, which removes specified identifiers with no actual knowledge that the rest could identify the person, and **expert determination**. Removing a last name is not de-identification; details, dates, and rare facts identify people. In consultation, counselors withhold identifying information unless the client consents or disclosure cannot be avoided (**ACA B.7.b**).

Breach Notification

Step	Rule
Presumption (164.402)	An impermissible use or disclosure is **presumed a breach** unless a documented risk assessment shows a **low probability that the PHI was compromised**. The assessment weighs the nature and extent of the PHI, who received it, whether it was actually viewed or acquired, and how far the risk was mitigated.
History	The **2013 Omnibus Rule** replaced the "risk of harm" standard; proof of misuse is **not** required.
Clients (164.404)	Notice **without unreasonable delay** and **no later than 60 days** after discovery
Business associates (164.410)	The vendor must notify the practice. **The practice remains responsible** for making sure affected clients are notified.
Others	HHS must also be notified, and media notice applies to large breaches. State breach laws may add duties.
Encryption	PHI encrypted to HHS standards is not "unsecured," so losing an encrypted device generally does not trigger notice.

FERPA, HIPAA, and 42 CFR Part 2

Law	Applies to	Exam point
FERPA	**Education records** at schools receiving federal education funds, including a school-employed K–12 counselor's records	**Excluded from HIPAA**. Parents generally have access rights. Private memory-aid notes kept solely by the counselor are treated differently.

Law	Applies to	Exam point
HIPAA	Covered providers, including a community counselor who sees a student	A school referral does **not** make a private counselor's records FERPA records.
42 CFR Part 2	Records of **federally assisted substance use disorder programs**	Stricter than HIPAA; generally requires patient consent and limits use of records in proceedings against the patient. A 2024 rule aligned parts of Part 2 with HIPAA; verify current requirements.

Part 5 — Telemental Health (6.4, 6.9)

Before the First Video Session (6.4)

1. **Suitability.** Can this client use the technology, and is distance counseling appropriate for their needs (**ACA H.4.c**)? A diagnosis alone, such as a substance use disorder, does not rule telehealth out. High acute risk or no private space may rule it out for now.
2. **Jurisdiction. ACA H.1.b**: distance counselors may be subject to the laws of both their own location and the client's place of residence. Boards generally regulate telehealth **where the client is located during the session**, so the counselor needs authorization there.
3. **Distance-counseling informed consent (H.2.a).** Cover risks and benefits, what happens if the technology fails, response times, emergency procedures, privacy limits, and possible insurance differences. **The intake consent does not automatically cover video.**
4. **Identity (H.3).** Verify who is on the screen, at the start and throughout.
5. **Emergency plan.** The client's usual location, a local emergency contact, and **crisis resources local to the**

client. Your own city's 911 cannot reach a client three states away.

6. **Disconnection plan.** If the call drops, call the backup number; if a high-risk client cannot be reached, follow the safety protocol.

Intimate partner violence changes the setup. Before going to video, confirm a private space and a **device the partner cannot access**, agree on a code word and interruption plan, and use only safe channels for links. In-person sessions may be safer.

At the Start of Every Session

- **Confirm the current physical location** and privacy. Clients travel.
- A **parked**, private car can work; a client may **not** join while driving.
- Keep cameras on; camera-off sessions lose risk-relevant observation.

Licensure Follows the Client

A client's consent **cannot waive** another state's licensure law, a home-state license does not travel with the platform, and audio-only sessions are not exempt. A client away for the summer means the counselor needs a license, temporary permit, or Compact privilege there.

Platform Compliance (6.9)

Requirement	Owner	Exam point
BAA with the vendor	164.502(e), 164.504(e)	The platform is a business associate.

Requirement	Owner	Exam point
Encryption in transit	164.312(e) transmission security; **ACA H.2.d**	Live video is PHI in transit even if nothing is recorded. **Encryption alone is not compliance**, because a BAA is still required.
Access controls	164.312(a)(1)	**Unique session links or a locked waiting room** so one client cannot walk into another's session. A reusable public link fails, whatever the BAA says.
No pandemic exception	HHS OCR	Consumer-app leniency **ended August 9, 2023.**
Medicare telehealth flexibilities	Medicare payment rules	These concern **coverage and payment**, not HIPAA platform compliance. They have been extended repeatedly; verify their current status.
Recording	**ACA B.6.c**; state recording laws	The **client's** informed consent (or a legal representative's if the client lacks capacity), plus any state-required consent; secure recordings as records. A caregiver without legal authority cannot consent, and a compliant platform does not supply consent.

When the compliant platform fails, **troubleshoot it or reschedule.** A parent's consent, a client's preference, or

a signed waiver does not make a consumer app compliant. HIPAA has no looser rule for minors.

Part 6 — Licensure, Certification, and Portability (6.6, 6.13)

Keeping the License Active (6.6)

The state board owns the license. Rules vary by state, so the right answer is usually "check and follow the board's rules."

Duty	What to know
Renewal and CE	The board sets required hours, content areas such as ethics, and **approved providers**. CE that counts for NCC renewal may not count for the license. Courses labeled "for counselors" may not meet board rules. Ongoing CE is also an ethical duty (**ACA C.2.f**).
CE audits	Renewal usually involves **attesting** to completion. An audit asks for **proof**. Keep certificates for as long as board rules require, not just until the renewal is approved.
Change of information	Report a name or address change within the board's deadline, which is often shorter than the renewal cycle. Updating an NPI record in NPPES does **not** notify the board.
Self-reporting	Many boards require reporting arrests, charges, or convictions within a set period. Some require reporting **charges**, not just convictions. The duty runs to the board, not to clients.

Duty	What to know
Associates and supervision	Associates practice only under a **board-approved supervisor**. When a supervisor leaves, register a replacement under board rules before continuing. Peer consultation with a fellow associate is not supervision, and hours without approved supervision may not count.
Lapsed license	**Stop practicing** until the board reinstates the license; otherwise it is unlicensed practice, and claims may be improper. A colleague's cosignature confers nothing. Any grace period depends on the state.

Licensure versus certification. The license is state permission to practice; the **NCC** is a **voluntary** national credential. Holding one never satisfies the other.

License Portability (6.13)

Route	How it works
Traditional multistate licensure	A separate full license in each state, often by endorsement. This route remains available everywhere.
Temporary or limited permits	Some states allow short-term practice for clients temporarily located there. Rules vary by state.
Counseling Compact privilege to practice	Practice in **member states** under a privilege based on the home-state license

Counseling Compact requirements (Counseling Compact Commission, n.d.): - The **home state** is the counselor's **primary state of residence**. - The counselor holds an **active, unencumbered home-state license** and has had **no encumbrance on any license in the prior two years**. - The counselor meets the remote state's notice, fee, and **jurisprudence** requirements. -

Member states require a national examination for licensure. The Compact does **not** require the NCC. - The counselor practices under the laws of the **remote state where the client is located.**

Situation	Rule
Client moves to a **non-member** state	The privilege does not reach that state. Meet its own licensure or authorization rules. There is no general continuity-of-care exception; any temporary allowance comes from that state's law.
Counselor **moves** between member states	Apply for a license in the **new home state** under Compact rules. The former home-state license converts to a privilege once the new one is active. A privilege cannot serve as the license where the counselor lives.

Verify the current status. The Compact has been enacted in many states, and privileges are being issued state by state as member states go live. Before relying on a privilege, check that both states are active members issuing or accepting privileges.

Part 7 — Billing, Reimbursement, and Compliance (6.7, 6.8, 6.10)

Identifiers and Enrollment

Owner	What it is	Exam point
NPI Type 1	Individual provider identifier, issued once per person for life	A counselor has **one** Type 1 NPI.

Owner	What it is	Exam point
NPI Type 2	Organizational identifier	A PLLC or group that bills as an entity gets a Type 2. The counselor stays the **rendering provider** under her Type 1.
NPPES	The federal system that issues and maintains NPIs	It is not a credentialing application, and it does not update the licensing board.
CAQH ProView	The shared credentialing profile many commercial payers pull from	Providers **re-attest** periodically (currently every 120 days).
NPDB	Federal data bank of malpractice payments and adverse licensure and privileging actions	Payers and hospitals query it; providers do not build profiles there.
Medicare MHC enrollment	Under the **Consolidated Appropriations Act, 2023**, eligible **mental health counselors** may enroll and bill Medicare directly **as of January 1, 2024**	Not “incident to” billing under a physician
Medicare mandatory claim submission	Enrolled providers submit claims for covered services. A provider eligible for Medicare must **enroll or formally opt out** before privately contracting with beneficiaries.	Billing a beneficiary privately without enrolling or opting out is the error.
“Incident to”	A narrow Medicare rule for services under a supervising practitioner, with specific supervision conditions	It is not a workaround for pending commercial credentialing.

Owner	What it is	Exam point
Locum tenens (substitute) billing	A substitute fills in for an **absent** enrolled provider for a limited time	It does not cover a new clinician waiting to be credentialed.

Billing under a colleague's NPI while your own panel application is pending is a false claim: it misrepresents who rendered the service, whatever the reason.

Before the First Claim (6.7)

- **Verify eligibility and benefits** first: active coverage, mental health benefits, cost-sharing, and any **prior authorization**. Recheck after a job loss or plan change. **Billing a terminated plan is a false claim.**
- **Patient vs. subscriber.** A child on a parent's plan is the **patient**; the parent is the **subscriber**.
- **Network status.** Tell an out-of-network client the likely costs and in-network options. A **superbill** is an optional statement for the client's own reimbursement claim.
- **Good faith estimate (No Surprises Act, 45 CFR 149.610).** Give **uninsured and self-pay** clients a written estimate of expected charges when services are scheduled or on request.
- **Advance Beneficiary Notice (ABN).** Given to **Original Medicare** beneficiaries before a service Medicare may not cover. It is not for self-pay clients generally.
- **Self-pay to keep treatment from the plan.** Honor it, document the restriction, submit **no** claim (**164.522(a)(1)(vi)**).
- **Explanation of benefits (EOB).** EOBs name the provider and service and go to the **policyholder**. A client on an abusive partner's plan can ask the plan to send communications elsewhere; the plan must honor it if she says disclosure could endanger her

(**164.522(b)**). The policyholder's permission is never needed.

- **Fees.** Consider clients' finances; adjust fees for hardship when legally permissible or help find affordable care (**A.10.c**). Pausing care when coverage lapses, especially in a crisis, skips the pretermination counseling **A.11.c** requires.
- **No-shows. Never bill a payer for a missed session**, at any code. Charge a no-show fee only as the signed policy and payer contract allow. A probation office is not a payer without an agreement.

The Codes (6.10)

Code	Service	Rule
90832	Psychotherapy, 30 minutes	**16–37** minutes
90834	Psychotherapy, 45 minutes	**38–52** minutes
90837	Psychotherapy, 60 minutes	**53 or more** minutes
90839	Psychotherapy for crisis, first 60 minutes	**30–74** minutes of urgent assessment and crisis intervention
90840	Crisis add-on, each additional 30 minutes	Used with 90839 once the crisis service reaches **75 minutes** or more. A 90-minute crisis session is 90839 + 90840.
90846	Family psychotherapy **without the patient present**	Billed for the **identified patient's** treatment. The family member need not be a client.
90847	Family psychotherapy **with the patient present** (conjoint)	The identified patient is the client.

Code	Service	Rule
90785	**Interactive complexity** add-on	Requires a **documented** complicating factor in **that** session. It attaches to diagnostic evaluation and to individual and group psychotherapy. It is **not** reported with crisis codes 90839/90840.

Interactive complexity factors (any one, documented): maladaptive communication among participants, such as heated disagreement; a caregiver's emotions or behavior interfering with the plan; a sentinel event requiring a mandated report, discussed in session; or play equipment, devices, or an interpreter for a client lacking expressive or receptive language. **Age alone is not a factor**, and neither is a language difference alone.

On U.S. claims, diagnoses are reported with **ICD-10-CM** codes.

Documentation That Supports a Claim

The record supports the claim. The claim does not support itself.

Claim	What the record must show
Any psychotherapy code	**Medical necessity**: notes linking **diagnosis**, current **functional impairment**, and **plan goals** to **each session's interventions** and the **client's response**
Time-based and crisis codes	**Actual start and stop times**; for crisis codes, the urgent **risk assessment** and **interventions**. A calendar block shows a reserved slot, not a delivered service.

Claim	What the record must show
90846	**Who attended, why the patient was absent**, and **how the session served the patient's plan**. A diagnosis for the family member would make that person the patient.
90785	The specific complicating factor and how it was managed

Errors the exam punishes: a diagnosis code standing in for necessity (**diagnosis-as-justification**); attendance-only notes; verbatim quotes sent to payers, which expose more than needed and belong, if anywhere, in psychotherapy notes; **cloned** notes; and notes written after the claim or only when audited.

Fraud and Abuse Patterns (6.8)

Pattern	Why it is wrong
Upcoding	Billing 90837 for 45 minutes, or a higher code because the diagnosis seems "intensive." Complexity does not add minutes.
Undercoding	Billing 90832 for 45 minutes "to be safe." Inaccurate in either direction is inaccurate.
Diagnosis chosen for coverage	Bipolar II for more sessions, anxiety to hide an alcohol diagnosis, or a vague unspecified code. Violates **ACA C.6.b** and **E. 5.a**; it is insurance fraud. Noting "doubts" or the client's preference does not cure it.
Billing for services not rendered	No-shows, sessions after coverage ended, sessions by someone else
Misidentifying the rendering provider	Using a colleague's NPI

Pattern	Why it is wrong
Routine copay waivers	Misstates the true fee and can violate payer contracts and fraud laws. A documented case-by-case hardship adjustment (**A.10.c**) differs. Pro bono (**C.6.e**) is free service, not billing an insurer while waiving the client's share.
Fee splitting and referral payments	**ACA A.10.b**: no fee splitting, and no commissions, rebates, or other payment for referring clients

Compliance Training

HIPAA requires workforce privacy training (164.530(b)) and security awareness training (164.308(a)(5)). The **OIG General Compliance Program Guidance (OIG, 2023)** lists compliance program elements: written policies, a compliance officer, training, open reporting lines, auditing and monitoring, consistent enforcement, and prompt correction. For a solo counselor, that means written billing procedures, periodic self-audits of notes against claims, and refunding overpayments.

Part 8 — Professional Liability Insurance (6.12)

Coverage	What it covers	Exam point
Occurrence policy	Incidents that **occur** during the policy period, whenever the claim is filed	No tail needed when it ends
Claims-made policy	Claims **made and reported** while the policy is active, for incidents after the retroactive date	When it ends (retirement, switching carriers), the counselor needs **tail coverage**
Tail (extended reporting endorsement)	Extends the time to report claims after a claims-made policy ends	The answer for a retiring counselor with a claims-made policy

Coverage	What it covers	Exam point
Prior acts ("nose") coverage	A new claims-made carrier covers incidents before the new policy began	The alternative to buying a tail when switching carriers
General (premises) liability	Slip-and-fall and other injuries at the office	Not professional negligence
Personal umbrella	Extra limits over personal policies	Generally **excludes professional services**
Property insurance	Damage to the office and equipment	It does not restore records or access to them.
Cyber liability	Costs of data breaches	A separate policy from professional liability

Employer coverage is not enough. An agency policy protects the agency first. It has its own limits and exclusions and may not cover off-duty work or **licensing board defense**. Under **respondeat superior** the employer can be liable for an employee's acts, but the counselor can still be named individually. A personal policy follows the counselor across jobs.

When a complaint or claim arrives, first **notify the carrier promptly**, as the policy's **notice-of-claim condition** requires; many policies include board defense and legal consultation. Do **not** call the complaining client (it can look retaliatory) or send the board a rushed, unadvised response.

Part 9 — Secure Records and EHR Use (6.16, 6.18)

Paper and Electronic Records (6.16)

ACA B.6.b requires secure records with access limited to authorized persons. HIPAA requires safeguards for PHI in **any form** (**164.530(c)**); for electronic PHI the Security Rule adds physical safeguards (**164.310**), including

device and media controls, and technical safeguards (**164.312**).

Situation	Secure practice	Error
Client brings **original documents** (school records, rating scales) that inform a diagnosis	Add **copies** to the secure record, return the originals, and limit access to those with a need	Desk tray in open view (fails **164.310** physical safeguards); shredding documents the diagnosis relied on (**A.1.b**)
Client-made **media** (phone videos of symptoms)	Upload through the practice's **secure portal** and keep in the record, limited to the care team	Texts to a personal phone; unlisted video links; a shared office drive everyone can open
Finishing notes **at home**	Work **only through the practice's encrypted remote access**, leaving no local copies	Emailing a draft to a personal account (**B. 3.e**); saving to a home computer and "deleting later"; carrying paper charts home
Retention period has **ended**	**Destroy** so the record cannot be read or reconstructed: shred, burn, or pulp paper; sanitize or physically destroy electronic media (**B.6.h**)	Office recycling bin (fails **164.530(c)**); mailing an unrequested chart to an old address

Retention periods come from state law and payer rules, not HIPAA. HIPAA requires keeping its own compliance documentation, such as policies and signed authorizations, for six years, but sets no clinical-record period. Minors' records are often kept longer. Check your state.

Using the EHR (6.18)

Safeguard	Owner	Meaning
Unique user ID	164.312(a)(2)(i)	Every user has their own login, so every action is traceable. **Never share credentials**, with a colleague or front-desk staff.
Role-based access	164.502(b), 164.514(d)	System access is not authorization. Staff open a chart only as their role in that client's care or operations requires.
Audit controls	164.312(b)	Logs of who viewed and changed what; snooping shows up and calls for sanctions and a breach risk assessment.
Access control	164.312(a)(1)	Automatic logoff, encryption, and emergency access procedures

EHR documentation rules: - **Correct by amendment.** Fix a wrong date in a signed note with a **dated, signed amendment that leaves the original visible** (**A.1.b**). Deleting and re-creating the note, or a hidden vendor edit, destroys the audit trail; leaving the error is also wrong. - **Edit copy-forward text** so the note records **only this session** (**A.1.b**; **C.6.b**). Signing cloned text, or "sign now, fix next week," attests to care that did not occur. - **No shadow records or convenience printouts.** Side files in a drawer, notes at home, and pages left in a mailbox sit outside the record's safeguards.

Part 10 — Accommodating Clients With Disabilities (6.14)

A counselor's office is a **public accommodation** under **ADA Title III** (government agencies fall under Title II, and recipients of federal funds under Section 504 of the Rehabilitation Act). **Psychiatric disabilities are covered.** Referring a client because of a disability rather than clinical need is discrimination (**ACA C.5**).

Rule	Owner	Application
No surcharges	**28 CFR 36.301(c)**	The client never pays for an accommodation: no fee for reading forms aloud, captioning, extra time, or an after-hours slot.
Reasonable modifications	28 CFR 36.302	Change policies and practices when needed, such as a **quieter waiting area** for a client whose voices worsen in a busy room or an accessible ground-floor room. Do not demand needless proof of a known need before making a simple change.
Auxiliary aids and effective communication	**28 CFR 36.303**	Provide aids such as **qualified interpreters**, captioning, and accessible formats at no cost, subject only to the narrow **undue burden** and fundamental-alteration limits. **Ask the client** what works.

Rule	Owner	Application
Companion interpreters	**28 CFR 36.303(c)**	A practice may **not require** a client to bring their own interpreter and may rely on an accompanying adult only in narrow circumstances. Family interpreting also compromises confidentiality.
Written notes	36.303	Notes may work for brief, simple exchanges. They are **not** effective communication for counseling.
Informed consent	**ACA A.2.c**	Consent must be communicated in a way the client can understand, which may mean accessible formats.
Online access	**ACA H.5.d**	Websites and online services are accessible to people with disabilities.

Accommodate rather than remove. A client with dyslexia gets audio or app-based self-monitoring logs with verbal review, not a plan with self-monitoring dropped. A client who now uses a wheelchair is seen in an accessible room, not switched to phone-only care, paused, or carried up the stairs.

Part 11 — Court Proceedings (6.17)

Beyond the ethics material on privilege and subpoenas, the practice tasks add three layers: **legal advice first, HIPAA's conditions, and the treating role.**

Step 1 — Get advice and notify the client. Consult an attorney or the liability carrier's risk-management service and **notify the client**. Do not simply send re-

cords to meet the attorney's deadline, and do not ignore the demand: privilege must be **asserted**, and silence risks sanctions.

Step 2 — Apply HIPAA 164.512(e).

Demand	What HIPAA permits
Court order (signed by a judge)	Disclose **only what the order expressly authorizes**
Subpoena or discovery request without a court order	Disclose only with the client's authorization, or with **satisfactory assurances** that the client was notified and could object, or that a **qualified protective order** was sought. Stricter **state privilege law** may require more, often a court order.

Step 3 — Apply the ethics standards. - **ACA B.2.d:** when ordered to disclose without the client's permission, seek the client's written consent, or ask that disclosure be prohibited or **limited as narrowly as possible.** - **ACA B.2.e:** release **only what is required**. Sending the whole file "to show cooperation" over-discloses. - Refusing a valid court order risks **contempt**. Any privilege claim is raised **with the court**, which decides it. A minor's own refusal does not override a court order.

The treating role. Counselors **do not evaluate current or former clients for forensic purposes (ACA E.13.c).** A treating counselor does not opine that a client is the better parent, poses no danger, or is sane; those opinions need an objective evaluator. **A release permits disclosure; it does not make the opinion appropriate.** If compelled, the counselor testifies as a **fact witness** about treatment. An unsolicited letter to the judge volunteers PHI and blurs roles.

Part 12 — Technology-Assisted Tools and AI (6.11, 6.19)

Technology-Assisted Clinical Tools (6.11)

Apps, wearables, virtual reality, and online platforms follow the same sequence: **vet the tool, get informed consent, build it into the plan, and review it.**

1. **Vet privacy.** Popularity says nothing about how an app stores or sells data, and many consumer apps fall outside HIPAA.
2. **Informed consent (H.2.a; H.4.a).** Explain **benefits and limitations**, data handling, and what the counselor will and will not do with the data.
3. **Fit (H.4.c).** Confirm the client can use the tool and that it suits their needs.
4. **Integrate.** As with CBT homework (**Beck, 2021**), **rehearse** in session, **link** to triggers or targets, and **review** use each session. A bare download link is rarely used well.
5. **Supplement, never substitute.** A calming app does not replace sessions.

Device data. Consumer wearables show **trends**, not validated diagnoses; heart-rate or sleep scores supplement assessment but do not diagnose panic disorder or mark hypomania. Client-shared data may be used in treatment and, once documented, is PHI.

Monitoring expectations. If an app shares entries, clarify **when** the counselor reviews them, that they are **not monitored live**, and where to turn in a crisis. Never hold a client's app password; it creates security risks and an unstated duty to monitor.

Consent and capacity. A tracking wearable for a client with memory loss is discussed **with the client**, seeking consent or **assent** (**ACA A.2.d**), reviewing data privacy, and tying it to the safety plan. **Covert monitoring** is paternalism; refusing all tracking is too rigid; one device

never replaces the safety plan. A parent-training reward app is likewise set up **in session** around positive targets and reviewed, not left to the child or turned into a misbehavior log.

Artificial Intelligence (6.19)

The 2014 ACA Code does not address AI by name, so "the ACA Code prohibits AI" is an invented rule. The existing standards decide AI items: informed consent (**A.2**), confidentiality (**B**), competence (**C.2**), technology (**H**, including **H.4.a** benefits and limitations), and accurate records (**A.1.b**). The **ACA AI Work Group recommendations (ACA, n.d.)** apply them to practice. Some states have passed laws limiting AI-delivered therapy, so check your state.

AI use	Before	During and after
AI note-taker (records sessions, drafts notes)	A **BAA** with the vendor (164.502(e), 164.504(e)); the client's **informed consent** to recording and AI processing. The NPP is not consent.	The counselor **reviews, corrects, and signs**, and remains accountable.
AI drafts (treatment plan summaries, accommodation suggestions)	Use only practice-approved, compliant tools	**Verify each item** against the assessment and record before it enters care or goes to a school team. Disclose AI's role; an "AI-generated" label does **not** shift liability.
Consumer chatbots	**Never** enter client details. Removing a last name is not de-identification.	—

AI use	Before	During and after
Client asks about a mental health chatbot	Help weigh its **evidence, privacy, and crisis limits**	Frame it as a **supplement** reviewed in session; chatbots are not licensed or bound by ethics codes. Keep human contacts and 988 in the safety plan.

Automation bias is trusting output because a machine produced it (Parasuraman & Manzey, 2010): signing AI drafts unread, letting app scores decide stability, or letting a chatbot set a sleep window. **The counselor answers for the clinical content.**

Part 13 — Marketing the Practice (6.20)

Owner	Rule	Application
ACA C.3.a Accurate Advertising	Advertising is accurate, not false, misleading, deceptive, or fraudulent	Specialties claimed only with real training; outcomes not promised
ACA C.3.b Testimonials	**Do not solicit** testimonials from current or former clients or anyone vulnerable to undue influence; discuss implications and get permission for any used	Do not ask satisfied clients to post reviews.
ACA C.3.c Statements by Others	Make reasonable efforts to keep others' statements about you accurate	Correct a directory listing that overstates credentials.
ACA C.4.a Accurate Representation; **C. 4.b** Credentials	Claim only qualifications actually completed, and only licenses and certifications that are **current and in good standing**	No "certified" for a certification still in progress; no lapsed credentials

Owner	Rule	Application
ACA H.6.a Virtual Professional Presence	**Separate professional and personal** profiles	A practice page, not a personal account, for marketing
ACA H.6.c Client Virtual Presence	Do not view clients' social media without consent	No searching clients online
ACA H.6.d Use of Public Social Media	Take precautions not to disclose confidential information	No case stories that could identify a client
ACA B.1.c; HIPAA	The **fact that someone is a client is protected**	**Do not reply publicly to reviews**, even with thanks; any reply confirms the relationship. At most, post a general policy statement that confirms nothing.
HIPAA 164.508(a)(3)	Using PHI for marketing generally requires **authorization**	A client's story, photo, or name in an ad needs a valid authorization, along with the ethics limits on testimonials.

What the Exam Rewards / What It Punishes

Rewards: - Following the written plan or protocol, staying with the client, and documenting each step; updating the plan after a move or drill - Matching each HIPAA request to its right - A BAA, encryption, and access controls before a vendor touches PHI, plus consent for recording and AI - Authorization where the client is located before telehealth - Following state board rules for CE, reporting, supervision, and renewal - Claims that match documented time, provider, and an accurate diagnosis - Free accommodations chosen with the client; legal advice before answering legal process

Punishes: - Waiting for proof of an emergency, or saving files before people - Personal devices, personal email, consumer apps, or shared logins for PHI - Consent, a

waiver, or a BAA treated as compliance - Billing no-shows, lapsed coverage, a colleague's NPI, or a coverage-driven diagnosis - Routine copay waivers, referral fees, or practicing on a lapsed license - Forensic opinions about one's own client - Soliciting testimonials or replying publicly to reviews

Swap-Pair Drill

If you see…	It's…	Not…
Data backup, disaster recovery, emergency mode	HIPAA contingency plan, 164.308(a)(7)	Building evacuation plan
"Who has received my information?"	Accounting of disclosures, 164.528	Access (164.524) or audit logs (164.312(b))
"Contact me only on my cell"	Confidential communications, 164.522(b)	Authorization (164.508)
Paid in full, keep it from the plan	Mandatory restriction, 164.522(a)(1)(vi)	Mandatory-submission myth
Insurer wants process notes	Specific authorization, 164.508(a)(2)	TPO (164.506) or minimum necessary (164.502(b))
Consumer video apps allowed	Ended August 9, 2023 (OCR)	Medicare telehealth flexibilities
Client in another state for the summer	Authorization where the client is located (H.1.b)	Client waiver
Practice privilege in member states	Counseling Compact	Separate full license in each state
Self-pay client, expected charges	Good faith estimate, 45 CFR 149.610	ABN
LPC billing Medicare directly since 2024	Medicare MHC enrollment (CAA 2023)	"Incident to"
90-minute crisis session	90839 + 90840	90837
Waiving every copay	Fraud risk	Hardship adjustment (A.10.c)

If you see…	It's…	Not…
"Bring a family member to interpret"	Prohibited as a requirement, 36.303(c)	Effective communication
"Testify I'm the better parent"	No forensic opinion of one's own client (E.13.c)	A release cures it
"ACA bans AI"	Invented rule; apply H.4.a, A.2, B	The 2014 Code
Asking clients for reviews	Prohibited solicitation (C.3.b)	Marketing

Quick Checks

1. A counselor wants to present a difficult case at a regional conference. Which step lets her share the case without a client authorization? A. Removing the client's last name from the slides B. Removing identifiers so the case meets HIPAA's de-identification standard and no remaining detail could identify the client C. Asking conference attendees to sign a confidentiality pledge D. Presenting only to licensed professionals, who are bound by confidentiality

2. A counselor has carried an **occurrence** professional liability policy for ten years and is retiring. To be covered for incidents during those years that are reported after she retires, she needs: A. No additional coverage, because the policy covers incidents that occurred while it was in force B. Tail coverage, because every policy stops covering claims once it ends C. A personal umbrella policy D. Prior acts coverage from a new carrier

3. A practice laptop containing client records is stolen from a staff member's car. Which fact most affects whether breach notification is required? A. Whether the thief is ever identified B. Whether any client later reports identity theft C. Whether the staff member had permission to take the laptop home D. Whether the laptop's data was encrypted to HHS standards

4. During an individual psychotherapy session with a 10-year-old, the child discloses abuse by an adult in the home. The counselor explains the mandated report to the child and the accompanying parent and discusses it with them during the session. Which add-on code may this support, if documented? A. 90840 B. 90847 C. 90785 D. 90846

5. A Medicare beneficiary with Original Medicare asks for a service the counselor believes Medicare will not cover. Before providing it, the counselor should give: A. A good faith estimate under the No Surprises Act B. An Advance Beneficiary Notice of Noncoverage C. A superbill D. A Notice of Privacy Practices

6. A parent asks a public high school counselor, who is employed by the school, for the counselor's records about the parent's 15-year-old. Which law primarily governs these records? A. FERPA B. HIPAA C. 42 CFR Part 2 D. The No Surprises Act

7. A malpractice payment is made on a counselor's behalf to settle a claim. This payment is reported to: A. CAQH ProView B. NPPES C. The National Practitioner Data Bank D. The DEA

8. A counselor is halfway through the requirements for a specialty certification. On her practice website she may: A. List the certification, since she will complete it soon B. List it with an asterisk explaining it is pending C. Describe herself as "certified" in the specialty but not name the certifying body D. Describe her completed training accurately without claiming the certification

9. A residential treatment center offers a counselor a payment for each client she refers to its program. The counselor should: A. Decline, because accepting payment for referrals is prohibited B. Accept, as long as she discloses the arrangement to clients C. Accept, as long as the center is the best fit clinically D. Accept and donate the payments as pro bono contributions

10. A solo counselor hires an outside billing service that will receive client names, diagnoses, and dates of service. Before sharing any information, she must: A. Get each client's authorization, since billing is not treatment B. Give the billing service a copy of her Notice of Privacy Practices C. Execute a business associate agreement with the billing service D. Remove all diagnoses from the information sent

11. A counselor licensed in her home state holds a Counseling Compact privilege to practice in a second member state and sees a client located there by video. Whose practice laws govern the sessions? A. The home state's, because privileges rest on the home-state license B. Whichever state's laws are stricter, chosen by the counselor C. The Compact's own uniform scope of practice, which replaces state law D. The remote state's, because the client is located there

12. An EHR report shows that a staff member opened the chart of a client she does not work with, who is her neighbor. The report was generated by which safeguard, and what follows? A. The minimum necessary standard; no action, since staff may view charts in the system B. Audit controls; apply the sanctions policy and assess the access as a possible breach C. The contingency plan; restore the chart from backup D. Transmission security; change the client's password

Answers and Rationales

1 — B. Owner: HIPAA de-identification (45 CFR 164.514(b)); ACA B.7.b. De-identified information is not PHI, and B.7.b bars identifying details in consultation without consent. - A: 164.514(b): removing a name is not de-identification; case details can still identify the client. - C: 164.508: only the client's authorization permits disclosure of identifiable PHI; a recipient's pledge does not. - D: Privacy Rule and B.7.b: a professional audience does not make identifiable disclosure permissible.

2 — A. Owner: occurrence policy. An occurrence policy covers incidents during the policy period whenever the claim is filed. - B: Tail coverage belongs to claims-made policies. - C: Umbrella policies generally exclude professional services. - D: Prior acts coverage is for switching between claims-made carriers.

3 — D. Owner: HIPAA Breach Notification Rule (164.402). PHI encrypted to HHS standards is not "unsecured," so its loss generally does not require notice. - A: 164.402 risk assessment: identifying the thief does not by itself show the PHI was not compromised. - B: The pre-2013 harm standard, replaced by the 2013 Omnibus Rule; proof of misuse is not required. - C: Practice device policy: permission bears on policy compliance, not on whether the PHI was compromised.

4 — C. Owner: CPT 90785, interactive complexity. Discussing a sentinel event and mandated report with the client and others in the session is a qualifying factor. - A: 90840 is the crisis add-on, used only with 90839. - B: 90847 is family psychotherapy with the patient present, not an add-on. - D: 90846 is family psychotherapy without the patient.

5 — B. Owner: Medicare Advance Beneficiary Notice (ABN). It tells an Original Medicare beneficiary that Medicare may not pay, before the service. - A: The good faith estimate is for uninsured and self-pay clients (45 CFR 149.610). - C: A superbill helps a client seek out-of-network reimbursement. - D: The NPP describes privacy practices (164.520), not coverage.

6 — A. Owner: FERPA. Records of a school-employed counselor are education records, which are excluded from HIPAA. - B: HIPAA covers health care providers, not education records. - C: Part 2 covers federally assisted substance use disorder programs. - D: The No Surprises Act governs billing estimates and surprise bills.

7 — C. Owner: National Practitioner Data Bank. It collects malpractice payments and adverse actions. - A:

CAQH ProView is a credentialing profile the provider maintains. - B: NPPES issues NPIs. - D: The DEA registers controlled-substance prescribers.

8 — D. Owner: ACA C.4.a and C.4.b. Counselors claim only qualifications completed and credentials current and in good standing. - A: C.4.b: claims a credential not yet earned. - B: C.3.a: an asterisk does not cure listing an unearned credential. - C: C.4.a: “certified” without the credential is still a misrepresentation.

9 — A. Owner: ACA A.10.b. Counselors do not give or receive commissions, rebates, or other payment for referrals. - B: A.10.b has no disclosure exception; disclosure does not make a referral fee acceptable. - C: A.10.b: clinical fit does not justify payment for the referral. - D: Pro bono (C.6.e) is service offered freely; it does not launder a referral fee.

10 — C. Owner: HIPAA business associate rules (164.502(e), 164.504(e)). A vendor handling PHI for the practice needs a BAA. - A: Payment is part of TPO (164.506); authorization is not required for it. - B: The NPP informs clients; it does not bind a vendor. - D: Minimum necessary (164.502(b)): claims require diagnoses; the safeguard is the BAA, not stripping data.

11 — D. Owner: Counseling Compact. A privilege holder practices under the laws of the remote state where the client is located. - A: Home-state license: the basis for the privilege, not the governing law of the session. - B: Compact rules: the counselor does not choose the governing law. - C: Counseling Compact: it creates no uniform scope of practice.

12 — B. Owner: HIPAA Security Rule audit controls (164.312(b)). Logs reveal access without a job-related need; the practice applies sanctions and assesses a possible breach. - A: Minimum necessary forbids this access; it does not permit it. - C: The contingency plan restores data after disasters. - D: Transmission security protects data in transit.

What the Exam Rewards / What It Punishes

Rewards: - Informed consent as an ongoing process, including limits of confidentiality - Consulting and documenting before difficult decisions - Responding to a subpoena by contacting the client and asserting privilege - Minimal necessary disclosure - Referral based on competence, not values - Pretermination counseling and referrals; never abandonment - Trying informal resolution with a colleague first when appropriate - Self-monitoring for impairment

Punishes: - Releasing records in response to a subpoena without client consent or a court order - Any sexual or romantic relationship with a current client, or with a former client within 5 years - Bartering initiated by the counselor - Ignoring a colleague's harmful violation - Practicing while impaired - Altering records - Terminating abruptly during a crisis

Swap-Pair Drill

If you see...	It's...	Not...
Keeping promises	Fidelity	Veracity
Telling the truth	Veracity	Fidelity
Client's right to choose	Autonomy	Beneficence
Doing good	Beneficence	Nonmaleficence
Avoiding harm	Nonmaleficence	Beneficence
Fairness and equal access	Justice	Autonomy
Legal right in court; client holds it	Privileged communication	Confidentiality
Ethical duty not to disclose	Confidentiality	Privilege
Approves programs	Accreditation (CACREP)	Certification
State permission to practice	Licensure	Certification

If you see…	It's…	Not…
Attorney-issued document requesting records	Subpoena	Court order
Judge-signed order	Court order	Subpoena
Former client romantic relationship	Prohibited for 5 years (A.5.c)	2 years
Evaluative, ongoing, shared responsibility	Supervision	Consultation
Supervisor liable for supervisee	Vicarious liability	Direct liability
Duty, dereliction, direct causation, damages	Malpractice	Ethical complaint
Would I want this in the news?	Publicity test	Universality test
Seven-step ethical decision-making model	Forester-Miller & Davis	Kitchener
Extra protection for process notes	HIPAA psychotherapy notes	42 CFR Part 2

Quick Checks

1. A counselor promises to call a client back by Friday and does. Which principle does this reflect? A. Veracity B. Fidelity C. Justice D. Beneficence

2. A counselor receives a subpoena from an attorney requesting a client's records. The counselor should first: A. Send the records immediately B. Ignore the subpoena C. Contact the client (and/or the client's attorney) and assert privilege on the client's behalf unless the client consents D. Destroy the records

3. Who holds privilege in a counseling relationship? A. The counselor B. The client C. The court D. The agency

4. A counselor wants to date a former client whom she last saw 3 years ago. Under the ACA Code, this is: A. Permitted after 2 years B. Prohibited until at least 5 years

after the last professional contact C. Permitted with the former client's consent D. Permitted if they meet outside the office

5. A client who can't afford fees offers to paint the counselor's office in exchange for sessions. Bartering may be acceptable if: A. The counselor proposed it B. The client requests it, it isn't exploitative, it's accepted in the community, and it's documented in a contract C. It's kept confidential D. The work is worth more than the sessions

6. A counselor notices she's been distracted, tearful, and missing details in sessions since her divorce. The best action is to: A. Keep working; clients will understand B. Seek help and limit or suspend practice until she can practice safely C. Tell clients about her divorce D. Refer all clients immediately and close her practice permanently

7. A counselor learns that a colleague has been seeing clients while intoxicated. No client has yet been harmed, but the risk is substantial. The best action is to: A. Ignore it to protect the colleague B. Report to the licensing board or appropriate body, since informal resolution isn't sufficient for a risk of substantial harm C. Tell the colleague's clients D. Post about it on social media

8. Which pair correctly matches the concept? A. Supervision: voluntary, nonevaluative B. Consultation: evaluative, shared responsibility C. Supervision: evaluative, ongoing, shared responsibility D. Consultation: supervisor is vicariously liable

9. A client with a terminal illness is considering hastening his death. The ACA Code allows the counselor to: A. Always break confidentiality B. Choose to maintain confidentiality, depending on law and circumstances, after consultation C. Encourage the decision D. End counseling

10. A client stops paying and owes several sessions. Before sending the account to collections, the counselor must: A. Do nothing; collections can proceed B. Have informed the client in advance and give the client an oppor-

tunity to pay C. Disclose the client's diagnosis to the agency D. Terminate without notice

11. A counselor wants to use an AI tool to draft session notes. The most appropriate first step is to: A. Paste session details into a free online chatbot B. Verify the tool meets HIPAA requirements, explain its use to clients, and obtain consent C. Use it without telling clients, since notes are the counselor's property D. Let the AI finalize notes without review

12. Which question applies the universality test? A. Would I want this reported in the news? B. Would I treat others the same way? C. Would I recommend this action to another counselor in the same situation? D. Is this legal in my state?

Answers and Rationales

1 — B. Keeping commitments is fidelity. A: veracity is truthfulness. C: justice is fairness. D: beneficence is doing good.

2 — C. A subpoena isn't a court order; contact the client and assert privilege (and consult legal guidance). A: releases records without authorization. B: ignoring a subpoena can bring legal consequences. D: destroying records is illegal and unethical.

3 — B. The client holds privilege and can waive it. A, C, D: incorrect holders.

4 — B. ACA A.5.c prohibits such relationships for 5 years, and even then requires forethought and documentation that it isn't exploitative. A: wrong time frame. C and D: consent and setting don't change the rule.

5 — B. ACA A.10.e conditions. A: counselor-initiated bartering is inappropriate. C: confidentiality isn't the issue. D: unequal exchange risks exploitation.

6 — B. ACA C.2.g: seek help and limit or suspend services when impaired. A: puts clients at risk. C: burdens

clients. D: an overreaction; she can arrange coverage and return when ready.

7 — B. Substantial risk of harm calls for reporting (ACA I. 2.b). A: fails to protect clients. C: violates confidentiality and proper process. D: inappropriate.

8 — C. Supervision is evaluative, ongoing, and involves shared responsibility. A and B: reversed. D: vicarious liability applies to supervisors, not consultants.

9 — B. ACA B.2.b. A: ignores the option the Code provides. C: imposes the counselor's view. D: abandonment.

10 — B. ACA A.10.d. A: skips required notice. C: discloses unnecessary information. D: abandonment.

11 — B. Confidentiality, informed consent, and counselor responsibility apply to AI (ACA B, A.2, H; ACA AI Work Group recommendations). A: breaches confidentiality. C: omits informed consent. D: the counselor remains responsible for records.

12 — C. Universality asks whether you'd recommend the action to others. A: publicity. B: justice. D: law, not one of Stadler's tests.

Audio Companion

Track	Covers	Target length
17.1	Code and Law: principles, consent, confidentiality, boundaries, records, decision-making	~6 min

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Chapter 18 — Research, Statistics, and Program Evaluation

2027 task map: 2B.11 Select, administer, and/or interpret appropriate instruments/assessments that align with contextual, cultural, and developmental considerations · 3B.8 Evaluate treatment effectiveness and client response · CACREP: Research and Program Evaluation

Why Research Matters on a Counseling Exam

Research items are a small share of the NCE, but they're among the most predictable. They test definitions and matching: which design fits, which statistic fits, which threat explains a result. Chapter 3 gave you the owners and a quick "which test" table. This chapter fills in the rest and connects research to practice: measuring whether counseling works.

Strategy: for statistics items, identify (1) how many groups, (2) what kind of data, and (3) whether the question is about **difference** or **relationship**. That narrows almost every option.

Part 1 — Research Basics

Variables

Term	Meaning	Example
Independent variable (IV)	What the researcher manipulates or compares	Type of treatment (CBT vs. waitlist)
Dependent variable (DV)	What's measured as the outcome	Depression score

Term	Meaning	Example
Confounding variable	An uncontrolled variable that offers an alternative explanation	Some participants also started medication
Moderator	Changes the **strength or direction** of a relationship (for whom, or when)	CBT works better for adults than adolescents
Mediator	**Explains how** or why the IV affects the DV	CBT reduces depression **by** reducing negative thoughts
Operational definition	How a variable is measured in a study	"Anxiety" = score on the GAD-7

Hypotheses

- **Null hypothesis (H_0):** no difference or no relationship.
- **Alternative (research) hypothesis (H_1):** there is a difference or relationship.
- **Directional** (one-tailed): predicts the direction ("CBT will **lower** scores").
- **Nondirectional** (two-tailed): predicts a difference without direction.

Researchers **reject** or **fail to reject** the null hypothesis. They never "prove" it.

Sampling

Method	Description
Simple random	Every member of the population has an equal chance of selection
Stratified random	Population divided into subgroups (strata), then randomly sampled from each, often in proportion
Cluster	Randomly select whole groups (e.g., schools), then study everyone in them

Method	Description
Systematic	Every *n*th person from a list
Convenience	Whoever is available; weakest generalizability
Purposive	Chosen for specific characteristics (common in qualitative research)
Snowball	Participants recruit others; useful for hard-to-reach populations

Random sampling supports **external validity** (generalizing to the population). **Random assignment** to groups supports **internal validity** (cause and effect). They're different things.

Part 2 — Research Designs

Quantitative Designs

Design	Key feature	Can it show cause and effect?
True experimental	**Random assignment** to groups; manipulation of the IV; control group	**Yes** (strongest)
Quasi-experimental	Manipulation of the IV **without random assignment** (intact groups, such as two classrooms)	Limited
Correlational	Measures relationships between variables without manipulation	**No**: correlation doesn't prove causation
Descriptive / survey	Describes characteristics of a group	No

Design	Key feature	Can it show cause and effect?
Ex post facto (causal-comparative)	Compares groups that already differ on a variable (e.g., people who were or weren't abused)	No
Single-subject (single-case)	One person or small group measured repeatedly across phases	Can suggest causation through replication
Longitudinal	Same participants measured over time	Shows change over time
Cross-sectional	Different age groups measured at one time	Faster, but cohort effects can confound
Meta-analysis	Statistically combines results of many studies, usually reporting an overall effect size	Summarizes evidence

Single-Subject Designs

Design	Phases
AB	Baseline (A), then intervention (B)
ABA (reversal/withdrawal)	Baseline, intervention, withdrawal of intervention
ABAB	Baseline, intervention, withdrawal, reintroduction; stronger evidence the intervention caused the change
Multiple baseline	Intervention introduced at **staggered times** across behaviors, settings, or people; used when withdrawing treatment would be unethical or impossible

Qualitative Traditions

Qualitative research explores meaning and experience, usually through interviews, observation, or documents.

Tradition	Focus	Owner / association
Phenomenology	The lived experience of a phenomenon; its essence	Husserl; Moustakas
Grounded theory	Developing a **theory** from data	**Glaser & Strauss**
Ethnography	A **culture** or group, through immersion and observation	Anthropology tradition
Case study	In-depth study of a bounded case	Stake; Yin
Narrative	Individuals' stories	—
Consensual qualitative research (CQR)	A team reaches consensus on themes	**Clara Hill**
Action / participatory action research	Researchers and participants collaborate to solve a practical problem and create change	Kurt Lewin (action research)

Trustworthiness in qualitative research (Lincoln & Guba): **credibility** (parallels internal validity), **transferability** (parallels external validity), **dependability** (parallels reliability), and **confirmability** (parallels objectivity). Strategies include **member checking**, **triangulation**, **peer debriefing**, **thick description**, and **audit trails**.

Mixed methods combine quantitative and qualitative data.

Part 3 — Validity in Research

Type	Question
Internal validity	Did the IV really cause the change in the DV?
External validity	Do the results generalize to other people, settings, and times?

Threats to Internal Validity (Campbell & Stanley)

Threat	What happens
History	An outside event during the study affects results (e.g., a natural disaster during a stress study)
Maturation	Natural changes in participants over time (growing older, tired, recovering on their own)
Testing	Taking a pretest affects posttest scores (practice effects)
Instrumentation	Changes in the measure or the raters between testings
Statistical regression	Participants selected for **extreme** scores drift toward the mean on retest
Selection	Groups differ at the start
Experimental mortality (attrition)	Participants drop out unevenly across groups
Selection–maturation interaction	Groups change at different rates
Diffusion of treatment	Control group learns about or receives part of the treatment (added to the list later by Cook & Campbell, 1979; not one of Campbell & Stanley's original eight)

Threats to External Validity and Other Effects

Effect	Description
Hawthorne effect	Participants change because they know they're being studied (Mayo)
Experimenter expectancy (Rosenthal effect)	Researcher's expectations influence results
Placebo effect	Improvement from the belief that one is receiving treatment
Demand characteristics	Participants guess the hypothesis and act accordingly

Effect	Description
John Henry effect	Control group works harder to compete with the treatment group
Reactivity of pretesting	The pretest sensitizes participants to the treatment (addressed by the Solomon four-group design)

Controls: random assignment, control groups, **double-blind** procedures (neither participants nor researchers know group assignments), and placebo conditions.

Part 4 — Descriptive Statistics

Central Tendency

Measure	Definition	Best for
Mean	Arithmetic average	Interval/ratio data with a roughly symmetrical distribution; **most affected by outliers**
Median	Middle score	Skewed data; ordinal data
Mode	Most frequent score	Nominal data; can have more than one (bimodal)

Variability

Measure	Definition
Range	Highest minus lowest; affected by extremes
Variance	Average of squared deviations from the mean
Standard deviation (SD)	Square root of the variance; the typical distance of scores from the mean
Interquartile range	Range of the middle 50% of scores

Distributions

- **Normal curve:** symmetrical; mean = median = mode; about **68%** of scores within ±1 SD, **95%** within ±2 SD, **99.7%** within ±3 SD.
- **Positive (right) skew:** tail to the right; most scores are low; **mean > median > mode** (the pattern after a hard test).
- **Negative (left) skew:** tail to the left; most scores are high; **mean < median < mode** (the pattern after an easy test).
- **Kurtosis:** peakedness. **Leptokurtic** (tall, narrow), **mesokurtic** (normal), **platykurtic** (flat).

Memory aid: the tail tells the tale. The skew is named for the direction of the tail, and the mean gets pulled toward the tail.

Part 5 — Inferential Statistics

Correlation

- **Pearson r** ranges from **−1.00 to +1.00**.
- The **sign** shows direction (positive: both rise together; negative: one rises as the other falls).
- The **absolute value** shows strength. **−.80 is stronger than +.50.**
- **Coefficient of determination (r^2):** the proportion of variance shared. An r of .60 means $r^2 = .36$, so 36% of the variance is shared.
- **Correlation does not imply causation.**

Choosing a Test

Question	Test
Two group means	**t-test** (William Gosset; independent for different groups; dependent/paired for the same people measured twice)
Three or more group means, one IV	**One-way ANOVA** (Ronald Fisher)

Question	Test
Two or more IVs	**Factorial ANOVA** (e.g., 2 × 3); can show **interaction effects**
Means, controlling for a covariate	**ANCOVA**
Two or more DVs	**MANOVA**
Frequencies in categories (nominal data)	**Chi-square**
Relationship between two interval/ratio variables	**Pearson r**
Relationship between two ranked variables	**Spearman rho**
Predicting one variable from another	**Regression** (multiple regression with several predictors)
Nonparametric alternative to independent t-test	**Mann–Whitney U**
Nonparametric alternative to one-way ANOVA	**Kruskal–Wallis**
Nonparametric alternative to paired t-test	**Wilcoxon signed-rank**
Reducing many variables into underlying factors	**Factor analysis**

Parametric tests (t-test, ANOVA, Pearson r) assume interval/ratio data and roughly normal distributions. **Nonparametric** tests (chi-square, Mann–Whitney, Kruskal–Wallis, Spearman) work with nominal or ordinal data or when assumptions aren't met.

Post hoc tests (e.g., Tukey, Scheffé) follow a significant ANOVA to find **which** groups differ.

Significance, Errors, and Effect Size

Term	Meaning
Alpha (α)	The chosen risk of a Type I error, usually **.05**

Term	Meaning
p-value	The probability of obtaining results at least this extreme **if the null hypothesis were true**. If $p < \alpha$, the result is statistically significant
Type I error	Rejecting a **true** null: a **false positive**
Type II error (β)	Failing to reject a **false** null: a **false negative**
Power ($1 - \beta$)	Probability of detecting a real effect; increased by **larger samples**, larger effects, and a higher alpha
Effect size	How large the effect is. **Cohen's d:** about **.2 small, .5 medium, .8 large**
Degrees of freedom	Related to sample size; used to evaluate test statistics

Lowering alpha (e.g., from .05 to .01) **reduces Type I errors** but **increases Type II errors**.

Statistical vs. clinical significance: a large study can find a statistically significant but tiny difference that doesn't matter in clients' lives. **Clinical significance** asks whether change is meaningful, often whether a client moved from a dysfunctional range to a functional range. The **reliable change index** (Jacobson & Truax) tests whether an individual's change is larger than measurement error.

Part 6 — Evaluating Counseling (3B.8)

Measuring Outcomes

- **Pretest and posttest measures** compare a client's scores before and after counseling (e.g., PHQ-9 at intake and at week 8).
- **Measurement-based care:** routinely administering brief measures (often every session) and using the results to guide treatment.

- **Feedback-informed treatment** tools, such as the **Outcome Rating Scale (ORS)** and **Session Rating Scale (SRS)** (Miller & Duncan), track progress and the alliance session by session.
- **Goal Attainment Scaling** (Kiresuk & Sherman): rates progress on individualized goals.

Common Factors

Research on what makes counseling work points to **common factors** shared across approaches, especially the **therapeutic alliance**, client factors, counselor empathy, and expectancy. **Saul Rosenzweig** (1936) introduced the idea, quoting the Dodo from *Alice in Wonderland* ("Everybody has won, and all must have prizes"); Luborsky and colleagues (1975) later named it the **"Dodo bird verdict"**: many bona fide therapies produce similar outcomes. **Bruce Wampold** extended this research.

Evidence-Based Practice

The American Psychological Association's definition of **evidence-based practice** integrates three components: 1. **Best available research** 2. **Clinical expertise** 3. **Client characteristics, culture, and preferences**

Program Evaluation

Term	Meaning
Needs assessment	Identifies needs and gaps **before** a program is designed
Formative evaluation	Ongoing evaluation **during** a program to improve it
Summative evaluation	Evaluation **at the end** to judge effectiveness
Process evaluation	Was the program delivered as planned (fidelity)?
Outcome evaluation	Did the program achieve its goals?
Cost-benefit analysis	Compares costs with benefits measured in dollars

Term	Meaning
Cost-effectiveness analysis	Compares costs with outcomes measured in nonmonetary units (e.g., cost per client improved)
Logic model	Maps inputs → activities → outputs → outcomes
Accountability	Demonstrating to stakeholders (funders, schools, agencies) that services work

Part 7 — Research Ethics (ACA Section G)

- **Institutional review board (IRB)** approval for research with human participants.
- **Informed consent** (G.2.a): purpose, procedures, risks, benefits, the right to withdraw **without penalty**, and confidentiality.
- **Deception** (G.2.b): used only when alternative procedures aren't feasible and the prospective value justifies it; participants are **debriefed** as soon as possible.
- **Voluntary participation**: declining to participate doesn't affect clients' services or students' grades.
- **Confidentiality** of data.
- **Accurate reporting** (G.4): report results accurately, including unfavorable results; no fabrication or plagiarism.
- **Vulnerable populations**: extra protections for minors and those unable to consent.

Historical cases that shaped research ethics: the **Tuskegee syphilis study**, Milgram's obedience studies, and the Stanford prison experiment. The **Belmont Report** (1979) set three principles: **respect for persons, beneficence, and justice**.

What the Exam Rewards / What It Punishes

Rewards: - Matching the test to the number of groups and type of data - Knowing random assignment is what allows causal conclusions - Naming the specific threat to validity - Interpreting correlation strength by absolute value - Distinguishing statistical from clinical significance - Using pre/post and session-by-session measures to evaluate counseling

Punishes: - Concluding causation from correlation - Confusing random sampling with random assignment - Mixing up Type I and Type II errors - Reading a negative correlation as “weak” - Treating a significant p-value as proof of a large effect

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Swap-Pair Drill

If you see…	It’s…	Not…
Explains **how** the IV affects the DV	Mediator	Moderator
Changes **when or for whom** an effect holds	Moderator	Mediator
Random assignment	True experiment	Quasi-experiment
Intact groups, no random assignment	Quasi-experiment	True experiment
Staggered intervention across behaviors	Multiple baseline	ABAB
Building theory from data	Grounded theory (Glaser & Strauss)	Phenomenology
Essence of lived experience	Phenomenology	Ethnography
Team consensus on themes	CQR (Hill)	Grounded theory
Outside event during the study	History	Maturation

If you see…	It's…	Not…
Extreme scorers move toward the mean	Statistical regression	Testing
Changes because they know they're observed	Hawthorne effect	Placebo effect
Control group tries harder	John Henry effect	Hawthorne effect
Tail to the right; mean > median	Positive skew	Negative skew
Compare two group means	t-test	ANOVA
Compare group means on two or more IVs	Factorial ANOVA	One-way ANOVA
Compare groups on two or more DVs	MANOVA	ANCOVA
Nominal frequencies	Chi-square	t-test
False positive	Type I	Type II
Improve a program while running	Formative	Summative
Identify gaps before designing	Needs assessment	Outcome evaluation
Program evaluation: outcomes in dollars	Cost-benefit	Cost-effectiveness
Respect for persons, beneficence, justice	Belmont Report	ACA principles

Quick Checks

1. A researcher randomly assigns 80 clients to either CBT or a waitlist and measures anxiety after 10 weeks. The design is: A. Correlational B. Quasi-experimental C. True experimental D. Ex post facto

2. A study finds that the alliance explains why a treatment reduces symptoms. The alliance is a: A. Moderator B. Mediator C. Confound D. Dependent variable

3. A researcher compares anxiety scores of clients in three different treatment groups. The appropriate test is: A. Independent t-test B. Chi-square C. One-way ANOVA D. Pearson r

4. Which correlation shows the strongest relationship? A. +.45 B. −.72 C. +.60 D. −.10

5. If r = .50, what proportion of variance is shared? A. 50% B. 25% C. 5% D. 75%

6. On a very difficult exam, most students score low and a few score high. The distribution is: A. Normal B. Positively skewed C. Negatively skewed D. Bimodal

7. Participants chosen because they scored extremely high on an anxiety scale score lower on retest, even without treatment. This is: A. History B. Maturation C. Statistical regression D. Attrition

8. A researcher concludes a program reduced dropout when, in fact, it had no effect. This is a: A. Type I error B. Type II error C. Lack of power D. Mediator effect

9. A counselor gives clients the PHQ-9 at intake and again at session 8 to see whether depression decreased. This is an example of: A. Pretest/posttest outcome measurement B. A needs assessment C. A cost-benefit analysis D. Phenomenological research

10. A study with 10,000 participants finds a statistically significant difference of half a point on a 60-point scale. The best interpretation is: A. The treatment has a large effect B. The difference is statistically significant but may not be clinically meaningful C. The study has low power D. A Type II error occurred

11. A school counselor wants to know what supports students need before designing a new program. She should conduct a: A. Summative evaluation B. Needs assessment C. Outcome evaluation D. Meta-analysis

12. Which qualitative tradition aims to develop a theory grounded in the data? A. Phenomenology B. Ethnography C. Grounded theory D. Narrative

Answers and Rationales

1 — C. Random assignment plus manipulation is a true experiment. A: correlational designs don't manipulate. B: quasi-experiments lack random assignment. D: ex post facto compares preexisting groups.

2 — B. A variable that explains how an effect occurs is a mediator. A: a moderator changes when or for whom. C: a confound offers an alternative explanation. D: the DV is the outcome (symptoms).

3 — C. Three or more group means: one-way ANOVA. A: a t-test compares two means. B: chi-square is for frequencies. D: Pearson r measures relationships.

4 — B. Strength is the absolute value; $|-.72|$ is the largest. A, C, D: smaller absolute values.

5 — B. $r^2 = .25$, so 25%. A: confuses r with r^2. C and D: incorrect.

6 — B. Most scores low with a tail to the right is positive skew. A: normal is symmetrical. C: negative skew has a tail to the left (most scores high). D: bimodal has two peaks.

7 — C. Extreme scores drift toward the mean: statistical regression. A: history is an outside event. B: maturation is natural change over time. D: attrition is dropout.

8 — A. A false positive is a Type I error. B: a Type II error misses a real effect. C: low power increases Type II risk. D: not relevant.

9 — A. Measuring before and after counseling (3B.8). B: needs assessment precedes a program. C: cost-benefit compares dollars. D: phenomenology is qualitative.

10 — B. Large samples can make trivial differences statistically significant. A: the effect is tiny. C: power is very

high with 10,000 participants. D: the null was rejected, so a Type II error didn't occur.

11 — B. Needs assessment identifies gaps before design. A and C: evaluate after or during implementation. D: meta-analysis combines existing studies.

12 — C. Grounded theory (Glaser & Strauss) builds theory from data. A: phenomenology describes lived experience. B: ethnography studies a culture. D: narrative focuses on stories.

Audio Companion

Track	Covers	Target length
18.1	Pick the Test: designs, validity threats, statistics, evaluation	~5 min

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Chapter 19 — Intake, Interview, Mental Status Exam, and Diagnosis

2027 task map: 2B.4 Conduct biopsychosocial assessments · 2B.12 Use all available data to select the appropriate diagnosis from the most recent DSM/ICD code · 2B.11 Select, administer, and/or interpret appropriate instruments/assessments that align with contextual, cultural, and developmental considerations · 2A.1 Discuss and obtain informed consent/assent from client or guardian · 2B.5 Conduct mental status examination · 2B.6 Determine co-occurring diagnoses, differential diagnoses, and other comorbidities · 2B.7 Determine the appropriate level of care · 3A.3 Develop an initial treatment plan · 2B.8 Use formal (e.g., assessment results) and informal (e.g., body language) observations · 2B.9 Use screening instruments · 2B.1 Assess for trauma · 2B.3 Assess substance use · 2A.3 Obtain client self-reports · 2B.2 Assess risk to self and others · CACREP: Assessment and Testing; Counseling and Helping Relationships

The Intake Is Where Everything Starts

Intake & Assessment is 25 of the 140 scored items. The domain asks what a counselor gathers at the start of counseling, in what order, and what to do with it. Several other chapters supply the content (risk in Chapter 6, culture in Chapter 9, presentations in Chapters 10–16). This chapter covers the **process**: the interview, the mental status exam, diagnosis, and matching clients to the right level and kind of care.

The guiding sequence: safety → relationship → assessment → diagnosis → plan. Assessment never stops after intake; it continues throughout counseling (2B.2).

Part 1 — The Initial Interview (2A.1, 2B.4, 2B.9, 2A.3)

Purposes of the First Session

1. **Informed consent** and limits of confidentiality (Chapter 17)
2. **Build rapport** (Chapter 4)
3. **Screen for safety**: suicide, homicide, abuse, violence (Chapter 6)
4. **Understand the presenting problem**: what brings the client in now
5. **Screen for fit**: are these the right services, at the right level of care, with the right counselor (2B.9)?
6. **Begin collaborative goal-setting**

Exploring the Presenting Problem

Ask about: - **Onset**: When did it start? What was happening then? - **Duration and frequency** - **Severity and distress** (2B.4): a **SUDS rating** (subjective units of distress, 0–100) or a 0–10 scale - **Impact on functioning**: work, school, relationships, self-care - **Precipitants**: Why now? - **Previous attempts** to cope and previous treatment - **The client's own explanation** of the problem (2B.11; the cultural formulation)

Types of Interviews

Type	Description	Strength	Limitation
Structured	Fixed questions in fixed order (e.g., **SCID-5**, Structured Clinical Interview for DSM-5)	High reliability; good for research and diagnosis	Less flexible; can feel impersonal
Semi-structured	Core questions with room for follow-up	Balance of reliability and flexibility	Requires training

Type	Description	Strength	Limitation
Unstructured	Conversational, follows the client	Builds rapport; flexible	Lower reliability; may miss information

Self-Report and Observation (2B.8, 2A.3)

- **Self-report**: interviews, questionnaires, symptom checklists, journals. Efficient, but subject to bias, insight, and honesty.
- **Formal observation**: structured methods (e.g., behavior rating scales, timed observations).
- **Informal observation**: what the counselor notices during the session — appearance, behavior, interaction, affect.
- **Collateral information** (with consent): family, teachers, prior providers, records.

Use several sources. Agreement among them increases confidence.

Part 2 — The Biopsychosocial Interview (2B.4)

The **biopsychosocial** assessment (based on Engel's model, Chapter 16) gathers a whole-person history:

Area	What to ask about
Biological	Medical conditions, medications, sleep, appetite, pain, substance use, family medical and psychiatric history, recent physical exam
Psychological	Presenting symptoms, mental health history, previous treatment and hospitalizations, trauma history, coping style, strengths

Area	What to ask about
Social	Family of origin, current relationships, support system, culture and identity, religion and spirituality, education, work, finances, housing, legal issues, military history
Developmental	Milestones, childhood experiences, school history
Risk	Suicide, homicide, self-harm, abuse, violence (Chapter 6)
Strengths	Resources, skills, values, supports (essential for treatment planning; Chapter 21)

The exam's sequencing rule: when a client arrives in crisis, a full history waits. Address safety first, then gather history.

The 4 Ps Case Formulation

A common way to organize intake information:

P	Question
Predisposing	What made the client vulnerable? (genetics, early experiences, temperament)
Precipitating	What triggered the current problem?
Perpetuating	What keeps it going? (avoidance, substance use, family patterns, negative thinking)
Protective	What strengths and supports help?

Part 3 — The Cultural Formulation Interview (2B.11)

The DSM-5-TR **Cultural Formulation Interview (CFI)** has **16 questions** in four domains:

1. **Cultural definition of the problem** ("What brings you here today? How would you describe the problem to your family and friends?")
2. **Cultural perceptions of cause, context, and support** ("Why do you think this is happening?" "What makes it better or worse?")
3. **Cultural factors affecting self-coping and past help-seeking**
4. **Cultural factors affecting current help-seeking** (including concerns about the counselor–client relationship)

There's also an **informant version** for family members and **supplementary modules** for specific topics (e.g., spirituality, immigrants and refugees, older adults). The CFI can be used with any client, not only clients from minority cultures.

Part 4 — The Mental Status Exam (2B.5)

The **MSE** is a snapshot of the client's current mental functioning, based mostly on **observation** during the interview. It's the psychological equivalent of a physical exam.

Component	What's assessed	Example terms
Appearance	Grooming, hygiene, dress, apparent age, notable features	Well-groomed; disheveled; appears older than stated age
Behavior / psychomotor activity	Eye contact, posture, movements, cooperation	Psychomotor **agitation** or **retardation**; tremor; guarded; cooperative

Component	What's assessed	Example terms
Speech	**Rate, volume, rhythm, tone**, amount	Pressured; slow; soft; monotone; poverty of speech
Mood	The client's **self-reported**, sustained emotional state, often in the client's own words	"I've been really down"
Affect	The counselor's **observation** of emotional expression in the moment	**Range**: full, restricted, **blunted**, **flat**. **Congruence**: congruent or incongruent with mood and content. **Lability**: rapid shifts
Thought process	**How** the client thinks: organization and flow	**Linear/goal-directed**; **circumstantial** (wanders but gets to the point); **tangential** (wanders and never returns); **flight of ideas** (rapid jumps between loosely connected topics); **loose associations** (unconnected ideas); **thought blocking**; **perseveration**; **clang associations**; **word salad**
Thought content	**What** the client thinks about	Suicidal or homicidal ideation; **delusions**; **obsessions**; preoccupations; phobias
Perception	Sensory experiences	**Hallucinations** (no stimulus present; auditory most common in psychotic disorders); **illusions** (misperception of a real stimulus); depersonalization and derealization

Component	What's assessed	Example terms
Cognition / sensorium	Orientation, attention, concentration, memory	**Oriented ×3** (person, place, time) or ×4 (plus situation); serial sevens; recall of three words
Insight	The client's understanding of their problem	Good, fair, poor, absent
Judgment	Ability to make sound decisions	Good, fair, poor

High-Yield MSE Distinctions

Pair	Difference
Mood vs. affect	Mood is **reported** by the client and sustained ("climate"); affect is **observed** by the counselor in the moment ("weather")
Circumstantial vs. tangential	Circumstantial speech **reaches the point** eventually; tangential speech **never does**
Flight of ideas vs. loose associations	Flight of ideas jumps quickly between **loosely connected** topics (common in mania); loose associations have **no logical connection** (common in psychosis)
Hallucination vs. illusion	Hallucination: **no stimulus**; illusion: **misperceived real stimulus** (a coat on a chair looks like a person)
Delusion vs. obsession	A delusion is a **fixed false belief** held despite evidence; an obsession is an intrusive thought the person usually recognizes as excessive
Thought process vs. thought content	Process is **how**; content is **what**
Blunted vs. flat affect	Blunted: significantly **reduced** expression; flat: virtually **no** expression

Screening tools for cognition: MoCA, MMSE, Mini-Cog (Chapter 12).

Part 5 — Diagnosis (2B.12, 2B.6)

How the DSM-5-TR Works

- The DSM-5 (2013) **eliminated the multiaxial system** (Axes I–V). Diagnoses are listed together, with medical conditions and psychosocial factors noted alongside.
- The **GAF** (Global Assessment of Functioning) was dropped. The DSM-5-TR suggests the **WHO Disability Assessment Schedule 2.0 (WHODAS 2.0)** for measuring functioning (2B.8).
- Diagnoses use **ICD-10-CM codes** for billing in the U.S.
- **Specifiers** add detail (e.g., severity; "with anxious distress"; "in early remission").
- **Provisional** diagnosis: used when there's a strong presumption the criteria will be met but not enough information yet.
- **Other specified** disorder: the clinician states why criteria aren't fully met (e.g., "other specified depressive disorder, depressive episode with insufficient symptoms").
- **Unspecified** disorder: the clinician chooses not to, or can't, specify why criteria aren't met (common in emergency settings).
- **Z codes**: other conditions that may be a focus of clinical attention, such as relationship distress, uncomplicated bereavement, problems with housing or employment, and moral, religious, or spiritual problems (Chapter 15). They aren't mental disorders.
- **Principal diagnosis** (inpatient): the condition chiefly responsible for the admission. **Reason for visit** (outpatient): the condition chiefly responsible for the outpatient services received. Either is listed first.

Differential Diagnosis

A widely taught sequence (adapted from Michael First's *DSM-5-TR Handbook of Differential Diagnosis*):

1. **Rule out malingering and factitious disorder**
2. **Rule out a substance or medication cause** (Chapter 11)
3. **Rule out a medical condition** (Chapter 10, Chapter 12, Chapter 16)
4. **Determine the specific primary disorder(s)**
5. **Differentiate adjustment disorder** from other specified/unspecified disorders
6. **Establish the boundary with no mental disorder**: is there clinically significant distress or impairment?

Term	Meaning
Malingering	**Intentionally** producing or exaggerating symptoms for an **external** reward (avoiding work or prosecution, getting money or drugs). A Z code, not a mental disorder
Factitious disorder	**Intentionally** falsifying symptoms **without obvious external reward**, to assume the sick role. **Imposed on self** or **imposed on another** (formerly "Munchausen by proxy")
Somatic symptom and functional neurological disorders	Symptoms are **not** intentionally produced (Chapter 16)

Anchoring Bias (3A.7)

Anchoring bias is fixing on an early diagnosis and discounting later data that doesn't fit. It often leads to **premature closure**: the search for other explanations stops once a diagnosis seems to fit. A client diagnosed with major depression mentions, in session 6, past weeks of little sleep and big spending. The anchored counselor keeps the depression diagnosis because it's already in the record. Don't confuse it with **confirmation bias**, which is

seeking out or favoring evidence that supports what you already believe. Anchoring is getting stuck on the first impression; confirmation bias is how you then gather evidence for it. The remedy for both: keep the differential open, treat every diagnosis as a working hypothesis, and **review the diagnosis with the client** when new information comes in (3A.7).

Co-Occurring Diagnoses (2B.6)

- Co-occurrence is common: depression with anxiety, substance use with trauma, ADHD with learning disorders.
- Diagnose every disorder for which criteria are met, unless the criteria say otherwise (some diagnoses exclude others: DMDD, for example, rules out a concurrent ODD diagnosis).
- Treat co-occurring substance use and mental health disorders together (integrated treatment; Chapter 9, Chapter 11).

Ethics of Diagnosis

- Explain the diagnosis and its implications (including insurance and records) as part of informed consent (ACA A.2.b, E.5).
- **Cultural sensitivity** in diagnosis (E.5.b): culture affects how problems are defined and expressed.
- **Historical and social prejudices** in diagnosis (E.5.c): be aware of how some groups have been misdiagnosed and pathologized.
- **Refraining from diagnosis** (E.5.d): counselors may refrain from making or reporting a diagnosis if they believe it would cause harm.
- **Never assign a diagnosis just for insurance reimbursement** when criteria aren't met. That's fraud.

Part 6 — Specialized Assessment

Area	Key points	Where covered
At-risk behaviors (2B.2)	Assess suicide, homicide, self-harm, and relationship violence at intake and **on an ongoing basis**	Chapter 6
Trauma (2B.1)	Ask about trauma with care and permission; screen (PC-PTSD-5, LEC-5, PCL-5, ACEs); don't push for detail at intake	Chapter 10
Substance use (2B.3)	Ask every client; screen (CAGE, AUDIT, CRAFFT, DAST); assess withdrawal risk	Chapters 9, 11
Functioning (2B.8)	WHODAS 2.0; daily activities; work, school, relationships	This chapter
Interactional dynamics (2B.8)	In couple and family sessions, observe who speaks, who's silent, alliances, conflict patterns, and nonverbal responses	Chapter 7

Part 7 — Level of Care and Modality (2B.7, 3A.3)

Level of Care

Match the intensity of services to the client's needs, using the **least restrictive** level that's safe and effective.

Level	Description	Typical indications
Outpatient	Regular sessions (e.g., weekly)	Mild to moderate symptoms; safe; functioning in daily life

Level	Description	Typical indications
Intensive outpatient (IOP)	Several hours a day, several days a week; client lives at home	More support needed; stepping down from a higher level
Partial hospitalization (PHP)	Day program, most of the day, most days; client returns home at night	Significant symptoms without need for 24-hour care
Residential	Living in a structured treatment setting	Need for a structured environment (often substance use or eating disorders)
Inpatient hospitalization	24-hour medically supervised care	**Imminent danger** to self or others; severe inability to care for self; acute psychosis or mania; medically unstable

Tools: the **ASAM Criteria** for substance use (six dimensions; Chapter 9); **LOCUS** (Level of Care Utilization System) for adults and **CALOCUS/CASII** for children and adolescents.

Involuntary hospitalization generally requires evidence of danger to self, danger to others, or grave disability (inability to meet basic needs), according to state law.

Choosing a Modality (3A.3)

Modality	Often indicated when...
Individual	Privacy is essential; individualized focus; safety concerns that make joint sessions risky
Couple	The relationship is the focus, and **no ongoing violence or coercive control** (Chapter 6)
Family	The problem is maintained by family patterns; a child or adolescent is the identified patient (Chapter 7)

Modality	Often indicated when…
Group	Social learning, universality, skill-building, and support would help, and the client has passed screening (Chapter 8)
Combined	Many clients benefit from more than one modality

What the Exam Rewards / What It Punishes

Rewards: - Safety screening first, especially in crisis - Asking every client about substance use and trauma, with care - Using multiple sources of information - Knowing the MSE terms precisely (mood vs. affect; tangential vs. circumstantial) - Ruling out substances and medical causes before primary diagnoses - Least restrictive level of care that's safe - Ongoing assessment of risk

Punishes: - Taking a full history before assessing safety in a crisis - Diagnosing from a single symptom or a single source - Confusing malingering (external gain) with factitious disorder (sick role) - Using the GAF or multiaxial system as current practice - Giving a diagnosis to obtain insurance payment when criteria aren't met - Hospitalizing when outpatient care is safe, or keeping outpatient care when danger is imminent

Swap-Pair Drill

If you see…	It's…	Not…
Client says "I feel down"	Mood	Affect
Counselor observes tearful, restricted expression	Affect	Mood
Speech wanders but gets to the point	Circumstantial	Tangential
Speech wanders and never gets to the point	Tangential	Circumstantial

If you see…	It's…	Not…
Rapid jumps between related topics	Flight of ideas	Loose associations
No logical connection between ideas	Loose associations	Flight of ideas
Hearing voices with no source	Hallucination	Illusion
Shadow mistaken for a person	Illusion	Hallucination
Person, place, time	Orientation ×3	Insight
Fixed questions in fixed order	Structured interview (SCID-5)	Unstructured
Replaced the GAF	WHODAS 2.0	Multiaxial Axis V
Fakes symptoms for money or to avoid prosecution	Malingering	Factitious disorder
Fakes symptoms to be seen as sick	Factitious disorder	Malingering
Clinician explains why criteria aren't met	Other specified	Unspecified
Holding to the first diagnosis despite new data	Anchoring bias	Confirmation bias
Predisposing, precipitating, perpetuating, protective	4 Ps case formulation	Biopsychosocial model
Day program, home at night	Partial hospitalization	Residential
Adults' level of care tool	LOCUS	ASAM

Quick Checks

1. A client says, "I'm fine, really," while crying and looking at the floor. Her affect is best described as: A. Congruent with her stated mood B. Incongruent with her stated mood C. Flat D. Labile

2. Asked about his job, a client begins describing his commute, his coworkers, and his lunch, and eventually answers the question. This is: A. Tangential B. Circumstantial C. Flight of ideas D. Loose associations

3. A client sees a coat hanging on a door and briefly thinks it's a person standing there. This is: A. A hallucination B. An illusion C. A delusion D. Depersonalization

4. A client arrives saying he plans to kill himself tonight. The counselor's first priority is to: A. Complete the biopsychosocial history B. Assess and respond to the immediate safety risk C. Administer the WHODAS 2.0 D. Explore his family of origin

5. In the DSM-5-TR, which tool is suggested for assessing functioning? A. GAF B. WHODAS 2.0 C. Axis V D. MMPI-3

6. A client reports pain symptoms that worsen whenever a disability hearing approaches and admits he needs "to look sick enough" to get benefits. This most suggests: A. Somatic symptom disorder B. Factitious disorder C. Malingering D. Functional neurological symptom disorder

7. A client's symptoms nearly meet criteria for a depressive disorder, and the counselor records why they don't fully meet them. The appropriate category is: A. Unspecified depressive disorder B. Other specified depressive disorder C. Provisional major depressive disorder D. No diagnosis

8. In the 4 Ps formulation, a client's daily drinking that keeps her anxiety from improving is a: A. Predisposing factor B. Precipitating factor C. Perpetuating factor D. Protective factor

9. Which is the first step in the differential diagnosis sequence? A. Determine the primary disorder B. Rule out malingering and factitious disorder C. Assign an adjustment disorder D. Establish the boundary with no disorder

10. A client with severe depression has no suicidal ideation, lives with supportive family, but has stopped functioning at work and needs more support than weekly

sessions provide. The most appropriate level of care is likely: A. Inpatient hospitalization B. Intensive outpatient or partial hospitalization C. No treatment D. Residential care for a year

11. A counselor is asked by a client to "put down something insurance will pay for" even though the client's symptoms don't meet criteria for any disorder. The counselor should: A. Assign a mild diagnosis to help the client B. Decline, explain that diagnoses must reflect actual criteria, and discuss other payment options C. Assign an "unspecified" diagnosis D. Bill under a family member's diagnosis

12. The Cultural Formulation Interview's first domain addresses: A. Cultural definition of the problem B. Medications and medical history C. Suicide risk D. Treatment goals

Answers and Rationales

1 — B. Her observed affect (crying) doesn't match her stated mood ("fine"). A: they don't match. C: flat affect shows virtually no expression. D: labile affect shifts rapidly.

2 — B. Wandering but eventually reaching the point is circumstantial. A: tangential never returns. C: flight of ideas involves rapid jumps. D: loose associations lack logical connection.

3 — B. Misperceiving a real stimulus is an illusion. A: a hallucination has no stimulus. C: a delusion is a fixed false belief. D: depersonalization is feeling detached from oneself.

4 — B. Safety comes first (Chapter 6). A, C, D: wait until safety is addressed.

5 — B. The DSM-5-TR suggests the WHODAS 2.0. A and C: the GAF and Axis V were dropped. D: the MMPI-3 is a personality inventory.

6 — C. Intentional symptom production for external gain (benefits) is malingering. A and D: symptoms aren't intentionally produced. B: factitious disorder involves no obvious external reward.

7 — B. "Other specified" is used when the clinician states why criteria aren't met. A: "unspecified" doesn't specify the reason. C: provisional is used when criteria are expected to be met with more information. D: a diagnosis is appropriate given clinically significant symptoms.

8 — C. Drinking that maintains the problem is perpetuating. A: predisposing factors are vulnerabilities. B: precipitating factors trigger the problem. D: protective factors help.

9 — B. The first step rules out malingering and factitious disorder (First). A, C, D: later steps.

10 — B. Needs exceed outpatient care but don't require 24-hour care; IOP or PHP is the least restrictive appropriate level. A: no imminent danger. C: untreated severe depression is harmful. D: excessive.

11 — B. Diagnosing to obtain payment when criteria aren't met is fraud and unethical. A, C, D: all misrepresent the client's condition.

12 — A. The CFI begins with the cultural definition of the problem. B, C, D: not CFI domains.

Audio Companion

Track	Covers	Target length
19.1	Intake and MSE: interview, MSE terms, diagnosis, level of care	~5 min

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Chapter 20 — Assessment and Testing

2027 task map: 2B.11 Select, administer, and/or interpret appropriate instruments/assessments that align with contextual, cultural, and developmental considerations · 3A.5 Integrate assessment results into treatment plans · 3B.8 Evaluate treatment effectiveness and client response · CACREP: Assessment and Testing

From Owners to Practice

Chapter 3 gave you the testing owners (Binet, Terman, Wechsler, Spearman, Cattell, Gardner, Sternberg), the types of reliability and validity, and the main score scales. This chapter covers what the NCE asks about using tests: choosing the right instrument, administering and scoring it, interpreting scores honestly, communicating results, and following the ethical and legal rules.

The principle that runs through the chapter: a test is one source of information. It informs decisions; it never makes them alone.

Part 1 — Types of Assessment

Distinction	Options
Standardized vs. nonstandardized	Standardized: uniform administration, scoring, and norms. Nonstandardized: informal checklists, observation, interviews
Norm-referenced vs. criterion-referenced vs. ipsative	Compared with **others** / with a **fixed standard** / with **oneself** (Chapter 3)

Distinction	Options
Maximal vs. typical performance	**Maximal**: best possible performance (intelligence, achievement, aptitude). **Typical**: usual behavior (personality, interests)
Speed vs. power	**Speed**: easy items, strict time limit. **Power**: items get harder; generous time
Objective vs. projective	**Objective**: structured responses, standard scoring (MMPI-3). **Projective**: ambiguous stimuli, open responses (Rorschach, TAT)
Individual vs. group	One examinee with one examiner, or many at once
Verbal vs. nonverbal	Language-dependent or language-reduced (e.g., Raven's Progressive Matrices)

What Each Category Measures

Category	Measures	Examples
Intelligence	General cognitive ability	WAIS, WISC, Stanford-Binet, KABC
Aptitude	Potential to learn or perform	ASVAB, GATB, SAT (often described as aptitude)
Achievement	What has been learned	WIAT, WRAT, Woodcock-Johnson Tests of Achievement
Personality	Traits, patterns, psychopathology	MMPI-3, MCMI, PAI, NEO-PI, 16PF
Interests	Preferences	Strong, SDS, Kuder (Chapter 14)
Values	What matters	MIQ (Chapter 14)
Symptoms and screening	Specific problems	PHQ-9, GAD-7, PCL-5, AUDIT
Behavior	Observed behavior, often by parents or teachers	CBCL, BASC, Conners

Category	Measures	Examples
Neuropsychological	Brain–behavior functioning	Halstead-Reitan, Bender-Gestalt II

Part 2 — Choosing an Instrument (2B.11)

Ask:

1. **Purpose:** screening, diagnosis, treatment planning, career decision, or outcome measurement?
2. **Reliability:** is it consistent enough? For important individual decisions, coefficients around **.90** are often expected; **.80** is considered acceptable for many uses.
3. **Validity:** does it measure what you need, for **this purpose**?
4. **Norms:** was it normed on people **like this client** (age, culture, language, disability)?
5. **Fairness:** is there evidence of bias for this client's group?
6. **Practicality:** time, cost, reading level, format, accommodations.
7. **Counselor competence:** are you trained and qualified to administer and interpret it (ACA E.2.a)?

User Qualification Levels

Publishers restrict instruments by user qualifications, often described in three levels:

Level	Typical requirement	Examples
A	Little special training	Some achievement and screening tools
B	Graduate coursework in testing; often a master's-level counselor	Interest inventories, many personality inventories, symptom scales
C	Advanced training, often a doctorate or specialized supervised training	Individual intelligence tests, projective tests

Publishers now use their own qualification codes, and state laws on test use vary. The principle is the same: **use only instruments you're qualified to use.**

Legal Cases on Testing Fairness

Case	Holding
Griggs v. Duke Power Co. (1971)	Employment tests must be **job-related**; tests that screen out protected groups without job relevance are discriminatory
Diana v. State Board of Education (1970)	Children must be tested in their **primary language** (and nonverbal measures used) before special education placement
Larry P. v. Riles (1979)	IQ tests could not be used to place Black children in classes for "educable mentally retarded" students in California because of cultural bias
Hobson v. Hansen (1967)	Ability-grouping ("tracking") based on biased group tests was unconstitutional
Debra P. v. Turlington (1981)	A graduation test must cover what students were actually taught (**curricular/instructional validity**)

Part 3 — Administration and Scoring

- Follow **standardized procedures** (instructions, time limits, materials). Departures make norms less meaningful.
- Provide **accommodations** for disability when appropriate (ACA E.7.a; ADA), and note them when interpreting results.
- Establish **rapport**; note factors that could affect performance (fatigue, illness, anxiety, language, medication).
- **Computer-administered** tests must also be validated, and the counselor remains responsible for interpretation (ACA E.7.c; Chapter 9).

Scores

A **raw score** means little on its own. It must be converted using norms or a criterion.

Score	Mean	SD	Notes
z-score	0	1	Basis for all standard scores
T-score	50	10	Common for personality inventories (MMPI-3: a T-score of **65 or above** is generally considered clinically elevated)
Deviation IQ	100	15	Wechsler; current Stanford-Binet also uses 15
Stanine	5	~2	Range 1–9
Normal curve equivalent (NCE)	50	21.06	Equal-interval scale from 1 to 99; used in education
Percentile rank	—	—	Percentage of the norm group scoring **at or below** a score. **Ordinal**: percentiles are **not equal intervals** (the difference between the 50th and 55th percentile is much smaller in raw-score terms than between the 90th and 95th)

Score	Mean	SD	Notes
Grade equivalent	—	—	Easily misinterpreted. A 3rd grader with a grade equivalent of 6.2 in math did as well as a typical 6th grader would **on 3rd-grade material**; it doesn't mean the child can do 6th-grade math

Converting: z = (score − mean) ÷ SD. A WAIS score of 130 is (130 − 100) ÷ 15 = **z of +2**, which is about the **98th percentile**, and equals a **T-score of 70**.

z	T	Deviation IQ	Approx. percentile
−2	30	70	2nd
−1	40	85	16th
0	50	100	50th
+1	60	115	84th
+2	70	130	98th

Standard Error of Measurement (SEM)

No test is perfectly reliable, so every observed score has error.

- **SEM = SD × √(1 − reliability)**
- **Higher reliability → smaller SEM.**
- **Confidence bands:** the true score falls within **±1 SEM about 68%** of the time and within **±2 SEM about 95%** of the time.
- **Example:** an IQ score of 100 with an SEM of 3 gives a 95% band of about **94 to 106**.

Exam principle: report and interpret scores as **ranges**, not single points.

Test Construction Terms

Term	Meaning
Item difficulty (p)	Proportion of test-takers who answer correctly; **higher p = easier** item; .50 is often ideal for discrimination
Item discrimination	How well an item separates high and low scorers on the whole test
Floor effect	Test is too hard; many score at the bottom; can't distinguish among low scorers
Ceiling effect	Test is too easy; many score at the top
Norm group	The sample whose scores provide comparison; must be **representative**, **recent**, and **relevant**

Diagnostic Accuracy

Term	Meaning
Sensitivity	Ability to correctly identify people **who have** the condition (true positive rate)
Specificity	Ability to correctly identify people **who don't** have it (true negative rate)
False positive	Test says the condition is present when it isn't
False negative	Test says it's absent when it's present
Base rate	How common the condition is in the population; low base rates produce more false positives even with good tests
Incremental validity	Whether a test adds useful information beyond what's already known

Screening tools prioritize **sensitivity** (don't miss cases); positives are then confirmed with more thorough assessment.

Part 4 — Major Instruments

Intelligence and Achievement

Instrument	Notes
Wechsler Adult Intelligence Scale (WAIS)	Ages 16+; Full Scale IQ and index scores (e.g., verbal comprehension, working memory, processing speed); current edition is the WAIS-5
Wechsler Intelligence Scale for Children (WISC-V)	Ages 6–16
WPPSI-IV	Ages 2½–7
Stanford-Binet (SB5)	Ages 2–85+; descended from Binet's scale
Kaufman Assessment Battery for Children (KABC-II)	Designed to reduce cultural and language bias
Raven's Progressive Matrices	**Nonverbal** reasoning; useful when language is a barrier
Woodcock-Johnson	Separate cognitive and achievement batteries
WIAT-4, **WRAT5**	Achievement

Personality: Objective

Instrument	Owner	Notes
MMPI-3 (2020); **MMPI-2**; **MMPI-A-RF** (adolescents)	Hathaway & McKinley (original)	Most researched personality and psychopathology inventory; **validity scales** detect inconsistent, overreported, or underreported responding
Millon Clinical Multiaxial Inventory (MCMI-IV)	Theodore Millon	Personality disorders and clinical syndromes
Personality Assessment Inventory (PAI)	Leslie Morey	Clinical, treatment, and interpersonal scales

Instrument	Owner	Notes
NEO Personality Inventory (NEO-PI-3)	Costa & McCrae	**Big Five**: **O**penness, **C**onscientiousness, **E**xtraversion, **A**greeableness, **N**euroticism (**OCEAN**)
16PF	Raymond Cattell	16 personality factors
California Psychological Inventory (CPI)	Harrison Gough	Normal personality
MBTI	Myers & Briggs (based on Jung)	Type, not pathology (Chapter 14)

Personality: Projective

Instrument	Owner	Task
Rorschach Inkblot Test	Hermann Rorschach; scoring by **Exner's Comprehensive System** and now **R-PAS**	Describe what inkblots look like
Thematic Apperception Test (TAT)	Henry Murray & Christiana Morgan	Tell stories about ambiguous pictures; needs and press
Rotter Incomplete Sentences Blank	Julian Rotter	Complete sentence stems
House-Tree-Person (H-T-P)	John Buck	Drawings
Draw-A-Person	Goodenough; Machover	Drawings
Kinetic Family Drawing	Burns & Kaufman	Draw family members doing something

Projective tests have lower reliability and validity evidence than objective tests, and scoring requires advanced training.

Behavior Rating Scales

Instrument	Notes
Child Behavior Checklist (CBCL)	Thomas Achenbach (ASEBA system); parent, teacher, and youth forms
BASC-3	Behavior Assessment System for Children
Conners	ADHD-related behaviors

Symptom and Outcome Measures

Instrument	Notes
SCL-90-R / BSI	Leonard Derogatis; broad symptom checklists
OQ-45	Michael Lambert; Outcome Questionnaire for tracking progress
ORS / SRS	Miller & Duncan; brief session-by-session outcome and alliance measures (Chapter 18)
PHQ-9, GAD-7, BDI-II, BAI, PCL-5	Chapter 10

Neuropsychological Screening

Instrument	Notes
Bender Visual-Motor Gestalt Test (Bender-Gestalt II)	Lauretta Bender; copying designs; visual-motor integration
Halstead-Reitan and **Luria-Nebraska** batteries	Comprehensive neuropsychological assessment (specialist-administered)
MoCA, MMSE	Cognitive screening (Chapter 12)

Part 5 — Interpreting and Communicating Results (3A.5)

Interpretation

- Interpret within the **context** of the client's history, culture, current state, and other data.

- Consider the **norm group's** fit and any **accommodations** given.
- Use **confidence bands**.
- **Integrate multiple sources**; no single score should drive a major decision.
- Watch for **validity scale** problems on instruments like the MMPI-3 before interpreting clinical scales.

Communicating Results (ACA E.1, E.3, E.6, E.9)

- Explain results in **language the client understands** (E.3.a).
- Explain the purpose, limits, and meaning of the results before and after testing.
- Invite questions and the client's reactions; check that the client understands.
- **Use results to support the client's own decisions** (3A.5): "Your results show strong Social and Artistic interests. How does that fit with what you know about yourself?"
- Avoid labels that overstate certainty.
- Note any reservations about validity or reliability in reports (E.9.a).

Ethics and Security

ACA standard	Rule
E.2.a Limits of competence	Use only tests you're trained to use
E.4 Release of data to qualified personnel	Release raw data (responses, test protocols) only to persons qualified to interpret them, with client consent
E.5 Diagnosis	Cultural sensitivity; refraining from diagnosis if harmful (Chapter 19)
E.6.a Appropriateness of instruments	Consider validity, reliability, psychometric limits, and appropriateness

ACA standard	Rule
E.7 Conditions of administration	Standard conditions; note irregularities; technological administration validated
E.8 Multicultural issues	Use caution with instruments normed on populations other than the client's
E.10 Assessment security	Maintain integrity and security of tests; don't reproduce or modify without permission
E.11 Obsolete assessment and outdated results	Don't use outdated instruments or results for current decisions

Test security: never photocopy test booklets for clients or post items online. That damages the test's validity for everyone and violates copyright.

What the Exam Rewards / What It Punishes

Rewards: - Choosing instruments by purpose, reliability, validity, norms, and the counselor's competence - Testing in the client's primary language; using nonverbal measures when appropriate - Reporting scores as ranges using the SEM - Explaining results clearly and using them to support the client's own decisions - Checking validity scales before interpreting clinical scales

Punishes: - Using one score to make a major decision - Interpreting percentile differences as equal intervals - Taking grade equivalents literally - Using outdated tests or norms that don't fit the client - Giving raw test data to unqualified people - Using tests beyond one's training

Swap-Pair Drill

If you see…	It's…	Not…
Test of best possible performance	Maximal performance	Typical performance
Test of usual (typical) behavior	Typical performance	Maximal performance

If you see…	It's…	Not…
Test with easy items, strict time	Speed test	Power test
Test with harder items, generous time	Power test	Speed test
Test of what's been learned	Achievement	Aptitude
Test of potential to learn	Aptitude	Achievement
Percentage scoring at or below	Percentile rank	Percentage correct
Mean 50, SD 10	T-score	Normal curve equivalent (SD 21.06)
True score range around observed	SEM confidence band	Standard deviation
Test identifies those who **have** the condition	Sensitivity	Specificity
Test identifies those who **don't** have the condition	Specificity	Sensitivity
Test too hard; everyone scores low	Floor effect	Ceiling effect
OCEAN	NEO-PI (Costa & McCrae)	16PF
Stories about ambiguous pictures	TAT (Murray)	Rorschach
Parent/teacher behavior ratings	CBCL (Achenbach)	MMPI-A
Nonverbal reasoning	Raven's	WAIS verbal index
Employment tests must be job-related	*Griggs v. Duke Power*	*Larry P. v. Riles*
Test in the primary language	*Diana v. State Board of Education*	*Hobson v. Hansen*
Test must cover what was taught	*Debra P. v. Turlington*	*Griggs*

Quick Checks

1. A score of 115 on the WAIS is about how many standard deviations above the mean? A. 0.5 B. 1 C. 1.5 D. 2

2. A T-score of 70 corresponds to a z-score of: A. +1 B. +2 C. +3 D. −2

3. A test has an SEM of 4. A client scores 90. The 95% confidence band is about: A. 86–94 B. 82–98 C. 88–92 D. 70–110

4. Which change would reduce a test's SEM? A. Lower reliability B. Higher reliability C. A smaller norm group D. Shorter time limits

5. A 4th grader's reading grade equivalent is 7.0. The best interpretation is: A. She can read 7th-grade books B. She should skip to 7th grade C. She scored as a typical 7th grader would on 4th-grade material D. The test is invalid

6. A recent immigrant from Haiti who speaks primarily Haitian Creole needs a cognitive assessment. The counselor's best choice is to: A. Use the English WAIS as usual B. Use a nonverbal measure and/or assessment in his primary language, interpreting with caution about norms C. Skip assessment entirely D. Use his 10-year-old as an interpreter

7. A screening tool for suicide risk should prioritize: A. Specificity B. Sensitivity C. Ceiling effects D. Low reliability

8. On the MMPI-3, before interpreting clinical scales, the counselor should check: A. The T-score of the first clinical scale B. The validity scales C. The client's IQ D. The client's Holland code

9. A counselor with only a master's-level testing course wants to administer and interpret the Rorschach. The best guidance is: A. Proceed, since it's a common test B. Don't, unless the counselor has the advanced specialized training the instrument requires C. Proceed with the cli-

ent's consent D. Use an online scoring program to compensate

10. A client asks for a copy of the MMPI-3 items to study. The counselor should: A. Provide a copy B. Explain that test materials must be kept secure, while offering to discuss the results C. Post the items online for other clients D. Let the client take the test home

11. Which court case held that IQ tests could not be used to place Black children in classes for students with intellectual disabilities in California because of cultural bias? A. *Griggs v. Duke Power* B. *Larry P. v. Riles* C. *Diana v. State Board of Education* D. *Debra P. v. Turlington*

12. A career client's interest inventory shows strong Investigative interests. The best way to use the result is to: A. Tell the client to become a scientist B. Explore with the client how the results fit his experience and goals, and use them to guide further exploration C. Ignore the result D. Retest until a different result appears

Answers and Rationales

1 — B. (115 − 100) ÷ 15 = 1 SD. A, C, D: incorrect calculations.

2 — B. T = 50 + 10z, so 70 is z = +2. A: z = +1 is T = 60. C: z = +3 is T = 80. D: z = −2 is T = 30.

3 — B. ±2 SEM = ±8, so 82–98. A: ±1 SEM is the 68% band. C: too narrow. D: too wide.

4 — B. SEM = SD × √(1 − r); higher reliability shrinks it. A: lower reliability increases SEM. C and D: don't directly lower SEM.

5 — C. Grade equivalents compare performance on the test's own material. A and B: common misinterpretations. D: no evidence of invalidity.

6 — B. Primary-language and nonverbal assessment, with cautious interpretation (*Diana*; ACA E.8). A: English testing would measure language, not ability. C: assessment can still help. D: a child shouldn't interpret (Chapter 9).

7 — B. Screening prioritizes not missing cases (sensitivity). A: specificity matters for confirming. C and D: undesirable properties.

8 — B. Validity scales detect inconsistent, overreported, or underreported responding. A, C, D: not the first check.

9 — B. Use only instruments within your competence (ACA E.2.a); projective scoring requires advanced training. A and C: competence isn't granted by popularity or consent. D: software doesn't replace training.

10 — B. Test security (ACA E.10). A, C, D: compromise test integrity.

11 — B. *Larry P. v. Riles*. A: *Griggs* concerns employment testing. C: *Diana* concerns language of testing. D: *Debra P.* concerns curricular validity.

12 — B. Results support the client's own decision-making (3A.5). A: test-and-tell. C: discards useful information. D: inappropriate.

Audio Companion

Track	Covers	Target length
20.1	Score It: test types, scores, SEM, instruments, ethics	~5 min

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Chapter 21 — Treatment Planning, Referral, and Termination

2027 task map: 3A.9 Collaborate with client to establish treatment goals and objectives · 3A.2 Develop SMART (Specific, Measurable, Attainable, Realistic, and Timely) counseling goals and objectives · 3A.4 Identify strengths and barriers affecting client goal attainment · 3B.1 Refer clients and/or family members to community resources or to specialists · 3A.3 Develop an initial treatment plan · 3B.9 Collaborate with client to determine counseling termination criteria · 4.5 Foster the group therapeutic process · 3B.2 Develop an aftercare plan · 3A.5 Integrate assessment results into treatment plans · 3A.10 Collaborate with client to revise treatment plan as needed · 3B.7 Engage with client in review of progress toward treatment goals at planned intervals · 5.3 Collaborate with other treatment providers (e.g., care coordination, interdisciplinary treatment team) · 3B.3 Discuss with clients how they will maintain progress · 4.11 Provide psychoeducation · CACREP: Counseling and Helping Relationships

The Plan Connects Everything

Treatment Planning & Continuity of Care is 21 of the 140 scored items, and the plan is where every other domain lands. Intake findings (Chapter 19) become problem statements. Theory and interventions (Chapters 2, 5) become methods. Assessment results (Chapter 20) become baselines and outcome measures. Ethics (Chapter 17) governs consent, referral, and termination.

The exam's core principle: the plan is **collaborative**, **measurable**, **sequenced by priority**, and **revised** as counseling unfolds.

Part 1 — Building the Plan (3A.9, 3A.2, 3A.3)

Components of a Treatment Plan

Component	Description	Example
Problem statement	The concern, stated in **behavioral** terms, often linked to the diagnosis	“Client reports depressed mood most days, sleeping 4 hours a night, and missing work 2 days a week”
Long-term goal	The broad outcome	“Client will experience reduced depression and return to full functioning”
Short-term objectives	**Measurable** steps toward the goal	“Client will score below 10 on the PHQ-9 within 12 weeks”
Interventions	What the counselor will do, tied to theory and evidence	“Behavioral activation and cognitive restructuring in weekly sessions”
Responsible parties	Who does what (client, counselor, other providers)	“Client will complete activity log daily”
Target dates and review dates	When objectives should be met and when the plan will be reviewed	“Review at session 6 and session 12”
Strengths and resources	What the client brings	“Supportive sister; previous recovery from depression”
Barriers	What may get in the way	“Night-shift work schedule; transportation”

SMART Objectives

Letter	Meaning	Weak	Strong
S	Specific	"Feel better"	"Attend one social activity a week"
M	Measurable	"Be less anxious"	"Reduce GAD-7 score from 16 to below 10"
A	Achievable	"Never feel anxious again"	"Use a coping skill in 4 of 5 anxious moments, by self-report"
R	Relevant	Unrelated to the client's concern	Tied to the client's stated goal
T	Time-bound	"Eventually"	"Within 8 weeks"

NBCC's 2027 task wording spells the acronym Specific, Measurable, Attainable, Realistic, and Timely; the meaning is the same.

Goals in the client's words: de Shazer's solution-focused criteria for **well-formed goals**: small, concrete, behavioral, stated as the **presence** of something (not the absence), important to the client, and within the client's control.

- Weak: "Stop fighting with my son."
- Strong: "Have three calm conversations with my son this week about his day."

Collaboration (3A.9)

- The client helps choose goals. Goals the counselor sets alone are less likely to be met, and they conflict with **autonomy**.
- Explore the client's **readiness** (stages of change; Chapter 5). A precontemplative client's first goal may be exploring whether change is wanted.

- Respect **cultural** values about goals, family involvement, and help-seeking.
- With minors, involve parents while respecting the young client's voice.

Sequencing Priorities

When a client has several problems, order them:

1. **Safety** (suicide, violence, abuse, medical danger)
2. **Basic needs and stabilization** (housing, food, acute substance withdrawal, severe symptoms)
3. **Problems that block other work** (e.g., substance use that undermines therapy)
4. **The client's most pressing concerns**
5. **Longer-term growth and prevention**

This mirrors Maslow and the crisis principle from Chapter 6.

Part 2 — Strengths and Barriers (3A.4)

Strengths (3A.4)	Barriers (3A.4)
Support system	Lack of transportation, childcare, or money
Past successes and coping skills	Work schedules
Motivation and readiness	Low motivation; ambivalence
Faith and community	Stigma; cultural mistrust
Education, intelligence, humor	Limited literacy or language access
Health	Co-occurring medical or mental health conditions
Insight	Substance use
	Unsafe living situation; family resistance

Strengths-based practice builds the plan on what's already working (SFBT, Chapter 2; the strengths perspective in social work, Dennis Saleebey). Barriers are

named early so the plan can address them, for example with telehealth for transportation problems or a sliding scale for cost.

Part 3 — Referral (3B.1)

Referring to a Different Level of Care (3B.1)

Use the level-of-care options from Chapter 19 (outpatient, IOP, PHP, residential, inpatient), choosing the **least restrictive** level that's safe and effective. **Step up** when symptoms worsen or outpatient care isn't enough; **step down** as the client stabilizes.

Referring for Concurrent Treatment (3B.1)

Clients often need more than one provider at the same time:

Need	Refer to
Medication evaluation	Psychiatrist, psychiatric nurse practitioner, or primary care provider
Medical evaluation	Primary care or specialist (Chapter 16)
Substance use treatment	Specialized program (Chapter 9)
Eating disorder	Medical provider and dietitian (Chapter 16)
Specialized trauma, sex, or couples therapy	Specialist with the needed training
Legal, financial, housing	Appropriate community resources
Case management	Social services

How to refer well: - Explain **why**, and involve the client in the decision. - Offer **several options** where possible. - Get a **signed release of information** before sharing information. - **Follow up** to confirm the client connected. - Avoid abandonment: when appropriate, continue care until the transition is complete (Chapter 17).

Part 4 — Collaboration and Documentation (5.3)

Working with Other Providers and Supports

- With a **release of information**, coordinate with physicians, psychiatrists, schools, probation officers, and family.
- Share only what the purpose requires (ACA B.2.e, minimal disclosure; Chapter 17). HIPAA's "minimum necessary" rule doesn't apply to disclosures for treatment or disclosures the client has authorized, but the ethical standard of sharing only what's needed still applies.
- **Integrated care** teams (behavioral health in primary care) improve outcomes for many clients.
- Involve **natural supports** (family, faith community, friends) when the client wants that.

Progress Note Formats

Format	Sections
SOAP	**S**ubjective (client's report), **O**bjective (counselor's observations, test scores), **A**ssessment (counselor's clinical impression and progress), **P**lan (next steps)
DAP	**D**ata (subjective and objective), **A**ssessment, **P**lan
BIRP	**B**ehavior (client's presentation), **I**ntervention (what the counselor did), **R**esponse (how the client responded), **P**lan
GIRP	**G**oal, **I**ntervention, **R**esponse, **P**lan

Exam favorite: in SOAP, the client's own words go in **S**; what the counselor observes or measures (affect, MSE findings, PHQ-9 score) goes in **O**.

Reports

Write reports that are clear, relevant to the reader's purpose, free of jargon, and limited to what's necessary. Note limitations of the data and any reservations about test validity (ACA E.9.a). Distinguish facts from clinical opinions.

Part 5 — Monitoring and Revising the Plan (3A.5, 3A.10, 3B.7, 4.11)

Measuring Progress

- Use **baseline** and repeated measures (PHQ-9, GAD-7, OQ-45, ORS/SRS; Chapters 18, 20).
- **Review progress with the client** (3B.7) at planned intervals: "Here's where your scores were when we started, and here's where they are now. What do you notice?"
- Use **assessment results** to help the client make decisions (3A.5), such as whether to add group therapy or step down to biweekly sessions.

When Progress Stalls (3A.10)

Don't keep doing the same thing, and don't switch approaches without discussion. Instead:

1. **Review the plan with the client.**
2. **Check the alliance** (goals, tasks, bond; Chapter 4). A stalled plan is often an alliance problem.
3. **Reassess**: missed diagnosis, substance use, medical issue, trauma, safety concern, new stressor?
4. **Check readiness**: has the client's stage of change shifted?
5. **Consider adjustments**: different interventions, frequency, modality, level of care, or referral for concurrent treatment.
6. **Consult** or seek supervision.

Adherence (4.11)

What older outlines called “treatment plan compliance” is increasingly termed **adherence**, which emphasizes collaboration over obedience.

- Explain the **rationale** for homework, attendance, and medication.
- Explore **barriers** to adherence without judgment.
- Use **motivational interviewing** when ambivalence is the barrier (Chapter 5).
- Respect the client’s right to refuse treatment, and explain the possible consequences (ACA A.2.b).

Part 6 — Termination (3B.9, 3B.3, 3B.2)

Types of Termination

Type	Description
Planned / natural	Goals are met; termination is discussed in advance
Client-initiated	The client decides to end, sometimes abruptly
Counselor-initiated	The counselor ends services (e.g., client isn’t benefiting, counselor leaving, competence limits, safety threat)
Premature (dropout)	The client stops before goals are met; meta-analytic estimates put dropout around **one in five** clients (Swift & Greenberg, 2012)
Forced	External limits: insurance, agency policy, a move, end of a training placement

The Termination Process

Raise termination early. Introduce the idea in informed consent and revisit it as goals are approached, not in the last session.

Tasks in the final phase: 1. **Review progress** and credit the client's work. 2. **Process feelings** about ending: pride, sadness, anxiety, sometimes anger. Endings can stir up past losses. 3. **Consolidate and generalize** learning (3B.3): what helped, and how to keep using it. 4. **Relapse prevention**: warning signs, coping plan, support system, what to do if problems return. 5. **Plan for the future**: referrals, support groups, booster sessions. 6. **Discuss the open door**: how the client can return if needed.

Tapering sessions (weekly to biweekly to monthly) can ease the transition.

Follow-Up After Discharge (3B.2)

- Follow-up contacts or **booster sessions** support maintenance.
- After hospital or higher-level discharge, the **first days and weeks** are a period of elevated suicide risk. A prompt follow-up appointment, an updated safety plan (Chapter 6), and caring contacts are important.
- A **discharge summary** documents the reason for services, the treatment provided, progress, the status at discharge, and recommendations.

Group Membership Transitions (4.5)

See Chapter 8: prepare new members and the group, encourage departing members to say goodbye, and process endings with the whole group.

Ethical Reminders

- **Pretermination counseling** and referrals (ACA A.11.c).
- **No abandonment** (A.12).
- Termination for nonpayment only after the fee policy was explained and with referrals; never in the middle of a crisis.
- When a counselor is in danger from a client, termination is permitted (A.11.c), with appropriate safety steps.

Continuity of Care

2027 task map: 3B.1 Refer clients and/or family members to community resources or to specialists · 3B.2 Develop an aftercare plan · 3B.3 Discuss with clients how they will maintain progress · 3B.4 Develop a discharge plan · 3B.5 Develop a relapse prevention plan · 3B.6 Develop a plan for disruption of service · 3B.7 Engage with client in review of progress toward treatment goals at planned intervals · 3B.8 Evaluate treatment effectiveness and client response · 3B.9 Collaborate with client to determine counseling termination criteria

Care That Outlasts the Session

A treatment plan describes what happens while the client is in your office. Continuity of care describes what happens at every seam: between you and another provider, between one level of care and the next, between this week and the week you are sick, and between the last session and the rest of the client's life.

The 2027 outlines list nine tasks here. They share one test: **does the client stay connected to help, and does the help keep working, when something changes?** Wrong answers leave a gap: a phone number with no appointment, a termination with no plan, a treatment never checked, a practice with no plan for the day the counselor cannot show up.

This section builds on the treatment-planning material on goals, referral, and termination types, and on the ethics chapter's coverage of abandonment.

Part 1 — Who Owns It

Owner	What it owns
ACA A.1.c Counseling Plans	Counselor and client devise plans jointly and **regularly review and revise** them for viability and effectiveness

Owner	What it owns
ACA A.2.b Types of Information Needed	Informed consent covers goals, procedures, and **continuation of services upon the counselor's incapacitation or death**
ACA A.11.c Appropriate Termination	Terminate when the client **no longer needs help, is not likely to benefit, or is being harmed**; provide **pretermination counseling** and referrals
ACA A.11.d Appropriate Transfer of Services	Complete clinical and administrative processes and keep **open communication** with client and new provider
ACA A.12 Abandonment and Client Neglect	Do not abandon or neglect clients; arrange continuation during **vacations, illness, and after termination**
ACA A.1.d Support Network Involvement	With client consent, enlist family, friends, and community supports
ACA B.1.c Respect for Confidentiality	Disclose client information only with **appropriate consent** or sound legal or ethical justification
ACA B.6.e Client Access	Clients may have reasonable access to their records on request
ACA B.6.g Disclosure or Transfer	**Written permission** before disclosing or transferring records to third parties
ACA B.6.h Storage and Disposal After Termination	Keep records after services end for reasonable future access, per law; dispose of them securely
ACA B.6.i Reasonable Precautions	Protect confidentiality if the counselor ends practice, is incapacitated, or dies; appoint a **records custodian** when appropriate
ACA C.2.d Monitor Effectiveness	Counselors **continually monitor their effectiveness** and take steps to improve

Owner	What it owns
ACA C.2.g Impairment	Refrain from providing services when physical, mental, or emotional problems are likely to harm clients
ACA C.2.h Counselor Incapacitation, Death, Retirement, or Termination of Practice	A **plan to transfer clients and records** to an identified colleague or **records custodian** (the professional will)
ACA H.2.a Informed Consent and Disclosure	For distance counseling: **possibility of technology failure and alternate methods of service delivery**, emergency procedures
HIPAA 45 CFR 164.308(a)(7)	Security Rule **contingency plan**: data backup, disaster recovery, emergency-mode operation
988, 211, 311	Three different phone lines: **crisis**, **community resources**, **city services**
Marlatt & Gordon (1985)	**Relapse prevention** model: high-risk situations, apparently irrelevant decisions, **abstinence violation effect**, lapse vs. relapse
Witkiewitz & Marlatt (2004)	Revised **dynamic** model of relapse
Leon Festinger (1957)	**Cognitive dissonance**: discomfort from conflicting beliefs or actions
Stokes & Baer (1977)	**Generalization**: gains do not transfer automatically; they must be programmed
Lewis et al. (2019)	**Measurement-based care**: routine measures, reviewed with the client, used to adjust treatment
Michael Lambert (2010)	**Routine outcome monitoring** with the OQ-45; feedback on **not-on-track** clients reduces deterioration

Owner	What it owns
Miller & Duncan (Miller et al., 2003; Duncan et al., 2003)	**PCOMS**: the Outcome Rating Scale (ORS) and Session Rating Scale (SRS)
Jacobson & Truax (1991)	**Reliable change index** and **clinically significant change**

Part 2 — Referral to Resources and Specialists (3B. 1)

Active vs. Passive Referral

The treatment-planning chapter covers *when* to refer. This part covers *how*. The exam keys the referral that makes the connection actually happen.

	Passive referral	Active referral
What the counselor does	Hands over a name, list, or number	With consent, makes contact, books or helps book the appointment, sends records with a release, confirms attendance
Who carries the burden	The client	Shared
Typical outcome	Lower follow-through	Higher follow-through

A **warm handoff** is the strongest form: the counselor introduces the client to the new provider directly, in person or on a joint call. Active referral to mutual-help groups increases attendance compared with simply suggesting them (Timko et al., 2006).

The key distinction: an active referral **closes the loop**. The counselor knows whether the client arrived. When the referral is a **transfer**, **A.11.d** adds the duty to complete the clinical and administrative steps and keep **open communication** with client and new provider.

Verifying the Resource

A referral to the wrong place is a dead end. Before referring, confirm:

- The service exists and is **accepting new clients**
- **Eligibility**, cost, waitlist, hours, and location or telehealth access
- **Language** and disability access
- **Competence** for this specific problem, including the evidence-based treatment the client needs

988, 211, 311

Line	What it is	Use it for
988	The national **Suicide & Crisis Lifeline** (call, text, or chat)	Suicidal thoughts, mental health or substance crisis, worried family members
211	**Community information and referral** for health and human services	Food, housing, utilities, transportation, childcare, local programs
311	**Non-emergency city services**, where a city offers it	Potholes, noise, trash, city information. Not a clinical resource

Put 988 in a crisis plan. Put 211 in a resource plan. Never treat 311 as a mental health line. Exact services vary by area, so check what your local 211 and 311 actually provide.

Referring Family Members

Task 3B.1 names **family members**, not only clients. A spouse, parent, or adult child may need support of their own: a caregiver group, a family education program, a

mutual-help group for families of people with substance use problems, or their own counselor.

- A family member who is not your client is referred **out**, not quietly added to your caseload.
- Your client's confidentiality still applies. Share clinical information with family only with the client's **consent** (B.1.c), unless a confidentiality exception applies (the ethics chapter).

Part 3 — Discharge Plans and Aftercare Plans (3B.4, 3B.2)

The exam treats these as two different documents. Know which is which.

	Discharge plan	Aftercare plan
Question it answers	How does the client **leave this service** safely?	How does the client **keep recovery going** after formal treatment?
Time frame	The **transition**: last days of this episode and first days after	The **months after**: ongoing, often open-ended
Starts	**At admission**	Drafted before discharge; owned by the client after
Core contents	Booked next appointment, signed release, records sent, medication bridge, crisis plan, who to call	Continuing services (step-down care, boosters), mutual-help or peer support, relapse prevention plan, daily routines, supports, return plan
Main risk it prevents	**Falling through the gap** between levels of care	**Losing gains** once support thins out

The key distinction: the **discharge plan** gets the client **to** the next provider. The **aftercare plan** says what the client **does** once the intensive help ends. A strong discharge plan includes the aftercare plan; it does not replace it.

Building a Discharge Plan

1. **Begin at admission**: where is this client going next, and what will they need there?
2. **Book the follow-up** before the client leaves: a date and time, not "call when you're ready."
3. **Secure the release** (B.6.g) so records reach the next provider.
4. **Bridge the gaps**: medication, transportation, authorization.
5. **Give a crisis plan** with 988 and named supports. Risk rises after leaving a higher level of care (the crisis chapter).
6. **Confirm the connection** after discharge.

An **aftercare plan** is written in the client's words. Beyond services and the relapse prevention plan (Part 5), it names the supports the client wants involved (A.1.d) and the daily structure that protects gains: sleep, work or school, routines.

Part 4 — Maintaining Progress (3B.3)

Gains made in counseling do not automatically last or spread to new settings. **Stokes & Baer** (1977) made this point about behavior change: **generalization must be planned**, not hoped for. That is the logic behind task 3B.3.

The Maintenance Conversation

Step	What it sounds like
Name what helped	"Which skills made the biggest difference?"
Attribute change to the client	"You did the exposures. The skill is yours." Clients who credit their own effort are better placed to keep using it
Plan practice	When, where, and how often the client will keep using each skill

Step	What it sounds like
Plan for new settings	Rehearse using skills at work, with family, under stress
Name warning signs	Early signs that things are slipping, and the first response
Taper and boost	Spread sessions out; schedule **booster sessions** after ending
Keep the door open	How to return if needed

Two wrong answers recur. **False reassurance** ("your gains will last") dismisses a real question. **Indefinite sessions "just in case"** foster dependency; when the client no longer needs counseling, **A.11.c** says to terminate. Tapering and booster sessions are the middle path.

Part 5 — Relapse Prevention (3B.5)

Marlatt & Gordon's Model (1985)

Relapse prevention is a cognitive-behavioral model developed by **G. Alan Marlatt and Judith Gordon** for addictive behaviors. It is now applied to many other problems. The model treats relapse as a **process** with identifiable steps, each a point where the client can intervene.

The sequence:

1. **Lifestyle imbalance**: too many "shoulds" and too few "wants" builds a desire for relief or indulgence.
2. **Apparently irrelevant decisions (AIDs)**, later renamed **seemingly irrelevant decisions (SIDs)**: small choices that look unrelated to use but move the client toward risk. Taking the route past the old bar. Keeping one bottle "for guests." Agreeing to watch the game at a friend's house where everyone drinks.
3. **High-risk situation**: a situation that threatens the client's sense of control. Marlatt's research found the most common are **negative emotional states**, **interpersonal conflict**, and **social pressure**.

4. **Coping response or not**:
   - **Coping** → increased **self-efficacy** → lower chance of relapse.
   - **No coping** → lower self-efficacy plus **positive outcome expectancies** ("a drink will calm me down") → **initial use**.
5. **The abstinence violation effect (AVE)** → full relapse, or a recovered lapse.

Lapse vs. Relapse

Lapse	Relapse
A single slip: one drink, one binge, one cigarette	A return to the old pattern
An event	A process
A **learning opportunity**	What the plan tries to prevent

The key distinction: a lapse does not have to become a relapse. **What the client thinks about the lapse decides which way it goes.**

The Abstinence Violation Effect

The **AVE** is the reaction that turns one slip into full relapse: "I've blown it, so I might as well keep going." It has two parts:

- **Dissonance**: the use conflicts with the client's identity as someone in recovery. The fastest way to end that conflict is to give up the identity ("I guess I'm not someone who can stay sober").
- **Personal attribution**: the client blames the slip on a stable, global, internal flaw ("I have no willpower") instead of a specific, changeable situation ("I was at a party without a plan").

The stronger the all-or-nothing belief, the stronger the AVE. Relapse prevention **reframes the lapse** as a mis-

take to learn from: what was the high-risk situation, what decision set it up, what coping was missing?

Festinger's Cognitive Dissonance (1957)

Leon Festinger, a social psychologist, described **cognitive dissonance**: the discomfort people feel when they hold two conflicting beliefs, or act against their own beliefs. People reduce it by changing the behavior, changing the belief, adding justifying thoughts, or minimizing the conflict.

Know the boundary. Marlatt drew on dissonance to explain *why* the AVE happens. But "cognitive dissonance" is Festinger's **general social psychology** term. When an item describes the "I've blown it, why stop now" reaction **inside a relapse prevention plan**, the answer is the **abstinence violation effect**, owned by **Marlatt & Gordon**.

Writing the Relapse Prevention Plan

Plan element	Example (Owen, six months sober)
High-risk situations	Arguments with his brother; Friday payday; work happy hours
Warning signs	Sleeping badly, isolating, romanticizing past drinking
Coping responses	Leave and call his sponsor; urge surfing; text a sober friend before Friday plans
If a lapse happens	Stop, call his sponsor within the hour, review what happened at the next session. One drink is information, not failure
Supports and crisis contacts	Sponsor, sister, counselor, 988

Witkiewitz & Marlatt (2004) later revised the model into a **dynamic** one: risk factors interact, and small changes can trigger sudden shifts. Mindfulness-based relapse prevention (Bowen et al., 2010) builds on Marlatt's

urge surfing: watching a craving rise and fall without acting on it.

Part 6 — Planning for Disruption of Service (3B.6)

Counselors get sick, take leave, move, retire, and die. Offices flood. Video platforms go down. Task 3B.6 asks for a **plan made before** the disruption, not an improvised response after.

The Standards

- **A.12**: no abandonment. Arrange **continuation of treatment** during vacations and illness. **A.2.b**: clients learn at intake how services continue if the counselor is **incapacitated or dies**.
- **C.2.h**: transfer **clients and records** to an **identified colleague or records custodian**. This is the **professional will**. **B.6.i**: protect confidentiality when practice ends.
- **B.6.h**: keep records for **reasonable future access**, for the period law requires, then dispose of them securely. Retention periods vary. Check your state.

The Professional Will

A professional will is a written plan, separate from a personal will, that tells a named colleague exactly what to do. It names a **records custodian** (and a backup) and gives them what they need to act: where records, devices, and **passwords** are; the current **client list** and contact information; a **notification script**; a **referral list**; steps to cancel appointments and close billing; who else to notify (malpractice carrier, licensing board); and how records will be kept and later destroyed (B.6.h).

Client Access Is Not a Continuity Plan

Under **B.6.e**, clients may request reasonable access to their records, and counselors provide it unless there is compelling evidence that access would cause harm. That is a client right. It is **not** a plan for disruption. Handing every client a full record does not arrange coverage or transfer care. Continuity → **C.2.h**, not B.6.e.

Types of Disruption

Disruption	The plan
Planned leave or vacation	Advance notice; a named **covering counselor**; crisis instructions; high-risk clients seen or contacted before you leave
Illness, incapacity, or death	Step back when health would impair services (C.2.g); the professional will is activated; the custodian notifies clients, protects records, and arranges transfer (C.2.h, B.6.i)
Relocation (counselor or client) or **closing practice**	Notice in time to process the ending; transfers with releases (A.11.d); records kept per B.6.h. Continuing across state lines depends on licensure: verify current rules, including Counseling Compact status in both states
Disaster (fire, flood, storm, pandemic)	Off-site **backup** of records; a way to reach clients; emergency-mode operation. HIPAA's Security Rule requires a **contingency plan** (45 CFR 164.308(a)(7))
Platform or internet outage	Agreed at intake: the **backup method** (usually phone), who calls whom, and what to do in an emergency if contact is lost (**H. 2.a**)

The key distinction: the exam rewards the plan that exists **before** the crisis and that **a named person** can carry out. “Clients will find someone else” is abandonment (A. 12).

Part 7 — Reviewing Progress at Planned Intervals (3B.7)

Measurement-Based Care

Measurement-based care (Lewis et al., 2019) has three parts:

1. Give a **standardized measure** routinely, often every session.
2. **Review the result with the client** at the time it is collected.
3. **Use the data** to keep or change the plan.

A measure filed without being discussed is paperwork, not measurement-based care.

Routine outcome monitoring is the measuring part. **Michael Lambert’s** work with the **OQ-45** (Lambert, 2010) showed that when counselors get feedback that a client is **not on track**, outcomes improve for those at-risk clients, and fewer of them deteriorate. **Miller & Duncan’s PCOMS** (Miller et al., 2003) uses two brief scales: the **Outcome Rating Scale (ORS)** for how the client is doing, and the **Session Rating Scale (SRS)** for how the client experienced the session and the alliance.

Why Planned Intervals

- **A.1.c**: counselor and client **regularly review and revise** the plan to check that it is still viable and effective.
- **C.2.d**: counselors **continually monitor their effectiveness** and take steps to improve. Routine measures are one concrete way to do it.

- Counselors' unaided judgment misses most clients who are getting worse (Hannan et al., 2005). Scheduled review catches what impressions miss.

Set review points **in the plan itself**, plus any time something major changes. Review is done **with** the client: show the scores, ask what the client notices and what has helped, ask about the relationship, then decide together.

Part 8 — Evaluating Effectiveness and Client Response (3B.8)

Review asks "what are the numbers?" Evaluation asks "**is this treatment working for this client, and what do we do about it?**"

Reliable and Clinically Significant Change

Jacobson & Truax (1991) gave two tests:

Concept	Meaning
Reliable change	The change in score is **larger than measurement error** would produce (the **reliable change index**, RCI). A small score shift may be noise
Clinically significant change	Reliable change **plus** moving from the clinical range into the **functional (nonclinical) range**
Reliable deterioration	A reliable change in the **worse** direction

Use each measure's published reliable-change value and cutoff, not an eyeball judgment.

Four Possible Answers

Pattern	Meaning	Response
Clinically significant improvement	The client is better and in the functional range	Consolidate; move toward the agreed termination criteria (Part 9)

Pattern	Meaning	Response
Reliable improvement, still in clinical range	Working, not finished	Continue; check whether goals or dose need adjusting
No reliable change after an adequate trial	Not working yet	Review with the client; check alliance, diagnosis, dose, barriers (the treatment-planning chapter); revise the plan
Reliable deterioration	Getting worse	**Act now**: check safety first, discuss with the client, consult, consider a **higher level of care** or referral

Evaluate **client response** broadly: symptoms, functioning, goals in the client's own terms, engagement, and the client's view of the work. A score that improves while the client says she feels worse needs a conversation.

Revision and Level of Care

- **Revise the plan** when evaluation shows it is not working (A.1.c). Change with the client, not to the client.
- **Step up** when deterioration or severity exceeds what this level can safely manage.
- **Step down** when gains are stable and less intensive care will hold them, with a discharge plan (Part 3).
- **Refer or terminate** when the client is **not likely to benefit** from continued counseling with you (A.11.c), with pretermination counseling and referrals.

The key distinction: an ineffective plan is **revised or escalated**, never continued unchanged and never dropped without a referral.

Part 9 — Collaborating on Termination Criteria (3B.9)

Termination criteria answer one question: **how will we both know we are done?** Task 3B.9 says the counselor sets them **with** the client. The rewarded timing is **early**, as part of informed consent and initial planning, then revisited at each review.

Three standards support this. **A.2.b** puts goals and the nature of services in informed consent. **A.1.c** makes the plan a joint product. **A.11.c** defines when termination is appropriate; criteria agreed in advance make that standard visible to both people. Criteria set late, or by the counselor alone, make ending feel like rejection.

Writing Termination Criteria

Good criteria are **observable** and tied to the plan's goals:

Type	Example
Measure-based	GAD-7 in the mild range or below at three sessions in a row
Functional	Back at work full time for a month
Client-defined	"I can drive on the highway without pulling over."
Skill-based	Uses the relapse prevention plan on her own when warning signs appear
Maintenance	Gains hold through two biweekly and one monthly session

Also agree on the **other ways counseling can end**, because A.11.c names them: the client is **not benefiting** (with referral), continued counseling is **harmful**, or a **fee arrangement** is not kept.

When the Criteria Are Met

Review progress against the criteria, process the ending, finalize the maintenance, aftercare, and relapse preven-

tion plans, taper, and confirm the open door. If the client wants to continue anyway, explore why. A new goal can justify continuing. Fear of ending alone does not; work on that fear as part of termination.

What the Exam Rewards / What It Punishes

Rewards: - Active referrals that close the loop: consent, release, booked appointment, confirmation - Discharge planning that starts at admission; a booked follow-up and crisis plan at every step-down - Aftercare and maintenance plans built with the client: skills, practice, boosters, open door - Relapse prevention that names high-risk situations and SIDs and reframes a lapse as learning - A professional will with a named records custodian, made before anything goes wrong - A backup contact method agreed at intake for telehealth - Routine measures reviewed with the client; plans revised when data say so - Termination criteria set early, together, in observable terms

Punishes: - Lists, pamphlets, and “call when you’re ready” - Discharge planning on the last day - False reassurance; indefinite sessions with no clinical reason - Treating a lapse as proof of failure - “Clients will find someone else”; no coverage during leave - Measures collected and never discussed - Continuing a plan unchanged when the client is not improving or is getting worse - Termination criteria announced by the counselor near the end

Swap-Pair Drill

If you see…	It’s…	Not…
Counselor books the appointment and confirms attendance	Active referral	Passive referral
Booked follow-up, release, crisis plan at step-down	Discharge plan	Aftercare plan

If you see…	It's…	Not…
Ongoing supports and practice after treatment ends	Aftercare plan	Discharge plan
Planning so skills carry into new settings	Generalization (Stokes & Baer)	Reassurance
Small choice that sets up risk ("I'll just drive past")	Apparently/seemingly irrelevant decision (Marlatt & Gordon)	High-risk situation
"I slipped once, so I've blown it"	Abstinence violation effect (Marlatt & Gordon)	Cognitive dissonance (Festinger)
One slip	Lapse	Relapse
Plan for counselor's death or incapacity	Professional will (ACA C.2.h)	Client access to records (B.6.e)
Leaving clients without coverage during illness	Abandonment (A.12)	Appropriate termination (A.11.c)
Backup method if the video platform fails	Distance-counseling informed consent (H. 2.a)	Professional will
Data backup and disaster recovery	HIPAA contingency plan, 164.308(a)(7)	ACA C.2.h
Crisis line	988	211
Housing and food resources	211	311
Score change bigger than measurement error	Reliable change (Jacobson & Truax)	Clinically significant change
Plan reviewed and revised with the client	ACA A.1.c	ACA A.11.c
End point agreed at the start	Collaborative termination criteria	Counselor's decision at the last session

Quick Checks

1. Danielle is leaving a partial hospitalization program next week. Which step most reduces the chance she will fall through the gap? A. Giving her a printed list of outpa-

tient counselors in her area B. Booking her first outpatient appointment and sending her records with her signed release C. Telling her to call the program if she needs anything D. Writing a detailed discharge summary for her chart

2. A counselor wants a structure for asking clients each session how they are doing and how the session went, using two brief scales. The counselor is using: A. The reliable change index B. The OQ-45 alone C. A professional will D. PCOMS (ORS and SRS)

3. Grant, three months abstinent from alcohol, agrees to drive a coworker home and takes the route past his old bar “because it’s faster.” In relapse prevention terms, this choice is: A. An apparently irrelevant decision B. The abstinence violation effect C. A lapse D. Cognitive dissonance

4. A counselor will be on medical leave for six weeks. The best plan is to: A. Close all cases and ask clients to call when she returns B. Give clients copies of their records so they can find someone new C. Tell clients in advance, name a covering counselor, and review crisis plans with higher-risk clients D. Continue seeing clients by phone from the hospital

5. Which belongs in an aftercare plan rather than only a discharge plan? A. The date and time of the first appointment with the next provider B. The signed release to send records C. Weekly mutual-help meetings, a relapse prevention plan, and how to return for help D. A bridge supply of medication until the next prescriber visit

6. A client’s PHQ-9 score drops by two points after eight sessions. Before calling this improvement, the counselor should check whether the change: A. Exceeds the measure’s reliable change value B. Matches the counselor’s clinical impression C. Was noticed by the client’s family D. Appears on the SRS

7. A parent of an adult client asks the counselor how she can cope with her son’s drinking. The counselor’s best re-

sponse is to: A. Share the son’s treatment details so she understands the situation B. Add the mother to the son’s sessions without asking him C. Tell her the counselor cannot discuss anything at all D. Without disclosing anything about her son, refer her to a family support group or a counselor of her own

8. Which ACA standard requires counselors to plan for transferring clients and records to an identified colleague or records custodian if they die or become incapacitated? A. A.11.c B. C.2.h C. B.6.e D. A.1.c

9. At intake, Mei-Ling and her counselor agree that counseling will end when her panic attacks have stopped for six weeks and she can ride the train to work. This is an example of: A. Collaboratively setting termination criteria early B. Premature termination C. A discharge plan D. Passive referral

10. A client’s outcome scores have reliably worsened over four sessions. The counselor’s first step is to: A. Continue the current plan for another month B. Terminate because the client is not benefiting C. Wait until the scheduled six-session review D. Assess safety and discuss the change with the client

11. Near the end of counseling, Felipe uses assertiveness skills easily in role-plays with his counselor but has not tried them anywhere else. The best next step for maintaining progress is to: A. Tell him the skills will carry over on their own once he feels ready B. Plan specific practice at work and with family, then review how it went C. Keep role-playing in session until he never makes a mistake D. Refer him to 211

12. A counselor tells a telehealth client at intake, “If the video drops, I’ll call your cell within two minutes.” This practice is grounded most directly in: A. ACA C.2.h B. ACA A.11.d C. ACA H.2.a D. HIPAA 164.524

Answers and Rationales

1 — B. Owner: discharge planning with linkage (A.11.d). A booked appointment and a release get her to the next provider. - A: Passive referral; no linkage. - C: Puts the burden on the client; no confirmed next step. - D: Documentation, not a transition plan.

2 — D. Owner: Miller & Duncan (PCOMS). The ORS tracks outcome; the SRS tracks the session and alliance. - A: Jacobson & Truax; a way to judge score change, not a feedback system. - B: Lambert's OQ-45 is a single outcome measure; it does not rate the session. - C: C.2.h; a continuity plan for counselor incapacity or death.

3 — A. Owner: Marlatt & Gordon (apparently, later "seemingly," irrelevant decisions). A small choice sets up a high-risk situation before any use. - B: Marlatt & Gordon's AVE follows a lapse; no use has occurred. - C: A lapse is actual use. - D: Festinger's general social psychology concept, not the relapse prevention term.

4 — C. Owner: ACA A.12. Arrange continuation of care during illness. - A: Leaving clients without coverage is abandonment (A.12). - B: Client access (B.6.e) is a right, not a coverage plan. - D: Practicing while seriously ill risks impairment (C.2.g) and is not a plan.

5 — C. Owner: aftercare planning. Ongoing supports, relapse prevention, and a return plan sustain recovery after treatment. - A: Discharge plan; linkage to the next provider. - B: Discharge plan; B.6.g release for the transition. - D: Discharge plan; bridging the transition.

6 — A. Owner: Jacobson & Truax (reliable change index). Small changes may be measurement error. - B: Clinical impression misses change (Hannan et al., 2005); it does not replace a reliability check. - C: Family observation is useful context, not a test of reliable change. - D: The SRS (Miller & Duncan) rates the session, not symptom change.

7 — D. Owner: 3B.1 referral of family members, with B.1.c confidentiality. The mother gets her own support; the

son's information stays protected. - A: Disclosure without the son's consent violates B.1.c. - B: Changes the son's treatment without his input; plans are devised jointly (A.1.c). - C: Confidentiality (B.1.c) limits what is shared about the son; it does not bar helping the mother find resources.

8 — B. Owner: ACA C.2.h, the professional will. - A: A.11.c governs appropriate termination. - C: B.6.e governs client access to records. - D: A.1.c governs jointly devised and reviewed counseling plans.

9 — A. Owner: collaborative termination criteria (3B.9; A.11.c, A.2.b). Observable criteria agreed at the start. - B: Premature termination is ending before goals are met. - C: A discharge plan covers the transition out of a service. - D: Passive referral is handing over a resource without linkage.

10 — D. Owner: Jacobson & Truax (reliable deterioration) with measurement-based care (Lewis et al., 2019). Deterioration calls for prompt action, safety first. - A: Continuing an ineffective plan unchanged violates A.1.c. - B: Termination without pretermination counseling and referral is not appropriate (A.11.c, A.12). - C: Planned intervals are the minimum, not a reason to delay.

11 — B. Owner: Stokes & Baer (generalization must be programmed). Planned practice in real settings carries gains beyond the office. - A: False reassurance; Stokes & Baer showed transfer does not happen on its own. - C: Office-only practice does not program transfer, and continuing without clinical need fosters dependency (A.11.c). - D: 211 is a community-resource line, not a maintenance plan.

12 — C. Owner: ACA H.2.a. Distance-counseling informed consent covers technology failure and alternate service delivery. - A: C.2.h is the professional will. - B: A.11.d governs transfer to another provider. - D: 164.524 is the HIPAA right of access to records.

What the Exam Rewards / What It Punishes

Rewards: - Collaborative goals in measurable, behavioral terms - Safety first in sequencing - Naming strengths and barriers early - Referring with releases, options, and follow-up - Reviewing progress with the client using data - Revisiting the alliance and reassessing when progress stalls - Introducing termination early; relapse prevention; open door - Prompt follow-up after discharge from a higher level of care

Punishes: - Counselor-written goals the client didn't help choose - Vague goals ("feel better") - Treating long-term growth before safety and stabilization - Continuing an ineffective plan unchanged - Switching approaches without discussing it with the client - Raising termination for the first time in the last session - Ending services without referrals

Swap-Pair Drill

If you see…	It's…	Not…
Treatment plan: broad outcome	Long-term goal	Short-term objective
Treatment plan: measurable step	Short-term objective	Long-term goal
Small, concrete, presence of something	Well-formed goal (de Shazer)	SMART
Client's own words in the note	S in SOAP	O
Counselor-observed affect; test score	O in SOAP	S
Behavior, intervention, response, plan	BIRP	DAP
Treatment progress stalls	Review plan, check alliance, reassess	Repeat the same intervention
Client's collaborative follow-through with treatment	Adherence	Compliance (older term)

If you see…	It's…	Not…
Termination: raise the ending early	Planned termination	Last-session announcement
Reducing frequency before ending	Tapering	Abrupt termination
High-risk period after hospital discharge	Prompt follow-up	Discharge without plan
Strengths perspective	Saleebey	Maslow

Quick Checks

1. Which is the best short-term objective? A. "Client will be happier." B. "Client will reduce panic attacks from 5 a week to 1 or fewer within 8 weeks, by self-report log." C. "Client will understand his anxiety." D. "Client will never panic again."

2. A client's goal is "stop yelling at my kids." Rewritten as a well-formed goal, it would be: A. "Never yell again." B. "Use a calm voice and take a 5-minute break when frustrated with the kids, at least 4 times this week." C. "Be a better parent." D. "Understand why I yell."

3. A client presents with suicidal ideation with a plan, alcohol use, marital conflict, and career dissatisfaction. The first treatment priority is: A. Career exploration B. Marital counseling C. Safety D. Alcohol education

4. In SOAP notes, the client's statement "I haven't slept more than four hours all week" belongs in: A. Subjective B. Objective C. Assessment D. Plan

5. After 10 sessions, a client's PHQ-9 scores haven't changed. The counselor's best next step is to: A. Continue the same plan B. Terminate counseling C. Review progress with the client, check the alliance, and reassess D. Switch to a new theory without telling the client

6. A counselor refers a client to a psychiatrist for a medication evaluation. Before sharing clinical information with

the psychiatrist, the counselor needs: A. Nothing; providers can always share B. A signed release of information from the client C. Permission from the counselor's supervisor D. A court order

7. When should termination first be discussed? A. In the last session B. Only when the client brings it up C. Early, beginning with informed consent, and revisited as goals approach D. After the client stops attending

8. A client who just completed a 5-day inpatient stay for a suicide attempt is being discharged to outpatient counseling. The most important step is: A. Scheduling the first outpatient appointment within a month B. Arranging a prompt follow-up appointment and safety planning, since risk is elevated after discharge C. Waiting for the client to call D. Closing the inpatient record

9. The older outline phrase "treatment plan compliance" is increasingly framed as adherence because it: A. Emphasizes obedience B. Emphasizes collaboration and the client's active role C. Requires a court order D. Applies only to medication

10. During termination, helping a client identify early warning signs and a plan for coping if symptoms return is called: A. Relapse prevention B. Tapering C. Pretermination counseling D. Step-down care

11. A client with severe depression isn't improving in weekly outpatient sessions and is struggling to function, but has no imminent risk. An appropriate option is: A. Inpatient hospitalization B. Stepping up to intensive outpatient or partial hospitalization, discussed with the client C. Terminating counseling D. Reducing sessions to monthly

12. Which is the best example of identifying a strength in a treatment plan? A. "Client has poor coping skills." B. "Client successfully stopped smoking 3 years ago and has a supportive sister." C. "Client is resistant." D. "Client misses appointments."

Answers and Rationales

1 — B. Specific, measurable, achievable, relevant, and time-bound. A and C: vague. D: unachievable.

2 — B. Behavioral, concrete, stated as the presence of something, and within the client's control (de Shazer). A: absence-based and unrealistic. C: vague. D: insight, not behavior.

3 — C. Safety first. A, B, D: come after safety is addressed.

4 — A. The client's report goes in Subjective. B: Objective holds counselor observations and measurements. C: Assessment is the counselor's clinical impression. D: Plan is next steps.

5 — C. Review, check the alliance, reassess, then adjust (3A.10). A: repeats what isn't working. B: abandons a client who still needs help. D: changes without collaboration.

6 — B. A signed release is required (ACA B.6.g). HIPAA itself permits disclosures for treatment, but the ethics code and many state laws require the client's written consent. A: incorrect. C and D: not what's required.

7 — C. Termination is part of informed consent and is revisited as goals approach. A, B, D: too late.

8 — B. Risk is elevated after discharge; prompt follow-up and safety planning matter (3B.2; Chapter 6). A and C: dangerous delays. D: administrative, not clinical.

9 — B. Adherence emphasizes collaboration. A: the opposite. C and D: incorrect.

10 — A. Planning for warning signs and coping is relapse prevention (3B.3). B: tapering reduces frequency. C: pretermination counseling is the broader ethical process. D: step-down is a level-of-care change.

11 — B. Step up to the least restrictive appropriate higher level, collaboratively (3B.1). A: no imminent risk. C and D: reduce care when more is needed.

12 — B. A specific, usable strength (3A.4). A, C, D: deficits or labels.

Audio Companion

Track	Covers	Target length
21.1	Plan It: goals, priorities, referral, notes, revision, termination	~4 min

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Chapter 22 — The Counselor as Instrument: Core Counseling Attributes

2027 task map: 1B.11 Practice self-reflection · 4.14 Use core counseling skills to facilitate treatment · 4.2 Explore the impact of the client's culture, context, values, and worldview · 4.4 Facilitate clients' ability to navigate interpersonal relationships · 4.5 Foster the group therapeutic process · 4.8 Integrate contextual and cultural influences into counselor–client relationship · CACREP: Counseling and Helping Relationships; Professional Counseling Orientation

Not a Domain, Still the Biggest Influence

Core counseling attributes are no longer a separate domain. In the 2027 specifications, they are knowledge and skills inside Provision of Counseling Interventions, and counselor self-awareness is its own subdomain (1B) of Professional Development & Counselor Self-Awareness. But they describe the ingredient that runs through every domain: **the counselor's own presence.** Research on what makes counseling work keeps pointing to the relationship and the counselor's qualities as strongly as to specific techniques (Chapter 18, common factors).

On the exam, these items often look like skills items (Chapter 4), but the right answer turns on an attribute: the response that is most **genuine**, most **empathic**, most **accepting**, or most **self-aware**.

Part 1 — Who Owns the Attributes

Owner	Contribution
Carl Rogers (1957)	**Six necessary and sufficient conditions** for therapeutic change (below)
Truax & Carkhuff	Operationalized Rogers's conditions as **accurate empathy, nonpossessive warmth, and genuineness**; Carkhuff's five-level empathy scale (Chapter 4)
Heinz Kohut	Self psychology; empathy as "**vicarious introspection**"
Norcross & Lambert	APA Task Forces on **evidence-based therapy relationships**: elements judged **demonstrably effective** include the **alliance**, **empathy**, **positive regard and affirmation**, and **collecting client feedback**; **congruence/genuineness**, **repairing alliance ruptures**, and **managing countertransference** were also found effective or probably effective
Jeffrey Hayes, Charles Gelso	Managing countertransference: **self-insight, self-integration, anxiety management, empathy, conceptualizing skills**
Derald Wing Sue; Hook et al.	Multicultural competence and cultural humility (Chapter 9)
Gerald Corey	Personal characteristics of effective counselors

Rogers's Six Necessary and Sufficient Conditions (1957)

1. Two people are in **psychological contact**.
2. The **client** is in a state of **incongruence** (vulnerable or anxious).
3. The **counselor** is **congruent** (integrated) in the relationship.

4. The counselor experiences **unconditional positive regard** for the client.
5. The counselor experiences **empathic understanding** of the client's internal frame of reference and tries to communicate it.
6. The client **perceives**, at least to a minimal degree, the counselor's empathy and unconditional positive regard.

Exam point: condition 6 means it isn't enough for the counselor to feel empathy and acceptance; **the client has to perceive them.**

Part 2 — The Attributes, One by One

Self-Awareness and Impact on Clients (1B.11)

Counselors bring their own history, values, biases, emotions, and blind spots into every session.

- **Countertransference**: the counselor's emotional reactions to the client, often from the counselor's own history (Chapter 5). Managing it is the counselor's responsibility, through self-reflection, **supervision**, **consultation**, and **personal counseling**.
- **The Johari Window** (Luft & Ingham; Chapter 3) shows how feedback from supervisors and colleagues reveals the counselor's **blind** area.
- **Power and privilege**: the counselor's role and identity carry power in the relationship. Awareness of that power is part of ethical practice (MSJCC; Chapter 9).
- **Self-care**: an exhausted counselor's presence suffers (Chapter 14, Chapter 17).

Exam pattern: when a counselor notices a strong personal reaction to a client, the scored answer is **reflect and seek supervision**, not disclose it to the client, act on it, or immediately refer the client out.

Genuineness and Congruence (4.14)

Term	Meaning
Genuineness	Being real and authentic, not hiding behind a professional role or façade
Congruence	The counselor's **inner experience matches** what the counselor outwardly communicates

The terms are close; Rogers often used them together. **Congruence doesn't mean saying everything you feel.** It means not pretending to feel something you don't, and sharing reactions when doing so serves the client (immediacy; Chapter 4).

Incongruent: a counselor who is bored says, "This is so interesting!" **Congruent:** a counselor notices drifting attention, refocuses, and, if useful, uses immediacy: "I notice we keep circling the same story. I wonder what we might be avoiding."

Empathic Attunement vs. Empathic Responding (4.14)

Attribute	What it is
Empathic attunement	The **internal** process of sensing and resonating with the client's experience, moment to moment, including what's unspoken
Empathic responding	**Communicating** that understanding back to the client, through reflection, tone, and nonverbal behavior

Attunement without responding leaves the client unaware they've been understood (Rogers's condition 6). Responding without attunement sounds formulaic.

Empathy vs. sympathy:

Empathy	Sympathy
Understanding the client's experience **from the client's frame of reference**	Feeling sorry **for** the client from one's own frame of reference
"It sounds like you feel completely alone with this."	"Oh, you poor thing. I feel so bad for you."

Empathy is the scored answer. Sympathy, reassurance, and identification ("I know exactly how you feel") are distractors.

Positive Regard and the Nonjudgmental Stance (4.14)

- **Unconditional positive regard (UPR):** valuing the client as a person, **without conditions**. It doesn't mean approving of everything the client does.
- **Nonjudgmental stance:** accepting the client's experience and choices without moral evaluation, while still being able to address harmful behavior.
- **Warmth** (Truax & Carkhuff's "nonpossessive warmth"): caring that doesn't create dependence.

Exam pattern: a client discloses something the counselor finds distasteful (an affair, a past crime, a choice that conflicts with the counselor's values). The scored response accepts the **person**, explores the client's experience, and, where relevant, addresses consequences or safety, without lecturing or showing disapproval.

Conflict Tolerance and Resolution (4.4)

Effective counselors can **stay present with conflict**: a client's anger at the counselor, disagreement, criticism, or tension in a couple or group.

- **Don't avoid, appease, or retaliate.**
- **Name the conflict** nondefensively and explore it.
- **Repair alliance ruptures** (Safran & Muran; Chapter 4): a well-handled rupture often strengthens the relationship.

- In couples, families, and groups, **model** respectful conflict (Chapter 7, Chapter 8).

Exam pattern: “You’re not helping me at all.” The scored response: “It sounds like you’re frustrated with how counseling is going. I’d really like to understand what’s not working for you.” Wrong answers defend credentials, offer reassurance, or refer the client away.

Gender, Multicultural Sensitivity, and Respect for Diversity (4.2, 4.8)

Covered in depth in Chapters 9 and 15. The attribute side: - **Cultural humility**: curiosity, openness, and awareness of the limits of one’s knowledge. - **Awareness of one’s own gender-role and cultural assumptions**, and their effect on the client. - **Using the client’s name, pronouns, and language** for their identity. - **Broaching** culture respectfully. - **Not imposing values** (ACA A.4.b).

Fostering Group Therapeutic Factors (4.5)

The leader’s attributes shape the group: **modeling** genuineness and feedback, building **cohesiveness** (Yalom’s group equivalent of the alliance), **instilling hope**, and protecting members (Chapter 8).

Foundational Listening, Attending, and Reflecting (4.14)

The microskills of Chapter 4 express the attributes: **attending** shows presence, **reflecting feelings** shows empathy, **paraphrasing and summarizing** show understanding, and **comfortable silence** shows acceptance.

Part 3 — Personal Characteristics of Effective Counselors

Drawing on Corey and others, effective counselors typically: - Have an identity of their own and know what they value - Respect and appreciate themselves, and can give and receive help - Are open to change and to feedback -

Make choices that are life-oriented - Are authentic, sincere, and honest - Have a sense of humor - Make mistakes and are willing to admit them - Live in the present - Appreciate the influence of culture - Have a sincere interest in others' welfare - Become deeply involved in their work and derive meaning from it, while keeping healthy boundaries - Maintain healthy relationships and self-care

Personal counseling for counselors is widely encouraged (though not universally required) as a way to increase self-awareness and manage countertransference.

Part 4 — How Attribute Items Look on the Exam

Most items give you several plausible responses. Ask: **which one best shows the attribute?**

If the response…	It shows…	Watch for distractors that…
Reflects the client's feeling from the client's view	Empathy	Sympathize, reassure, or say "I know how you feel"
Matches the counselor's real reaction, used for the client's benefit	Congruence	Fake enthusiasm, or vent the counselor's feelings
Accepts the person while exploring the behavior	Positive regard, nonjudgment	Lecture, moralize, or show disgust
Explores the client's anger nondefensively	Conflict tolerance	Defend, apologize excessively, or end the session
Seeks supervision about a strong reaction	Self-awareness	Disclose it to the client, or refer out immediately
Invites the client's cultural meaning	Cultural humility	Assume, stereotype, or ignore culture

Counselor Self-Awareness and Professional Development

2027 task map: 1A.1 Seek consultation from a colleague for assistance with a specific skill or modality · 1A.2 Seek supervision from a counseling supervisor · 1A.3 Complete continuing education · 1A.4 Develop knowledge of empirically supported counseling interventions or approaches · 1A.5 Engage in advocacy for the client and the counseling profession · 1A.6 Engage in training related to counseling ethics · 1A.7 Engage in training related to ethical and legal use of artificial intelligence (AI) · 1A.8 Engage in training related to working with different cultures or populations · 1B.1 Address your compassion fatigue (e.g., burnout) potential · 1B.2 Assess your exposure to vicarious trauma · 1B.3 Assess your need for specialized training and/or certification · 1B.4 Assess your strengths and limitations as a counselor · 1B.5 Assess your suitability to work with a specific client based on their needs and sociocultural considerations · 1B.6 Utilize self-regulation techniques · 1B.7 Determine the client-counselor suitability (e.g., counselor has the skills and experience needed, availability, no conflict of interest) · 1B.8 Determine your competency for administering assessments · 1B.9 Maintain appropriate boundaries · 1B.10 Practice self-care · 1B.11 Practice self-reflection

The Counselor Is Part of the Treatment

Every other task assumes a counselor who is competent, regulated, honest about limits, and current. This unit keeps that counselor in working order. **Self-awareness** (1B) looks inward: Am I right for this client? Am I worn down? Is this reaction about the client or about me? **Professional development** (1A) looks outward: Whom do I consult? What training do I need? What does the research support?

Items here share one shape. A counselor hits a limit, a reaction, or a gap. The wrong options **ignore** the problem, **overcorrect** it (usually with an abrupt referral), or **push**

it onto the client (through disclosure or by making the client the teacher). The keyed answer **names the problem and takes a proportionate step to fix it** while care continues.

Part 1 — Who Owns It

Owner	What it owns
ACA Code of Ethics (2014), Section C	**C.2.a** boundaries of competence · **C.2.b** new specialty areas · **C.2.d** monitor effectiveness · **C.2.e** consultations on ethical obligations · **C.2.f** continuing education · **C.2.g** impairment · **C.3.a** accurate advertising · **C.4.a** accurate representation · **C.4.b** credentials · **C.7.a–c** scientific basis, innovation, harmful practices · Section C introduction: **self-care** · **C.1** knowledge of and compliance with standards
ACA Code (2014), Sections A and B	**A.1.a** primary responsibility · **A.1.b** records · **A.2.c** developmental and cultural sensitivity · **A.4.b** personal values · **A.5.d** friends or family members · **A.6, A.6.a, A.6.e** boundaries, previous relationships, nonprofessional relationships · **A.7.a–b** advocacy and consent · **A.10.e** bartering · **A.10.f** receiving gifts · **A.11.a, A.11.b, A.11.d** competence- and values-based referral, transfer · **A.12** abandonment · **B.1.c** confidentiality · **B.7.a–b** case consultation

Owner	What it owns
ACA Code (2014), Sections D, E, F, H, I	**D.1.c** interdisciplinary teamwork · **D.2.a** consultant competency · **E.2.a** limits of assessment competence · **E.2.b** appropriate use · **E.7** conditions of administration · **F.1.a** supervision and client welfare · **F.2.a** supervisor preparation · **F. 4.a** informed consent for supervision · **H.1.a** technology knowledge and competency · **H. 2.a** technology informed consent · **I.1.b** ethical decision making · **I.1.c** conflicts between ethics and laws
Christina Maslach & Susan Jackson (1981)	**Burnout** and the **Maslach Burnout Inventory (MBI)**: emotional exhaustion, depersonalization (cynicism), reduced personal accomplishment
Charles Figley (1995)	**Compassion fatigue** (secondary traumatic stress): the “cost of caring”
Lisa McCann & Laurie Anne Pearlman (1990)	**Vicarious traumatization**: cumulative change in the counselor’s beliefs about safety, trust, esteem, intimacy, and control
Beth Hudnall Stamm (2010)	**Compassion satisfaction**; the **Professional Quality of Life Scale (ProQOL)**: compassion satisfaction, burnout, secondary traumatic stress
Thomas Skovholt (2001)	**The resilient practitioner**; the **cycle of caring**; balancing other-care with self-care
John Norcross & Gary VandenBos (2018)	Evidence-based **self-care strategies** for therapists; personal therapy as one strategy
Sigmund Freud (1910); **Hayes, Gelso, & Hummel** (2011)	**Countertransference** (Freud) and its **management** (Hayes et al.)
Donald Schön (1983)	**Reflective practice**: reflection-in-action and reflection-on-action

Owner	What it owns
Walfish et al. (2012)	**Self-assessment bias**: clinicians overrate their own effectiveness
James Gross (1998)	**Emotion regulation**: cognitive reappraisal vs. expressive suppression
Gerald Caplan (1970)	**Mental health consultation**: a coordinate, nonhierarchical relationship
Bernard & Goodyear (2019)	Definition of **clinical supervision**: evaluative, hierarchical, extends over time
Janine Bernard (1979)	**Discrimination model** of supervision
Stoltenberg & Delworth (1987)	**Integrated Developmental Model (IDM)** of supervision
Elizabeth Holloway (1995)	**Systems approach** to supervision
Ladany et al. (1996)	**Supervisee nondisclosure**
Sackett et al. (1996); **APA Presidential Task Force on Evidence-Based Practice** (2006)	**Evidence-based practice**: best available research + clinical expertise + client characteristics, culture, and preferences
Dianne Chambless & Steven Hollon (1998)	**Empirically supported therapy criteria**: efficacious and specific, efficacious, possibly efficacious
Jacobson & Truax (1991)	**Clinical significance**; the Reliable Change Index
Paul Meehl (1954)	**Clinical vs. statistical (actuarial) prediction**
Jeffrey Mitchell (1983)	**Critical incident stress debriefing (CISD)**
Rose, Bisson, Churchill, & Wessely (Cochrane review, 2002)	Evidence that **single-session debriefing** does not prevent PTSD and may harm; **WHO** (2013) guidance against routine debriefing
Lewis, Arnold, House, & Toporek (2002); **Toporek & Daniels** (2018)	**ACA Advocacy Competencies**: two dimensions, six domains

Owner	What it owns
Karen Kitchener (1984)	**Moral principles** (autonomy, nonmaleficence, beneficence, justice, fidelity); intuitive vs. critical-evaluative levels
ACA Code Preamble	Six principles, adding **veracity**
Forester-Miller & Davis (1996)	**Practitioner's seven-step** decision-making model
Vilia Tarvydas (1998)	**Integrative decision-making model**: four themes, four stages
Meara, Schmidt, & Day (1996)	**Principle ethics vs. virtue ethics**
Mandatory vs. aspirational ethics	Minimum compliance vs. the highest standard
Automation bias; algorithm aversion (Dietvorst et al., 2015)	Two opposite errors with AI and decision aids
HIPAA	**Business associate agreement** (45 CFR 164.504(e)); **Safe Harbor** de-identification
ADA Title III	**Effective communication** and auxiliary aids, including interpreters (28 CFR 36.303)
Hook et al. (2013)	**Cultural humility**
Ratts et al. (2016)	**Multicultural and Social Justice Counseling Competencies (MSJCC)**

Part 2 — Strengths, Limitations, and Specialized Training (1B.3, 1B.4)

Boundaries of Competence

ACA C.2.a limits practice to the counselor's education, training, supervised experience, credentials, and appropriate professional experience. Self-assessment of strengths and limitations is how you apply C.2.a to a real client.

Error	What it looks like	Why it fails
Overestimating competence	"My general training and a good alliance cover everything."	Ignores needs outside the counselor's scope, such as medication management or acute crisis care
Underestimating competence	"This diagnosis needs a specialist. I'll decline."	Counselors treat many serious conditions within a collaborative plan; refusal is not required and can delay care

The keyed answer **names what the counselor offers and where others must fill in**: for a serious mood disorder, the counselor may own relapse-prevention planning and mood monitoring while psychiatric and crisis services handle medication and acute episodes. A clear role plus clear referrals.

When the gap is too large to close with consultation or training in time, **ACA A.11.a** directs referral. When the only "gap" is a values conflict, referral is not allowed (Part 3).

Checking Self-Assessment Against Data

Walfish et al. (2012) found that clinicians, on average, rate their own effectiveness well above their peers', and almost none rate themselves below average. **Felt confidence and years in practice are poor gauges of skill** with a specific disorder. **ACA C.2.d** (monitor effectiveness) directs counselors to keep checking their efficacy, including through peer supervision. So **compare self-ratings with outside data**: outcome measures and supervisor or peer feedback. A client's or relative's opinion cannot carry that weight.

Reflective Practice and the Near Miss

Donald Schön (1983) described the **reflective practitioner**. **Reflection-in-action** happens during the session ("I'm pushing and she's pulling back"). **Reflection-on-action** happens afterward ("Why did I miss that?").

When a counselor finds a near miss, such as almost skipping a screen for past hypomania in a client presenting with depression, reflection-on-action gives the keyed answer: **name the gap and change the routine** (screen at every intake). Wrong answers **overcorrect** (transfer the client; one fixable gap is not incompetence), **externalize** (blame the client's self-report; asking is the counselor's job), or **lean on a strength** (rapport cannot replace a systematic screen).

Specialized Training and Certification

ACA C.2.b (new specialty areas): counselors practice in new areas only after appropriate education, training, and supervised experience, and protect clients while skills develop. Assessing your need for specialized training starts from the **caseload**: what do these clients need, what training do I have, and what credential rules apply where I practice?

Idea	Rule
License ≠ competence in every specialty	A license sets a general floor. It does not confer competence with every population, problem, or instrument

Idea	Rule
Training is the basis for practice; a credential is a claim	Many modality certifications, such as EMDR certification, are **voluntary** advanced credentials. A counselor with completed training and its required consultation may use the method but may not **claim** the certification. **ACA C.4.a** (accurate representation) and **C. 4.b** (claim only credentials that are current and in good standing) govern the claim; **C. 3.a** (accurate advertising) governs what goes on a website
A certificate is only as good as its content	Collecting any available certificate does not build competence unless its training matches what clients need

Some roles do require specific preparation. **ACA F.2.a** (supervisor preparation): counselors are trained in supervision methods before offering supervision, and many state boards add their own supervisor requirements. Years of practice do not substitute. Verify your board's requirements first; the supervision agreement (**F.4.a**) comes after.

Part 3 — Suitability for a Specific Client (1B.5, 1B.7)

The Three Suitability Tests

The 2027 outline names them directly. A counselor is suitable for a client when the counselor has:

1. **The skills and experience the client needs** (or a realistic path to them through consultation and supervision)
2. **Availability** at the level the client needs (session frequency, crisis access, scheduling)
3. **No conflict of interest** (no competing role or tie)

Not suitability tests: shared background or interests (rapport, not competence), the fee the insurer pays, or whether the client already has insight (limited insight is often a target of treatment).

Values Conflicts: Serve, Don't Refer

- **ACA A.4.b (personal values):** counselors are aware of and avoid imposing their own values, attitudes, beliefs, and behaviors. They **seek training** in areas where they risk imposing values, especially when their values conflict with the client's goals.
- **ACA A.11.b (values within termination and referral):** counselors **do not refer** prospective or current clients **based solely on the counselor's personally held values**.

When a counselor's personal or religious beliefs conflict with a client's identity, relationship, or goals, the keyed answer is to **serve the client and seek training, consultation, or supervision as needed.** Sharing the beliefs so the client can "decide" is value imposition by self-disclosure. Referral for **lack of competence** (A.11.a) is permitted; referral for values is not.

Conflicts of Interest and Prior Contact

ACA A.6 (managing and maintaining boundaries) and **A.6.a** (previous relationships) direct counselors to weigh the risks and benefits of accepting a client with whom they have had another relationship. **A.5.d** prohibits counseling friends or family members when the counselor cannot remain objective. **A.6.e** asks counselors to avoid nonprofessional relationships with former clients and their families when the interaction is potentially harmful.

Prior or current tie	Typical judgment
The counselor holds **ongoing evaluative power** over the client (instructor, employer, grader)	A real conflict of interest: discuss it and arrange a referral. Having someone else grade leaves the power relationship in place; asking the client to drop the class shifts the problem onto the client
A **single brief transaction** years ago with no contact since	Rarely a conflict. Acknowledge it, weigh the risk, document the decision. Automatic referral overreacts; silence risks trust if the client recognizes the counselor later
A close friend or relative	Not suitable when objectivity is compromised (A.5.d)

Sociocultural and Language Fit

ACA A.2.c (developmental and cultural sensitivity) directs counselors to communicate in ways clients understand and, when needed, to arrange **a qualified interpreter or translator**. For a Deaf client who uses American Sign Language, offer a qualified interpreter or a signing counselor, with the client's input. **ADA Title III** requires **effective communication** through auxiliary aids, including qualified interpreters (**28 CFR 36.303**). Written notes rarely meet that bar for counseling; a family member as interpreter compromises confidentiality, accuracy, and candor.

Cultural difference alone is not a reason to refer. The counselor builds the needed competence (Part 14).

Part 4 — Competence to Administer Assessments (1B.8)

ACA E.2.a (limits of competence): counselors use only the testing and assessment services for which they have been trained and are competent. **ACA E.2.b** (appropriate use): counselors are responsible for the appropriate application, scoring, interpretation, and use of instruments,

whether they score them themselves or use technology or other services. **ACA E.7** asks that instruments be administered under the conditions established in their standardization.

Four traps recur:

Trap	Why it fails
"I read the manual."	Reading instructions is not training. Structured rating scales and risk instruments need instrument-specific training to give reliable scores
"The publisher let me buy it."	Publisher **qualification levels** set a minimum to purchase. They do not show skill with a given instrument
"Front-desk staff can give it."	The counselor stays responsible for who administers the instrument (E.2.b). Untrained administration threatens validity and privacy
"I was trained years ago."	Skill decays without use. Before relying on new scores, refresh the manual and scoring rules and **co-rate with a trained colleague** to confirm **interrater reliability**

Also wrong: interpreting by impression instead of norms, changing the administration format (parents completing a clinician-rated scale), skipping assessment, or leaving scores out of the record (**A.1.b**, records). If structured assessment is needed before training is complete, **consult or refer to a qualified evaluator** in the meantime.

Part 5 — Maintaining Appropriate Boundaries (1B.9)

The ethics chapter covers boundary crossings and violations in full. Two patterns carry most items here.

Gifts (ACA A.10.f). The Code does not ban gifts. Counselors weigh **the therapeutic relationship, the monet-**

ary value of the gift, the client's motivation, and the counselor's motivation. A young child's drawing usually fits: accept it warmly and explore what it means. Refusing can feel like rejection to a child. Matching it with a gift of equal value turns the relationship into a social exchange. Displaying it with the child's name reveals that the child is a client.

Services and second roles. Free labor for the counselor's business creates an obligation and a second relationship; paying the client makes the client a vendor. Decline warmly and explore what the offer means. When parents ask the counselor to take a second paid role, such as tutoring their child, decline the role (**A.6**), explain why, and **refer** to someone who can meet the need. Changing the location or billing leaves the dual role intact; the clinical goal stays on the counseling agenda.

Keep **bartering** (**A.10.e**) separate: it is an exchange of goods or services for counseling that the **client requests**, that is an accepted practice among professionals in the community, that is not exploitative, and that is documented in a written contract. A thank-you offer of free labor is not bartering.

Part 6 — The Costs of Caring: Burnout, Compassion Fatigue, and Vicarious Trauma (1B.1, 1B.2)

The exam asks you to tell three conditions apart, match each to its owner, and choose the response that fits its cause.

Burnout (Maslach & Jackson)

Christina Maslach defined **burnout** as a prolonged response to **chronic interpersonal stressors on the job**. With **Susan Jackson** she built the **Maslach Burnout Inventory (MBI; Maslach & Jackson, 1981)**, which measures three dimensions:

Dimension	What it looks like in a counselor
Emotional exhaustion	Drained, depleted, nothing left to give at the end of the day
Depersonalization (later called **cynicism**)	Detached, callous, or irritable toward clients; seeing them as cases or labels
Reduced personal accomplishment (later called **inefficacy**)	Feeling ineffective; doubting the work makes a difference

Two exam points: **cynicism toward clients is a burnout dimension, not healthy professional distance.** Because burnout grows from **work conditions**, the fix targets the **workload** as well as the counselor. A week off eases exhaustion briefly, but burnout returns if the caseload is unchanged. The keyed answer adjusts the caseload with the supervisor **and** builds in recovery.

Compassion Fatigue (Figley)

Charles Figley (1995) described **compassion fatigue** as the "cost of caring" for people who are suffering. He used it interchangeably with **secondary traumatic stress (STS)**: stress reactions that come from **helping or wanting to help a traumatized or suffering person**. STS can resemble PTSD: intrusive thoughts, avoidance, and arousal, picked up from clients' accounts rather than from direct exposure. Compassion fatigue can come on **quickly**, after a run of intense cases, where burnout builds slowly.

Early signs include **numbness, going through the motions**, dreading certain sessions, and reduced empathy. The exam rewards acting **at the early sign**: self-care, consultation or supervision, and caseload limits **now**, not "once the semester ends" or "once clients complain."

Vicarious Traumatization (McCann & Pearlman)

Lisa McCann and Laurie Anne Pearlman (1990) described **vicarious traumatization**: a **cumulative transformation** of the counselor's inner experience that comes from **empathic engagement with clients' trauma** over time. It is grounded in their **constructivist self-development theory**, and its signature is **disrupted cognitive schemas**, the counselor's core beliefs about **safety, trust, esteem, intimacy, and control**. Intrusive imagery from clients' stories can also occur.

Signs include intrusive images, dread before sessions, hypervigilance **outside work**, loss of trust, and reading ordinary client events as hidden danger. A counselor whose sense of danger **outruns the client's data** should **check the reaction against what the client reports and assess the counselor's own trauma exposure**. Treating it as "clinical intuition" and adding sessions over-treats the client.

Compassion Satisfaction and the ProQOL (Stamm)

Beth Hudnall Stamm added the positive side. **Compassion satisfaction** is the pleasure and meaning a helper draws from doing the work well. In her model, **compassion fatigue** has two parts, **burnout** and **secondary traumatic stress**, and compassion satisfaction can offset them.

Her **Professional Quality of Life Scale (ProQOL)** measures all three: **compassion satisfaction, burnout, and secondary traumatic stress**. It is a standard tool for assessing the effects of trauma exposure systematically, and its results belong in supervision. Compassion satisfaction comes from **meaningful work, not more work**; adding clients to rebuild purpose deepens fatigue.

Keep Them Apart

	Burnout	Compassion fatigue / STS	Vicarious traumatization
Owner	Maslach (& Jackson, MBI)	Figley	McCann & Pearlman
Source	Chronic work demands and conditions	Caring for suffering or traumatized clients	Cumulative empathic exposure to client trauma
Onset	Gradual	Can be sudden	Cumulative
Signature	Exhaustion, cynicism, inefficacy	PTSD-like intrusion, avoidance, arousal; numbness	Changed beliefs about safety, trust, esteem, intimacy, control
Trauma-specific?	No	Mostly (trauma or suffering)	Yes

Distractors: **countertransference** (Part 7) comes from the counselor's own history, not trauma exposure. The **Beck Depression Inventory-II (BDI-II; Beck et al., 1996)** measures depression severity. The **MBI** measures burnout only.

When Fatigue Touches Clinical Judgment

ACA C.2.g (impairment): counselors **monitor themselves for signs of impairment** and refrain from offering services **when impaired**, seeking help and limiting or suspending work if needed. The duty is to act **early**, not wait for impairment to show.

The most tested scenario: a drained counselor feels a pull to accept a client's "I'd never do it" without further questions. Keyed sequence: **name the fatigue, finish the safety work now, then seek consultation and support.** Accepting the denial lets exhaustion lower the bar; handing the client off mid-session without assessing risk is neglect (**A.12**).

General order: notice and name the sign → finish urgent client work → assess (ProQOL) and take it to supervision → change the conditions (caseload, recovery, personal counseling) → C.2.g steps if impairment is present or likely. Wrong answers dismiss the sign as "part of the job," tell the client, ask the client to share less trauma detail, or transfer the client at once.

Part 7 — Self-Care, Self-Regulation, and Self-Reflection (1B.6, 1B.10, 1B.11)

Self-Care Is Ongoing, Preventive, and Ethical

The **introduction to Section C** of the ACA Code says counselors engage in **self-care activities** to maintain their emotional, physical, mental, and spiritual well-being so they can meet their professional responsibilities. Self-care is part of ethical practice, not a crisis response.

Thomas Skovholt (*The Resilient Practitioner*) frames counseling as **one-way caring**: the counselor gives care without expecting it back, so resilience requires **balancing other-care with self-care**. His **cycle of caring** describes each helping relationship in four phases, repeated across a career:

1. **Empathic attachment** (forming the bond)
2. **Active involvement** (doing the work)
3. **Felt separation** (ending well and letting go)
4. **Re-creation** (restoring energy before the next attachment)

Skipping felt separation and re-creation carries each client into the next. The resilient practitioner builds renewal into ordinary weeks.

John Norcross (with VandenBos, *Leaving It at the Office*) lists research-informed self-care strategies, such as minding the body, nurturing relationships, setting boundaries, and **personal therapy**, which is **one** strategy, not the whole of self-care.

Three wrong views: self-care only when stress peaks (it is **ongoing and preventive**), only after impairment (self-care **prevents** it), or only personal therapy.

Self-Regulation in Session

Alarm, a rising pulse, irritation, the urge to escape: **self-regulation** keeps these from driving the response. The rule: **regulate first, then respond.** Techniques include **paced breathing** with a slow exhale, slowing speech, noticing body cues, **grounding** for a few minutes between sessions, and **cognitive reappraisal** ("This anger is fear, not about me").

James Gross separates **cognitive reappraisal**, which changes the emotion, from **expressive suppression**, which hides the expression but leaves physiological arousal high and impairs memory. "Push it down and start the next session" is suppression, not regulation.

Situation	Keyed response	Common wrong answers
Client speeds up and grows irritable; counselor's pulse rises	Slow own breathing, keep a calm, measured tone	Match the client's volume (escalates); tell the client to stop (reactive enactment); end the session abruptly
Client says quietly that her family would be better off without her; counselor feels alarm	Slow breathing and speech, then **ask directly about suicide**	Reassure and move on; change the topic until the counselor calms; call emergency services because the counselor feels alarmed (risk comes from assessment, not the counselor's emotion)

Situation	Keyed response	Common wrong answers
Counselor is still keyed up after a hostile session; next client is waiting	Brief grounding pause, then begin	Vent to front-desk staff or in a hallway (breaches confidentiality, ACA Section B, **B.1.c**); suppress and start at once; cancel the hostile client's next appointments

Asking directly about suicide does not increase risk; regulation makes the direct question possible.

Self-Reflection and Countertransference

Freud introduced **countertransference** as the analyst's unconscious reactions to the patient, an obstacle to overcome. Contemporary views treat it as reactions shaped by the counselor's own history and needs, useful once examined. **Hayes, Gelso, and colleagues** name the qualities that **manage** it: **self-insight, self-integration, anxiety management, empathy, and conceptualizing skills**.

Self-reflection keeps countertransference from steering care: What am I feeling? Where does it come from? How is it shaping what I do?

- **A belief leaks into a reaction.** The counselor feels impatient and thinks the client "isn't trying." Reflect on the belief and **bracket** it. Telling the client to try harder voices the bias as feedback.
- **Personal history drives a reaction.** A counselor who struggled in school feels fiercely protective of a child client and angry at her teacher. Reflect on the link; anger is not evidence about the school, and long self-disclosure centers the counselor's story.
- **A session went sideways.** The counselor kept pushing suggestions while the client tried to describe a worry. Reflect on what drove the push (a need to show

progress) and plan to listen more. Labeling the client "resistant" externalizes; a long apology is self-focus.

Across all three, **reflection comes before any referral**, and supervision is the place to bring what reflection turns up.

Part 8 — Consultation and Supervision (1A.1, 1A.2)

Two Different Relationships

	Consultation	Clinical supervision
Owner	**Caplan**: a **coordinate, nonhierarchical** relationship; the consultee is **free to accept or reject** the advice	**Bernard & Goodyear**: an intervention by a more senior member of the profession that is **evaluative, hierarchical, and extends over time**
Who is responsible for the client	The treating counselor keeps **primary responsibility** (A. 1.a)	Responsibility is **shared**; the supervisor monitors client welfare (**F.1.a**) and can be held vicariously liable
Evaluation	None	Yes, including gatekeeping
Typical use	A licensed counselor needs help with a specific skill or modality	A counselor working toward licensure, or anyone in a supervised role

Remember Caplan's core point: **no authority, free to accept or reject**. Referral differs again: it **moves the client's care** to someone else (**A.11.d**, appropriate transfer of services).

Consultation Done Right

When. A counselor with some training but little experience in a modality (behavioral activation, imaginal expos-

ure, sleep restriction) consults an experienced colleague **while building skill** (**ACA C.2.b**). Online summaries mean learning the method on the client; referral is premature with training underway.

Whom. Match the consultant to the **specific skill gap**. Seniority or friendship is not expertise. A physician coordinates medical care (**D.1.c**, interdisciplinary teamwork) but is not the source for a counseling technique. **ACA D.2.a** (consultant competency) is the bar the consultant should meet.

What to share. ACA B.7.b: no identifying information without consent unless unavoidable, and **only what the consultation needs**. **B.7.a** limits consultation discussion to professional purposes. A full chart is too much; a public forum can identify a client even without a name; dinner conversation is not consultation. A release is **not** needed for a de-identified consultation.

What to do with the advice. Weigh it against your own assessment, keep responsibility (A.1.a), and document it. Advice does not transfer responsibility.

When the plan stalls. One consultation is not standing approval. If outcome scores stay flat, **return to the consultant with the data** (C.2.d).

Supervision Done Right

A supervisee unsure about a risk judgment, or shaken by a client's content, brings **both the clinical question and the counselor's own reactions** to supervision, so the supervisor can monitor client welfare (F.1.a). A supervisee facing an unfamiliar diagnosis reviews the plan with the **assigned supervisor before using it**. A peer's approval cannot replace the supervisor, and reporting after the fact removes the oversight.

Ladany et al. (1996) found that most supervisees withhold something, such as mistakes or negative reactions, often to manage impressions. Hiding doubts defeats su-

pervision; referring a treatable client out to escape discomfort shifts the problem onto the client.

Model	Owner	Signature
Discrimination model	**Bernard**	Three supervisor **roles** (teacher, counselor, consultant) crossed with three **foci** (intervention, conceptualization, personalization)
Integrated Developmental Model (IDM)	**Stoltenberg & Delworth**	Supervisees move through **levels**, tracked by **self/other awareness, motivation, and autonomy**
Systems approach to supervision (SAS)	**Holloway**	The **supervisory relationship** at the center, surrounded by contextual factors

Part 9 — Continuing Education (1A.3)

ACA C.2.f (continuing education): counselors pursue continuing education to stay current with scientific and professional information, maintain competence in the skills they use, and stay informed about best practices with diverse populations. It owns the **duty to stay current**; do not confuse it with **C.2.a** (practice within competence) or **C.2.g** (impairment). Four rules:

1. **Plan from needs.** Choose CE from a review of the **current caseload and practice gaps**. Comfort-zone topics, free courses, or the most hours in one sitting satisfy a number, not the purpose.
2. **Check the provider first.** Credentialing bodies accept hours only from **approved or qualifying providers**. Price says nothing about whether hours count.
3. **Check each credential separately.** Each board and certifying body sets its **own** hours, required topics (often ethics), and accepted providers. The issuing

jurisdiction's rules apply, not the state where you trained. Check your state board and certifying body for current requirements.

4. **Keep records; report honestly.** Retain **certificates of completion** (provider, date, hours) for **audits**; recall or a catalog listing proves nothing. If short at renewal, **contact the board about its options before attesting**. A false attestation is misrepresentation (**C.4.a**); uncredited reading does not count.

Part 10 — Empirically Supported Interventions (1A.4)

Evidence-Based Practice Has Three Legs

David Sackett and colleagues (1996) defined evidence-based medicine as integrating the **best available research evidence** with **clinical expertise** and **patient values**. The **APA Presidential Task Force on Evidence-Based Practice (2006)** carried this into psychology: **best available research + clinical expertise + client characteristics, culture, and preferences**.

Two errors cut off a leg: **theoretical allegiance** (loyalty to an orientation, a known bias) and **ranking by effect size alone** (research only, no client). Client preference shapes the choice among supported options but does not **define** what is supported.

ACA C.7.a (scientific basis for treatment) directs counselors to use techniques grounded in theory and/or with an empirical or scientific foundation. **C.7.b** (development and innovation) requires explaining the risks, benefits, and ethical considerations of developing or innovative techniques and minimizing harm. **C.7.c** (harmful practices) bars techniques when **substantial evidence suggests harm**, even if requested.

Chambless & Hollon (1998): What Counts as Empirically Supported

Dianne Chambless and Steven Hollon (1998) set criteria for **empirically supported therapies (ESTs)**. Evidence must come from **randomized controlled trials** (or rigorous controlled single-case experiments), with a **treatment manual**, a **clearly defined population**, reliable and valid outcome measures, and appropriate analysis. They defined three levels:

Level	Meaning
Efficacious and specific	Shown **superior to a placebo or to an alternative bona fide treatment** in at least two independent research settings
Efficacious	Shown **superior to no treatment** (e.g., wait-list), or equivalent to an already established treatment, in at least two independent research settings
Possibly efficacious	Support from only **one study or one research group**, without independent replication

A related, earlier scheme from the APA Division 12 task force (Chambless and colleagues) used "**well-established**" and "**probably efficacious**." If an item says "efficacious and specific," the owner is Chambless & Hollon (1998).

Efficacy vs. Effectiveness vs. Clinical Significance

Term	Question	Typical evidence
Efficacy	Does it work under **ideal research conditions**?	Tightly controlled RCTs with screened samples, manuals, and trained therapists

Term	Question	Typical evidence
Effectiveness	Does it work in **routine practice** with typical, often comorbid clients?	Studies in community clinics with few exclusions
Clinical significance (**Jacobson & Truax, 1991**)	Is **an individual's** change meaningful?	Reliable Change Index; movement into the functional range

Beating placebo in controlled trials with screened participants shows **efficacy**, not yet effectiveness in your setting, and not that any one client changed meaningfully.

Weighing the Evidence

A common **hierarchy of evidence**, strongest first: systematic reviews and meta-analyses (and **practice guidelines** built on them) → individual RCTs → quasi-experimental and observational studies → case series and **single case reports** → expert opinion, testimonials, and anecdote. Then apply three checks:

- **External validity (generalizability).** Rigorous evidence from people like the client weighs most. For a veteran at risk of suicide, guidelines that include at-risk veterans (such as the VA/DoD suicide-risk guideline) outweigh studies of adolescents.
- **Publication bias (the file-drawer problem).** Positive results get published more often. Publication does not prove efficacy, and article counts are not strength of evidence.
- **Best available evidence.** A thin base, such as the small adjustment-disorder literature (some support for problem-solving approaches and brief CBT), is **not zero evidence**. Appraise what exists rather than falling back on intuition or borrowing a protocol from a different diagnosis.

Build knowledge from guidelines, systematic reviews, and peer-reviewed outcome research, not from popularity or

social media trends, testimonials, workshop handouts, or aging graduate training (**C.2.f**).

Meehl: Clinical vs. Statistical Prediction

Paul Meehl (1954), in *Clinical Versus Statistical Prediction*, compared **unaided clinical judgment** with **statistical (actuarial) prediction**, which combines the same data by formula. Statistical prediction **matched or outperformed** clinical judgment in nearly every comparison, and later meta-analyses agreed. Lesson: structured data check intuition, and "thin research, so I'll trust my gut" is the weaker choice. Meehl's "**broken leg**" exception allows overriding a formula when the clinician holds decisive information it does not capture, which is why a counselor whose interview contradicts a tool's low-risk rating acts on the **full assessment** and documents the discrepancy (Part 13).

A Popular Practice That Failed: Critical Incident Stress Debriefing

Jeffrey Mitchell (1983) developed **critical incident stress debriefing (CISD)**, a structured single-session group meeting held within days of a traumatic event, originally for emergency workers, moving through set phases (introduction, facts, thoughts, reactions, symptoms, teaching, re-entry). It spread widely.

Controlled research did not support it. The **Cochrane review by Rose, Bisson, Churchill, and Wessely (2002)** found that **single-session psychological debriefing** did **not prevent PTSD**, and some trials found **worse outcomes** at follow-up. International trauma guidance, including the **World Health Organization (2013)**, now recommends **against** routine psychological debriefing. Current practice favors **psychological first aid**, monitoring, and **trauma-focused CBT** for persistent symptoms (the crisis chapter covers these). Mitchell's supporters argue

the trials tested debriefing outside its intended group format; the exam's point is unchanged.

CISD shows three errors at once: **bandwagon** (adoption is not evidence), **timing as rescue** (holding it at the "right" time does not save an unsupported intervention), and **testimonial** (a colleague's good experience is not outcome research). When evidence suggests harm, **ACA C.7.c** applies.

Part 11 — Advocacy for the Client and the Profession (1A.5)

ACA A.7.a (advocacy): when appropriate, counselors advocate at individual, group, institutional, and societal levels to address barriers that limit client access, growth, and development. **ACA A.7.b (confidentiality and advocacy):** counselors obtain **client consent** before advocating on behalf of an identifiable client.

The Advocacy Competencies

The **ACA Advocacy Competencies** were developed by **Lewis, Arnold, House, and Toporek (2002)** and endorsed by ACA. **Toporek and Daniels (2018)** led the update. The model has two dimensions: **extent of client involvement** (**collaborating with** clients vs. **acting on behalf of** clients) and **level of intervention** (individual, school/community, public arena). Crossing them gives six domains:

Level	Collaborating with clients	Acting on behalf of clients
Client/student (individual)	**Client/student empowerment**: helping the client build skills to self-advocate	**Client/student advocacy**: negotiating for a specific client, such as appealing an insurer's denial

Level	Collaborating with clients	Acting on behalf of clients
School/community	**Community collaboration**: supporting a community group that leads its own effort	**Systems advocacy**: changing a barrier within a school, agency, or community system
Public arena	**Collective action** (formerly *public information*): working with groups to change public awareness or policy	**Social/political advocacy**: working to change law or public policy, such as testifying for legislation

Advocacy Within Ethical Bounds

Situation	Keyed advocacy	Wrong answers
Insurer denies sessions; outcome scores show continued need	With consent, **appeal using outcome data** showing medical necessity (client/student advocacy)	Bill a more severe diagnosis (**upcoding is fraud**); accept the denial without challenge; post about the denial publicly (exposes the client)
A probation officer calls the client's medication for opioid use disorder "just another drug"	With **written consent**, give the officer information on medication treatment as evidence-based care	Advise the client to hide treatment; tell the client to drop medication; contact the judge without the client's consent

Advocacy for the **profession** (licensure, insurance recognition, scope of practice) usually works in the public arena, through law and policy.

Part 12 — Training in Counseling Ethics and Decision-Making Models (1A.6)

Counselors know the Code and the law (**C.1**, **I.1.a**) and, facing a dilemma, **use and document an ethical decision-making model** (**I.1.b**). The ethics chapter covers the Code's content; this part owns the models.

Kitchener's Principles

Karen Kitchener (1984) described the moral principles underlying ethical reasoning: **autonomy, nonmaleficence, beneficence, justice, and fidelity**. The **ACA Code Preamble** lists these and adds **veracity**.

Principle	Meaning
Beneficence	Working for the good of the individual and society by promoting well-being
Fidelity	Honoring commitments and **keeping promises**
Veracity	Dealing **truthfully** with people

Kitchener also separated an **intuitive** level of moral reasoning from a **critical-evaluative** level (rules, principles, theory) used when intuition is not enough.

Mandatory vs. Aspirational Ethics

Mandatory ethics is the minimum: complying with required standards. **Aspirational ethics** strives for the **highest standards** and the spirit behind the Code.

Principle Ethics vs. Virtue Ethics (Meara, Schmidt, & Day, 1996)

Naomi Meara, Lyle Schmidt, and Jeanne Day (1996) argued that principles alone are not enough.

	Principle ethics	Virtue ethics
Question	"**What shall I do?**"	"**Who shall I be?**"
Focus	Solving a specific dilemma using obligations and principles	The counselor's **character**, across all situations
Tools	Principles such as autonomy and beneficence	Core virtues: **prudence, integrity, respectfulness, benevolence**

The two work together: principles guide the decision; virtues shape the person and the small choices no rule covers. Aspirational ethics is about **how high** the standard is; virtue ethics is about **character**.

Forester-Miller & Davis (1996): The Practitioner's Seven Steps

Holly Forester-Miller and Thomas Davis set out seven steps: **(1) identify the problem, (2) apply the ACA Code, (3) determine the nature and dimensions of the dilemma, (4) generate courses of action, (5) consider consequences and choose, (6) evaluate the selected action, (7) implement it.**

Tarvydas (1998): The Integrative Decision-Making Model

Vilia Tarvydas built an **integrative** model that joins principle-based reasoning with attention to the counselor's values and the setting. It rests on **four themes** the counselor maintains throughout: a **reflective attitude**, **balance** among the issues and the parties involved, close attention to **context**, and **collaboration** with all rightful parties, including the client.

It moves through **four stages**:

Stage	Name	Key work
I	Interpreting the situation through **awareness and fact finding**	Heighten sensitivity; identify the stakeholders and their ethical claims; gather facts
II	**Formulating** an ethical decision	Review the dilemma; identify the codes, laws, principles, and policies that apply; generate courses of action; weigh consequences; **consult**; select the best ethical course

Stage	Name	Key work
III	**Selecting an action** by weighing competing **nonmoral values, personal blind spots, or prejudices**	Reflect on the counselor's own competing values and biases; consider pressures from colleagues, team, institution, and society
IV	**Planning and executing** the action	Sequence the steps; anticipate barriers and plan countermeasures; carry out, document, and evaluate

Stage III is the signature: Tarvydas builds **self-examination of the decision-maker** into the model.

When the Code and the Law Conflict

ACA I.1.c: counselors **make known their commitment to the Code and take steps to resolve the conflict**; if it cannot be resolved, they may adhere to the law. Ignoring the law is wrong; so is complying silently. **C.2.e**: consult when unsure of ethical obligations.

Part 13 — Training in Ethical and Legal Use of AI (1A.7)

The **2014 ACA Code does not mention artificial intelligence by name.** Its technology, confidentiality, and responsibility standards apply directly:

- **H.1.a (knowledge and competency):** counselors who use technology develop knowledge and skills about its **technical, ethical, and legal** considerations. Build that competence **before** using an AI tool with client data.
- **H.2.a (informed consent and disclosure):** clients are informed about the technologies used in their care, including risks and limits, and can choose. Consent cannot depend on whether the client thinks to ask.

- **B.1.c (confidentiality)** and **HIPAA:** a vendor that handles protected health information on the counselor's behalf is a **business associate** and must sign a **business associate agreement (BAA)** (45 CFR 164.504(e)). Free public chatbots generally do not. Removing a name is **not** de-identification: HIPAA's **Safe Harbor** method requires removing 18 categories of identifiers, and session details can still identify a client.
- **A.1.a (primary responsibility):** AI informs; the counselor decides. The counselor is responsible for every note and clinical decision, including AI-drafted ones.

Agency approval does not supply competence or settle consent and privacy. AI guidance from professional bodies is evolving; verify current recommendations from ACA, NBCC, and your state board.

Two Opposite Errors

Error	Meaning	Correction
Automation bias	Trusting automated output because it is automated, even over contrary evidence	Verify AI output; AI tools can be inaccurate and can carry bias
Algorithm aversion (Dietvorst et al., 2015)	Rejecting a tool wholesale after seeing it err once	Keep verifying; don't abandon a useful aid after one error

When an AI risk rating conflicts with the interview, the counselor relies on the **full clinical assessment and documents the discrepancy**.

Part 14 — Training to Work With Different Cultures and Populations (1A.8)

Cultural Humility (Hook et al., 2013)

Joshua Hook and colleagues (2013) defined **cultural humility** as an **other-oriented** interpersonal stance: openness to the client's cultural identity, curiosity about what it means to the client, and **no assumption of superiority**. They found that clients' perceptions of their counselor's cultural humility were associated with a stronger working alliance and better outcomes.

Faced with a norm the counselor knows little about, the counselor **explores what it means to this client** and **seeks training** to close the gap. Treating a cultural explanation as denial pathologizes it; referring for difference alone dodges responsibility.

The Multicultural and Social Justice Counseling Competencies (Ratts et al., 2016)

The **MSJCC** (**Ratts, Singh, Nassar-McMillan, Butler, & McCullough, 2016**) organize competence into four developmental domains: **counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions**. Within them, counselors build **attitudes and beliefs, knowledge, skills, and action**. The model attends to privilege and marginalization in both counselor and client. The multicultural material elsewhere in this book covers it in depth.

What Real Training Looks Like

Choice	Verdict
A **research-based course with skills practice and follow-up consultation** (e.g., on gender-responsive substance use care drawing on SAMHSA guidance)	**Builds competence**: knowledge, practice, and feedback transfer to sessions

Choice	Verdict
Reputable population-specific training applied to each individual (e.g., on graduate students' stressors and help-seeking)	**Builds competence** without stereotyping
A one-hour webinar with a certificate of completion	Documents **attendance, not skill**
A podcast of first-person stories	Builds empathy and awareness, not clinical skill
Online lists of "typical traits"	**Overgeneralization**: invites seeing the group instead of the client
The general multicultural course from graduate school	Treats competence as fixed; **C. 2.f** says stay current on best practices with diverse populations
Making the client the main teacher	Shifts the counselor's training burden onto the client's paid session time

What the Exam Rewards / What It Punishes

Rewards: - Naming a limit or reaction and taking a proportionate step while care continues - Skill-matched, de-identified consultation; doubts brought to the supervisor before acting - Serving clients despite values conflicts, with training and consultation - Acting on early signs of burnout, compassion fatigue, or vicarious trauma - Regulating yourself first, then responding (including asking directly about suicide) - Evidence weighed by design, replication, and fit to the client - Advocacy with consent, within ethical and legal bounds

Punishes: - Referring out to escape discomfort or because of the counselor's values - Learning a new method on the client - Disclosing the counselor's state or beliefs to the client; hiding doubts from a supervisor - Treating popularity, testimonials, or a single case as evidence - Treating a license, a purchase approval, or a certificate as competence - Upcoding, false CE attestation, or claiming

an unearned credential - Trusting AI output blindly, or pasting client data into a public tool

Swap-Pair Drill

If you see…	It's…	Not…
Exhaustion, cynicism, reduced accomplishment from workload	Burnout → Maslach (MBI)	Vicarious trauma
Numbness and PTSD-like symptoms from caring for suffering clients	Compassion fatigue → Figley	Burnout
Changed beliefs about safety and trust from cumulative trauma exposure	Vicarious traumatization → McCann & Pearlman	Countertransference
Measures compassion satisfaction, burnout, and STS	ProQOL → Stamm	MBI
Reaction rooted in the counselor's own history	Countertransference → Freud	Vicarious trauma
Cycle of caring; one-way caring; resilient practitioner	Skovholt	Norcross
Reflection-in-action, reflection-on-action	Schön	Kitchener
Clinicians overrate their own effectiveness	Walfish et al. (2012)	Meehl
Nonhierarchical; free to accept or reject advice	Consultation → Caplan	Supervision
Evaluative, hierarchical, over time	Supervision → Bernard & Goodyear	Consultation
Teacher/counselor/ consultant × three foci	Discrimination model → Bernard	IDM

If you see…	It's…	Not…
Levels; self/other awareness, motivation, autonomy	IDM → Stoltenberg & Delworth	Discrimination model
Superior to placebo or another treatment in two independent settings	Efficacious and specific → Chambless & Hollon	Possibly efficacious
Ideal conditions vs. routine practice	Efficacy vs. effectiveness	Clinical significance
Is one client's change meaningful?	Clinical significance → Jacobson & Truax	Efficacy
Research + expertise + client	Evidence-based practice → Sackett; APA (2006)	EST criteria
Formulas match or beat unaided judgment	Meehl	Sackett
Single-session debriefing after trauma	Mitchell (CISD) → not supported (Rose et al., Cochrane)	Psychological first aid
Testifying for legislation	Social/political advocacy (public arena)	Systems advocacy
"What shall I do?" vs. "Who shall I be?"	Principle vs. virtue ethics → Meara, Schmidt, & Day	Mandatory vs. aspirational
Seven steps starting with the Code	Forester-Miller & Davis	Tarvydas
Reflection, balance, context, collaboration; four stages	Tarvydas	Forester-Miller & Davis
Trust the tool because it's a tool	Automation bias	Algorithm aversion (rejecting it after one error)
Other-oriented, no superiority	Cultural humility → Hook et al.	MSJCC

Quick Checks

1. In Stamm's model of professional quality of life, compassion fatigue is made up of: A. Burnout and secondary traumatic stress B. Emotional exhaustion and depersonalization C. Disrupted beliefs about safety and trust D. Compassion satisfaction and burnout

2. A treatment outperformed an alternative bona fide therapy in randomized trials run by two independent research teams. Under Chambless and Hollon's (1998) criteria, it is: A. Possibly efficacious B. Efficacious C. Efficacious and specific D. Clinically significant

3. During an ethics workshop, a counselor is asked to reflect less on how to resolve dilemmas and more on what kind of person the counselor wants to be across all of practice. This reflects: A. Principle ethics B. Mandatory ethics C. Forester-Miller and Davis's first step D. Virtue ethics

4. Using Tarvydas's integrative model, a counselor has chosen the best ethical course but now examines whether personal biases and pressure from the agency are pulling toward a different choice. The counselor is in: A. Stage I B. Stage II C. Stage III D. Stage IV

5. Research comparing clinicians' unaided predictions with predictions made by statistical formulas from the same data has generally found that: A. Statistical prediction matches or outperforms clinical judgment B. Clinical judgment outperforms statistical prediction C. The two are equal only for experienced clinicians D. Statistical prediction is accurate only for diagnosis

6. After a long course of treatment ends, a counselor deliberately marks the goodbye with the client and then takes time to restore energy before taking a new intake. This best illustrates: A. Figley's compassion fatigue B. Gross's expressive suppression C. Maslach's depersonalization D. Skovholt's cycle of caring

7. A counselor supports a neighborhood parents' group that is leading its own campaign for safer routes to school, offering skills and resources at the group's request. Under the ACA Advocacy Competencies, this is: A. Client/student empowerment B. Community collaboration C. Systems advocacy D. Social/political advocacy

8. After a staff member's sudden death at work, an agency director plans a mandatory single-session group debriefing for all employees the next day. The counselor's best recommendation is: A. Hold the debriefing within 72 hours so it is effective B. Offer voluntary practical support, monitor staff, and refer those with persistent symptoms for trauma-focused treatment C. Hold the debriefing, since crisis teams widely use it D. Require each employee to describe the event in detail to prevent PTSD

9. A client snaps, "You're just like every other counselor." The counselor silently reframes the remark as the client's fear of being let down, and feels calmer before responding. This is: A. Cognitive reappraisal B. Expressive suppression C. Depersonalization D. Countertransference enactment

10. A counselor plans to use an AI transcription service for telehealth sessions. Which 2014 ACA standard most directly requires the counselor to first build knowledge of the tool's technical, ethical, and legal considerations? A. C.2.f Continuing Education B. B.7.b Disclosure of Confidential Information C. A.10.f Receiving Gifts D. H.1.a Knowledge and Competency

11. A study tests a manualized therapy in community clinics, enrolls clients with multiple diagnoses, and uses few exclusion criteria. It mainly addresses the therapy's: A. Efficacy B. Clinical significance C. Effectiveness D. Specificity

12. A counselor finds herself referring to clients by diagnosis ("my 3 o'clock borderline") and feeling detached from them after a year of heavy caseloads. On the MBI, this most reflects: A. Emotional exhaustion B. Depersonal-

ization C. Reduced personal accomplishment D. Vicarious traumatization

Answers and Rationales

1 — A. Owner: Stamm (ProQOL): compassion fatigue = burnout + secondary traumatic stress. B: Maslach & Jackson (MBI): two of the three burnout dimensions. C: McCann & Pearlman: vicarious traumatization, not Stamm's components. D: Stamm: compassion satisfaction is the positive counterweight, not part of compassion fatigue.

2 — C. Owner: Chambless & Hollon (1998): superiority to placebo or an alternative bona fide treatment in two independent settings = efficacious and specific. A: Chambless & Hollon: possibly efficacious means one study or one research group. B: Chambless & Hollon: efficacious means superior to no treatment. D: Jacobson & Truax (1991): clinical significance concerns meaningful change for an individual, not a comparison between treatments.

3 — D. Owner: Meara, Schmidt, & Day (1996): virtue ethics asks "Who shall I be?" A: Meara, Schmidt, & Day: principle ethics asks "What shall I do?" about a dilemma. B: Mandatory ethics: minimum compliance with rules. C: Forester-Miller & Davis: step 1 is identifying the problem, a dilemma-solving step.

4 — C. Owner: Tarvydas (1998): Stage III weighs competing nonmoral values, personal blind spots, and contextual pressures. A: Tarvydas Stage I: awareness and fact finding. B: Tarvydas Stage II: formulating the decision (codes, options, consequences, consultation). D: Tarvydas Stage IV: planning and executing the action.

5 — A. Owner: Meehl (1954): statistical prediction matched or beat clinical judgment. B: Reverses Meehl's finding. C: Meehl and later meta-analyses: clinician experience did not erase the advantage. D: Meehl: the advantage held across many kinds of prediction.

6 — D. Owner: Skovholt: felt separation and re-creation complete the cycle of caring. A: Figley: compassion fatigue is a cost of caring, not a renewal practice. B: Gross: suppression hides emotion; it is not renewal. C: Maslach: depersonalization is a burnout dimension.

7 — B. Owner: ACA Advocacy Competencies (Lewis et al., 2002; Toporek & Daniels, 2018): collaborating with a community group that leads = community collaboration. A: Advocacy Competencies: empowerment works with one client on self-advocacy. C: Advocacy Competencies: systems advocacy acts on behalf of clients within a system. D: Advocacy Competencies: social/political advocacy acts in the public arena on law or policy.

8 — B. Owner: Rose et al. (Cochrane review) and current trauma guidance: support, monitoring, and trauma-focused treatment for persistent symptoms; ACA C.7.c. A: Mitchell (CISD): timing does not rescue an unsupported intervention. C: Bandwagon error: adoption is not evidence. D: Mitchell (CISD) detail-recounting format: reviews found no prevention of PTSD and possible harm.

9 — A. Owner: Gross: reappraisal changes the meaning and the emotion. B: Gross: suppression hides expression and leaves arousal high. C: Maslach: depersonalization is burnout-driven detachment. D: Countertransference (Freud): acting on the counselor's reaction, which the counselor avoided.

10 — D. Owner: ACA H.1.a: technical, ethical, and legal knowledge before using technology. A: ACA C.2.f: general duty to stay current, not technology competence. B: ACA B.7.b: disclosure in case consultation. C: ACA A.10.f: gifts from clients.

11 — C. Owner: effectiveness research: routine settings, typical comorbid clients. A: Efficacy research: ideal conditions and screened samples. B: Jacobson & Truax: meaningful change for individuals. D: Chambless & Hollon: specificity requires beating placebo or an alternative treatment.

12 — B. Owner: Maslach & Jackson (MBI): depersonalization (cynicism) toward clients. A: Maslach & Jackson: exhaustion is depletion, not detachment. C: Maslach & Jackson: reduced accomplishment is feeling ineffective. D: McCann & Pearlman: vicarious traumatization involves trauma-driven belief change, not workload cynicism.

What the Exam Rewards / What It Punishes

Rewards: - Empathy communicated so the client can perceive it - Congruence that serves the client - Acceptance of the person, even when addressing harmful behavior - Staying present and nondefensive in conflict; repairing ruptures - Supervision, consultation, and personal counseling for countertransference - Cultural humility

Punishes: - Sympathy or reassurance in place of empathy - “I know exactly how you feel” - Pretending to feel something (incongruence) - Judgment, lecturing, or visible disapproval - Defensiveness when the client is angry with the counselor - Acting on or disclosing countertransference without reflection

Swap-Pair Drill

If you see…	It’s…	Not…
Inner experience matches outward expression	Congruence	Positive regard
Being real, no façade	Genuineness	Empathy
Sensing the client’s experience internally	Empathic attunement	Empathic responding
Communicating understanding	Empathic responding	Empathic attunement
Feeling with, from the client’s frame	Empathy	Sympathy
Feeling sorry for the client	Sympathy	Empathy
Valuing the person without conditions	Unconditional positive regard	Approval of all behavior

If you see…	It's…	Not…
Six necessary and sufficient conditions	Rogers (1957)	Truax & Carkhuff
Accurate empathy, nonpossessive warmth, genuineness	Truax & Carkhuff	Rogers's six conditions
Vicarious introspection	Kohut	Rogers
Demonstrably effective relationship elements	Norcross & Lambert	Wampold
Self-insight, self-integration, anxiety management	Managing countertransference (Hayes & Gelso)	Burnout prevention
Client must perceive empathy and UPR	Rogers's sixth condition	Rogers's first condition

Quick Checks

1. A client says, "My mother died last month, and I can't stop crying at work." The most empathic response is: A. "I'm so sorry. That's terrible." B. "I know exactly how you feel; I lost my mother too." C. "Losing your mother so recently, and then having the grief come up at work, sounds overwhelming." D. "Time heals all wounds."

2. A counselor feels bored as a client repeats the same story but says, "This is fascinating." This counselor is being: A. Congruent B. Incongruent C. Empathic D. Nonjudgmental

3. According to Rogers, which condition requires that the client perceive the counselor's empathy and regard? A. The first B. The third C. The fifth D. The sixth

4. A client discloses that he cheated on his wife and feels terrible. The best response reflecting positive regard is: A. "Cheating is wrong; you need to tell her." B. "It sounds like you're carrying a lot of guilt about what happened. Tell me more about what you're feeling." C. "Everyone

makes mistakes; don't worry about it." D. Saying nothing and changing the subject

5. A client yells, "You're useless! I've wasted months here!" The best response shows: A. Defending the counselor's credentials B. Conflict tolerance: acknowledging the frustration and exploring what isn't working C. Ending the session D. Agreeing that counseling is useless

6. A counselor notices strong irritation toward a client who reminds her of her father. The best action is to: A. Tell the client about the resemblance B. Seek supervision and reflect on the countertransference C. Refer the client immediately D. Ignore the feeling

7. "Empathic attunement" differs from "empathic responding" in that attunement is: A. Verbal B. The internal process of sensing the client's experience C. Always shared with the client D. A group therapeutic factor

8. Which set of conditions did Truax and Carkhuff identify as core to effective counseling? A. Interpretation and insight B. Accurate empathy, nonpossessive warmth, and genuineness C. Confrontation and directives D. Free association and transference

9. A counselor working with a client from a different cultural background asks, "How do you and your family understand what's happening?" This reflects: A. Cultural encapsulation B. Cultural humility C. Countertransference D. Sympathy

10. Which relationship element did the Norcross and Lambert task forces judge demonstrably effective? A. Counselor self-disclosure as a primary intervention B. Empathy C. Advice-giving D. Interpretation

11. Unconditional positive regard means the counselor: A. Approves of everything the client does B. Values the client as a person regardless of the client's behavior C. Avoids discussing harmful behavior D. Agrees with the client's views

12. A counselor says, “I feel so sorry for you. You poor thing.” This is an example of: A. Empathy B. Sympathy C. Immediacy D. Congruence

Answers and Rationales

1 — C. Reflects the client’s experience from the client’s frame of reference (empathic responding). A: sympathy. B: identification that shifts focus to the counselor. D: a cliché that dismisses the feeling.

2 — B. Saying the opposite of what one feels is incongruent. A: congruence would match inner and outer. C: no empathy is shown. D: the issue is genuineness, not judgment.

3 — D. The sixth condition: the client perceives empathy and UPR. A: psychological contact. B: counselor congruence. C: the counselor’s empathic understanding.

4 — B. Accepts the person and explores his experience. A: judgment and directive. C: minimizing reassurance. D: avoidance.

5 — B. Nondefensive exploration of the conflict (4.4; rupture repair). A: defensive. C: avoids. D: unhelpful and untrue.

6 — B. Countertransference is managed through reflection and supervision (1B.11). A: burdens the client. C: premature. D: unmanaged countertransference can harm.

7 — B. Attunement is internal sensing; responding communicates it. A and C: describe responding. D: unrelated.

8 — B. Truax and Carkhuff’s conditions. A: psychodynamic aims. C: directive techniques. D: psychoanalytic concepts.

9 — B. Curiosity about the client’s cultural meaning reflects cultural humility. A: encapsulation imposes the counselor’s assumptions. C: countertransference is the counselor’s emotional reaction. D: sympathy is feeling sorry for.

10 — B. Empathy was judged demonstrably effective. A, C, D: not among the demonstrably effective relationship elements.

11 — B. UPR values the person, not every behavior. A, C, D: misunderstandings of UPR.

12 — B. Feeling sorry for someone from one's own frame is sympathy. A: empathy understands from the client's frame. C: immediacy addresses the present relationship. D: congruence is about genuineness.

Audio Companion

Track	Covers	Target length
22.1	Be the Instrument: Rogers's conditions, empathy, congruence, regard, conflict, self-awareness	~4 min

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Chapter 23 — Indirect Client Care

2027 task map: 5.1 Collaborate with client's relevant support systems (e.g., advocate, family, wraparound services, legal services) with goals and objectives · 5.2 Collaborate with non-clinical stakeholders (e.g., legal counselor, community resources) · 5.3 Collaborate with other treatment providers (e.g., care coordination, interdisciplinary treatment team) · 5.4 Document the self-reported effects of client's medication · 5.5 Maintain an updated list of referral resources · 5.6 Review clinical documentation before client sessions · 5.7 Secure appropriate translation services for clients' preferred language · 5.8 Write clinical documentation during or after client sessions

The Work Around the Session

Most of what this unit covers happens when the client is not in the chair. The counselor reads the chart before the session, writes the note after it, calls the prescriber, books the interpreter, updates the referral list, and agrees on roles with a sister, a school counselor, or a domestic violence advocate. The 2027 outlines group these tasks as indirect client care.

Indirect does not mean minor. A missed line in a pre-session review can hide a risk flag. A family member asked to interpret can change what a safety plan says. A note that says "the medication is too strong for her" puts a medical judgment in the record that the counselor is not qualified to make.

Every item in this area turns on three questions, asked in order:

1. **Consent.** Has the client agreed, in writing where it matters, to this contact and this much sharing?

2. **Scope.** Is this my role, or someone else's: the prescriber's, the attorney's, the interpreter's, the advocate's?
3. **Record.** Does the record show what happened accurately, on time, and in the client's words where the client is the source?

The rewarded answer keeps the counselor responsible for the client's care, shares only what the client agreed to and the purpose needs, and connects the client to the right person instead of doing that person's job.

Part 1 — Who Owns It

Owner	What it owns
ACA A.1.b; **B.6.a**	Timely, accurate records needed for services and continuity; amendments noted per policy
ACA A.1.d Support Network Involvement	Family, friends, and community leaders as resources, **with client consent**
ACA A.2.c Developmental and Cultural Sensitivity	Culturally appropriate communication; **a qualified interpreter or translator** when language is a barrier
ACA A.2.e Mandated Clients	Telling mandated clients what is shared, with whom
ACA A.3 Clients Served by Others	Requesting a release to inform the client's other mental health professionals and collaborate
ACA A.7.a / A.7.b	Advocacy to remove barriers; client consent before advocating for an identifiable client
ACA A.10.b	No payment for referrals (no fee splitting)
ACA A.11.a	Knowing appropriate referral resources and suggesting alternatives when lacking competence
ACA A.11.d	Completing transfer steps and keeping communication open

Owner	What it owns
ACA A.12	No abandonment
ACA B.1.c Respect for Confidentiality	Disclosing only with consent or sound legal or ethical justification
ACA B.2.d / B.2.e	Limiting court-ordered disclosure; sharing only essential information
ACA B.3.b / B.3.e	Informing clients about interdisciplinary teams; protecting transmitted information
ACA B.6.g Disclosure or Transfer	Written client permission before records go to a third party
ACA B.7.a / B.7.b Case Consultation	Consultation for professional purposes with only germane data; no identifying information without consent unless unavoidable
ACA C.2.a Boundaries of Competence	The basis of the medication scope line
ACA D.1.c Interdisciplinary Teamwork	Collaborating on teams with a counseling perspective
HIPAA Privacy Rule (45 CFR Part 164)	Authorization (164.508), psychotherapy notes (164.508(a)(2)), treatment disclosures (164.506), minimum necessary (164.502(b), 164.514(d)), judicial proceedings (164.512(e)), access (164.524), amendment (164.526)
42 CFR Part 2	Added protection for federally assisted substance use disorder program records
FERPA	Education records held by schools
VAWA and FVPSA confidentiality provisions	Domestic violence programs generally share survivor information only with a written, time-limited release
ADA Title III (28 CFR 36.303; 36.301(c))	Qualified interpreters and other auxiliary aids for effective communication, at no charge to the client

Owner	What it owns
Title VI; Section 1557 of the Affordable Care Act	Language access in federally funded and health programs
National CLAS Standards (HHS Office of Minority Health, 2013)	Competent interpreters; no untrained individuals or minors as interpreters
Wraparound (Bruns et al., 2004); **systems of care** (Stroul & Friedman, 1986)	Family-driven team planning for youth across systems; the coordinated network it works within
ACT (Stein & Test, 1980); **MST** (Henggeler et al., 2009)	Community team for adults with serious mental illness; home-based treatment for youth antisocial behavior
Family-focused therapy (Miklowitz, 2008)	Family roles and early warning signs in bipolar disorder
Peer recovery support (SAMHSA)	Lived-experience support, not clinical treatment
ACA Advocacy Competencies (Toporek et al., 2009)	Client-, community-, and public-level advocacy
Collaborative care (Unützer et al., 2002)	Team-based depression care in primary care
Measurement-based care (Lewis et al., 2019); **PHQ-9** (Kroenke et al., 2001)	Repeated measures reviewed to guide treatment
SOAP (Weed, 1968); DAP; BIRP; GIRP	Progress note formats
FDA boxed warnings; **Stahl** (2020, 2021)	Drug safety warnings; mechanisms and side-effect profiles
FINISH (Berber, 1998); **AIMS** (Guy, 1976)	Antidepressant discontinuation symptoms; tardive dyskinesia screening
AGS Beers Criteria (American Geriatrics Society, 2023); **CBT-I guideline** (Qaseem et al., 2016)	Risky medications in older adults; CBT-I first-line for chronic insomnia
988; **211**; **311**	Crisis line; social service referral; city non-emergency services

Part 2 — Collaborating With Support Systems (5.1)

Consent Comes First

ACA A.1.d encourages counselors to consider enlisting family, friends, and community or spiritual leaders as positive resources, **with client consent**. **ACA B.1.c** sets the limit: disclose only with appropriate consent or a sound legal or ethical justification. A client's wish to involve family is an invitation to get a release, not a license to call.

The rewarded sequence:

1. Explore what the client wants the support person to do, and what the client wants kept private.
2. Get a **signed release** naming the person, the information, the purpose, and when it ends. A HIPAA authorization also carries the client's signature and the right to revoke (45 CFR 164.508).
3. Plan the contact or joint session around the client's goals and warning signs.
4. Document the release, the contact, and what was shared.

Risk information does not skip step 2. Disclosing a client's diagnosis or risk to family without consent requires an exception such as serious and foreseeable harm (the ethics chapter covers the exceptions). When a client in ongoing care asks for family support, the protective move is to bring family in **with** the client's agreement.

The Client Sets the Scope

Consent is not all-or-nothing. A client may involve a partner in a new bedtime routine while keeping a past trauma or a diagnosis private. The release records that scope, and the counselor stays inside it.

Error	Why it fails
Overbroad disclosure: treating any release as opening the whole record	A release covers only what it names (ACA B.1.c; ACA B.2.e)
All-or-nothing consent: no involvement unless the client discloses everything	Coercive; blocks a support the client chose
Rigid separation: keeping family out because it "blurs the frame"	Wastes a resource the client asked to use (ACA A.1.d)

A client may widen the scope later or revoke it. Revocation stops future disclosures; it cannot undo past ones.

Give Supports a Role Tied to Goals

Education alone leaves a willing helper unsure what to do. Surveillance turns a helper into a monitor. The rewarded middle is **specific roles, agreed on by the client and the support person together, tied to treatment goals.** In bipolar disorder, family-focused therapy (Miklowitz, 2008) teaches families to spot early warning signs, communicate with less criticism, and follow a relapse-prevention plan the client helped write.

Pattern	Verdict
Collaborative role clarity: client and support person agree on what to notice and how to raise it	Rewarded
Covert monitoring: the support person reports to the counselor without the client's knowledge	Erodes trust and autonomy
Psychoeducation alone: reading material, no agreed roles	Helpful but incomplete
Handing over the goals: family sets goals because of past impairment	Removes the client's voice

Advocates, Peers, and Community Helpers

Support	What they do	What they do not do
Domestic violence advocate	Safety planning, shelter, legal and protective-order advocacy	Share survivor information without the survivor's release
Certified peer recovery specialist	Shares lived experience of recovery; offers hope, mutual support, and help with recovery goals (SAMHSA)	Diagnose, write the treatment plan, or prescribe (medications for opioid use disorder come from a prescriber)
Home-visiting nurse; community health worker	Health education, home visits, links to services	Replace the client's counseling goals
Case manager	Links to housing, benefits, transportation	Provide counseling

Domestic violence advocates. Programs funded under the Violence Against Women Act (VAWA) and the Family Violence Prevention and Services Act (FVPSA) generally may share a survivor's identifying information only with an informed, written, time-limited release. With the client's release, the counselor and advocate align on **one safety plan** that follows the client's choices. Calling for updates without consent breaches confidentiality; handing all safety planning to the advocate leaves gaps.

Shared plans with any helper. Agree on shared goals, each person's role, and how you will stay in contact. Parallel plans duplicate or contradict each other. Deferring the counseling goals to the helper's priorities abandons the counseling role. Sending full progress notes exceeds what shared goals require.

Wraparound and Other Team Models

Wraparound is a planning process, not a level of care. A facilitator brings the family, natural supports, and systems such as school, child welfare, and juvenile court into

one team that writes **one individualized, strengths-based plan**. Its ten principles (Bruns et al., 2004) are family voice and choice, team-based, natural supports, collaboration, community-based, culturally competent, individualized, strengths-based, persistence, and outcome-based. Wraparound grew within the **systems of care** framework for children's mental health (Stroul & Friedman, 1986).

Model	Who it serves	Core feature
Wraparound	Youth with needs across several systems, and their families	Family-driven team; one plan; natural supports
Assertive community treatment (ACT) (Stein & Test, 1980)	Adults with serious mental illness and frequent hospital use	Multidisciplinary team delivers services in the community, around the clock
Multisystemic therapy (MST) (Henggeler et al., 2009)	Youth with serious antisocial behavior	Intensive home-based treatment of family, peer, and school systems
Partial hospitalization (PHP)	People needing structured daily treatment without an overnight stay	A level of care: a day program

Exam point: "family-driven," "natural supports," and "one plan across systems" signal wraparound. "Adults with serious mental illness" plus "a team that comes to them" signals ACT.

Part 3 — Collaborating With Non-Clinical Stakeholders (5.2)

Non-clinical stakeholders include attorneys, courts, probation officers, schools, employers, child welfare workers, and community agencies. Many have their own legal duties, so their promise of confidentiality does not protect the client. The client's informed, documented consent does.

Attorneys and Courts

A request from an attorney, **even the client's own attorney**, does not authorize release. **ACA B.6.g** requires written client permission before records are disclosed or transferred to a legitimate third party, unless an exception applies. HIPAA requires a valid authorization for disclosures that are not treatment, payment, operations, or another permitted purpose (45 CFR 164.508).

Before the client signs, talk through how release could affect the case; records sent for a custody matter may reach the other side. Share only what the purpose needs (ACA B.2.e).

Document	What to do
Client's signed authorization	Release what it names, no more
Attorney's letter or request	Not an authorization; contact the client
Subpoena	Do not ignore it and do not simply comply; contact the client, consult legal counsel, assert privilege if the client has not waived it (see the ethics chapter)
Court order	Comply, seek the client's consent or ask to limit disclosure, and release as little as possible (ACA B.2.d; 45 CFR 164.512(e))

A blanket refusal is also wrong. With a valid authorization or court order, records can go to an attorney.

Probation, Parole, and Mandated Clients

ACA A.2.e requires counselors working with mandated clients to explain what information will be shared, with whom, and the limits of confidentiality. A probation officer may have to report to the court what he hears. Before an officer joins a session or receives anything, confirm a **current signed release names the officer and states what may be shared**. If the client is in a federally assisted substance use disorder program, 42 CFR Part 2 adds its own consent rules, including rules for criminal

justice referrals; verify the current Part 2 requirements, which have been revised in recent years.

Schools

With a parent's signed consent (and the student's assent where appropriate), the counselor may coordinate with a school counselor about classroom strategies. Share **only what the strategies need**: the goal, what helps, what to watch for. Full session notes are not needed (ACA B.2.e).

HIPAA does not bar consented coordination. Records a school holds are generally **education records under FERPA**, not protected health information under HIPAA; the counselor's own records stay under HIPAA and the ACA Code.

Employers and Other Third Parties

An employer, landlord, or human resources office asking about a client gets nothing without the client's authorization, not even confirmation that the person is a client. Tell the client about the request and let the client decide.

Advocacy Within Role

ACA A.7.a supports advocacy at the individual, group, institutional, and societal levels to address barriers to client growth. **ACA A.7.b** requires client consent before advocating for an identifiable client. The ACA Advocacy Competencies (Toporek et al., 2009) run from acting with the client, to acting on the client's behalf, to community and public-arena work.

Advocacy has a boundary. With consent, the counselor documents needs and functioning, writes support letters within competence, and **links the client to legal aid** or an attorney. Giving legal advice, filing complaints, and representing the client belong to attorneys. Refusing to help at all because "advocacy is outside counseling" ignores A.7.a.

Part 4 — Collaborating With Other Treatment Providers (5.3)

Care Coordination

When counselors learn a client is seeing another mental health professional, **ACA A.3** directs them to request a release to inform that professional and to build a collaborative relationship. The same logic applies to primary care, psychiatry, substance use treatment, and dietitians.

Good coordination has four parts:

1. **One plan, not parallel plans.** Agree on shared goals and who owns which piece.
2. **Clear roles.** The prescriber manages medication; the counselor provides counseling; the dietitian manages nutrition.
3. **A contact plan.** How often, by what secure channel (ACA B.3.e), and who calls whom when something changes.
4. **Documentation.** Each contact, what was shared, and what was decided.

When a referral moves care to another provider, **ACA A.11.d** requires completing the clinical and administrative steps and keeping communication open with both the client and the new provider. Transitions between levels of care are covered in the continuity-of-care section.

Interdisciplinary Teams

ACA B.3.b requires that clients be told when an interdisciplinary team serves them: that it exists, who is on it, what is shared, and why. **ACA D.1.c** asks counselors on teams to work toward the client's welfare while contributing the counseling profession's perspective.

Integrated care places behavioral health inside primary care. In the collaborative care model (Unützer et al., 2002), a care manager, the primary care provider, and a consulting psychiatrist track symptoms with repeated

measures and adjust treatment when the client is not improving.

What May Be Shared, Under Which Rule

Rule	What it says	Exam trap
HIPAA treatment disclosures (45 CFR 164.506)	Covered providers may share PHI with each other for treatment without authorization	The ACA Code (A.3, B.1.c) and many state laws still expect client consent; the rewarded answer gets a release
HIPAA minimum necessary (164.502(b), 164.514(d))	Applies to most uses and disclosures, but not to disclosures to a provider for treatment or those the client authorizes	The ethical duty to share only what is needed (ACA B.2.e) still applies
Psychotherapy notes (164.508(a)(2))	Separate process notes need a specific authorization, even when disclosed to another treating provider	Do not send them as part of routine coordination
42 CFR Part 2	Federally assisted SUD program records need consent that meets Part 2	Verify current requirements
State law	Some states are stricter than HIPAA for mental health records	HIPAA sets a floor; stricter state law applies

Consultation: Advice In, Responsibility Stays

Consultation brings in a colleague's expertise while the **treating counselor keeps responsibility** for the client's care. **ACA B.7.a** keeps consultation to professional purposes, with only the data germane to the question and every effort to protect the client's identity. **ACA B.7.b** bars sharing identifying information unless the client consents or it is unavoidable, and limits shared information to what the consultation needs. Contrast **supervision**, which is evaluative and shares responsibility, and **refer-**

ral or transfer, which moves responsibility to the new provider once complete.

Consultation for a specific skill or for ethical questions is covered in the self-awareness section.

Part 5 — Medication: Notice, Ask, Document, Pass It On (5.4)

Counselors do not prescribe. They often see the client more than anyone else on the team, though, so they are the first to hear that a medication causes drowsiness, kills appetite, ends sexual interest, or has not been taken in a week. The task is narrow: **notice the effect, ask about it, document it as the client's report, and, with consent, communicate it to the prescriber.** The prescriber decides what changes.

The Scope Line

ACA C.2.a limits practice to the counselor's education, training, and credentials, and counselor scope-of-practice laws do not include prescribing. Prescribers include psychiatrists, primary care physicians, psychiatric-mental health nurse practitioners, physician assistants, and, in a few states, psychologists with prescriptive authority.

Inside counselor scope	Outside counselor scope
Asking what the client takes and how it is going	Recommending, starting, adjusting, or stopping a medication
Recording reported benefits, side effects, timing, and adherence	Changing when a dose is taken, even "just take it at bedtime"
Recognizing possible side effects and passing them on	Deciding a dose is too high or too low, or that a different drug is needed
General psychoeducation (some medications take weeks to work); "talk with your prescriber before stopping"	Predicting that a side effect will fade

Inside counselor scope	Outside counselor scope
Referring for a medication evaluation; exploring ambivalence with motivational interviewing	Fluid, diet, or supplement advice that alters a medication's effect
Getting urgent medical help when danger signs appear	Interpreting lab values or diagnosing toxicity

Asking About Medication

Ask at intake and whenever the client mentions a change: what the client takes (including over-the-counter drugs, supplements such as St. John's wort, alcohol, and cannabis) and who prescribed it; when it started or last changed; what it helps; what side effects occur and when; how it affects work, school, sleep, driving, eating, relationships, and sex; whether doses are missed or the client is thinking of stopping; and whether the client will let the counselor share this with the prescriber.

Sexual side effects are a common reason people stop antidepressants and are rarely volunteered. Ask directly and without judgment.

Documenting the Self-Report

Element	Example
Name as the client gives it	"my anxiety pill," or the name read off the bottle
Prescriber	Psychiatric nurse practitioner
Start or change date	Started about three weeks ago, per client
Reported benefits	"I'm not lying awake worrying as much"
Reported side effects and timing	Nausea for an hour after the morning dose
Functional impact	Skipping breakfast meetings at work
Adherence	Reports missing two doses this week
Counselor action	With client's signed release, counselor will inform prescriber

Record benefits as well as side effects; the prescriber weighs both. “Client is on medication” is too vague to use. Record the report now; the client’s report is the data, and it does not need a physician’s confirmation first.

Self-report versus attribution. Document what the client reports, label it as the client’s report, and do not assign a cause.

Entry type	Example	Verdict
Self-report	“Client reports feeling ‘wired’ at bedtime since her ADHD medication was changed.”	Correct
Counselor observation	“Client yawned repeatedly; speech slowed late in session.”	Correct, labeled as observation
Counselor attributing cause	“The new medication is causing her insomnia.”	A medical judgment
Dosing opinion	“Dose appears too high.”	Prescriber’s role
Reframing as a psychiatric symptom	“Restlessness is likely anxiety, not a medication effect.”	Speculation in the record
Omission	Leaving the report out because medication “belongs in the physician’s chart”	Reported effects belong in the counseling record too

Passing It On: Routine, Prompt, Emergency

Urgency	Examples	Counselor action
Routine	Mild nausea, sexual side effects, daytime drowsiness, weight gain, questions	Document; with consent, inform the prescriber; encourage the client to raise it

Urgency	Examples	Counselor action
Prompt	New or worse suicidal thoughts after starting an antidepressant; akathisia; signs of mania after an antidepressant; stopping a medication abruptly; new rash on lamotrigine	Assess risk; contact the prescriber the same day; document
Emergency	The patterns in the emergency table below	Help the client get immediate medical evaluation (911 or emergency department); document what was reported and done

Consent governs routine communication with the prescriber. In an emergency, getting the client to medical care comes first; the ethics chapter covers when disclosure without consent is justified.

The antidepressant boxed warning. All antidepressants carry an FDA boxed warning for increased suicidal thoughts and behaviors in **children, adolescents, and young adults under age 25**, especially early in treatment and after dose changes. New suicidal thoughts, agitation, or restlessness in a young client who recently started an antidepressant call for a risk assessment, documentation, and prompt prescriber contact. Reassurance and advice to skip doses are both wrong; skipping doses is a medication decision and can cause discontinuation symptoms.

Antidepressant-induced mania. In a client with unrecognized bipolar disorder, an antidepressant can trigger hypomania or mania. Decreased need for sleep, racing thoughts, or risky spending soon after starting one call for prompt prescriber contact and a review of the diagnosis.

Drug-Class Reference

These tables give mechanisms, uses, side effects clients report, and what the counselor watches for. Doses are left out on purpose; they are outside counselor scope. Every agent in the antidepressant table carries the under-25 boxed warning described above. Mechanisms and side-effect profiles follow Stahl (2020, 2021) and FDA labeling.

Antidepressants

Class and agents	Mechanism and uses	Side effects clients report	Counselor watches for
SSRIs: fluoxetine (Prozac), sertraline (Zoloft), escitalopram (Lexapro), citalopram (Celexa), paroxetine (Paxil), fluvoxamine	Block the serotonin reuptake transporter. First-line for depression, persistent depressive disorder, GAD, social anxiety, panic, OCD, PTSD	Sexual dysfunction (desire, arousal, orgasm), nausea, diarrhea, insomnia or sedation, sweating, headache, weight change	Full effect often takes 4–6 weeks; a client who quits at week 2 "because it isn't working" needs general psychoeducation and a prescriber conversation. Fluoxetine's long half-life means fewer discontinuation symptoms
SNRIs: venlafaxine (Effexor), desvenlafaxine (Pristiq), duloxetine (Cymbalta)	Block serotonin and norepinephrine reuptake. Depression and anxiety disorders; duloxetine also for fibromyalgia and nerve and musculoskeletal pain	SSRI-like effects plus raised blood pressure (especially venlafaxine)	Discontinuation symptoms are often stronger than with SSRIs. Duloxetine is common when depression and chronic pain co-occur

Class and agents	Mechanism and uses	Side effects clients report	Counselor watches for
Bupropion (Wellbutrin; Zyban)	Blocks dopamine and norepinephrine reuptake. Depression, seasonal depression, smoking cessation	Insomnia, agitation, dry mouth; few sexual side effects	Lowers the seizure threshold, a concern with eating disorders, alcohol or sedative withdrawal, or seizure history (a key warning, not boxed). Sometimes added to offset SSRI sexual effects
Mirtazapine (Remeron)	Blocks alpha-2 and some serotonin receptors. Depression with insomnia and poor appetite	Sedation, increased appetite, weight gain	Often chosen for older adults with poor sleep and appetite
Trazodone	Serotonin antagonist and reuptake inhibitor. Often used off-label for sleep	Sedation, dizziness, lightheadedness on standing	**Priapism** is rare but an emergency
Tricyclics (TCAs): amitriptyline, nortriptyline, imipramine, desipramine	Block serotonin and norepinephrine reuptake plus cholinergic, histamine, and alpha-1 receptors. Now used mostly for pain, migraine prevention, and sleep	Dry mouth, blurred vision, constipation, urinary retention, confusion, dizziness on standing, sedation, weight gain, cardiac conduction effects	**Overdose is far more lethal than with SSRIs**: a suicidal client on a TCA calls for risk assessment, means-safety counseling, and prompt prescriber contact. Anticholinergic confusion is dangerous in older adults

Class and agents	Mechanism and uses	Side effects clients report	Counselor watches for
MAOIs: phenelzine (Nardil), tranylcypromine (Parnate), selegiline patch (Emsam)	Inhibit monoamine oxidase, which breaks down serotonin, norepinephrine, dopamine, and tyramine. Treatment-resistant and atypical depression	Dizziness on standing, insomnia, weight gain	Tyramine-rich foods (aged cheese, cured meats, some fermented foods, tap beer, soy sauce) can cause a **hypertensive crisis**. Serotonergic drugs (SSRIs, SNRIs, tramadol, some cough medicines) can cause **serotonin syndrome**; prescribers build in a washout period when switching, longer after fluoxetine
Dextromethorphan-bupropion (Auvelity)	NMDA receptor antagonist plus bupropion. Major depressive disorder in adults	Dizziness, headache, dry mouth	Antidepressant boxed warning; bupropion's seizure caution

Newer Agents Clients May Mention

Agent	What it is	What the counselor should know
Esketamine (Spravato) nasal spray	NMDA receptor antagonist for adults with treatment-resistant depression or with major depression and acute suicidal thoughts or behavior	Given only in a certified health care setting under a REMS program, with monitoring after each dose; never taken home. Boxed warnings: sedation, dissociation, respiratory depression, abuse and misuse, suicidal thoughts and behaviors. No driving until the next day after restful sleep, so transportation is a planning issue. Feeling “detached” after treatment is a reportable effect
Zuranolone (Zurzuvae)	Oral neuroactive steroid that enhances GABA-A activity; a short course for postpartum depression in adults	Boxed warning for impaired driving: no driving or hazardous activity for at least 12 hours after each dose. Coordinate with obstetric and psychiatric prescribers
Xanomeline-trospium (Cobenfy)	Oral antipsychotic for schizophrenia in adults that acts on muscarinic receptors instead of blocking dopamine D2 receptors; trospium limits effects outside the brain	Clients may report nausea, vomiting, indigestion, constipation, fast heartbeat, or raised blood pressure; urinary retention is a warning. It lacks the usual D2-blocker movement profile, but any new effect is still reported, not interpreted

Mood Stabilizers

Agent	Uses	Key risks	Counselor watches for
Lithium	Bipolar I and II, especially mania and maintenance; depression augmentation; linked to lower suicide risk	Boxed warning: **toxicity**. Narrow therapeutic range; the prescriber monitors blood levels. Long term: hypothyroidism, weight gain, thirst and frequent urination, acne, kidney effects	**Toxicity:** coarse tremor, vomiting, diarrhea, confusion, unsteady gait, slurred speech; in severe cases seizures and heart rhythm problems. Levels rise with dehydration, vomiting or diarrhea, heavy sweating, low-salt diets, NSAIDs (ibuprofen, naproxen), and some blood-pressure medicines. Mild, steady side effects are routine reports; a coarse tremor with GI symptoms or confusion is an emergency. Never advise on fluids or salt
Valproate (Depakote)	Bipolar disorder (including mixed features and rapid cycling), epilepsy, migraine prevention	Boxed warnings: **liver toxicity, pancreatitis, major birth-defect risk** (neural tube defects, lower IQ in exposed children)	Coordinate with the prescriber if a client who could become pregnant plans or reports a pregnancy

Agent	Uses	Key risks	Counselor watches for
Lamotrigine (Lamictal)	Bipolar maintenance, especially preventing depression; epilepsy	Boxed warning: **serious rash, including Stevens-Johnson syndrome**, more likely with rapid dose increases or with valproate	Report any new rash while starting it at once. Often preferred over valproate for people of reproductive age
Carbamazepine (Tegretol)	Bipolar disorder, epilepsy, trigeminal neuralgia	Boxed warnings: **serious skin reactions** (HLA-B*1502 screening in at-risk ancestry); **aplastic anemia and agranulocytosis**. Also liver effects and birth-defect risk	Lowers levels of many other drugs, including hormonal contraceptives. Fever, sore throat, or unusual bruising is reported promptly

Antipsychotics

Group and agents	Mechanism	Key side effects	Counselor watches for
First-generation (typical): haloperidol (Haldol), chlorpromazine, fluphenazine, perphenazine	Dopamine D2 blockade	**Extrapyramidal symptoms (EPS)**: **akathisia** (inner restlessness and urge to move, often mistaken for anxiety or worsening psychosis, and linked to distress and suicidality), **acute dystonia** (sudden spasms of neck, eyes, or jaw), **drug-induced parkinsonism** (tremor, rigidity, shuffling gait). **Tardive dyskinesia (TD)**: involuntary lip, tongue, or limb movements after long-term use; may be permanent. **Neuroleptic malignant syndrome (NMS)**	Report every movement change. TD is screened with the AIMS (Guy, 1976) and treated with VMAT2 inhibitors such as valbenazine (Ingrezza) and deutetrabenazine (Austedo). NMS is an emergency

Group and agents	Mechanism	Key side effects	Counselor watches for
Second-generation (atypical): risperidone (Risperdal), olanzapine (Zyprexa), quetiapine (Seroquel), aripiprazole (Abilify), ziprasidone (Geodon), lurasidone (Latuda), brexpiprazole (Rexulti), cariprazine (Vraylar), lumateperone (Caplyta)	D2 and serotonin 5-HT2A blockade, or D2 partial agonism (aripiprazole, brexpiprazole, cariprazine)	**Metabolic effects**: weight gain, raised blood sugar and lipids (highest with olanzapine and clozapine, notable with quetiapine); sedation; EPS and akathisia are possible	Report significant weight gain, new thirst, or frequent urination; the prescriber monitors weight, glucose, and lipids. Quetiapine is often used off-label for sleep. Agents approved for depression carry the antidepressant boxed warning

Group and agents	Mechanism	Key side effects	Counselor watches for
Clozapine (Clozaril)	Multiple receptors; weak D2 blockade	Boxed warnings: **severe neutropenia**, orthostatic hypotension and fainting, **seizures**, **myocarditis and cardiomyopathy**, mortality in older adults with dementia-related psychosis. High metabolic risk; drooling; constipation that can become dangerous	The most effective antipsychotic for treatment-resistant schizophrenia, with evidence for reducing suicidal behavior in schizophrenia. The prescriber orders regular blood (ANC) monitoring. Report fever, sore throat, chest pain, or a racing heart promptly. The FDA ended the clozapine REMS program in 2025, so registry enrollment and ANC reporting to the registry are no longer required; the monitoring itself continues

First-generation, second-generation, and clozapine labels carry a boxed warning for **increased mortality in older adults with dementia-related psychosis**; xanomeline-trospium has no boxed warning. TD and NMS are major warnings but not boxed.

Anxiolytics and Sleep Medications

Class and agents	Mechanism	Key risks	Counselor watches for
Benzodiazepines: diazepam (Valium), lorazepam (Ativan), alprazolam (Xanax), clonazepam (Klonopin), chlordiazepoxide (Librium)	Enhance GABA-A receptor activity, depressing the central nervous system	Boxed warnings: **abuse, misuse, and addiction; dependence and withdrawal reactions; profound sedation, slowed breathing, and death when combined with opioids**. Sedation, memory problems, falls in older adults	Not a long-term anxiety treatment; dependence can develop within weeks. **Withdrawal after regular use can cause seizures and can be life-threatening**, so stopping needs medical management. Ask about alcohol and opioids. Used medically for alcohol withdrawal
Buspirone	Serotonin 5-HT1A partial agonist	Dizziness, nausea, headache	For GAD; takes 2–4 weeks; useless as-needed or for panic. No dependence. Clients used to fast benzodiazepine relief may call it ineffective

Class and agents	Mechanism	Key risks	Counselor watches for
Z-drugs: zolpidem (Ambien), zaleplon (Sonata), eszopiclone (Lunesta)	Act at GABA-A receptors	Boxed warning: **complex sleep behaviors** (sleepwalking, sleep-driving, sometimes with no memory and serious injury)	Report complex sleep behavior promptly. Long-term use is a reason to suggest CBT-I, the first-line treatment for chronic insomnia (Qaseem et al., 2016), and to support a prescriber conversation

ADHD Medications

Class and agents	Mechanism	Key side effects	Counselor watches for
Stimulants: amphetamine salts (Adderall), lisdexamfetamine (Vyvanse, a prodrug with lower misuse potential), methylphenidate (Ritalin, Concerta), dexmethylphenidate (Focalin)	Increase dopamine and norepinephrine release and block reuptake	**Reduced appetite** (growth monitored in children), **trouble falling asleep**, raised heart rate and blood pressure, irritability, tics, rebound as doses wear off	Schedule II. Boxed warning: **abuse, misuse, and addiction**. Watch for taking extra, use to stay awake or study, and diversion. Cardiac conditions need medical coordination
Atomoxetine (Strattera)	Selective norepinephrine reuptake inhibitor	GI upset, decreased appetite, fatigue	Not controlled; takes weeks. Boxed warning: **suicidal thoughts in children and adolescents**

Class and agents	Mechanism	Key side effects	Counselor watches for
Viloxazine (Qelbree)	Norepinephrine reuptake inhibition, among other actions	Sleepiness, decreased appetite	Boxed warning: suicidal thoughts and behaviors
Alpha-2 agonists: guanfacine ER (Intuniv), clonidine ER (Kapvay)	Stimulate alpha-2 receptors	Sedation, low blood pressure; rebound high blood pressure if stopped abruptly	Helpful with tics or sleep problems. Clonidine, guanfacine, and the alpha-1 blocker prazosin are also used off-label for PTSD nightmares

Substance Use and Neurocognitive Medications

Medication	Use	Counselor watches for
Buprenorphine (alone or with naloxone); **methadone**	Opioid use disorder	Treatment, not "trading one drug for another." Methadone for OUD comes through opioid treatment programs. Benzodiazepines or alcohol add overdose risk
Naltrexone (oral or monthly injection)	Opioid (after withdrawal) and alcohol use disorders	Lower opioid tolerance after stopping raises overdose risk if use resumes
Disulfiram (Antabuse)	Alcohol use disorder; alcohol causes a severe reaction (flushing, nausea, pounding heart)	Boxed warning: never given to an intoxicated person or without full knowledge. Hidden alcohol (mouthwash, cooking) can trigger it

Medication	Use	Counselor watches for
Naloxone (Narcan)	Reverses opioid overdose	Belongs in the safety plan of anyone at risk; available over the counter
Cholinesterase inhibitors: donepezil (Aricept), rivastigmine (Exelon), galantamine (Razadyne); **memantine** (Namenda, an NMDA antagonist)	Alzheimer's disease; cholinesterase inhibitors also in Lewy body dementia	They give modest, temporary symptom benefit and do not stop or reverse the disease, which helps set realistic expectations with families. Nausea, diarrhea, slow heart rate; dizziness and confusion with memantine

In older adults, the AGS Beers Criteria (American Geriatrics Society, 2023) flag benzodiazepines and strongly anticholinergic drugs as medications to avoid or use with caution. Report new confusion or falls. The substance use chapter covers withdrawal management and medication decisions in addiction.

Emergencies to Recognize

The counselor does not diagnose these. The counselor recognizes the pattern well enough to get the client to medical care, then documents what was reported and done.

Emergency	Typical cause	What the client may report or show
Serotonin syndrome	MAOI with SSRIs, SNRIs, tramadol, or other serotonergic drugs; St. John's wort or MDMA combinations	Agitation, confusion, sweating, fever, fast heartbeat, muscle twitching or rigidity, diarrhea
Hypertensive crisis	MAOI with tyramine-rich foods or some decongestants	Sudden severe headache, stiff neck, pounding heart, nausea

Emergency	Typical cause	What the client may report or show
Lithium toxicity	Dehydration, GI illness, low salt, NSAIDs, some blood-pressure drugs	Coarse tremor, vomiting, diarrhea, confusion, unsteady gait, slurred speech
Neuroleptic malignant syndrome	Antipsychotics	High fever, severe rigidity, confusion, unstable pulse and blood pressure
Respiratory depression	Benzodiazepines with opioids or alcohol	Extreme drowsiness, slow or shallow breathing, unresponsiveness
Sedative or alcohol withdrawal	Abrupt stop after regular use	Tremor, sweating, agitation, hallucinations, seizures
Serious skin reaction	Lamotrigine, carbamazepine	Spreading rash, blisters, mouth sores, fever
Priapism	Trazodone, rarely others	Prolonged, painful erection

Discontinuation symptoms are not an emergency but are often mistaken for relapse. Stopping an SSRI or SNRI abruptly, especially paroxetine or venlafaxine, can cause the FINISH symptoms (Berber, 1998): flu-like symptoms, insomnia, nausea, imbalance, sensory disturbances ("brain zaps"), and hyperarousal. Document the report and encourage the client to talk with the prescriber before stopping any medication.

Part 6 — The Referral Resource List (5.5)

Why the List Is an Ethical Duty

ACA A.11.a expects counselors to know **culturally and clinically appropriate referral resources** and to suggest them when they lack the competence to help; if the client declines the referral, the counselor discontinues the relationship. An outdated list fails that duty at the mo-

ment it matters. **ACA A.12** bars abandonment, so ending services without arranging alternatives is not an option. **ACA A.10.b** prohibits fees, commissions, or rebates for referrals.

What Each Entry Holds

Field	Why it matters
Name, location, telehealth, contact	The client can reach it
Specialty, populations, and treatments offered	Fit for the need (eating disorders, trauma, adolescents, OCD)
License status, verified with the state board	The provider is qualified
Languages offered; accessibility	Language and disability access
Insurance accepted, sliding scale, cost	The client can afford it
Openings or waitlist	Avoids dead-end referrals
Level of care (outpatient, IOP, PHP, residential, inpatient)	Matches the assessed need
Date last verified	Shows when the entry was last checked

Keeping It Current

- **Verify on a schedule** (specialty, license, openings, insurance) and whenever a client reports a problem.
- **Vet before adding.** A name a colleague mentions in passing is a lead, not a listing.
- **Retire dead entries** promptly, and check what replaced them.
- **Avoid reciprocity as a criterion.** Keeping only providers who refer back is a conflict of interest.

When a listed resource turns out to be closed or full: secure a current option for the client **now**, then update the list and set a routine check. Handing over the dead listing, dropping to a lower level of care because the list failed, telling the client to search alone, or deleting

the entry and waiting for an annual review all shift the cost of a stale list onto the client.

Making the Referral

Explain why the referral is recommended and involve the client in the decision. Offer **several vetted options** that fit the need so the client chooses; naming one preferred colleague limits choice and can look like a conflict of interest, and sending the client to search online unaided skips the vetting. Get a signed release before sharing information, and **follow up** to confirm the client connected (ACA A.11.d).

Matching the Resource to the Client

Resource	Best fit
Community mental health center	Uninsured or low-income clients needing ongoing care; sliding-scale fees
Federally qualified health center (FQHC)	Integrated primary and behavioral care with sliding-fee discounts
Employee assistance program (EAP)	Employed clients; short-term, employer-sponsored; not available to someone unemployed
Out-of-network private practice	Clients who can pay or have out-of-network benefits
University training clinic	Low-cost care from supervised trainees
Mutual-help groups (AA, NA, SMART Recovery, NAMI programs)	Peer support alongside treatment
Legal aid; vocational rehabilitation; housing and benefits agencies	Legal, work, and basic-needs barriers

Crisis and National Lines

Line	What it does
988 Suicide & Crisis Lifeline	National three-digit line for suicidal and mental health crises; call, text, or chat. Veterans and service members press 1 after dialing 988 for the Veterans Crisis Line
911	Immediate danger to life; medical emergencies
Crisis Text Line	Text HOME to 741741
211	Community information and referral: housing, food, utilities, social services
311	Non-emergency city services in some cities
National Domestic Violence Hotline	1-800-799-7233
SAMHSA National Helpline	Treatment referral and information; 1-800-662-4357
Poison Control	1-800-222-1222, including medication overdoses and mix-ups

Every referral list carries 988. Do not confuse it with 211 (social services) or 311 (municipal services).

Part 7 — Reviewing the Record Before the Session (5.6)

Pre-session review turns last week's session into this week's plan. It is a clinical task, not a billing one, done before the client arrives.

What to Review

Item	Why
Last progress note and its plan	Continuity: what was assigned, what was left open
Treatment plan goals and objectives	Keeps the session tied to the client's goals

Item	Why
Homework and self-monitoring data (drinking log, thought records, activity schedule)	Shows what happened between sessions
Repeated measures (PHQ-9, GAD-7, others)	Trend over time
Risk history and current safety plan	Prior risk carries forward
Medication changes reported	Something to follow up
Collateral contacts and messages since last session	New information from family, providers, or the client
Releases on file: who, what scope, expiration	Determines who may join or be contacted
Pending tasks: referral follow-up, letters, coordination calls	Closes loops

Memory misses details and trends. Intake paperwork alone misses recent progress. Skimming notes in session spends the client's time on the counselor's preparation.

Turning Review Into a Plan

Risk carries forward. When a recent note records suicidal thinking, plan to reassess risk and review the safety plan **early** in the session. Waiting for the client to bring it up risks missing a change; jumping to a higher level of care before a current assessment goes beyond the evidence. The crisis chapter covers risk assessment.

Measurement-based care. Reviewing repeated scores lets the counselor see the trend and adjust treatment, ideally reviewing that trend with the client (Lewis et al., 2019). The PHQ-9 (Kroenke et al., 2001) is a screening and severity measure; a score does not confirm a diagnosis without further assessment.

Consent check. If someone else will attend, confirm a **current signed release** names that person and states what may be shared. An expired release is not a release.

Covering for a Colleague

Before seeing a colleague's client during a leave, review the treatment plan, recent notes, and risk history. "Starting fresh" risks missing risk factors; asking for a full retelling burdens the client with what the record already holds. The continuity-of-care section covers transfers and handoffs.

Part 8 — Language Access (5.7)

Interpreter or Translator

Role	Works with	Example
Interpreter	**Spoken or signed** language, in real time	Session with a client whose preferred language is not English; ASL interpreting for a Deaf client
Translator	**Written** text	Informed consent form, safety plan handout, intake forms
Sight translation (by an interpreter)	Reading a written document aloud in another language	Reviewing a consent form during the session

Who Is Qualified, and Who Is Not

A **qualified interpreter** is proficient in both languages, trained in interpreting, bound by confidentiality and an interpreter code of ethics, and able to interpret accurately and impartially, ideally with mental health experience. Being bilingual is not the same skill.

Do not use	Why
The client (interpreting for a family member)	Dual role; filtered or inaccurate messages; the client cannot both participate and interpret
Family members	Bias, protectiveness, secrets, and confidentiality problems; they may soften or omit

Do not use	Why
Minor children	Burdens the child, exposes the child to adult clinical content, and risks inaccuracy; only in an emergency when no qualified interpreter is available
Untrained bilingual staff (such as a receptionist)	Not trained in accuracy or role; confidentiality issues
Consumer translation apps	Inaccurate for clinical content and a privacy risk
Speaking English slowly	Does not provide meaningful access

When no in-person interpreter is available, a **qualified telephone or video remote interpreter** gives accurate access right away.

The Rules

ACA A.2.c directs counselors to communicate in culturally and developmentally appropriate ways and, when clients have difficulty understanding the language counselors use, to arrange **a qualified interpreter or translator** to ensure comprehension.

ADA Title III treats private counseling offices as places of public accommodation. They must provide auxiliary aids and services, including qualified interpreters, for **effective communication** with people who are deaf, hard of hearing, or have other communication disabilities (28 CFR 36.303). The provider may not charge the client for them (28 CFR 36.301(c)). Under 36.303(c), the provider may not require the person to bring someone to interpret; an accompanying adult may interpret only in an emergency involving an imminent threat to safety or welfare when no interpreter is available, or when the person specifically requests it, the adult agrees, and reliance is appropriate; a minor child may interpret only in that kind of emergency.

Title VI of the Civil Rights Act and **Section 1557 of the Affordable Care Act** require programs that receive

federal financial assistance, including many health programs, to provide meaningful access for people with limited English proficiency. Implementing rules have changed over time; verify current requirements.

The **National CLAS Standards** (HHS Office of Minority Health, 2013) call for free, timely language assistance, notice that it is available, competent interpreters rather than untrained individuals or minors, and materials in the languages common in the service area.

Deaf and Hard-of-Hearing Clients

For a Deaf client whose preferred language is American Sign Language, arrange a **qualified ASL interpreter**. Written notes are too slow and limited for counseling, and English may be the client's second language. Lip-reading (speechreading) captures only part of spoken English. Ask the client what works; preferences vary.

Running an Interpreted Session

Before: - Book the interpreter in advance, in the client's preferred language and dialect, and check that the interpreter does not know the client personally. - **Brief the interpreter**: the session's purpose, sensitive topics, key terms, confidentiality, and the expectation to interpret everything in the first person.

During: - Seat yourself facing the client; an ASL interpreter sits near the counselor so the client can see both. - **Speak directly to the client** in the first person ("How have you been sleeping?"), not to the interpreter ("Ask her how she has been sleeping"). - Speak in **short segments** so the interpreter can render everything accurately. - Expect everything to be interpreted. **Summaries lose meaning and emotional content.** - If the interpreter adds advice, edits, or side conversations, pause and restate the interpreter's role.

After: - Debrief the interpreter: clarify cultural or language points, and check in after distressing content, since interpreters are exposed to traumatic material too.

Written Materials and Consent

A signature on a form the client cannot read does not make consent informed. Simple English still does not reach a client whose preferred language is not English. Provide a **qualified translation** of the consent form, or have a qualified interpreter sight-translate it, and **review it together**, answering questions through the interpreter (ACA A.2.c).

Documenting Language Access

Record the client's preferred spoken and written language, the interpreter's name or ID number, the mode (in person, phone, video), and any translated documents. If a client declines a professional interpreter, document the offer and the refusal.

Part 9 — Writing Clinical Documentation (5.8)

Why the Note Exists

ACA A.1.b requires sufficient, timely documentation that supports the delivery and continuity of services and accurately reflects progress and services; **ACA B.6.a** restates the duty to keep necessary records. The note serves care first, telling the next session, a covering counselor, and coordinating providers what happened. It also supports billing, meets legal requirements, and shows the reasoning behind decisions.

What Every Progress Note Contains

- Client identifier, date, start and end time, service type, setting (telehealth included)
- Who attended, including interpreters and support persons
- The client's report, quoted where the words matter; the counselor's observations and any measure scores
- Interventions, tied to the treatment plan, and the client's response

- **Risk assessment** and any safety actions
- Progress toward goals; reported medication effects and adherence
- Collateral contacts, consultations, and releases
- The plan; the counselor's signature, credentials, and date signed

Write in plain, objective, behavioral language. Leave out judgmental labels ("manipulative"), speculation about people who are not clients ("her mother is clearly narcissistic"), and details irrelevant to care.

Note Formats

Format	Sections	Where things go
SOAP (Weed, 1968)	**S**ubjective, **O**bjective, **A**ssessment, **P**lan	Client's report in S; counselor observations and scores in O; clinical impression and progress in A; next steps in P
DAP	**D**ata, **A**ssessment, **P**lan	Client report and counselor observations combined in D
BIRP	**B**ehavior, **I**ntervention, **R**esponse, **P**lan	Client's presentation in B; what the counselor did in I; how the client responded in R
GIRP	**G**oal, **I**ntervention, **R**esponse, **P**lan	Starts from the treatment-plan goal addressed

Exam favorites: the client's quoted statement goes in S (SOAP) or D (DAP). A score of 14 on the PHQ-9 goes in O (SOAP) or D (DAP). "Client rated anxiety 7 before and 4 after the grounding exercise" is the client's response to an intervention: R in BIRP or GIRP.

Self-Report, Observation, and Attribution

The note keeps three kinds of statements distinct:

Kind	Signal words	Example
Client self-report	“Client reports,” “client states,” quotation marks	Client states, “I’ve been up every night since Sunday.”
Counselor observation	“Counselor observed,” “client presented with”	Speech slowed; affect flat; eye contact limited
Clinical assessment	“Assessment,” “impression,” “consistent with”	Symptoms consistent with continued moderate depression; progress toward objective 2 limited

Counselors may record clinical impressions within their competence, including diagnostic impressions. They do not record medical conclusions, such as what caused a side effect or whether a dose is right. Those stay as the client’s report plus the action taken.

When to Write

- **During the session (collaborative documentation):** the counselor and client write part of the note together in the last minutes, reviewing what was covered and the plan. It can improve accuracy and engagement. Explain it in informed consent, and do not let typing replace attending.
- **After the session:** write as soon as practical, while memory is accurate, and within the deadline your agency, payer, or state sets. Always finish before the next session and before any handoff.

Corrections, Late Entries, and Addenda

Situation	Correct action	Wrong action
Something omitted from a signed note	A **dated, signed addendum** that references the original session date and states the added information	Rewriting the signed note (can look like falsifying the record); adding it to next week’s note as new information (misdates when the client said it)

Situation	Correct action	Wrong action
An error in a note	Mark the error as corrected (in paper records, a single line through it, initialed and dated; in an EHR, the amendment function), keeping the original readable	Erasing, obliterating, or deleting
A note written late	Label it a late entry with the actual date written and the date of service	Backdating

ACA A.1.b expects amendments to be noted according to agency or institutional policy. Under HIPAA, a client may request an amendment to the record (45 CFR 164.526). The provider may deny a request when the record is accurate and complete, and the client may then file a statement of disagreement.

Psychotherapy Notes and the Clinical Record

HIPAA separates the **clinical record** (dates and times of service, diagnosis, treatment plan, progress summaries, medications, test results) from **psychotherapy notes**: the counselor's analysis of session content, kept separately from the rest of the record. Psychotherapy notes need a specific authorization for most disclosures (45 CFR 164.508(a)(2)) and are excluded from the client's right of access under HIPAA (45 CFR 164.524), though state law may differ. Medication reports, risk assessments, and anything another provider needs belong in the clinical record, not only in psychotherapy notes. Clients' access to their records under the ACA Code is covered in the ethics chapter.

Documenting Coordination

Each collateral contact gets an entry: date, who, by what means, the release that authorized it, what was shared, and what was decided.

What the Exam Rewards / What It Punishes

Rewards: - A signed release naming the person, the scope, and the purpose before any contact - Specific, client-endorsed roles for supports, tied to goals - One coordinated plan with clear roles and a contact plan - Advocacy within the counselor's role, plus links to legal services - Recording medication effects as self-report and passing them to the prescriber with consent - Immediate medical evaluation for toxicity or emergency signs - A verified referral list, several options, and active follow-up - Pre-session review of the last note, goals, data, risk, and releases - Qualified, briefed interpreters; speaking directly to the client - Timely, objective notes; dated addenda for corrections

Punishes: - Calling family, attorneys, officers, or advocates without consent - Treating a release as opening the whole record - Turning supports into covert monitors - Doing someone else's job (filing legal complaints, setting family goals for the client, adjusting medication) - Medication advice, dosing opinions, or attributing a cause in the note - Reassuring away new suicidal thoughts in a young client on an antidepressant - Handing a client a dead or unvetted referral - Walking into a session without reviewing the record - Family members, children, apps, or slow English in place of an interpreter - Rewriting signed notes or backdating entries

Swap-Pair Drill

If you see…	It's…	Not…
Client wants family involved	Signed release first, then a goal-focused session (ACA A.1.d)	Calling family directly
Client limits what may be shared	Release defines the scope (ACA B.1.c)	All-or-nothing consent

If you see…	It's…	Not…
Family-driven team, one plan across school, court, and home	Wraparound	ACT
Adults with serious mental illness; team delivers services in the community	ACT	Wraparound
Lived-experience recovery support	Peer recovery specialist	Licensed clinician
Client's own attorney requests records	Client's signed authorization (ACA B.6.g; 45 CFR 164.508)	Release because attorneys have access
Officer joining a mandated client's session	Current release naming the officer and scope (ACA A.2.e)	Officer's promise of confidentiality
Records the school holds	FERPA	HIPAA
Counselor learns client sees another therapist	Request a release to inform and collaborate (ACA A.3)	Proceed alone
Consultant advises; counselor keeps responsibility	Consultation	Supervision
Client reports a side effect	Document as self-report; inform prescriber with consent	Advise a change
Under 25, new suicidal thoughts after starting an antidepressant	FDA boxed warning: assess risk, contact prescriber	Wait for it to pass
Lithium plus coarse tremor, vomiting, confusion	Possible toxicity: immediate medical care	Common side effects
Child with poor appetite and late sleep onset on a new medication	Stimulant side effects	Antipsychotic side effects
Inner restlessness on an antipsychotic	Akathisia	Anxiety disorder

If you see…	It's…	Not…
Involuntary lip and tongue movements after years on an antipsychotic	Tardive dyskinesia	Acute dystonia
Severe headache after aged cheese on an MAOI	Hypertensive crisis	Serotonin syndrome
Counselor lacks competence	Know resources, suggest alternatives (ACA A.11.a)	Abandonment (ACA A.12)
National mental health crisis line	988	211
Uninsured client needs ongoing care	Sliding-scale community mental health center	EAP
Spoken words in real time	Interpreter	Translator
Deaf client who uses ASL	Qualified ASL interpreter (ADA Title III, 28 CFR 36.303)	Written notes or lip-reading
Data, Assessment, Plan	DAP	SOAP
Behavior, Intervention, Response, Plan	BIRP	DAP
Omission found after signing	Dated, signed addendum	Rewriting the note

Quick Checks

1. A client says, "When I forget my new antidepressant for a day, I get dizzy and feel little electric zaps." Which progress note entry is most appropriate? A. "Client reports dizziness and 'electric zap' sensations on days she misses a dose; with her consent, counselor will inform the prescriber." B. "Client is experiencing antidepressant discontinuation syndrome due to missed doses." C. "Counselor advised client to take a half dose whenever she

misses one.” D. “Client’s dizziness is most likely a somatic symptom of her anxiety disorder.”

2. A client who started haloperidol last month says he feels an “inner restlessness” and paces throughout the session. The counselor should recognize a possible: A. Worsening of psychosis that calls for a higher level of care B. Akathisia, which the counselor documents and reports to the prescriber C. Generalized anxiety that relaxation training alone will resolve D. Tardive dyskinesia, which is permanent and needs no report

3. A client taking phenelzine calls an hour after eating aged cheese and cured meats. She reports a sudden, severe headache, a stiff neck, and a pounding heart. The counselor should: A. Note the symptoms and recheck them at the next session B. Suggest a pain reliever and rest until the headache eases C. Help her get emergency medical care now and document the call D. Leave a message asking the prescriber to consider a dose change

4. A client’s employer calls to ask whether the client has been attending counseling. No release is on file. The counselor should: A. Confirm attendance only, since attendance is not clinical content B. Share a general progress summary so the employer can support her C. Tell the employer to send a subpoena if the information is needed D. Neither confirm nor deny that she is a client, and tell her about the call

5. At intake, a client mentions she also sees a psychiatrist. The counselor’s best next step is to: A. Proceed independently, since the two practices are separate B. Call the psychiatrist without asking her, since HIPAA permits treatment disclosures C. Discuss coordination with her and request a release to contact the psychiatrist D. Ask her to choose between the psychiatrist and counseling

6. During pre-session review, a counselor sees that the release allowing a client’s partner to join sessions expired last month. The partner is coming today. The counselor should: A. Let the partner join, since the client agreed to

involvement once before B. Raise it with the client at the start and get a new signed release before sharing information C. Cancel the session until a renewed release arrives by mail D. Send the partner the recent notes so he is prepared

7. Midway through an interpreted session, the counselor realizes the interpreter is adding her own advice. The counselor should: A. Continue, since the advice seems helpful B. Ask the interpreter to summarize the client's answers instead C. Ask the client's 12-year-old, who came along, to interpret instead D. Pause, restate that the interpreter renders everything without additions, and debrief afterward

8. A private-practice counselor arranges a qualified ASL interpreter for a Deaf client. Under ADA Title III, the interpreter's cost: A. Is borne by the practice, with no surcharge to the client B. May be added to the client's session fee C. Is split between the client and the practice D. Is waived only if the client requests it in writing

9. In a BIRP note, "Client rated anxiety 8 before and 5 after practicing paced breathing" belongs in: A. Behavior B. Intervention C. Response D. Plan

10. A client says she receives esketamine for treatment-resistant depression. Which statement is accurate? A. She self-administers the nasal spray at home each morning B. It is given in a certified health care setting with monitoring after each dose C. It is an SSRI with no boxed warning beyond the antidepressant warning D. It is a benzodiazepine prescribed for anxiety

Answers and Rationales

1 — A. Owner: documenting self-report, with consent-based communication to the prescriber (task 5.4). B: Counselor attributing cause; naming the syndrome is a medical judgment. C: Medication advice; dosing belongs to the prescriber. D: Reframing a reported effect as a psychiatric symptom; speculation in the record.

2 — B. Owner: extrapyramidal symptoms of first-generation antipsychotics (akathisia). A: Level of care beyond the evidence; a new effect after starting an antipsychotic is reported, not assumed to be psychosis. C: Misattributing a side effect to an anxiety disorder. D: Tardive dyskinesia means involuntary movements after long-term use, and every movement change is reported.

3 — C. Owner: MAOI–tyramine hypertensive crisis; emergency response. A: Minimizing a medical emergency. B: Medication advice outside scope that delays care. D: Routine prescriber contact is too slow, and dose decisions are not the counselor's.

4 — D. Owner: ACA B.1.c and HIPAA authorization (45 CFR 164.508). A: Confirming that someone is a client is itself a disclosure. B: Overbroad disclosure without consent. C: Steering a third party toward legal process is not the counselor's role and does not protect the client.

5 — C. Owner: ACA A.3, Clients Served by Others. A: Parallel, uncoordinated care. B: HIPAA permits treatment disclosures (164.506), but the ACA Code expects a release request and collaboration with the client's knowledge. D: Coercive and against client welfare.

6 — B. Owner: current, written, scoped consent (ACA B.1.c; 45 CFR 164.508). A: An expired release is not consent. C: Overcorrection; the client can sign a new release on site. D: Disclosure without authorization.

7 — D. Owner: qualified interpreter practice, complete first-person interpretation (ACA A.2.c). A: Additions distort the client's communication. B: Summarized interpretation loses meaning and emotional content. C: A minor as interpreter; the National CLAS Standards call for avoiding minors as interpreters.

8 — A. Owner: ADA Title III, 28 CFR 36.301(c) and 36.303. B: A surcharge for an auxiliary aid is prohibited. C: Cost-sharing is still a charge to the client. D: No written request is needed; the obligation is the provider's.

9 — C. Owner: BIRP format (Response). A: Behavior is the client's presentation. B: Intervention is what the counselor did. D: Plan is next steps.

10 — B. Owner: esketamine REMS and FDA labeling. A: Esketamine is not dispensed for home use. C: Esketamine is an NMDA receptor antagonist with its own boxed warnings. D: It is not a benzodiazepine.

PART THREE

Practice

NCE Practice Exam 1

170 items · 3 options each · 3 hours 45 minutes · Answer every item.

Take this exam under timed conditions before checking the key. Use the four-step routine on every item: read the last line first, name the owner, predict, then eliminate by owner.

Pacing checkpoints: item 43 by 0:50 · item 85 by 1:39 (end of the first half: review it, submit it, take the optional break) · item 128 by 2:29 · item 170 by 3:18.

Original items written for this book. Not affiliated with or endorsed by NBCC. No content from any live examination.

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1. A woman repeatedly visits emergency rooms with symptoms she secretly causes by misusing medication. She gains no money, time off, or legal advantage and seems to want to be cared for as a patient. This most suggests:

A. Somatic symptom disorder
B. Factitious disorder imposed on self
C. Malingering

2. A client with severe depression needs structured treatment most of each weekday. He has no suicidal intent, is medically stable, and has a supportive home to return to each evening. Which level of care fits best?

A. Residential treatment
B. Weekly outpatient counseling
C. Partial hospitalization

3. During an in-office session, the building’s fire alarm sounds. The counselor’s most appropriate first action is to:

A. Follow the practice’s evacuation plan and leave with the client
B. Finish the session unless smoke or flames become visible
C. Lock the client’s paper chart in the file cabinet first

4. A client who worked hard in counseling invites the counselor to her college graduation. The counselor’s most appropriate approach is to:

A. Attend and bring a gift, with no need to document
B. Weigh risks and benefits and document the rationale
C. Decline, since any outside contact is a violation

5. A counselor sees that a colleague’s website features a glowing testimonial from one of the colleague’s current clients. No client appears to have been harmed. The counselor’s best first step is to:

A. Ignore it, since advertising isn’t a clinical issue
B. File a complaint with the state licensing board
C. Raise the concern with the colleague informally

6. A counselor has completed an introductory workshop on behavioral activation but has little experience using it. Before using it with a depressed client, the counselor’s best step is to:

A. Use it and rely on brief online summaries to guide sessions
B. Consult a colleague experienced in behavioral activation
C. Refer the client to a counselor with more experience

7. A client in an intensive outpatient program now has fewer symptoms, is back at work, and is using her coping skills consistently. The most appropriate level-of-care decision is to:

A. Continue IOP until all symptoms resolve
B. Step down to standard outpatient
C. Step up to partial hospitalization

8. During outpatient counseling for depression, a client discloses daily purging and recent weight loss. Along with continuing counseling, the counselor should refer her to:

A. A medical provider and a dietitian
B. A case manager for housing resources
C. A substance use treatment program

9. A client asks the counselor to involve his family in supporting his recovery from depression. The counselor's best first step is to:

A. Call his family to explain his diagnosis and risk level
B. Tell him family involvement can blur the therapy frame
C. Obtain his signed release, then plan a family session

10. A counselor with no eating disorder training learns that a client needs specialized eating disorder treatment soon, and supervision can't close the gap in time. The counselor should:

A. End services, since treating her would be unethical
B. Refer the client to an appropriately trained provider
C. Continue treatment while reading about eating disorders

11. Midway through an intake, a client says quietly, "Something happened when I was deployed," and looks away. The counselor's best response is to:

A. Ask the client to describe exactly what happened, in full detail
B. Skip trauma questions for now so rapport is not disrupted
C. Acknowledge it, ask permission to return to it, and offer a brief screen

12. A client reports depressed mood, poor sleep, job stress, and passing thoughts that her family would be better off without her. How should the counselor help her prioritize these problems?

A. Address safety first, then rank the rest with her by distress

B. Start with job stress, since it triggered the other symptoms
C. Rank the problems privately by severity and share the order

13. A client's panic attacks began at age 14, three years before she started drinking, and continued during a 6-month period of sobriety. This history best supports:

A. Alcohol withdrawal
B. An independent anxiety disorder
C. Alcohol-induced anxiety disorder

14. A client says her partner has never hit her but monitors her phone, controls all the money, and has cut her off from friends. Which concept best fits?

A. Enmeshment
B. Coercive control
C. The cycle of violence

15. A client asks for a copy of her counseling record. Under HIPAA, the counselor should:

A. Require a subpoena before releasing any part of the record
B. Provide timely access, excluding separately kept psychotherapy notes
C. Write a treatment summary in place of the record she requested

16. After an assault, a client says, "I used to believe the world was basically fair and that I was a good person. Now none of that makes sense." This best reflects:

A. Shattered assumptions
B. Posttraumatic growth
C. Remembrance and mourning

17. A client sent by his spouse says his drinking "isn't really a problem" and he isn't sure why he's here. The most appropriate first treatment goal is to:

A. Confront his denial with his spouse present
B. Explore whether he wants anything to change
C. Set a goal of complete abstinence in 30 days

18. When selecting continuing education to maintain a national counseling certification, a counselor's first check should be whether the course:

A. Is the lowest-cost option that meets the hour total
B. Awards the most clock hours in a single sitting
C. Comes from a provider the credentialing body accepts

19. A client's primary care physician requests records from the federally assisted substance use disorder program where the client was treated. What is generally required before these records are released?

A. Only the minimum necessary, with no consent
B. A parent's signature under student records law
C. The client's specific written consent

20. Viewing crisis as a time-limited state, often about four to six weeks, within a framework of primary, secondary, and tertiary prevention, is associated with:

A. Albert Roberts
B. Erich Lindemann
C. Gerald Caplan

21. A client says the medication his physician started last week makes him drowsy at work. The counselor should:

A. Document his report and, with consent, inform the prescriber
B. Suggest he take the medication at bedtime instead
C. Tell him the drowsiness means he needs a different drug

22. A client asks whether the diagnosis on her insurance claim will appear in her health record. The counselor explains plainly that it will and what the diagnosis means. Which ethical principle does this most directly reflect?

A. Fidelity
B. Veracity
C. Nonmaleficence

23. A client reports new episodes of racing heart and dread. She drinks several energy drinks a day and recently started a stimulant medication. Malingering and factitious disorder have been ruled out. In First's differential diagnosis sequence, the next step is to:

A. Determine the specific primary disorder
B. Rule out a general medical condition
C. Rule out a substance or medication cause

24. A client's substance use assessment shows strong cravings, withdrawal symptoms, and loss of activities he once valued. These results should be used to:

A. File in the chart while the plan follows only his stated goals
B. Link each finding to a plan objective, such as a medication referral
C. Assign the program's standard package for his severity level

25. Teaching parents to use I-messages and active listening instead of "you" messages is most associated with:

A. Virginia Satir
B. Thomas Gordon
C. John Gottman

26. A newly licensed counselor wants to be sure that continuing education hours earned this renewal cycle will count toward license renewal. The counselor should first:

A. Rely on the hours already used to renew NCC certification
B. Take any training labeled "for counselors" and submit it later
C. Check the state board's CE rules and approved-provider criteria

27. The PTSD specifier “with delayed expression” applies when:

A. Full criteria aren’t met until at least 6 months after the event
B. Symptoms begin within 3 months of a stressor
C. Symptoms occur from 3 days to 1 month after the event

28. The strengths perspective, which builds treatment plans on what is already working in a client’s life, is most associated with:

A. Abraham Maslow
B. Dennis Saleebey
C. Saul Rosenzweig

29. A counselor wants to build current knowledge of empirically supported interventions for generalized anxiety. Which source is most appropriate?

A. Current practice guidelines and peer-reviewed outcome studies
B. A popular self-help book recommended by several clients
C. The approach the counselor learned in graduate school

30. A community agency wants a brief alcohol screen to give every adult client at intake. Which instrument fits that purpose?

A. AUDIT
B. GAD-7
C. PCL-5

31. Situation selection, attentional deployment, cognitive reappraisal, and response modulation are points of intervention in which framework?

A. Linehan’s biosocial theory
B. Gross’s process model
C. Siegel’s window of tolerance

32. Which practice best keeps a counselor’s referral list current?

A. Adding any provider a colleague happens to mention in passing
B. Regularly checking specialty, license, openings, and insurance
C. Using a directory printed when the counselor was first hired

33. A motivated client has missed four sessions because the bus route to the clinic was canceled. The counselor's best response is to:

A. Document the client as resistant to treatment
B. Address the barrier, such as offering telehealth
C. Terminate services for repeated no-shows

34. Which term names the legal right that protects a client's communications from being disclosed in court, a right the client holds and can waive?

A. Privileged communication
B. Privacy
C. Confidentiality

35. A client in treatment for opioid use is on probation, has chronic back pain, recently lost his job, and is grieving a friend's overdose. His treatment plan should integrate:

A. His opioid use only, since the other issues fall outside counseling
B. His grief only, since loss is the factor most likely to drive use
C. His legal, medical, work, and grief context alongside his use

36. A counselor in private practice is relocating across the country in two months. The most ethical way to handle her current clients is to:

A. Provide pretermination counseling and arrange transfers
B. Keep seeing clients by video from the new state
C. Notify clients by letter in the final week

37. A counseling intern's practicum placement ends in May, so her clients' services with her must end even though some goals remain unmet. This type of termination is:

A. Premature
B. Planned
C. Forced

38. A newly licensed counselor joining several insurance panels is asked to create and regularly re-attest one online credentialing profile that many payers share. This profile is maintained through:

A. CAQH ProView
B. NPPES
C. NPDB

39. A client sues a counselor for harm. Which set of elements must generally be shown for a malpractice claim to succeed?

A. Respect for persons, beneficence, justice
B. Duty, dereliction, direct causation, damages
C. Serious threat, identifiable victim, credibility

40. A current client sends her counselor a friend request on the counselor's personal social media account. The counselor should:

A. Accept after documenting it as a boundary extension
B. Accept, but avoid commenting on her posts
C. Decline; personal virtual relationships are prohibited

41. In an ethics training, a counselor learns a seven-step model that begins with identifying the problem and applying the ACA Code. This model was developed by:

A. Karen Kitchener
B. Forester-Miller and Davis
C. Vilia Tarvydas

42. A counselor whose father died recently notices she changes the subject whenever a client talks about his own dying father. Her best next step is to:

A. Reflect on her own grief and bring it to supervision
B. Tell the client about her father so he feels understood
C. Refer the client to a colleague right away

43. Asked what brought him in, a client speaking at an ordinary pace moves from his car repairs to his mechanic's dog to a TV show. Each topic follows from the last, but he never answers the question. His thought process is best described as:

A. Circumstantial
B. Tangential
C. Loose associations

44. Before a session with a client in treatment for alcohol use, the best use of the counselor's preparation time is to:

A. Check the billing record to confirm his copay
B. Wait to read his notes until after the session
C. Review his last note, goals, and drinking log

45. While building a safety plan, a client writes, "Go to my cousin's to watch the game. I don't have to talk about how I feel." This belongs in which step?

A. People and places that provide distraction
B. People I can ask for help with the crisis
C. Internal coping strategies I can use alone

46. A counselor realizes a progress note written last week lists the wrong medication dosage. The counselor should:

A. Add a dated correction to the record
B. Leave it, since the next note will be accurate
C. Rewrite the note so the error no longer appears

47. After four sessions, a client treated for major depressive disorder describes past periods of elevated mood and little need for sleep. The counselor should next:

A. Revisit the diagnosis with the client and explain the possible change
B. Keep the original diagnosis, since it is already in the record

C. Change the diagnosis in the record without discussing it with her

48. Prealcoholic, prodromal, crucial, and chronic are phases described by:

A. Alan Marlatt
B. Prochaska & DiClemente
C. E. M. Jellinek

49. A client in ongoing video counseling will spend two months at a relative's home in another state. Before continuing sessions, the counselor must confirm:

A. That the client signs a waiver of the other state's rules
B. Legal authorization to practice where the client will be located
C. That the home-state license allows video sessions

50. When assessing a client's risk of harming someone else, which factor is the strongest single predictor of future violence?

A. Recent substance use
B. A previous suicide attempt
C. A history of violence

51. Perceived susceptibility, perceived severity, perceived benefits, perceived barriers, and cues to action are elements of:

A. The transactional model of stress
B. The biopsychosocial model
C. The health belief model

52. A counselor's agency plans to adopt an AI tool that drafts progress notes from session audio. What should the counselor do first?

A. Start using it, since the agency has approved the tool
B. Use it only with clients who do not ask how notes are made
C. Train on its accuracy, privacy safeguards, and consent needs

53. In a family with an alcoholic parent, the oldest daughter earns top grades, cares for her siblings, and makes the family look successful to outsiders. This best fits:

A. “Don’t talk, don’t trust, don’t feel”
B. The family hero role
C. The crucial phase

54. A client says her faith guides her decision to stay in a difficult but nonviolent marriage. The counselor personally believes she should leave. The counselor should:

A. Explore how her faith shapes the choice, without imposing values
B. Share the counselor’s view that leaving would be healthier
C. Point out the research on outcomes of unhappy marriages

55. A client asks to bring her mother, whose preferred language is not English, to a session about her safety plan. The counselor should:

A. Arrange a qualified interpreter and brief them beforehand
B. Ask the client to interpret for her mother
C. Book an interpreter to arrive just as the session starts

56. At her final session, a client offers her counselor a small handmade bracelet, explaining that giving such gifts is an important custom in her family. Under the ACA Code, the counselor should consider:

A. Whether a five-year waiting period has passed
B. Whether a written barter contract was signed
C. Its monetary value and the client’s motivation

57. A couples approach that teaches communication and problem-solving skills while also helping partners accept differences that likely won’t change is:

A. Emotionally focused therapy
B. The Gottman Method
C. Integrative behavioral couple therapy

58. A client with PTSD attempted suicide last week and is now without stable housing. Regarding exposure-based trauma treatment, the counselor should:

A. Begin prolonged exposure now so avoidance does not strengthen
B. Stabilize safety and coping before starting trauma exposure
C. Use flooding at full intensity to speed up habituation

59. Goals that are small, concrete, behavioral, stated as the presence of something, and within the client's control are the well-formed goals described by:

A. de Shazer (solution-focused)
B. Doran (SMART objectives)
C. Saleebey (strengths perspective)

60. A client is referred by his probation officer as a condition of release. Before counseling begins, the counselor should:

A. Promise that sessions will stay fully confidential
B. Explain what information will be shared with probation
C. Wait to discuss reporting until probation requests it

61. A counselor bills a 90-minute crisis psychotherapy session. Which documentation best supports the claim?

A. A brief note stating the client was seen and is stable
B. The appointment calendar showing a 90-minute block
C. Start and stop times, risk assessment, and interventions

62. A counselor is updating an old agency intake form that still asks for a functioning rating on Axis V. To align the form with the DSM-5-TR, the functioning section should use:

A. The SCID-5 interview
B. A GAF rating on Axis V
C. The WHODAS 2.0

63. In a SOAP note, the counselor writes, "Client is making moderate progress toward Objective 1; depressive

symptoms appear to be lessening.” This entry belongs in which section?

A. Objective
B. Subjective
C. Assessment

64. A client describes evening drinking as normal in his family and community, a context the counselor knows little about. The counselor should:

A. Explore its meaning to him and seek training on the context
B. Rely on the client to teach the counselor about it in sessions
C. Refer him to a counselor who shares his background

65. The father of a depressed teen asks what he can do at home to help keep his son safe. The counselor’s most important recommendation is to:

A. Treat a sudden calm as a sign danger has passed
B. Limit access to firearms and stockpiled medications
C. Have his son sign a no-suicide contract

66. A supervision model describes the supervisor shifting among the roles of teacher, counselor, and consultant, with foci on intervention, conceptualization, and personalization. This model belongs to:

A. Forester-Miller and Davis
B. Bernard (discrimination model)
C. Stoltenberg and Delworth (IDM)

67. A client with bipolar disorder agrees to let her brother support her treatment goals. The most useful way to involve him is to:

A. Ask him to monitor her daily and report back privately
B. Agree with both on specific roles, such as noticing sleep changes
C. Give him reading on bipolar disorder and leave roles open

68. A counselor licensed only in Georgia has a client who will spend three months working in Colorado and wants to continue video sessions from there. The key issue is that:

A. The client must be offered face-to-face sessions
B. Licensure generally follows the client's location
C. Her Georgia license covers sessions she provides

69. A counselor wants to measure whether a client's symptoms improve from the first session to the twelfth, checking progress along the way. Which instrument best serves this purpose?

A. OQ-45
B. Rorschach Inkblot Test
C. WAIS-5

70. A counselor wants a client to use a mood-tracking smartphone app between sessions. Before starting, the counselor should:

A. Choose the most downloaded app, since popularity signals safety
B. Ask the client for the app password to check entries daily
C. Review its privacy terms and obtain the client's informed consent

71. A counselor moves a client who has met her goals from weekly to biweekly and then monthly sessions before ending. This practice is called:

A. Booster sessions
B. Tapering
C. Step-down care

72. A 19-year-old says, "I haven't really thought about what I believe or what I want to do, and I'm not worried about it." Marcia would classify her identity status as:

A. Identity diffusion
B. Moratorium
C. Identity vs. role confusion

73. After months of an overloaded caseload, a counselor feels exhausted, cynical toward clients, and ineffective. Which response best addresses this burnout?

A. Adjust the caseload with the supervisor and build in recovery time
B. Take a week off, then return to the same caseload
C. Keep working, since cynicism is normal professional detachment

74. In Roberts's seven-stage crisis intervention model, the first stage is to:

A. Assess lethality and imminent danger
B. Define the problem
C. Make psychological contact

75. A client reports feeling foggy each morning since starting a muscle relaxant. Which progress note entry is most appropriate?

A. "Client is oversedated by the new muscle relaxant each morning."
B. "Client reports morning fogginess since starting the relaxant."
C. "Client appears foggy in the morning; the dose is likely too high."

76. A client's attorney, who has no testing training, asks for the client's MMPI-3 item responses and answer sheet. The client has signed a release. The counselor should:

A. Refuse to share anything about the testing with anyone
B. Release raw data only to a qualified professional the client designates
C. Send the item responses to the attorney because consent was given

77. A 15-year-old with serious antisocial behavior receives intensive home- and community-based treatment that targets his family, peers, school, and neighborhood. This approach is:

A. Problem-solving skills training
B. Multisystemic therapy
C. Functional family therapy

78. A counselor with a claims-made professional liability policy is retiring. To stay covered for claims filed after the policy ends, she needs:

A. An umbrella personal liability policy
B. A general (premises) liability policy
C. Extended reporting (“tail”) coverage

79. A counselor computes Cronbach’s alpha for a 20-item anxiety scale given once to a sample of clients. What is she evaluating?

A. Internal consistency
B. Test-retest reliability
C. Alternate forms reliability

80. An immigrant father keeps his social life entirely within his heritage community and avoids contact with the host culture. Berry would call this strategy:

A. Marginalization
B. Immersion-emersion
C. Separation

81. After months of hearing a client describe self-harm, a counselor has intrusive images of her injuries and dread before sessions. The counselor should assess for:

A. Vicarious trauma
B. Countertransference
C. Burnout

82. Two days after a methamphetamine binge, a client is sleeping 14 hours a day, eating constantly, having vivid unpleasant dreams, and feeling deeply depressed. The counselor should recognize this as:

A. Stimulant withdrawal, with possible suicide risk
B. Opioid withdrawal
C. Cannabis withdrawal

83. Which case held that a therapist's duty can extend to a foreseeable victim based on a client's history of violence, even without an explicit threat?

A. Ewing v. Goldstein (2004)
B. Tarasoff v. Regents, Tarasoff II (1976)
C. Jablonski v. United States (1983)

84. A client agrees to an intensive outpatient program, but the counselor learns the program on the referral list closed months ago. The counselor should:

A. Give the client the old listing and let her follow up
B. Find a current program now, then update the list
C. Delete the listing and wait for the annual review

85. A 4-year-old insists the moon follows her car because it wants to play with her. Which concept best describes her thinking?

A. Animism
B. Initiative vs. guilt
C. Personal fable

86. A counselor uses an AI note-taking tool that meets HIPAA requirements. A new client says she doesn't want the tool used in her sessions. The counselor should:

A. Refer her to a counselor who doesn't use AI tools
B. Respect her refusal and document without the tool
C. Use it anyway, since notes are the counselor's responsibility

87. To obtain a privilege to practice in another Counseling Compact member state, a counselor must hold:

A. A separate full license in each remote state
B. Current national certification as an NCC through NBCC
C. An active, unencumbered license in the home state

88. A counselor gives 25 clients the GAD-7 at intake and again at week 10 and wants to know whether the mean score changed. The appropriate test is:

A. One-way ANOVA
B. Dependent (paired) t-test
C. Independent t-test

89. A client snaps, “You’re judging me, just like my father always did,” though the counselor has said nothing critical. The client’s reaction is best understood as:

A. Reaction formation
B. Transference
C. Countertransference

90. A counselor completed an approved EMDR basic training, including its consultation hours, but has not pursued voluntary EMDR certification. Which statement is most accurate?

A. The counselor may use EMDR but should not claim certification
B. The counselor may not use EMDR until fully certified
C. The counselor may list EMDR certification on a website now

91. Over the past year, a client has had cravings, drunk more than intended, tried and failed to cut down, and given up softball to drink. No other criteria apply. The alcohol use disorder severity is:

A. Mild
B. Severe
C. Moderate

92. A parent notes that one child rose from the 50th to the 55th percentile and another from the 90th to the 95th, and concludes both gained the same amount. The counselor’s most accurate explanation is that:

A. Both children gained five points of the same ability
B. Percentile ranks are ordinal, not equal intervals
C. The difference is explained by the standard error of measurement

93. While reviewing notes before a session, a counselor sees the client reported passive suicidal thoughts last week. The counselor should plan to:

A. Wait to see whether the client raises the topic again
B. Revisit risk and the safety plan early in the session
C. Arrange inpatient admission before they meet

94. A counselor receives an order signed by a judge requiring release of a client's treatment records. The counselor's best response is to:

A. Comply, releasing only what the order requires
B. Refuse and assert privilege until the client consents
C. Send the complete record so nothing is withheld

95. During a video session, a client suddenly says she just swallowed a handful of pills. Which step, taken at the start of the session, matters most now?

A. Verifying her identity with a code word
B. Confirming her physical location
C. Confirming the platform has a BAA

96. A client with dyslexia says the practice's written intake and consent forms are hard to complete. The counselor should:

A. Refer the client to a provider who specializes in learning disabilities
B. Read the forms aloud, adding a fee for the extra time
C. Offer accessible formats, such as reading forms aloud, at no cost

97. Near the end of counseling, a client asks, "How do I keep this going once we stop meeting?" The counselor's best response is to:

A. Review which skills helped and how she'll keep using them
B. Refer her for a medication evaluation to protect gains
C. Add a new long-term goal to extend counseling

98. When planning care for a client with bipolar I disorder, a counselor best uses self-assessment of strengths and limitations by:

A. Naming what the counselor offers and where psychiatric care fits
B. Assuming general training and a strong alliance cover all needs
C. Declining the client, since bipolar disorder requires a specialist

99. At intake a client says, "I've felt empty and irritable for weeks." In the mental status exam, this statement is recorded under which component?

A. Mood
B. Thought content
C. Affect

100. A client calls hours after losing her job and being evicted, crying that she doesn't know where she and her children will sleep tonight. The counselor's best response is to:

A. Take a directive role to secure shelter tonight
B. Explore how early losses shape her reaction
C. Reflect her feelings and let her lead

101. A parent whose preferred language is not English arrives for her son's session, and her 10-year-old daughter offers to interpret. The counselor should:

A. Let the daughter interpret to avoid a delay
B. Use a qualified telephone or video interpreter
C. Continue in English and speak more slowly

102. Which DSM-5-TR term applies when the clinician strongly presumes a client will meet full criteria but does not yet have enough information?

A. Unspecified disorder
B. Other specified disorder
C. Provisional diagnosis

103. After being sent to his room, a boy deliberately breaks his sister's favorite toy. His father says he feels deeply hurt. According to Dreikurs, the boy's most likely goal is:

A. Revenge
B. Power
C. Attention

104. A counselor's practice moves to a new office building. The practice's emergency preparedness plan should be:

A. Kept as written, since it was reviewed last year
B. Replaced by the building manager's fire plan
C. Updated for new exits, contacts, and procedures

105. At a planned session 6 review, the best way for the counselor to engage a client in reviewing progress is to:

A. Show baseline and current scores and ask what she notices
B. Readminister the full intake assessment battery
C. Tell her she is improving and move on to new material

106. Before a job interview, a client asks herself, "Do I have the skills and support to handle this?" In Lazarus and Folkman's model, she is making a:

A. Primary appraisal
B. Secondary appraisal
C. Hardiness judgment of control

107. A client begins talking rapidly over the counselor and grows irritable at each question. The counselor notices a rising pulse. The best response is to:

A. Slow the counselor's own breathing and keep a calm tone
B. Match the client's pace and volume to show engagement
C. Firmly tell the client to stop interrupting

108. A researcher wants to show that a social skills intervention caused improvement, but withdrawing the inter-

vention would be unethical. She introduces it at staggered times across three settings. This design is:

A. Multiple baseline
B. Quasi-experimental
C. ABAB reversal

109. A client reports active thoughts of suicide and a vague plan, with no clear intent and a supportive partner. The best response matched to this level of risk is:

A. Immediate involuntary hospitalization until risk passes
B. A no-suicide contract with routine weekly sessions as before
C. A safety plan, means restriction, and more frequent sessions

110. A progress note organized into Data, Assessment, and Plan sections follows which format?

A. SOAP
B. DAP
C. BIRP

111. In a children's divorce-adjustment group, the leader runs a sentence-completion activity. Right afterward, the leader should:

A. Move straight to the next activity
B. Process it so members can share reactions
C. Require each child to read an answer aloud

112. A client is terrified of being judged. Her counselor assigns her to wear mismatched shoes to the grocery store on purpose and notice that being stared at is not awful. This homework is:

A. Acting as if
B. A shame-attacking exercise
C. Paradoxical intention

113. A client brings original school records and rating scales that inform an ongoing diagnostic evaluation. The counselor should:

A. Keep the originals in a desk tray until the evaluation ends
B. Shred them after reading, since only the counselor's notes count
C. Add copies to the secure record and return the originals

114. At intake three months ago, a client denied suicidal thoughts. Today she reports a breakup, nightly drinking, and pulling away from friends. The counselor should:

A. Ask her directly about suicidal thoughts today
B. Rely on the intake screen, which found no ideation
C. Arrange inpatient admission because of the new stressors

115. Directives, ordeals, and a focus on power within the family hierarchy are most associated with:

A. Jay Haley
B. Salvador Minuchin
C. Murray Bowen

116. At intake, a counselor learns the client is a student in a course the counselor teaches. The most appropriate step is to:

A. Discuss the conflict with her and arrange a referral
B. Continue counseling and have someone else grade her work
C. Ask her to drop the course so counseling can continue

117. A supervisor points out that a new counselor often finishes clients' sentences, something the counselor had never noticed. In the Johari Window, this behavior was in the counselor's:

A. Blind area
B. Hidden area
C. Unknown area

118. Immigrant parents and their 16-year-old, raised mostly in the U.S., argue constantly about curfew and dating. The best systemic framing is:

A. The parents' overly rigid family boundaries
B. An emotional cutoff between the generations
C. A gap in acculturation between generations

119. A client agrees to involve his wife in rebuilding household routines but wants his feelings of shame kept private. The counselor should:

A. Share all of his treatment once any release is signed
B. Define on the release what may be shared, then proceed
C. Decline to involve his wife unless he discloses fully

120. A client of a different racial background than the counselor begins describing racism at work, then hesitates. Which counselor response best reflects multicultural sensitivity?

A. "Many people from your background deal with that at work."
B. "I try to treat everyone the same, so race won't matter here."
C. "I wonder what it's like to talk about this with someone of a different race."

121. A researcher lowers alpha from .05 to .01. What is the result?

A. Fewer Type I errors, more Type II errors
B. Fewer Type II errors, more Type I errors
C. Greater statistical power

122. A counselor receives an attorney-issued subpoena for a client's records. The most appropriate first step is to:

A. Send the full record by the subpoena's deadline
B. Disregard it, since counselors' records are privileged
C. Consult an attorney and notify the client first

123. A five-level empathy scale on which level 3 is the interchangeable, minimally facilitative response was developed by:

A. Robert Carkhuff
B. Gerard Egan
C. Clara Hill

124. A treatment team asks a counselor to administer a structured psychosis rating scale the counselor has never used. What should the counselor do first?

A. Confirm meeting the scale's training and user qualifications
B. Administer it by following the manual's instructions
C. Have front-desk staff give it to save the counselor time

125. Which progress note format organizes entries as Goal, Intervention, Response, and Plan?

A. DAP
B. BIRP
C. GIRP

126. A couple requests couples counseling. During a private screening, one partner discloses that the other controls the money, monitors the phone, and makes threats. The counselor's best decision is to:

A. Decline joint sessions and offer individual counseling with safety planning
B. Begin couples counseling that focuses on communication skills
C. Raise the disclosure with both partners in the next joint session

127. A colleague who is not involved in a client's care asks the counselor to pull up that client's chart in the shared EHR. The counselor should:

A. Show the chart, since all practice staff use the same EHR
B. Decline, since chart access requires a job-related need
C. Lend the colleague login credentials to view it directly

128. In Joiner's interpersonal theory of suicide, what makes an attempt possible, as distinct from what creates the desire for death?

A. Psychache
B. Acquired capability
C. Perceived burdensomeness

129. A client who has barely slept in three days talks rapidly, jumping from her business plan to a song lyric to a trip to Paris. Each jump has a loose link to the one before. Her thought process is best described as:

A. Tangential thinking
B. Loose associations
C. Flight of ideas

130. As thanks for his progress, a client offers to redesign the counselor's practice website for free. The counselor should:

A. Accept, since free design work is a harmless gift
B. Accept the work but pay him his usual rate
C. Decline, thank him, and explore what the offer means

131. With ten minutes left, a client begins describing a sexual assault for the first time, then goes quiet and stares blankly. The counselor's best action is to:

A. Reassure her that she is completely safe now
B. Slow down and help her ground in the room
C. Encourage her to keep going while the memory is fresh

132. Which attribute describes a counselor being real and authentic rather than hiding behind a professional role or façade?

A. Genuineness
B. Unconditional positive regard
C. Empathic attunement

133. A counselor wants to use an AI tool that records sessions and drafts progress notes. Before using it, the counselor should:

A. Confirm a business associate agreement and obtain client consent
B. Use a free consumer AI tool and delete the audio afterward

C. Rely on the Notice of Privacy Practices as client consent

134. Death, freedom, isolation, and meaninglessness are the four givens of existence described by:

A. Irvin Yalom
B. Viktor Frankl
C. Fritz Perls

135. Two families with very different histories both come to counseling with a teen who refuses to attend school. This illustrates:

A. Homeostasis
B. Multifinality
C. Equifinality

136. A counselor practices self-care only during weeks when stress feels overwhelming. Which view best corrects this approach?

A. Self-care belongs only in the counselor's personal therapy
B. Self-care is an ongoing, preventive part of ethical practice
C. Self-care means time off only after impairment occurs

137. A new depression screen correctly flags 95% of people who truly have depression. This property of the screen is its:

A. Sensitivity
B. Specificity
C. Base rate

138. A 45-year-old client discloses that an uncle sexually abused her when she was a child. The uncle died years ago, and no child is currently at risk. The counselor should generally:

A. Support her in processing it; no mandated report applies
B. Report the abuse to adult protective services
C. Report the abuse to child protective services today

139. A counselor launching a private practice wants to build its social media presence. Which practice is appropriate?

A. Ask satisfied current clients to post online reviews
B. Reply publicly to a client's review to thank her
C. Keep separate professional and personal profiles

140. A member who has felt useless since retiring brightens after helping a newer member find a job lead. Which of Yalom's therapeutic factors is at work?

A. Instillation of hope
B. Universality
C. Altruism

141. A client has not completed his thought records for three weeks and says, "Part of me wants to, part of me doesn't see the point." The counselor's best response is to:

A. Explore his ambivalence using motivational interviewing
B. Remind him that homework is required to continue
C. Drop homework from the plan to avoid any further conflict

142. A counselor seeks a colleague's consultation on building exposure exercises for a client's intrusive harm obsessions. Which approach best protects the client?

A. Share only the de-identified details the question requires
B. Share the full chart so the advice can be fully specific
C. Avoid any consultation until the client signs a release

143. A client with panic attacks mentions that his mother and grandmother both had panic disorder. In a 4 Ps case formulation, this family history is a:

A. Predisposing factor
B. Perpetuating factor
C. Precipitating factor

144. A treatment outperformed a placebo condition in tightly controlled randomized trials with carefully screened participants. These results best establish the treatment's:

A. Effectiveness in routine community practice
B. Efficacy under ideal research conditions
C. Clinical significance for individual clients

145. A tornado warning is issued for the area while a client is in session. The counselor should:

A. End the session early so the client can drive home quickly
B. Move with the client to the plan's designated shelter area
C. Continue the session, since most warnings pass without harm

146. A client with a confirmed contagious, life-threatening disease says he has no plans to tell his partner and will continue high-risk behavior. Under the ACA Code, the counselor:

A. May keep it confidential as an end-of-life decision
B. Must keep this confidential as he is a competent adult
C. May be justified in disclosing to the identifiable partner

147. A counselor working toward licensure feels shaken by a client's intrusive harm thoughts and unsure of the risk judgment. The best step is to:

A. Refer the client out so the content is no longer heard
B. Keep the doubts private to appear competent to the supervisor
C. Review the risk formulation and own reactions in supervision

148. A counselor asks, "Suppose tonight while you sleep the problem is solved. When you wake up, what is the first thing you'd notice?" This is:

A. An externalizing question
B. Adler's "the Question"
C. The miracle question

149. After eight sessions, a client's anxiety scores haven't improved. She mentions she has been drinking every night since starting a stressful new job. The counselor's best next step is to:

A. Refer her out, since drinking is outside the plan
B. Reassess and revise the plan with her
C. Continue the plan, since change takes time

150. A client who disclosed recent suicidal ideation misses a session, and the practice has a written no-show protocol for at-risk clients. The counselor should:

A. Follow the protocol's outreach steps and document each one
B. Wait for the client to reschedule, respecting adult autonomy
C. Call the client's workplace to ask about the client's safety

151. In the second session, a client gently corrects the counselor: "I use they/them pronouns." The counselor's best response is to:

A. Apologize at length for the mistake
B. Ask why they decided to use those pronouns
C. Thank them briefly and use their pronouns consistently

152. A client denies any suicidal thoughts but faces eviction this week and has no food at home. She wants to explore how her childhood shaped her relationships. The first treatment priority is:

A. Exploring childhood relationship patterns
B. Basic needs and stabilization
C. A full trauma history in the next session

153. Which ACA Code of Ethics standard directs counselors to keep current with scientific and professional information through ongoing education?

A. C.2.a Boundaries of Competence
B. C.2.f Continuing Education
C. C.2.g Impairment

154. A treatment plan objective reads: “Client will attend two support group meetings per week, as shown by attendance sheets.” Which SMART element is missing?

A. Relevant
B. Specific
C. Time-bound

155. An adolescent survivor of sexual abuse says she feels “dirty and damaged” compared with other kids. Finkelhor and Browne would describe this as:

A. Stigmatization
B. Betrayal
C. Powerlessness

156. A client asks that appointment reminders go only to his personal cell phone, not his work email. Under HIPAA, the counselor must:

A. Ask him to explain why before changing his contact method
B. Obtain his signed authorization before using the new number
C. Honor the reasonable request without requiring a reason

157. Self-insight, self-integration, anxiety management, empathy, and conceptualizing skills are factors in which counselor task?

A. Building the working alliance
B. Managing countertransference
C. Repairing alliance ruptures

158. The W-curve of cultural adjustment adds which experience to the U-curve?

A. An initial honeymoon period
B. Reverse culture shock on returning home
C. Marginalization from both cultures

159. A counselor discovers that her license lapsed last month because a renewal step was missed. The counselor should:

A. Stop practicing until the board restores the license
B. Keep seeing clients and complete renewal when convenient
C. Have a licensed colleague cosign her notes until renewal

160. In an individual session, one partner in couples counseling discloses an affair. The counselor explained a no-secrets policy at intake. The best response is to:

A. Agree to keep the affair private
B. Reveal it at the next joint session without warning
C. Support the partner in disclosing, per the policy

161. In a treatment plan, “Client will experience reduced anxiety and return to full participation at work” is best classified as a:

A. Short-term objective
B. Long-term goal
C. Problem statement

162. A client with insurance asks to pay out of pocket in full so the health plan will not learn of the client’s treatment. The counselor should:

A. Bill the plan anyway, since covered services must be submitted
B. Honor the request, document it, and submit no claim to the plan
C. Bill under a less sensitive diagnosis code instead

163. A client says, “My wife wants me to quit drinking, but I’m only here to get her off my back.” In Bordin’s model, the alliance is weakest in:

A. The bond
B. Agreement on goals
C. Agreement on tasks

164. Which statement from one group member to another best models effective feedback?

A. “A while back you seemed distracted at some point.”
B. “When you checked your phone as I spoke, I felt brushed off.”
C. “Everyone in here thinks you’re kind of selfish, honestly.”

165. A counselor in solo practice forms a professional limited liability company and will bill payers under the business. In addition to her individual NPI, the business should obtain:

A. A second Type 1 NPI issued to the business
B. A DEA registration number for the entity
C. A Type 2 (organizational) NPI for the entity

166. Which of Yalom’s therapeutic factors is considered the group equivalent of the therapeutic alliance?

A. Group cohesiveness
B. Universality
C. Altruism

167. In a group, a member snaps at the leader, “You always let Dana talk the most. You obviously like her better.” The leader’s best response is:

A. “I’m so sorry. I never meant to make anyone feel that way.”
B. “You’re feeling overlooked here. I’d like to hear more about that.”
C. “I give every member equal time, so that isn’t accurate.”

168. A counselor in private practice is choosing a video platform for telehealth. Which option meets HIPAA requirements?

A. An encrypted platform with a signed business associate agreement
B. Any video app the client already uses and trusts on her phone

C. A free consumer app, since live video is never stored as a record

169. After six weeks without gambling, a client places one bet and says, “I’ve already blown it.” Using Marlatt’s relapse prevention, the counselor’s best response is to:

A. Start treatment over at precontemplation
B. Confront him about breaking his commitment
C. Treat the lapse as a learning event

170. A psychotherapy session lasts 45 minutes, documented from start to stop. Which CPT code does this time support?

A. 90832 (16–37 minutes)
B. 90834 (38–52 minutes)
C. 90837 (53 or more minutes)

NCE Practice Exam 1 — Answer Key with Ownership Map

Every rationale names who or what **owns** each option. Read all three, not just the correct one: each item teaches three pairings.

How to review: for every item you missed *or guessed*, write in your error log (Workbook, Part 4) the owner you picked and the owner of the correct answer.

Quick Key

1. B	2. C	3. A	4. B	5. C	6. B	7. B	8. A	9. C	10. B
11. C	12. A	13. B	14. B	15. B	16. A	17. B	18. C	19. C	20. C
21. A	22. B	23. C	24. B	25. B	26. C	27. A	28. B	29. A	30. A
31. B	32. B	33. B	34. A	35. C	36. A	37. C	38. A	39. B	40. C
41. B	42. A	43. B	44. C	45. A	46. A	47. A	48. C	49. B	50. C
51. C	52. C	53. B	54. A	55. A	56. C	57. C	58. B	59. A	60. B
61. C	62. C	63. C	64. A	65. B	66. B	67. B	68. B	69. A	70. C
71. B	72. A	73. A	74. A	75. B	76. B	77. B	78. C	79. A	80. C
81. A	82. A	83. C	84. B	85. A	86. B	87. C	88. B	89. B	90. A
91. C	92. B	93. B	94. A	95. B	96. C	97. A	98. A	99. A	100. A
101. B	102. C	103. A	104. C	105. A	106. B	107. A	108. A	109. C	110. B
111. B	112. B	113. C	114. A	115. A	116. A	117. A	118. C	119. B	120. C

121. A	122. C	123. A	124. A	125. C	126. A	127. B	128. B	129. C	130. C
131. B	132. A	133. A	134. A	135. C	136. B	137. A	138. A	139. C	140. C
141. A	142. A	143. A	144. B	145. B	146. C	147. C	148. C	149. B	150. A
151. C	152. B	153. B	154. C	155. A	156. C	157. B	158. B	159. A	160. C
161. B	162. B	163. B	164. B	165. C	166. A	167. B	168. A	169. C	170. B

Rationales

1 — B

Intake & Assessment · Task 2B.12 · Chapter 19

- **B. Factitious disorder imposed on self — Owner: Factitious disorder (sick role, no external reward).** Intentionally falsifying symptoms without obvious external reward, to assume the sick role, is factitious disorder; she is doing it to herself.
- A. Somatic symptom disorder — Owner: Somatic symptom disorder (not intentional). In somatic symptom disorder the symptoms are not intentionally produced, but she secretly causes hers.
- C. Malingering — Owner: Malingering (external gain). Malingering requires an external reward such as money or avoiding work, which the stem rules out.
- **Pair to learn:** Fakes symptoms to be seen as sick → Factitious disorder (not malingering)

2 — C

Intake & Assessment · Task 2B.7 · Chapter 19

- **C. Partial hospitalization — Owner: Partial hospitalization (PHP): day program, home at night.** PHP is a day program most of the day, most days, with the client returning home at night, for significant symptoms that don’t need 24-hour care.

- A. Residential treatment — Owner: Residential treatment (living in a structured setting). Residential care means living in the treatment setting; this client can safely go home each night.
- B. Weekly outpatient counseling — Owner: Outpatient (mild to moderate symptoms). Weekly outpatient care fits mild to moderate symptoms, but this client needs structured treatment most of each day.
- **Pair to learn:** Day program, home at night → Partial hospitalization (not residential)

3 — A

Legal & Ethical Compliance · Task 6.1 · Chapter 17

- **A. Follow the practice's evacuation plan and leave with the client — Owner: Practice emergency preparedness plan (evacuation).** An alarm activates the written plan: the counselor leaves with the client by the posted route to the meeting point and documents the disruption afterward.
- B. Finish the session unless smoke or flames become visible — Owner: Clinical continuity over safety (error pattern). Waiting for proof of a fire delays evacuation; the plan, not the counselor's judgment of the alarm, decides the response.
- C. Lock the client's paper chart in the file cabinet first — Owner: HIPAA physical safeguards (45 CFR 164.310), misapplied. Securing records matters in daily operations, but no record outranks anyone's personal safety during an evacuation.
- **Pair to learn:** Alarm sounds mid-session → evacuate with the client per the emergency plan

4 — B

Provision of Counseling Interventions · Task 4.15 · Chapter 17

- **B. Weigh risks and benefits and document the rationale — Owner: ACA A.6 boundary extension (A.6.b).** Attending a graduation can be a boundary

extension; the counselor considers risks and benefits and documents the reasoning beforehand when feasible.

- A. Attend and bring a gift, with no need to document — Owner: Undocumented boundary crossing. Skipping the risk–benefit review and documentation ignores A. 6.b.
- C. Decline, since any outside contact is a violation — Owner: Rigid avoidance (not the ACA standard). The Code distinguishes crossings from violations; not every outside contact is harmful.
- **Pair to learn:** Graduation invitation → Boundary extension, documented (ACA A.6)

5 — C

Professional Development & Counselor Self-Awareness · Task 1A.5 · Chapter 17

- **C. Raise the concern with the colleague informally — Owner: ACA I.2.a informal resolution.** When appropriate and no substantial harm is involved, counselors first try to resolve the issue with the colleague directly.
- A. Ignore it, since advertising isn't a clinical issue — Owner: Ignoring a violation (ACA I.2). Using a current client's testimonial conflicts with C.3.b, which bars soliciting testimonials from current or former clients, so ignoring it fails the duty to address violations.
- B. File a complaint with the state licensing board — Owner: ACA I.2.b reporting ethical violations. Formal reporting fits when informal resolution fails or substantial harm is likely.
- **Pair to learn:** Minor colleague violation, no harm → Informal resolution first (ACA I.2.a)

6 — B

Professional Development & Counselor Self-Awareness · Task 1A.1 · Chapter 22

- **B. Consult a colleague experienced in behavioral activation — Owner: ACA C.2.b — consultation while building skill.** While developing skill in a new approach, the counselor takes steps to ensure competent work; targeted consultation with an experienced colleague does that.
- A. Use it and rely on brief online summaries to guide sessions — Owner: Practicing beyond competence (ACA C.2.a). Informal summaries do not build competence; the counselor would be learning the approach on the client.
- C. Refer the client to a counselor with more experience — Owner: Premature referral (ACA A.11.a). Referral fits when a counselor cannot become competent; with training underway, consultation keeps care with the client's counselor.
- **Pair to learn:** Some training, little experience in a modality → consult an experienced colleague (ACA C. 2.b)

7 — B

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 21

- **B. Step down to standard outpatient — Owner: Step down (least restrictive level).** As the client stabilizes, care steps down to the least restrictive level that is safe and effective.
- A. Continue IOP until all symptoms resolve — Owner: Overly restrictive care. Staying at a higher level than needed ignores the least-restrictive principle.
- C. Step up to partial hospitalization — Owner: Step up (higher level of care). Stepping up is for worsening symptoms, not improvement.

- **Pair to learn:** Client stabilizing → Step down to least restrictive level

8 — A

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 21

- **A. A medical provider and a dietitian — Owner: Concurrent referral (eating disorder).** Eating disorders call for concurrent referral to a medical provider and dietitian while counseling continues.
- B. A case manager for housing resources — Owner: Concurrent referral (social services). Case management meets social needs, not eating disorder medical needs.
- C. A substance use treatment program — Owner: Concurrent referral (substance use). Nothing in the stem indicates substance use.
- **Pair to learn:** Eating disorder in counseling → Concurrent medical and dietitian referral

9 — C

Indirect Client Care · Task 5.1 · Chapter 23

- **C. Obtain his signed release, then plan a family session — Owner: ACA A.1.d support network involvement, with consent.** ACA A.1.d supports enlisting family as a resource with client consent, so a signed release comes first and the session then targets his goals and warning signs.
- A. Call his family to explain his diagnosis and risk level — Owner: Unauthorized disclosure (ACA B.1.c). Calling family about his diagnosis and risk without consent breaches confidentiality; there is no imminent-danger exception here.
- B. Tell him family involvement can blur the therapy frame — Owner: Rigid separation of supports. Shutting out a willing support system wastes a resource the client himself asked to use.

- **Pair to learn:** Client wants family involved → signed release first, then family session (ACA A.1.d)

10 — B

Professional Development & Counselor Self-Awareness · Task 1B.5 · Chapter 17

- **B. Refer the client to an appropriately trained provider — Owner: ACA A.11.a competence-based referral.** When needs exceed competence and the gap can't be closed in time, referral is the ethical response.
- A. End services, since treating her would be unethical — Owner: Abandonment (ACA A.12). Ending without a referral abandons the client.
- C. Continue treatment while reading about eating disorders — Owner: Practicing outside competence (ACA C.2.a). Self-study alone doesn't establish competence for a specialized, urgent need.
- **Pair to learn:** Needs exceed competence → Competence-based referral (ACA A.11.a)

11 — C

Intake & Assessment · Task 2B.1 · Chapter 19

- **C. Acknowledge it, ask permission to return to it, and offer a brief screen — Owner: Trauma assessment with care and permission (PC-PTSD-5 screening).** Trauma is assessed with care and permission, using a screen such as the PC-PTSD-5, without pushing for detail at intake.
- A. Ask the client to describe exactly what happened, in full detail — Owner: Pushing for trauma detail at intake. Pressing for a detailed account at intake is what the exam punishes and can flood the client before safety and rapport exist.
- B. Skip trauma questions for now so rapport is not disrupted — Owner: Avoiding trauma assessment. Every client should be asked about trauma, with care; skipping it misses information the client has just signaled.

- **Pair to learn:** Trauma at intake → Ask with care and permission, screen, don't push for detail

12 — A

Treatment Planning & Continuity of Care · Task 3A.1 · Chapter 21

- **A. Address safety first, then rank the rest with her by distress — Owner: Safety-first target hierarchy (life-threatening concerns lead).** Thoughts that others would be better off without her signal possible suicide risk, so risk is assessed and addressed first. The remaining concerns are then ranked collaboratively by distress, impairment, and her own priorities.
- B. Start with job stress, since it triggered the other symptoms — Owner: Precipitant-as-priority error. The event that triggered an episode is not automatically the first target. Fixing the stressor will not necessarily lift mood or sleep, and it skips the safety concern.
- C. Rank the problems privately by severity and share the order — Owner: Unilateral planning (contrary to ACA A.1.c). ACA A.1.c calls for counselors and clients to devise counseling plans jointly. Ranking problems without her removes her voice and weakens buy-in.
- **Pair to learn:** Possible suicide risk among several concerns → Safety first, then collaborative ranking

13 — B

Intake & Assessment · Task 2B.3 · Chapter 11

- **B. An independent anxiety disorder — Owner: Substance-induced vs. independent rule.** Symptoms that began before use and persisted during abstinence point to an independent disorder.
- A. Alcohol withdrawal — Owner: DSM-5-TR alcohol withdrawal. Withdrawal follows stopping heavy use; her attacks predate drinking entirely.
- C. Alcohol-induced anxiety disorder — Owner: Substance-induced disorder. Substance-induced

symptoms begin during or soon after use and improve with sustained abstinence.

- **Pair to learn:** Symptoms predate use and persist in abstinence → independent disorder

14 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Coercive control — Owner: Stark.** A pattern of monitoring, financial control, and isolation is coercive control, even without physical violence.
- A. Enmeshment — Owner: Minuchin (structural). Enmeshment describes diffuse family boundaries, not one partner dominating the other.
- C. The cycle of violence — Owner: Walker. Walker's cycle describes tension-building, an acute battering incident, and a honeymoon phase.
- **Pair to learn:** Coercive control → Stark

15 — B

Legal & Ethical Compliance · Task 6.3 · Chapter 17

- **B. Provide timely access, excluding separately kept psychotherapy notes — Owner: HIPAA right of access (45 CFR 164.524).** HIPAA gives clients access to their designated record set, currently within 30 days (one 30-day extension) and sooner if state law requires; psychotherapy notes are excluded.
- A. Require a subpoena before releasing any part of the record — Owner: Subpoena/legal process (misapplied to routine access). Clients have a federal right to their own records; legal process governs disclosures to third parties, not a client's own request.
- C. Write a treatment summary in place of the record she requested — Owner: Summary in lieu of access (164.524, client must agree). A summary may replace the record only if the client agrees in advance; the counselor cannot substitute one on her own.

- **Pair to learn:** Client asks for her own record → HIPAA right of access (psychotherapy notes excluded)

16 — A

Intake & Assessment · Task 2B.1 · Chapter 10

- **A. Shattered assumptions — Owner: Ronnie Janoff-Bulman.** Trauma breaking basic beliefs that the world is benevolent and meaningful and that the self is worthy is Janoff-Bulman's concept.
- B. Posttraumatic growth — Owner: Tedeschi & Calhoun. Posttraumatic growth is positive change after struggle, which she isn't describing.
- C. Remembrance and mourning — Owner: Judith Herman (stage 2). This is Herman's second recovery stage, a treatment phase, not the experience of lost beliefs.
- **Pair to learn:** Shattered assumptions → Janoff-Bulman

17 — B

Treatment Planning & Continuity of Care · Task 3A.9 · Chapter 21

- **B. Explore whether he wants anything to change — Owner: Readiness (stages of change: precontemplation).** A precontemplative client's first goal may be exploring whether change is wanted, respecting readiness and autonomy.
- A. Confront his denial with his spouse present — Owner: Premature confrontation. Confrontation doesn't build collaboration with a client who isn't yet considering change.
- C. Set a goal of complete abstinence in 30 days — Owner: Counselor-set goal (conflicts with autonomy). A goal the client didn't help choose is less likely to be met.
- **Pair to learn:** Precontemplative client → Explore whether change is wanted (readiness)

18 — C

Professional Development & Counselor Self-Awareness · Task 1A.3 · Chapter 22

- **C. Comes from a provider the credentialing body accepts — Owner: Approved-provider requirement.** Credentialing bodies accept hours only from approved or qualifying providers, so provider status is the first screen.
- A. Is the lowest-cost option that meets the hour total — Owner: Cost-driven selection (not a CE standard). Price says nothing about whether the hours will count; hours from a nonaccepted provider may be rejected at renewal.
- B. Awards the most clock hours in a single sitting — Owner: Hour stacking (credits are not competence). Packing hours into one event satisfies a number, not the purpose of CE, and still fails if the provider is not accepted.
- **Pair to learn:** CE hours that will count → a provider the credentialing body accepts

19 — C

Indirect Client Care · Task 5.2 · Chapter 17

- **C. The client's specific written consent — Owner: 42 CFR Part 2.** Federally assisted SUD program records carry extra protection and generally require specific written consent to disclose.
- A. Only the minimum necessary, with no consent — Owner: HIPAA minimum necessary rule. HIPAA's minimum necessary rule doesn't apply to treatment disclosures and never overrides Part 2's written-consent requirement.
- B. A parent's signature under student records law — Owner: FERPA. FERPA governs student education records, not SUD treatment records.
- **Pair to learn:** SUD program records → 42 CFR Part 2 (specific written consent)

20 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Gerald Caplan — Owner: Caplan (preventive psychiatry).** Caplan described crisis as time-limited and framed intervention as levels of prevention.
- A. Albert Roberts — Owner: Roberts (seven-stage model). Roberts owns the seven-stage crisis intervention model.
- B. Erich Lindemann — Owner: Lindemann (crisis intervention). Lindemann studied acute grief after the Cocoanut Grove fire and founded crisis intervention.
- **Pair to learn:** Crisis lasts about 4 to 6 weeks; prevention levels → Caplan

21 — A

Indirect Client Care · Task 5.4 · Chapter 23

- **A. Document his report and, with consent, inform the prescriber — Owner: Counselor scope: document and communicate with prescriber.** Counselors record client-reported effects and, with consent, pass them to the prescriber, who decides on any change.
- B. Suggest he take the medication at bedtime instead — Owner: Medication advice (outside counselor scope). Changing the timing of a dose is a prescribing decision, even when it sounds harmless.
- C. Tell him the drowsiness means he needs a different drug — Owner: Medical judgment (outside counselor scope). Deciding whether a medication is wrong for him is a prescriber judgment; the counselor reports the effect rather than drawing conclusions.
- **Pair to learn:** Client reports a side effect → document and inform the prescriber

22 — B

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **B. Veracity — Owner: Veracity (ACA principle).** Dealing truthfully with the client, even about unwelcome facts like a diagnosis on record, is veracity.
- A. Fidelity — Owner: Fidelity (ACA principle). Fidelity is keeping promises and commitments; no promise is being honored here.
- C. Nonmaleficence — Owner: Nonmaleficence (ACA principle). Nonmaleficence is avoiding harm; the defining act here is telling the truth.
- **Pair to learn:** Truthful disclosure → Veracity

23 — C

Intake & Assessment · Task 2B.12 · Chapter 19

- **C. Rule out a substance or medication cause — Owner: First's differential sequence, step 2.** After malingering and factitious disorder, the sequence rules out a substance or medication cause, which the energy drinks and stimulant make especially relevant.
- A. Determine the specific primary disorder — Owner: First's differential sequence, step 4. The primary disorder is determined only after substance and medical causes are ruled out.
- B. Rule out a general medical condition — Owner: First's differential sequence, step 3. Ruling out a medical condition comes after ruling out substances and medications.
- **Pair to learn:** Rule out substances before primary diagnosis → First's differential sequence, step 2

24 — B

Treatment Planning & Continuity of Care · Task 3A.5 · Chapter 21

- **B. Link each finding to a plan objective, such as a medication referral — Owner: Assessment-driven treatment planning.** Specific findings shape specific

objectives: cravings and withdrawal point toward a medication evaluation, and lost activities toward rebuilding valued routines.

- A. File in the chart while the plan follows only his stated goals — Owner: Unused assessment (assessment–planning disconnect). Client goals matter, but filing results without using them leaves key needs unaddressed.
- C. Assign the program's standard package for his severity level — Owner: One-size-fits-all programming. A severity label alone misses the client's particular profile. Packaged plans ignore what the assessment actually found.
- **Pair to learn:** Cravings, withdrawal, lost activities → Each finding drives a plan objective

25 — B

Provision of Counseling Interventions · Task 4.3 · Chapter 5

- **B. Thomas Gordon — Owner: Gordon (Parent Effectiveness Training).** I-messages and active listening come from Gordon's Parent Effectiveness Training.
- A. Virginia Satir — Owner: Satir (experiential). Satir is known for communication stances and family sculpting, not I-messages.
- C. John Gottman — Owner: Gottman Method. Gottman is known for the Four Horsemen, softened startup, and repair attempts.
- **Pair to learn:** I-messages → Thomas Gordon

26 — C

Legal & Ethical Compliance · Task 6.6 · Chapter 17

- **C. Check the state board's CE rules and approved-provider criteria — Owner: State licensure board CE rules; ACA C.2.f.** Board rules, which vary by state, set required hours, content areas such as ethics, and accepted providers; ACA C.2.f calls for ongoing CE.

- A. Rely on the hours already used to renew NCC certification — Owner: NBCC certification renewal (separate from licensure). The NCC is a voluntary national credential; the state board sets its own CE hours, topics, and provider rules, which may differ.
- B. Take any training labeled “for counselors” and submit it later — Owner: Unvetted CE provider (error pattern). Boards may accept only approved providers or required content areas, so unvetted courses may not count.
- **Pair to learn:** CE that counts for renewal → the state board’s rules (ACA C.2.f)

27 — A

Intake & Assessment · Task 2B.1 · Chapter 10

- **A. Full criteria aren’t met until at least 6 months after the event — Owner: DSM-5-TR PTSD specifier.** Delayed expression means full PTSD criteria are not met until at least 6 months after the trauma.
- B. Symptoms begin within 3 months of a stressor — Owner: DSM-5-TR adjustment disorder. Onset within 3 months is the adjustment disorder rule.
- C. Symptoms occur from 3 days to 1 month after the event — Owner: DSM-5-TR acute stress disorder. That window defines acute stress disorder.
- **Pair to learn:** Delayed expression → criteria met 6+ months after trauma

28 — B

Treatment Planning & Continuity of Care · Task 3A.4 · Chapter 21

- **B. Dennis Saleebey — Owner: Saleebey (strengths perspective).** Saleebey is the owner of the strengths perspective in social work.
- A. Abraham Maslow — Owner: Maslow (hierarchy of needs). Maslow’s hierarchy guides sequencing basic needs before growth.

- C. Saul Rosenzweig — Owner: Rosenzweig (common factors, Dodo bird verdict). Rosenzweig introduced the common factors idea, not the strengths perspective.
- **Pair to learn:** Strengths perspective → Saleebey

29 — A

Professional Development & Counselor Self-Awareness · Task 1A.4 · Chapter 22

- **A. Current practice guidelines and peer-reviewed outcome studies — Owner: Evidence-based practice (hierarchy of evidence).** Guidelines and controlled outcome research are where empirical support is established and updated.
- B. A popular self-help book recommended by several clients — Owner: Testimonial evidence (popularity heuristic). Popularity and client praise do not establish that an intervention works.
- C. The approach the counselor learned in graduate school — Owner: Knowledge obsolescence (ACA C.2.f). Training ages; counselors keep current with scientific information rather than relying on what they first learned.
- **Pair to learn:** Building knowledge of supported interventions → guidelines + peer-reviewed outcome research

30 — A

Intake & Assessment · Task 2B.3 · Chapter 19

- **A. AUDIT — Owner: AUDIT (alcohol use screening).** The AUDIT is a substance use screen listed among the tools (with CAGE, CRAFFT, and DAST) for asking every client about substance use.
- B. GAD-7 — Owner: GAD-7 (anxiety screen). The GAD-7 screens anxiety symptoms, not substance use.
- C. PCL-5 — Owner: PCL-5 (trauma symptom measure). The PCL-5 screens trauma symptoms, not alcohol use.
- **Pair to learn:** Alcohol screen for every client → AUDIT (not PCL-5)

31 — B

Provision of Counseling Interventions · Task 4.3 · Chapter 10

- **B. Gross's process model — Owner: James Gross (emotion regulation).** Gross described emotion regulation as occurring at successive points in the emotion process, from situation selection to response modulation.
- A. Linehan's biosocial theory — Owner: Marsha Linehan (DBT). Linehan's theory explains how dysregulation develops from emotional vulnerability plus an invalidating environment.
- C. Siegel's window of tolerance — Owner: Daniel Siegel. The window of tolerance is the arousal zone in which a person can function.
- **Pair to learn:** Process model of emotion regulation → Gross

32 — B

Indirect Client Care · Task 5.5 · Chapter 23

- **B. Regularly checking specialty, license, openings, and insurance — Owner: Scheduled resource verification.** Checking specialty, license status, availability, and accepted insurance on a schedule keeps referrals usable.
- A. Adding any provider a colleague happens to mention in passing — Owner: Unvetted referrals. An unchecked name gives no assurance the provider is qualified or available.
- C. Using a directory printed when the counselor was first hired — Owner: Outdated resource list. Providers move, close, and change services, so a static directory soon becomes wrong.
- **Pair to learn:** Referral list kept current → verify specialty, license, openings, insurance on a schedule

33 — B

Treatment Planning & Continuity of Care · Task 3A.4 · Chapter 21

- **B. Address the barrier, such as offering telehealth — Owner: Barriers (3A.4).** Transportation is a barrier; naming it and adapting the plan, for example with telehealth, supports goal attainment.
- A. Document the client as resistant to treatment — Owner: Labeling (deficit language). Calling a transportation problem resistance mislabels a barrier as a client trait.
- C. Terminate services for repeated no-shows — Owner: Premature termination (abandonment risk). Ending services ignores a solvable barrier and leaves the client without care.
- **Pair to learn:** Lack of transportation → Barrier to address (3A.4)

34 — A

Intake & Assessment · Task 2A.1 · Chapter 17

- **A. Privileged communication — Owner: Privileged communication (legal; client holds it).** Privilege is a legal protection from disclosure in court, and the client is its holder.
- B. Privacy — Owner: Privacy (constitutional and legal right of the person). Privacy is the broad right to decide what personal information is shared, not a court-specific protection.
- C. Confidentiality — Owner: Confidentiality (ethical duty of the counselor). Confidentiality is the counselor's ethical duty not to disclose, not a courtroom right the client waives.
- **Pair to learn:** Client-held legal right in court → Privileged communication

35 — C

Treatment Planning & Continuity of Care · Task 3A.6 · Chapter 21

- **C. His legal, medical, work, and grief context alongside his use — Owner: Engel's biopsychosocial model.** Engel's biopsychosocial model holds that biological, psychological, and social factors interact. Pain, probation, job loss, and grief all shape his recovery and belong in the plan.
- A. His opioid use only, since the other issues fall outside counseling — Owner: Reductionist biomedical model. Treating the substance use apart from its context leaves major relapse drivers unaddressed. Engel proposed his model as a corrective to this narrow focus.
- B. His grief only, since loss is the factor most likely to drive use — Owner: Single-factor conceptualization. Grief matters, but it is one factor among several. Fixing on one factor misses pain, legal, and work stressors.
- **Pair to learn:** Pain, probation, job loss, grief → Biopsychosocial plan (Engel)

36 — A

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 17

- **A. Provide pretermination counseling and arrange transfers — Owner: ACA A.11.c and A.11.d appropriate termination and transfer.** Counselors provide pretermination counseling, recommend other providers, and coordinate transfers so care continues.
- B. Keep seeing clients by video from the new state — Owner: Unauthorized telehealth (licensure follows the client). Practicing across state lines requires authorization where clients are located.
- C. Notify clients by letter in the final week — Owner: Abrupt termination (abandonment, A.12). Last-minute

notice leaves clients without time to process or connect with new care.

- **Pair to learn:** Counselor relocating → Pretermination counseling and transfer (A.11.c, A.11.d)

37 — C

Treatment Planning & Continuity of Care · Task 3B.9 · Chapter 21

- **C. Forced — Owner: Forced termination.** External limits, such as the end of a training placement, create forced termination.
- A. Premature — Owner: Premature termination (dropout). Premature termination is a client stopping before goals are met.
- B. Planned — Owner: Planned/natural termination. Planned termination happens when goals are met and is discussed in advance.
- **Pair to learn:** End of training placement → Forced termination

38 — A

Legal & Ethical Compliance · Task 6.8 · Chapter 17

- **A. CAQH ProView — Owner: CAQH ProView (payer credentialing database).** Many commercial payers pull credentialing data from CAQH ProView, which providers must re-attest periodically (currently every 120 days).
- B. NPPES — Owner: NPPES (National Plan and Provider Enumeration System). NPPES issues and maintains National Provider Identifiers; it is not the shared credentialing application payers review.
- C. NPDB — Owner: National Practitioner Data Bank (NPDB). The National Practitioner Data Bank collects reports of malpractice payments and adverse licensure actions; payers query it, but providers do not build profiles there.
- **Pair to learn:** One shared credentialing profile, re-attested → CAQH ProView

39 — B

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **B. Duty, dereliction, direct causation, damages — Owner: Malpractice (the four Ds).** Malpractice requires a professional duty, a breach of the standard of care, a causal link, and actual harm.
- A. Respect for persons, beneficence, justice — Owner: Belmont Report principles. These are research ethics principles from the Belmont Report.
- C. Serious threat, identifiable victim, credibility — Owner: Duty to protect triggers (Tarasoff). These trigger the duty to protect; they aren't elements of a malpractice claim.
- **Pair to learn:** Duty, dereliction, direct causation, damages → Malpractice

40 — C

Provision of Counseling Interventions · Task 4.9 · Chapter 17

- **C. Decline; personal virtual relationships are prohibited — Owner: ACA A.5.e personal virtual relationships.** The ACA Code prohibits personal virtual relationships, such as social media friendships, with current clients.
- A. Accept after documenting it as a boundary extension — Owner: ACA A.6 boundary extension. Boundary extensions cover events like a graduation; they don't permit a prohibited relationship.
- B. Accept, but avoid commenting on her posts — Owner: Boundary violation (A.5.e). Accepting creates the very personal virtual relationship the Code prohibits.
- **Pair to learn:** Social media friendship with a current client → Prohibited (ACA A.5.e)

41 — B

Professional Development & Counselor Self-Awareness · Task 1A.6 · Chapter 22

- **B. Forester-Miller and Davis — Owner: Forester-Miller & Davis (practitioner's model).** Forester-Miller and Davis's practitioner's guide sets out seven steps, starting with identifying the problem and applying the ACA Code.
- A. Karen Kitchener — Owner: Kitchener (moral principles). Kitchener described the moral principles — autonomy, beneficence, nonmaleficence, justice, fidelity — that underlie ethical reasoning.
- C. Vilia Tarvydas — Owner: Tarvydas (integrative model). Tarvydas's integrative model adds reflection, balancing, and attention to context to principle-based reasoning.
- **Pair to learn:** Seven steps, start by applying the Code → Forester-Miller & Davis

42 — A

Professional Development & Counselor Self-Awareness · Task 1B.11 · Chapter 22

- **A. Reflect on her own grief and bring it to supervision — Owner: Self-awareness: managing countertransference through supervision.** When a counselor notices a strong personal reaction, the scored answer is to reflect and seek supervision so the reaction doesn't steer the session.
- B. Tell the client about her father so he feels understood — Owner: Self-disclosure that meets the counselor's needs. Disclosing her own loss here serves her needs and shifts the focus to the counselor.
- C. Refer the client to a colleague right away — Owner: Premature referral. Immediate referral is premature; countertransference is first managed through reflection and supervision.

- **Pair to learn:** Counselor's own reaction steering the session → Self-awareness: reflect and seek supervision

43 — B

Intake & Assessment · Task 2B.5 · Chapter 19

- **B. Tangential — Owner: Tangential thought process.** Tangential thinking wanders away from the question and never returns to the point.
- A. Circumstantial — Owner: Circumstantial thought process. Circumstantial thinking wanders but eventually reaches the point; this client never does.
- C. Loose associations — Owner: Loose associations (no logical connection). Loose associations have no logical connection between ideas, but here each topic follows from the one before.
- **Pair to learn:** Never gets to the point → Tangential (not circumstantial)

44 — C

Indirect Client Care · Task 5.6 · Chapter 23

- **C. Review his last note, goals, and drinking log — Owner: Pre-session clinical review.** Reviewing the last note, treatment goals, and his self-monitoring data lets the counselor plan a focused session.
- A. Check the billing record to confirm his copay — Owner: Administrative review. Billing details do not help plan the clinical focus of the session.
- B. Wait to read his notes until after the session — Owner: Unprepared session. Reading notes afterward means the session starts without continuity from last week.
- **Pair to learn:** Preparing for a session → review last note, treatment goals, and homework data

45 — A

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **A. People and places that provide distraction — Owner: Stanley & Brown: step 3.** Contacts and settings that help without discussing the crisis are the distraction step.
- B. People I can ask for help with the crisis — Owner: Stanley & Brown: step 4. That step lists people who can be told about the crisis, which the client isn't planning to do here.
- C. Internal coping strategies I can use alone — Owner: Stanley & Brown: step 2. Internal coping strategies are things the client does alone.
- **Pair to learn:** Distraction without disclosing → Stanley & Brown step 3

46 — A

Indirect Client Care · Task 5.8 · Chapter 17

- **A. Add a dated correction to the record — Owner: Correcting records (never alter the original).** Errors are corrected with a dated addition; the original entry is never altered or destroyed.
- B. Leave it, since the next note will be accurate — Owner: Inaccurate documentation (ACA A.1.b). Records must be accurate; leaving a known error uncorrected falls short.
- C. Rewrite the note so the error no longer appears — Owner: Altering records (punished on the exam). Changing the original entry is altering a record, which is prohibited.
- **Pair to learn:** Error in a note → Dated correction, never alter the original

47 — A

Treatment Planning & Continuity of Care · Task 3A.7 · Chapter 21

- **A. Revisit the diagnosis with the client and explain the possible change — Owner: Diagnosis as ongoing and shared (DSM-5-TR bipolar differential; ACA A.2.b).** Past hypomanic or manic periods raise a bipolar differential under DSM-5-TR. Diagnosis is reviewed with the client as new information emerges, since ACA A.2.b includes the implications of diagnosis in informed consent; coordination with any prescriber also matters.
- B. Keep the original diagnosis, since it is already in the record — Owner: Anchoring bias (premature diagnostic closure). Holding to a first impression despite new data is anchoring. A diagnosis on file is a working hypothesis, not a final answer.
- C. Change the diagnosis in the record without discussing it with her — Owner: Unilateral record change (informed consent breach). Clients have a right to understand their diagnosis and its implications. Changing it silently excludes her from decisions that affect her treatment.
- **Pair to learn:** New hypomanic history after MDD diagnosis → Revisit diagnosis with client

48 — C

Intake & Assessment · Task 2B.3 · Chapter 11

- **C. E. M. Jellinek — Owner: Jellinek (disease concept of alcoholism).** Jellinek described these phases as part of the disease concept of alcoholism.
- A. Alan Marlatt — Owner: Marlatt (relapse prevention). Marlatt is known for relapse prevention and the abstinence violation effect.
- B. Prochaska & DiClemente — Owner: Stages of change. Their stages run from precontemplation to maintenance and apply to any behavior change.

- **Pair to learn:** Prealcoholic → chronic phases → Jellinek

49 — B

Legal & Ethical Compliance · Task 6.9 · Chapter 17

- **B. Legal authorization to practice where the client will be located — Owner: ACA H.1.b; state licensure law.** Telehealth is generally regulated where the client is located, so the counselor needs a license, temporary permit, or Compact privilege as that state's rules allow.
- A. That the client signs a waiver of the other state's rules — Owner: Consent misapplied (licensure cannot be waived). Licensure laws protect the public in each state; a client cannot consent away another state's requirements.
- C. That the home-state license allows video sessions — Owner: Home-state assumption (error pattern). A home-state license covers clients located in that state; the platform does not carry it across state lines.
- **Pair to learn:** Client temporarily in another state → authorization where the client is located (ACA H.1.b)

50 — C

Intake & Assessment · Task 2B.2 · Chapter 6

- **C. A history of violence — Owner: History of violence (strongest violence predictor).** A history of violence is the strongest predictor of future violence.
- A. Recent substance use — Owner: Substance use (a violence risk factor). Substance use is one factor to ask about, but history of violence is the strongest single predictor.
- B. A previous suicide attempt — Owner: Prior attempt (strongest suicide-attempt predictor). A prior attempt is the strongest single predictor of a future suicide attempt, not of violence toward others.
- **Pair to learn:** Strongest predictor of future violence → History of violence (not prior suicide attempt)

51 — C

Intake & Assessment · Task 2B.6 · Chapter 16

- **C. The health belief model — Owner: Rosenstock and colleagues.** The health belief model explains health behavior through these perceptions, with self-efficacy added later.
- A. The transactional model of stress — Owner: Lazarus & Folkman. The transactional model centers on primary and secondary appraisal.
- B. The biopsychosocial model — Owner: George Engel. Engel's model says illness involves biological, psychological, and social factors.
- **Pair to learn:** Susceptibility, severity, benefits, barriers, cues → health belief model (Rosenstock)

52 — C

Professional Development & Counselor Self-Awareness · Task 1A.7 · Chapter 22

- **C. Train on its accuracy, privacy safeguards, and consent needs — Owner: ACA H.1.a knowledge and competency.** Before using AI with client data, the counselor builds competence in its limits, data security, and how to obtain informed consent.
- A. Start using it, since the agency has approved the tool — Owner: Institutional deference (approval is not competence). Agency approval does not give the counselor competence with the tool or settle its privacy and consent issues.
- B. Use it only with clients who do not ask how notes are made — Owner: Informed consent omission (ACA H.2.a). Clients are told about technologies used in their care; consent cannot depend on whether they think to ask.
- **Pair to learn:** Adopting an AI note tool → train on accuracy, privacy, and consent first

53 — B

Intake & Assessment · Task 2B.3 · Chapter 11

- **B. The family hero role — Owner: Wegscheider-Cruse (family roles).** The hero overachieves and brings outside validation to the family in Wegscheider-Cruse's roles.
- A. "Don't talk, don't trust, don't feel" — Owner: Claudia Black (family rules). Black's rules describe the family's unspoken norms, not one child's role.
- C. The crucial phase — Owner: E. M. Jellinek. Jellinek's crucial phase describes the drinker's progression, not a child's role.
- **Pair to learn:** Hero role → Wegscheider-Cruse

54 — A

Provision of Counseling Interventions · Task 4.8 · Chapter 22

- **A. Explore how her faith shapes the choice, without imposing values — Owner: Not imposing values (ACA A.4.b).** Respect for diversity means exploring the client's own values and meaning without imposing the counselor's.
- B. Share the counselor's view that leaving would be healthier — Owner: Imposing counselor values. Steering the client toward the counselor's preference imposes values, which ACA A.4.b prohibits.
- C. Point out the research on outcomes of unhappy marriages — Owner: Persuasion disguised as information-giving. Using information to push the counselor's preferred outcome still imposes values.
- **Pair to learn:** Client's values differ from the counselor's → Don't impose values (ACA A.4.b)

55 — A

Indirect Client Care · Task 5.7 · Chapter 23

- **A. Arrange a qualified interpreter and brief them beforehand — Owner: Qualified, briefed**

interpreter (ACA A.2.c). A trained interpreter bound by confidentiality, briefed on the session's purpose, gives accurate communication on a high-stakes topic.

- B. Ask the client to interpret for her mother — Owner: Family member as interpreter. Asking the client to interpret puts her in a dual role and risks filtered or inaccurate messages.
- C. Book an interpreter to arrive just as the session starts — Owner: Unprepared interpreter (no briefing). Without a briefing on purpose, terms, and confidentiality, the interpreter enters a high-stakes safety discussion unprepared.
- **Pair to learn:** High-stakes session needing an interpreter → qualified interpreter, briefed beforehand

56 — C

Legal & Ethical Compliance · Task 6.7 · Chapter 17

- **C. Its monetary value and the client's motivation — Owner: ACA A.10.f gifts.** For gifts, counselors weigh the relationship, monetary value, and both parties' motivations; small, culturally meaningful gifts may be accepted.
- A. Whether a five-year waiting period has passed — Owner: ACA A.5.c former clients. The five-year rule governs romantic relationships with former clients, not gifts.
- B. Whether a written barter contract was signed — Owner: ACA A.10.e bartering. Bartering exchanges goods or services for counseling; a farewell gift is not payment.
- **Pair to learn:** Client gift → ACA A.10.f (value, motivation, relationship)

57 — C

Provision of Counseling Interventions · Task 4.4 · Chapter 7

- **C. Integrative behavioral couple therapy — Owner: Christensen & Jacobson (IBCT).** IBCT combines

change strategies with acceptance strategies such as unified detachment and tolerance building.

- A. Emotionally focused therapy — Owner: Johnson (EFT). EFT de-escalates the pursue-withdraw cycle and accesses primary attachment emotions.
- B. The Gottman Method — Owner: Gottman. Gottman targets the Four Horsemen, repair, and the couple's friendship.
- **Pair to learn:** Acceptance plus change → IBCT (Christensen & Jacobson)

58 — B

Treatment Planning & Continuity of Care · Task 3A.8 · Chapter 21

- **B. Stabilize safety and coping before starting trauma exposure — Owner: Phase-oriented trauma treatment (Herman: safety and stabilization first).** Herman's first stage is establishing safety; the recent attempt and unstable housing call for stabilizing now and reassessing readiness, not ruling out exposure.
- A. Begin prolonged exposure now so avoidance does not strengthen — Owner: Foa's prolonged exposure (premature timing). Comorbidity alone does not rule out prolonged exposure, but a suicide attempt last week and loss of housing call for addressing safety first.
- C. Use flooding at full intensity to speed up habituation — Owner: Flooding/implosive therapy (Stampfl; contraindicated here). Maximal-intensity flooding can overwhelm a client in acute crisis and heighten risk. Intensity without stabilization is the contraindication.
- **Pair to learn:** Recent suicide attempt + crisis, trauma work planned → Stabilize before exposure

59 — A

Treatment Planning & Continuity of Care · Task 3A.9 · Chapter 21

- **A. de Shazer (solution-focused) — Owner: de Shazer (well-formed goals, SFBT).** de Shazer's well-formed goal criteria match the stem exactly.
- B. Doran (SMART objectives) — Owner: SMART objectives (Doran). SMART stresses specific, measurable, achievable, relevant, time-bound, not presence over absence.
- C. Saleebey (strengths perspective) — Owner: Saleebey (strengths perspective). Saleebey's strengths perspective builds on client assets, not goal-wording criteria.
- **Pair to learn:** Small, concrete, presence of something → Well-formed goal (de Shazer)

60 — B

Intake & Assessment · Task 2A.1 · Chapter 17

- **B. Explain what information will be shared with probation — Owner: ACA A.2.e mandated clients.** Mandated clients must know, before services begin, what will be shared with the referral source, and that they may refuse with consequences explained.
- A. Promise that sessions will stay fully confidential — Owner: Overpromising confidentiality. Information will be shared with probation, so a promise of full confidentiality is false.
- C. Wait to discuss reporting until probation requests it — Owner: Delayed informed consent. Consent is required before counseling begins, not after a report is requested.
- **Pair to learn:** Court or probation referral → ACA A.2.e (explain what is shared first)

61 — C

Legal & Ethical Compliance · Task 6.10 · Chapter 17

- **C. Start and stop times, risk assessment, and interventions — Owner: Crisis psychotherapy codes 90839/90840 (time-based).** Crisis codes are time-based, so the record must show actual time plus the urgent assessment and interventions delivered.
- A. A brief note stating the client was seen and is stable — Owner: Insufficient documentation (vague note). A note that omits time, assessment, and interventions cannot support a time-based crisis code.
- B. The appointment calendar showing a 90-minute block — Owner: Scheduling record (not clinical documentation). A calendar shows a reserved slot, not the service actually delivered or its clinical content.
- **Pair to learn:** Time-based crisis code → documented times, risk assessment, interventions

62 — C

Intake & Assessment · Task 2B.8 · Chapter 19

- **C. The WHODAS 2.0 — Owner: WHODAS 2.0 (WHO Disability Assessment Schedule).** The DSM-5-TR dropped the GAF and the multiaxial system and suggests the WHODAS 2.0 for measuring functioning.
- A. The SCID-5 interview — Owner: SCID-5 (structured diagnostic interview). The SCID-5 is a structured interview for making diagnoses, not the DSM-5-TR's suggested functioning measure.
- B. A GAF rating on Axis V — Owner: GAF / multiaxial Axis V (eliminated in DSM-5). The GAF and Axis V were dropped; using them treats an outdated system as current practice.
- **Pair to learn:** Replaced the GAF → WHODAS 2.0 (not multiaxial Axis V)

63 — C

Indirect Client Care · Task 5.3 · Chapter 21

- **C. Assessment — Owner: SOAP: Assessment.** The counselor's clinical impression and judgment of progress belong in Assessment.
- A. Objective — Owner: SOAP: Objective. Objective holds observations and measurements, such as affect or a PHQ-9 score.
- B. Subjective — Owner: SOAP: Subjective. Subjective holds the client's own report and words.
- **Pair to learn:** Counselor's clinical impression of progress → SOAP: Assessment

64 — A

Professional Development & Counselor Self-Awareness · Task 1A.8 · Chapter 22

- **A. Explore its meaning to him and seek training on the context — Owner: Cultural humility (Hook et al., 2013); MSJCC.** Asking what the norm means to this client respects his view, and seeking training closes the counselor's own knowledge gap.
- B. Rely on the client to teach the counselor about it in sessions — Owner: Client-as-educator burden. Clients can share their meaning, but making them the counselor's main teacher shifts the counselor's training burden onto paid session time.
- C. Refer him to a counselor who shares his background — Owner: Matching-as-solution referral. Cultural difference alone is not grounds for referral; the counselor is responsible for building the needed competence.
- **Pair to learn:** Unfamiliar cultural norm → explore meaning and seek own training

65 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Limit access to firearms and stockpiled medications — Owner: Lethal means counseling.** Means restriction is a strongly supported prevention strategy because many suicidal crises are brief.
- A. Treat a sudden calm as a sign danger has passed — Owner: Suicide myth. A sudden calm can follow a decision to die and calls for reassessment.
- C. Have his son sign a no-suicide contract — Owner: No-suicide contract (lacks evidence). No-suicide contracts haven't been shown to prevent suicide and can give false reassurance.
- **Pair to learn:** Means restriction → strongly supported prevention

66 — B

Professional Development & Counselor Self-Awareness · Task 1A.2 · Chapter 17

- **B. Bernard (discrimination model) — Owner: Bernard's discrimination model.** Bernard's discrimination model defines the three supervisor roles and three foci named in the stem.
- A. Forester-Miller and Davis — Owner: Forester-Miller & Davis seven-step model. This is an ethical decision-making model, not a supervision model.
- C. Stoltenberg and Delworth (IDM) — Owner: Integrated Developmental Model (Stoltenberg & Delworth). The IDM describes supervisee development through levels of awareness, motivation, and autonomy.
- **Pair to learn:** Teacher, counselor, consultant roles → Bernard (discrimination model)

67 — B

Indirect Client Care · Task 5.1 · Chapter 23

- **B. Agree with both on specific roles, such as noticing sleep changes — Owner: Collaborative role clarity tied to goals.** Concrete roles agreed on by the client, such as noticing early warning signs like reduced sleep, link family support directly to her treatment goals.
- A. Ask him to monitor her daily and report back privately — Owner: Covert monitoring (undermines autonomy). Secret reporting turns a support into surveillance and erodes the client's trust and autonomy.
- C. Give him reading on bipolar disorder and leave roles open — Owner: Psychoeducation alone. Education helps, but without agreed roles the brother does not know how to support her specific goals.
- **Pair to learn:** Consenting family support in bipolar disorder → agreed roles tied to early warning signs

68 — B

Legal & Ethical Compliance · Task 6.4 · Chapter 9

- **B. Licensure generally follows the client's location — Owner: ACA H.1.b laws of the client's location.** Counselors must follow the laws where the client is physically located and generally need authorization to practice there.
- A. The client must be offered face-to-face sessions — Owner: ACA H.4.c-d fit and effectiveness of distance counseling. Fit for distance counseling is a clinical question; the barrier here is legal authorization.
- C. Her Georgia license covers sessions she provides — Owner: Counselor-location licensure (misconception). Being licensed where the counselor sits isn't enough; the client's location governs.
- **Pair to learn:** Client in another state for telehealth → Licensure follows the client (H.1.b)

69 — A

Treatment Planning & Continuity of Care · Task 3B.8 · Chapter 20

- **A. OQ-45 — Owner: OQ-45 (Lambert; Outcome Questionnaire).** The OQ-45 is an outcome questionnaire designed for tracking progress over treatment, which supports pretest and posttest comparison.
- B. Rorschach Inkblot Test — Owner: Rorschach (projective; Exner/R-PAS). The Rorschach is a projective personality test with lower reliability evidence, not a repeated outcome measure.
- C. WAIS-5 — Owner: WAIS (Wechsler intelligence test). The WAIS measures general cognitive ability, which is not what counseling outcomes track.
- **Pair to learn:** Tracking progress across sessions → OQ-45 (Lambert)

70 — C

Legal & Ethical Compliance · Task 6.11 · Chapter 17

- **C. Review its privacy terms and obtain the client's informed consent — Owner: ACA H.2.a (informed consent and disclosure).** Counselors explain the benefits, risks, and data-handling limits of technology they bring into care and obtain informed consent before using it.
- A. Choose the most downloaded app, since popularity signals safety — Owner: Popularity heuristic (not privacy vetting). Download counts say nothing about how an app stores, sells, or shares health data.
- B. Ask the client for the app password to check entries daily — Owner: Credential sharing (security and boundary risk). Holding a client's login creates security risks and an unstated duty to monitor between sessions.
- **Pair to learn:** Adding a client-facing app → privacy review + informed consent (ACA H.2.a)

71 — B

Treatment Planning & Continuity of Care · Task 3B.9 · Chapter 21

- **B. Tapering — Owner: Tapering.** Gradually reducing session frequency before ending is tapering.
- A. Booster sessions — Owner: Booster sessions (follow-up). Boosters are follow-up contacts after discharge, not the wind-down schedule.
- C. Step-down care — Owner: Step-down (level-of-care change). Step-down moves a client to a less intensive program, like IOP to outpatient.
- **Pair to learn:** Weekly to biweekly to monthly → Tapering

72 — A

Intake & Assessment · Task 2B.11 · Chapter 3

- **A. Identity diffusion — Owner: Marcia (identity statuses).** No exploration (crisis) and no commitment defines Marcia's diffusion status.
- B. Moratorium — Owner: Marcia (identity statuses). Moratorium requires active exploration, which she isn't doing.
- C. Identity vs. role confusion — Owner: Erikson (psychosocial stage 5). This is Erikson's adolescent crisis; Marcia's statuses, not Erikson's stage name, classify her current position.
- **Pair to learn:** Diffusion (no crisis, no commitment) → Marcia

73 — A

Professional Development & Counselor Self-Awareness · Task 1B.1 · Chapter 22

- **A. Adjust the caseload with the supervisor and build in recovery time — Owner: Maslach (burnout from chronic work demands).** Burnout grows from chronic work demands, so the fix targets the workload as well as the counselor's recovery.

- B. Take a week off, then return to the same caseload — Owner: Short-term respite only (demands unchanged). Time off eases exhaustion briefly, but burnout returns when the workload driving it is unchanged.
- C. Keep working, since cynicism is normal professional detachment — Owner: Depersonalization mistaken for detachment. Cynicism toward clients is a core burnout dimension (Maslach), not healthy professional distance.
- **Pair to learn:** Burnout from workload → change the workload, plus recovery (Maslach)

74 — A

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **A. Assess lethality and imminent danger — Owner: Roberts: stage 1.** Roberts begins with a biopsychosocial and lethality/imminent danger assessment.
- B. Define the problem — Owner: James & Gilliland: step 1. Defining the problem is the first step of James and Gilliland's six-step model.
- C. Make psychological contact — Owner: Roberts: stage 2. Establishing contact and the relationship is Roberts's second stage.
- **Pair to learn:** Seven stages starting with lethality assessment → Roberts

75 — B

Indirect Client Care · Task 5.4 · Chapter 23

- **B. "Client reports morning fogginess since starting the relaxant." — Owner: Documenting self-report.** Recording the effect and its timing as the client's own report is accurate and within scope.
- A. "Client is oversedated by the new muscle relaxant each morning." — Owner: Counselor attributing cause. Stating that the drug causes oversedation is a medical judgment the counselor cannot make.

- C. “Client appears foggy in the morning; the dose is likely too high.” — Owner: Dosing opinion (outside counselor scope). Commenting on whether a dose is too strong belongs to the prescriber.
- **Pair to learn:** Medication effect in a note → record it as the client’s self-report

76 — B

Intake & Assessment · Task 2B.11 · Chapter 20

- **B. Release raw data only to a qualified professional the client designates — Owner: ACA E.4 (release of data to qualified personnel).** Raw test data and protocols go only to persons qualified to interpret them, with client consent.
- A. Refuse to share anything about the testing with anyone — Owner: Over-restriction (ignores client’s right to results). Results can still be explained to the client and raw data released to qualified personnel; a blanket refusal goes too far.
- C. Send the item responses to the attorney because consent was given — Owner: Consent alone (ignores ACA E.4 qualification rule). Client consent is required but not sufficient; the recipient must also be qualified to interpret the data.
- **Pair to learn:** Raw test data → ACA E.4: qualified personnel only

77 — B

Treatment Planning & Continuity of Care · Task 3A.11 · Chapter 16

- **B. Multisystemic therapy — Owner: Henggeler (MST).** MST is intensive home- and community-based treatment across the systems around the youth.
- A. Problem-solving skills training — Owner: Kazdin (PSST). PSST teaches the youth problem-solving skills rather than targeting multiple systems.

- C. Functional family therapy — Owner: Alexander & Parsons (FFT). FFT is a family-focused treatment, not intensive work across peers, school, and neighborhood.
- **Pair to learn:** Home- and community-based, multiple systems → MST (Henggeler)

78 — C

Legal & Ethical Compliance · Task 6.12 · Chapter 17

- **C. Extended reporting ("tail") coverage — Owner: Claims-made policy extended reporting endorsement.** Claims-made policies cover claims reported while active; tail coverage extends the reporting period after the policy ends.
- A. An umbrella personal liability policy — Owner: Personal umbrella insurance. Umbrella policies extend personal liability limits; they generally exclude professional services.
- B. A general (premises) liability policy — Owner: General liability insurance. General liability covers incidents such as slip-and-fall injuries at the office, not professional negligence.
- **Pair to learn:** Claims-made policy ending → tail (extended reporting) coverage

79 — A

Intake & Assessment · Task 2B.11 · Chapter 3

- **A. Internal consistency — Owner: Internal consistency (Cronbach's alpha; KR-20).** Cronbach's alpha tests whether the items hang together in a single administration.
- B. Test-retest reliability — Owner: Test-retest reliability (stability over time). Test-retest requires giving the same test twice; this scale was given once.
- C. Alternate forms reliability — Owner: Alternate (parallel) forms reliability. Alternate forms compares two versions of a test; only one version is involved here.

- **Pair to learn:** Items hang together → Internal consistency (not test-retest)

80 — C

Provision of Counseling Interventions · Task 4.2 · Chapter 15

- **C. Separation — Owner: Berry (acculturation strategies).** Keeping heritage while avoiding the host culture is Berry's separation.
- A. Marginalization — Owner: Berry (acculturation strategies). Marginalization means connecting with neither culture; he is strongly connected to his heritage.
- B. Immersion-emersion — Owner: Cross (Nigrescence). Immersion-emersion is a stage of Black racial identity development, not an acculturation strategy.
- **Pair to learn:** Keep heritage, avoid host → separation (Berry)

81 — A

Professional Development & Counselor Self-Awareness · Task 1B.2 · Chapter 22

- **A. Vicarious trauma — Owner: McCann & Pearlman (vicarious traumatization).** McCann and Pearlman described vicarious trauma as cumulative change from empathic exposure to clients' trauma, including intrusive imagery.
- B. Countertransference — Owner: Freud (countertransference). Countertransference is the counselor's reaction shaped by the counselor's own history; it is not defined by trauma exposure.
- C. Burnout — Owner: Maslach (burnout). Burnout is exhaustion, cynicism, and reduced accomplishment from work demands, not intrusive trauma images.
- **Pair to learn:** Intrusive images from clients' trauma → vicarious trauma (McCann & Pearlman)

82 — A

Intake & Assessment · Task 2B.3 · Chapter 11

- **A. Stimulant withdrawal, with possible suicide risk — Owner: DSM-5-TR stimulant withdrawal (crash).** The stimulant crash brings fatigue, hypersomnia, increased appetite, and depressed mood; it's not usually medically dangerous, but suicide risk can be high.
- B. Opioid withdrawal — Owner: DSM-5-TR opioid withdrawal. Opioid withdrawal is flu-like, with runny nose, yawning, goosebumps, and diarrhea.
- C. Cannabis withdrawal — Owner: DSM-5-TR cannabis withdrawal. Cannabis withdrawal brings insomnia and decreased appetite, not the hypersomnia and increased eating seen here.
- **Pair to learn:** Crash (hypersomnia, hunger, low mood) → stimulant withdrawal

83 — C

Intake & Assessment · Task 2A.1 · Chapter 6

- **C. Jablonski v. United States (1983) — Owner: Jablonski v. United States (1983).** Jablonski extended the duty to foreseeable victims based on violent history, without a specific stated threat.
- A. Ewing v. Goldstein (2004) — Owner: Ewing v. Goldstein (2004). Ewing held that a threat reported by a client's family member can trigger the duty.
- B. Tarasoff v. Regents, Tarasoff II (1976) — Owner: Tarasoff v. Regents (1976), duty to protect. Tarasoff II broadened warning to protecting an intended victim of a client's threat.
- **Pair to learn:** Violent history, no explicit threat → Jablonski v. United States

84 — B

Indirect Client Care · Task 5.5 · Chapter 23

- **B. Find a current program now, then update the list — Owner: Active referral + list maintenance.** The counselor secures a current option for the client first, then updates the list and sets a routine check.
- A. Give the client the old listing and let her follow up — Owner: Passive referral. Handing over a dead listing leaves the client without care at the point she agreed to it.
- C. Delete the listing and wait for the annual review — Owner: Deferred maintenance. Removing the entry does nothing for the client and leaves the list unchecked for months.
- **Pair to learn:** Referral resource found closed → place the client now, then update and recheck the list

85 — A

Intake & Assessment · Task 2B.11 · Chapter 3

- **A. Animism — Owner: Piaget: preoperational stage.** Attributing life and intentions to objects is animism, a hallmark of Piaget's preoperational stage (ages 2 to 7).
- B. Initiative vs. guilt — Owner: Erikson (psychosocial stage 3). Initiative vs. guilt is Erikson's preschool psychosocial crisis; it names a crisis, not a cognitive feature.
- C. Personal fable — Owner: Elkind (adolescent egocentrism). The personal fable is an adolescent's sense of being unique or invulnerable, not a preschooler's belief that objects are alive.
- **Pair to learn:** Animism → Piaget (preoperational)

86 — B

Legal & Ethical Compliance · Task 6.4 · Chapter 17

- **B. Respect her refusal and document without the tool — Owner: Informed consent for AI (ACA A.2;**

ACA AI Work Group). Clients must be told when AI is used and may decline; the counselor honors the refusal.

- A. Refer her to a counselor who doesn't use AI tools — Owner: Unnecessary referral. Simply not using the tool meets her request; transferring her care is not needed.
- C. Use it anyway, since notes are the counselor's responsibility — Owner: Counselor responsibility for records (misapplied). The counselor is responsible for notes, but that doesn't override the client's right to refuse.
- **Pair to learn:** Client declines AI note-taker → Informed consent; respect refusal

87 — C

Legal & Ethical Compliance · Task 6.13 · Chapter 17

- **C. An active, unencumbered license in the home state — Owner: Counseling Compact privilege requirements.** The privilege rests on an active home-state license with no encumbrance or restriction on any license in the prior two years, plus notice, fees, and jurisprudence rules.
- A. A separate full license in each remote state — Owner: Traditional multistate licensure (pre-Compact route). Separate licenses remain an option, but the Compact's privilege exists so a counselor does not need one in each member state.
- B. Current national certification as an NCC through NBCC — Owner: NBCC national certification (voluntary credential). The Compact requires no NCC; member states require a Commission-approved national exam for licensure, and the privilege rests on the home-state license.
- **Pair to learn:** Compact privilege → unencumbered home-state license

88 — B

Intake & Assessment · Task 2B.11 · Chapter 18

- **B. Dependent (paired) t-test — Owner: Dependent/ paired t-test.** Two means from the same people measured twice call for a dependent (paired) t-test.
- A. One-way ANOVA — Owner: One-way ANOVA. One-way ANOVA compares three or more group means.
- C. Independent t-test — Owner: Independent t-test. The independent t-test compares two different groups, not the same clients twice.
- **Pair to learn:** Same people measured twice → Dependent (paired) t-test

89 — B

Provision of Counseling Interventions · Task 4.11 · Chapter 5

- **B. Transference — Owner: Freud (psychoanalysis).** Redirecting feelings and expectations from an important past relationship onto the counselor is transference, which the counselor can explore tentatively.
- A. Reaction formation — Owner: Freud / A. Freud (defense mechanisms). Reaction formation is expressing the opposite of what one feels; the client is expressing anger openly.
- C. Countertransference — Owner: Freud (psychoanalysis). Countertransference is the counselor's own emotional reaction to the client, handled in supervision, not taught to the client here.
- **Pair to learn:** Feelings from past relationships onto counselor → transference (Freud)

90 — A

Professional Development & Counselor Self-Awareness · Task 1B.3 · Chapter 22

- **A. The counselor may use EMDR but should not claim certification — Owner: ACA C.4.a accurate**

representation. Competence rests on education, training, and supervised experience; counselors claim only qualifications actually completed (C.4.a).

- B. The counselor may not use EMDR until fully certified — Owner: Credential-required error (training is the competence basis). EMDR certification is a voluntary, advanced credential; completed training with consultation is the usual basis for practice.
- C. The counselor may list EMDR certification on a website now — Owner: Misrepresented credential (ACA C.3.a). Advertising a certification not earned is inaccurate advertising and misrepresents qualifications.
- **Pair to learn:** Trained but not certified → practice within training; don't claim the credential (ACA C.4.a)

91 — C

Intake & Assessment · Task 2B.3 · Chapter 11

- **C. Moderate — Owner: DSM-5-TR severity (4–5 criteria).** Four criteria fall in the moderate range of 4 to 5.
- A. Mild — Owner: DSM-5-TR severity (2–3 criteria). Mild requires only 2 to 3 criteria; she meets 4.
- B. Severe — Owner: DSM-5-TR severity (6+ criteria). Severe requires 6 or more criteria.
- **Pair to learn:** 4–5 criteria → moderate SUD

92 — B

Intake & Assessment · Task 2B.11 · Chapter 20

- **B. Percentile ranks are ordinal, not equal intervals — Owner: Percentile rank (ordinal scale).** Percentiles are not equal intervals; the gap from the 90th to 95th is much larger in raw-score terms than from the 50th to 55th.
- A. Both children gained five points of the same ability — Owner: Equal-interval assumption (interpreting percentiles as interval data). Treating percentile

differences as equal intervals is the exact error the exam punishes.

- C. The difference is explained by the standard error of measurement — Owner: Standard error of measurement (score error band). SEM describes error around one score; it does not explain why equal percentile gaps differ in size.
- **Pair to learn:** Percentile differences are unequal → Percentile rank is ordinal (not interval)

93 — B

Indirect Client Care · Task 5.6 · Chapter 23

- **B. Revisit risk and the safety plan early in the session — Owner: Ongoing risk monitoring.** Prior risk noted in the record calls for checking current risk and reviewing the safety plan early in the session.
- A. Wait to see whether the client raises the topic again — Owner: Passive follow-up. Leaving it to the client risks missing a change in risk.
- C. Arrange inpatient admission before they meet — Owner: Level of care beyond the evidence. Passive thoughts last week do not by themselves indicate a need for inpatient care without a current assessment.
- **Pair to learn:** Prior risk noted in the record → revisit risk and safety plan early in the session

94 — A

Indirect Client Care · Task 5.2 · Chapter 17

- **A. Comply, releasing only what the order requires — Owner: Court order (ACA B.2.d).** A judge-signed order must be followed, while disclosing as little as possible and asking the court to limit disclosure if needed.
- B. Refuse and assert privilege until the client consents — Owner: Subpoena response (assert privilege). Asserting privilege is the response to an attorney-issued subpoena, not to an order signed by a judge.

- C. Send the complete record so nothing is withheld — Owner: Overdisclosure (violates ACA B.2.e). Sending everything ignores the minimal disclosure standard.
- **Pair to learn:** Judge-signed order → Comply with minimal disclosure (ACA B.2.d)

95 — B

Legal & Ethical Compliance · Task 6.4 · Chapter 9

- **B. Confirming her physical location — Owner: Telehealth practice (location check for emergencies).** Knowing where the client is lets the counselor send local emergency help right away.
- A. Verifying her identity with a code word — Owner: ACA H.3 (client verification). Verification confirms who the client is but doesn't help emergency responders find her.
- C. Confirming the platform has a BAA — Owner: HIPAA (business associate agreement). A BAA protects privacy but does nothing for an emergency in progress.
- **Pair to learn:** Telehealth emergency → confirm client's location each session

96 — C

Legal & Ethical Compliance · Task 6.14 · Chapter 17

- **C. Offer accessible formats, such as reading forms aloud, at no cost — Owner: ADA Title III effective communication; ACA A.2.c.** Practices provide accessible ways to give and receive information at no added charge so consent is truly informed.
- A. Refer the client to a provider who specializes in learning disabilities — Owner: Disability-based referral (ACA C.5). Referring because of a disability rather than clinical need is discriminatory; ACA C.5 lists disability.
- B. Read the forms aloud, adding a fee for the extra time — Owner: Accommodation surcharge (prohibited, 28 CFR 36.301(c)). ADA Title III bars charging clients with disabilities for the cost of accommodations.

- **Pair to learn:** Disability affects access to forms → accessible formats at no cost (ADA Title III)

97 — A

Treatment Planning & Continuity of Care · Task 3B.3 · Chapter 21

- **A. Review which skills helped and how she'll keep using them — Owner: Consolidate and generalize learning (3B.3).** Consolidating what helped and planning how to apply it independently supports maintenance of progress.
- B. Refer her for a medication evaluation to protect gains — Owner: Concurrent referral (medication). Nothing suggests a need for medication; the question is about maintaining gains.
- C. Add a new long-term goal to extend counseling — Owner: Long-term goal (new). Adding goals delays ending rather than preparing her to maintain gains.
- **Pair to learn:** Maintaining gains after ending → Consolidate and generalize (3B.3)

98 — A

Professional Development & Counselor Self-Awareness · Task 1B.4 · Chapter 22

- **A. Naming what the counselor offers and where psychiatric care fits — Owner: ACA C.2.a boundaries of competence (with collaboration).** Honest self-assessment defines the counselor's role, such as relapse-prevention work, and where medication or crisis care must come from others.
- B. Assuming general training and a strong alliance cover all needs — Owner: Overestimating competence. Treating a serious mood disorder as fully covered by general skills ignores needs outside the counselor's scope.
- C. Declining the client, since bipolar disorder requires a specialist — Owner: Underestimating competence. Counselors can treat clients with bipolar disorder

within a collaborative plan; automatic refusal is not required.

- **Pair to learn:** Self-assessment in planning → name your role and your referrals

99 — A

Intake & Assessment · Task 2B.5 · Chapter 19

- **A. Mood — Owner: Mood (client-reported).** Mood is the client's self-reported, sustained emotional state, often recorded in the client's own words. The client is describing weeks of feeling, which is the sustained "climate."
- B. Thought content — Owner: Thought content (what the client thinks). Thought content covers what the client thinks about, such as delusions, obsessions, or suicidal ideation, not a reported emotional state.
- C. Affect — Owner: Affect (counselor-observed). Affect is what the counselor observes of emotional expression in the moment, not what the client reports.
- **Pair to learn:** Client-reported emotional state → Mood (not affect)

100 — A

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **A. Take a directive role to secure shelter tonight — Owner: Crisis intervention (directive stance).** In an acute crisis the counselor is more directive and focuses on immediate safety and restoring functioning.
- B. Explore how early losses shape her reaction — Owner: Psychodynamic insight (Freud). Exploring history during an active crisis is a wrong answer; stabilization comes first.
- C. Reflect her feelings and let her lead — Owner: Nondirective stance (Rogers). A fully nondirective stance doesn't address the urgent need for a safe place tonight.

- **Pair to learn:** Acute crisis → more directive, here-and-now

101 — B

Indirect Client Care · Task 5.7 · Chapter 23

- **B. Use a qualified telephone or video interpreter — Owner: Remote qualified interpreter.** When no in-person interpreter is available, a qualified phone or video interpreter provides accurate access right away.
- A. Let the daughter interpret to avoid a delay — Owner: Minor as interpreter. Children should not interpret clinical conversations; it burdens them and risks inaccurate communication.
- C. Continue in English and speak more slowly — Owner: Ignoring language need. Speaking slowly does not give a parent with limited English meaningful access.
- **Pair to learn:** Child offers to interpret → use a qualified phone or video interpreter

102 — C

Intake & Assessment · Task 2B.12 · Chapter 19

- **C. Provisional diagnosis — Owner: Provisional diagnosis (DSM-5-TR).** A provisional diagnosis is used when there is a strong presumption criteria will be met but information is still incomplete.
- A. Unspecified disorder — Owner: Unspecified (reason not specified). Unspecified is used when the clinician chooses not to, or cannot, say why criteria aren't met, often in emergency settings.
- B. Other specified disorder — Owner: Other specified (clinician states why criteria aren't met). Other specified is used when the clinician records the specific reason criteria are not fully met, not when information is pending.
- **Pair to learn:** Criteria expected but information incomplete → Provisional diagnosis (not other specified)

103 — A

Provision of Counseling Interventions · Task 4.1 · Chapter 13

- **A. Revenge — Owner: Dreikurs (goals of misbehavior).** In Dreikurs's model, an adult who feels hurt is a clue that the child's goal is revenge.
- B. Power — Owner: Dreikurs (goals of misbehavior). Power struggles leave adults feeling angry and challenged, not hurt.
- C. Attention — Owner: Dreikurs (goals of misbehavior). Attention-seeking leaves adults feeling annoyed.
- **Pair to learn:** Adult feels hurt → revenge (Dreikurs)

104 — C

Legal & Ethical Compliance · Task 6.15 · Chapter 17

- **C. Updated for new exits, contacts, and procedures — Owner: Emergency preparedness plan review.** Major changes such as relocation trigger a review so the plan matches the new site and current contacts.
- A. Kept as written, since it was reviewed last year — Owner: Stale plan (calendar-only review). Plans are reviewed on a schedule and whenever conditions change; a move changes exits, contacts, and procedures.
- B. Replaced by the building manager's fire plan — Owner: Building evacuation plan (not a practice plan). A building plan covers evacuation, not client safety, records, or continuity of care for the practice.
- **Pair to learn:** Office move → update the emergency preparedness plan

105 — A

Treatment Planning & Continuity of Care · Task 3B.7 · Chapter 21

- **A. Show baseline and current scores and ask what she notices — Owner: Reviewing progress with the**

client (3B.7). Sharing data from baseline to now and inviting the client's reflection makes the review collaborative.

- B. Readminister the full intake assessment battery — Owner: Reassessment (used when progress stalls). Full reassessment is a response to stalled progress, not a routine planned review.
- C. Tell her she is improving and move on to new material — Owner: Unilateral counselor judgment. Announcing progress without her input skips her active role in the review.
- **Pair to learn:** Planned progress review → Share data and invite client reflection (3B.7)

106 — B

Provision of Counseling Interventions · Task 4.11 · Chapter 16

- **B. Secondary appraisal — Owner: Lazarus & Folkman (transactional model).** Secondary appraisal asks whether one has the resources to cope.
- A. Primary appraisal — Owner: Lazarus & Folkman (transactional model). Primary appraisal asks whether the situation is a threat, harm, or challenge.
- C. Hardiness judgment of control — Owner: Kobasa (hardiness). Control is one of Kobasa's three Cs, a personality trait rather than an appraisal step.
- **Pair to learn:** Do I have the resources? → secondary appraisal (Lazarus & Folkman)

107 — A

Professional Development & Counselor Self-Awareness · Task 1B.6 · Chapter 22

- **A. Slow the counselor's own breathing and keep a calm tone — Owner: Counselor self-regulation (paced breathing).** Regulating one's own arousal keeps the response therapeutic instead of reactive.
- B. Match the client's pace and volume to show engagement — Owner: Mirroring escalation. Matching

an escalating client amplifies the escalation rather than calming it.

- C. Firmly tell the client to stop interrupting — Owner: Countertransference enactment (reactivity). Acting on the counselor's frustration escalates the exchange and ruptures rapport.
- **Pair to learn:** Counselor's rising arousal in session → self-regulate first

108 — A

Intake & Assessment · Task 2B.11 · Chapter 18

- **A. Multiple baseline — Owner: Multiple baseline (single-subject design).** Staggering the intervention across behaviors, settings, or people is multiple baseline, used when withdrawal is unethical.
- B. Quasi-experimental — Owner: Quasi-experimental design. Quasi-experiments compare intact groups without random assignment, not staggered phases in one case.
- C. ABAB reversal — Owner: ABAB (single-subject design). ABAB requires withdrawing the intervention, which the stem rules out.
- **Pair to learn:** Staggered intervention, withdrawal unethical → Multiple baseline

109 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. A safety plan, means restriction, and more frequent sessions — Owner: Moderate-risk response (level of care matching).** Active ideation with a vague plan and no clear intent is moderate risk, matched with a safety plan, means restriction, and closer contact.
- A. Immediate involuntary hospitalization until risk passes — Owner: Imminent-risk response. Involuntary holds are for imminent risk with stated intent and means; this over-responds.

- B. A no-suicide contract with routine weekly sessions as before — Owner: No-suicide contract (lacks evidence). Contracts lack evidence and routine scheduling under-responds to active ideation.
- **Pair to learn:** Moderate risk → safety plan + means restriction (least restrictive appropriate care)

110 — B

Indirect Client Care · Task 5.8 · Chapter 23

- **B. DAP — Owner: DAP (Data, Assessment, Plan).** DAP combines client report and observations into one Data section, then Assessment and Plan.
- A. SOAP — Owner: SOAP (Subjective, Objective, Assessment, Plan). SOAP separates subjective and objective information before the assessment and plan.
- C. BIRP — Owner: BIRP (Behavior, Intervention, Response, Plan). BIRP organizes the note around client behavior, the counselor's intervention, and the client's response.
- **Pair to learn:** Data, Assessment, Plan → DAP note

111 — B

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **B. Process it so members can share reactions — Owner: Processing structured activities.** Exercises should serve a purpose and be processed afterward.
- A. Move straight to the next activity — Owner: Overuse of structured exercises. Stacking exercises without processing makes the group leader-dependent.
- C. Require each child to read an answer aloud — Owner: Forced participation. Members can decline to participate in an exercise.
- **Pair to learn:** Structured activity → process it afterward

112 — B

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **B. A shame-attacking exercise — Owner: Ellis (REBT).** Deliberately doing something mildly embarrassing to attack the belief that disapproval is awful is Ellis's shame-attacking exercise.
- A. Acting as if — Owner: Adler (individual psychology). Acting as if asks the client to behave like the person she wishes to be, not to provoke the judgment she fears.
- C. Paradoxical intention — Owner: Frankl (logotherapy). Paradoxical intention asks a client to intend the feared symptom itself, such as trying to stay awake, not to court disapproval.
- **Pair to learn:** Shame-attacking exercise → Ellis (REBT)

113 — C

Legal & Ethical Compliance · Task 6.16 · Chapter 17

- **C. Add copies to the secure record and return the originals — Owner: ACA B.6.b; HIPAA designated record set.** Materials used in care become part of the secured record with access limited to authorized staff.
- A. Keep the originals in a desk tray until the evaluation ends — Owner: Unsecured paper records (HIPAA 45 CFR 164.310). Leaving client documents in open view fails the physical safeguards that protect records.
- B. Shred them after reading, since only the counselor's notes count — Owner: Improper disposal of relied-upon documents (ACA A.1.b). Documents that inform a diagnosis belong in the record so the clinical basis for decisions is preserved.
- **Pair to learn:** Client-supplied documents used in care → secure copies in the record

114 — A

Intake & Assessment · Task 2B.2 · Chapter 6

- **A. Ask her directly about suicidal thoughts today — Owner: Ongoing risk assessment (2B.2).** Risk assessment is ongoing, not a one-time intake task; a recent loss, intoxication, and isolation call for direct reassessment now.
- B. Rely on the intake screen, which found no ideation — Owner: One-time screening (ignores ongoing assessment). Risk changes; the intake result does not cover new risk factors that have appeared since.
- C. Arrange inpatient admission because of the new stressors — Owner: Over-response (ignores least restrictive care). Hospitalizing before assessing the level of risk skips the step of matching care to risk.
- **Pair to learn:** New risk factors after intake → Ongoing risk assessment (not one-time screening)

115 — A

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **A. Jay Haley — Owner: Haley (strategic).** Haley's strategic therapy uses directives, ordeal therapy, and attention to power in the hierarchy.
- B. Salvador Minuchin — Owner: Minuchin (structural). Minuchin also cares about hierarchy, but his tools are boundaries, joining, and enactment, not ordeals.
- C. Murray Bowen — Owner: Bowen (multigenerational). Bowen works on differentiation and triangles as a calm coach, not through directives.
- **Pair to learn:** Directives / ordeal → Haley (strategic)

116 — A

Professional Development & Counselor Self-Awareness · Task 1B.7 · Chapter 22

- **A. Discuss the conflict with her and arrange a referral — Owner: Conflict of interest — multiple**

relationship (ACA A.6). The counselor's evaluative role over her creates a conflict of interest, so the pairing is unsuitable and a referral is warranted.

- B. Continue counseling and have someone else grade her work — Owner: Partial role separation. Removing grading leaves the teacher–student power relationship in place alongside counseling.
- C. Ask her to drop the course so counseling can continue — Owner: Shifting the conflict onto the client. Asking the client to give up her course to fix the counselor's conflict burdens her.
- **Pair to learn:** Counselor also client's instructor → conflict of interest, refer

117 — A

Professional Development & Counselor Self-Awareness · Task 1B.11 · Chapter 22

- **A. Blind area — Owner: Luft & Ingham (Johari Window): blind area.** The blind area holds what others see but the person doesn't; feedback from supervisors reveals it.
- B. Hidden area — Owner: Luft & Ingham (Johari Window): hidden area. The hidden area holds what the person knows but keeps from others; the counselor didn't know about this habit.
- C. Unknown area — Owner: Luft & Ingham (Johari Window): unknown area. The unknown area is unseen by self and others; the supervisor clearly saw this behavior.
- **Pair to learn:** Supervisor feedback reveals what the counselor can't see → Blind area (Johari Window)

118 — C

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **C. A gap in acculturation between generations — Owner: Berry (acculturation).** Framing the conflict

as an acculturation difference avoids labeling either side as defiant or rigid.

- A. The parents' overly rigid family boundaries — Owner: Minuchin (structural). Labeling the parents' norms as rigid applies one culture's standard before exploring theirs.
- B. An emotional cutoff between the generations — Owner: Bowen (multigenerational). A cutoff is a severed relationship; this family is highly engaged, just in conflict.
- **Pair to learn:** Immigrant parent-teen conflict → acculturation gap

119 — B

Indirect Client Care · Task 5.1 · Chapter 23

- **B. Define on the release what may be shared, then proceed — Owner: Client-defined scope of consent (ACA B.1.c).** Clients can limit what a release covers, so the counselor records that scope and involves his wife only in the routine goals he chose.
- A. Share all of his treatment once any release is signed — Owner: Overbroad disclosure. A release does not open the whole record; disclosure is limited to what the client authorizes.
- C. Decline to involve his wife unless he discloses fully — Owner: All-or-nothing consent. Consent is not all-or-nothing; requiring full disclosure as a condition of involvement is coercive.
- **Pair to learn:** Partial consent → release defines exactly what may be shared

120 — C

Provision of Counseling Interventions · Task 4.2 · Chapter 22

- **C. "I wonder what it's like to talk about this with someone of a different race." — Owner: Broaching (cultural humility).** Broaching culture respectfully

invites the client's experience and acknowledges the counselor's own position and power.

- A. "Many people from your background deal with that at work." — Owner: Stereotyping (cultural encapsulation). Generalizing about the client's group assumes rather than inviting the client's own meaning.
- B. "I try to treat everyone the same, so race won't matter here." — Owner: Ignoring culture (not cultural humility). Declaring race irrelevant ignores culture and the power the counselor's identity carries.
- **Pair to learn:** Naming cultural difference respectfully → Broaching / cultural humility (not ignoring culture)

121 — A

Intake & Assessment · Task 2B.11 · Chapter 18

- **A. Fewer Type I errors, more Type II errors — Owner: Alpha and error trade-off.** Lowering alpha reduces false positives but makes it harder to detect real effects, increasing Type II errors.
- B. Fewer Type II errors, more Type I errors — Owner: Reversed trade-off (Type I vs. Type II confusion). This reverses the relationship; a stricter alpha raises Type II risk.
- C. Greater statistical power — Owner: Power ($1 - \beta$). A lower alpha decreases power; a higher alpha increases it.
- **Pair to learn:** Lower alpha → Fewer Type I, more Type II errors

122 — C

Legal & Ethical Compliance · Task 6.17 · Chapter 17

- **C. Consult an attorney and notify the client first — Owner: Subpoena response (HIPAA 164.512(e); state law).** Counselors seek legal advice and inform the client, releasing records only with consent, a court order, or the assurances HIPAA 164.512(e) and stricter state privilege law require.

- A. Send the full record by the subpoena's deadline — Owner: Treating a subpoena as a court order. A subpoena is not a court order; releasing records without consent or proper legal assurances can breach confidentiality.
- B. Disregard it, since counselors' records are privileged — Owner: Ignoring legal process (contempt risk). Privilege must be asserted, not ignored; failing to respond can expose the counselor to sanctions.
- **Pair to learn:** Attorney's subpoena → legal consultation and client notice first

123 — A

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **A. Robert Carkhuff — Owner: Carkhuff (empathy scale).** Carkhuff's scale runs from levels that subtract (1 and 2) to interchangeable (3) to additive (4 and 5).
- B. Gerard Egan — Owner: Egan (The Skilled Helper). Egan owns SOLER and a three-stage helping model.
- C. Clara Hill — Owner: Hill (helping skills model). Hill's model moves from exploration to insight to action.
- **Pair to learn:** Five-level empathy scale → Carkhuff

124 — A

Professional Development & Counselor Self-Awareness · Task 1B.8 · Chapter 22

- **A. Confirm meeting the scale's training and user qualifications — Owner: ACA E.2.a limits of competence (assessment).** Counselors use only the instruments they are trained and qualified to administer and interpret.
- B. Administer it by following the manual's instructions — Owner: Manual-reading as competence. Reading the instructions is not training; structured rating scales require training to administer reliably.
- C. Have front-desk staff give it to save the counselor time — Owner: Unqualified administration (ACA E.2.b).

Counselors remain responsible for the appropriate use of assessments, including who administers them.

- **Pair to learn:** Unfamiliar instrument → verify qualifications first (ACA E.2.a)

125 — C

Indirect Client Care · Task 5.3 · Chapter 21

- **C. GIRP — Owner: GIRP note format.** GIRP stands for Goal, Intervention, Response, Plan.
- A. DAP — Owner: DAP note format. DAP stands for Data, Assessment, Plan.
- B. BIRP — Owner: BIRP note format. BIRP begins with Behavior, the client's presentation, not Goal.
- **Pair to learn:** Goal, Intervention, Response, Plan → GIRP

126 — A

Treatment Planning & Continuity of Care · Task 3A.3 · Chapter 19

- **A. Decline joint sessions and offer individual counseling with safety planning — Owner: Individual modality when coercive control is present.** Couple work is indicated only when there is no ongoing violence or coercive control; individual sessions allow safety assessment and planning.
- B. Begin couples counseling that focuses on communication skills — Owner: Couple modality (contraindicated with coercive control). Conjoint work while coercive control is ongoing can increase danger and keep the at-risk partner from speaking freely.
- C. Raise the disclosure with both partners in the next joint session — Owner: Conjoint confrontation (breaks private IPV screening). IPV is screened privately; exposing the disclosure in a joint session can put the at-risk partner in danger.
- **Pair to learn:** Coercive control present → Individual modality (couples counseling contraindicated)

127 — B

Legal & Ethical Compliance · Task 6.18 · Chapter 17

- **B. Decline, since chart access requires a job-related need — Owner: HIPAA minimum necessary / role-based access (45 CFR 164.514(d)).** Workforce members view records only as their role in the client's care or operations requires.
- A. Show the chart, since all practice staff use the same EHR — Owner: System access mistaken for authorization. Having EHR access does not create a reason to view a particular client's chart.
- C. Lend the colleague login credentials to view it directly — Owner: Credential sharing (HIPAA unique user ID, 164.312(a)(2)(i)). Each user needs a unique login so access is traceable; sharing credentials defeats the audit trail.
- **Pair to learn:** Uninvolved colleague asks for a chart → no job-related need, no access

128 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Acquired capability — Owner: Joiner (interpersonal theory).** Reduced fear of death and increased pain tolerance make an attempt possible.
- A. Psychache — Owner: Shneidman (suicidology). Psychache is Shneidman's term for intolerable psychological pain.
- C. Perceived burdensomeness — Owner: Joiner (interpersonal theory). Burdensomeness, with thwarted belongingness, produces the desire for death, not the capability.
- **Pair to learn:** Acquired capability → Joiner

129 — C

Intake & Assessment · Task 2B.5 · Chapter 19

- **C. Flight of ideas — Owner: Flight of ideas (common in mania).** Flight of ideas is rapid jumping between loosely connected topics and is common in mania, which fits the reduced sleep and rapid talk.
- A. Tangential thinking — Owner: Tangential thought process. Tangential thinking drifts off the question without returning, but the rapid, loosely linked jumps in a manic picture define flight of ideas.
- B. Loose associations — Owner: Loose associations (common in psychosis). Loose associations have no logical connection at all; here each jump still has a loose link.
- **Pair to learn:** Rapid jumps between loosely connected topics → Flight of ideas (not loose associations)

130 — C

Professional Development & Counselor Self-Awareness · Task 1B.9 · Chapter 22

- **C. Decline, thank him, and explore what the offer means — Owner: Boundary maintenance with exploration of meaning.** Declining warmly keeps the relationship professional, and exploring the gesture can be useful clinical material.
- A. Accept, since free design work is a harmless gift — Owner: Accepting services as a gift (ACA A.10.f). Labor for the counselor's business is not a small token; it creates an ongoing obligation and a second relationship.
- B. Accept the work but pay him his usual rate — Owner: Business relationship with a client (multiple relationship). Paying him turns him into a vendor, adding a business role to the counseling relationship.
- **Pair to learn:** Client offers work for the counselor → decline and explore meaning

131 — B

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **B. Slow down and help her ground in the room — Owner: Containment and grounding.** A flooded client needs grounding and stabilization before the session ends; detail can wait.
- A. Reassure her that she is completely safe now — Owner: Reassurance that dismisses the feeling. Reassurance skips the client's present experience and doesn't ground her.
- C. Encourage her to keep going while the memory is fresh — Owner: Pushing for trauma detail. Pushing for detail while a client is flooded increases dysregulation.
- **Pair to learn:** Flooded client → contain and ground before closing

132 — A

Provision of Counseling Interventions · Task 4.14 · Chapter 22

- **A. Genuineness — Owner: Genuineness (Rogers; Truax & Carkhuff).** Genuineness is being real and authentic, not hiding behind a professional role or façade.
- B. Unconditional positive regard — Owner: Unconditional positive regard (Rogers). UPR is valuing the client as a person without conditions, not the counselor's authenticity.
- C. Empathic attunement — Owner: Empathic attunement (internal sensing). Attunement is the internal process of sensing the client's experience, not being free of a façade.
- **Pair to learn:** Being real, no façade → Genuineness (not empathy)

133 — A

Legal & Ethical Compliance · Task 6.19 · Chapter 17

- **A. Confirm a business associate agreement and obtain client consent — Owner: HIPAA business associate agreement (45 CFR 164.502(e), 164.504(e)).** A vendor handling client information needs a BAA, and clients should give informed consent to recording and AI processing.
- B. Use a free consumer AI tool and delete the audio afterward — Owner: Non-compliant vendor use (no BAA). Deleting audio later does not cure sending client information to a vendor with no BAA or security assurances.
- C. Rely on the Notice of Privacy Practices as client consent — Owner: Notice of Privacy Practices (45 CFR 164.520). The privacy notice describes practices; it is not informed consent to recording sessions or using an AI tool.
- **Pair to learn:** AI note-taker → BAA + informed consent before use

134 — A

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **A. Irvin Yalom — Owner: Yalom (existential).** Yalom named these four givens; existential counseling helps clients confront them.
- B. Viktor Frankl — Owner: Frankl (logotherapy). Frankl's focus is the search for meaning, with techniques such as paradoxical intention and dereflection.
- C. Fritz Perls — Owner: Perls (Gestalt). Perls focused on here-and-now awareness and unfinished business.
- **Pair to learn:** Four givens → Yalom

135 — C

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **C. Equifinality — Owner: Systems theory.** Different starting points leading to the same outcome is equifinality.
- A. Homeostasis — Owner: Don Jackson. Homeostasis is a family's tendency to resist change and restore balance.
- B. Multifinality — Owner: Systems theory. Multifinality is the reverse: the same starting point leading to different outcomes.
- **Pair to learn:** Different starts, same outcome → equifinality

136 — B

Professional Development & Counselor Self-Awareness · Task 1B.10 · Chapter 22

- **B. Self-care is an ongoing, preventive part of ethical practice — Owner: Skovholt (resilient practitioner); ACA Section C.** Self-care is a sustained practice to maintain well-being so counselors can meet their responsibilities, not a crisis response.
- A. Self-care belongs only in the counselor's personal therapy — Owner: Personal therapy (one strategy — Norcross). Personal therapy is one valuable self-care strategy, not the whole of self-care.
- C. Self-care means time off only after impairment occurs — Owner: Impairment response (ACA C.2.g). Stepping back when impaired is required, but self-care aims to prevent reaching impairment.
- **Pair to learn:** Self-care only in crisis → ongoing preventive practice (Skovholt; ACA Section C)

137 — A

Intake & Assessment · Task 2B.11 · Chapter 20

- **A. Sensitivity — Owner: Sensitivity (true positive rate).** Sensitivity is the ability to correctly identify people who have the condition, and screening tools prioritize it.
- B. Specificity — Owner: Specificity (true negative rate). Specificity is the ability to correctly identify people who don't have the condition.
- C. Base rate — Owner: Base rate (how common the condition is). Base rate describes how common the condition is in a population, not how well a test detects it.
- **Pair to learn:** Identifies those who have the condition → Sensitivity (not specificity)

138 — A

Intake & Assessment · Task 2A.1 · Chapter 6

- **A. Support her in processing it; no mandated report applies — Owner: Past adult-disclosed abuse (generally not reportable).** Past abuse disclosed by an adult, with no child currently at risk, is generally not a mandated report; the work is clinical.
- B. Report the abuse to adult protective services — Owner: Adult protective services reporting. APS addresses current abuse of older or dependent adults, not a competent adult's childhood history.
- C. Report the abuse to child protective services today — Owner: Mandated child abuse reporting. Child abuse reporting protects children currently at risk; none is identified here.
- **Pair to learn:** Adult's past childhood abuse, no child at risk → Generally not reportable

139 — C

Legal & Ethical Compliance · Task 6.20 · Chapter 17

- **C. Keep separate professional and personal profiles — Owner: ACA H.6.a (virtual professional presence).** Counselors who keep both kinds of presence create separate professional and personal pages to distinguish them.
- A. Ask satisfied current clients to post online reviews — Owner: ACA C.3.b (testimonials). Counselors do not solicit testimonials from current or former clients or others vulnerable to undue influence.
- B. Reply publicly to a client's review to thank her — Owner: Confirming the relationship (ACA B.1.c; HIPAA). A public reply confirms that the person is a client, disclosing protected information without consent.
- **Pair to learn:** Practice social media → separate professional and personal profiles (ACA H.6.a)

140 — C

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **C. Altruism — Owner: Yalom: altruism.** Helping another member raises one's own sense of worth.
- A. Instillation of hope — Owner: Yalom: instillation of hope. Hope comes from seeing other members improve, not from helping them.
- B. Universality — Owner: Yalom: universality. Universality is the relief of learning "I'm not the only one."
- **Pair to learn:** Helping others raises self-worth → altruism (Yalom)

141 — A

Provision of Counseling Interventions · Task 4.11 · Chapter 21

- **A. Explore his ambivalence using motivational interviewing — Owner: Adherence: motivational interviewing for ambivalence.** When ambivalence is the barrier to adherence, MI explores it without judgment.
- B. Remind him that homework is required to continue — Owner: Compliance (obedience framing). Demanding compliance replaces collaboration with obedience.
- C. Drop homework from the plan to avoid any further conflict — Owner: Abandoning the rationale. Removing the task skips exploring barriers and explaining its value.
- **Pair to learn:** Ambivalence about homework → MI to support adherence (4.11)

142 — A

Professional Development & Counselor Self-Awareness · Task 1A.1 · Chapter 22

- **A. Share only the de-identified details the question requires — Owner: ACA B.7.b disclosure in consultation.** Under B.7.b, counselors share no identifying information without consent unless unavoidable, and disclose only what the consultation needs.
- B. Share the full chart so the advice can be fully specific — Owner: Over-disclosure in consultation. A full chart exceeds what a skill consultation requires and exposes identifying details the consultant does not need.
- C. Avoid any consultation until the client signs a release — Owner: Overly rigid confidentiality (misread of ACA B.7.b). A release is needed to disclose

identifying information; a de-identified consultation can proceed without one.

- **Pair to learn:** Consulting on a case → de-identified, need-to-know details (ACA B.7.b)

143 — A

Intake & Assessment · Task 2B.4 · Chapter 19

- **A. Predisposing factor — Owner: 4 Ps: predisposing (vulnerability).** Predisposing factors are what made the client vulnerable, such as genetics, early experiences, and temperament.
- B. Perpetuating factor — Owner: 4 Ps: perpetuating (maintains the problem). Perpetuating factors keep the problem going, such as avoidance or substance use, not inherited risk.
- C. Precipitating factor — Owner: 4 Ps: precipitating (trigger). A precipitating factor is what triggered the current problem; family history is a background vulnerability, not a trigger.
- **Pair to learn:** Genetic vulnerability → Predisposing factor (4 Ps)

144 — B

Professional Development & Counselor Self-Awareness · Task 1A.4 · Chapter 22

- **B. Efficacy under ideal research conditions — Owner: Efficacy research (RCTs; Chambless & Hollon, 1998).** Controlled randomized trials with selected samples show efficacy: the treatment works under ideal conditions. Such evidence is part of the empirical foundation ACA C.7.a asks counselors to ground their techniques in.
- A. Effectiveness in routine community practice — Owner: Effectiveness research (real-world transportability). Effectiveness asks whether a treatment works in everyday settings with typical, often comorbid clients — screened trial samples do not answer that.

- C. Clinical significance for individual clients — Owner: Clinical significance (Jacobson & Truax, 1991). Clinical significance asks whether an individual's change is meaningful (e.g., the Reliable Change Index), not whether a group mean beat placebo.
- **Pair to learn:** Controlled trials, screened samples → efficacy (real-world settings → effectiveness)

145 — B

Legal & Ethical Compliance · Task 6.1 · Chapter 17

- **B. Move with the client to the plan's designated shelter area — Owner: Practice emergency preparedness plan (shelter in place).** A warning activates the plan's shelter-in-place procedure; the counselor stays with the client and checks in once the warning ends.
- A. End the session early so the client can drive home quickly — Owner: Improvised dismissal (error pattern). Sending a client onto the road during a warning increases danger; severe-weather plans direct people to shelter in place.
- C. Continue the session, since most warnings pass without harm — Owner: Normalcy bias (error pattern). Discounting an official warning delays protective action that the plan already specifies.
- **Pair to learn:** Tornado warning mid-session → shelter in place with the client per the plan

146 — C

Intake & Assessment · Task 2A.1 · Chapter 17

- **C. May be justified in disclosing to the identifiable partner — Owner: ACA B.2.c contagious, life-threatening diseases.** After assessing whether the client intends to inform the partner, disclosure to an identifiable third party at serious and foreseeable risk may be justified, subject to state law.
- A. May keep it confidential as an end-of-life decision — Owner: ACA B.2.b end-of-life decisions. B.2.b covers

terminally ill clients considering hastening death, not risk to a partner.

- B. Must keep this confidential as he is a competent adult — Owner: Absolute confidentiality (not the ACA standard). Confidentiality has limits, and B.2.c names this situation as one of them.
- **Pair to learn:** Identifiable partner at risk of contagious disease → ACA B.2.c

147 — C

Professional Development & Counselor Self-Awareness · Task 1A.2 · Chapter 22

- **C. Review the risk formulation and own reactions in supervision — Owner: Clinical supervision — client welfare (ACA F.1.a).** Supervisors monitor client welfare; bringing both the risk judgment and the counselor's reactions lets supervision do its job.
- A. Refer the client out so the content is no longer heard — Owner: Referral to escape counselor discomfort. Referring out because the counselor is uncomfortable shifts the counselor's problem onto a client who is still treatable.
- B. Keep the doubts private to appear competent to the supervisor — Owner: Supervisee nondisclosure (Ladany et al.). Supervisees often withhold doubts to manage impressions, which defeats the purpose of supervision and risks the client.
- **Pair to learn:** Supervisee unsure about risk → bring it to supervision

148 — C

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **C. The miracle question — Owner: de Shazer & Berg (SFBT).** Imagining a solved problem and its first small signs is the SFBT miracle question.
- A. An externalizing question — Owner: White & Epston (narrative). Externalizing questions separate the

person from the problem by naming it as an outside entity.

- B. Adler's "the Question" — Owner: Adler (individual psychology). Adler asks what would be different if the client were well, to learn what the symptom helps the client avoid.
- **Pair to learn:** Miracle question → de Shazer & Berg (SFBT)

149 — B

Treatment Planning & Continuity of Care · Task 3A.10 · Chapter 21

- **B. Reassess and revise the plan with her — Owner: Reassess when progress stalls (3A.10).** New substance use and a new stressor may be blocking progress; reassessing and revising the plan together is the scored response.
- A. Refer her out, since drinking is outside the plan — Owner: Premature referral. Drinking calls for reassessment and possibly concurrent referral, not ending the relationship.
- C. Continue the plan, since change takes time — Owner: Continuing an ineffective plan. Repeating what isn't working is what the exam punishes.
- **Pair to learn:** Stalled progress plus new drinking → Reassess and revise the plan (3A.10)

150 — A

Legal & Ethical Compliance · Task 6.2 · Chapter 17

- **A. Follow the protocol's outreach steps and document each one — Owner: Practice SOP (at-risk no-show protocol).** The written protocol sets timely outreach through the client's consented contact methods; documenting each step shows it was followed.
- B. Wait for the client to reschedule, respecting adult autonomy — Owner: Autonomy over safety protocol (error pattern). Autonomy matters, but the practice has

already defined outreach for at-risk clients, and ignoring it leaves risk unchecked.

- C. Call the client's workplace to ask about the client's safety — Owner: Unauthorized disclosure (ACA B.1.c; HIPAA). Contacting an employer without consent reveals the client is in treatment; outreach uses consented contacts.
- **Pair to learn:** At-risk client no-shows → follow the written outreach protocol and document

151 — C

Provision of Counseling Interventions · Task 4.2 · Chapter 22

- **C. Thank them briefly and use their pronouns consistently — Owner: Respect for gender identity (using the client's pronouns).** Using the client's name, pronouns, and language for their identity is the attribute the exam rewards.
- A. Apologize at length for the mistake — Owner: Excessive apology (centers the counselor). A long apology shifts attention to the counselor's discomfort instead of the client.
- B. Ask why they decided to use those pronouns — Owner: "Why" question (invites justification). A "why" question asks the client to justify their identity and invites defensiveness.
- **Pair to learn:** Client states pronouns → Use them (gender sensitivity, 4.2)

152 — B

Treatment Planning & Continuity of Care · Task 3A.3 · Chapter 21

- **B. Basic needs and stabilization — Owner: Sequencing: basic needs and stabilization (Maslow).** With safety addressed, housing and food come before insight-oriented work.

- A. Exploring childhood relationship patterns — Owner: Sequencing: longer-term growth. Insight work is longer-term growth and comes after stabilization.
- C. A full trauma history in the next session — Owner: Premature trauma exploration. A detailed history doesn't address urgent basic needs.
- **Pair to learn:** Eviction and no food → Basic needs before insight (sequencing)

153 — B

Professional Development & Counselor Self-Awareness · Task 1A.3 · Chapter 22

- **B. C.2.f Continuing Education — Owner: ACA C.2.f continuing education.** C.2.f states that counselors recognize the need for continuing education to stay current and maintain competence in the skills they use.
- A. C.2.a Boundaries of Competence — Owner: ACA C. 2.a boundaries of competence. C.2.a limits practice to the counselor's education, training, and supervised experience; it does not set the ongoing-learning duty.
- C. C.2.g Impairment — Owner: ACA C.2.g impairment. C.2.g addresses refraining from services when a counselor's own problems are likely to harm clients.
- **Pair to learn:** Duty to stay current → ACA C.2.f continuing education

154 — C

Treatment Planning & Continuity of Care · Task 3A.2 · Chapter 21

- **C. Time-bound — Owner: SMART: Time-bound.** The objective has no target date or time frame, such as within 8 weeks.
- A. Relevant — Owner: SMART: Relevant. Nothing suggests the objective is unrelated to the client's concern.
- B. Specific — Owner: SMART: Specific. Two support group meetings per week is already specific.

- **Pair to learn:** No target date → SMART: Time-bound

155 — A

Legal & Ethical Compliance · Task 6.2 · Chapter 13

- **A. Stigmatization — Owner: Finkelhor & Browne (traumagenic dynamics).** Shame, guilt, and a sense of being damaged becoming part of the self-image is the stigmatization dynamic.
- B. Betrayal — Owner: Finkelhor & Browne (traumagenic dynamics). Betrayal is broken trust in someone the child depended on.
- C. Powerlessness — Owner: Finkelhor & Browne (traumagenic dynamics). Powerlessness is the repeated overriding of the child's will and control.
- **Pair to learn:** Feeling damaged → stigmatization (Finkelhor & Browne)

156 — C

Legal & Ethical Compliance · Task 6.3 · Chapter 17

- **C. Honor the reasonable request without requiring a reason — Owner: Right to request confidential communications (45 CFR 164.522(b)).** Providers must accommodate reasonable requests to receive communications by alternative means or at alternative locations, without asking why.
- A. Ask him to explain why before changing his contact method — Owner: Requiring justification (prohibited for providers). A provider may not require a client to explain the reason for a confidential-communication request.
- B. Obtain his signed authorization before using the new number — Owner: HIPAA authorization (45 CFR 164.508). Authorizations cover uses and disclosures outside treatment, payment, and operations — not how the client wants to be contacted.
- **Pair to learn:** Contact me only by this phone → HIPAA confidential communications (164.522(b))

157 — B

Professional Development & Counselor Self-Awareness · Task 1B.11 · Chapter 22

- **B. Managing countertransference — Owner: Hayes & Gelso (countertransference management).** Hayes and Gelso identified these five factors in managing countertransference.
- A. Building the working alliance — Owner: Bordin (goals, tasks, bond). Bordin's working alliance has three parts: goals, tasks, and bond.
- C. Repairing alliance ruptures — Owner: Safran & Muran (rupture and repair). Safran and Muran describe withdrawal and confrontation ruptures repaired by naming them openly, not these five factors.
- **Pair to learn:** Self-insight, self-integration, anxiety management → Managing countertransference (Hayes & Gelso)

158 — B

Provision of Counseling Interventions · Task 4.2 · Chapter 15

- **B. Reverse culture shock on returning home — Owner: Gullahorn & Gullahorn (W-curve).** The W-curve adds a second dip in adjustment when the person returns to the home culture.
- A. An initial honeymoon period — Owner: Oberg (culture shock phases). The honeymoon period is already part of Oberg's sequence and the U-curve.
- C. Marginalization from both cultures — Owner: Berry (acculturation strategies). Marginalization is one of Berry's acculturation strategies.
- **Pair to learn:** Second dip on returning home → W-curve (Gullahorn & Gullahorn)

159 — A

Legal & Ethical Compliance · Task 6.6 · Chapter 17

- **A. Stop practicing until the board restores the license — Owner: State licensure law (practice requires an active license).** Once a license lapses, practice stops until the board reinstates it; any grace period or late-renewal rule depends on the state.
- B. Keep seeing clients and complete renewal when convenient — Owner: Unlicensed practice (state licensure law). Practicing on a lapsed license is unlicensed practice and can also make claims for those sessions improper.
- C. Have a licensed colleague cosign her notes until renewal — Owner: Cosignature misapplied (not licensure). A colleague's signature does not confer a license; it cannot cover independent practice on a lapsed credential.
- **Pair to learn:** Lapsed license → stop practice and reinstate per board rules

160 — C

Provision of Counseling Interventions · Task 4.4 · Chapter 7

- **C. Support the partner in disclosing, per the policy — Owner: ACA A.8 and B.4.b (no-secrets policy).** Following the policy explained during informed consent, the counselor encourages and supports the partner in disclosing.
- A. Agree to keep the affair private — Owner: Violating the stated secrets policy. Keeping a secret contradicts the policy both partners agreed to.
- B. Reveal it at the next joint session without warning — Owner: Disclosure without preparation. Revealing it without first working with the partner blindsides the client.
- **Pair to learn:** No-secrets policy → support disclosure (ACA A.8, B.4.b)

161 — B

Treatment Planning & Continuity of Care · Task 3A.2 · Chapter 21

- **B. Long-term goal — Owner: Long-term goal.** A broad outcome statement is a long-term goal.
- A. Short-term objective — Owner: Short-term objective. Objectives are measurable steps, such as a target score by a date.
- C. Problem statement — Owner: Problem statement. A problem statement describes the current concern in behavioral terms.
- **Pair to learn:** Broad outcome → Long-term goal

162 — B

Legal & Ethical Compliance · Task 6.7 · Chapter 17

- **B. Honor the request, document it, and submit no claim to the plan — Owner: HIPAA restriction right (45 CFR 164.522(a)(1)(vi)).** A covered entity must agree to withhold information from a health plan for services the client pays for in full out of pocket.
- A. Bill the plan anyway, since covered services must be submitted — Owner: Mandatory-submission myth. HIPAA requires honoring this restriction once the client pays in full, so the plan need not receive a claim.
- C. Bill under a less sensitive diagnosis code instead — Owner: Fraudulent coding (ACA C.6.b). Submitting a diagnosis that does not reflect the client's condition is inaccurate reporting to a third party and insurance fraud.
- **Pair to learn:** Client self-pays in full to keep it from the plan → honor the restriction (45 CFR 164.522)

163 — B

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **B. Agreement on goals — Owner: Bordin (working alliance): goals.** The client and the counselor's work

aren't yet aimed at an outcome the client wants, which is a goals problem.

- A. The bond — Owner: Bordin (working alliance): bond. Nothing here shows distrust of or conflict with the counselor.
- C. Agreement on tasks — Owner: Bordin (working alliance): tasks. A tasks problem is disagreement about methods, such as doubting that worksheets help.
- **Pair to learn:** Client wants a different outcome → Bordin: goals

164 — B

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **B. "When you checked your phone as I spoke, I felt brushed off." — Owner: Effective feedback: specific, timely, owned.** It is specific to behavior, recent, and owned by the speaker.
- A. "A while back you seemed distracted at some point." — Owner: Vague, untimely feedback. It isn't specific or timely, so it's hard to act on.
- C. "Everyone in here thinks you're kind of selfish, honestly." — Owner: Unowned, character-based feedback. It speaks for others and labels character instead of behavior.
- **Pair to learn:** Specific, timely, owned → effective feedback

165 — C

Legal & Ethical Compliance · Task 6.8 · Chapter 17

- **C. A Type 2 (organizational) NPI for the entity — Owner: Type 2 NPI (organizational providers).** Practice entities such as a PLLC that bill as an organization use a Type 2 NPI, while the counselor keeps her Type 1 NPI as the rendering provider.
- A. A second Type 1 NPI issued to the business — Owner: Type 1 NPI (individual providers only). Type 1

NPIs identify individual practitioners, and each individual receives only one.

- B. A DEA registration number for the entity — Owner: DEA registration (controlled-substance prescribers). DEA numbers authorize prescribing or handling controlled substances; they are not billing identifiers for counseling entities.
- **Pair to learn:** Billing under a practice entity → Type 2 (organizational) NPI

166 — A

Provision of Counseling Interventions · Task 4.5 · Chapter 22

- **A. Group cohesiveness — Owner: Yalom: group cohesiveness.** Cohesiveness is described as Yalom's group equivalent of the alliance, and leaders build it through their own attributes.
- B. Universality — Owner: Yalom: universality. Universality is realizing one is not alone in one's problems, not the group's version of the alliance.
- C. Altruism — Owner: Yalom: altruism. Altruism is benefiting from helping other members, not the group's version of the alliance.
- **Pair to learn:** Group equivalent of the alliance → Group cohesiveness (Yalom)

167 — B

Provision of Counseling Interventions · Task 4.4 · Chapter 22

- **B. "You're feeling overlooked here. I'd like to hear more about that." — Owner: Conflict tolerance: name and explore non-defensively.** Staying present, naming the conflict, and exploring it non-defensively models respectful conflict for the group.
- A. "I'm so sorry. I never meant to make anyone feel that way." — Owner: Excessive apology (appeasement). Appeasing with an apology closes the conflict instead of exploring it.

- C. “I give every member equal time, so that isn’t accurate.” — Owner: Defensiveness (not conflict tolerance). Defending the leader escalates the conflict and is what the exam punishes.
- **Pair to learn:** Member anger at the leader → Conflict tolerance: explore non-defensively

168 — A

Legal & Ethical Compliance · Task 6.9 · Chapter 17

- **A. An encrypted platform with a signed business associate agreement — Owner: HIPAA business associate rules (45 CFR 164.502(e), 164.504(e)).** A vendor that transmits or stores client information is a business associate; HIPAA requires a BAA plus appropriate safeguards such as encryption.
- B. Any video app the client already uses and trusts on her phone — Owner: Client preference (not compliance). Comfort with an app does not supply a BAA or the safeguards HIPAA requires of the practice.
- C. A free consumer app, since live video is never stored as a record — Owner: Transmission security (45 CFR 164.312(e)), misread. HIPAA protects information in transit as well as at rest, so live sessions still need a compliant platform.
- **Pair to learn:** Choosing a telehealth platform → BAA + encryption (HIPAA business associate rules)

169 — C

Provision of Counseling Interventions · Task 4.3 · Chapter 5

- **C. Treat the lapse as a learning event — Owner: Marlatt (relapse prevention).** Reframing a slip as information counters the abstinence violation effect and identifies the high-risk situation.
- A. Start treatment over at precontemplation — Owner: Misapplied stages of change (Prochaska & DiClemente). Relapse is a normal part of change; the client hasn’t stopped recognizing the problem.

- B. Confront him about breaking his commitment — Owner: Confrontation (contrary to MI). Confrontation adds shame and feeds the all-or-nothing reaction to the lapse.
- **Pair to learn:** Abstinence violation effect → Marlatt (relapse prevention)

170 — B

Legal & Ethical Compliance · Task 6.10 · Chapter 17

- **B. 90834 (38–52 minutes) — Owner: CPT 90834 (45-minute psychotherapy).** 90834 covers 38–52 minutes of psychotherapy, which matches the documented 45 minutes.
- A. 90832 (16–37 minutes) — Owner: CPT 90832 (30-minute psychotherapy). 90832 covers 16–37 minutes; billing it would undercode a 45-minute session.
- C. 90837 (53 or more minutes) — Owner: Upcoding (billing fraud). 90837 requires at least 53 minutes; billing it for 45 minutes claims time the record does not support.
- **Pair to learn:** 45 documented minutes → 90834 (38–52)

Score by Domain

Domain	Items	Your correct	%
Professional Development & Counselor Self-Awareness	25		
Intake & Assessment	35		
Treatment Planning & Continuity of Care	23		
Provision of Counseling Interventions	39		
Indirect Client Care	17		
Legal & Ethical Compliance	31		
Total	**170**		

Scaled NCE scores are not percentages, and no practice percentage maps onto a pass or fail. Use this table to find your two weakest domains and give them extra study time next week.

Item numbers by domain

- **Professional Development & Counselor Self-Awareness:** 5, 6, 10, 18, 29, 41, 42, 52, 64, 66, 73, 81, 90, 98, 107, 116, 117, 124, 130, 136, 142, 144, 147, 153, 157
- **Intake & Assessment:** 1, 2, 11, 13, 16, 23, 27, 30, 34, 43, 48, 50, 51, 53, 60, 62, 72, 76, 79, 82, 83, 85, 88, 91, 92, 99, 102, 108, 114, 121, 129, 137, 138, 143, 146
- **Treatment Planning & Continuity of Care:** 7, 8, 12, 17, 24, 28, 33, 35, 36, 37, 47, 58, 59, 69, 71, 77, 97, 105, 126, 149, 152, 154, 161
- **Provision of Counseling Interventions:** 4, 14, 20, 25, 31, 40, 45, 54, 57, 65, 74, 80, 89, 100, 103, 106, 109, 111, 112, 115, 118, 120, 123, 128, 131, 132, 134, 135, 140, 141, 148, 151, 158, 160, 163, 164, 166, 167, 169
- **Indirect Client Care:** 9, 19, 21, 32, 44, 46, 55, 63, 67, 75, 84, 93, 94, 101, 110, 119, 125
- **Legal & Ethical Compliance:** 3, 15, 22, 26, 38, 39, 49, 56, 61, 68, 70, 78, 86, 87, 95, 96, 104, 113, 122, 127, 133, 139, 145, 150, 155, 156, 159, 162, 165, 168, 170

NCE Practice Exam 2

170 items · 3 options each · 3 hours 45 minutes · Answer every item.

Take this exam under timed conditions before checking the key. Use the four-step routine on every item: read the last line first, name the owner, predict, then eliminate by owner.

Pacing checkpoints: item 43 by 0:50 · item 85 by 1:39 (end of the first half: review it, submit it, take the optional break) · item 128 by 2:29 · item 170 by 3:18.

Original items written for this book. Not affiliated with or endorsed by NBCC. No content from any live examination.

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1. Without using any checklist, a counselor notices that a client wears sunglasses indoors and glances at the door each time someone passes in the hall. This information comes from:

A. Formal observation
B. Informal observation
C. Client self-report

2. Three days after stopping years of heavy daily drinking, a client is confused and disoriented, sees insects crawling on the wall, is sweating heavily, and has a racing pulse. This most likely reflects:

A. Stimulant withdrawal
B. Cannabis withdrawal
C. Delirium tremens

3. During an intake, the building loses power and the practice’s electronic records system goes down. What should the counselor do first?

A. Confirm the client's safety and use the plan's downtime procedures
B. Type the client's answers into a personal phone app for now
C. Send the client home with no set time to finish the intake

4. A counselor asks each client to complete a brief rating at every session to learn how the work is landing. The APA task forces that judged collecting client feedback demonstrably effective were led by:

A. Norcross and Lambert
B. Safran and Muran
C. Truax and Carkhuff

5. A client meets full criteria for both generalized anxiety disorder and alcohol use disorder. Neither diagnosis's criteria exclude the other. The counselor should:

A. Diagnose both and plan integrated treatment
B. Diagnose only the principal disorder
C. Treat the drinking first, then the anxiety

6. A counselor consulted once with a trained colleague before starting a new therapy for chronic depression. Six sessions later, the client's PHQ-9 scores are unchanged. What is the best next step?

A. Continue as planned, since the colleague approved the approach
B. Return to the consultant with the outcome data to review the plan
C. Switch to a different therapy without consulting anyone

7. A counselor brings a difficult case to her peer consultation group. The client has not consented to being identified. The counselor should:

A. Share the full chart so peers can help
B. Share only needed, nonidentifying details
C. Hand clinical responsibility to the group

8. A new mother signs a release so her home-visiting nurse can support her counseling goals. The best way to collaborate is to:

A. Let the nurse set separate goals on her own visit plan
B. Defer counseling goals to the nurse's health priorities
C. Agree on shared goals, roles, and a plan for staying in touch

9. A client stopped drinking 14 months ago. She lives at home, takes no medication for alcohol use, and reports no use disorder criteria except occasional cravings. The correct specifier is:

A. In early remission
B. In a controlled environment
C. In sustained remission

10. A client asks to read her counseling records. There is no evidence that reading them would harm her. The counselor should:

A. Show her the psychotherapy notes only
B. Allow access and help her understand them
C. Refuse, since records belong to the counselor

11. A client names four concerns at intake, and he wants to work on all of them at once. The counselor's best response is to:

A. Rank them by impact and readiness, then start with one or two
B. Work on all four equally so no concern feels set aside
C. Wait to set priorities until his spouse can choose them

12. A client arrives for his first appointment smelling of alcohol and slurring his words, and he is eager to sign the consent forms. The counselor's best action is to:

A. Delay consent until he is sober
B. Have him sign now and review it later
C. Proceed, since he is an adult volunteer

13. A 23-year-old has changed jobs and apartments three times in two years. She says she is exploring who she is,

focusing on herself, and feels “not a teenager, but not really an adult either.” Which concept fits best?

A. Emerging adulthood
B. Intimacy vs. isolation
C. Age-30 transition

14. Before beginning exposure-based treatment with an adult client, the practice’s informed consent procedure requires the counselor to:

A. Begin exposures and get written consent once progress shows
B. Review purpose, likely distress, options, and the right to refuse
C. Skip formal consent, since the method is evidence-based

15. Under IDEA, which provision serves infants and toddlers with developmental delays from birth to age 3?

A. A Section 504 plan
B. Part C early intervention
C. An Individualized Education Program

16. A counselor holds a state license and a national certification, each with its own renewal cycle. To meet continuing education requirements, the counselor should:

A. Assume the national certification’s hours satisfy every licensing board
B. Follow the rules of the state where the counselor completed graduate school
C. Verify each credential’s hour, topic, and provider rules separately

17. While building a plan with a client who has recurring depression, which counselor question best identifies strengths to include?

A. Which symptom bothers you most right now?
B. How did your childhood shape this pattern?
C. What helped you get through hard times before?

18. An adult child of an alcoholic father says, “Nobody ever mentioned Dad’s drinking. We learned not to count on anyone and to keep our feelings to ourselves.” Who described these family rules?

A. Claudia Black
B. E. M. Jellinek
C. Sharon Wegscheider-Cruse

19. When a client reports on a new sleep medication from his physician, the counselor’s note should include:

A. Its name, start date, benefits, side effects, and impact
B. The counselor’s view of whether the dose is right
C. Side effects only, since benefits are not relevant

20. At a first session in a small outpatient practice, a counselor learns that a client with an eating disorder needs a structured treatment setting the practice can’t provide. She is not in imminent danger. The best response is to:

A. Begin weekly sessions and reassess in a month
B. Arrange inpatient hospitalization today
C. Discuss the fit and refer to a suitable setting

21. A counselor recommends that a client add weekly group therapy. The client declines, saying she wants to stay in individual sessions only. The counselor should:

A. Make group attendance a condition of counseling
B. Respect her choice and explain any consequences
C. Enroll her in group and revisit consent later

22. A military veteran recently lost his job and separated from his spouse. To integrate his military background into his treatment plan, the counselor should first:

A. Assume he will want practical, task-focused goals only
B. Explore what his service means to him and plan from that
C. Leave his service out, since his current stressors are civilian

23. Early in an intake, a counselor asks a client, “On a scale from 0 to 100, how upsetting is this for you right now?” The counselor is using:

A. A SUDS rating
B. A Likert attitude scale
C. The WHODAS 2.0

24. During a file audit, a counselor finds that a current client never received the practice’s Notice of Privacy Practices. The counselor should:

A. Have the client sign an authorization for treatment disclosures
B. Rely on the notice posted in the waiting room as sufficient
C. Provide the notice now and document that it was provided

25. In a treatment plan, “Client works rotating night shifts and has no reliable childcare” is best listed under:

A. Strengths and resources
B. Problem statement
C. Barriers

26. After a factory closes and thousands lose their jobs overnight, the town’s suicide rate rises. Durkheim would classify these deaths as:

A. Egoistic
B. Fatalistic
C. Anomic

27. When reviewing research on suicide-prevention interventions for a client who is a military veteran, which evidence should carry the most weight?

A. Controlled trials whose samples included at-risk veterans
B. A single case report describing a dramatic recovery
C. Any published study, since publication confirms efficacy

28. As planned termination nears, a client whose father left when she was young says, “Of course you’re done with me too.” The counselor’s best response is to:

A. Explore her feelings about ending and earlier losses
B. Shift the session to building her relapse prevention plan
C. Remind her the end date was set together at the start

29. A repeating pattern of tension-building, an acute battering incident, and then a honeymoon of apology and calm describes:

A. The Power and Control Wheel
B. Coercive control
C. Walker’s cycle of violence

30. Which number belongs on every referral list as the national three-digit line for suicidal or mental health crises?

A. 211
B. 988
C. 311

31. A mother’s adult son refuses any help for his drinking. She wants to learn to reward his sober behavior and encourage him to enter treatment without confrontation. Which approach was designed for her situation?

A. Community Reinforcement and Family Training
B. Brief Strategic Family Therapy
C. Multidimensional Family Therapy

32. Reviewing a diagnosis with a client, the counselor explains that it carries the DSM-5-TR “provisional” specifier. This means:

A. Full criteria are not met, and no specific reason is recorded
B. Full criteria are not met, and the specific reason is recorded
C. Full criteria are expected, but information is not yet sufficient

33. Who described compassion fatigue, the emotional and physical exhaustion of caring for suffering people that includes secondary traumatic stress?

A. Charles Figley
B. Christina Maslach
C. McCann and Pearlman

34. Informed consent tells clients how their care will continue if the counselor dies or becomes incapacitated. The plan that names a records custodian for this purpose is often called a:

A. Release of information
B. Business associate agreement
C. Professional will

35. A licensed counselor moves to a new home address in the same state and changes her legal name. Regarding her license, she should:

A. Notify the board within the time its rules require
B. Wait and report both changes at her next renewal
C. Update only her NPI record, which the board will receive

36. Which progress note format puts both the client's report and the counselor's observations into a single first section?

A. BIRP
B. DAP
C. SOAP

37. A Deaf client who uses American Sign Language requests counseling at a small agency. The agency's best response is to:

A. Communicate through written notes only
B. Provide a qualified ASL interpreter
C. Have her hearing daughter interpret

38. In an ethics training, the principle that obligates counselors to keep promises and honor commitments to clients is:

A. Beneficence
B. Fidelity
C. Veracity

39. A supervisor suggests that a client at moderate suicide risk sign a no-suicide contract. The best evidence-based alternative is to:

A. Rely on the SAD PERSONS score alone
B. Build a written safety plan with her
C. Have her sign the contract and file a copy

40. During a session with an interpreter, the counselor should:

A. Address the interpreter and ask what the client feels
B. Ask the interpreter to summarize the client's answers
C. Speak directly to the client in short segments

41. A telehealth client begins having severe dissociative episodes during video sessions, and family members often walk into her room. Under the ACA Code, the counselor should:

A. Reassess fit and consider in-person sessions
B. Verify her identity with a code word each session
C. Continue unchanged, since she consented to telehealth

42. Which type of termination describes a client who stops attending before treatment goals are met?

A. Forced termination
B. Planned termination
C. Premature termination

43. During an individual screening for couples counseling, a woman discloses that her partner hits her and she is afraid of him. The counselor should:

A. Defer joint sessions and address her safety individually
B. Begin joint sessions to improve the couple's communication
C. Raise her disclosure in the next joint session for honesty

44. During conflict, a wife's heart races, she goes silent, and she turns away from her husband. Using Gottman's antidotes, the counselor coaches her in:

A. Physiological self-soothing
B. Gentle startup
C. Taking responsibility

45. Parents describe their 14-year-old as "rough" with his 8-year-old brother. He repeatedly hits him and takes his things, and the younger boy is now afraid to be home alone with him. The parents call it normal fighting. The counselor should recognize this as possible:

A. Power-seeking misbehavior
B. Sibling abuse
C. Sibling rivalry

46. A licensed professional counselor wants to begin seeing Medicare beneficiaries. Since January 2024, the counselor can:

A. Bill only incident to a supervising physician's services
B. Enroll in Medicare as a mental health counselor (MHC)
C. Bill beneficiaries privately without enrolling or opting out

47. A husband snaps angrily whenever his wife works late. The counselor helps him find, beneath the anger, a fear of being unimportant to her. This move is central to:

A. The Gottman Method
B. Bowen family systems therapy
C. Emotionally focused therapy

48. A counselor wants to paste session details into a free public AI chatbot to help draft a treatment plan. Which principle should guide this decision?

A. Chatbot output is reliable because it draws on vast data
B. Removing the client's name alone makes the disclosure safe

C. Client data should go only to a secure tool covered by a BAA

49. Which case held that a credible threat reported to a therapist by a client's family member can trigger the duty to protect?

A. Jablonski v. United States
B. Ewing v. Goldstein
C. Tarasoff v. Regents (1976)

50. A client who stopped using heroin two days ago has a runny nose, watery eyes, frequent yawning, goosebumps, dilated pupils, muscle aches, and diarrhea. This presentation is most consistent with:

A. Alcohol withdrawal
B. Opioid withdrawal
C. Stimulant intoxication

51. A child has needs across home, school, and juvenile court. Which approach uses a family-driven team to build one individualized plan with natural supports?

A. Wraparound process
B. Assertive community treatment
C. Partial hospitalization

52. When a 13-year-old's parents give legal consent for counseling, the young client's own agreement to take part is called:

A. Informed consent
B. Assent
C. Authorization

53. Which is the best problem statement for a treatment plan?

A. Client has a great deal of anxiety and simply wants to feel calmer overall
B. Client will reduce panic attacks to 1 or fewer a week within 8 weeks
C. Client reports 4 panic attacks a week and missed 3 workdays this month

54. A 7-year-old has frequent tantrums at home and at school. In developing a behavioral management plan, the counselor should:

A. Lengthen time-outs after each tantrum until the behavior stops
B. Define the target behavior, reinforce alternatives, and track it
C. Ignore all behavior, including aggression, so it extinguishes

55. In Forester-Miller and Davis's seven-step ethical decision-making model, which step comes right after applying the ACA Code of Ethics?

A. Apply the publicity test to the choice
B. Evaluate the selected course of action
C. Determine the nature of the dilemma

56. The practice's telehealth vendor reports a security incident that may have exposed client session data. The practice must:

A. Leave all notice to the vendor, since its systems were affected
B. Notify clients only if misuse of their data is later proven
C. Assess the risk and notify clients promptly, within 60 days at most

57. For 15 years, a couple has argued about his wish for quiet weekends at home and her love of busy social plans. Neither wants to change who they are. Gottman would classify this conflict as a:

A. Pursue-withdraw cycle
B. Perpetual problem
C. Solvable problem

58. A client's panic attacks began two weeks after he was laid off from a job he had held for 15 years. In a 4 Ps case formulation, the layoff is a:

A. Predisposing factor
B. Precipitating factor
C. Perpetuating factor

59. A counselor's referrals increasingly come from graduate students, a population the counselor has little experience serving. The best training step is:

A. Reputable training on their stressors, applied to each person
B. Online lists of typical traits used as a guide to each client
C. Reliance on the general multicultural course from graduate school

60. A woman reports that her husband checks her phone, controls all the money, keeps her from seeing friends, and has hit her several times, with the violence getting worse. Using Johnson's typology, this pattern is:

A. Violent resistance
B. Intimate terrorism
C. Situational couple violence

61. An 8-year-old has had severe temper outbursts about four times a week for the past 14 months, starting at age 7, and is irritable and angry most of the day between outbursts. The best fit is:

A. Intermittent explosive disorder
B. Disruptive mood dysregulation disorder
C. Oppositional defiant disorder

62. A client's attorney asks the counselor for records to use in a custody case. The counselor should first:

A. Send the full record, since attorneys have access
B. Obtain the client's signed authorization to release
C. Refuse, since attorneys can never receive records

63. A man who has repeatedly hit his partner asks his counselor for help changing. The standard referral is:

A. A batterer intervention program
B. Conjoint couples counseling
C. An anger management class alone

64. As planned termination nears, a client asks how he will keep the gains he has made. The counselor should:

A. Reassure him that gains always last once counseling ends
B. Continue weekly sessions indefinitely to prevent backsliding
C. Review what helped, plan ongoing practice, and offer boosters

65. After a tense meeting with his supervisor, a client goes home and yells at his son over a small mess. The counselor gently helps him see the link. She is educating him about:

A. Projection
B. Reaction formation
C. Displacement

66. A client who uses a wheelchair was denied a reasonable accommodation at work. Following the ACA Advocacy Competencies, the counselor's first move should be to:

A. Help her prepare to advocate for herself
B. Call her employer and negotiate for her
C. Launch a public campaign about workplace access

67. An insurer audits a counselor's weekly psychotherapy claims. Which progress note content best supports medical necessity?

A. Links among diagnosis, impairment, and plan goals
B. A diagnosis code entered on every claim form
C. A verbatim account of what the client said

68. A counselor learns her client was denied housing assistance and wants to call the housing office to advocate on the client's behalf. First, the counselor should:

A. Get the client's consent
B. Stay out of it, since housing isn't counseling
C. Call the housing office right away

69. Which tool is designed to guide level-of-care decisions for children and adolescents?

A. The ASAM Criteria
B. CALOCUS/CASII
C. LOCUS

70. During a busy term with many clients in acute distress, a counselor notices feeling numb and going through the motions in sessions. What is the best response?

A. Wait to act until clients begin to complain about care
B. Treat it as an early sign of compassion fatigue and act now
C. Take on more clients to rebuild a sense of purpose

71. On a self-report checklist, a client denies any alcohol problems. With his consent, his wife and his previous counselor both describe nightly heavy drinking. The counselor's best approach is to:

A. Accept the checklist, since self-report is most direct
B. Diagnose alcohol use disorder from the collateral reports
C. Weigh all sources and explore the discrepancy with him

72. A client taking lithium says she has a coarse tremor, vomiting, diarrhea, and confusion. The counselor should:

A. Note these as common side effects to recheck next week
B. Suggest she drink less water to ease the nausea
C. Help her get immediate medical evaluation and document it

73. During company layoffs, an executive stays deeply involved in her work, believes her actions can shape what happens, and treats the upheaval as a chance to grow. Her stress buffer best fits:

A. Hardiness
B. Career adaptability
C. Type A behavior

74. A counselor believes that making or reporting a diagnosis would harm a particular client. Which ACA standard addresses this situation?

A. E.5.b, Cultural sensitivity
B. E.5.d, Refraining from diagnosis
C. E.5.c, Historical and social prejudices

75. A client is completing an intensive outpatient program and will step down to weekly counseling. A sound discharge plan includes:

A. A booked follow-up, a release for the next provider, and a crisis plan
B. A list of local resources and an invitation to call when ready
C. A planning meeting on the final day once progress is confirmed

76. After six sessions, a client's ORS scores are flat. Her SRS ratings show she feels the session tasks don't fit her goals. The counselor's best next step is to:

A. Discuss the alliance and adjust tasks with her
B. Continue the plan, since six sessions is early
C. Switch to a new approach without discussing it

77. An agency adds a sliding-scale fee so that clients with low incomes can receive the same services as everyone else. Which ethical principle does this most directly reflect?

A. Beneficence
B. Justice
C. Autonomy

78. A counselor recommends a calming app with guided breathing for a client's panic symptoms. Which implementation is best?

A. Send a download link and let the client explore it alone
B. Practice it in session, tie it to triggers, and review its use
C. Replace weekly sessions with the app to reduce costs

79. Two co-leaders openly disagree in front of members about whether to continue an activity, and the session ends tense. The best next step is for the co-leaders to:

A. Never mention it, so members aren't upset
B. Meet to process the conflict and plan ahead
C. Let the senior leader make all decisions from now on

80. Which instrument assesses a counselor's compassion satisfaction, burnout, and secondary traumatic stress?

A. Beck Depression Inventory-II
B. Maslach Burnout Inventory
C. Professional Quality of Life Scale

81. A year after a fire destroyed her home, a client says she now appreciates each day more, feels closer to her sisters, and has found strengths she didn't know she had. This best illustrates:

A. Posttraumatic growth
B. Resilience
C. Shattered assumptions

82. In a first family session, the counselor notices that the father answers every question asked of his 13-year-old daughter, while the mother stays silent and looks away. The counselor is assessing:

A. The cultural definition of the problem
B. The daughter's affect
C. Interactional dynamics

83. Under the ACA Code, a counselor who lacks the competence to help a client is expected to:

A. Know referral resources and suggest appropriate alternatives
B. End services immediately and let the client find help
C. Refer only to providers who pay a referral fee

84. In James and Gilliland's six-step crisis model, which step is the first of the acting steps?

A. Provide support
B. Examine alternatives
C. Make psychological contact

85. After one drink at a party, a client in recovery says, "I've blown it, so I might as well keep drinking." In her relapse prevention plan, this reaction is called:

A. An apparently irrelevant decision
B. The abstinence violation effect
C. Cognitive dissonance

86. Someone the counselor dated briefly in college, many years ago, calls and asks to begin counseling with her. Under the ACA Code, the counselor should:

A. Decline and refer him to another counselor
B. Accept after documenting the risks first
C. Accept, since more than 5 years have passed

87. A counselor wants to post mental health tips online to reach potential clients. Right now she has only a personal account with family photos. The best approach is to:

A. Share brief client success stories
B. Create a separate professional account
C. Post the tips from her personal account

88. An agency counselor covered by the agency's liability policy asks whether she also needs her own professional liability policy. The best answer is:

A. No, because the agency policy fully protects every employee
B. No, because employees cannot be sued as individuals
C. Yes, because the agency policy protects the agency first

89. A client's GAD-7 score has fallen from 16 at intake to 6 over ten weeks. She asks whether she is ready to move to every-other-week sessions. The counselor's best response is to:

A. Review the scores with her and decide together
B. Tell her the scores mean she must stay weekly
C. End counseling now, since her score is under 10

90. During a couples session, the partners begin talking over each other with rising voices. The counselor's best response is to:

A. End the session until both partners have calmed down
B. Slow things down so each partner can speak and be heard
C. Let them argue freely so pent-up anger is released

91. A counselor's caseload increasingly includes clients with gambling problems. What is the best way to assess the need for specialized certification?

A. Weigh client needs against training and applicable rules
B. Assume the license already covers every presenting problem
C. Obtain any certificate available, whatever its content

92. Studying survivors' grief after the 1942 Cocoanut Grove nightclub fire, this theorist described acute grief reactions and helped found crisis intervention:

A. Gerald Caplan
B. Erich Lindemann
C. Kenneth Doka

93. A counselor is seeing a colleague's client for the first time while the colleague is on leave. Before meeting her, the counselor should:

A. Start fresh so prior notes do not bias the work
B. Review the plan, recent notes, and risk history
C. Plan to have the client retell her full history

94. A leader who names an experience shared across the whole group, such as "Several of you have felt invisible at home," is using:

A. Linking
B. Process observation
C. Universalizing

95. Intense imagined exposure to a feared situation, often exaggerated, without a gradual hierarchy, is:

A. Implosive therapy
B. Systematic desensitization
C. Flooding

96. To protect clients if she suddenly becomes incapacitated or dies, a counselor in private practice should:

A. Rely on clients to find a new counselor if it is ever needed
B. Give each client a copy of the full case record right now
C. Prepare a professional will naming a records custodian

97. A client agrees to a psychiatric medication evaluation while continuing counseling. Which plan best reflects a well-made referral?

A. Fax her records to the nearest psychiatrist today
B. Tell her to find a prescriber through her insurer
C. Offer options, get a release, and follow up

98. A counselor is about to base a current placement decision on results from an outdated test edition given several years ago. Which ACA standard most directly applies?

A. E.11, Obsolete assessment and outdated results
B. E.10, Assessment security
C. E.8, Multicultural issues in assessment

99. A counselor using a Counseling Compact privilege has a telehealth client who moves to a state that has not joined the Compact. To keep seeing the client, the counselor must:

A. Meet that state’s own licensure or authorization requirements
B. Rely on the home-state license, since the client is

already in care
C. Use the existing privilege, which extends to non-member states

100. A counselor realizes she tends to steer female clients toward caregiving careers and male clients toward leadership roles, whatever their stated interests. Her best response is to:

A. Disclose the pattern to each client and apologize
B. Examine her gender-role assumptions in supervision
C. Refer her male clients to a male colleague

101. Reviewing an intake, a counselor sees that a history of elevated mood was nearly missed in a client presenting with depression. The counselor should:

A. Conclude the counselor cannot treat mood disorders and transfer
B. Name the gap and add routine screening for past elevated mood
C. Attribute the miss to the client's incomplete self-report

102. A client checks the stove dozens of times before leaving home. The counselor has her check once, leave, and resist going back while her anxiety drops. This intervention is:

A. Systematic desensitization
B. Interoceptive exposure
C. Exposure and response prevention

103. A client wants help choosing a new career, but he arrives high for most sessions and can't recall what was discussed. No safety risks are present. The first treatment priority is:

A. Career exploration
B. His substance use
C. Long-term personal growth

104. A client whose preferred written language is not English must sign an informed consent form. The counselor should:

A. Have the client sign the English form to meet the rule
B. Summarize the form aloud in simple English
C. Provide a qualified translation and review it together

105. A test has a standard deviation of 15 and a reliability coefficient of .91. Its standard error of measurement is:

A. 1.35
B. 4.5
C. 14.3

106. A projective test asks a client to finish sentence stems such as “I wish…” and “My mother…” This instrument is the:

A. Thematic Apperception Test
B. House-Tree-Person
C. Rotter Incomplete Sentences Blank

107. Reviewing her emergency preparedness plan, a solo counselor realizes a flood could destroy her paper records and office server. The plan should add:

A. Encrypted off-site backups and a records-access procedure
B. A flood rider on her professional liability policy
C. A rule to close the practice until records are rebuilt

108. A member has been quiet all session but leaned forward when others discussed caring for aging parents. Which response best draws her out?

A. “Ana, everyone else has shared, so now it’s your turn.”
B. “Ana, I noticed you leaned in. Want to add anything?”
C. “Ana’s situation sounds just like Ben’s, I’m guessing.”

109. A couple in a same-sex relationship requests couples counseling. The counselor’s personal religious values conflict with the relationship. What is most appropriate?

A. Refer the couple elsewhere, citing the values conflict
B. Serve them, seeking training and consultation as needed
C. Share the counselor’s beliefs so the couple can decide

110. Under HIPAA, a counselor's process notes, kept apart from the clinical record and containing the counselor's analysis of session content, are called:

A. The designated record set
B. 42 CFR Part 2 records
C. Psychotherapy notes

111. A client insists that his neighbor implanted a tracking device in his tooth. He holds this belief firmly even after dental X-rays show nothing. In the MSE, this is best recorded as:

A. A delusion, under thought content
B. An obsession, under thought content
C. A hallucination, under perception

112. After signing a progress note, a counselor realizes she left out something the client reported. She should:

A. Rewrite the signed note to include the detail
B. Add it to next week's note as new information
C. Add a dated, signed addendum to the note

113. A counselor at a community agency also has a private practice. A client whose agency sessions are ending asks to keep seeing her privately. Under the ACA Code, this is acceptable only if:

A. Agency policy allows it and she knows other options
B. Her supervisor gives informal verbal approval
C. The client pays the same fee charged at the agency

114. Kiresuk and Sherman developed which method for rating a client's progress on individualized goals?

A. Reliable change index
B. Outcome Rating Scale
C. Goal Attainment Scaling

115. A counselor plans to finish a progress note at home that evening. The most secure approach is to:

A. Work only through the practice's encrypted remote record access
B. Email the draft to a personal account to finish at home

C. Save the draft to the home computer and delete it after upload

116. A couples technique in which one partner speaks briefly and the other must paraphrase before replying comes from:

A. PREP (Markman, Stanley, & Blumberg)
B. The Gottman Method
C. Thomas Gordon’s Parent Effectiveness Training

117. As a client quietly says her family would be better off without her, the counselor feels a jolt of alarm and begins speaking quickly. What is best?

A. Reassure her that her family loves her, then move on
B. Slow breathing and speech, then ask directly about suicide
C. Shift topics until the counselor feels calmer again

118. A client with complex trauma has been safe and stable for months and has solid coping skills. She now begins telling the full story of her abuse and grieving what it cost her. In Herman’s model, she has entered:

A. Safety and stabilization
B. Remembrance and mourning
C. Processing the pain of grief

119. Hypothesizing, circularity, and neutrality, with a team observing from behind a mirror, are hallmarks of:

A. Bowen family systems therapy
B. MRI brief strategic therapy
C. Milan systemic therapy

120. A client experiencing intimate partner violence is working with a domestic violence advocate. The counselor’s most appropriate approach is to:

A. Call the advocate for updates without asking the client
B. Leave all safety planning to the advocate’s program
C. With her release, align one safety plan with the advocate

121. During an intensive meditation retreat, a client with no history of psychosis had overwhelming visions and a sense of merging with the universe. She is frightened and struggling to function. Which concept was developed for experiences like this?

A. Existential isolation
B. Spiritual emergency
C. Spiritual bypassing

122. A trainee wants to confront a client about a discrepancy in their very first session. Based on Ivey's microskills hierarchy, the trainee should first:

A. Share a similar personal story to bond
B. Offer an interpretation to show expertise
C. Attend and listen before trying to influence

123. A client in a custody dispute asks his counselor to testify that he is the better parent. The counselor should:

A. Explain that treating counselors do not give forensic opinions
B. Agree, since the counselor knows the client best
C. Agree once the client signs a release for the testimony

124. A client says, "My landlord raised the rent again, and I had to pick up weekend shifts to cover it." Which response is a paraphrase?

A. "Having a stable home really matters to you."
B. "So the rent went up, and now you're working weekends."
C. "You sound stretched thin and anxious about money."

125. Which factors best determine whether a counselor is suitable to take on a new client?

A. Shared background and interests with the client
B. Skills, availability, and no conflict of interest
C. Whether insurance pays the counselor's full fee

126. After months of 50-hour weeks, a counselor notices she is short with clients and drifts during sessions. Her best next step is to:

A. Tell her clients she is burned out and on edge
B. Push through, since her skills are still adequate
C. Address her self-care and raise it in supervision

127. Which framework uses six dimensions, including withdrawal potential and recovery environment, to match clients with substance use problems to a level of care?

A. SBIRT
B. ASAM Criteria
C. Addiction Severity Index

128. A counselor finds that a signed progress note in the EHR lists the wrong date of service. The counselor should:

A. Delete the note and re-create it with the correct date
B. Leave the error, since signed notes can never be changed
C. Add a dated, signed amendment that keeps the original visible

129. A counselor joins an integrated care team with a physician and a psychiatric nurse practitioner who will discuss shared clients. Before discussing a client with the team, the counselor should:

A. Tell the client about the team and what is shared
B. Share the full record so care is coordinated
C. Keep all client information from the team

130. Earlier, a client described waiting at the window for a father who often didn't show up. Now she says she keeps dating people who don't call back. The counselor says, "I wonder if that pattern connects to waiting for your dad." This is:

A. Reframe
B. Reflection of meaning
C. Interpretation

131. A counselor meets a publisher's qualification level to purchase a structured violence-risk instrument but has never been trained on it. The counselor should:

A. Complete instrument-specific training before using it
B. Use it, since purchase eligibility confirms competence
C. Use it, but leave the scores out of the client's record

132. A client mentions that she is observing a religious fast this month, one the counselor knows little about. The response that best reflects cultural humility is:

A. "I'd like to understand what this fast means for you."
B. "I've read about that fast. It's mainly about discipline."
C. "A counselor who shares your faith might suit you better."

133. A practice-approved AI tool drafts a treatment plan summary for a client. The counselor's responsibility is to:

A. Sign it as written, since the vendor answers for its accuracy
B. Verify and edit it, remaining accountable for its content
C. Label it AI-generated so that liability shifts to the tool

134. In the mental status exam, affect that shows virtually no emotional expression is described as:

A. Incongruent
B. Blunted
C. Flat

135. In a busy emergency department, a counselor sees clear, impairing depressive symptoms but has no time to sort out which specific disorder, if any, fully fits. The most fitting DSM-5-TR designation is:

A. Provisional major depressive disorder
B. Unspecified depressive disorder
C. Other specified depressive disorder

136. A child client's parents ask the counselor to also tutor their daughter in reading at their home for extra pay. The counselor should:

A. Accept, since knowing her well may speed her reading progress
B. Accept if tutoring is held at the office and billed separately

C. Decline the second role and refer to a reading specialist

137. A clinic director wants to know whether clients referred by courts, schools, and physicians differ in how many complete treatment versus drop out. The data are counts in each category. The appropriate test is:

A. Chi-square
B. Pearson r
C. One-way ANOVA

138. Two sisters live through the same parental divorce. One becomes anxious and withdrawn; the other becomes more confident and independent. This illustrates:

A. Circular causality
B. Multifinality
C. Equifinality

139. Mid-session, a client suddenly reports crushing chest pain spreading down the left arm. The counselor should first:

A. Call 911 and stay with the client, per the medical emergency plan
B. Lead a grounding exercise to see whether the pain eases
C. Advise the client to see a physician after the session ends

140. A group leader invites a member near the end of treatment to tell newer members how her depression has lifted. For the newer members, the leader is mainly fostering which of Yalom's therapeutic factors?

A. Universality
B. Altruism
C. Instillation of hope

141. A counselor skilled in CBT for depression is new to applying it to somatic symptom presentations. Who is the best consultant to approach?

A. The agency director, who has the most seniority
B. A colleague experienced in CBT for somatic symptoms
C. The client's physician, who manages the physical complaints

142. Which instrument was developed to assess lethality risk in intimate partner violence?

A. The Danger Assessment
B. The Power and Control Wheel
C. The C-SSRS

143. A document that records the reason for services, the treatment provided, progress, status at discharge, and recommendations is a:

A. Discharge summary
B. Treatment plan
C. BIRP note

144. Worried about privacy, a current client asks the counselor to shred the intake notes. The counselor should:

A. Shred the notes, since clients own their clinical records
B. Rewrite the notes to remove the details the client dislikes
C. Explain the retention policy and how the records are secured

145. A 15-year-old's parents want counseling to focus on his grades. He says he mostly wants help with constant fights at home. In setting goals, the counselor should:

A. Adopt the parents' goal, since they consented
B. Involve his parents while honoring his goals
C. Adopt only his goal and leave the parents out

146. A client describes himself as "Deaf, part of the Deaf community." His referral form says "hearing impaired." The counselor's best approach is to:

A. Use his own language for his identity
B. Treat his hearing loss as the presenting problem
C. Use the referral's clinical term for accuracy

147. A supervisee has never treated acute stress disorder and is drafting a client's treatment plan. What is most appropriate?

A. Review the plan with the assigned supervisor before using it
B. Have a former classmate informally approve the plan
C. Finalize it and mention the case at a later supervision meeting

148. A counselor demonstrates how to ask a landlord for repairs, then has the client rehearse the request until she feels sure she can do it. The concept the counselor most aims to build is:

A. Unconditional self-acceptance
B. Reciprocal inhibition
C. Self-efficacy

149. After reading her record, a client asks in writing that the counselor delete an accurate diagnosis she finds stigmatizing. Under HIPAA, the counselor should:

A. Delete the diagnosis, since clients control their records
B. Deny in writing and explain her right to file a disagreement
C. Refer her to the licensing board, which decides amendments

150. A widowed mother of three says nothing is going right. The counselor asks, "With so much on your plate, how have you managed to keep going each day?" This is:

A. A unique outcome question
B. A coping question
C. A scaling question

151. A new client asks whether she can text her counselor between sessions. The informed consent discussion should explain:

A. Its uses, reply times, and no emergency use
B. That texts are fully secure and private
C. That she should text if she feels suicidal at night

152. At license renewal, a counselor realizes the required continuing education hours are six hours short. The counselor should:

A. Attest to full completion and finish the hours afterward
B. Count recent journal reading that earned no CE credit
C. Contact the board about its options before attesting

153. Seeing many clients unable to afford care, a counselor testifies before a state legislative committee for a bill expanding public mental health funding. Under the ACA Advocacy Competencies, this is:

A. Public arena: social/political advocacy
B. School/community: systems advocacy
C. Client/student: advocacy for one client

154. A client says, “I agree with what we’re working toward, and the exercises make sense, but I’m not sure I can trust you with the hard stuff.” Which part of Bordin’s working alliance needs attention?

A. The bond
B. Unconditional positive regard
C. Agreement on goals

155. A licensed counselor is arrested and charged with an offense unrelated to work. Regarding licensure, the counselor should:

A. Review board rules and self-report within any required period
B. Report the matter only if it ends in a conviction
C. Disclose the charge to current clients at their next session

156. A client says she plans to overdose this weekend on pills she has stockpiled at home and intends to do it. She is willing to accept help. The counselor’s best action is to:

A. Increase sessions to twice weekly and involve her partner
B. Write a safety plan together and see her again next week

C. Stay with her and arrange an immediate hospital evaluation

157. A colleague urges a single-session psychological debriefing for a client seen two days after a car crash. What does current evidence indicate?

A. It is effective, given its wide use by crisis teams
B. It is not supported and may worsen outcomes for some
C. It is effective if held within 72 hours of the event

158. Which type of interview uses fixed questions in a fixed order and offers the highest reliability for research and diagnosis?

A. A semi-structured interview with open follow-up
B. An unstructured interview that follows the client
C. A structured interview such as the SCID-5

159. A client says, “Lately I just want everything to stop.” Which question best assesses suicide risk?

A. “You’re not thinking of hurting yourself, are you?”
B. “Are you having thoughts of killing yourself?”
C. “What do you mean by ‘everything’?”

160. A client reports losing employer health coverage last month but wants to keep attending sessions. The counselor should:

A. Keep billing the former plan until claims start being denied
B. Pause services until the client has new insurance in place
C. Verify current eligibility and discuss fee or coverage options

161. A clinic’s AI screening app rates a client as low suicide risk. In session, the client mentions she has written goodbye letters to her children. The counselor should:

A. Rely on the app’s low-risk rating
B. Assess her suicide risk directly
C. Hospitalize her involuntarily at once

162. A client says she plans to overdose this weekend, has pills saved at home, and intends to act. She refuses to go to the hospital voluntarily. The best response is to:

A. Stay with her and arrange a hospital evaluation
B. Ask her to sign a no-suicide contract
C. Complete a safety plan and see her next week

163. A new client plans to pay out of pocket rather than use insurance. Under federal law, the counselor must provide:

A. A good faith estimate of expected charges
B. A superbill for out-of-network reimbursement
C. An advance beneficiary notice of noncoverage

164. During an argument about money, a husband answers his wife's tears with calm statistics and detached logic, showing no feeling. Which concept best describes his communication?

A. Emotional cutoff
B. Stonewalling
C. Super-reasonable stance

165. An older adult client's daughter asks the counselor to record each video session so she can replay it for her mother. Before any recording, the counselor needs:

A. The daughter's written permission as the family caregiver
B. The client's informed consent, plus any state-required consent
C. Only confirmation that the platform is HIPAA compliant

166. Research showing that family rejection of LGBTQ youth is linked to much higher risk of depression, suicide attempts, and substance use, while family acceptance is protective, comes from:

A. Ilan Meyer's minority stress model
B. Anthony D'Augelli's life-span model
C. Caitlin Ryan's Family Acceptance Project

167. With the parents' signed release, a school asks for a written summary of a student's counseling assessment. The counselor's report should:

A. Present clinical opinions as established fact
B. Include every test score and session detail
C. Use plain language and note the data's limits

168. A counselor meets alone with a client's spouse about the client's treatment and bills family psychotherapy without the patient present (90846). The note should show:

A. That the spouse is also an active client of the practice
B. The spouse's own diagnosis and treatment plan
C. How the session served the client's plan goals

169. Since her mother died two months ago, a colleague has seemed exhausted, has missed appointments, and once fell asleep in a team meeting. No client harm is known. The counselor's best first step is to:

A. File a complaint with the licensing board
B. Say nothing, since grief is private
C. Consult with and support the colleague

170. A client offers to share heart-rate data from her consumer smartwatch to help track her anxiety. The counselor should treat the data as:

A. One trend source alongside her self-reported symptoms
B. A validated measure for diagnosing panic disorder
C. Information the counselor may not use under HIPAA

NCE Practice Exam 2 — Answer Key with Ownership Map

Every rationale names who or what **owns** each option. Read all three, not just the correct one: each item teaches three pairings.

How to review: for every item you missed *or guessed*, write in your error log (Workbook, Part 4) the owner you picked and the owner of the correct answer.

Quick Key

1. B	2. C	3. A	4. A	5. A	6. B	7. B	8. C	9. C	10. B
11. A	12. A	13. A	14. B	15. B	16. C	17. C	18. A	19. A	20. C
21. B	22. B	23. A	24. C	25. C	26. C	27. A	28. A	29. C	30. B
31. A	32. C	33. A	34. C	35. A	36. B	37. B	38. B	39. B	40. C
41. A	42. C	43. A	44. A	45. B	46. B	47. C	48. C	49. B	50. B
51. A	52. B	53. C	54. B	55. C	56. C	57. B	58. B	59. A	60. B
61. B	62. B	63. A	64. C	65. C	66. A	67. A	68. A	69. B	70. B
71. C	72. C	73. A	74. B	75. A	76. A	77. B	78. B	79. B	80. C
81. A	82. C	83. A	84. B	85. B	86. A	87. B	88. C	89. A	90. B
91. A	92. B	93. B	94. C	95. A	96. C	97. C	98. A	99. A	100. B
101. B	102. C	103. B	104. C	105. B	106. C	107. A	108. B	109. B	110. C
111. A	112. C	113. A	114. C	115. A	116. A	117. B	118. B	119. C	120. C

121. B	122. C	123. A	124. B	125. B	126. C	127. B	128. C	129. A	130. C
131. A	132. A	133. B	134. C	135. B	136. C	137. A	138. B	139. A	140. C
141. B	142. A	143. A	144. C	145. B	146. A	147. A	148. C	149. B	150. B
151. A	152. C	153. A	154. A	155. A	156. C	157. B	158. C	159. B	160. C
161. B	162. A	163. A	164. C	165. B	166. C	167. C	168. C	169. C	170. A

Rationales

1 — B

Intake & Assessment · Task 2B.8 · Chapter 19

- **B. Informal observation — Owner: Informal observation (2B.8).** Informal observation is what the counselor notices during the session, such as appearance, behavior, and interaction, without a structured method.
- A. Formal observation — Owner: Formal observation (rating scales, timed observations). Formal observation uses structured methods such as behavior rating scales or timed observations; none was used here.
- C. Client self-report — Owner: Self-report (2A.3). Self-report comes from what the client says or writes in interviews, questionnaires, or checklists, not from what the counselor sees.
- **Pair to learn:** Noticed in session without a structured method → Informal observation (not formal)

2 — C

Intake & Assessment · Task 2B.3 · Chapter 11

- **C. Delirium tremens — Owner: Alcohol withdrawal (potentially life-threatening).** Confusion, hallucinations, and autonomic instability about 2 to 4

days after stopping alcohol indicate delirium tremens, which requires emergency medical care.

- A. Stimulant withdrawal — Owner: Stimulants (crash: fatigue, hypersomnia, hunger). The stimulant crash brings fatigue, increased sleep and appetite, and low mood, not delirium.
- B. Cannabis withdrawal — Owner: Cannabis (irritability, restlessness). Cannabis withdrawal is uncomfortable but not medically dangerous and doesn't cause delirium.
- **Pair to learn:** Confusion, hallucinations, autonomic instability 2 to 4 days after drinking stops → delirium tremens

3 — A

Legal & Ethical Compliance · Task 6.1 · Chapter 17

- **A. Confirm the client's safety and use the plan's downtime procedures — Owner: HIPAA contingency plan (45 CFR 164.308(a)(7)).** The Security Rule requires backup and emergency-mode procedures; the practice plan sets how to continue the intake on paper or reschedule safely.
- B. Type the client's answers into a personal phone app for now — Owner: Unsecured ePHI on a personal device (HIPAA Security Rule). An unapproved personal app creates an unprotected copy of health information outside the practice's safeguards.
- C. Send the client home with no set time to finish the intake — Owner: Unplanned dismissal (no follow-up step). Ending without a rescheduled time leaves the client without a clear path back to care; the plan's backup steps cover this.
- **Pair to learn:** Power and EHR outage → safety first, then the plan's downtime procedures (HIPAA contingency plan)

4 — A

Professional Development & Counselor Self-Awareness · Task 1B.11 · Chapter 22

- **A. Norcross and Lambert — Owner: Norcross & Lambert (evidence-based therapy relationships).** The Norcross and Lambert task forces judged the alliance, empathy, positive regard and affirmation, and collecting client feedback demonstrably effective.
- B. Safran and Muran — Owner: Safran & Muran (alliance ruptures and repair). Safran and Muran described withdrawal and confrontation ruptures and their repair.
- C. Truax and Carkhuff — Owner: Truax & Carkhuff (operationalized Rogers's conditions). Truax and Carkhuff defined accurate empathy, nonpossessive warmth, and genuineness.
- **Pair to learn:** Demonstrably effective relationship elements, including client feedback → Norcross & Lambert

5 — A

Intake & Assessment · Task 2B.6 · Chapter 19

- **A. Diagnose both and plan integrated treatment — Owner: Co-occurring diagnoses; integrated treatment (2B.6).** Counselors diagnose every disorder whose criteria are met unless the criteria say otherwise, and co-occurring substance use and mental health disorders are treated together.
- B. Diagnose only the principal disorder — Owner: Principal diagnosis (listed first, not alone). The principal diagnosis (inpatient) or reason for visit (outpatient) is the condition chiefly responsible for the admission or services and is listed first; it doesn't replace other diagnoses whose criteria are met.
- C. Treat the drinking first, then the anxiety — Owner: Sequential (non-integrated) treatment. Treating one

disorder and postponing the other runs against integrated treatment of co-occurring disorders.

- **Pair to learn:** Criteria met for two disorders → Diagnose both, integrated treatment (2B.6)

6 — B

Professional Development & Counselor Self-Awareness · Task 1A.1 · Chapter 22

- **B. Return to the consultant with the outcome data to review the plan — Owner: Ongoing, data-informed consultation (ACA C.2.d).** Counselors monitor effectiveness (C.2.d); bringing outcome data back to the consultant lets the plan be adjusted with expert input.
- A. Continue as planned, since the colleague approved the approach — Owner: One-time consultation treated as standing approval. Consultation advice fits the facts at the time; it does not cover a later lack of progress.
- C. Switch to a different therapy without consulting anyone — Owner: Unreviewed change of treatment. Abandoning the therapy without review may discard a fixable approach and skips the consultation the counselor still needs.
- **Pair to learn:** No progress on a newly learned therapy → return to consultation with outcome data

7 — B

Professional Development & Counselor Self-Awareness · Task 1A.2 · Chapter 17

- **B. Share only needed, nonidentifying details — Owner: ACA B.7 (consultation).** In consultation, counselors don’t reveal identifying information without consent and share only what’s needed.
- A. Share the full chart so peers can help — Owner: Excess disclosure (minimal disclosure, B.2.e). The full chart reveals far more than the consultation needs.
- C. Hand clinical responsibility to the group — Owner: Supervision (evaluative, shared responsibility), not

consultation. In consultation, the consulting counselor keeps full responsibility for the client.

- **Pair to learn:** Peer consultation → Share only what's needed, no identifiers (ACA B.7)

8 — C

Indirect Client Care · Task 5.1 · Chapter 23

- **C. Agree on shared goals, roles, and a plan for staying in touch — Owner: Care coordination with a community support.** Shared goals, clear roles, and an agreed way to stay in contact let the nurse's visits reinforce counseling instead of running alongside it.
- A. Let the nurse set separate goals on her own visit plan — Owner: Parallel, uncoordinated care. Separate, unconnected plans risk duplicated or conflicting work for the client.
- B. Defer counseling goals to the nurse's health priorities — Owner: Abdicating the counseling role. The nurse's health priorities do not replace the client's counseling goals.
- **Pair to learn:** Community support helping the client → one shared plan with roles and contact, not parallel plans

9 — C

Intake & Assessment · Task 2B.3 · Chapter 11

- **C. In sustained remission — Owner: DSM-5-TR (12+ months, craving allowed).** Twelve months or more with no criteria met, except craving, is sustained remission.
- A. In early remission — Owner: DSM-5-TR (3 to under 12 months). Early remission covers at least 3 but less than 12 months; she has passed 12 months.
- B. In a controlled environment — Owner: DSM-5-TR (restricted access, e.g., locked facility). She lives at home with normal access to alcohol.
- **Pair to learn:** 12+ months without criteria except craving → sustained remission

10 — B

Indirect Client Care · Task 5.6 · Chapter 17

- **B. Allow access and help her understand them — Owner: ACA B.6.e (client access to records).** Counselors provide reasonable access to records unless there is compelling evidence of harm, and help clients understand what they read.
- A. Show her the psychotherapy notes only — Owner: HIPAA psychotherapy notes (kept separate, extra protection). Psychotherapy notes are a separately protected category; her request is for reasonable access to her clinical record.
- C. Refuse, since records belong to the counselor — Owner: Counselor ownership of records (inconsistent with B.6.e and HIPAA access rights). Clients have a right to access their records, with limited exceptions; none applies here.
- **Pair to learn:** Client asks to see records → Reasonable access (ACA B.6.e)

11 — A

Treatment Planning & Continuity of Care · Task 3A.1 · Chapter 21

- **A. Rank them by impact and readiness, then start with one or two — Owner: Collaborative prioritization informed by readiness (Prochaska & DiClemente).** Ranking concerns by their impact and by the client's readiness to change gives early wins and a workable plan. Readiness reflects the stages of change model.
- B. Work on all four equally so no concern feels set aside — Owner: Diffuse, unfocused treatment plan. Spreading effort evenly across every concern dilutes progress and can overwhelm the client. Doing everything is not the same as honoring his wishes.
- C. Wait to set priorities until his spouse can choose them — Owner: Violation of client autonomy

(Kitchener's ethical principles). An adult client sets his own treatment priorities. Handing that choice to a spouse undercuts autonomy, one of Kitchener's core ethical principles.

- **Pair to learn:** Too many concerns at once → Rank by impact and readiness, start small

12 — A

Intake & Assessment · Task 2A.1 · Chapter 17

- **A. Delay consent until he is sober — Owner: Assessing capacity to consent (2A.1): intoxication.** Intoxication is one of the factors that can impair capacity, so consent should wait until he can understand what he's agreeing to.
- B. Have him sign now and review it later — Owner: Consent without capacity (not cured later; A.2.a). A signature given without capacity isn't meaningful informed consent.
- C. Proceed, since he is an adult volunteer — Owner: Assuming capacity (ignores capacity factors). Being an adult who wants services doesn't establish capacity when he is visibly intoxicated.
- **Pair to learn:** Intoxicated at intake → Capacity to consent (2A.1)

13 — A

Intake & Assessment · Task 2B.11 · Chapter 12

- **A. Emerging adulthood — Owner: Arnett.** Arnett's emerging adulthood (roughly 18 to 29) is marked by identity exploration, instability, self-focus, feeling in-between, and possibilities.
- B. Intimacy vs. isolation — Owner: Erikson (psychosocial stage 6). Erikson's young-adult crisis centers on forming close relationships, not on instability and feeling in-between.
- C. Age-30 transition — Owner: Levinson (Seasons of a Man's Life). Levinson's age-30 transition is a

reappraisal within his life structure model, not the in-between period of the early twenties.

- **Pair to learn:** Feeling in-between, ages 18 to 29 → emerging adulthood (Arnett, not Levinson)

14 — B

Legal & Ethical Compliance · Task 6.2 · Chapter 17

- **B. Review purpose, likely distress, options, and the right to refuse — Owner: ACA A.2.a–A.2.b (informed consent).** Clients receive information on techniques, risks, benefits, and the right to refuse a modality, and the consent is documented.
- A. Begin exposures and get written consent once progress shows — Owner: Retroactive consent (violates ACA A.2.a). Consent must come before the intervention begins, not after the client sees results.
- C. Skip formal consent, since the method is evidence-based — Owner: Evidence-base exemption myth. Strong research support does not remove the duty to obtain informed consent, especially when temporary distress is expected.
- **Pair to learn:** New procedure with expected distress → informed consent first (ACA A.2)

15 — B

Intake & Assessment · Task 2B.11 · Chapter 12

- **B. Part C early intervention — Owner: IDEA Part C (birth to 3).** IDEA Part C provides early intervention services for children from birth to age 3.
- A. A Section 504 plan — Owner: Rehabilitation Act of 1973. A 504 plan gives accommodations to students whose disability limits a major life activity; it is not the birth-to-3 program.
- C. An Individualized Education Program — Owner: IDEA special education (ages 3 to 21). An IEP sets special education goals and services for eligible students ages 3 to 21.

- **Pair to learn:** Birth to 3 services → IDEA Part C (not IEP or 504)

16 — C

Professional Development & Counselor Self-Awareness · Task 1A.3 · Chapter 22

- **C. Verify each credential's hour, topic, and provider rules separately — Owner: Credential-specific CE requirements (each issuing body sets its own).** Each licensing board and certifying body sets its own hours, required topics (often ethics), and accepted providers, so each must be checked.
- A. Assume the national certification's hours satisfy every licensing board — Owner: Credential-overlap assumption. Hours may overlap, but boards often add their own topic or provider rules, so one credential's hours are not automatically accepted by another.
- B. Follow the rules of the state where the counselor completed graduate school — Owner: Training-state confusion (jurisdiction error). Requirements come from the jurisdiction that issued the license, not where the counselor trained.
- **Pair to learn:** License plus certification → check each body's CE rules separately

17 — C

Treatment Planning & Continuity of Care · Task 3A.4 · Chapter 21

- **C. What helped you get through hard times before? — Owner: Strengths-based practice (SFBT; Saleebey).** Asking about past coping and successes surfaces strengths the plan can build on.
- A. Which symptom bothers you most right now? — Owner: Problem-focused symptom assessment. This clarifies the problem statement but doesn't identify strengths.
- B. How did your childhood shape this pattern? — Owner: Insight-oriented exploration (psychodynamic).

This seeks insight into origins, not resources to use in the plan.

- **Pair to learn:** Past coping and successes → Strengths (3A.4)

18 — A

Intake & Assessment · Task 2B.3 · Chapter 11

- **A. Claudia Black — Owner: Black (don't talk, don't trust, don't feel).** Claudia Black described the unspoken rules of alcoholic families: don't talk, don't trust, don't feel.
- B. E. M. Jellinek — Owner: Jellinek (disease concept; phases of alcoholism). Jellinek described the phases of alcoholism in the drinker, not rules in the family.
- C. Sharon Wegscheider-Cruse — Owner: Wegscheider-Cruse (family roles in addiction). Wegscheider-Cruse described family roles such as hero, scapegoat, lost child, mascot, and enabler, not family rules.
- **Pair to learn:** Don't talk, don't trust, don't feel → Claudia Black (not Wegscheider-Cruse)

19 — A

Indirect Client Care · Task 5.4 · Chapter 23

- **A. Its name, start date, benefits, side effects, and impact — Owner: Complete self-report documentation.** Name as the client gives it, start date, reported benefits and side effects, and effects on functioning give the prescriber a full picture.
- B. The counselor's view of whether the dose is right — Owner: Dosing opinion (outside counselor scope). Judging whether a dose is right is the prescriber's role.
- C. Side effects only, since benefits are not relevant — Owner: Incomplete record. Reported benefits matter as much as side effects for judging how the medication affects functioning.
- **Pair to learn:** Documenting medication report → name, start, benefits, side effects, functional impact

20 — C

Intake & Assessment · Task 2B.9 · Chapter 19

- **C. Discuss the fit and refer to a suitable setting — Owner: Screening for fit (2B.9); least restrictive appropriate level of care.** The first session screens whether these are the right services at the right level of care. A client who needs a structured environment should be referred to a setting that provides it.
- A. Begin weekly sessions and reassess in a month — Owner: Under-matching the level of care. Weekly outpatient care doesn't meet her need for a structured setting, so it keeps her at a level below what she needs.
- B. Arrange inpatient hospitalization today — Owner: Inpatient hospitalization (imminent danger, medical instability). Inpatient care is for imminent danger, severe inability to care for oneself, or medical instability; none is described, so this is more restrictive than needed.
- **Pair to learn:** Needs exceed what the service offers → Screen for fit and refer (2B.9)

21 — B

Intake & Assessment · Task 2A.5 · Chapter 17

- **B. Respect her choice and explain any consequences — Owner: ACA A.2.b (right to refuse services or modality changes).** Clients have the right to refuse any service or change in modality and to be advised of the consequences of refusing.
- A. Make group attendance a condition of counseling — Owner: Coercion (violates autonomy and A.2.b). Conditioning services on a modality she refused overrides her right to decline.
- C. Enroll her in group and revisit consent later — Owner: Consent after the fact (informed consent precedes services, A.2.a). Informed consent comes before a service begins, not after.

- **Pair to learn:** Client declines a modality change → Right to refuse (ACA A.2.b)

22 — B

Treatment Planning & Continuity of Care · Task 3A.6 · Chapter 21

- **B. Explore what his service means to him and plan from that — Owner: Cultural humility (client-defined meaning).** Cultural humility means asking how the client defines his own identity and context. What service means to him, and any resources tied to it, are then built into the plan.
- A. Assume he will want practical, task-focused goals only — Owner: Stereotyping (group-based assumption). Assuming a veteran will reject emotional work is a stereotype, not assessment. Plans built on group assumptions miss the individual.
- C. Leave his service out, since his current stressors are civilian — Owner: Context-blind planning. Service history may shape identity, coping, and available resources. Dismissing context the client may see as central weakens the plan.
- **Pair to learn:** Veteran identity in planning → Explore client's meaning (cultural humility)

23 — A

Intake & Assessment · Task 2B.4 · Chapter 19

- **A. A SUDS rating — Owner: Subjective units of distress (SUDS).** A SUDS rating is the client's own 0 to 100 rating of current distress, a quick way to assess the level of distress (2B.4).
- B. A Likert attitude scale — Owner: Rensis Likert. A Likert scale rates agreement with a set of statements; it does not ask for a single 0 to 100 rating of current distress.
- C. The WHODAS 2.0 — Owner: WHO Disability Assessment Schedule 2.0 (functioning). The WHODAS 2.0 measures functioning across daily life; it is the

DSM-5-TR's suggested functioning measure, not a single in-the-moment distress rating.

- **Pair to learn:** 0 to 100 rating of current distress → SUDS (not WHODAS 2.0)

24 — C

Legal & Ethical Compliance · Task 6.3 · Chapter 17

- **C. Provide the notice now and document that it was provided — Owner: HIPAA Notice of Privacy Practices (45 CFR 164.520).** Direct providers give the notice by first service and, under the current rule, make a good-faith effort to get written acknowledgment; a missed notice is given and documented now.
- A. Have the client sign an authorization for treatment disclosures — Owner: HIPAA authorization (45 CFR 164.508). Treatment, payment, and operations disclosures need no authorization; an authorization is not the same as the privacy notice.
- B. Rely on the notice posted in the waiting room as sufficient — Owner: NPP posting requirement (164.520(c)). Posting at the office is required in addition to — not instead of — giving the notice directly to each client.
- **Pair to learn:** Client never got the privacy notice → NPP + good-faith acknowledgment (164.520)

25 — C

Treatment Planning & Continuity of Care · Task 3A.4 · Chapter 21

- **C. Barriers — Owner: Treatment plan component: barriers (3A.4).** Work schedules and lack of childcare are barriers, named early so the plan can address them.
- A. Strengths and resources — Owner: Treatment plan component: strengths (3A.4). Strengths are what help, such as supports and past successes; these factors get in the way.

- B. Problem statement — Owner: Treatment plan component: problem statement. The problem statement describes the clinical concern, not obstacles to treatment.
- **Pair to learn:** Work schedule, no childcare → Barriers (3A.4)

26 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Anomic — Owner: Durkheim (anomic suicide).** Anomic suicide follows sudden disruption of social norms or regulation, such as economic collapse.
- A. Egoistic — Owner: Durkheim (egoistic suicide). Egoistic suicide reflects too little social integration.
- B. Fatalistic — Owner: Durkheim (fatalistic suicide). Fatalistic suicide reflects excessive regulation and oppression.
- **Pair to learn:** Sudden disruption of norms (economic collapse) → anomic (Durkheim)

27 — A

Professional Development & Counselor Self-Awareness · Task 1A.4 · Chapter 22

- **A. Controlled trials whose samples included at-risk veterans — Owner: External validity (generalizability).** Evidence weighs most when it is rigorous and drawn from people like the client.
- B. A single case report describing a dramatic recovery — Owner: Anecdotal evidence (lowest level of evidence). One case cannot show that an intervention works or that results generalize.
- C. Any published study, since publication confirms efficacy — Owner: Publication bias (file-drawer problem). Publication does not prove efficacy; the literature skews toward positive results and may not fit this client.

- **Pair to learn:** Does the research apply to this client? → external validity (similar samples)

28 — A

Treatment Planning & Continuity of Care · Task 3B.9 · Chapter 21

- **A. Explore her feelings about ending and earlier losses — Owner: Termination task: process feelings about ending.** Endings can stir up past losses; processing these feelings is a core task of the final phase.
- B. Shift the session to building her relapse prevention plan — Owner: Relapse prevention (a later termination task). Relapse prevention matters, but skipping her feelings misses what's happening now.
- C. Remind her the end date was set together at the start — Owner: Defending the plan (misses the feeling). Restating the plan answers the logistics but not the hurt she just expressed.
- **Pair to learn:** Ending stirs past losses → Process feelings about termination (3B.9)

29 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Walker's cycle of violence — Owner: Lenore Walker.** Walker described this three-phase cycle in intimate partner violence.
- A. The Power and Control Wheel — Owner: Domestic Abuse Intervention Programs (Duluth). The Duluth wheel maps tactics of power and control, not a three-phase cycle.
- B. Coercive control — Owner: Evan Stark. Stark's concept describes ongoing domination such as monitoring and isolation, not a cycle.
- **Pair to learn:** Tension → battering → honeymoon → Walker (not Duluth)

30 — B

Indirect Client Care · Task 5.5 · Chapter 23

- **B. 988 — Owner: 988 Suicide & Crisis Lifeline.** 988 is the national Suicide & Crisis Lifeline for call, text, or chat during a mental health crisis.
- A. 211 — Owner: 211 (community information and referral). 211 connects people to social services such as housing and food help, not crisis counseling.
- C. 311 — Owner: 311 (non-emergency city services). 311 handles non-emergency municipal services in some cities.
- **Pair to learn:** National mental health crisis line → 988

31 — A

Intake & Assessment · Task 2B.3 · Chapter 11

- **A. Community Reinforcement and Family Training — Owner: Robert Meyers (CRAFT).** CRAFT teaches concerned family members to reinforce sobriety and engage a treatment-refusing loved one.
- B. Brief Strategic Family Therapy — Owner: Szapocznik (adolescent substance use). BSFT is a family therapy for adolescent substance use, and her son is an adult who won't attend.
- C. Multidimensional Family Therapy — Owner: Liddle (adolescent substance use). MDFT treats adolescent substance use with the teen involved, not a family member working alone with an adult.
- **Pair to learn:** Family member engaging a treatment-refusing loved one → CRAFT (Meyers)

32 — C

Treatment Planning & Continuity of Care · Task 3A.7 · Chapter 21

- **C. Full criteria are expected, but information is not yet sufficient — Owner: DSM-5-TR provisional specifier.** DSM-5-TR uses "provisional" when there is a strong presumption that full criteria will be met but

not enough information is yet available. Explaining this helps the client understand the diagnosis may change.

- A. Full criteria are not met, and no specific reason is recorded — Owner: DSM-5-TR unspecified disorder. An unspecified diagnosis is used when criteria for a specific disorder are not met and the clinician chooses not to, or cannot, state why.
- B. Full criteria are not met, and the specific reason is recorded — Owner: DSM-5-TR other specified disorder. An other specified diagnosis is used when criteria are not fully met and the clinician records the specific reason, such as short duration.
- **Pair to learn:** Strong presumption, information still incomplete → "Provisional" specifier (DSM-5-TR)

33 — A

Provision of Counseling Interventions · Task 4.11 · Chapter 14

- **A. Charles Figley — Owner: Figley (compassion fatigue).** Figley described compassion fatigue as exhaustion from caring for those who suffer, including secondary traumatic stress.
- B. Christina Maslach — Owner: Maslach (burnout). Maslach described burnout: exhaustion, depersonalization, and reduced accomplishment from chronic workplace stress.
- C. McCann and Pearlman — Owner: McCann & Pearlman (vicarious trauma). Vicarious trauma refers to lasting changes in the helper's beliefs and worldview.
- **Pair to learn:** Compassion fatigue → Figley (not Maslach)

34 — C

Intake & Assessment · Task 2A.1 · Chapter 17

- **C. Professional will — Owner: Professional will (ACA C.2.h, B.6.i: plan for incapacity or death).** A professional will names a records custodian and

arranges transfer of records and care if the counselor can't continue.

- A. Release of information — Owner: Release of information (client authorization to share). A release authorizes sharing specific information; it doesn't name a records custodian.
- B. Business associate agreement — Owner: HIPAA business associate agreement (vendor contract). A BAA is a contract with a vendor who handles protected health information, not a continuity plan.
- **Pair to learn:** Plan for counselor's death or incapacity → Professional will / records custodian

35 — A

Legal & Ethical Compliance · Task 6.6 · Chapter 17

- **A. Notify the board within the time its rules require — Owner: State board change-of-information rules.** Boards require licensees to update name and contact information within a set period; deadlines vary by state, and missed notices can mean missed renewal mail.
- B. Wait and report both changes at her next renewal — Owner: Deferred reporting (error pattern). Many boards set reporting deadlines shorter than the renewal cycle, so waiting can itself be a violation.
- C. Update only her NPI record, which the board will receive — Owner: NPPES/NPI record (federal, separate from licensure). NPI data goes to the federal NPPES registry for billing identity; it does not update the state licensing board.
- **Pair to learn:** Name or address change → notify the board per its rules

36 — B

Indirect Client Care · Task 5.3 · Chapter 21

- **B. DAP — Owner: DAP (Data, Assessment, Plan).** In DAP, the Data section combines subjective and objective information.

- A. BIRP — Owner: BIRP (Behavior, Intervention, Response, Plan). BIRP begins with the client's behavior and presentation, then separates intervention and response.
- C. SOAP — Owner: SOAP (Subjective, Objective, Assessment, Plan). SOAP separates the client's report (S) from the counselor's observations (O).
- **Pair to learn:** Subjective and objective together in Data → DAP

37 — B

Legal & Ethical Compliance · Task 6.14 · Chapter 9

- **B. Provide a qualified ASL interpreter — Owner: ADA reasonable accommodation (6.14).** A qualified ASL interpreter is a reasonable accommodation that gives the client full access in her own language.
- A. Communicate through written notes only — Owner: Written English (ASL is a full, separate language). ASL is a full language, not signed or written English; notes don't give equal access.
- C. Have her hearing daughter interpret — Owner: Family member as interpreter (compromises accuracy, confidentiality, roles). Family interpreters compromise accuracy, confidentiality, and family roles.
- **Pair to learn:** Deaf client → Qualified ASL interpreter (ADA accommodation)

38 — B

Professional Development & Counselor Self-Awareness · Task 1A.6 · Chapter 22

- **B. Fidelity — Owner: Fidelity (ACA Preamble; Kitchener).** Fidelity means honoring commitments and keeping promises, including fulfilling responsibilities of trust.
- A. Beneficence — Owner: Beneficence (ACA Preamble; Kitchener). Beneficence means working for the good of the individual and society by promoting well-being.

- C. Veracity — Owner: Veracity (ACA Preamble). Veracity means dealing truthfully with individuals the counselor contacts professionally.
- **Pair to learn:** Keeping promises → fidelity

39 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Build a written safety plan with her — Owner: Stanley & Brown (Safety Planning Intervention).** A collaborative safety plan, plus means restriction, is the supported approach; contracts lack evidence.
- A. Rely on the SAD PERSONS score alone — Owner: Patterson et al. (SAD PERSONS). SAD PERSONS is a teaching tool, not validated to predict risk on its own.
- C. Have her sign the contract and file a copy — Owner: No-suicide contract (no supporting evidence). Contracts haven't been shown to prevent suicide and can give false reassurance.
- **Pair to learn:** Safety plan (Stanley & Brown) over no-suicide contract

40 — C

Indirect Client Care · Task 5.7 · Chapter 23

- **C. Speak directly to the client in short segments — Owner: First-person interpreting practice.** Addressing the client directly in short segments keeps the relationship with the client and helps the interpreter stay accurate.
- A. Address the interpreter and ask what the client feels — Owner: Third-person communication. Speaking to the interpreter sidelines the client and weakens the therapeutic relationship.
- B. Ask the interpreter to summarize the client's answers — Owner: Summarized interpretation. Interpreters should render everything said; summaries lose meaning and emotional content.

- **Pair to learn:** Interpreted session → speak directly to the client in short segments

41 — A

Legal & Ethical Compliance · Task 6.4 · Chapter 9

- **A. Reassess fit and consider in-person sessions — Owner: ACA H.4.c–d (appropriateness and effectiveness of distance counseling).** Counselors decide whether distance counseling is appropriate; severe dissociation and no privacy may call for face-to-face services.
- B. Verify her identity with a code word each session — Owner: ACA H.3 (client verification). Verification matters but doesn't address safety or privacy.
- C. Continue unchanged, since she consented to telehealth — Owner: Consent is not the same as fit. Consent doesn't settle whether distance counseling remains appropriate.
- **Pair to learn:** Telehealth not working for this client → H.4.d: consider face-to-face

42 — C

Treatment Planning & Continuity of Care · Task 3B.9 · Chapter 21

- **C. Premature termination — Owner: Premature termination (dropout; Swift & Greenberg).** Stopping before goals are met is premature termination, estimated to occur in about one in five clients.
- A. Forced termination — Owner: Forced termination (external limits). Forced termination comes from outside limits such as insurance, agency policy, or a training placement ending.
- B. Planned termination — Owner: Planned (natural) termination. Planned termination happens when goals are met and the ending is discussed in advance.
- **Pair to learn:** Stops before goals are met → Premature termination (dropout)

43 — A

Treatment Planning & Continuity of Care · Task 3A.8 · Chapter 21

- **A. Defer joint sessions and address her safety individually — Owner: IPV screening (conjoint sessions generally contraindicated).** Active violence with fear of the partner is widely regarded as a contraindication to joint sessions, so individual safety planning and referral to domestic violence resources come first.
- B. Begin joint sessions to improve the couple's communication — Owner: Premature conjoint therapy during active IPV. Joint sessions can expose her to retaliation for what she says and imply shared responsibility for the abuse.
- C. Raise her disclosure in the next joint session for honesty — Owner: No-secrets policy misapplied (endangers the client). Revealing her disclosure in front of the partner could provoke violence. Safety outweighs a general no-secrets policy.
- **Pair to learn:** Active IPV disclosed in couples screening → No joint sessions; individual safety plan

44 — A

Provision of Counseling Interventions · Task 4.4 · Chapter 7

- **A. Physiological self-soothing — Owner: Gottman (antidote to stonewalling).** Self-soothing is the antidote to stonewalling when a partner is flooded.
- B. Gentle startup — Owner: Gottman (antidote to criticism). Gentle startup replaces criticism, not shutting down.
- C. Taking responsibility — Owner: Gottman (antidote to defensiveness). Taking responsibility is the antidote to defensiveness.
- **Pair to learn:** Stonewalling → physiological self-soothing (Gottman)

45 — B

Intake & Assessment · Task 2B.2 · Chapter 13

- **B. Sibling abuse — Owner: Family abuse (power imbalance, repeated victimization, fear).** A power imbalance, repeated victimization of the same child, and fear are the markers that separate sibling abuse from ordinary fighting.
- A. Power-seeking misbehavior — Owner: Dreikurs (goals of misbehavior). Dreikurs's goals explain a child's misbehavior toward adults; they don't capture repeated victimization of a younger sibling.
- C. Sibling rivalry — Owner: Normal sibling conflict. Ordinary rivalry lacks the power imbalance, repeated targeting of one child, and fear described here.
- **Pair to learn:** Power imbalance + repeated victimization + fear → sibling abuse (not rivalry)

46 — B

Legal & Ethical Compliance · Task 6.8 · Chapter 17

- **B. Enroll in Medicare as a mental health counselor (MHC) — Owner: Medicare MHC enrollment (Consolidated Appropriations Act, 2023).** Since January 1, 2024, counselors who meet Medicare's MHC requirements can enroll and bill Medicare directly.
- A. Bill only incident to a supervising physician's services — Owner: Medicare "incident to" billing. Incident-to billing was the main pathway before 2024; eligible counselors can now enroll and bill in their own name.
- C. Bill beneficiaries privately without enrolling or opting out — Owner: Medicare mandatory claim submission rule. A practitioner eligible for Medicare must enroll and submit claims or formally opt out before privately contracting with beneficiaries.
- **Pair to learn:** LPC seeing Medicare clients since 2024 → enroll as a Medicare MHC

47 — C

Provision of Counseling Interventions · Task 4.4 · Chapter 7

- **C. Emotionally focused therapy — Owner: Sue Johnson (EFT).** EFT accesses primary emotions beneath secondary ones to de-escalate the cycle and restructure the bond.
- A. The Gottman Method — Owner: Gottman. Gottman targets the Four Horsemen and friendship through coaching, not accessing primary attachment emotion.
- B. Bowen family systems therapy — Owner: Bowen. Bowen coaches calm differentiation and detriangulation, not access to attachment emotions.
- **Pair to learn:** Primary emotion beneath secondary anger → EFT (Johnson), not Gottman

48 — C

Professional Development & Counselor Self-Awareness · Task 1A.7 · Chapter 22

- **C. Client data should go only to a secure tool covered by a BAA — Owner: HIPAA business associate agreement; ACA B.1.c.** A vendor handling protected health information must sign a business associate agreement; free public chatbots generally do not, and confidentiality applies regardless.
- A. Chatbot output is reliable because it draws on vast data — Owner: Automation bias. Trusting automated output because it is automated is a recognized error; AI output requires verification.
- B. Removing the client's name alone makes the disclosure safe — Owner: Incomplete de-identification (HIPAA Safe Harbor identifiers). HIPAA's Safe Harbor method removes 18 identifiers, not just a name; session details can still identify a client.
- **Pair to learn:** Client data into an AI tool → secure vendor with a BAA

49 — B

Intake & Assessment · Task 2A.1 · Chapter 6

- **B. Ewing v. Goldstein — Owner: Ewing v. Goldstein (2004, California).** Ewing extended the duty to protect to credible threats relayed by a client's family member.
- A. Jablonski v. United States — Owner: Jablonski (duty from history of violence, no explicit threat). Jablonski based the duty on a client's history of violence, not a family member's report.
- C. Tarasoff v. Regents (1976) — Owner: Tarasoff II (duty to protect an identifiable victim). Tarasoff II broadened the duty to warn into a duty to protect; it involved the client's own threat.
- **Pair to learn:** Threat reported by family member → Ewing v. Goldstein

50 — B

Intake & Assessment · Task 2B.3 · Chapter 11

- **B. Opioid withdrawal — Owner: Opioids (flu-like withdrawal; rarely life-threatening).** Runny nose, tearing, yawning, goosebumps, dilated pupils, aches, and diarrhea form the flu-like opioid withdrawal picture.
- A. Alcohol withdrawal — Owner: Alcohol (tremor, sweating, seizures). Alcohol withdrawal brings tremor, sweating, rapid pulse, and possible seizures, not yawning, tearing, and goosebumps.
- C. Stimulant intoxication — Owner: Stimulants (euphoria, dilated pupils, agitation). Stimulant intoxication also dilates pupils but brings euphoria, rapid heart rate, and agitation rather than flu-like symptoms.
- **Pair to learn:** Yawning, tearing, goosebumps, dilated pupils → opioid withdrawal

51 — A

Indirect Client Care · Task 5.1 · Chapter 23

- **A. Wraparound process — Owner: Wraparound (family-driven, team-based planning).** Wraparound brings family, natural supports, and systems such as school and court into one strengths-based, family-driven plan.
- B. Assertive community treatment — Owner: ACT (adults with serious mental illness). ACT is a multidisciplinary team that delivers services directly to adults with serious mental illness in the community.
- C. Partial hospitalization — Owner: Partial hospitalization (structured day program). Partial hospitalization is an intensive day-treatment level of care, not a cross-system planning process.
- **Pair to learn:** Youth across multiple systems, family-driven team plan → wraparound

52 — B

Intake & Assessment · Task 2A.1 · Chapter 17

- **B. Assent — Owner: Assent (minors; ACA A.2.d, B. 5.b).** Parents or guardians generally consent; the minor gives assent.
- A. Informed consent — Owner: Informed consent (ACA A.2; given by those legally able). Legal consent here comes from the parents, so the child's agreement isn't informed consent.
- C. Authorization — Owner: HIPAA authorization (permission to release information). An authorization permits release of information, not participation in counseling.
- **Pair to learn:** Minor's agreement alongside parental consent → Assent

53 — C

Treatment Planning & Continuity of Care · Task 3A.2 · Chapter 21

- **C. Client reports 4 panic attacks a week and missed 3 workdays this month — Owner: Problem statement (behavioral terms).** A problem statement describes the concern in concrete, behavioral terms that can serve as a baseline.
- A. Client has a great deal of anxiety and simply wants to feel calmer overall — Owner: Vague goal language (not behavioral). This is vague and doesn't describe observable behavior or frequency.
- B. Client will reduce panic attacks to 1 or fewer a week within 8 weeks — Owner: Short-term objective (measurable step). This is a measurable short-term objective, not a statement of the problem.
- **Pair to learn:** Concern in behavioral terms → Problem statement

54 — B

Treatment Planning & Continuity of Care · Task 3A.11 · Chapter 21

- **B. Define the target behavior, reinforce alternatives, and track it — Owner: Behavior management plan (operant conditioning; Skinner).** An effective plan defines the behavior in observable terms, reinforces alternative behaviors, and tracks data consistently across home and school settings.
- A. Lengthen time-outs after each tantrum until the behavior stops — Owner: Escalating punishment. Escalating punishment may briefly suppress tantrums but teaches no replacement skill and can increase conflict.
- C. Ignore all behavior, including aggression, so it extinguishes — Owner: Extinction/planned ignoring misapplied. Planned ignoring can reduce attention-

seeking behavior, but aggression or unsafe behavior cannot be ignored, and extinction bursts must be anticipated.

- **Pair to learn:** Tantrums across settings → Define target, reinforce alternatives, track data

55 — C

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **C. Determine the nature of the dilemma — Owner: Forester-Miller & Davis: step 3.** After identifying the problem and applying the Code, the counselor examines the principles involved, relevant literature, and consultation.
- A. Apply the publicity test to the choice — Owner: Stadler's three tests (justice, publicity, universality). The publicity test belongs to Stadler and is used to check a chosen action, not as step 3.
- B. Evaluate the selected course of action — Owner: Forester-Miller & Davis: step 6. Evaluation comes after a course of action has been chosen.
- **Pair to learn:** Step 3 → Determine nature and dimensions of the dilemma (Forester-Miller & Davis)

56 — C

Legal & Ethical Compliance · Task 6.9 · Chapter 17

- **C. Assess the risk and notify clients promptly, within 60 days at most — Owner: HIPAA Breach Notification Rule (45 CFR 164.402, 164.404).** Unless a risk assessment shows low probability of compromise, clients get notice without unreasonable delay and no later than 60 days after discovery; state laws may add duties.
- A. Leave all notice to the vendor, since its systems were affected — Owner: Delegated-duty assumption (45 CFR 164.410). The vendor must notify the practice, but the practice remains responsible for making sure affected clients are notified.

- B. Notify clients only if misuse of their data is later proven — Owner: Harm standard (replaced by the 2013 Omnibus Rule). An impermissible disclosure is presumed a breach unless a risk assessment shows a low probability of compromise; proof of misuse is not required.
- **Pair to learn:** Vendor security incident → risk assessment, then notice within 60 days (HIPAA Breach Notification Rule)

57 — B

Provision of Counseling Interventions · Task 4.1 · Chapter 13

- **B. Perpetual problem — Owner: Gottman (lasting differences).** Most couple conflicts are perpetual, rooted in lasting differences; the counseling goal is dialogue and acceptance, not a final solution.
- A. Pursue-withdraw cycle — Owner: Emotionally focused therapy. EFT's pursue-withdraw cycle describes one partner pressing and the other retreating in conflict; the stem describes a lasting difference in preferences.
- C. Solvable problem — Owner: Gottman (problems that can be settled). A difference rooted in personality that has lasted 15 years isn't the kind of problem that gets settled.
- **Pair to learn:** Lasting differences that won't be resolved → perpetual problem (Gottman)

58 — B

Intake & Assessment · Task 2B.4 · Chapter 19

- **B. Precipitating factor — Owner: 4 Ps: precipitating.** Precipitating factors answer the question of what triggered the current problem. The layoff came right before the panic began.
- A. Predisposing factor — Owner: 4 Ps: predisposing. Predisposing factors are vulnerabilities such as

genetics, early experiences, and temperament, not the recent trigger.

- C. Perpetuating factor — Owner: 4 Ps: perpetuating. Perpetuating factors keep the problem going, such as avoidance, substance use, or negative thinking.
- **Pair to learn:** What triggered the current problem → Precipitating factor (4 Ps)

59 — A

Professional Development & Counselor Self-Awareness · Task 1A.8 · Chapter 22

- **A. Reputable training on their stressors, applied to each person — Owner: MSJCC (Ratts et al., 2016) — knowledge and skills.** Population-specific training builds knowledge, and applying it while assessing each individual keeps it from becoming a stereotype.
- B. Online lists of typical traits used as a guide to each client — Owner: Overgeneralization (stereotyping). Trait lists flatten individual differences and invite the counselor to see the group rather than the client.
- C. Reliance on the general multicultural course from graduate school — Owner: One-time training (competence as fixed). A single course years ago cannot cover a new population; cultural competence is developed over time.
- **Pair to learn:** New population → reputable training, applied individually (MSJCC)

60 — B

Intake & Assessment · Task 2B.2 · Chapter 13

- **B. Intimate terrorism — Owner: Michael P. Johnson (coercive controlling violence).** Violence used within a broader pattern of coercive control, usually frequent and escalating, is intimate terrorism.
- A. Violent resistance — Owner: Michael P. Johnson (IPV typology). Violent resistance is violence used in

response to a partner's intimate terrorism; she is the one being controlled.

- C. Situational couple violence — Owner: Michael P. Johnson (IPV typology). Situational couple violence arises from specific conflicts without a broader pattern of control; here control is pervasive.
- **Pair to learn:** Violence within a pattern of coercive control → intimate terrorism (Johnson)

61 — B

Provision of Counseling Interventions · Task 4.3 · Chapter 10

- **B. Disruptive mood dysregulation disorder — Owner: DSM-5-TR (12 months; diagnosed ages 6 to 18; onset before 10).** Severe outbursts 3 or more times a week with persistently irritable mood between them for at least 12 months, with onset before age 10, defines DMDD.
- A. Intermittent explosive disorder — Owner: DSM-5-TR (outbursts out of proportion, age 6+). IED involves outbursts, but not the persistently irritable mood between them that defines DMDD.
- C. Oppositional defiant disorder — Owner: DSM-5-TR (4 symptoms for 6 months). When criteria for both are met, only DMDD is diagnosed; severe recurrent outbursts with chronic irritability point to DMDD.
- **Pair to learn:** Outbursts 3+/week + chronic irritability for 12 months → DMDD

62 — B

Indirect Client Care · Task 5.2 · Chapter 23

- **B. Obtain the client's signed authorization to release — Owner: Written authorization to disclose (ACA B.6.g / HIPAA).** The client's signed authorization is needed before records go to any third party, and she should understand how release could affect the custody case.

- A. Send the full record, since attorneys have access — Owner: Disclosure without authorization. An attorney's request alone, even from the client's own attorney, does not authorize release.
- C. Refuse, since attorneys can never receive records — Owner: Blanket refusal. With valid authorization or a court order, records can be released; refusal is not the rule.
- **Pair to learn:** Even the client's own attorney requests records → client's signed authorization first

63 — A

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **A. A batterer intervention program — Owner: Batterer intervention (IPV standard).** Batterer intervention programs, not anger management alone, are the standard referral for those who use violence.
- B. Conjoint couples counseling — Owner: Couples counseling (contraindicated with ongoing violence). Couples work is contraindicated while violence or coercive control is ongoing.
- C. An anger management class alone — Owner: Anger management (insufficient alone). Anger management alone is not the standard referral for partner violence.
- **Pair to learn:** Person using violence → batterer intervention program (not anger management alone)

64 — C

Treatment Planning & Continuity of Care · Task 3B.3 · Chapter 21

- **C. Review what helped, plan ongoing practice, and offer boosters — Owner: Maintenance and generalization planning (booster sessions).** Naming the skills that helped, planning how to keep using them, and scheduling booster sessions supports maintenance after counseling ends.

- A. Reassure him that gains always last once counseling ends — Owner: False reassurance. Gains can fade without a plan. Blanket reassurance dismisses his question and leaves him unprepared.
- B. Continue weekly sessions indefinitely to prevent backsliding — Owner: Fostering dependency (ACA A.11.c). ACA A.11.c directs counselors to terminate when clients no longer need or benefit from counseling. Open-ended sessions foster dependency.
- **Pair to learn:** "How will I keep my gains?" → Review skills, practice plan, booster sessions

65 — C

Provision of Counseling Interventions · Task 4.11 · Chapter 2

- **C. Displacement — Owner: Defense mechanism (Freud; A. Freud).** Displacement redirects feelings toward a safer target, here from the supervisor to the son.
- A. Projection — Owner: Defense mechanism (Freud; A. Freud). Projection attributes one's own unacceptable feelings to someone else; he isn't claiming his son is angry.
- B. Reaction formation — Owner: Defense mechanism (Freud; A. Freud). Reaction formation expresses the opposite of what one feels, such as excessive warmth toward the supervisor.
- **Pair to learn:** Redirecting feelings to a safer target → displacement

66 — A

Provision of Counseling Interventions · Task 4.8 · Chapter 9

- **A. Help her prepare to advocate for herself — Owner: ACA Advocacy Competencies (client level, acting with).** Empowering clients to self-advocate comes before acting on their behalf.

- B. Call her employer and negotiate for her — Owner: Client advocacy (acting on behalf of). Acting on her behalf is appropriate after empowerment is tried or when she can't act herself.
- C. Launch a public campaign about workplace access — Owner: Social/political advocacy (public arena). Public-arena advocacy is a broader level and not the first step for one client.
- **Pair to learn:** Advocacy → empower the client first (acting with, then on behalf of)

67 — A

Legal & Ethical Compliance · Task 6.10 · Chapter 17

- **A. Links among diagnosis, impairment, and plan goals — Owner: Medical necessity documentation.** Each note should connect the diagnosis and current functional impairment to plan goals, the interventions used, and the client's response.
- B. A diagnosis code entered on every claim form — Owner: Diagnosis-as-justification error. A diagnosis code alone does not show why each session was needed or what it accomplished.
- C. A verbatim account of what the client said — Owner: Process-note detail (minimum necessary). Verbatim content belongs, if anywhere, in psychotherapy notes; it does not show necessity and exposes more than needed.
- **Pair to learn:** Audit of weekly claims → notes linking diagnosis, impairment, goals, interventions

68 — A

Professional Development & Counselor Self-Awareness · Task 1A.5 · Chapter 17

- **A. Get the client's consent — Owner: ACA A.7.b (consent before advocating for a specific client).** Counselors obtain client consent before advocating on a specific client's behalf.

- B. Stay out of it, since housing isn't counseling — Owner: Ignoring client barriers (contrary to A.7.a). Counselors advocate to address barriers clients face; housing is one of them.
- C. Call the housing office right away — Owner: Advocacy without consent (contrary to A.7.b). Acting before the client agrees discloses her situation and skips her voice.
- **Pair to learn:** Advocating for a specific client → Client consent first (ACA A.7.b)

69 — B

Intake & Assessment · Task 2B.7 · Chapter 19

- **B. CALOCUS/CASII — Owner: CALOCUS/CASII (children and adolescents).** The CALOCUS/CASII is the level-of-care tool for children and adolescents.
- A. The ASAM Criteria — Owner: ASAM Criteria (substance use, six dimensions). The ASAM Criteria guide level of care for substance use, using six dimensions.
- C. LOCUS — Owner: LOCUS (Level of Care Utilization System, adults). LOCUS is the level-of-care tool for adults.
- **Pair to learn:** Level of care for youth → CALOCUS/CASII (LOCUS is for adults)

70 — B

Professional Development & Counselor Self-Awareness · Task 1B.1 · Chapter 22

- **B. Treat it as an early sign of compassion fatigue and act now — Owner: Figley (compassion fatigue).** Numbness after heavy exposure to suffering is an early warning of compassion fatigue; prompt self-care, consultation, and caseload limits follow.
- A. Wait to act until clients begin to complain about care — Owner: Waiting for impairment (misread of ACA C.2.g). Counselors monitor themselves for early signs and act before impairment harms clients.

- C. Take on more clients to rebuild a sense of purpose — Owner: Compassion satisfaction (Stamm) misapplied. Stamm's compassion satisfaction comes from meaningful work, not added load, which deepens fatigue.
- **Pair to learn:** Numbness, going through the motions → early compassion fatigue (Figley); act now

71 — C

Intake & Assessment · Task 2A.3 · Chapter 19

- **C. Weigh all sources and explore the discrepancy with him — Owner: Multiple sources of information (self-report plus collateral).** Self-report is efficient but subject to bias, insight, and honesty. Using several sources, and exploring where they disagree, increases confidence in the assessment.
- A. Accept the checklist, since self-report is most direct — Owner: Single-source assessment (self-report only). Relying on self-report alone ignores conflicting collateral information; self-report is vulnerable to bias and limited insight.
- B. Diagnose alcohol use disorder from the collateral reports — Owner: Diagnosing from collateral alone (skipping integration). Collateral reports inform the assessment but don't replace it; diagnosing from them alone skips his own account and direct assessment.
- **Pair to learn:** Self-report conflicts with collateral → Integrate multiple sources (2A.3)

72 — C

Indirect Client Care · Task 5.4 · Chapter 23

- **C. Help her get immediate medical evaluation and document it — Owner: Lithium toxicity response.** GI symptoms with coarse tremor and confusion suggest lithium toxicity, which needs immediate medical evaluation; the counselor connects her to care and documents.

- A. Note these as common side effects to recheck next week — Owner: Minimizing signs of toxicity. A fine tremor and mild nausea can be common, but coarse tremor with vomiting, diarrhea, and confusion signal possible toxicity.
- B. Suggest she drink less water to ease the nausea — Owner: Dangerous advice (dehydration raises lithium levels). Vomiting, diarrhea, and low fluid intake can raise lithium levels, and fluid advice is outside counselor scope.
- **Pair to learn:** Lithium + coarse tremor, GI upset, confusion → possible toxicity, immediate medical care

73 — A

Provision of Counseling Interventions · Task 4.11 · Chapter 16

- **A. Hardiness — Owner: Kobasa (commitment, control, challenge).** Kobasa's hardiness is made up of commitment, control, and challenge, which buffer stress.
- B. Career adaptability — Owner: Savickas (concern, control, curiosity, confidence). Savickas's four Cs are career construction resources; commitment, control, and challenge as a stress buffer are Kobasa's.
- C. Type A behavior — Owner: Friedman & Rosenman. Type A is a competitive, impatient, hostile pattern, and hostility is the harmful part, not a buffer.
- **Pair to learn:** Commitment, control, challenge → hardiness (Kobasa, not Savickas)

74 — B

Intake & Assessment · Task 2B.12 · Chapter 19

- **B. E.5.d, Refraining from diagnosis — Owner: ACA Code E.5.d.** E.5.d allows counselors to refrain from making or reporting a diagnosis if they believe it would cause harm to the client.
- A. E.5.b, Cultural sensitivity — Owner: ACA Code E. 5.b. E.5.b concerns how culture affects the way

problems are defined and expressed in diagnosis, not withholding a diagnosis.

- C. E.5.c, Historical and social prejudices — Owner: ACA Code E.5.c. E.5.c concerns awareness of how some groups have been misdiagnosed and pathologized, not refraining from diagnosis.
- **Pair to learn:** Diagnosis would cause harm → Refraining from diagnosis (ACA E.5.d)

75 — A

Treatment Planning & Continuity of Care · Task 3B.4 · Chapter 21

- **A. A booked follow-up, a release for the next provider, and a crisis plan — Owner: Discharge planning with linkage to the next level of care.** Good discharge planning confirms a scheduled appointment, secures a signed release so records reach the next provider, and gives a crisis plan for the transition.
- B. A list of local resources and an invitation to call when ready — Owner: Passive referral (no linkage). Handing over a list without a confirmed appointment raises the risk that the client falls through the gap between levels of care.
- C. A planning meeting on the final day once progress is confirmed — Owner: Last-minute discharge planning. Discharge planning begins at admission. Waiting until the last day leaves no time to arrange appointments, releases, or supports.
- **Pair to learn:** Step-down from IOP → booked follow-up + release + crisis plan

76 — A

Treatment Planning & Continuity of Care · Task 3A.10 · Chapter 21

- **A. Discuss the alliance and adjust tasks with her — Owner: Checking the alliance when progress stalls (3A.10).** A stalled plan is often an alliance

problem; agreement on tasks is part of the alliance and can be revised together.

- B. Continue the plan, since six sessions is early — Owner: Repeating an ineffective plan. Flat scores plus alliance feedback call for review, not more of the same.
- C. Switch to a new approach without discussing it — Owner: Unilateral change (switching without collaboration). Changing approaches without discussion leaves her out of decisions about her care.
- **Pair to learn:** Stalled progress plus low task agreement → Check and repair the alliance (3A.10)

77 — B

Legal & Ethical Compliance · Task 6.7 · Chapter 17

- **B. Justice — Owner: Justice (ACA principle; Kitchener).** Fairness and equal access to services, such as a sliding scale, is justice.
- A. Beneficence — Owner: Beneficence (ACA principle; Kitchener). Beneficence is working for the client's good; the defining feature here is equitable access.
- C. Autonomy — Owner: Autonomy (ACA principle; Kitchener). Autonomy is the client's right to direct their own life, not fair distribution of services.
- **Pair to learn:** Sliding scale, equal access → Justice

78 — B

Legal & Ethical Compliance · Task 6.11 · Chapter 17

- **B. Practice it in session, tie it to triggers, and review its use — Owner: Between-session practice with review (CBT homework).** Rehearsing the skill in session, linking it to the client's triggers, and reviewing practice makes the tool part of treatment.
- A. Send a download link and let the client explore it alone — Owner: Unintegrated technology (no follow-through). An app handed over without teaching or review is rarely used well and is disconnected from treatment targets.

- C. Replace weekly sessions with the app to reduce costs — Owner: Substituting a self-help tool for treatment. A calming app supports counseling for panic symptoms; it is not a replacement for clinical care.
- **Pair to learn:** Calming app → rehearse in session, link to triggers, review use

79 — B

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **B. Meet to process the conflict and plan ahead — Owner: Co-leadership (meet before and after sessions).** Co-leaders must meet to plan and process; open disagreement without repair can harm the group.
- A. Never mention it, so members aren't upset — Owner: Avoiding repair. Leaving a visible conflict unaddressed models avoidance.
- C. Let the senior leader make all decisions from now on — Owner: Abandoning shared leadership. This removes the benefits of co-leadership instead of repairing it.
- **Pair to learn:** Co-leader disagreement → meet before and after sessions to process

80 — C

Professional Development & Counselor Self-Awareness · Task 1B.2 · Chapter 22

- **C. Professional Quality of Life Scale — Owner: Stamm (ProQOL).** The ProQOL measures compassion satisfaction, burnout, and secondary traumatic stress, making it a fit for assessing trauma exposure.
- A. Beck Depression Inventory-II — Owner: Beck (BDI-II). The BDI-II measures depressive symptom severity, not the effects of trauma exposure at work.
- B. Maslach Burnout Inventory — Owner: Maslach & Jackson (MBI). The MBI measures burnout dimensions

only; it does not assess secondary traumatic stress or compassion satisfaction.

- **Pair to learn:** Compassion satisfaction + burnout + STS → ProQOL (Stamm)

81 — A

Intake & Assessment · Task 2B.1 · Chapter 10

- **A. Posttraumatic growth — Owner: Tedeschi & Calhoun.** Positive change that follows a struggle with trauma is posttraumatic growth.
- B. Resilience — Owner: Bonanno. Bonanno's resilience describes the common pattern of steady functioning after potential trauma; she describes positive change beyond where she started.
- C. Shattered assumptions — Owner: Janoff-Bulman. Shattered assumptions are broken beliefs about a benevolent world and a worthy self, not positive change.
- **Pair to learn:** Positive change after struggling with trauma → posttraumatic growth (Tedeschi & Calhoun, not Bonanno)

82 — C

Intake & Assessment · Task 2B.8 · Chapter 19

- **C. Interactional dynamics — Owner: Interactional dynamics (2B.8).** In couple and family sessions, counselors observe who speaks, who stays silent, alliances, conflict patterns, and nonverbal responses.
- A. The cultural definition of the problem — Owner: CFI domain 1. The CFI's first domain asks how the client describes the problem; it is gathered by asking, not by observing who speaks for whom.
- B. The daughter's affect — Owner: MSE affect (one person's observed emotional expression). Affect is one person's emotional expression in the moment; this observation concerns how members interact.
- **Pair to learn:** Who speaks, who's silent, alliances → Interactional dynamics (2B.8)

83 — A

Indirect Client Care · Task 5.5 · Chapter 23

- **A. Know referral resources and suggest appropriate alternatives — Owner: ACA A.11.a competence within termination and referral.** Counselors are expected to know culturally and clinically appropriate referral resources and suggest them when they cannot help.
- B. End services immediately and let the client find help — Owner: Client abandonment (ACA A.12). Ending services without arranging alternatives risks abandoning the client.
- C. Refer only to providers who pay a referral fee — Owner: Fee splitting (ACA A.10.b). Receiving payment for referrals is prohibited as an unacceptable business practice.
- **Pair to learn:** Lacks competence → know referral resources and suggest alternatives (ACA A.11.a)

84 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Examine alternatives — Owner: James & Gilliland (step 4).** Steps 1 to 3 (define the problem, ensure safety, provide support) are listening steps; examining alternatives begins the acting steps.
- A. Provide support — Owner: James & Gilliland (step 3). Providing support is the last of the listening steps.
- C. Make psychological contact — Owner: Roberts (stage 2). Making psychological contact is a stage in Roberts's seven-stage model.
- **Pair to learn:** Listening steps then acting steps → James & Gilliland (not Roberts)

85 — B

Treatment Planning & Continuity of Care · Task 3B.5 · Chapter 21

- **B. The abstinence violation effect — Owner: Marlatt & Gordon (abstinence violation effect).** The abstinence violation effect is the all-or-nothing reaction that turns a lapse into a full relapse. Relapse prevention plans reframe a lapse as a learning event.
- A. An apparently irrelevant decision — Owner: Marlatt & Gordon (apparently irrelevant decisions). Apparently (later "seemingly") irrelevant decisions are small choices, such as driving past a bar, that set up a high-risk situation before any use.
- C. Cognitive dissonance — Owner: Leon Festinger (cognitive dissonance theory). Festinger described the discomfort of holding conflicting beliefs or acting against them. It is a social psychology concept, not the relapse prevention term for this reaction.
- **Pair to learn:** "I've blown it, so why stop?" → Abstinence violation effect (Marlatt & Gordon)

86 — A

Provision of Counseling Interventions · Task 4.9 · Chapter 17

- **A. Decline and refer him to another counselor — Owner: ACA A.5.b (previous romantic partners).** Counselors may not become counselors to their previous romantic or sexual partners, no matter how much time has passed.
- B. Accept after documenting the risks first — Owner: Boundary extension (A.6), not applicable here. Documented boundary extensions apply to limited nonprofessional contact, not to a standard that prohibits the relationship.
- C. Accept, since more than 5 years have passed — Owner: ACA A.5.c 5-year rule (applies to former

clients). The 5-year rule covers relationships with former clients, not counseling a former partner.

- **Pair to learn:** Former romantic partner seeks counseling → Prohibited (ACA A.5.b)

87 — B

Legal & Ethical Compliance · Task 6.20 · Chapter 9

- **B. Create a separate professional account — Owner: ACA H.6.a (virtual professional presence).** Counselors keep professional and personal social media presences separate.
- A. Share brief client success stories — Owner: Testimonials and confidentiality (C.3.b, H.6.d). Testimonials from current clients aren't allowed, and posting client stories risks disclosing confidential information.
- C. Post the tips from her personal account — Owner: Mixing personal and professional presence (contrary to H.6.a). Using her personal account blends her private life with her professional role.
- **Pair to learn:** Professional online presence → Separate accounts (ACA H.6.a)

88 — C

Legal & Ethical Compliance · Task 6.12 · Chapter 17

- **C. Yes, because the agency policy protects the agency first — Owner: Individual professional liability policy.** A personal policy protects the counselor's own interests, follows her across jobs, and often includes licensing board defense.
- A. No, because the agency policy fully protects every employee — Owner: Assumed employer coverage. Agency policies have their own limits, exclusions, and priorities, and may not cover off-duty work or licensing board defense.
- B. No, because employees cannot be sued as individuals — Owner: Respondeat superior (misread). The employer can be held liable for employees' acts,

but the individual counselor can still be named in a suit.

- **Pair to learn:** Covered by employer's policy → still carry your own

89 — A

Treatment Planning & Continuity of Care · Task 3A.5 · Chapter 21

- **A. Review the scores with her and decide together — Owner: Using assessment results to facilitate client decisions (3A.5).** Assessment results are a tool for the client's decision; reviewing them together supports an informed, collaborative choice.
- B. Tell her the scores mean she must stay weekly — Owner: Counselor-directed decision (undercuts collaboration, 3A.9). Her scores have improved, and deciding for her bypasses her role in the plan.
- C. End counseling now, since her score is under 10 — Owner: Termination without process (A.11.c pretermination counseling). She asked about reducing frequency, and ending abruptly skips termination work.
- **Pair to learn:** Client decision about frequency → Use assessment results together (3A.5)

90 — B

Provision of Counseling Interventions · Task 4.4 · Chapter 22

- **B. Slow things down so each partner can speak and be heard — Owner: Conflict tolerance; modeling respectful conflict (4.4).** Effective counselors stay present with conflict, name it non-defensively, and model respectful conflict in couples and families.
- A. End the session until both partners have calmed down — Owner: Conflict avoidance. Ending the session avoids the conflict instead of staying present with it.
- C. Let them argue freely so pent-up anger is released — Owner: Catharsis (emotional release, Yalom).

Emotional release alone doesn't model respectful conflict and lets the escalation continue.

- **Pair to learn:** Staying present with escalating conflict → Conflict tolerance; model respectful conflict (4.4)

91 — A

Professional Development & Counselor Self-Awareness · Task 1B.3 · Chapter 22

- **A. Weigh client needs against training and applicable rules — Owner: Competence self-assessment (ACA C.2.a).** The counselor weighs what these clients need against current training and whatever credential rules apply in that jurisdiction or setting.
- B. Assume the license already covers every presenting problem — Owner: Licensure-equals-competence error. A license sets a general floor; it does not confer competence in every specialty.
- C. Obtain any certificate available, whatever its content — Owner: Credentials over competence. A credential helps only if its training meets the caseload's needs.
- **Pair to learn:** Shifting caseload → match training to client needs and applicable credential rules

92 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Erich Lindemann — Owner: Lindemann.** Lindemann's Cocoanut Grove work founded the field of crisis intervention.
- A. Gerald Caplan — Owner: Caplan (preventive psychiatry). Caplan built on crisis work with preventive psychiatry and the time-limited view of crisis.
- C. Kenneth Doka — Owner: Doka. Doka described disenfranchised grief, loss that isn't socially recognized.

- **Pair to learn:** Cocoanut Grove fire; acute grief → Lindemann (not Caplan)

93 — B

Indirect Client Care · Task 5.6 · Chapter 23

- **B. Review the plan, recent notes, and risk history — Owner: Continuity of care through record review.** Reviewing the plan, recent notes, and risk history keeps care continuous during coverage.
- A. Start fresh so prior notes do not bias the work — Owner: Ignoring the record. Skipping the record risks missing risk factors and repeating work already done.
- C. Plan to have the client retell her full history — Owner: Redundant reassessment. Asking for a full retelling burdens the client and wastes time the record could save.
- **Pair to learn:** Covering another counselor's client → review plan, recent notes, and risk history first

94 — C

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **C. Universalizing — Owner: Group leader skill: universalizing.** Universalizing highlights a theme that many members share.
- A. Linking — Owner: Group leader skill: linking. Linking connects two specific members' concerns.
- B. Process observation — Owner: Group leader skill: process observation. Process observation comments on how members are interacting, not what they share.
- **Pair to learn:** Leader names a shared theme for the group → universalizing (not linking)

95 — A

Provision of Counseling Interventions · Task 4.6 · Chapter 5

- **A. Implosive therapy — Owner: Stampfl (implosive therapy).** Stampfl's implosive therapy is intense, imagined, often exaggerated exposure.
- B. Systematic desensitization — Owner: Wolpe. Wolpe pairs relaxation with imagined scenes climbed gradually up a hierarchy.
- C. Flooding — Owner: Behavioral (flooding). Flooding is typically prolonged, intense exposure to the actual feared situation; the imagined, deliberately exaggerated version is Stampfl's implosive therapy.
- **Pair to learn:** Intense imagined exposure → implosive therapy (Stampfl), not flooding

96 — C

Treatment Planning & Continuity of Care · Task 3B.6 · Chapter 21

- **C. Prepare a professional will naming a records custodian — Owner: Professional will (ACA C.2.h).** ACA C.2.h requires a plan to transfer clients and records to an identified colleague or records custodian if the counselor is incapacitated, dies, or ends practice.
- A. Rely on clients to find a new counselor if it is ever needed — Owner: Client abandonment (ACA A.12). Leaving clients to manage an unexpected disruption alone risks the abandonment and neglect that ACA A. 12 prohibits.
- B. Give each client a copy of the full case record right now — Owner: Client access to records misapplied (ACA B.6.e). Clients may request access to their records, but handing out full records now is not a plan for continuity and does not arrange transfer of care.
- **Pair to learn:** Sudden incapacity or death → Professional will with records custodian (ACA C.2.h)

97 — C

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 21

- **C. Offer options, get a release, and follow up — Owner: Referral process (3B.1).** Good referrals involve the client, offer options, use a signed release before sharing, and confirm the client connected.
- A. Fax her records to the nearest psychiatrist today — Owner: Sharing without a release (ACA B.6.g). Ethically, information isn't shared without the client's signed release.
- B. Tell her to find a prescriber through her insurer — Owner: Passive referral without follow-up. Leaving the search to her skips options, coordination, and follow-up.
- **Pair to learn:** Concurrent medication referral → Options, release, follow-up (3B.1)

98 — A

Intake & Assessment · Task 2B.11 · Chapter 20

- **A. E.11, Obsolete assessment and outdated results — Owner: ACA Code E.11.** E.11 directs counselors not to use outdated instruments or results for current decisions.
- B. E.10, Assessment security — Owner: ACA Code E. 10. E.10 concerns keeping tests secure and not reproducing or modifying them without permission.
- C. E.8, Multicultural issues in assessment — Owner: ACA Code E.8. E.8 urges caution with instruments normed on populations other than the client's; age of the results is a different issue.
- **Pair to learn:** Outdated tests or results for current decisions → ACA E.11 (not E.10)

99 — A

Legal & Ethical Compliance · Task 6.13 · Chapter 17

- **A. Meet that state's own licensure or authorization requirements — Owner: State licensure law (non-member state).** A non-member state controls who may practice with clients located there, so its own licensure or authorization rules apply.
- B. Rely on the home-state license, since the client is already in care — Owner: Continuity-of-care exception myth. There is no general continuity exception; any temporary allowance comes only from that state's own law.
- C. Use the existing privilege, which extends to non-member states — Owner: Compact scope misread (member states only). A privilege to practice is valid only in Compact member states.
- **Pair to learn:** Client moves to a non-Compact state → that state's own licensure rules

100 — B

Provision of Counseling Interventions · Task 4.2 · Chapter 22

- **B. Examine her gender-role assumptions in supervision — Owner: Awareness of one's own gender-role assumptions (4.2).** Sensitivity to gender issues includes awareness of one's own gender-role assumptions and their effect on clients; supervision supports that reflection.
- A. Disclose the pattern to each client and apologize — Owner: Self-disclosure that shifts focus to the counselor. This centers the counselor's process and burdens clients instead of changing her practice.
- C. Refer her male clients to a male colleague — Owner: Premature referral (avoiding self-work). Referral sidesteps the counselor's own bias rather than addressing it.

- **Pair to learn:** Counselor's gender-role bias steering clients → Examine own assumptions (4.2)

101 — B

Professional Development & Counselor Self-Awareness · Task 1B.4 · Chapter 22

- **B. Name the gap and add routine screening for past elevated mood — Owner: Reflective practice (Schön).** Reflecting on the miss and changing practice turns an identified limitation into a routine safeguard.
- A. Conclude the counselor cannot treat mood disorders and transfer — Owner: Overcorrection. One near miss is a fixable gap, not proof of incompetence; transferring the client overreacts.
- C. Attribute the miss to the client's incomplete self-report — Owner: Externalizing a limitation. Clients often do not link past highs to mood; screening is the counselor's job, not the client's.
- **Pair to learn:** Near miss in assessment → name the gap, change the routine

102 — C

Provision of Counseling Interventions · Task 4.6 · Chapter 5

- **C. Exposure and response prevention — Owner: Behavioral (ERP for OCD).** ERP pairs facing the obsession with blocking the compulsion, here the repeated checking.
- A. Systematic desensitization — Owner: Wolpe. Systematic desensitization uses relaxation and imagined scenes; it doesn't block a compulsion.
- B. Interoceptive exposure — Owner: CBT for panic. Interoceptive exposure targets feared body sensations in panic, not compulsions.
- **Pair to learn:** Face the obsession, block the compulsion → ERP

103 — B

Treatment Planning & Continuity of Care · Task 3A.3 · Chapter 21

- **B. His substance use — Owner: Sequencing: problems that block other work.** With safety and basic needs secure, a problem that undermines counseling itself comes before his other goals.
- A. Career exploration — Owner: Sequencing: client's most pressing concern (comes later). His career goal matters, but substance use blocks progress on it.
- C. Long-term personal growth — Owner: Sequencing: longer-term growth and prevention (last). Growth work comes after problems that block counseling are addressed.
- **Pair to learn:** Substance use undermining therapy → Problems that block other work first (3A.3)

104 — C

Indirect Client Care · Task 5.7 · Chapter 23

- **C. Provide a qualified translation and review it together — Owner: ACA A.2.c culturally appropriate informed consent.** The Code calls for arranging qualified interpreters or translators so clients understand what they consent to.
- A. Have the client sign the English form to meet the rule — Owner: Signature without comprehension. A signature on a form the client cannot read does not make consent informed.
- B. Summarize the form aloud in simple English — Owner: Plain-language summary in English. Simple English still does not reach a client whose preferred language is not English.
- **Pair to learn:** Consent form, non-English preferred language → qualified translation, reviewed with the client (ACA A.2.c)

105 — B

Intake & Assessment · Task 2B.11 · Chapter 20

- **B. 4.5 — Owner: SEM = SD × √(1 − reliability).** SEM = 15 × √(1 − .91) = 15 × √.09 = 15 × .3 = 4.5.
- A. 1.35 — Owner: Error: SD × (1 − r) without the square root. 15 × .09 = 1.35 forgets to take the square root of (1 − reliability).
- C. 14.3 — Owner: Error: SD × √r. 15 × √.91 is about 14.3, which uses reliability instead of (1 − reliability); higher reliability should shrink the SEM.
- **Pair to learn:** SEM = SD × √(1 − r); higher reliability → smaller SEM

106 — C

Intake & Assessment · Task 2B.11 · Chapter 20

- **C. Rotter Incomplete Sentences Blank — Owner: Julian Rotter.** Rotter's Incomplete Sentences Blank is a projective test built on completing sentence stems.
- A. Thematic Apperception Test — Owner: Henry Murray and Christiana Morgan. The TAT asks clients to tell stories about ambiguous pictures; it is scored for needs and press.
- B. House-Tree-Person — Owner: John Buck. The House-Tree-Person is a projective drawing task, not sentence completion.
- **Pair to learn:** Completing sentence stems → Rotter Incomplete Sentences Blank (not TAT)

107 — A

Legal & Ethical Compliance · Task 6.15 · Chapter 17

- **A. Encrypted off-site backups and a records-access procedure — Owner: HIPAA contingency plan (45 CFR 164.308(a)(7)).** The Security Rule requires a data backup plan, disaster recovery plan, and emergency mode operation plan for electronic records.

- B. A flood rider on her professional liability policy — Owner: Property insurance vs. professional liability. Professional liability covers claims of negligence; insurance does not restore records or access to them.
- C. A rule to close the practice until records are rebuilt — Owner: Abandonment (ACA A.12). Stopping services without arranging care risks abandoning clients; the plan should keep care going.
- **Pair to learn:** Disaster could destroy records → off-site backup + recovery plan (HIPAA contingency plan)

108 — B

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **B. "Ana, I noticed you leaned in. Want to add anything?" — Owner: Drawing out.** It gently invites a quiet member in without forcing her to speak.
- A. "Ana, everyone else has shared, so now it's your turn." — Owner: Forcing a silent member to speak. Pressuring participation is punishing and ignores cultural norms about speaking.
- C. "Ana's situation sounds just like Ben's, I'm guessing." — Owner: Linking (premature). Linking connects members, but here it speaks for her before she has shared.
- **Pair to learn:** Inviting a quiet member in gently → drawing out

109 — B

Professional Development & Counselor Self-Awareness · Task 1B.5 · Chapter 22

- **B. Serve them, seeking training and consultation as needed — Owner: ACA A.4.b personal values.** A. 4.b directs counselors to avoid imposing values and to seek training where they risk doing so, while serving the couple's goals.
- A. Refer the couple elsewhere, citing the values conflict — Owner: Values-based referral (prohibited by

ACA A.11.b). A.11.b bars referring clients based solely on the counselor's personally held values.

- C. Share the counselor's beliefs so the couple can decide — Owner: Value imposition through self-disclosure. Disclosing the beliefs shifts the counselor's values onto the clients and harms the alliance.
- **Pair to learn:** Values conflict with a client → serve, with training and consultation (ACA A.4.b, A.11.b)

110 — C

Indirect Client Care · Task 5.8 · Chapter 17

- **C. Psychotherapy notes — Owner: HIPAA psychotherapy notes (extra protection; specific authorization).** Psychotherapy notes are kept separate from the clinical record and generally require specific authorization to disclose.
- A. The designated record set — Owner: HIPAA designated record set (clinical record). The designated record set is the clinical record itself: dates, diagnosis, plan, progress summaries.
- B. 42 CFR Part 2 records — Owner: 42 CFR Part 2 (federally assisted SUD program records). Part 2 protects substance use disorder program records, not a counselor's separate process notes.
- **Pair to learn:** Separate process notes with extra protection → HIPAA psychotherapy notes

111 — A

Intake & Assessment · Task 2B.5 · Chapter 19

- **A. A delusion, under thought content — Owner: Delusion (thought content).** A delusion is a fixed false belief held despite evidence. Delusions are recorded under thought content, which covers what the client thinks.
- B. An obsession, under thought content — Owner: Obsession. An obsession is an intrusive thought the person usually recognizes as excessive; this client is fully convinced.

- C. A hallucination, under perception — Owner: Hallucination (perception). A hallucination is a sensory experience with no stimulus present; this is a belief, not a sensory experience.
- **Pair to learn:** Fixed false belief despite evidence → Delusion (not obsession)

112 — C

Indirect Client Care · Task 5.8 · Chapter 23

- **C. Add a dated, signed addendum to the note — Owner: Record integrity (late entry / addendum).** A dated, signed addendum adds the information while keeping the original record intact.
- A. Rewrite the signed note to include the detail — Owner: Altering a signed record. Changing a signed note after the fact can look like falsifying the record.
- B. Add it to next week's note as new information — Owner: Misdated documentation. Entering it as new information next week misrepresents when the client said it.
- **Pair to learn:** Omission found after signing → dated, signed addendum, never rewrite

113 — A

Intake & Assessment · Task 2A.5 · Chapter 17

- **A. Agency policy allows it and she knows other options — Owner: ACA A.10.a (self-referral).** Agency counselors may refer clients to their own practices only when the agency allows it and clients are informed of other options.
- B. Her supervisor gives informal verbal approval — Owner: Consultation (useful, but not the A.10.a condition). Consulting is wise, but the standard turns on agency policy and the client's knowledge of alternatives.
- C. The client pays the same fee charged at the agency — Owner: Establishing fees (A.10.c), not the self-referral test. Fee setting is a separate standard;

matching the fee doesn't satisfy the self-referral conditions.

- **Pair to learn:** Agency client to counselor's private practice → Self-referral rules (ACA A.10.a)

114 — C

Treatment Planning & Continuity of Care · Task 3B.7 · Chapter 18

- **C. Goal Attainment Scaling — Owner: Goal Attainment Scaling (Kiresuk & Sherman).** Goal Attainment Scaling rates progress on goals tailored to each client.
- A. Reliable change index — Owner: Reliable change index (Jacobson & Truax). The reliable change index tests whether change exceeds measurement error.
- B. Outcome Rating Scale — Owner: ORS (Miller & Duncan; feedback-informed treatment). The ORS tracks general functioning session by session; it belongs to Miller and Duncan.
- **Pair to learn:** Individualized goal ratings → Goal Attainment Scaling (Kiresuk & Sherman)

115 — A

Legal & Ethical Compliance · Task 6.16 · Chapter 17

- **A. Work only through the practice's encrypted remote record access — Owner: HIPAA Security Rule; ACA B.6.b.** Authorized, encrypted access keeps the note within the practice's safeguards with no stray copies.
- B. Email the draft to a personal account to finish at home — Owner: Unsecured transmission (ACA B.3.e). Personal email moves client information outside protected systems; ACA B.3.e requires precautions for any transmission.
- C. Save the draft to the home computer and delete it after upload — Owner: Residual local copies (HIPAA device and media controls). Files saved to a personal

computer can persist after deletion and sit outside the practice's controls.

- **Pair to learn:** Finishing notes off-site → only through secure, authorized record access

116 — A

Provision of Counseling Interventions · Task 4.3 · Chapter 7

- **A. PREP (Markman, Stanley, & Blumberg) — Owner: PREP (speaker-listener technique).** The speaker-listener technique is PREP's structured communication tool.
- B. The Gottman Method — Owner: Gottman. Gottman's tools include softened startup, repair attempts, and love maps.
- C. Thomas Gordon's Parent Effectiveness Training — Owner: Gordon (I-messages). Gordon taught I-messages and active listening to parents.
- **Pair to learn:** Speaker-listener technique → PREP (not Gottman)

117 — B

Professional Development & Counselor Self-Awareness · Task 1B.6 · Chapter 22

- **B. Slow breathing and speech, then ask directly about suicide — Owner: Self-regulation before direct suicide inquiry.** Calming first lets the counselor ask clearly and directly; asking about suicide does not increase risk.
- A. Reassure her that her family loves her, then move on — Owner: Premature reassurance. Reassurance dismisses the statement and skips the risk inquiry it calls for.
- C. Shift topics until the counselor feels calmer again — Owner: Counselor avoidance (anxiety-driven). Avoiding the statement lets the counselor's anxiety leave risk unassessed.

- **Pair to learn:** Alarm at a suicidal statement → regulate, then ask directly

118 — B

Intake & Assessment · Task 2B.1 · Chapter 10

- **B. Remembrance and mourning — Owner: Herman (stage 2 of trauma recovery).** After safety and stabilization, Herman's second stage is remembrance and mourning: telling the trauma story and grieving its losses.
- A. Safety and stabilization — Owner: Herman (stage 1 of trauma recovery). She has already completed stage 1; telling and grieving the story marks stage 2.
- C. Processing the pain of grief — Owner: Worden (second task of mourning). This is Worden's task for mourning a death, not a stage of Herman's trauma recovery model.
- **Pair to learn:** Telling and grieving the trauma story → remembrance and mourning (Herman)

119 — C

Provision of Counseling Interventions · Task 4.1 · Chapter 2

- **C. Milan systemic therapy — Owner: Selvini Palazzoli, Boscolo, Cecchin, Prata (Milan).** These are the Milan team's guiding principles, along with circular questioning and positive connotation.
- A. Bowen family systems therapy — Owner: Bowen. Bowen's terms are differentiation, triangles, and multigenerational transmission.
- B. MRI brief strategic therapy — Owner: Watzlawick, Weakland, Jackson, Fisch (MRI). MRI focuses on attempted solutions that maintain the problem and on second-order change.
- **Pair to learn:** Hypothesizing, circularity, neutrality → Milan (not MRI)

120 — C

Indirect Client Care · Task 5.1 · Chapter 23

- **C. With her release, align one safety plan with the advocate — Owner: Collaboration with an advocate on client goals.** With her written consent, coordinating with the advocate keeps one safety plan that is consistent and centered on her choices.
- A. Call the advocate for updates without asking the client — Owner: Disclosure without consent (ACA B. 1.c). Contacting the advocate without her consent breaches confidentiality, and many domestic violence programs can share information only with the survivor's written release.
- B. Leave all safety planning to the advocate's program — Owner: Siloed care (role abdication). Safety stays a shared concern; handing it off entirely leaves gaps between the two services.
- **Pair to learn:** IPV client with an advocate → signed release, one client-directed safety plan

121 — B

Provision of Counseling Interventions · Task 4.2 · Chapter 15

- **B. Spiritual emergency — Owner: Stanislav & Christina Grof.** The Grofs described spiritual emergency: intense spiritual experiences that can resemble psychosis.
- A. Existential isolation — Owner: Yalom (ultimate concerns). Existential isolation is loneliness that persists even among people, one of Yalom's ultimate concerns.
- C. Spiritual bypassing — Owner: Welwood. Spiritual bypassing is using spiritual ideas to avoid emotional pain, not an overwhelming spiritual experience.
- **Pair to learn:** Intense spiritual experience resembling psychosis → spiritual emergency (Grof)

122 — C

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **C. Attend and listen before trying to influence — Owner: Ivey (microskills hierarchy).** Ivey's sequence is attending, then listening, then influencing; you earn the right to influence.
- A. Share a similar personal story to bond — Owner: Self-disclosure (influencing skill). Self-disclosure is also an influencing skill and can shift focus to the counselor.
- B. Offer an interpretation to show expertise — Owner: Interpretation (influencing skill). Interpretation is an influencing skill and is premature early in counseling.
- **Pair to learn:** Attend → listen → influence → Ivey

123 — A

Legal & Ethical Compliance · Task 6.17 · Chapter 17

- **A. Explain that treating counselors do not give forensic opinions — Owner: ACA E.13.c (client evaluation prohibited).** Counselors do not evaluate current or former clients for forensic purposes; the counselor may testify only to facts of treatment if compelled.
- B. Agree, since the counselor knows the client best — Owner: Dual role (treating and forensic). A treating relationship is built on advocacy, not the objectivity a custody opinion requires.
- C. Agree once the client signs a release for the testimony — Owner: Consent misread as curing a role conflict. A release permits disclosure; it does not make a forensic opinion about one's own client appropriate.
- **Pair to learn:** Client wants custody testimony → no forensic opinions about one's own client (ACA E.13.c)

124 — B

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **B. “So the rent went up, and now you’re working weekends.” — Owner: Paraphrase (basic listening sequence, Ivey).** It restates the facts in fresh words without naming an emotion or value.
- A. “Having a stable home really matters to you.” — Owner: Reflection of meaning. It names the value underneath the statement.
- C. “You sound stretched thin and anxious about money.” — Owner: Reflection of feeling. It names an emotion under the content.
- **Pair to learn:** Restates facts → paraphrase (not reflection of feeling)

125 — B

Professional Development & Counselor Self-Awareness · Task 1B.7 · Chapter 22

- **B. Skills, availability, and no conflict of interest — Owner: Client–counselor suitability (competence, availability, no conflict).** Suitability rests on whether the counselor has the needed skills, can be available at the needed level, and has no conflicting tie.
- A. Shared background and interests with the client — Owner: Similarity matching. Similarity can help rapport but is not the basis for determining suitability.
- C. Whether insurance pays the counselor’s full fee — Owner: Fee-driven intake decisions. Payment matters to business, but it says nothing about whether the counselor can serve the client well.
- **Pair to learn:** Suitability → skills, availability, no conflict of interest

126 — C

Professional Development & Counselor Self-Awareness · Task 1B.11 · Chapter 22

- **C. Address her self-care and raise it in supervision — Owner: Self-awareness and self-care (1B.11).** An exhausted counselor's presence suffers. Noticing it, attending to self-care, and using supervision are the counselor's responsibility.
- A. Tell her clients she is burned out and on edge — Owner: Self-disclosure that burdens the client. This meets the counselor's needs and shifts the focus and burden onto clients.
- B. Push through, since her skills are still adequate — Owner: Ignoring impairment. Her presence is already suffering, which affects clients regardless of her skill level.
- **Pair to learn:** Counselor fatigue affecting presence → Self-care and supervision (1B.11)

127 — B

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 9

- **B. ASAM Criteria — Owner: ASAM Criteria (American Society of Addiction Medicine).** The ASAM Criteria's six dimensions guide placement at the least restrictive level of care that's safe.
- A. SBIRT — Owner: SBIRT (screening, brief intervention, referral to treatment). SBIRT is a public health process for identifying and responding to risky use, not a six-dimension placement framework.
- C. Addiction Severity Index — Owner: ASI (measures addiction severity). The ASI assesses severity; it isn't the level-of-care placement framework.
- **Pair to learn:** Six dimensions for level of care → ASAM Criteria

128 — C

Legal & Ethical Compliance · Task 6.18 · Chapter 17

- **C. Add a dated, signed amendment that keeps the original visible — Owner: ACA A.1.b (amendments noted per policy).** Corrections are made by transparent amendment, preserving the original entry in the audit trail.
- A. Delete the note and re-create it with the correct date — Owner: Record alteration (erases the original entry). Deleting signed entries hides the original and undermines the record's integrity.
- B. Leave the error, since signed notes can never be changed — Owner: Rigid-record myth. Signed notes can be corrected; leaving a known error keeps the record inaccurate.
- **Pair to learn:** Error in a signed EHR note → dated amendment, original preserved (ACA A.1.b)

129 — A

Indirect Client Care · Task 5.2 · Chapter 17

- **A. Tell the client about the team and what is shared — Owner: ACA B.3.b (interdisciplinary teams).** Counselors inform clients of the team's existence and composition and what information will be shared.
- B. Share the full record so care is coordinated — Owner: Minimal disclosure ignored (ACA B.2.e; B.3.b teams). Only the information needed should be shared, and the client should know about it first.
- C. Keep all client information from the team — Owner: Absolute confidentiality (ignores the B.3 team model). Team care allows appropriate sharing once the client is informed.
- **Pair to learn:** Interdisciplinary team → Inform client of team and what's shared (ACA B.3.b)

130 — C

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **C. Interpretation — Owner: Interpretation (influencing skill).** It tentatively proposes a connection to the past that the client hasn't made.
- A. Reframe — Owner: Reframe (influencing skill). A reframe offers a new angle on the same facts; it doesn't link the present to the past.
- B. Reflection of meaning — Owner: Reflection of meaning (listening skill). Reflection of meaning names the value underneath; it doesn't propose a new connection.
- **Pair to learn:** Proposes a connection outside awareness → interpretation (not reframe)

131 — A

Professional Development & Counselor Self-Awareness · Task 1B.8 · Chapter 22

- **A. Complete instrument-specific training before using it — Owner: ACA E.2.a limits of competence.** Competence with a structured instrument requires training on that instrument.
- B. Use it, since purchase eligibility confirms competence — Owner: Purchase-qualification fallacy. Publisher qualification levels set a minimum to buy, not proof of skill with a given test.
- C. Use it, but leave the scores out of the client's record — Owner: Record-keeping evasion (ACA A.1.b). Leaving results out of the record does not fix the competence gap and violates documentation duties.
- **Pair to learn:** Eligible to buy ≠ trained to use → instrument-specific training (ACA E.2.a)

132 — A

Provision of Counseling Interventions · Task 4.2 · Chapter 22

- **A. "I'd like to understand what this fast means for you." — Owner: Cultural humility (Hook et al.).** Cultural humility shows curiosity, openness, and awareness of the limits of one's knowledge, inviting the client's own meaning.
- B. "I've read about that fast. It's mainly about discipline." — Owner: Assuming (cultural encapsulation). Telling the client what her practice means imposes the counselor's assumptions.
- C. "A counselor who shares your faith might suit you better." — Owner: Premature referral. Difference alone isn't a reason to refer; it skips the counselor's responsibility to learn.
- **Pair to learn:** Invites the client's cultural meaning → Cultural humility (not assuming)

133 — B

Legal & Ethical Compliance · Task 6.19 · Chapter 17

- **B. Verify and edit it, remaining accountable for its content — Owner: Clinical accountability for documentation (ACA A.1.b).** The counselor checks AI output against the assessment and record and remains responsible for what is signed.
- A. Sign it as written, since the vendor answers for its accuracy — Owner: Automation bias (misplaced accountability). Vendors do not assume clinical responsibility; deferring to AI output risks errors in the record.
- C. Label it AI-generated so that liability shifts to the tool — Owner: Disclosure misread as transferring responsibility. Labeling the source may be useful, but it does not shift accountability for the clinical content.
- **Pair to learn:** AI-drafted documentation → counselor verifies and stays accountable

134 — C

Intake & Assessment · Task 2B.5 · Chapter 19

- **C. Flat — Owner: Flat affect (virtually no expression).** Flat affect means virtually no emotional expression, the far end of the range of affect.
- A. Incongruent — Owner: Incongruent affect (mismatch with mood or content). Incongruent affect doesn't match the client's stated mood or the content of what is said; it describes fit, not the amount of expression.
- B. Blunted — Owner: Blunted affect (significantly reduced expression). Blunted affect is significantly reduced expression, but some expression remains.
- **Pair to learn:** Virtually no expression → Flat affect (not blunted)

135 — B

Intake & Assessment · Task 2B.12 · Chapter 19

- **B. Unspecified depressive disorder — Owner: Unspecified disorder (common in emergency settings).** Unspecified is used when the clinician chooses not to, or can't, specify why full criteria aren't met. It is common in emergency settings.
- A. Provisional major depressive disorder — Owner: Provisional diagnosis. Provisional requires a strong presumption that full criteria for a specific disorder will be met; this counselor has no such basis yet.
- C. Other specified depressive disorder — Owner: Other specified disorder. Other specified requires the clinician to state why criteria aren't fully met, which there is no time to do here.
- **Pair to learn:** Can't or won't specify why criteria aren't met → Unspecified (not other specified)

136 — C

Professional Development & Counselor Self-Awareness · Task 1B.9 · Chapter 22

- **C. Decline the second role and refer to a reading specialist — Owner: Avoiding a second professional role (ACA A.6) with referral.** Declining protects the counseling relationship, and a referral still meets the family's real need.
- A. Accept, since knowing her well may speed her reading progress — Owner: Rationalized multiple relationship. A plausible benefit does not offset the risk of blending a paid tutoring role with counseling.
- B. Accept if tutoring is held at the office and billed separately — Owner: Structural fix that keeps the dual role. Changing the location and billing leaves the counselor in two roles with the same family.
- **Pair to learn:** Family asks for a second paid role → decline and refer

137 — A

Intake & Assessment · Task 2B.11 · Chapter 18

- **A. Chi-square — Owner: Chi-square (frequencies in nominal categories).** Completed versus dropped out, by referral source, is a set of frequency counts in categories, which is what chi-square analyzes.
- B. Pearson r — Owner: Pearson r (relationship between two interval/ratio variables). Pearson r measures a relationship between two continuous variables, not frequencies in categories.
- C. One-way ANOVA — Owner: One-way ANOVA (three or more group means, one IV). ANOVA compares group means on an interval or ratio score; these data are counts, not means.
- **Pair to learn:** Nominal frequencies in categories → Chi-square

138 — B

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **B. Multifinality — Owner: Systems concept (multifinality).** The same starting point leading to different outcomes is multifinality.
- A. Circular causality — Owner: Systems concept (circular causality). Circular causality is behaviors causing each other in loops.
- C. Equifinality — Owner: Systems concept (equifinality). Equifinality is different starting points leading to the same outcome.
- **Pair to learn:** Same start, different outcomes → multifinality (not equifinality)

139 — A

Legal & Ethical Compliance · Task 6.1 · Chapter 17

- **A. Call 911 and stay with the client, per the medical emergency plan — Owner: Practice emergency plan (medical emergency).** New acute cardiac-type symptoms activate the plan: summon emergency services, stay with the client, and have staff meet responders.
- B. Lead a grounding exercise to see whether the pain eases — Owner: Diagnostic overshadowing (error pattern). Treating new physical symptoms as anxiety delays urgent medical care that the symptoms call for.
- C. Advise the client to see a physician after the session ends — Owner: Routine medical referral (wrong level of urgency). A routine referral fits non-urgent concerns; possible cardiac symptoms require an emergency response now.
- **Pair to learn:** Sudden chest pain in session → 911 per the medical emergency plan

140 — C

Provision of Counseling Interventions · Task 4.5 · Chapter 22

- **C. Instillation of hope — Owner: Yalom: instillation of hope.** Seeing someone further along improve gives newer members hope that change is possible; leaders foster this factor deliberately.
- A. Universality — Owner: Yalom: universality. Universality is the relief of learning one isn't alone in a problem; the focus here is evidence of improvement.
- B. Altruism — Owner: Yalom: altruism. Altruism benefits the member who helps others, here the speaker, not the newer members listening.
- **Pair to learn:** Seeing others improve → Instillation of hope (Yalom), not universality

141 — B

Professional Development & Counselor Self-Awareness · Task 1A.1 · Chapter 22

- **B. A colleague experienced in CBT for somatic symptoms — Owner: Expertise-matched consultation (ACA D.2.a consultant competency).** Useful consultation comes from someone competent in the exact skill in question — here, CBT for somatic presentations.
- A. The agency director, who has the most seniority — Owner: Authority bias (rank over expertise). Seniority does not mean expertise in this skill; consultation should target the specific gap.
- C. The client's physician, who manages the physical complaints — Owner: Interdisciplinary teamwork (ACA D.1.c). Coordinating with the physician (with a release) supports medical care, but the physician is not the source for CBT technique.
- **Pair to learn:** Specific skill gap → consultant with that specific expertise

142 — A

Intake & Assessment · Task 2B.2 · Chapter 6

- **A. The Danger Assessment — Owner: Jacquelyn Campbell.** Campbell's Danger Assessment is the lethality risk instrument for intimate partner violence.
- B. The Power and Control Wheel — Owner: Domestic Abuse Intervention Programs (Duluth). The Power and Control Wheel describes abusive tactics; it is a model, not a lethality instrument.
- C. The C-SSRS — Owner: Posner and colleagues (Columbia-Suicide Severity Rating Scale). The C-SSRS rates suicide severity, not lethality risk from a violent partner.
- **Pair to learn:** IPV lethality instrument → Danger Assessment (Campbell), not C-SSRS (Posner)

143 — A

Treatment Planning & Continuity of Care · Task 3B.2 · Chapter 21

- **A. Discharge summary — Owner: Discharge summary (3B.2).** The discharge summary documents why services began, what was done, how the client progressed, and what comes next.
- B. Treatment plan — Owner: Treatment plan (goals, objectives, interventions). A treatment plan guides future care; it isn't the end-of-services record.
- C. BIRP note — Owner: BIRP progress note (behavior, intervention, response, plan). A BIRP note documents a single session, not the whole course of treatment.
- **Pair to learn:** Reason, treatment, progress, status, recommendations → Discharge summary

144 — C

Legal & Ethical Compliance · Task 6.2 · Chapter 17

- **C. Explain the retention policy and how the records are secured — Owner: Practice record-retention SOP (state/federal law; ACA B.6).**

Records are kept for the period that law and policy set, which varies by state; explaining safeguards addresses the client's worry.

- A. Shred the notes, since clients own their clinical records — Owner: Record-ownership myth (HIPAA 45 CFR 164.524, 164.526). Clients have rights to access records and request amendments, not to have the practice destroy them.
- B. Rewrite the notes to remove the details the client dislikes — Owner: Record alteration (ACA A.1.b). Changing original entries to suit a client misrepresents the record of care that was provided.
- **Pair to learn:** Client asks to destroy records → explain retention policy and safeguards

145 — B

Treatment Planning & Continuity of Care · Task 3A.9 · Chapter 21

- **B. Involve his parents while honoring his goals — Owner: Collaborative goal-setting with minors (3A.9).** With minors, counselors involve parents while respecting the young client's voice.
- A. Adopt the parents' goal, since they consented — Owner: Parental consent treated as goal ownership. Consent doesn't make the parents the only voice in goal-setting; ignoring his goal weakens collaboration.
- C. Adopt only his goal and leave the parents out — Owner: Excluding parents (ignores their legal rights, ACA B.5.b). Parents have legal rights the counselor respects, even while protecting the minor's voice.
- **Pair to learn:** Minor and parents disagree on goals → Involve parents, respect the youth's voice (3A.9)

146 — A

Provision of Counseling Interventions · Task 4.8 · Chapter 22

- **A. Use his own language for his identity — Owner: Respect for diversity: the client's language for**

identity (4.8). Respect and acceptance for diversity includes using the client's name, pronouns, and language for his identity.

- B. Treat his hearing loss as the presenting problem — Owner: Pathologizing identity (ACA E.5.c concerns). He described a community and identity, not a complaint; framing it as the problem pathologizes it.
- C. Use the referral's clinical term for accuracy — Owner: Imposing outside terminology. Choosing the referral's term over the client's own description puts the counselor's frame ahead of his identity.
- **Pair to learn:** Client names his identity → Use the client's own language (4.8)

147 — A

Professional Development & Counselor Self-Awareness · Task 1A.2 · Chapter 22

- **A. Review the plan with the assigned supervisor before using it — Owner: Clinical supervision (supervisor oversight and accountability).** The supervisor shares responsibility for the supervisee's cases, so a new presentation should be reviewed with that supervisor before the plan is used.
- B. Have a former classmate informally approve the plan — Owner: Peer consultation (no evaluative authority). Peer consultation can add ideas but does not replace the assigned supervisor, who holds evaluative and legal responsibility.
- C. Finalize it and mention the case at a later supervision meeting — Owner: After-the-fact reporting. Informing the supervisor after decisions are made removes the oversight supervision is meant to provide.
- **Pair to learn:** Supervisee, unfamiliar diagnosis → assigned supervisor, not peer consultation

148 — C

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **C. Self-efficacy — Owner: Bandura (social cognitive).** Modeling, rehearsal, and mastery experiences build Bandura's self-efficacy.
- A. Unconditional self-acceptance — Owner: Ellis (REBT). Ellis targets irrational beliefs and self-acceptance, not confidence from modeling and rehearsal.
- B. Reciprocal inhibition — Owner: Wolpe (systematic desensitization). Reciprocal inhibition pairs relaxation with anxiety cues; no relaxation hierarchy is used here.
- **Pair to learn:** Modeling + rehearsal → self-efficacy (Bandura)

149 — B

Legal & Ethical Compliance · Task 6.3 · Chapter 17

- **B. Deny in writing and explain her right to file a disagreement — Owner: HIPAA right to amend (45 CFR 164.526).** A provider may deny amendment of accurate, complete information but must respond in writing, generally within 60 days, and explain her right to submit a statement of disagreement.
- A. Delete the diagnosis, since clients control their records — Owner: Record alteration (misreading the amendment right). HIPAA gives a right to request amendment, not to remove accurate entries; altering an accurate record undermines its integrity.
- C. Refer her to the licensing board, which decides amendments — Owner: State licensing board (complaints, not amendments). Licensing boards handle complaints about conduct; amendment requests are answered by the provider holding the record.
- **Pair to learn:** Request to delete an accurate diagnosis → HIPAA amendment denial + statement of disagreement (164.526)

150 — B

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **B. A coping question — Owner: de Shazer & Berg (SFBT).** Coping questions are an SFBT technique that draws out the client's existing strengths and resources.
- A. A unique outcome question — Owner: White & Epston (narrative). Unique outcomes are times the externalized problem didn't win; no problem is being externalized here.
- C. A scaling question — Owner: de Shazer & Berg (SFBT). A scaling question asks for a 0 to 10 rating; this question asks how she copes.
- **Pair to learn:** Coping question → SFBT (de Shazer & Berg)

151 — A

Legal & Ethical Compliance · Task 6.4 · Chapter 9

- **A. Its uses, reply times, and no emergency use — Owner: Electronic communication in informed consent (ACA H.2.a; Chapter 9).** Clients need to know how texts will and won't be used, how fast to expect replies, and that texting isn't an emergency channel.
- B. That texts are fully secure and private — Owner: Guaranteed security (no system is completely secure). Counselors inform clients of the limits of confidentiality with technology rather than promising full security.
- C. That she should text if she feels suicidal at night — Owner: Texting as an emergency channel (contrary to H.2.a emergency procedures). Emergency procedures must not depend on a counselor seeing a text.
- **Pair to learn:** Texting between sessions → Consent covers uses, response time, not for emergencies

152 — C

Professional Development & Counselor Self-Awareness · Task 1A.3 · Chapter 22

- **C. Contact the board about its options before attesting — Owner: Honest reporting; board-specific remedies.** Boards differ in remedies such as extensions or inactive status, so the counselor asks before attesting and reports accurately.
- A. Attest to full completion and finish the hours afterward — Owner: False attestation (ACA C.4.a misrepresentation). Attesting to hours not yet completed misrepresents qualifications and can bring board discipline.
- B. Count recent journal reading that earned no CE credit — Owner: Non-qualifying activity claimed as CE. Only activities that meet the board's CE rules count; uncredited reading does not.
- **Pair to learn:** Short on CE at renewal → contact the board; never attest falsely

153 — A

Professional Development & Counselor Self-Awareness · Task 1A.5 · Chapter 22

- **A. Public arena: social/political advocacy — Owner: ACA Advocacy Competencies, public arena level (Lewis et al., 2002; Toporek & Daniels, 2018).** Working to change law or public policy for many people is social/political advocacy, the public arena level of the Advocacy Competencies (consistent with ACA A.7.a).
- B. School/community: systems advocacy — Owner: ACA Advocacy Competencies, school/community level. Systems advocacy targets barriers within the counselor's own school, agency, or community system — not state legislation.
- C. Client/student: advocacy for one client — Owner: ACA Advocacy Competencies, client/student level.

Client/student-level advocacy acts with or for a specific client, such as negotiating with an agency on that client's behalf.

- **Pair to learn:** Testifying for legislation → public arena, social/political advocacy (Toporek et al.)

154 — A

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **A. The bond — Owner: Bordin (working alliance: bond).** Distrust or distance from the counselor is a bond problem, repaired through empathy, reliability, and openness about ruptures.
- B. Unconditional positive regard — Owner: Rogers (core conditions). This is a Rogerian core condition, not one of Bordin's three alliance parts.
- C. Agreement on goals — Owner: Bordin (working alliance: goals). The client agrees with what they're working toward.
- **Pair to learn:** Distrust of the counselor → Bordin: bond

155 — A

Legal & Ethical Compliance · Task 6.6 · Chapter 17

- **A. Review board rules and self-report within any required period — Owner: State board self-reporting rules.** Many boards require licensees to report arrests, charges, or convictions within a set period; what must be reported, and when, varies by state.
- B. Report the matter only if it ends in a conviction — Owner: Conviction-only assumption (varies by state). Some boards require reporting of charges, not just convictions, so waiting for an outcome risks a separate reporting violation.
- C. Disclose the charge to current clients at their next session — Owner: Counselor-focused self-disclosure (error pattern). The reporting duty runs to the board;

telling clients shifts the session's focus onto the counselor without meeting that duty.

- **Pair to learn:** Arrest or charge → check board rules and self-report as required

156 — C

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 6

- **C. Stay with her and arrange an immediate hospital evaluation — Owner: High-risk response (Chapter 6: matching level of care to risk).** A specific plan, intent, and access to means signal high risk: don't leave her alone, and arrange immediate evaluation, voluntary if possible.
- A. Increase sessions to twice weekly and involve her partner — Owner: Moderate-risk response (Chapter 6). More frequent sessions fit moderate risk, not a client with plan, intent, and means.
- B. Write a safety plan together and see her again next week — Owner: Low-risk response (under-responds to high risk). A safety plan alone with a week's wait doesn't match a specific plan, intent, and means.
- **Pair to learn:** Plan, intent, means → Stay with client; immediate evaluation for hospitalization

157 — B

Professional Development & Counselor Self-Awareness · Task 1A.4 · Chapter 22

- **B. It is not supported and may worsen outcomes for some — Owner: Cochrane review (Rose et al.); trauma practice guidelines.** Reviews find single-session debriefing does not prevent PTSD and may worsen outcomes for some; trauma-focused CBT is favored for persistent symptoms.
- A. It is effective, given its wide use by crisis teams — Owner: Bandwagon error (adoption is not evidence). How often a method is used says nothing about whether controlled research supports it.

- C. It is effective if held within 72 hours of the event — Owner: Critical incident stress debriefing (Mitchell). CISD guidance placed debriefing within days of the event, but timing does not rescue an intervention that trials do not support.
- **Pair to learn:** Single-session debriefing → not supported (Cochrane, Rose et al.); trauma-focused CBT if symptoms persist

158 — C

Intake & Assessment · Task 2A.1 · Chapter 19

- **C. A structured interview such as the SCID-5 — Owner: Structured interview (SCID-5, First et al.).** Structured interviews ask fixed questions in a fixed order, which gives them high reliability and makes them a good fit for research and diagnosis.
- A. A semi-structured interview with open follow-up — Owner: Semi-structured interview. Semi-structured interviews use core questions plus flexible follow-up, balancing reliability and flexibility rather than maximizing reliability.
- B. An unstructured interview that follows the client — Owner: Unstructured interview. Unstructured interviews build rapport and flexibility but have lower reliability and may miss information.
- **Pair to learn:** Fixed questions in fixed order → Structured interview (SCID-5), not unstructured

159 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. “Are you having thoughts of killing yourself?” — Owner: Direct closed question (standard of care).** Asking directly and in plain language is correct and doesn’t plant the idea.
- A. “You’re not thinking of hurting yourself, are you?” — Owner: Leading question with a euphemism. It signals

the expected answer, and “hurting yourself” may not be heard as suicide.

- C. “What do you mean by ‘everything’?” — Owner: Clarification. Clarifying is useful, but it doesn’t directly assess suicide.
- **Pair to learn:** Ask directly about suicide; avoid euphemisms

160 — C

Legal & Ethical Compliance · Task 6.7 · Chapter 17

- **C. Verify current eligibility and discuss fee or coverage options — Owner: Eligibility verification; ACA A.10.c (establishing fees).** Confirming coverage and openly discussing options such as an adjusted fee or new coverage keeps care going; A.10.c asks counselors to consider clients’ finances.
- A. Keep billing the former plan until claims start being denied — Owner: Billing a terminated plan (insurance fraud). Submitting claims to a plan that no longer covers the client is a false claim, whatever the outcome.
- B. Pause services until the client has new insurance in place — Owner: Abrupt interruption of care (ACA A. 11.c). A.11.c permits ending services for nonpayment only with pretermination counseling and referrals; a unilateral pause skips that discussion.
- **Pair to learn:** Coverage lost mid-treatment → verify eligibility and discuss fee options (ACA A.10.c)

161 — B

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **B. Assess her suicide risk directly — Owner: ACA AI Work Group: AI is never the sole basis for risk decisions.** The counselor is responsible for clinical judgment; goodbye letters are preparatory behavior that calls for the counselor’s own direct risk assessment, whatever the app says.

- A. Rely on the app's low-risk rating — Owner: AI as sole basis for a risk decision (contrary to ACA AI Work Group). AI output can't replace the counselor's judgment, especially when the client reports a clear warning sign.
- C. Hospitalize her involuntarily at once — Owner: Over-response without assessment (least restrictive care, Chapter 6). Level of care should follow an assessment of risk, which hasn't happened yet.
- **Pair to learn:** AI risk rating vs. clinical warning sign → Counselor's own assessment (AI not sole basis)

162 — A

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **A. Stay with her and arrange a hospital evaluation — Owner: High risk → immediate evaluation, involuntary if she refuses.** A specific plan, intent, and access to means is high risk: she shouldn't be left alone, and refusal of voluntary care calls for an immediate evaluation for involuntary hospitalization.
- B. Ask her to sign a no-suicide contract — Owner: No-suicide contract (no supporting evidence). Contracts lack evidence and give false reassurance.
- C. Complete a safety plan and see her next week — Owner: Low/moderate-risk response (under-responding). A safety plan alone under-responds to plan, intent, and means.
- **Pair to learn:** Plan + intent + means → high risk: stay with client, immediate evaluation

163 — A

Legal & Ethical Compliance · Task 6.8 · Chapter 17

- **A. A good faith estimate of expected charges — Owner: No Surprises Act good faith estimate (45 CFR 149.610).** Providers must give uninsured or self-pay clients a good faith estimate of expected charges when services are scheduled or on request.

- B. A superbill for out-of-network reimbursement — Owner: Superbill (client-requested billing statement). A superbill helps a client seek out-of-network reimbursement; it is optional and does not replace the required estimate.
- C. An advance beneficiary notice of noncoverage — Owner: Medicare Advance Beneficiary Notice (ABN). An ABN warns Medicare beneficiaries that Medicare may not pay for a service; it does not apply to self-pay clients generally.
- **Pair to learn:** Self-pay client → good faith estimate (No Surprises Act)

164 — C

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **C. Super-reasonable stance — Owner: Satir (communication stances).** Satir's super-reasonable (computer) stance hides feeling behind detached logic.
- A. Emotional cutoff — Owner: Bowen. Emotional cutoff is distancing from family of origin to manage unresolved attachment, not a conversational style.
- B. Stonewalling — Owner: Gottman (Four Horsemen). Stonewalling is shutting down and withdrawing; he is still engaging, just without feeling.
- **Pair to learn:** Detached logic, no feeling → super-reasonable (Satir)

165 — B

Legal & Ethical Compliance · Task 6.9 · Chapter 17

- **B. The client's informed consent, plus any state-required consent — Owner: ACA B.6.c; state recording laws.** Counselors obtain client permission before recording; state law may add consent rules, and recordings are secured as records.
- A. The daughter's written permission as the family caregiver — Owner: Unauthorized proxy consent. A

relative without legal authority cannot consent for a client who can make her own decisions.

- C. Only confirmation that the platform is HIPAA compliant — Owner: Platform compliance (not consent to record). A compliant platform protects data but does not supply the client's permission to be recorded.
- **Pair to learn:** Recording a session → client's informed consent (ACA B.6.c) + state law

166 — C

Provision of Counseling Interventions · Task 4.2 · Chapter 15

- **C. Caitlin Ryan's Family Acceptance Project — Owner: Ryan (Family Acceptance Project).** Caitlin Ryan's Family Acceptance Project documented the risk of family rejection and the protective effect of acceptance.
- A. Ilan Meyer's minority stress model — Owner: Meyer (distal and proximal stressors). Meyer explains how external and internal minority stressors raise risk, not the specific effect of family acceptance.
- B. Anthony D'Augelli's life-span model — Owner: D'Augelli (LGB identity processes). D'Augelli described identity development through six interrelated processes.
- **Pair to learn:** Family acceptance protects LGBTQ youth → Caitlin Ryan (not Meyer)

167 — C

Indirect Client Care · Task 5.3 · Chapter 21

- **C. Use plain language and note the data's limits — Owner: Report writing (ACA E.9.a; 5.3).** Reports should be clear, free of jargon, limited to what's needed, and note limitations of the data.
- A. Present clinical opinions as established fact — Owner: Blurring fact and opinion. Good reports distinguish facts from clinical opinions.

- B. Include every test score and session detail — Owner: Excess disclosure (ACA B.2.e minimal disclosure). Reports share only what is necessary for the reader's purpose.
- **Pair to learn:** Report for another party → Clear, necessary, limits noted (ACA E.9.a)

168 — C

Legal & Ethical Compliance · Task 6.10 · Chapter 17

- **C. How the session served the client's plan goals — Owner: CPT 90846 (family psychotherapy without the patient).** The record should show who attended, why the patient was absent, and how the session advanced the identified client's treatment plan.
- A. That the spouse is also an active client of the practice — Owner: Conjoint vs. individual client confusion. The spouse need not be a client; 90846 is billed for the identified patient's treatment.
- B. The spouse's own diagnosis and treatment plan — Owner: Billing the family member as the patient. Documenting a diagnosis for the spouse would make the spouse the patient, which is a different service and claim.
- **Pair to learn:** Family session without the patient → document how it served the client's plan (90846)

169 — C

Professional Development & Counselor Self-Awareness · Task 1B.10 · Chapter 17

- **C. Consult with and support the colleague — Owner: ACA C.2.g, I.2 (assisting an impaired colleague).** Colleagues who notice signs of impairment consult with and assist the colleague, who may need to limit services until it's safe.
- A. File a complaint with the licensing board — Owner: Formal report (I.2.b; for substantial harm or failed informal resolution). A formal report fits substantial harm or failed informal efforts; neither applies yet.

- B. Say nothing, since grief is private — Owner: Ignoring impairment (contrary to C.2.g). Ignoring signs of impairment leaves clients at risk and the colleague unsupported.
- **Pair to learn:** Colleague showing impairment, no harm yet → Consult with and assist (C.2.g)

170 — A

Legal & Ethical Compliance · Task 6.11 · Chapter 17

- **A. One trend source alongside her self-reported symptoms — Owner: Supplemental wearable data (not a validated measure).** Consumer wearables can show useful trends but are not validated diagnostic tools; they supplement clinical assessment.
- B. A validated measure for diagnosing panic disorder — Owner: Overreliance on consumer device output. Diagnosis rests on clinical criteria and assessment, not on heart-rate readings from a consumer device.
- C. Information the counselor may not use under HIPAA — Owner: Misreading HIPAA (use for treatment is permitted). HIPAA permits using health information for treatment; once shared and documented, it is protected, not forbidden.
- **Pair to learn:** Smartwatch data → supplemental trend, not diagnostic

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Score by Domain

Domain	Items	Your correct	%
Professional Development & Counselor Self-Awareness	25		
Intake & Assessment	35		
Treatment Planning & Continuity of Care	23		
Provision of Counseling Interventions	39		
Indirect Client Care	17		
Legal & Ethical Compliance	31		

Domain	Items	Your correct	%
Total	**170**		

Scaled NCE scores are not percentages, and no practice percentage maps onto a pass or fail. Use this table to find your two weakest domains and give them extra study time next week.

Item numbers by domain

- **Professional Development & Counselor Self-Awareness:** 4, 6, 7, 16, 27, 38, 48, 59, 68, 70, 80, 91, 101, 109, 117, 125, 126, 131, 136, 141, 147, 152, 153, 157, 169
- **Intake & Assessment:** 1, 2, 5, 9, 12, 13, 15, 18, 20, 21, 23, 31, 34, 45, 49, 50, 52, 58, 60, 69, 71, 74, 81, 82, 98, 105, 106, 111, 113, 118, 134, 135, 137, 142, 158
- **Treatment Planning & Continuity of Care:** 11, 17, 22, 25, 28, 32, 42, 43, 53, 54, 64, 75, 76, 85, 89, 96, 97, 103, 114, 127, 143, 145, 156
- **Provision of Counseling Interventions:** 26, 29, 33, 39, 44, 47, 57, 61, 63, 65, 66, 73, 79, 84, 86, 90, 92, 94, 95, 100, 102, 108, 116, 119, 121, 122, 124, 130, 132, 138, 140, 146, 148, 150, 154, 159, 162, 164, 166
- **Indirect Client Care:** 8, 10, 19, 30, 36, 40, 51, 62, 72, 83, 93, 104, 110, 112, 120, 129, 167
- **Legal & Ethical Compliance:** 3, 14, 24, 35, 37, 41, 46, 55, 56, 67, 77, 78, 87, 88, 99, 107, 115, 123, 128, 133, 139, 144, 149, 151, 155, 160, 161, 163, 165, 168, 170

NCE Practice Exam 3

170 items · 3 options each · 3 hours 45 minutes · Answer every item.

Take this exam under timed conditions before checking the key. Use the four-step routine on every item: read the last line first, name the owner, predict, then eliminate by owner.

Pacing checkpoints: item 43 by 0:50 · item 85 by 1:39 (end of the first half: review it, submit it, take the optional break) · item 128 by 2:29 · item 170 by 3:18.

Original items written for this book. Not affiliated with or endorsed by NBCC. No content from any live examination.

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1. At session four, a client's Session Rating Scale shows a sharp drop in her alliance rating, though she says, "Everything's fine." The counselor's best response is to:

A. Offer to refer her to a colleague she might prefer
B. Mention the drop and ask what hasn't felt right to her
C. Accept her statement and continue with the planned work

2. A school counselor wants a substance use screen for a 15-year-old who was caught with alcohol at a dance. Which screen is recommended for adolescents?

A. The CRAFFT
B. The OQ-45
C. The CBCL

3. An overnight fire closes the office building, and clients are scheduled for sessions the next day. Under the practice's emergency plan, the counselor should:

A. Notify clients and hold sessions securely from the backup site

B. Cancel all sessions until the building reopens
C. Wait for clients to call, so the closure does not alarm them

4. A client's treatment goal is to reduce panic attacks. After a weekend workshop, her counselor is eager to try a new sand tray technique with her. The best course is to:

A. Choose interventions tied to her panic goal
B. Try the technique and see how she responds to it
C. Help her set a new goal that suits the technique

5. In a skills group, a client role-plays talking to a neighbor whose dog barks all night. She smiles and says, "Must be nice to sleep so soundly while the rest of us listen to your dog." Her response is best labeled:

A. Aggressive
B. Assertive
C. Passive-aggressive

6. After consulting a colleague about adapting interpersonal therapy for a client's role transition, how should the counselor use the advice?

A. Follow it exactly, since the consultant now shares responsibility
B. Weigh it against the counselor's own assessment and document it
C. Ask the consultant to co-sign the revised treatment plan

7. A 9-year-old has had frequent eye blinking, shoulder shrugging, and throat-clearing sounds for about 18 months. Which presentation best fits?

A. ADHD, combined presentation
B. Tourette's disorder
C. OCD with tic-related specifier

8. A client who recently lost her job asks whether she could do the counselor's bookkeeping in exchange for sessions. Under the ACA Code, this barter is acceptable only if:

A. It isn't exploitative, fits local norms, and is put in writing
B. The agency allows it and she is told about her other options
C. The counselor proposes the terms as a fair way to help

9. A client in early recovery wants his certified peer recovery specialist involved in his plan. The peer specialist's role is best described as:

A. Diagnosing and revising the treatment plan
B. Prescribing medications for opioid use disorder
C. Sharing lived experience to support recovery goals

10. A licensed counselor holds a master's degree in counseling and a PhD in medieval history. She wants to advertise herself as "Dr." in her counseling practice. Under the ACA Code, she:

A. May, since any earned doctorate allows the title in advertising
B. May not, since her doctorate isn't in a counseling-related field
C. May, as long as her NCC certification is listed next to it

11. Which theorist is associated with the finding that hopelessness predicts suicide more strongly than depression itself?

A. Thomas Joiner
B. David Jobes
C. Aaron Beck

12. A client wants to start with feeling like a failing parent, but the counselor sees her insomnia as more central. With no safety concern present, the counselor should:

A. Begin with her concern, share the sleep view, and revisit the order
B. Begin with the insomnia, since the counselor sets the order of focus
C. Postpone priorities until her symptoms ease enough for her to choose

13. After years of her husband monitoring her phone, controlling the money, and beating her, a woman struck him with a lamp during an attack. In Johnson's typology, her violence is best classified as:

A. Situational couple violence
B. Violent resistance
C. Intimate terrorism

14. A client with schizophrenia has stopped eating, sleeps outside in freezing weather, and cannot say how he would get food. He denies suicidal or homicidal thoughts and refuses help. Involuntary hospitalization would most likely rest on:

A. His poor insight into the illness
B. Grave disability
C. Imminent danger to self through suicidal intent

15. A client asks for a list of everyone outside the practice who has received his health information. HIPAA requires the counselor to:

A. Give him a full copy of his record instead of a list
B. Give an accounting that excludes treatment, payment, and operations
C. Decline, since disclosure logs are internal security records

16. Near the end of counseling, a counselor helps a client list early warning signs that depression is returning, coping steps to take, and whom to call. This termination task is:

A. Relapse prevention
B. Pretermination counseling
C. Booster sessions

17. Retirement, the birth of a child, and entering adolescence can each set off which type of crisis?

A. Situational
B. Existential
C. Developmental

18. A counselor's caseload has shifted toward older adults. The best basis for choosing this year's continuing education is:

A. Topics the counselor already knows well and enjoys
B. Whatever courses the employer offers at no cost
C. A review of current caseload needs and practice gaps

19. A woman has taken a prescribed benzodiazepine exactly as directed for a year. She now needs more for the same effect and feels shaky when a dose is late. She meets no other criteria. She has:

A. Mild sedative use disorder, with two criteria met
B. Sedative use disorder in early remission
C. No disorder; prescribed tolerance doesn't count

20. A counselor wants to show whether ten sessions of counseling reduced a client's depressive symptoms. Which plan best measures the outcome?

A. Rely on her own impression of how much the client has improved
B. Give the PHQ-9 at intake and again at the tenth session
C. Give the PHQ-9 once, at the tenth session, and compare it to norms

21. A 19-year-old client says she has had restlessness, poor sleep, and new suicidal thoughts since starting an SSRI two weeks ago. The counselor should first:

A. Assess risk, document, and contact the prescriber promptly
B. Reassure her these effects usually fade within a month
C. Advise her to skip doses until she sees her physician

22. Two days after a tornado destroys several neighborhoods, a counselor is sent to a shelter. Which approach fits best?

A. Prolonged exposure to process each survivor's memories
B. A required debriefing where each survivor retells the

event
C. Psychological first aid for safety and practical needs

23. A counselor wants a diagnostic interview that keeps a set of core questions for reliability but still lets her follow up when a client says something unexpected. Which format fits best?

A. A semi-structured interview
B. A structured interview such as the SCID-5
C. An unstructured, conversational interview

24. A client with PTSD after a car crash says she is motivated and has supportive family nearby, but she takes long detours to avoid the crash site and constantly scans traffic. Which pairing is correct?

A. Strength: her detour routes; barrier: her family's involvement
B. Strength: her family support; barrier: avoiding the crash route
C. Strength: scanning traffic; barrier: her motivation to change

25. A memory test has a mean of 60 and a standard deviation of 12. A client's raw score is 36. What are the client's z-score and the equivalent score on the deviation IQ scale?

A. $z = -2$; deviation IQ score of 70
B. $z = -2.4$; deviation IQ score of 64
C. $z = -2$; deviation IQ score of 80

26. In a SOAP note, which entry belongs in the Objective section?

A. Client says, "I can't get out of bed."
B. PHQ-9 score of 14; affect constricted
C. Progress toward Objective 2 is moderate

27. After renewing, a counselor is selected for a random continuing education audit by the state board. She is best prepared if she has:

A. Listed her course titles on the renewal application
B. Kept certificates only until her renewal was approved
C. Kept certificates for as long as board rules require

28. A client in counseling for anxiety learns that her application for disability benefits was denied, and she doesn't know how to appeal. Along with continuing counseling, the counselor should:

A. Advise her on the legal steps to file her appeal
B. Refer her to legal or benefits resources, then follow up
C. Call the benefits office to argue her case on her behalf

29. Which brief measures track a client's outcome and the alliance session by session?

A. The OQ-45 (Lambert)
B. The SCL-90-R and the BSI (Derogatis)
C. The ORS and SRS (Miller & Duncan)

30. A counselor finds only a few small trials on treating adjustment disorder. How should the counselor proceed in building knowledge of supported approaches?

A. Appraise the few trials and reviews that exist for quality
B. Treat it as having no evidence and rely on clinical intuition
C. Apply PTSD protocols, since both are stress-related disorders

31. A counselor who has seen a 15-year-old individually begins family sessions with the teen and both parents. Which shift does the new modality call for?

A. Promising each member that private disclosures stay private
B. Keeping the focus on the teen's intrapersonal insight
C. Viewing the teen's symptoms as part of a family pattern

32. A client arrives agitated and suspicious, with dilated pupils, a racing heart, and heavy sweating. Which is most likely?

A. Stimulant intoxication
B. Sedative-hypnotic intoxication
C. Opioid intoxication

33. An unemployed, uninsured client needs ongoing counseling the counselor cannot provide. The most useful resource on the referral list is:

A. An out-of-network private group practice
B. A sliding-scale community mental health center
C. An employee assistance program (EAP)

34. A man with alcohol use disorder has met no criteria except craving during 5 months in a locked residential facility. Which specifiers apply?

A. Early remission, on maintenance therapy
B. Early remission, in a controlled environment
C. Sustained remission, in a controlled environment

35. A client in a manic episode has not slept in four days, believes she can fly, and has been walking into traffic. Her family cannot keep her safe at home. Which level of care fits?

A. Inpatient hospitalization
B. Partial hospitalization
C. An intensive outpatient program

36. A functional assessment shows a client's binge drinking mostly follows late work shifts and paydays. The treatment plan should:

A. Keep general abstinence goals for every day of the week
B. Require her to quit her job, since work causes her drinking
C. Target those antecedents, such as a coping plan after late shifts

37. As a trauma survivor begins describing the assault, she goes numb, her voice flattens, and she says she feels "far away." Which concept best describes her state?

A. Hyperarousal outside her window of tolerance
B. Exhaustion stage of general adaptation
C. Hypoarousal outside her window of tolerance

38. Six weeks after her husband's death, a client feels sad and misses him, but she sleeps, works, and sees friends. She meets criteria for no disorder and wants support. How is the focus of counseling best documented?

A. Adjustment disorder with depressed mood
B. A Z code for uncomplicated bereavement
C. Unspecified depressive disorder

39. While a counselor's insurance panel application is pending, her group practice suggests billing her sessions under a credentialed colleague's NPI. This practice is best described as:

A. Misrepresenting the rendering provider on a claim
B. Acceptable incident-to billing for supervised staff
C. A permitted locum tenens billing arrangement

40. With her signed release, a client asks her counselor to coordinate with her primary care physician about her sleep problems. What should the counselor share?

A. Her psychotherapy notes, which explain session content
B. Her full counseling record, so the picture is complete
C. Only what the physician needs to help with her sleep

41. CACREP's approval of a university's master's program in clinical mental health counseling is an example of:

A. Accreditation
B. Licensure
C. Certification

42. An ethics training covers conflicts between the ACA Code and the law. When the two conflict, counselors should first:

A. Follow the Code and disregard the conflicting law
B. State their commitment to the Code and seek to re-

solve it
C. Comply with the law without raising the conflict at all

43. A client who has been severely depressed for months arrives smiling and says, “I finally feel at peace. Everything is settled.” She mentions she recently updated her will. The counselor should first:

A. Explore what helped her feel at peace this week
B. Ask her directly about thoughts of killing herself
C. Note the improvement and space sessions further apart

44. During chemotherapy, a counselor finds that she loses track of what clients said earlier in sessions and is too exhausted to focus. The most ethical response is to:

A. Close her practice and end all services by the end of the week
B. Seek help and limit or pause her caseload until she can resume
C. Keep her full caseload, since the illness isn’t her fault

45. A client on probation wants his probation officer to join the next session. During pre-session review, the counselor should confirm that:

A. The officer agrees to keep what he hears confidential
B. The officer has already read the prior notes
C. A current signed release names the officer and its scope

46. The 1998 study linking childhood adversity to later heart disease, substance use, and depression was conducted by:

A. Felitti and Anda
B. Judith Herman
C. Bessel van der Kolk

47. A 7-year-old who recently arrived from Guatemala and speaks mainly Spanish is referred for a special education evaluation. Which approach best follows the legal standard for testing her?

A. Give the English WISC-V, since it’s the most widely used test
B. Place her by ability group using last year’s group test scores
C. Test her in Spanish and add nonverbal measures before placement

48. A counselor reviews age, disability, religion, ethnicity, social class, sexual orientation, Indigenous heritage, national origin, and gender to bring cultural factors into a treatment plan. This framework is:

A. Hays’s ADDRESSING model
B. Sue’s tripartite model of multicultural competence
C. Bronfenbrenner’s ecological systems model

49. Define the problem, generate alternatives, decide, implement, and evaluate. This problem-solving sequence is credited to:

A. Jay Haley
B. James and Gilliland
C. D’Zurilla and Goldfried

50. Which instrument is a screen for posttraumatic stress that a counselor might use when assessing trauma at intake?

A. The PC-PTSD-5
B. The CRAFFT
C. The GAD-7

51. A counselor notices that the practice’s video platform uses one reusable meeting link for every client. The best action is to:

A. Keep the link, since the vendor signed a business associate agreement
B. Use unique session links or a locked waiting room for each client
C. Ask clients to keep cameras off until they see who is present

52. A family assessment model that rates families on cohesion, from disengaged to enmeshed, and on flexibility, from rigid to chaotic, was developed by:

A. Salvador Minuchin
B. David Olson
C. Nathan Epstein

53. Which type of program evaluation compares a program's costs with outcomes measured in nonmonetary units, such as the cost per client who improved?

A. Process evaluation
B. Cost-effectiveness analysis
C. Cost-benefit analysis

54. An AI decision-support tool rates a client as low suicide risk, but the counselor's interview raises concern. Training on ethical AI use would teach the counselor to:

A. Accept the tool's rating, since algorithms reduce clinician bias
B. Disregard all AI output in future clinical decisions
C. Rely on the full clinical assessment and document the discrepancy

55. A client with terminal cancer tells his counselor that he is thinking about ways to hasten his death. Under the ACA Code, the counselor:

A. Must break confidentiality, as she would for any other suicide risk
B. Should end counseling, since the decision is outside its scope
C. May keep it confidential after consulting and reviewing the law

56. A researcher wants to examine how treatment type (CBT or interpersonal therapy) and client gender each affect depression scores, and whether the two interact. The appropriate test is:

A. Factorial ANOVA
B. One-way ANOVA
C. ANCOVA

57. A counselor needs someone to convey a client's spoken words in another language in real time during sessions. The appropriate professional is a:

A. Qualified interpreter
B. Certified translator
C. Bilingual receptionist without training

58. A colleague confides that he has begun a sexual relationship with a current client and asks the counselor to keep it to herself. The counselor's best action is to:

A. Report it, since substantial harm to the client is likely
B. Talk with him privately and let him end it on his own terms
C. Keep his confidence, since he told her this as a friend

59. In Helms's White racial identity model, which status comes immediately after contact?

A. Reintegration
B. Disintegration
C. Encounter

60. When reviewing her diagnosis, a client asks the counselor to record a milder one so her record "looks better." The counselor should:

A. Record the milder diagnosis to protect her privacy
B. Explain the accurate diagnosis, its basis, and who sees it
C. Leave every diagnosis out of the record from now on

61. A counselor is considering telling a client's partner that the client has a contagious, life-threatening disease. Under the 2014 ACA Code, before disclosing, the counselor must:

A. Apply the Tarasoff duty to warn standard for violent threats

B. Assess whether the client intends to inform the partner
C. Confirm the diagnosis with the client's physician

62. At intake, a counselor hands a new client a finished treatment plan with goals she wrote in advance and asks him to sign it. The main problem is that:

A. Goals can't be set until a full ASAM review is done
B. Goals he didn't help choose conflict with his autonomy
C. The plan lists no barriers that could block his progress

63. A colleague suggests diagnosing a client with a more severe disorder because the client's plan covers more sessions for it. The counselor should:

A. Use the severe diagnosis but note doubts in the record
B. Ask the client which diagnosis he would prefer
C. Bill the diagnosis that the assessment supports

64. After a client meets her goals and ends regular counseling, the counselor schedules check-ins at one and three months to help her maintain her gains. These are:

A. Tapered sessions
B. Booster sessions
C. Relapse prevention

65. Sensitization, identity confusion, identity assumption, and commitment are stages in the model of:

A. Vivienne Cass
B. Anthony D'Augelli
C. Richard Troiden

66. A counselor plans training for a client population new to the counselor. Which option best builds competence?

A. A research-based course with skills practice and consultation
B. A one-hour webinar that grants a certificate of completion
C. A podcast of first-person recovery stories from that population

67. A counselor ended services with a client six years ago. They meet again at a community event and want to start dating. Under the ACA Code, the counselor:

A. Must still use forethought and document it isn't exploitative
B. Is permanently barred from dating this former client
C. Is free to proceed, since well over 2 years have passed

68. A counselor's HIPAA-compliant AI tool drafts her progress notes. One draft states that the client "denied suicidal ideation," but suicide never came up in that session. The counselor should:

A. Sign it as drafted, since the tool meets HIPAA requirements
B. Sign it now and add a dated correction to the note afterward
C. Fix the draft before signing; the note is her responsibility

69. A parent asks the counselor to coordinate with her son's school counselor about classroom strategies. After obtaining the parent's signed consent, the counselor should:

A. Send the full session notes to the school counselor
B. Share only what is needed for school strategies
C. Avoid contact, since school records fall under HIPAA

70. Whenever his wife raises a concern, a husband shoots back, "That's not my fault, you're the one who forgot." In Gottman's terms, the antidote the counselor would coach is:

A. Taking responsibility
B. A gentle startup
C. Physiological self-soothing

71. The terms "good-enough mother" and "holding environment" belong to which object relations theorist?

A. Donald Winnicott
B. John Bowlby
C. Melanie Klein

72. A family asks a counselor to help set up a location-tracking wearable for an older client with memory loss without telling him, to avoid upsetting him. The counselor should:

A. Agree, since family safety concerns override his consent
B. Refuse any tracking, since it always violates autonomy
C. Discuss it with him and seek his consent or assent

73. Which ACA Code standard calls for verifying a distance counseling client's identity, for example with code words?

A. H.4.c, technology-assisted services
B. H.3, client verification
C. H.1.b, laws and statutes

74. A counselor receives a subpoena from an attorney in a client's custody dispute requesting all counseling records. The client hasn't signed a release. The counselor's best first step is to:

A. Disregard the subpoena, since the client hasn't given consent
B. Contact the client and assert privilege on the client's behalf
C. Send the records, since a subpoena is a legal order to comply

75. After several high-risk sessions, a drained counselor notices a pull to accept a client's "I'd never do it" without further questions. What is best?

A. Finish the risk assessment, then seek consultation and support
B. Accept the denial and end the session early to recover energy
C. Refer the client to another counselor before assessing risk

76. An older client's daughter comes to the intake. With the client's consent, the counselor wants the daughter's view of how the family understands the problem and its cultural meaning. Which tool fits?

A. A 4 Ps formulation of the daughter's report
B. The CFI informant version
C. The WHODAS 2.0, filled out by the daughter

77. In the first meeting of a counseling group, a member asks whether everything said in the group will stay private. The leader's most accurate response is to explain that:

A. A signed group agreement guarantees that nothing will leave
B. Privileged communication protects what members say in court
C. She will protect it and ask members to, but can't guarantee it

78. A client is denied a workplace disability accommodation. With her consent, the counselor's most appropriate role is to:

A. File a legal complaint on her behalf
B. Write a support letter and link her to legal aid
C. Stay out of it, since advocacy is outside counseling

79. A family brings in their 12-year-old, Maya, for refusing to go to school. Which question best reflects a circular, systemic focus?

A. "Why do you think Maya keeps refusing to go to school lately?"
B. "When Maya stays home, who notices first? What does Dad do?"
C. "How many school days has Maya missed this month?"

80. A veteran says he can't forgive himself for an order he carried out overseas that went against everything he believed was right. This is best described as:

A. Military sexual trauma
B. Disenfranchised grief
C. Moral injury

81. A counselor receives notice that a client filed a complaint with the licensing board. Under a typical professional liability policy, the counselor's first step is to:

A. Call the client to resolve the complaint informally
B. Send the board a detailed response the same day
C. Notify the liability carrier promptly, per the policy

82. In an inpatient setting, when a client has more than one DSM-5-TR diagnosis, what is the condition chiefly responsible for the admission called?

A. The Axis I diagnosis
B. The provisional diagnosis
C. The principal diagnosis

83. The term psychache, meaning intolerable psychological pain, was introduced by:

A. Edwin Shneidman
B. Aaron Beck
C. Thomas Joiner

84. A counselor with a trauma-heavy caseload starts reading a client's ordinary low moods as signs of hidden danger. The best first step is to:

A. Check the reaction against client data and assess own exposure
B. Trust it as clinical intuition and add more frequent sessions
C. Tell the client about the caseload to explain the tension

85. A client scored 30 on a symptom measure at intake and 28 after eight sessions. The measure's SEM is 3. What is the most accurate interpretation?

A. The change is within one SEM, so it may reflect measurement error
B. A ceiling effect on the measure is hiding the client's

real change
C. The two-point drop confirms that counseling worked

86. A client wants help being more assertive at work. She mentions that her partner choked her last weekend and that she is afraid to go home tonight. The first treatment priority is:

A. Inviting her partner to join couples sessions
B. Assertiveness training for her concerns at work
C. Assessing lethality and safety planning with her

87. A parent reports her 9-year-old son has had reduced appetite and trouble falling asleep since starting a new medication. These effects are most typical of:

A. Second-generation antipsychotics
B. Stimulants
C. Benzodiazepines

88. At intake, a counselor wants to evaluate a client's level of mental health functioning. Which question does this most directly?

A. "How are these symptoms affecting your work and daily life?"
B. "How would you describe this problem to your family and friends?"
C. "From 0 to 100, how upsetting does this feel right now?"

89. A client with major depressive disorder has the long-term goal of returning to full functioning. Which entry is a short-term objective?

A. Client reports low mood and misses work two days a week
B. Client will experience reduced depression overall
C. Client will score below 10 on the PHQ-9 within 12 weeks

90. A counselor practicing under the Counseling Compact moves her primary residence from one member state to another. She must:

A. Keep her former home-state license as her home license
B. Practice in her new state on a privilege indefinitely
C. Apply for a license in the new home state under Compact rules

91. A manager assumes that an employee who uses a wheelchair must also struggle with complex projects and leading meetings. This error is best described as:

A. Cultural imperialism
B. The spread effect
C. Stereotype threat

92. A client from a family that makes major decisions together says she wants her parents involved in shaping her counseling goals. The counselor's best approach is to:

A. Encourage her to set her own goals independently first
B. Let her parents set her goals for her instead
C. Respect that value and explore how to involve them

93. A licensed counselor is asked to provide supervision to a new graduate seeking licensure. Before agreeing, the counselor should first:

A. Verify the board's supervisor training or credential requirements
B. Agree, since years of practice qualify a counselor to supervise
C. Draft a supervision contract and begin the following week

94. A client is being discharged after a week of inpatient care following a suicide attempt and will continue in outpatient counseling. The counselor should prioritize:

A. A first outpatient appointment sometime within the next month
B. A follow-up appointment within days, with safety planning
C. Waiting for the client to call once she feels ready

95. A counselor asks, “As you talk about your father right now, what are you noticing in your chest and hands?” The counselor makes no reference to the counseling relationship. This response is best described as:

A. Immediacy
B. Here-and-now awareness
C. An interpretation

96. When making a referral from the list, the counselor best supports client choice by:

A. Naming only the one colleague the counselor prefers
B. Offering several vetted options that fit the need
C. Asking the client to search online without any help

97. For 8 months, a 10-year-old has often lost his temper, argued with his mother and teachers, refused rules, and deliberately annoyed others. He hasn’t hurt anyone, destroyed property, or stolen. This best fits:

A. Disruptive mood dysregulation disorder
B. Conduct disorder
C. Oppositional defiant disorder

98. A supervisor is held legally responsible for harm caused by her supervisee’s actions, even though she was not negligent herself. This legal doctrine is:

A. Direct liability
B. Dereliction of duty
C. Vicarious liability

99. A Deaf client who uses American Sign Language requests an interpreter for counseling sessions. The counselor should:

A. Ask the client to bring a family member to interpret
B. Exchange written notes with the client during sessions
C. Arrange a qualified interpreter at no charge to the client

100. Family reconstruction, a method for exploring family-of-origin patterns, is associated with:

A. Murray Bowen
B. Virginia Satir
C. Carl Whitaker

101. An adult survivor of childhood sexual abuse says, “No matter what I said or did, it kept happening.” Finkelhor and Browne would link this to which traumagenic dynamic?

A. Stigmatization
B. Powerlessness
C. Betrayal

102. How can a counselor most accurately judge the counselor’s own strengths and limitations with a specific disorder?

A. Compare self-ratings with outcome data and supervisor feedback
B. Rely on how confident the counselor feels during sessions
C. Rely on the counselor’s total years of general practice

103. A client says, “My car broke down again, and I don’t know how I’ll get to work this week.” Which response is interchangeable, at about level 3 on Carkhuff’s scale?

A. “With your car out again, you’re really worried about getting to work.”
B. “You’re worried about getting to work, and maybe tired of always being one breakdown from falling behind.”
C. “Cars always seem to break down at the worst time.”

104. A client has repeated panic-like episodes, but each involves fewer symptoms than the criteria require. The counselor wants the record to state that reason. Which designation fits?

A. Panic disorder (provisional)
B. Unspecified anxiety disorder
C. Other specified anxiety disorder

105. Reviewing a client’s prior PHQ-9 scores before each session mainly helps the counselor:

A. Confirm the diagnosis without further assessment
B. Track progress and adjust the treatment focus
C. Meet billing rules for the session code

106. After session five, a member stays behind and tells the leader privately that she feels the leader cuts her off more than others. The leader's best response is to:

A. Listen non-defensively and invite her to raise it in group
B. Explain the reasons for each interruption so she understands
C. Agree to keep the complaint between the two of them

107. After two months of weekly outpatient sessions, a client's depression has worsened. She misses most work and is struggling to care for herself, but she has no suicidal intent. The most appropriate option to discuss with her is:

A. Stepping up to intensive outpatient or partial hospitalization
B. Continuing weekly sessions under the current treatment plan
C. Inpatient hospitalization, given how poorly she is functioning

108. Updating her practice's emergency preparedness plan, a solo counselor considers what would happen if she were suddenly incapacitated. The plan should include:

A. Permission for her family to open client files
B. A line in each consent form that services may end
C. A professional will naming a colleague to manage records

109. A 24-year-old client says she scratches her arms until they bleed when she feels overwhelmed, and that she has no wish to die. The counselor's best response is to:

A. Make continued counseling conditional on her stopping now
B. Arrange an inpatient admission today to keep her safe

C. Assess suicide and medical risk, then build coping skills

110. A client makes a credible threat to harm his counselor after she declines to write a letter he requested. Under the ACA Code, the counselor:

A. May end services, with safety steps and referrals
B. Must continue, since ending would be abandonment
C. Must warn his family under the Tarasoff duty to protect

111. A Deaf client who communicates in American Sign Language requests counseling, and the counselor does not sign. In assessing suitability, the counselor should:

A. Offer a qualified interpreter or a signing counselor
B. Use written notes so no third party is involved
C. Ask the client's adult child to interpret sessions

112. A client says calmly, "I think you've got my mother all wrong. She's not the problem." The response that best shows conflict tolerance is:

A. "I may have missed something. Help me see her the way you do."
B. "I've been doing this a long time, and the pattern is clear."
C. "You're right, I'm so sorry. I won't bring her up again."

113. Six weeks into a plan to cut back on gambling, a client who had been making steady changes says, "I'm not sure I even want to stop anymore." The counselor's best step is to:

A. Reassess his readiness and revise the plan with him
B. Keep the current plan and remind him of his original goal
C. Confront his denial so that he will recommit

114. A Deaf client whose preferred language is American Sign Language begins counseling. The counselor should arrange:

A. Written notes exchanged during each session
B. A qualified ASL interpreter for sessions
C. Seating so the client can lip-read clearly

115. Which progress note format records the client's presentation, what the counselor did, how the client reacted, and next steps?

A. GIRP
B. SOAP
C. BIRP

116. In a working-stage session, a member tells the leader, "When Ray talks over me, I feel dismissed." The leader's best response is:

A. "Ray, please let her finish from now on."
B. "It sounds like you feel dismissed by him."
C. "Could you say that directly to Ray?"

117. A former client's paper chart has passed the retention period that state law and practice policy require. The counselor should dispose of it by:

A. Placing it in the office recycling bin with the other paper
B. Mailing it to the client's last known address
C. Shredding it so the information cannot be reconstructed

118. The idea that awareness of death drives people to cling to cultural worldviews and self-esteem as buffers is called:

A. Neurotic anxiety
B. Terror management theory
C. Viktor Frankl's will to meaning

119. In Stanley and Brown's Safety Planning Intervention, what does the client identify first?

A. Internal coping strategies to use when alone
B. Warning signs that a crisis may be starting
C. Ways to make the environment safer

120. Minutes after a tense session in which a client accused the counselor of betrayal, the counselor still feels keyed up, and the next client is waiting. What is best?

A. Take a brief grounding pause before seeing the next client
B. Vent about the prior client to the front-desk staff
C. Push the feelings down and start the session at once

121. The view that a well-handled alliance rupture often strengthens the counseling relationship comes from:

A. Robert Carkhuff
B. Edward Bordin
C. Safran and Muran

122. In a quiet office, a client says the hum of the air vent briefly sounded like someone whispering her name. When she listened closely, she could tell it was only the vent. In the MSE, this is recorded as:

A. An auditory hallucination
B. A delusion
C. An illusion

123. A judge orders a counselor to release a client's records related to a specific incident. The counselor should:

A. Refuse, citing the client's privilege
B. Release only what the order specifies, seeking to limit it
C. Release the entire record to show full cooperation

124. In Cass's model of homosexual identity formation, which stage comes immediately after identity comparison?

A. Moratorium
B. Identity tolerance
C. Identity acceptance

125. A client who has drunk heavily every day for years says, "I'm quitting cold turkey tomorrow, on my own." The counselor's most important response is to:

A. Arrange medical evaluation; withdrawal could be life-threatening
B. Explore his ambivalence with a decisional balance exercise
C. Affirm his decision and schedule a relapse prevention session

126. At intake, a counselor realizes the client once drove the counselor on a single rideshare trip years ago, with no contact since. The best step is to:

A. Refer him elsewhere, since any prior contact rules out counseling
B. Say nothing, since mentioning it could harm early rapport
C. Acknowledge it, weigh any risk, and document the decision

127. A counselor notices she now refers to clients as “the 2 o’clock” and feels cynical and detached from them. This best reflects:

A. Depersonalization as a dissociative symptom
B. The depersonalization dimension of burnout
C. Reduced personal accomplishment

128. The 1974 Tarasoff decision, often called Tarasoff I, held that therapists have a duty to:

A. Act for a foreseeable victim based on violent history
B. Warn an identifiable victim of a serious threat
C. Take reasonable steps to protect an intended victim

129. When opening a client’s second-session note, the counselor sees that the EHR has copied forward the entire intake note. The counselor should:

A. Revise the note to record only what occurred in this session
B. Sign it as written, since the presentation has not changed much
C. Sign it now and add a session summary next week

130. A client describes a harsh inner voice that demands perfection and another part of her that whines and makes excuses to avoid the work. A Gestalt counselor would most likely frame this split as:

A. Parent and Child ego states
B. Superego and id
C. Top dog and underdog

131. When an ACA ethical standard conflicts with a law, the ACA Code directs counselors first to:

A. Follow the Code, since ethics always outrank the law
B. State their commitment to the Code and try to resolve it
C. Follow the law without raising the conflict with anyone

132. A counselor was trained years ago on a clinician-rated symptom scale but has not used it since. Before relying on new scores, the counselor should:

A. Rely on the original training, since competence is permanent
B. Review the manual and co-rate with a trained colleague
C. Interpret scores by clinical impression instead of norms

133. A client who uses a wheelchair comes in for depression and says it's about losing his job. The counselor suspects the disability is the real issue. The best approach is to:

A. Refer him to a counselor who specializes in disability
B. Start with the client's own account of what's wrong
C. Gently suggest that his disability may be the deeper issue

134. A counselor wants to screen for adversity in a client's childhood, a known risk factor for later substance use. Which tool fits this purpose?

A. The ACEs questionnaire
B. The Child Behavior Checklist
C. The Social Readjustment Rating Scale

135. A client asks whether an AI mental health chatbot could help between sessions. The counselor's best response is to:

A. Endorse it, since chatbots follow clinical care standards
B. Forbid it, since the ACA Code prohibits AI use
C. Review its privacy and crisis limits as a possible supplement

136. A counselor coaches an escalated teen to put words to what he feels ("I'm furious and embarrassed") because labeling emotions helps regulate them. This reflects:

A. Siegel's "name it to tame it"
B. Wells's metacognitive therapy
C. Gross's response modulation

137. Which federal law strengthened HIPAA enforcement and added breach notification requirements for electronic health records?

A. HITECH Act
B. 42 CFR Part 2
C. FERPA

138. At the end of a session, a young child shyly gives the counselor a drawing he made of the two of them playing a game. The counselor should:

A. Accept it warmly and explore its meaning with him
B. Decline it, since counselors may never accept gifts
C. Accept it and give him a gift of similar value next week

139. Before a first session, a counselor wants to look through a new client's public Instagram profile to learn more about her. Under the ACA Code, the counselor should:

A. View it, since the profile is public
B. Follow it from her professional account
C. Not view it unless the client consents

140. In First's differential diagnosis sequence, which question is asked at the final step?

A. Is there clinically significant distress or impairment?
B. Could a substance or medication explain the symptoms?
C. Are the symptoms being intentionally produced?

141. Building security announces a lockdown for a reported armed intruder during a session. The counselor should:

A. Leave with the client for the parking lot right away
B. Lock the door and stay out of sight with the client
C. Step into the hallway to confirm that the threat is real

142. A client who has described years of resentment toward his older brother now speaks of him with lavish, almost gushing praise. If this is a defense, it is most likely:

A. Rationalization
B. Sublimation
C. Reaction formation

143. A client whose drinking has led to two DUIs and abnormal liver tests says, "I don't have a drinking problem. Everyone else is overreacting." In the MSE, this statement speaks most directly to his:

A. Insight
B. Thought process
C. Judgment

144. A licensed counselor asks an expert colleague for advice on setting a client's sleep-restriction schedule. The colleague has no evaluative authority, and the counselor is free to accept or reject the advice. This arrangement is best described as:

A. Clinical supervision
B. Referral for treatment
C. Collegial consultation

145. Before acting on a hard ethical decision, a counselor asks herself, “Would I be comfortable seeing this choice reported in the newspaper?” This question is Stadler’s:

A. Justice test
B. Publicity test
C. Universality test

146. When should a counselor first raise the topic of termination with a client?

A. Only when the client brings up wanting to stop
B. In the final session, once the goals are met
C. In informed consent, revisiting it as goals near

147. A client begins sending several portal messages a day seeking reassurance, and the practice has a written between-session contact policy. The counselor should:

A. Review the contact policy and explore the pattern in session
B. Reply to each message right away with detailed reassurance
C. Give the client a personal cell number for faster replies

148. A client who takes high daily doses of nonprescribed benzodiazepines announces she will quit “cold turkey” this weekend. The counselor’s best response is to:

A. Explain that withdrawal is unpleasant but safe
B. Refer her for medically managed withdrawal
C. Affirm her plan and check in on Monday

149. A client grieving her father asks, “Have you ever lost a parent?” The counselor has. Which response is the most appropriate self-disclosure?

A. “Yes, and honestly, I’m still struggling with it most days.”
B. “Yes. Let me tell you about my father’s illness and how we coped.”

C. “I have, and it was disorienting. What has it been like for you?”

150. A supervisee notices the supervisor shifts among teacher, counselor, and consultant roles to target intervention, conceptualization, and personalization skills. This approach reflects:

A. The Integrated Developmental Model
B. The discrimination model
C. The systems approach to supervision

151. In a grief group, the leader asks, “Who else has felt guilty for laughing since the loss?” Several members nod in surprise. Which therapeutic factor is the leader fostering?

A. Catharsis
B. Instillation of hope
C. Universality

152. A counselor promises to hold a client’s Tuesday evening slot while he travels for work. When another client asks for that time, she declines. Which ethical principle does this most directly reflect?

A. Veracity
B. Justice
C. Fidelity

153. A health plan reviewing a client’s claims asks for the counselor’s separately kept psychotherapy notes. Under HIPAA, the counselor should:

A. Send them, since payment disclosures need no authorization
B. Send them once the plan limits its request to the minimum necessary
C. Send them only with the client’s specific written authorization

154. Midway through a study of a campus stress-reduction program, a hurricane closes the university for two

weeks, and stress scores rise in both the treatment and control groups. This threat to internal validity is:

A. Maturation
B. Experimental mortality
C. History

155. While teaching stress management, a counselor explains the relaxation response. This concept is owned by:

A. Joseph Wolpe
B. Herbert Benson
C. Edmund Jacobson

156. A counselor is randomly selected for a continuing education audit at renewal. Which best verifies the hours the counselor reported?

A. Retained certificates listing provider, date, and hours
B. A list of course titles the counselor recalls attending
C. The provider’s online catalog showing the courses offered

157. Brief Strategic Family Therapy for adolescent substance use was developed by:

A. Howard Liddle
B. José Szapocznik
C. Scott Henggeler

158. After an ethics training, a counselor resolves to go beyond simply avoiding Code violations and to practice at the highest standard the profession sets. This orientation is called:

A. Mandatory ethics
B. Principle ethics
C. Aspirational ethics

159. A provisionally licensed associate counselor learns that her board-approved supervisor will leave the agency next week. The associate should:

A. Keep seeing clients and log hours until someone new starts
B. Follow board rules to secure and register a new super-

visor
C. Arrange for a fellow associate to supervise her for now

160. A parent tells a 9-year-old that he can play outside as soon as he finishes his reading. This arrangement illustrates:

A. The Premack principle
B. A token economy
C. Negative reinforcement

161. A counselor is clearing out closed client files that are past the retention period her state requires. The appropriate step is to:

A. Dispose of them in a way that protects confidentiality
B. Place them in the office's regular paper recycling bin
C. Give them to the records custodian named in her will

162. Evidence-based practice in counseling integrates the best available research with clinical expertise and:

A. The counselor's preferred theoretical orientation
B. The treatment with the largest published effect size
C. Client characteristics, culture, and preferences

163. A researcher sorts a state's licensed counselors into urban, suburban, and rural groups, then randomly selects counselors from each group in proportion to its size. This sampling method is:

A. Systematic sampling
B. Stratified sampling
C. Cluster sampling

164. A counselor reflects on how her professional role and her identity carry power in her relationship with each client. This awareness belongs most directly to which core attribute?

A. Self-awareness and impact on clients
B. Nonpossessive warmth toward the client
C. Empathic attunement to the client

165. To attract clients, a counselor plans to routinely waive insurance copays for everyone. Compliance training would identify this as:

A. A possible fraud and payer contract violation
B. A fee adjustment for financial hardship
C. Pro bono service to the public good

166. A client who uses a wheelchair says his biggest problem is that his workplace has no ramp and his manager won't move meetings to the ground floor. His view of disability reflects the:

A. Biopsychosocial model
B. Social model
C. Medical model

167. A client says her goal is "to stop being so lonely." Using de Shazer's criteria for well-formed goals, the best rewrite is:

A. Never feel lonely on a weekend again
B. Understand why she keeps pulling away from others
C. Call or meet up with one friend twice a week

168. A client's insurer denies further sessions although standardized scores show symptoms remain elevated. The best advocacy step is to:

A. Bill under a more severe diagnosis the insurer will approve
B. With consent, appeal using outcome data showing medical necessity
C. Accept the denial and end at the last authorized session

169. A client hasn't paid for four sessions, and the counselor plans to use a collection agency. Under the ACA Code, the counselor must first:

A. Give the agency the client's diagnosis to support the claim
B. End services at once and then file with the agency

C. Have told the client in advance and offered a chance to pay

170. A colleague says counselors may still use consumer video apps for telehealth under the federal pandemic flexibility. The counselor should recognize that:

A. The flexibility lasts as long as Medicare telehealth extensions
B. Encryption alone makes a consumer app compliant
C. HHS OCR ended that enforcement discretion in 2023

NCE Practice Exam 3 — Answer Key with Ownership Map

Every rationale names who or what **owns** each option. Read all three, not just the correct one: each item teaches three pairings.

How to review: for every item you missed *or guessed*, write in your error log (Workbook, Part 4) the owner you picked and the owner of the correct answer.

Quick Key

1. B	2. A	3. A	4. A	5. C	6. B	7. B	8. A	9. C	10. B
11. C	12. A	13. B	14. B	15. B	16. A	17. C	18. C	19. C	20. B
21. A	22. C	23. A	24. B	25. A	26. B	27. C	28. B	29. C	30. A
31. C	32. A	33. B	34. B	35. A	36. C	37. C	38. B	39. A	40. C
41. A	42. B	43. B	44. B	45. C	46. A	47. C	48. A	49. C	50. A
51. B	52. B	53. B	54. C	55. C	56. A	57. A	58. A	59. B	60. B
61. B	62. B	63. C	64. B	65. C	66. A	67. A	68. C	69. B	70. A
71. A	72. C	73. B	74. B	75. A	76. B	77. C	78. B	79. B	80. C
81. C	82. C	83. A	84. A	85. A	86. C	87. B	88. A	89. C	90. C
91. B	92. C	93. A	94. B	95. B	96. B	97. C	98. C	99. C	100. B
101. B	102. A	103. A	104. C	105. B	106. A	107. A	108. C	109. C	110. A
111. A	112. A	113. A	114. B	115. C	116. C	117. C	118. B	119. B	120. A

121. C	122. C	123. B	124. B	125. A	126. C	127. B	128. B	129. A	130. C
131. B	132. B	133. B	134. A	135. C	136. A	137. A	138. A	139. C	140. A
141. B	142. C	143. A	144. C	145. B	146. C	147. A	148. B	149. C	150. B
151. C	152. C	153. C	154. C	155. B	156. A	157. B	158. C	159. B	160. A
161. A	162. C	163. B	164. A	165. A	166. B	167. C	168. B	169. C	170. C

Rationales

1 — B

Treatment Planning & Continuity of Care · Task 3B.8 · Chapter 4

- **B. Mention the drop and ask what hasn't felt right to her — Owner: Rupture repair: name and explore (Safran & Muran).** Feedback measures are meant to be used; naming the drop non-defensively and exploring it repairs a possible withdrawal rupture.
- A. Offer to refer her to a colleague she might prefer — Owner: Premature referral. Referral abandons the alliance instead of repairing it.
- C. Accept her statement and continue with the planned work — Owner: Ignoring a withdrawal rupture. Agreeing too easily can itself signal a withdrawal rupture; pressing on leaves it unrepaired.
- **Pair to learn:** Feedback shows an alliance drop → Name and explore the rupture (Safran & Muran)

2 — A

Intake & Assessment · Task 2B.3 · Chapter 11

- **A. The CRAFFT — Owner: Adolescent substance use screen.** The CRAFFT is the screen recommended for adolescents.

- B. The OQ-45 — Owner: Lambert (Outcome Questionnaire). The OQ-45 tracks counseling progress; it is not a substance screen.
- C. The CBCL — Owner: Achenbach (CBCL). The CBCL is a behavior rating checklist, not a substance use screen.
- **Pair to learn:** Adolescent substance use screen → CRAFFT (not CBCL)

3 — A

Legal & Ethical Compliance · Task 6.1 · Chapter 17

- **A. Notify clients and hold sessions securely from the backup site — Owner: Practice emergency plan (client notification and continuity of care).** The plan's client-notification and alternate-site steps keep care going without dropping privacy and security safeguards.
- B. Cancel all sessions until the building reopens — Owner: Client neglect (ACA A.12). An indefinite stop leaves clients without care when the plan already provides a way to continue it.
- C. Wait for clients to call, so the closure does not alarm them — Owner: Failure to notify (bypassing the communication plan). Silence leaves clients arriving at a closed building; the plan directs proactive contact through their listed methods.
- **Pair to learn:** Office closed by disaster → notify clients and continue from the backup site per the plan

4 — A

Provision of Counseling Interventions · Task 4.7 · Chapter 5

- **A. Choose interventions tied to her panic goal — Owner: Implementing the treatment plan (4.7).** Every intervention should trace back to a treatment goal; a valid technique unrelated to the goal is the wrong choice.

- B. Try the technique and see how she responds to it — Owner: Technique unrelated to stated goals. A technique chosen for the counselor's interest rather than the client's goal breaks the link to the plan.
- C. Help her set a new goal that suits the technique — Owner: Fitting the client to the technique. Goals come from the client's concern; they are not reshaped to fit a technique.
- **Pair to learn:** Every intervention traces back to a treatment goal (4.7)

5 — C

Provision of Counseling Interventions · Task 4.3 · Chapter 5

- **C. Passive-aggressive — Owner: Assertiveness training (behavioral tradition: Wolpe, Salter, Lazarus).** An indirect, sarcastic jab instead of a direct request is passive-aggressive.
- A. Aggressive — Owner: Assertiveness training. An aggressive reply is an open attack or insult.
- B. Assertive — Owner: Assertiveness training. An assertive reply would name the behavior, the feeling, the impact, and a request.
- **Pair to learn:** Sarcastic indirect jab → passive-aggressive (not assertive)

6 — B

Professional Development & Counselor Self-Awareness · Task 1A.1 · Chapter 22

- **B. Weigh it against the counselor's own assessment and document it — Owner: Consultee retains responsibility (ACA A.1.a).** The treating counselor keeps primary responsibility, decides how the advice applies, and documents the consultation.
- A. Follow it exactly, since the consultant now shares responsibility — Owner: Responsibility-transfer error (consultation is not supervision). Consultation informs

care; it does not move responsibility for the client to the consultant.

- C. Ask the consultant to co-sign the revised treatment plan — Owner: Clinical supervision (supervisor vicarious liability). Co-signing fits supervision, where the supervisor shares liability; a consultant holds no such role.
- **Pair to learn:** Consultant's advice → counselor decides, keeps responsibility, documents

7 — B

Intake & Assessment · Task 2B.11 · Chapter 12

- **B. Tourette's disorder — Owner: DSM-5-TR Tourette's disorder.** Tourette's requires multiple motor tics and at least one vocal tic for more than 1 year, with onset before age 18.
- A. ADHD, combined presentation — Owner: DSM-5-TR ADHD. ADHD involves inattention and hyperactivity-impulsivity, not motor and vocal tics.
- C. OCD with tic-related specifier — Owner: DSM-5-TR OCD (tic-related specifier). This specifier requires obsessions or compulsions, which are not described here.
- **Pair to learn:** Motor + vocal tics, over 1 year, onset before 18 → Tourette's disorder

8 — A

Legal & Ethical Compliance · Task 6.7 · Chapter 17

- **A. It isn't exploitative, fits local norms, and is put in writing — Owner: Bartering conditions (ACA A. 10.e).** A.10.e allows bartering only when the client requests it, it isn't exploitative or harmful, it's accepted in the community, and it's documented in a written contract.
- B. The agency allows it and she is told about her other options — Owner: Self-referral rule (ACA A.10.a). Agency permission plus informing clients of other

options is the rule for referring agency clients to a private practice, not for bartering.

- C. The counselor proposes the terms as a fair way to help — Owner: Counselor-initiated bartering (punished). Bartering must be requested by the client; counselor-initiated bartering is what the exam punishes.
- **Pair to learn:** Client-requested barter → ACA A.10.e conditions and written contract

9 — C

Indirect Client Care · Task 5.1 · Chapter 23

- **C. Sharing lived experience to support recovery goals — Owner: Peer support specialist.** Peer specialists use their own recovery experience to offer mutual support, hope, and help reaching the client's goals.
- A. Diagnosing and revising the treatment plan — Owner: Licensed clinician role. Diagnosis and treatment planning belong to the licensed clinician, not the peer specialist.
- B. Prescribing medications for opioid use disorder — Owner: Prescriber role. Medications for opioid use disorder are ordered by a prescriber such as a physician or nurse practitioner.
- **Pair to learn:** Lived-experience support in recovery → peer recovery specialist

10 — B

Professional Development & Counselor Self-Awareness · Task 1A.5 · Chapter 17

- **B. May not, since her doctorate isn't in a counseling-related field — Owner: Use of "Dr." (ACA C.4.d).** C.4.d permits counselors to use "Dr." only with a doctorate in counseling or a closely related field.
- A. May, since any earned doctorate allows the title in advertising — Owner: Misreading of ACA C.4.d. C.4.d

limits the title to doctorates in counseling or a closely related field, not any doctorate.

- C. May, as long as her NCC certification is listed next to it — Owner: Certification (NCC from NBCC). A national certification doesn't change which degree qualifies a counselor to use "Dr."
- **Pair to learn:** "Dr." in counseling practice → Doctorate in counseling or closely related field (C.4.d)

11 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Aaron Beck — Owner: Beck (hopelessness; Beck Hopelessness Scale).** Beck identified hopelessness as a stronger predictor of suicide than depression.
- A. Thomas Joiner — Owner: Joiner (interpersonal theory of suicide). Joiner explains suicidal desire through belongingness and burdensomeness.
- B. David Jobes — Owner: Jobes (CAMS). Jobes developed the Collaborative Assessment and Management of Suicidality.
- **Pair to learn:** Hopelessness predicts suicide → Beck (not Joiner)

12 — A

Treatment Planning & Continuity of Care · Task 3A.1 · Chapter 21

- **A. Begin with her concern, share the sleep view, and revisit the order — Owner: Bordin's working alliance (agreement on goals and tasks).** Bordin defined the working alliance as agreement on goals and tasks plus the bond. When safety does not override, the client's priority leads, the counselor shares a view openly, and the order is revisited.
- B. Begin with the insomnia, since the counselor sets the order of focus — Owner: Expert-directed planning. Overriding her stated priority with the counselor's

formulation weakens agreement on goals and reduces engagement.

- C. Postpone priorities until her symptoms ease enough for her to choose — Owner: Delayed planning error. Treatment priorities guide care now. Waiting until she feels better stalls the very work meant to help her feel better.
- **Pair to learn:** Client's priority differs from counselor's, no safety issue → Client leads; agree on goals (Bordin)

13 — B

Intake & Assessment · Task 2B.2 · Chapter 13

- **B. Violent resistance — Owner: Michael P. Johnson (IPV typology).** Violent resistance is violence used in response to a partner's intimate terrorism, often in self-defense.
- A. Situational couple violence — Owner: Michael P. Johnson (IPV typology). Situational couple violence arises from escalating conflicts without a broader pattern of control; here he exerts coercive control.
- C. Intimate terrorism — Owner: Michael P. Johnson (IPV typology). Intimate terrorism describes his pattern of coercive control, not her response to it.
- **Pair to learn:** Violence in response to a partner's coercive control → violent resistance (Johnson)

14 — B

Intake & Assessment · Task 2B.8 · Chapter 19

- **B. Grave disability — Owner: Involuntary criteria: grave disability (inability to meet basic needs).** His inability to meet basic needs for food and shelter is grave disability, one of the three general grounds for involuntary care.
- A. His poor insight into the illness — Owner: MSE: insight. Poor insight alone is not a basis for involuntary care; the grounds are danger to self, danger to others, or grave disability.

- C. Imminent danger to self through suicidal intent — Owner: Involuntary criteria: danger to self. He denies suicidal thoughts; the risk comes from his inability to meet basic needs.
- **Pair to learn:** Cannot meet basic needs → Grave disability (involuntary criterion)

15 — B

Legal & Ethical Compliance · Task 6.3 · Chapter 17

- **B. Give an accounting that excludes treatment, payment, and operations — Owner: HIPAA accounting of disclosures (45 CFR 164.528).** Clients may request an accounting of certain disclosures, generally answered within 60 days; disclosures for treatment, payment, operations, or by authorization are excluded.
- A. Give him a full copy of his record instead of a list — Owner: HIPAA right of access (45 CFR 164.524). Access to the record is a different right; it does not tell the client who received his information.
- C. Decline, since disclosure logs are internal security records — Owner: Security Rule audit controls (45 CFR 164.312(b)). Audit logs serve system security, but the accounting of disclosures is a separate right the client can exercise.
- **Pair to learn:** Who has seen my information? → HIPAA accounting of disclosures (164.528)

16 — A

Treatment Planning & Continuity of Care · Task 3B.3 · Chapter 21

- **A. Relapse prevention — Owner: Relapse prevention (termination task).** Identifying warning signs, a coping plan, supports, and what to do if problems return is relapse prevention.
- B. Pretermination counseling — Owner: Pretermination counseling (ACA A.11.c). Pretermination counseling is

the broader ethical process of preparing for ending, not this specific plan for warning signs.

- C. Booster sessions — Owner: Booster sessions (follow-up after ending, 3B.2). Booster sessions are follow-up contacts after ending; the stem describes building a plan for warning signs.
- **Pair to learn:** Warning signs and coping plan → Relapse prevention

17 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Developmental — Owner: Crisis types: developmental.** Developmental crises arise from predictable life transitions.
- A. Situational — Owner: Crisis types: situational. Situational crises follow sudden, unexpected events such as an accident or job loss.
- B. Existential — Owner: Crisis types: existential. Existential crises involve inner conflicts about meaning, purpose, or identity.
- **Pair to learn:** Predictable life transitions → developmental crisis (not situational)

18 — C

Professional Development & Counselor Self-Awareness · Task 1A.3 · Chapter 22

- **C. A review of current caseload needs and practice gaps — Owner: Needs-based CE planning (ACA C.2.f).** CE serves competence in the skills the counselor actually uses, so selection starts from who the counselor now serves and where skills are thin.
- A. Topics the counselor already knows well and enjoys — Owner: Comfort-zone selection. Repeating familiar topics earns hours but leaves the counselor's new practice gaps untouched.
- B. Whatever courses the employer offers at no cost — Owner: Convenience selection. Free courses can be

useful, but cost alone is not a basis for matching training to client needs.

- **Pair to learn:** Caseload shifts → choose CE from a practice-gap review

19 — C

Intake & Assessment · Task 2B.3 · Chapter 11

- **C. No disorder; prescribed tolerance doesn't count — Owner: DSM-5-TR exception for supervised prescribed use.** Tolerance and withdrawal from medication taken as prescribed under medical supervision don't count toward a substance use disorder.
- A. Mild sedative use disorder, with two criteria met — Owner: SUD severity (mild = 2–3). This counts tolerance and withdrawal from prescribed use, which the DSM-5-TR excludes.
- B. Sedative use disorder in early remission — Owner: Early remission specifier. Remission requires a prior use disorder, which she never met.
- **Pair to learn:** Tolerance/withdrawal from prescribed use → not counted toward SUD

20 — B

Treatment Planning & Continuity of Care · Task 3B.8 · Chapter 20

- **B. Give the PHQ-9 at intake and again at the tenth session — Owner: Pretest/posttest measurement (3B.8).** Using the same measure before and after counseling provides a baseline and shows change over time.
- A. Rely on her own impression of how much the client has improved — Owner: Informal observation alone. A single informal source is subject to bias; outcomes call for a standardized measure.
- C. Give the PHQ-9 once, at the tenth session, and compare it to norms — Owner: Posttest only (no

baseline). Without a pretest there is no baseline, so change cannot be shown.

- **Pair to learn:** Same measure before and after → Pretest/posttest outcome measurement (3B.8)

21 — A

Indirect Client Care · Task 5.4 · Chapter 23

- **A. Assess risk, document, and contact the prescriber promptly — Owner: FDA boxed warning: antidepressants, ages under 25.** Antidepressants carry a boxed warning for increased suicidal thinking in children, adolescents, and young adults, so new thoughts call for risk assessment and prompt prescriber contact.
- B. Reassure her these effects usually fade within a month — Owner: Minimizing activation symptoms. Early activation can occur, but new suicidal thoughts are not something to wait out.
- C. Advise her to skip doses until she sees her physician — Owner: Discontinuation advice (outside counselor scope). Skipping doses is a medication decision outside counselor scope and can cause discontinuation symptoms.
- **Pair to learn:** Under 25, new suicidal thoughts after starting an SSRI → assess risk, contact prescriber (boxed warning)

22 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Psychological first aid for safety and practical needs — Owner: Psychological first aid (NCTSN & National Center for PTSD).** PFA is the evidence-informed approach for the hours and days after a disaster: safety, comfort, practical help, and connection.
- A. Prolonged exposure to process each survivor's memories — Owner: Foa (prolonged exposure).

Prolonged exposure is an adult trauma treatment, not a first response days after a disaster.

- B. A required debriefing where each survivor retells the event — Owner: Single-session psychological debriefing. Forced detailed retelling soon after trauma is not recommended and may increase distress.
- **Pair to learn:** Days after a disaster → psychological first aid (not debriefing)

23 — A

Intake & Assessment · Task 2B.12 · Chapter 19

- **A. A semi-structured interview — Owner: Semi-structured interview.** Semi-structured interviews use core questions with room for follow-up, balancing reliability and flexibility, though they require training.
- B. A structured interview such as the SCID-5 — Owner: Structured interview (SCID-5). Fixed questions in a fixed order give high reliability but leave little room for unplanned follow-up.
- C. An unstructured, conversational interview — Owner: Unstructured interview. It is flexible and builds rapport, but its lower reliability does not give her the consistent core she wants.
- **Pair to learn:** Core questions plus room for follow-up → Semi-structured interview (not structured SCID-5)

24 — B

Treatment Planning & Continuity of Care · Task 3A.4 · Chapter 21

- **B. Strength: her family support; barrier: avoiding the crash route — Owner: Strengths-based assessment (Saleebey).** Saleebey's strengths perspective identifies resources such as support and motivation. Avoidance is the core barrier, since it maintains PTSD symptoms.
- A. Strength: her detour routes; barrier: her family's involvement — Owner: Safety behavior mislabeled (negative reinforcement of avoidance). Detours bring

short-term relief but maintain avoidance through negative reinforcement. Family support is a strength, not a barrier.

- C. Strength: scanning traffic; barrier: her motivation to change — Owner: Hypervigilance mislabeled (DSM-5-TR PTSD arousal symptom). Constant scanning is hypervigilance, a PTSD arousal symptom, and her motivation is a strength. This reverses which factors help or hinder.
- **Pair to learn:** Avoidance vs. support → Avoidance is a barrier; support is a strength

25 — A

Intake & Assessment · Task 2B.11 · Chapter 20

- **A. z = −2; deviation IQ score of 70 — Owner: z = (x − mean) ÷ SD; deviation IQ = 100 + 15z.** (36 − 60) ÷ 12 = −2, and 100 + 15(−2) = 70, about the 2nd percentile.
- B. z = −2.4; deviation IQ score of 64 — Owner: Divided by 10 instead of the test's SD of 12. −24 ÷ 10 = −2.4 uses the wrong SD; the test's SD is 12.
- C. z = −2; deviation IQ score of 80 — Owner: T-score SD of 10 used instead of 15. The z is right, but 100 + 10(−2) uses the T-score's SD; deviation IQ uses SD 15.
- **Pair to learn:** Raw 36 (mean 60, SD 12) → z = −2 → deviation IQ 70

26 — B

Indirect Client Care · Task 5.3 · Chapter 21

- **B. PHQ-9 score of 14; affect constricted — Owner: SOAP: Objective (counselor observations, test scores).** What the counselor observes or measures, such as affect and test scores, goes in Objective.
- A. Client says, "I can't get out of bed." — Owner: SOAP: Subjective (client's report). The client's own words belong in Subjective.
- C. Progress toward Objective 2 is moderate — Owner: SOAP: Assessment (clinical impression). The

counselor's impression of progress belongs in Assessment.

- **Pair to learn:** Counselor-observed affect and test score → SOAP: Objective

27 — C

Legal & Ethical Compliance · Task 6.6 · Chapter 17

- **C. Kept certificates for as long as board rules require — Owner: State board CE recordkeeping rules.** Boards set how long licensees must keep CE proof, which varies by state; kept certificates satisfy an audit.
- A. Listed her course titles on the renewal application — Owner: Renewal attestation (not proof of completion). Renewal usually involves attesting to completion; an audit asks for documentation that backs the attestation.
- B. Kept certificates only until her renewal was approved — Owner: Premature disposal of CE records (error pattern). Audits often come after approval, so discarding proof at renewal leaves nothing to submit.
- **Pair to learn:** CE audit → keep completion certificates for the board's required period

28 — B

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 21

- **B. Refer her to legal or benefits resources, then follow up — Owner: Concurrent referral to community resources (3B.1).** Legal and financial needs are referred to appropriate community resources, with follow-up to confirm she connected, while counseling continues.
- A. Advise her on the legal steps to file her appeal — Owner: Practicing outside competence (ACA C.2.a). Giving legal advice falls outside a counselor's competence.

- C. Call the benefits office to argue her case on her behalf — Owner: Advocacy without client consent (ACA A.7.b). Advocating for a specific client requires her consent first, and empowering her to advocate is preferred where possible.
- **Pair to learn:** Legal or benefits problem → Concurrent referral to community resources (3B.1)

29 — C

Treatment Planning & Continuity of Care · Task 3B.8 · Chapter 20

- **C. The ORS and SRS (Miller & Duncan) — Owner: Miller & Duncan (Outcome and Session Rating Scales).** The ORS and SRS are brief session-by-session outcome and alliance measures.
- A. The OQ-45 (Lambert) — Owner: Lambert (Outcome Questionnaire). The OQ-45 tracks progress but does not rate the alliance.
- B. The SCL-90-R and the BSI (Derogatis) — Owner: Derogatis (broad symptom checklists). These are broad symptom checklists, not session-by-session alliance measures.
- **Pair to learn:** Session-by-session outcome and alliance → ORS/SRS (Miller & Duncan), not OQ-45

30 — A

Professional Development & Counselor Self-Awareness · Task 1A.4 · Chapter 22

- **A. Appraise the few trials and reviews that exist for quality — Owner: Best available evidence (Sackett).** A thin research base still warrants appraising what exists — evidence-based practice uses the best available evidence.
- B. Treat it as having no evidence and rely on clinical intuition — Owner: Unaided clinical judgment (Meehl). Meehl found statistical prediction generally matches or beats unaided judgment; sparse evidence is not zero evidence.

- C. Apply PTSD protocols, since both are stress-related disorders — Owner: Overgeneralizing evidence across diagnoses. Sharing a DSM-5-TR chapter does not make PTSD findings apply to adjustment disorder.
- **Pair to learn:** Thin evidence base → appraise the best available evidence

31 — C

Provision of Counseling Interventions · Task 4.7 · Chapter 5

- **C. Viewing the teen's symptoms as part of a family pattern — Owner: Family modality: the system is the client.** In family work the system is the client; symptoms are seen as serving a function, with circular rather than linear causality.
- A. Promising each member that private disclosures stay private — Owner: Unplanned secrets promise (ACA B.4.b calls for confidentiality terms agreed at the start). A secrets policy must be set and explained at the start, not promised ad hoc.
- B. Keeping the focus on the teen's intrapersonal insight — Owner: Individual modality focus. An intrapersonal focus belongs to individual work, not the family modality.
- **Pair to learn:** Individual → family modality: the system is the client (circular, not linear)

32 — A

Intake & Assessment · Task 2B.3 · Chapter 11

- **A. Stimulant intoxication — Owner: Stimulant intoxication (DSM-5-TR).** Stimulant intoxication brings dilated pupils, rapid heart rate, sweating, agitation, and paranoia.
- B. Sedative-hypnotic intoxication — Owner: Sedative, hypnotic, or anxiolytic intoxication. Sedative intoxication resembles alcohol intoxication, with slurred speech, poor coordination, and memory problems, not dilated pupils and paranoia.

- C. Opioid intoxication — Owner: Opioid intoxication. Opioid intoxication causes pinpoint pupils, drowsiness, and slowed breathing.
- **Pair to learn:** Dilated pupils, racing heart, paranoia → stimulant intoxication (not opioids)

33 — B

Indirect Client Care · Task 5.5 · Chapter 23

- **B. A sliding-scale community mental health center — Owner: Community mental health center (sliding scale).** Sliding-scale community centers adjust fees to income and serve uninsured clients.
- A. An out-of-network private group practice — Owner: Out-of-network private care. Out-of-network fees are likely to be unaffordable without insurance.
- C. An employee assistance program (EAP) — Owner: Employee assistance program (employer-sponsored). EAPs are employer-sponsored and short term, so an unemployed client generally would not have access.
- **Pair to learn:** Uninsured client needs ongoing care → sliding-scale community mental health center

34 — B

Intake & Assessment · Task 2B.3 · Chapter 11

- **B. Early remission, in a controlled environment — Owner: DSM-5-TR remission specifiers.** Three to under 12 months with no criteria except craving is early remission, and a locked facility restricting access adds “in a controlled environment.”
- A. Early remission, on maintenance therapy — Owner: On maintenance therapy specifier. Maintenance therapy applies to opioid and tobacco use with agonist or antagonist medication, not described here.
- C. Sustained remission, in a controlled environment — Owner: Sustained remission specifier. Sustained remission requires 12 months or more.
- **Pair to learn:** Locked facility + 3 to 12 months → early remission, in a controlled environment

35 — A

Intake & Assessment · Task 2B.7 · Chapter 19

- **A. Inpatient hospitalization — Owner: Inpatient care: imminent danger, acute mania or psychosis.** Imminent danger plus acute mania calls for 24-hour medically supervised care; that is the least restrictive level that is safe here.
- B. Partial hospitalization — Owner: Partial hospitalization (day program, home at night). She would return home at night, where her family cannot keep her safe.
- C. An intensive outpatient program — Owner: Intensive outpatient (IOP). IOP offers several hours a day while living at home, far less than her imminent danger requires.
- **Pair to learn:** Acute mania with imminent danger → Inpatient hospitalization (not PHP)

36 — C

Treatment Planning & Continuity of Care · Task 3A.5 · Chapter 21

- **C. Target those antecedents, such as a coping plan after late shifts — Owner: Functional analysis (antecedent–behavior–consequence).** Functional analysis identifies the antecedents and consequences that maintain a behavior. Turning the identified triggers into objectives makes the plan targeted.
- A. Keep general abstinence goals for every day of the week — Owner: Generic plan that ignores assessment data. A uniform goal ignores the pattern the assessment found and wastes its guidance.
- B. Require her to quit her job, since work causes her drinking — Owner: Directive mandate (not collaborative planning). Triggers inform coping plans, not ultimatums. Mandating a job change is drastic and not built with the client.

- **Pair to learn:** Use tied to specific triggers → Functional analysis drives objectives

37 — C

Intake & Assessment · Task 2B.1 · Chapter 10

- **C. Hypoarousal outside her window of tolerance — Owner: Siegel (window of tolerance).** Siegel describes trauma narrowing the window of tolerance, so arousal swings into hypoarousal: numbness, shutdown, and dissociation.
- A. Hyperarousal outside her window of tolerance — Owner: Siegel (window of tolerance). Hyperarousal shows as anxiety, panic, or anger, not numbness and shutdown.
- B. Exhaustion stage of general adaptation — Owner: Selye (general adaptation syndrome). Selye's exhaustion stage follows prolonged stress, not a moment-to-moment shutdown when recalling trauma.
- **Pair to learn:** Numbness, shutdown, dissociation → hypoarousal (Siegel), not Selye's exhaustion

38 — B

Intake & Assessment · Task 2B.12 · Chapter 19

- **B. A Z code for uncomplicated bereavement — Owner: Z codes (conditions that may be a focus of clinical attention).** Z codes record conditions such as uncomplicated bereavement that are a focus of attention but are not mental disorders.
- A. Adjustment disorder with depressed mood — Owner: Adjustment disorder (First's step 5). Any disorder, adjustment disorder included, requires clinically significant distress or impairment, which she does not show.
- C. Unspecified depressive disorder — Owner: Unspecified disorder. An unspecified diagnosis still names a mental disorder; expected grief with intact functioning is not one.

- **Pair to learn:** Uncomplicated bereavement → Z code (not a mental disorder)

39 — A

Legal & Ethical Compliance · Task 6.8 · Chapter 17

- **A. Misrepresenting the rendering provider on a claim — Owner: False claims (misrepresentation of the rendering provider).** Claims must identify who actually provided the service; billing under another clinician's NPI misrepresents the service and can constitute fraud.
- B. Acceptable incident-to billing for supervised staff — Owner: Medicare "incident to" rules. Incident-to billing is a narrow Medicare rule with specific supervision conditions; it is not a workaround for pending commercial credentialing.
- C. A permitted locum tenens billing arrangement — Owner: Locum tenens (substitute provider) billing. Locum tenens covers a substitute filling in for an absent enrolled provider; it does not cover a new clinician awaiting credentialing.
- **Pair to learn:** Billing under a colleague's NPI while credentialing is pending → false claim

40 — C

Indirect Client Care · Task 5.3 · Chapter 21

- **C. Only what the physician needs to help with her sleep — Owner: Minimal disclosure (ACA B.2.e).** Even with a release, counselors share only the information essential to the purpose, here her sleep problems.
- A. Her psychotherapy notes, which explain session content — Owner: HIPAA psychotherapy notes (extra protection). Psychotherapy notes are kept separate and generally require specific authorization; they aren't needed here.
- B. Her full counseling record, so the picture is complete — Owner: Exceeds minimal disclosure (ACA

B.2.e). Sharing the whole record goes beyond what the physician needs.

- **Pair to learn:** Coordination with a release → Share the minimum necessary

41 — A

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **A. Accreditation — Owner: Accreditation (programs, e.g., CACREP).** Accreditation approves programs, not individuals; CACREP accredits counseling programs.
- B. Licensure — Owner: Licensure (state permission to practice). Licensure is a state's permission for an individual to practice.
- C. Certification — Owner: Certification (e.g., NCC from NBCC). Certification is a credential given to individuals by a professional body.
- **Pair to learn:** Approves programs → Accreditation (CACREP)

42 — B

Professional Development & Counselor Self-Awareness · Task 1A.6 · Chapter 22

- **B. State their commitment to the Code and seek to resolve it — Owner: ACA I.1.c conflicts between ethics and laws.** I.1.c says counselors make known their commitment to the Code and take steps to resolve the conflict, and may then adhere to the law if it cannot be resolved.
- A. Follow the Code and disregard the conflicting law — Owner: Ethics-over-law error (not the ACA standard). The Code does not direct counselors to ignore the law; it sets a process for resolving the conflict.
- C. Comply with the law without raising the conflict at all — Owner: Unexamined legalism. Complying may be the end point, but only after the counselor has stated the ethical commitment and tried to resolve the conflict.

- **Pair to learn:** Code vs. law conflict → state commitment, try to resolve (ACA I.1.c)

43 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Ask her directly about thoughts of killing herself — Owner: Sudden calm as a warning sign; ask directly.** A sudden calm can follow a decision to die, and putting affairs in order is preparatory behavior; reassess directly.
- A. Explore what helped her feel at peace this week — Owner: Exploring before assessing safety. Safety assessment comes before exploration when warning signs appear.
- C. Note the improvement and space sessions further apart — Owner: Myth: sudden improvement means danger has passed. Treating a warning sign as progress under-responds to risk.
- **Pair to learn:** Sudden calm + putting affairs in order → reassess suicide risk directly

44 — B

Professional Development & Counselor Self-Awareness · Task 1B.10 · Chapter 17

- **B. Seek help and limit or pause her caseload until she can resume — Owner: Impairment (ACA C. 2.g).** C.2.g directs counselors to seek help and limit, suspend, or terminate professional responsibilities until they can practice safely.
- A. Close her practice and end all services by the end of the week — Owner: Abandonment (ACA A.12). Abrupt closure without arranging continuation of care risks abandonment; she can limit services and return when ready.
- C. Keep her full caseload, since the illness isn’t her fault — Owner: Practicing while impaired (punished).

Fault is irrelevant; continuing while impaired puts clients at risk, which C.2.g prohibits.

- **Pair to learn:** Counselor's own impairment → Limit or suspend services (C.2.g)

45 — C

Indirect Client Care · Task 5.6 · Chapter 23

- **C. A current signed release names the officer and its scope — Owner: Written, scoped consent (ACA B.1.c; A.2.e).** Even where law may allow oral agreement for someone present, a signed release naming the officer and what may be shared documents informed consent before anything is disclosed.
- A. The officer agrees to keep what he hears confidential — Owner: Misplaced reliance on a third party's promise. A probation officer may have to report what he learns to the court, so his promise does not protect the client; the client's documented consent does.
- B. The officer has already read the prior notes — Owner: Disclosure without authorization (ACA B.1.c). The officer should not have seen any notes unless a release already allowed it.
- **Pair to learn:** Legal stakeholder joining a session → current signed release naming the person and scope

46 — A

Intake & Assessment · Task 2B.1 · Chapter 10

- **A. Felitti and Anda — Owner: Felitti & Anda (ACE Study).** Felitti and Anda's Adverse Childhood Experiences Study linked more childhood adversity to higher risk of later physical and mental health problems.
- B. Judith Herman — Owner: Herman (Trauma and Recovery). Herman described complex trauma and three stages of recovery.
- C. Bessel van der Kolk — Owner: van der Kolk (The Body Keeps the Score). Van der Kolk wrote about

trauma's effects on the body and brain, but did not conduct the ACE Study.

- **Pair to learn:** ACE Study → Felitti & Anda (not van der Kolk)

47 — C

Intake & Assessment · Task 2B.11 · Chapter 20

- **C. Test her in Spanish and add nonverbal measures before placement — Owner: Diana v. State Board of Education (1970).** Diana held that children must be tested in their primary language, with nonverbal measures, before special education placement.
- A. Give the English WISC-V, since it's the most widely used test — Owner: Ignoring primary language (contrary to Diana; ACA E.8). Testing in English would measure her English, not her ability.
- B. Place her by ability group using last year's group test scores — Owner: Tracking by biased group tests (Hobson v. Hansen). Hobson found ability tracking based on biased group tests unconstitutional.
- **Pair to learn:** Test in the primary language before special education placement → Diana v. State Board of Education

48 — A

Treatment Planning & Continuity of Care · Task 3A.6 · Chapter 21

- **A. Hays's ADDRESSING model — Owner: Pamela Hays (ADDRESSING framework).** Hays's ADDRESSING acronym lists these cultural influences so counselors can weigh each one with the client. It is a practical tool for integrating context into assessment and planning.
- B. Sue's tripartite model of multicultural competence — Owner: Derald Wing Sue (awareness, knowledge, skills). Sue's tripartite model describes the counselor's multicultural competence: awareness of own biases,

knowledge of the client's worldview, and culturally appropriate skills.

- C. Bronfenbrenner's ecological systems model — Owner: Urie Bronfenbrenner (micro- to macrosystem). Bronfenbrenner's model nests the person within microsystem, mesosystem, exosystem, macrosystem, and chronosystem. It maps environmental layers, not a list of identity factors.
- **Pair to learn:** Age, Disability, Religion, Ethnicity, SES, Sexual orientation, Indigenous, National origin, Gender → ADDRESSING (Hays)

49 — C

Provision of Counseling Interventions · Task 4.3 · Chapter 5

- **C. D'Zurilla and Goldfried — Owner: D'Zurilla & Goldfried (problem-solving training).** This five-step problem-solving sequence belongs to D'Zurilla and Goldfried.
- A. Jay Haley — Owner: Haley (strategic first interview). Haley's sequence is social, problem, interaction, goal-setting, task-setting.
- B. James and Gilliland — Owner: James & Gilliland (six-step crisis model). Their model has listening steps then acting steps for crisis intervention.
- **Pair to learn:** Define → alternatives → decide → implement → evaluate → D'Zurilla & Goldfried

50 — A

Intake & Assessment · Task 2B.1 · Chapter 19

- **A. The PC-PTSD-5 — Owner: Trauma screen (2B. 1).** The PC-PTSD-5 is one of the trauma screens counselors use, along with the LEC-5, PCL-5, and ACEs.
- B. The CRAFFT — Owner: Adolescent substance use screen. The CRAFFT screens adolescents for substance use, not trauma.

- C. The GAD-7 — Owner: Anxiety symptom screen. The GAD-7 screens for generalized anxiety, not posttraumatic stress.
- **Pair to learn:** Trauma screen at intake → PC-PTSD-5 (not GAD-7)

51 — B

Legal & Ethical Compliance · Task 6.9 · Chapter 17

- **B. Use unique session links or a locked waiting room for each client — Owner: HIPAA Security Rule access controls (45 CFR 164.312(a)(1)).** Access controls limit each session to authorized participants, preventing one client from entering another's session.
- A. Keep the link, since the vendor signed a business associate agreement — Owner: BAA-as-compliance myth. A BAA sets the vendor's duties but does not configure the practice's settings; the practice must still block unauthorized access.
- C. Ask clients to keep cameras off until they see who is present — Owner: Client-shifted safeguard (error pattern). This moves the security burden onto clients and still allows unauthorized people into the session.
- **Pair to learn:** One shared meeting link → unique links or a waiting room (HIPAA access controls)

52 — B

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **B. David Olson — Owner: Olson (Circumplex Model; FACES).** Cohesion and flexibility are the two dimensions of Olson's Circumplex Model.
- A. Salvador Minuchin — Owner: Minuchin (structural). Minuchin speaks of boundaries and enmeshment, but the two-dimension model is Olson's.
- C. Nathan Epstein — Owner: Epstein (McMaster Model). The McMaster Model uses six dimensions, including roles and behavior control.

- **Pair to learn:** Cohesion + flexibility → Olson's Circumplex (not McMaster)

53 — B

Intake & Assessment · Task 2B.11 · Chapter 18

- **B. Cost-effectiveness analysis — Owner: Cost-effectiveness analysis (program evaluation).** Cost-effectiveness analysis relates costs to nonmonetary outcomes, like cost per client improved.
- A. Process evaluation — Owner: Process evaluation (fidelity). Process evaluation asks whether the program was delivered as planned, not what it costs per outcome.
- C. Cost-benefit analysis — Owner: Cost-benefit analysis (program evaluation). Cost-benefit analysis measures the benefits in dollars, not in units such as clients improved.
- **Pair to learn:** Cost per nonmonetary outcome → Cost-effectiveness analysis (not cost-benefit)

54 — C

Professional Development & Counselor Self-Awareness · Task 1A.7 · Chapter 22

- **C. Rely on the full clinical assessment and document the discrepancy — Owner: Counselor retains clinical responsibility (ACA A.1.a).** AI informs but does not decide; the counselor acts on the full assessment and documents why.
- A. Accept the tool's rating, since algorithms reduce clinician bias — Owner: Automation bias. Deferring to the algorithm over contrary clinical data is automation bias; AI tools can also carry bias.
- B. Disregard all AI output in future clinical decisions — Owner: Algorithm aversion. Rejecting a tool wholesale after one error is algorithm aversion; the lesson is verification, not abandonment.
- **Pair to learn:** AI rating conflicts with the interview → counselor's judgment governs; document

55 — C

Intake & Assessment · Task 2A.1 · Chapter 17

- **C. May keep it confidential after consulting and reviewing the law — Owner: End-of-life decisions (ACA B.2.b).** B.2.b gives counselors the option to maintain confidentiality for terminally ill clients considering hastening death, after consultation and depending on law and circumstances.
- A. Must break confidentiality, as she would for any other suicide risk — Owner: Serious and foreseeable harm (ACA B.2.a). B.2.a permits disclosure to prevent serious and foreseeable harm, but B.2.b specifically gives counselors an option with terminally ill clients.
- B. Should end counseling, since the decision is outside its scope — Owner: Abandonment (ACA A.12). Ending services at such a moment abandons the client when support matters most.
- **Pair to learn:** Terminally ill client considering hastening death → Option to keep confidentiality after consultation (B.2.b)

56 — A

Intake & Assessment · Task 2B.11 · Chapter 18

- **A. Factorial ANOVA — Owner: Factorial ANOVA (two or more IVs; interaction effects).** Two independent variables with a question about their interaction call for a factorial ANOVA, such as a 2 × 2 design.
- B. One-way ANOVA — Owner: One-way ANOVA (one IV, three or more groups). A one-way ANOVA handles only one independent variable and can’t test an interaction.
- C. ANCOVA — Owner: ANCOVA (means compared while controlling a covariate). ANCOVA adjusts for a covariate; gender here is a second independent variable whose interaction is of interest.
- **Pair to learn:** Two or more IVs and their interaction → Factorial ANOVA

57 — A

Indirect Client Care · Task 5.7 · Chapter 23

- **A. Qualified interpreter — Owner: Interpreter (spoken or signed language).** Interpreters convey spoken or signed language in real time.
- B. Certified translator — Owner: Translator (written text). Translators convert written documents rather than live conversation.
- C. Bilingual receptionist without training — Owner: Untrained bilingual staff. Being bilingual is not the same as being a trained interpreter, and front-desk staff also raise confidentiality and role issues.
- **Pair to learn:** Spoken words in real time → interpreter; written documents → translator

58 — A

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **A. Report it, since substantial harm to the client is likely — Owner: Reporting colleague violations (ACA I.2.b).** When a violation has caused or is likely to cause substantial harm, counselors take further action, such as reporting to the licensing board.
- B. Talk with him privately and let him end it on his own terms — Owner: Informal resolution (ACA I.2.a). Informal resolution fits minor violations; a sexual relationship with a current client is likely to cause substantial harm.
- C. Keep his confidence, since he told her this as a friend — Owner: Ignoring a harmful violation (punished). Friendship doesn't outweigh the duty to protect clients; ignoring a harmful violation is what the exam punishes.
- **Pair to learn:** Colleague violation with substantial harm → Report (I.2.b), not informal resolution

59 — B

Provision of Counseling Interventions · Task 4.2 · Chapter 3

- **B. Disintegration — Owner: Helms (White racial identity).** Helms's order is contact, disintegration, reintegration, pseudo-independence, immersion/emersion, autonomy.
- A. Reintegration — Owner: Helms (White racial identity). Reintegration follows disintegration, so it is two steps after contact.
- C. Encounter — Owner: Cross (nigrescence). Encounter is the second stage of Cross's model of Black racial identity.
- **Pair to learn:** Contact → disintegration → reintegration → Helms (not Cross)

60 — B

Treatment Planning & Continuity of Care · Task 3A.7 · Chapter 21

- **B. Explain the accurate diagnosis, its basis, and who sees it — Owner: Proper diagnosis with informed consent (ACA E.5.a, A.2.b).** ACA E.5.a calls for proper diagnosis, and A.2.b includes the implications of diagnosis in informed consent. Explaining the basis and who will see it addresses her worry honestly.
- A. Record the milder diagnosis to protect her privacy — Owner: Misrepresentation of diagnosis. Recording an inaccurate diagnosis misrepresents the client's condition and can amount to fraud if billed.
- C. Leave every diagnosis out of the record from now on — Owner: Refraining from diagnosis misapplied (ACA E.5.d). ACA E.5.d allows refraining from a diagnosis when it would harm the client, not as a routine way to keep records looking better.
- **Pair to learn:** Client asks for a milder diagnosis → Accurate diagnosis, explain implications (ACA E.5.a)

61 — B

Indirect Client Care · Task 5.2 · Chapter 17

- **B. Assess whether the client intends to inform the partner — Owner: Contagious, life-threatening diseases (ACA B.2.c, 2014).** Before disclosing, counselors assess the client's intent to inform the third party or to engage in harmful behavior, and follow state law.
- A. Apply the Tarasoff duty to warn standard for violent threats — Owner: Tarasoff I (duty to warn of violence). Tarasoff concerns serious threats of physical violence; disease transmission falls under B.2.c and state law.
- C. Confirm the diagnosis with the client's physician — Owner: 2005 ACA Code requirement (dropped in 2014). Confirming the diagnosis was a step in the 2005 Code; the 2014 Code removed it.
- **Pair to learn:** Before disclosing contagious disease → Assess client's intent to inform (B.2.c, 2014)

62 — B

Treatment Planning & Continuity of Care · Task 3A.9 · Chapter 21

- **B. Goals he didn't help choose conflict with his autonomy — Owner: Collaborative goal setting (3A.9); autonomy.** Goals the counselor sets alone are less likely to be met and conflict with the client's autonomy.
- A. Goals can't be set until a full ASAM review is done — Owner: ASAM Criteria (substance use level of care). ASAM guides level-of-care placement for substance use; it isn't required before every treatment plan.
- C. The plan lists no barriers that could block his progress — Owner: Barriers (3A.4). Barriers do belong in a plan, but the central flaw is that the client had no voice in his own goals.
- **Pair to learn:** Counselor-written goals → Collaboration and client autonomy (3A.9)

63 — C

Legal & Ethical Compliance · Task 6.10 · Chapter 17

- **C. Bill the diagnosis that the assessment supports — Owner: ACA C.6.b (reports to third parties); E. 5.a (proper diagnosis).** Counselors report accurately to insurers and diagnose based on assessment, documenting the criteria and impairment that support it.
- A. Use the severe diagnosis but note doubts in the record — Owner: False claims (diagnosis chosen for coverage). Documenting doubts does not cure a claim built on a diagnosis the counselor does not believe is accurate.
- B. Ask the client which diagnosis he would prefer — Owner: Client collusion (misused informed consent). Diagnosis rests on assessment, not client preference; involving the client does not make an inaccurate claim honest.
- **Pair to learn:** Diagnosis picked for coverage → bill the accurate diagnosis (ACA C.6.b)

64 — B

Treatment Planning & Continuity of Care · Task 3B.2 · Chapter 21

- **B. Booster sessions — Owner: Booster sessions (follow-up after discharge, 3B.2).** Follow-up contacts after ending that support maintenance are booster sessions.
- A. Tapered sessions — Owner: Tapering (reducing frequency before ending). Tapering spaces out sessions before termination; these contacts happen after counseling has ended.
- C. Relapse prevention — Owner: Relapse prevention (termination task). Relapse prevention is the plan for warning signs and coping, built before ending; the stem describes scheduled follow-up contacts.

- **Pair to learn:** Scheduled check-ins after ending → Booster sessions (3B.2)

65 — C

Provision of Counseling Interventions · Task 4.2 · Chapter 15

- **C. Richard Troiden — Owner: Troiden (sexual identity model).** Troiden's model moves from sensitization through identity confusion and identity assumption to commitment.
- A. Vivienne Cass — Owner: Cass (homosexual identity formation). Cass's stages run from identity confusion to synthesis, with comparison, tolerance, acceptance, and pride between.
- B. Anthony D'Augelli — Owner: D'Augelli (life-span LGB model). D'Augelli described six interrelated processes rather than fixed stages.
- **Pair to learn:** Sensitization → commitment → Troiden (not Cass)

66 — A

Professional Development & Counselor Self-Awareness · Task 1A.8 · Chapter 22

- **A. A research-based course with skills practice and consultation — Owner: Training with practice and consultation (skill transfer).** Knowledge plus skills practice and follow-up consultation is what turns training into competent practice.
- B. A one-hour webinar that grants a certificate of completion — Owner: Certificate-of-attendance error. A brief webinar may raise awareness, but a certificate documents attendance, not skill.
- C. A podcast of first-person recovery stories from that population — Owner: Awareness-only exposure. Personal stories can build empathy and awareness but do not teach clinical skills on their own.
- **Pair to learn:** Training that builds skill → evidence-based content, practice, consultation

67 — A

Provision of Counseling Interventions · Task 4.9 · Chapter 17

- **A. Must still use forethought and document it isn't exploitative — Owner: Former clients after 5 years (ACA A.5.c).** After 5 years, A.5.c still requires forethought and documentation that the relationship isn't exploitative, and avoiding it if exploitation is possible.
- B. Is permanently barred from dating this former client — Owner: Previous partners rule (ACA A.5.b), misapplied. The permanent bar in A.5.b concerns counseling a former romantic partner; A.5.c sets a 5-year limit, not a lifetime ban, on dating former clients.
- C. Is free to proceed, since well over 2 years have passed — Owner: 2-year time frame (not the ACA standard). The ACA standard is 5 years, not 2, and even after 5 years conditions still apply.
- **Pair to learn:** Former client after 5 years → Forethought and documentation still required (A.5.c)

68 — C

Indirect Client Care · Task 5.8 · Chapter 17

- **C. Fix the draft before signing; the note is her responsibility — Owner: Counselor responsibility for AI output (ACA AI Work Group; Ch 17 Part 11).** The counselor is responsible for every note, including AI-drafted ones, so she must review and fix inaccurate content before it becomes part of the record.
- A. Sign it as drafted, since the tool meets HIPAA requirements — Owner: HIPAA compliance (privacy and security, not accuracy). HIPAA compliance addresses privacy and security; it doesn't make the tool's clinical content accurate or shift responsibility from the counselor.

- B. Sign it now and add a dated correction to the note afterward — Owner: Error correction rule for finalized records (dated correction). Dated corrections fix errors discovered in finalized entries; knowingly signing an inaccurate note is not the way to use that rule.
- **Pair to learn:** AI-drafted note → Counselor reviews and is responsible for it

69 — B

Indirect Client Care · Task 5.1 · Chapter 23

- **B. Share only what is needed for school strategies — Owner: Minimal disclosure (ACA B.2.e).** Consent permits contact, and sharing only what the school needs keeps the disclosure to the minimum.
- A. Send the full session notes to the school counselor — Owner: Excessive disclosure. The school does not need session content; sending full notes discloses far more than the purpose requires.
- C. Avoid contact, since school records fall under HIPAA — Owner: Misapplied HIPAA (FERPA governs education records). HIPAA does not bar consented coordination, and school-held records are generally governed by FERPA rather than HIPAA.
- **Pair to learn:** Coordinating with a school → consent, share only what the strategies need

70 — A

Provision of Counseling Interventions · Task 4.4 · Chapter 7

- **A. Taking responsibility — Owner: Gottman (antidote to defensiveness).** Defensiveness is one of the Four Horsemen; its antidote is taking responsibility.
- B. A gentle startup — Owner: Gottman (antidote to criticism). Gentle startup is the antidote to criticism.
- C. Physiological self-soothing — Owner: Gottman (antidote to stonewalling). Self-soothing is the antidote to stonewalling.

- **Pair to learn:** Defensiveness → antidote: taking responsibility (Gottman)

71 — A

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **A. Donald Winnicott — Owner: Winnicott (object relations).** The good-enough mother and the holding environment are Winnicott's concepts.
- B. John Bowlby — Owner: Bowlby (attachment). Bowlby owns the secure base and internal working models.
- C. Melanie Klein — Owner: Klein (object relations). Klein is an object relations founder, but these two terms are Winnicott's.
- **Pair to learn:** Good-enough mother; holding environment → Winnicott (not Bowlby)

72 — C

Legal & Ethical Compliance · Task 6.11 · Chapter 17

- **C. Discuss it with him and seek his consent or assent — Owner: ACA A.2.d (inability to give consent).** With clients whose capacity is limited, counselors seek assent, include them in decisions as appropriate, and review the device's data privacy.
- A. Agree, since family safety concerns override his consent — Owner: Covert monitoring (paternalism). Hiding the device bypasses the client's autonomy and his right to take part in decisions about his care.
- B. Refuse any tracking, since it always violates autonomy — Owner: Rigid avoidance (not the ACA standard). Monitoring technology can be appropriate when introduced with consent or assent and tied to a safety plan.
- **Pair to learn:** Tracking device for a client with memory loss → seek consent or assent (ACA A.2.d)

73 — B

Legal & Ethical Compliance · Task 6.4 · Chapter 9

- **B. H.3, client verification — Owner: ACA Code H.3.** H.3 requires verifying the client's identity at the start and throughout distance counseling.
- A. H.4.c, technology-assisted services — Owner: ACA Code H.4.c. H.4.c covers whether the client can use the technology and whether it fits the client's needs, not identity verification.
- C. H.1.b, laws and statutes — Owner: ACA Code H.1.b. H.1.b covers following the laws of the counselor's and client's locations.
- **Pair to learn:** Code words to confirm identity → ACA H.3 (client verification)

74 — B

Intake & Assessment · Task 2A.1 · Chapter 17

- **B. Contact the client and assert privilege on the client's behalf — Owner: Subpoena response (assert privilege; Jaffee v. Redmond, privileged communication).** A subpoena isn't a court order; the counselor contacts the client (and/or the client's attorney), seeks legal guidance, and asserts the client's privilege if it hasn't been waived.
- A. Disregard the subpoena, since the client hasn't given consent — Owner: Ignoring a subpoena (legal risk). Ignoring a subpoena can bring legal consequences; the counselor responds by asserting privilege, not by doing nothing.
- C. Send the records, since a subpoena is a legal order to comply — Owner: Court order (judge-signed), confused with a subpoena. Compliance with minimal disclosure applies to a judge-signed court order; releasing records on a subpoena without consent is what the exam punishes.
- **Pair to learn:** Attorney-issued subpoena → Contact client, assert privilege

75 — A

Professional Development & Counselor Self-Awareness · Task 1B.1 · Chapter 22

- **A. Finish the risk assessment, then seek consultation and support — Owner: Self-monitoring for impairment (ACA C.2.g).** Name the fatigue, keep the safety work intact, then address the fatigue through consultation and support.
- B. Accept the denial and end the session early to recover energy — Owner: Fatigue-driven shortcut (compassion fatigue). Exhaustion is lowering the bar for risk assessment; a denial alone is not an assessment.
- C. Refer the client to another counselor before assessing risk — Owner: Client neglect (ACA A.12). Handing off a client mid-session without assessing risk leaves the client unprotected.
- **Pair to learn:** Fatigue lowering the bar on risk → finish safety work, then get support

76 — B

Intake & Assessment · Task 2B.11 · Chapter 19

- **B. The CFI informant version — Owner: DSM-5-TR Cultural Formulation Interview, informant version.** The CFI has an informant version for family members, gathering their cultural view of the problem.
- A. A 4 Ps formulation of the daughter's report — Owner: 4 Ps case formulation. The 4 Ps organize intake information into predisposing, precipitating, perpetuating, and protective factors; they are not a cultural interview.
- C. The WHODAS 2.0, filled out by the daughter — Owner: WHODAS 2.0 (functioning). The WHODAS 2.0 measures functioning, not how the family culturally understands the problem.
- **Pair to learn:** Family member's cultural view of the problem → CFI informant version

77 — C

Intake & Assessment · Task 2A.1 · Chapter 17

- **C. She will protect it and ask members to, but can't guarantee it — Owner: Confidentiality in groups (ACA B.4).** Counselors explain that confidentiality can't be guaranteed in groups, while clarifying their own commitment and asking members to respect it.
- A. A signed group agreement guarantees that nothing will leave — Owner: Overpromising confidentiality. A signed agreement encourages privacy but can't guarantee it; promising otherwise misinforms members.
- B. Privileged communication protects what members say in court — Owner: Privileged communication (legal, client-held). Privilege is a legal right that varies by state and doesn't guarantee that group members keep what they hear private.
- **Pair to learn:** Group privacy → Can't be guaranteed (ACA B.4)

78 — B

Indirect Client Care · Task 5.2 · Chapter 23

- **B. Write a support letter and link her to legal aid — Owner: ACA A.7.a advocacy + legal-aid referral.** Counselors advocate within their role, such as documenting needs, and connect clients with legal services for the legal steps.
- A. File a legal complaint on her behalf — Owner: Attorney role (legal representation). Filing legal actions is the work of attorneys, not counselors.
- C. Stay out of it, since advocacy is outside counseling — Owner: Rejecting the advocacy role. The ACA Code supports advocacy to address barriers that limit client growth.
- **Pair to learn:** Client faces a legal or system barrier → advocate within role and link to legal aid

79 — B

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **B. "When Maya stays home, who notices first? What does Dad do?" — Owner: Milan (circular questioning).** Circular questions put the pattern, not one member, at the center.
- A. "Why do you think Maya keeps refusing to go to school lately?" — Owner: Linear "why" question. A linear question locates the cause inside one member.
- C. "How many school days has Maya missed this month?" — Owner: Closed question. A closed question gathers a fact but not the family pattern.
- **Pair to learn:** Who notices, what happens next → circular question (Milan)

80 — C

Provision of Counseling Interventions · Task 4.8 · Chapter 9

- **C. Moral injury — Owner: Veterans: moral injury.** Distress after acts that violate one's own moral code is moral injury.
- A. Military sexual trauma — Owner: Veterans: MST. MST involves sexual assault or harassment during service.
- B. Disenfranchised grief — Owner: Doka. Disenfranchised grief is loss that isn't socially recognized.
- **Pair to learn:** Violated one's own moral code → moral injury

81 — C

Legal & Ethical Compliance · Task 6.12 · Chapter 17

- **C. Notify the liability carrier promptly, per the policy — Owner: Policy notice-of-claim condition.** Policies require prompt notice of claims and

complaints, and many provide licensing board defense coverage and legal guidance.

- A. Call the client to resolve the complaint informally — Owner: Contacting the complainant (retaliation risk). Contacting the complainant can appear coercive and may worsen the complaint.
- B. Send the board a detailed response the same day — Owner: Unadvised response to the board. A rushed response without advice can include errors or unnecessary disclosures of client information.
- **Pair to learn:** Licensing board complaint → notify the liability carrier promptly

82 — C

Intake & Assessment · Task 2B.12 · Chapter 19

- **C. The principal diagnosis — Owner: Principal diagnosis (DSM-5-TR).** The principal diagnosis is the condition chiefly responsible for the admission, and it is listed first.
- A. The Axis I diagnosis — Owner: DSM-IV multiaxial system (eliminated in DSM-5). The DSM-5 eliminated the multiaxial system, so Axis I is no longer current practice.
- B. The provisional diagnosis — Owner: Provisional diagnosis. Provisional is used when criteria are expected to be met but information is incomplete; it does not mark the reason for the admission.
- **Pair to learn:** Chiefly responsible for the admission, listed first → Principal diagnosis (not Axis I)

83 — A

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **A. Edwin Shneidman — Owner: Shneidman (suicidology).** Shneidman, the founder of suicidology, introduced psychache.
- B. Aaron Beck — Owner: Beck (hopelessness). Beck linked hopelessness to suicide risk.

- C. Thomas Joiner — Owner: Joiner (interpersonal theory of suicide). Joiner owns thwarted belongingness, perceived burdensomeness, and acquired capability.
- **Pair to learn:** Psychache → Shneidman (not Joiner)

84 — A

Professional Development & Counselor Self-Awareness · Task 1B.2 · Chapter 22

- **A. Check the reaction against client data and assess own exposure — Owner: Vicarious trauma self-assessment (disrupted safety schemas).** Vicarious trauma can shift a counselor's sense of danger; client reports are the check, and the counselor's exposure is what to assess.
- B. Trust it as clinical intuition and add more frequent sessions — Owner: Mistaking a trauma-shaped view for intuition. Acting on an unexamined sense of danger can over-treat the client and reinforces the distortion.
- C. Tell the client about the caseload to explain the tension — Owner: Counselor-serving self-disclosure. Disclosure meant to explain the counselor's state serves the counselor, not the client.
- **Pair to learn:** Sense of danger outruns client data → assess own vicarious trauma

85 — A

Treatment Planning & Continuity of Care · Task 3B.8 · Chapter 20

- **A. The change is within one SEM, so it may reflect measurement error — Owner: SEM confidence band (interpret scores as ranges).** The 68% band around 30 is 27 to 33, and 28 falls inside it, so the two-point drop may be error rather than real change.
- B. A ceiling effect on the measure is hiding the client's real change — Owner: Ceiling effect (test too easy; many score at the top). Nothing suggests scores are bunched at the top of the scale.

- C. The two-point drop confirms that counseling worked — Owner: Treating scores as exact points (ignores SEM). Every observed score has error; a change smaller than one SEM does not confirm improvement.
- **Pair to learn:** Change smaller than the SEM → May be measurement error; interpret scores as ranges

86 — C

Treatment Planning & Continuity of Care · Task 3A.3 · Chapter 21

- **C. Assessing lethality and safety planning with her — Owner: Safety first (sequencing priorities; Campbell's lethality indicators).** Safety comes first in sequencing; prior strangulation is a high lethality indicator, so assessment and safety planning come before other goals.
- A. Inviting her partner to join couples sessions — Owner: Couples counseling (contraindicated with ongoing IPV). Conjoint work is contraindicated while violence is ongoing and could increase her danger.
- B. Assertiveness training for her concerns at work — Owner: Client's most pressing stated concern (sequencing step 4). Her stated concern matters, but it comes after safety.
- **Pair to learn:** Danger at home → Safety first in sequencing

87 — B

Indirect Client Care · Task 5.4 · Chapter 23

- **B. Stimulants — Owner: Stimulants (methylphenidate, amphetamines).** Reduced appetite and insomnia are common side effects of stimulants, which the counselor documents and shares with the prescriber.
- A. Second-generation antipsychotics — Owner: Second-generation antipsychotics (weight gain, metabolic effects). Many of these drugs increase appetite and

weight, with metabolic risk that varies by drug, so appetite loss would be atypical.

- C. Benzodiazepines — Owner: Benzodiazepines (sedation, dependence). Benzodiazepines mainly cause sedation and drowsiness, not appetite loss and trouble falling asleep.
- **Pair to learn:** Child with reduced appetite and insomnia on a new medication → stimulant side effects

88 — A

Intake & Assessment · Task 2B.8 · Chapter 19

- **A. "How are these symptoms affecting your work and daily life?" — Owner: Functioning (2B.8): impact on work, relationships, and self-care.** Functioning is judged by how symptoms affect daily activities such as work, school, relationships, and self-care.
- B. "How would you describe this problem to your family and friends?" — Owner: CFI domain 1 (cultural definition of the problem). This CFI question explores the client's cultural definition of the problem, not functioning.
- C. "From 0 to 100, how upsetting does this feel right now?" — Owner: SUDS (distress, 2B.4). A SUDS rating measures subjective distress, which can differ sharply from actual functioning.
- **Pair to learn:** Impact on work and daily life → Functioning (2B.8), not distress (SUDS)

89 — C

Treatment Planning & Continuity of Care · Task 3A.2 · Chapter 21

- **C. Client will score below 10 on the PHQ-9 within 12 weeks — Owner: Short-term objective (measurable step).** It is a measurable, time-bound step toward the long-term goal.
- A. Client reports low mood and misses work two days a week — Owner: Problem statement (concern in

behavioral terms). This describes the presenting concern in behavioral terms, which is the problem statement.

- B. Client will experience reduced depression overall — Owner: Long-term goal (broad outcome). This is a broad outcome, which describes a long-term goal rather than a measurable step.
- **Pair to learn:** Measurable, time-bound step → Short-term objective

90 — C

Legal & Ethical Compliance · Task 6.13 · Chapter 17

- **C. Apply for a license in the new home state under Compact rules — Owner: Counseling Compact (change of home state).** A licensee moving between member states applies for a new home-state license; the former home-state license converts to a privilege once the new license is active.
- A. Keep her former home-state license as her home license — Owner: Home state misread (defined by primary residence). Under the Compact, the home state is the primary state of residence, so the former state can no longer serve as home.
- B. Practice in her new state on a privilege indefinitely — Owner: Privilege used as a home license (Compact misread). A privilege covers remote states; it cannot stand in for a license in the state where the counselor lives.
- **Pair to learn:** Moving between Compact states → license in the new home state

91 — B

Intake & Assessment · Task 2B.6 · Chapter 16

- **B. The spread effect — Owner: Beatrice Wright (disability psychology).** Wright's spread effect is assuming that one limitation means global limitation.
- A. Cultural imperialism — Owner: Iris Marion Young (faces of oppression). Cultural imperialism is one of

Young's faces of oppression, not the specific error of generalizing one limitation.

- C. Stereotype threat — Owner: Steele & Aronson. Stereotype threat affects the target's own performance; the error here is the manager's assumption.
- **Pair to learn:** Assuming one limitation means global limitation → spread effect (Beatrice Wright)

92 — C

Treatment Planning & Continuity of Care · Task 3A.9 · Chapter 21

- **C. Respect that value and explore how to involve them — Owner: Respecting cultural values in goal setting (3A.9).** Collaboration includes respecting cultural values about family involvement in goals and help-seeking.
- A. Encourage her to set her own goals independently first — Owner: Cultural encapsulation (Wrenn). Pushing an individualistic standard substitutes the counselor's cultural assumptions for the client's.
- B. Let her parents set her goals for her instead — Owner: Overriding client autonomy. Involving her family doesn't mean handing them the decision; she still chooses her goals.
- **Pair to learn:** Collectivist family involvement → Respect cultural values in goals (3A.9)

93 — A

Professional Development & Counselor Self-Awareness · Task 1B.3 · Chapter 22

- **A. Verify the board's supervisor training or credential requirements — Owner: ACA F.2.a supervisor preparation.** Counselors are trained in supervision before offering it, and many boards set their own supervisor requirements, which vary by state.

- B. Agree, since years of practice qualify a counselor to supervise — Owner: Experience-equals-competence error. Supervision is a distinct competency; years of practice do not train someone to supervise.
- C. Draft a supervision contract and begin the following week — Owner: Supervision informed consent (ACA F. 4.a), premature. A supervision agreement is appropriate, but only after the counselor confirms being qualified to supervise.
- **Pair to learn:** Asked to supervise for licensure → verify supervisor training and credential (ACA F.2.a)

94 — B

Treatment Planning & Continuity of Care · Task 3B.2 · Chapter 21

- **B. A follow-up appointment within days, with safety planning — Owner: Follow-up after discharge (3B.2; elevated post-discharge risk).** The days and weeks after discharge carry elevated suicide risk, so prompt follow-up and safety planning matter most.
- A. A first outpatient appointment sometime within the next month — Owner: Delayed follow-up during a high-risk period. Waiting weeks leaves the client unsupported during the period of highest risk.
- C. Waiting for the client to call once she feels ready — Owner: Passive follow-up (no plan). Leaving contact up to the client provides no plan during an elevated-risk period.
- **Pair to learn:** High-risk period after hospital discharge → Prompt follow-up

95 — B

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **B. Here-and-now awareness — Owner: Perls (Gestalt here-and-now).** Directing attention to

present experience is the Gestalt here-and-now focus; it says nothing about the counselor-client relationship.

- A. Immediacy — Owner: Immediacy (microskill). Immediacy addresses what is happening between counselor and client right now.
- C. An interpretation — Owner: Interpretation (microskill). Interpretation proposes a connection outside the client's awareness.
- **Pair to learn:** Present-moment awareness, no relationship comment → here-and-now (Gestalt), not immediacy

96 — B

Indirect Client Care · Task 5.5 · Chapter 23

- **B. Offering several vetted options that fit the need — Owner: Client choice among vetted referrals (ACA A.11.a).** Several vetted options that match the need let the client choose while ensuring quality.
- A. Naming only the one colleague the counselor prefers — Owner: Single referral (limits choice). Giving only one name limits the client's choice and can look like a conflict of interest.
- C. Asking the client to search online without any help — Owner: Unsupported self-referral. Sending the client to search alone shifts the work to the client and skips vetting.
- **Pair to learn:** Making a referral → several vetted options so the client chooses

97 — C

Treatment Planning & Continuity of Care · Task 3A.11 · Chapter 16

- **C. Oppositional defiant disorder — Owner: DSM-5-TR ODD.** ODD involves angry or irritable mood, argumentative or defiant behavior, or vindictiveness, with at least 4 symptoms for 6 months.
- A. Disruptive mood dysregulation disorder — Owner: DSM-5-TR DMDD. DMDD requires severe outbursts 3

or more times a week with persistent irritability for 12 months.

- B. Conduct disorder — Owner: DSM-5-TR conduct disorder. Conduct disorder involves violating others' rights, such as aggression, destruction, or theft, which are absent.
- **Pair to learn:** Angry, defiant, 4+ symptoms, 6 months, no rights violations → ODD (not conduct disorder)

98 — C

Professional Development & Counselor Self-Awareness · Task 1A.2 · Chapter 17

- **C. Vicarious liability — Owner: Vicarious liability (respondeat superior).** Under vicarious liability, supervisors can be held legally responsible for their supervisees' actions.
- A. Direct liability — Owner: Direct liability (supervisor's own negligent supervision). Direct liability applies when the supervisor's own supervision was negligent, which the stem rules out.
- B. Dereliction of duty — Owner: Malpractice element: dereliction (breach). Dereliction is the breach element of a malpractice claim, not the doctrine that assigns responsibility to a supervisor.
- **Pair to learn:** Supervisor liable for supervisee's acts → Vicarious liability

99 — C

Legal & Ethical Compliance · Task 6.14 · Chapter 17

- **C. Arrange a qualified interpreter at no charge to the client — Owner: ADA Title III auxiliary aids (28 CFR 36.303).** Practices provide auxiliary aids, such as qualified interpreters, at no cost, subject only to the narrow undue-burden limit.
- A. Ask the client to bring a family member to interpret — Owner: Companion interpreting (restricted, 28 CFR 36.303(c)). The ADA bars requiring clients to bring

their own interpreter, and family interpreting compromises confidentiality.

- B. Exchange written notes with the client during sessions — Owner: Written notes (effective only for brief exchanges). Notes may work for simple exchanges, but counseling involves complex communication that notes do not convey effectively.
- **Pair to learn:** Deaf client requests an interpreter → qualified interpreter at no cost (ADA Title III)

100 — B

Provision of Counseling Interventions · Task 4.1 · Chapter 7

- **B. Virginia Satir — Owner: Satir (human validation / experiential).** Family reconstruction is one of Satir's signature techniques for family-of-origin work.
- A. Murray Bowen — Owner: Bowen (multigenerational). Bowen's family-of-origin tools are the genogram, process questions, and coaching differentiation.
- C. Carl Whitaker — Owner: Whitaker (symbolic-experiential). Whitaker is known for co-therapy, playfulness, and absurdity.
- **Pair to learn:** Family reconstruction → Satir (not Bowen)

101 — B

Legal & Ethical Compliance · Task 6.2 · Chapter 13

- **B. Powerlessness — Owner: Finkelhor & Browne (traumagenic dynamics).** Powerlessness is the dynamic in which the child's will and sense of control are repeatedly overridden.
- A. Stigmatization — Owner: Finkelhor & Browne (traumagenic dynamics). Stigmatization involves shame and feeling "damaged," not the inability to stop the abuse.

- C. Betrayal — Owner: Finkelhor & Browne (traumagenic dynamics). Betrayal concerns broken trust in someone the child depended on, not overridden control.
- **Pair to learn:** Will and control repeatedly overridden → powerlessness (Finkelhor & Browne)

102 — A

Professional Development & Counselor Self-Awareness · Task 1B.4 · Chapter 22

- **A. Compare self-ratings with outcome data and supervisor feedback — Owner: ACA C.2.d monitor effectiveness (outcome feedback).** External data such as client outcomes and supervisor or peer feedback check the blind spots of self-assessment.
- B. Rely on how confident the counselor feels during sessions — Owner: Self-assessment bias. Clinicians tend to overrate their own effectiveness (Walfish et al., 2012), so felt confidence is a poor gauge of skill.
- C. Rely on the counselor's total years of general practice — Owner: Experience-equals-expertise error. Years in practice do not show skill with a specific disorder without outcome evidence.
- **Pair to learn:** Judging your own skill → outcome data and feedback, not confidence

103 — A

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **A. "With your car out again, you're really worried about getting to work." — Owner: Carkhuff level 3 (interchangeable).** It captures the same content (the breakdown) and feeling (worry about getting to work) the client expressed, without adding or subtracting.
- B. "You're worried about getting to work, and maybe tired of always being one breakdown from falling behind." — Owner: Carkhuff level 4 (additive). It adds a meaning just below the surface (being tired of living

one setback from falling behind), which makes it level 4, not interchangeable.

- C. “Cars always seem to break down at the worst time.” — Owner: Carkhuff level 2 (subtracts). A generalization about cars loses the client’s specific worry about getting to work.
- **Pair to learn:** Same content and feeling → Carkhuff level 3 (interchangeable), not level 4

104 — C

Intake & Assessment · Task 2B.12 · Chapter 19

- **C. Other specified anxiety disorder — Owner: Other specified disorder (DSM-5-TR).** “Other specified” is used when the clinician records why full criteria are not met.
- A. Panic disorder (provisional) — Owner: Provisional diagnosis. Provisional assumes full criteria will likely be met once more information arrives; here the symptom count itself falls short.
- B. Unspecified anxiety disorder — Owner: Unspecified disorder (DSM-5-TR). “Unspecified” is chosen when the clinician cannot, or chooses not to, state why criteria are not met; this counselor wants to state it.
- **Pair to learn:** Clinician states why criteria aren’t met → Other specified (not unspecified)

105 — B

Indirect Client Care · Task 5.6 · Chapter 23

- **B. Track progress and adjust the treatment focus — Owner: Measurement-based care.** Reviewing repeated scores shows the trend in symptoms and guides changes to treatment.
- A. Confirm the diagnosis without further assessment — Owner: Screening misused as diagnosis. The PHQ-9 is a screening and severity measure; it does not confirm a diagnosis alone.

- C. Meet billing rules for the session code — Owner: Billing compliance. Billing is not the clinical purpose of reviewing outcome scores.
- **Pair to learn:** Reviewing repeated symptom scores → measurement-based care

106 — A

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **A. Listen non-defensively and invite her to raise it in group — Owner: Encouraging direct member-leader interaction (transition stage).** Leaders respond to challenges non-defensively and encourage members to express reactions directly in the group.
- B. Explain the reasons for each interruption so she understands — Owner: Defending the leader. Explaining or justifying is a defensive response to a challenge.
- C. Agree to keep the complaint between the two of them — Owner: Creating a secret with one member. A private side agreement keeps the reaction out of the group's process.
- **Pair to learn:** Member's reaction to the leader → receive non-defensively, bring it into group

107 — A

Treatment Planning & Continuity of Care · Task 3B.1 · Chapter 21

- **A. Stepping up to intensive outpatient or partial hospitalization — Owner: Step up to least restrictive higher level of care (3B.1).** Worsening symptoms that outpatient care isn't handling call for stepping up to the least restrictive level that's safe and effective, chosen with the client.
- B. Continuing weekly sessions under the current treatment plan — Owner: Continuing an ineffective plan unchanged (punished). Repeating a plan that isn't working is what the exam punishes.

- C. Inpatient hospitalization, given how poorly she is functioning — Owner: Inpatient care (over-restrictive without imminent risk). Without imminent risk, inpatient care is more restrictive than needed.
- **Pair to learn:** Worsening, no imminent risk → Step up to IOP or PHP

108 — C

Legal & Ethical Compliance · Task 6.15 · Chapter 17

- **C. A professional will naming a colleague to manage records — Owner: ACA C.2.h (counselor incapacitation, death, retirement, or termination).** Counselors prepare a plan for transferring clients and files to a designated colleague or records custodian.
- A. Permission for her family to open client files — Owner: Unauthorized disclosure (ACA B.1.c). Family members are not authorized to access client records simply because they are next of kin.
- B. A line in each consent form that services may end — Owner: Informed consent disclosure (ACA A.2.b). Disclosing that services may be interrupted is useful but does not arrange for anyone to act.
- **Pair to learn:** Counselor incapacitated → professional will with records custodian (ACA C.2.h)

109 — C

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **C. Assess suicide and medical risk, then build coping skills — Owner: NSSI response (assessment plus DBT-type skills).** NSSI still requires suicide and medical assessment, followed by work on alternative coping.
- A. Make continued counseling conditional on her stopping now — Owner: Ultimatum (not recommended for NSSI). Making counseling conditional on stopping harms the alliance and builds no skills.

- B. Arrange an inpatient admission today to keep her safe — Owner: Over-response without assessment. Hospitalizing before assessing risk is not matched to the level of risk.
- **Pair to learn:** NSSI → assess suicide and medical risk, build coping; no ultimatums

110 — A

Treatment Planning & Continuity of Care · Task 3B.9 · Chapter 21

- **A. May end services, with safety steps and referrals — Owner: Termination when the counselor is in danger (ACA A.11.c).** A.11.c permits termination when the counselor is in danger from the client, with appropriate safety steps and referrals to other providers.
- B. Must continue, since ending would be abandonment — Owner: Abandonment (ACA A.12), misapplied. Ending services for a legitimate A.11.c reason, with referrals, is not abandonment.
- C. Must warn his family under the Tarasoff duty to protect — Owner: Duty to protect (Tarasoff II). The duty to protect concerns a third party at risk; it doesn't require warning the client's family here.
- **Pair to learn:** Counselor in danger from client → Termination permitted (A.11.c)

111 — A

Professional Development & Counselor Self-Awareness · Task 1B.5 · Chapter 22

- **A. Offer a qualified interpreter or a signing counselor — Owner: ACA A.2.c developmental and cultural sensitivity.** Counselors communicate in ways clients understand, arranging a qualified interpreter when needed, and the client helps choose the option.
- B. Use written notes so no third party is involved — Owner: Inadequate accommodation (ADA effective communication). Written notes rarely give effective

communication for counseling and can shut out the client's first language.

- C. Ask the client's adult child to interpret sessions — Owner: Family interpreter error (confidentiality and dual roles). A relative as interpreter compromises confidentiality, accuracy, and what the client feels free to say.
- **Pair to learn:** Language mismatch → qualified interpreter or matched counselor (ACA A.2.c)

112 — A

Provision of Counseling Interventions · Task 4.4 · Chapter 22

- **A. "I may have missed something. Help me see her the way you do." — Owner: Conflict tolerance: explore non-defensively (4.4).** It stays present with the disagreement and explores the client's view without defending or retreating.
- B. "I've been doing this a long time, and the pattern is clear." — Owner: Defending the counselor. Defending credentials escalates the conflict and dismisses the client's view.
- C. "You're right, I'm so sorry. I won't bring her up again." — Owner: Appeasing (avoiding conflict). Appeasing avoids the conflict instead of exploring it, and closes off a possibly important topic.
- **Pair to learn:** Client disagrees with the counselor → Explore non-defensively (not defend or appease)

113 — A

Treatment Planning & Continuity of Care · Task 3A.10 · Chapter 21

- **A. Reassess his readiness and revise the plan with him — Owner: Check readiness when progress stalls (3A.10; stages of change).** When progress stalls, the counselor checks whether the client's stage of change has shifted and adjusts the plan collaboratively.

- B. Keep the current plan and remind him of his original goal — Owner: Continuing an ineffective plan unchanged. Continuing unchanged ignores what the client just said about his readiness.
- C. Confront his denial so that he will recommit — Owner: Confrontational approach in addiction work (punished). Confrontation tends to increase resistance; the exam rewards exploring readiness instead.
- **Pair to learn:** Readiness has shifted → Reassess stage of change, revise plan

114 — B

Indirect Client Care · Task 5.7 · Chapter 23

- **B. A qualified ASL interpreter for sessions — Owner: ADA effective communication (qualified ASL interpreter).** A qualified ASL interpreter provides effective communication in the client's preferred language.
- A. Written notes exchanged during each session — Owner: Written notes. Exchanging notes is too slow and limited for the complex back-and-forth of counseling.
- C. Seating so the client can lip-read clearly — Owner: Speechreading. Lip-reading captures only part of spoken English and should not be relied on.
- **Pair to learn:** Deaf client who uses ASL → qualified ASL interpreter

115 — C

Indirect Client Care · Task 5.3 · Chapter 21

- **C. BIRP — Owner: BIRP (Behavior, Intervention, Response, Plan).** BIRP covers the client's behavior (presentation), the counselor's intervention, the client's response, and the plan.
- A. GIRP — Owner: GIRP (Goal, Intervention, Response, Plan). GIRP opens with the treatment goal, not the client's presentation.

- B. SOAP — Owner: SOAP (Subjective, Objective, Assessment, Plan). SOAP separates the client's report from the counselor's observations; it has no intervention or response section.
- **Pair to learn:** Behavior, intervention, response, plan → BIRP

116 — C

Provision of Counseling Interventions · Task 4.5 · Chapter 8

- **C. "Could you say that directly to Ray?" — Owner: Facilitating member-to-member interaction.** Facilitating encourages members to talk to each other, not just to the leader.
- A. "Ray, please let her finish from now on." — Owner: Leader-centered rule-setting. The leader settles it for them instead of promoting direct interaction.
- B. "It sounds like you feel dismissed by him." — Owner: Reflection of feeling (individual counseling in group). A reflection keeps the exchange between member and leader.
- **Pair to learn:** Tell me → tell Ray directly: facilitating member interaction

117 — C

Legal & Ethical Compliance · Task 6.16 · Chapter 17

- **C. Shredding it so the information cannot be reconstructed — Owner: ACA B.6.h (storage and disposal after termination).** Counselors dispose of records in a way that protects confidentiality; shredding, burning, or pulping makes paper records unreadable.
- A. Placing it in the office recycling bin with the other paper — Owner: Improper disposal (HIPAA safeguards, 45 CFR 164.530(c)). Recycling bins are not secure; anyone handling the waste could read a discarded chart.

- B. Mailing it to the client's last known address — Owner: Unrequested record transfer (error pattern). Mailing an unrequested chart to an old address risks misdelivery and disclosure; secure disposal is the counselor's duty.
- **Pair to learn:** Retention period over → destroy records so they cannot be read (ACA B.6.h)

118 — B

Provision of Counseling Interventions · Task 4.2 · Chapter 15

- **B. Terror management theory — Owner: Greenberg, Solomon, & Pyszczynski.** Terror management theory proposes that death awareness motivates people to hold onto worldviews and self-esteem as buffers.
- A. Neurotic anxiety — Owner: Rollo May (normal vs. neurotic anxiety). May distinguished normal from neurotic anxiety; this isn't a theory of worldview defense.
- C. Viktor Frankl's will to meaning — Owner: Frankl (logotherapy). Frankl's will to meaning is the drive to find meaning, including in suffering, not worldview defense.
- **Pair to learn:** Fear of death drives worldview defense → terror management theory (not May or Frankl)

119 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Warning signs that a crisis may be starting — Owner: Stanley & Brown: step 1.** Step 1 is warning signs: thoughts, moods, situations, or behaviors that signal a crisis is starting.
- A. Internal coping strategies to use when alone — Owner: Stanley & Brown: step 2. Internal coping strategies are step 2.

- C. Ways to make the environment safer — Owner: Stanley & Brown: step 6. Reducing access to lethal means is the final step.
- **Pair to learn:** Safety plan step 1 → warning signs (Stanley & Brown)

120 — A

Professional Development & Counselor Self-Awareness · Task 1B.6 · Chapter 22

- **A. Take a brief grounding pause before seeing the next client — Owner: Between-session self-regulation (grounding).** A few minutes of regulation keeps one session from spilling into the next.
- B. Vent about the prior client to the front-desk staff — Owner: Confidentiality breach (ACA Section B). Discussing a client with staff who have no need to know breaches confidentiality.
- C. Push the feelings down and start the session at once — Owner: Expressive suppression. Suppression hides the expression but leaves physiological arousal high and impairs memory; it does not regulate the emotion.
- **Pair to learn:** Tension carrying into the next session → brief grounding pause first

121 — C

Provision of Counseling Interventions · Task 4.4 · Chapter 22

- **C. Safran and Muran — Owner: Safran & Muran: alliance ruptures and repair.** Safran and Muran described withdrawal and confrontation ruptures and showed that repair often strengthens the alliance.
- A. Robert Carkhuff — Owner: Carkhuff: five-level empathy scale. Carkhuff rated empathic responses, not ruptures.
- B. Edward Bordin — Owner: Bordin: working alliance (goals, tasks, bond). Bordin defined the parts of the alliance, not the rupture-and-repair model.

- **Pair to learn:** Repaired rupture strengthens the alliance → Safran & Muran (not Bordin)

122 — C

Intake & Assessment · Task 2B.5 · Chapter 19

- **C. An illusion — Owner: MSE perception: illusion (misperceived real stimulus).** An illusion is a misperception of a real stimulus, here the sound of the vent.
- A. An auditory hallucination — Owner: MSE perception: hallucination (no stimulus). A hallucination occurs with no stimulus present; this experience began with a real sound.
- B. A delusion — Owner: MSE thought content: delusion. A delusion is a fixed false belief; she corrected her perception as soon as she listened closely.
- **Pair to learn:** Misperceived real sound → Illusion (not hallucination)

123 — B

Legal & Ethical Compliance · Task 6.17 · Chapter 17

- **B. Release only what the order specifies, seeking to limit it — Owner: ACA B.2.d (court-ordered disclosure).** Counselors seek client consent or ask that disclosure be limited as narrowly as possible, then release only what the order requires.
- A. Refuse, citing the client's privilege — Owner: Privilege (decided by the court). Refusing a court order can mean contempt; any privilege claim is raised with the court, while B.2.d directs seeking consent or narrowing the order.
- C. Release the entire record to show full cooperation — Owner: Over-disclosure (ACA B.2.e minimal disclosure). Releasing more than the order specifies discloses information the court did not require.
- **Pair to learn:** Court order for records → release only what it specifies (ACA B.2.d)

124 — B

Provision of Counseling Interventions · Task 4.2 · Chapter 3

- **B. Identity tolerance — Owner: Cass (homosexual identity formation).** Cass's order is confusion, comparison, tolerance, acceptance, pride, synthesis, so tolerance follows comparison.
- A. Moratorium — Owner: Marcia (identity statuses). Moratorium is Marcia's status of exploration without commitment, not a stage of sexual identity formation.
- C. Identity acceptance — Owner: Cass (homosexual identity formation). Acceptance comes after tolerance in Cass's sequence, not directly after comparison.
- **Pair to learn:** Confusion → comparison → tolerance → acceptance → pride → synthesis → Cass (not Marcia)

125 — A

Intake & Assessment · Task 2B.3 · Chapter 11

- **A. Arrange medical evaluation; withdrawal could be life-threatening — Owner: Alcohol withdrawal is potentially life-threatening (assess withdrawal risk, 2B.3).** Alcohol withdrawal can cause seizures and delirium tremens, so stopping heavy daily drinking calls for medical evaluation and managed withdrawal.
- B. Explore his ambivalence with a decisional balance exercise — Owner: Motivational interviewing / stages of change (Prochaska & DiClemente). He has already decided; the priority is making withdrawal medically safe, not building motivation.
- C. Affirm his decision and schedule a relapse prevention session — Owner: Relapse prevention (Marlatt). Relapse prevention matters later, but it skips the immediate medical danger of withdrawal.
- **Pair to learn:** Heavy daily drinker stopping abruptly → Medical evaluation (alcohol withdrawal can be dangerous)

126 — C

Professional Development & Counselor Self-Awareness · Task 1B.7 · Chapter 22

- **C. Acknowledge it, weigh any risk, and document the decision — Owner: ACA A.6.a previous relationships.** A.6.a asks counselors to weigh the risks and benefits of a prior relationship; a single brief transaction rarely creates a conflict.
- A. Refer him elsewhere, since any prior contact rules out counseling — Owner: Rigid avoidance (not the ACA standard). Not every prior contact creates a conflict; automatic referral is unnecessary.
- B. Say nothing, since mentioning it could harm early rapport — Owner: Concealing a potential conflict. Hiding a known prior contact undermines transparency if the client recalls it later.
- **Pair to learn:** Brief past contact → weigh it, acknowledge, document (ACA A.6.a)

127 — B

Provision of Counseling Interventions · Task 4.11 · Chapter 14

- **B. The depersonalization dimension of burnout — Owner: Maslach (burnout dimensions).** In Maslach's model, depersonalization means cynicism and detachment from clients or work.
- A. Depersonalization as a dissociative symptom — Owner: Dissociative symptom. Dissociative depersonalization is feeling detached from oneself, not cynicism toward clients.
- C. Reduced personal accomplishment — Owner: Maslach (burnout dimensions). Reduced accomplishment is feeling ineffective, a different burnout dimension than detachment.
- **Pair to learn:** Cynicism and detachment from clients → depersonalization (Maslach burnout), not dissociation

128 — B

Provision of Counseling Interventions · Task 4.10 · Chapter 6

- **B. Warn an identifiable victim of a serious threat — Owner: Tarasoff I (1974): duty to warn.** Tarasoff I established the duty to warn an identifiable victim.
- A. Act for a foreseeable victim based on violent history — Owner: Jablonski v. United States (1983). Jablonski extended the duty to foreseeable victims without an explicit threat.
- C. Take reasonable steps to protect an intended victim — Owner: Tarasoff II (1976): duty to protect. The broader duty to protect came from the 1976 decision.
- **Pair to learn:** Duty to warn (1974) → Tarasoff I; duty to protect (1976) → Tarasoff II

129 — A

Legal & Ethical Compliance · Task 6.18 · Chapter 17

- **A. Revise the note to record only what occurred in this session — Owner: Accurate documentation (ACA A.1.b; C.6.b).** Each note must reflect the service actually provided; carried-over text can misstate care and the claim it supports.
- B. Sign it as written, since the presentation has not changed much — Owner: Cloned documentation (payer compliance risk). Signing copied text attests to content that did not occur in this session.
- C. Sign it now and add a session summary next week — Owner: Sign now, correct later (inaccurate attestation). Documentation should be accurate and timely; signing inaccurate text and fixing it later fails both.
- **Pair to learn:** EHR copy-forward → edit to this session's actual content

130 — C

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **C. Top dog and underdog — Owner: Perls (Gestalt).** Top dog/underdog is a Gestalt key term (Perls), and Gestalt works with such splits through techniques like two-chair dialogue.
- A. Parent and Child ego states — Owner: Berne (transactional analysis). Ego states are Berne's TA vocabulary, not Gestalt's.
- B. Superego and id — Owner: Freud (psychoanalysis). Id, ego, and superego are Freud's structures of personality, not a Gestalt framing.
- **Pair to learn:** Top dog / underdog → Perls (Gestalt), not Berne's ego states

131 — B

Intake & Assessment · Task 2A.1 · Chapter 17

- **B. State their commitment to the Code and try to resolve it — Owner: Conflicts between ethics and laws (ACA I.1.c).** I.1.c directs counselors to make their commitment to the Code known and take steps to resolve the conflict; if that fails, they may follow the law.
- A. Follow the Code, since ethics always outrank the law — Owner: Misreading of ACA I.1.c. I.1.c allows counselors to follow the law if the conflict can't be resolved; ethics don't automatically outrank law.
- C. Follow the law without raising the conflict with anyone — Owner: Mandatory ethics (minimum rules only). Following the law may be the end point, but I.1.c first asks counselors to state their commitment and try to resolve the conflict.
- **Pair to learn:** Ethics-law conflict → State commitment to the Code, seek resolution (I.1.c)

132 — B

Professional Development & Counselor Self-Awareness · Task 1B.8 · Chapter 22

- **B. Review the manual and co-rate with a trained colleague — Owner: Maintaining current competence (interrater reliability).** Refreshing the scoring rules and checking agreement with a trained rater confirms reliable use before scores guide care.
- A. Rely on the original training, since competence is permanent — Owner: Skill decay ignored. Competence fades without use; past training does not guarantee present accuracy.
- C. Interpret scores by clinical impression instead of norms — Owner: Nonstandard interpretation. Departing from the manual's scoring and norms undermines the scale's validity.
- **Pair to learn:** Rusty on an instrument → refresh and verify reliability

133 — B

Provision of Counseling Interventions · Task 4.8 · Chapter 22

- **B. Start with the client's own account of what's wrong — Owner: Respect and acceptance for diversity (4.8).** Respect means letting the client define his concern rather than assuming his disability explains it.
- A. Refer him to a counselor who specializes in disability — Owner: Premature referral. He came for job-related depression; disability alone is not a reason to refer.
- C. Gently suggest that his disability may be the deeper issue — Owner: Premature interpretation (counselor assumption). It imposes the counselor's assumption before exploring the client's own account.

- **Pair to learn:** Client defines his own problem → Respect for diversity (not assuming the disability is the issue)

134 — A

Intake & Assessment · Task 2B.1 · Chapter 19

- **A. The ACEs questionnaire — Owner: Adverse Childhood Experiences (ACEs) screen.** The ACEs screen targets childhood adversity, which raises risk for later substance use and other problems.
- B. The Child Behavior Checklist — Owner: Achenbach (CBCL). The CBCL gathers parent, teacher, and youth ratings of a child's behavior, not an adult's childhood history.
- C. The Social Readjustment Rating Scale — Owner: Holmes & Rahe (life change units). It totals life change units from stressful life events; it is not a childhood adversity screen.
- **Pair to learn:** Childhood adversity → ACEs (not Holmes & Rahe)

135 — C

Legal & Ethical Compliance · Task 6.19 · Chapter 17

- **C. Review its privacy and crisis limits as a possible supplement — Owner: Technology benefits and limitations (ACA H.4.a); informed consent.** Counselors help clients weigh a tool's privacy, evidence, and crisis limits and review its use in session as a supplement, not a substitute.
- A. Endorse it, since chatbots follow clinical care standards — Owner: Assuming AI tools meet clinical standards. Consumer chatbots are not licensed providers and may lack crisis safeguards or privacy protections.
- B. Forbid it, since the ACA Code prohibits AI use — Owner: Invented Code rule (ACA 2014 does not address AI). The 2014 ACA Code predates AI tools; it requires competence and informed consent, not a ban.

- **Pair to learn:** AI chatbot between sessions → vet privacy and crisis limits; supplement, not substitute

136 — A

Provision of Counseling Interventions · Task 4.3 · Chapter 10

- **A. Siegel's "name it to tame it" — Owner: Siegel (name it to tame it).** Siegel's phrase captures the idea that naming an emotion helps regulate it.
- B. Wells's metacognitive therapy — Owner: Wells (metacognitive model). Wells targets beliefs about worry, not labeling emotions in the moment.
- C. Gross's response modulation — Owner: Gross (process model: response modulation). Response modulation acts on the emotional response after it arises, such as suppression, rather than labeling it.
- **Pair to learn:** Labeling emotions to regulate them → Siegel (name it to tame it)

137 — A

Legal & Ethical Compliance · Task 6.4 · Chapter 17

- **A. HITECH Act — Owner: HITECH Act (2009).** The HITECH Act strengthened HIPAA enforcement and added breach notification requirements for electronic records.
- B. 42 CFR Part 2 — Owner: 42 CFR Part 2 (SUD program records). 42 CFR Part 2 gives extra protection to records of federally assisted substance use disorder programs.
- C. FERPA — Owner: FERPA (student education records). FERPA protects the privacy of student education records and parents' access to them.
- **Pair to learn:** Breach notification, stronger HIPAA enforcement → HITECH Act

138 — A

Professional Development & Counselor Self-Awareness · Task 1B.9 · Chapter 22

- **A. Accept it warmly and explore its meaning with him — Owner: ACA A.10.f receiving gifts.** A.10.f has counselors weigh the relationship, the gift's value, the client's motivation, and their own motivation; a child's drawing usually fits.
- B. Decline it, since counselors may never accept gifts — Owner: Rigid no-gift rule (not the ACA standard). The Code does not ban all gifts, and refusing a child's drawing can feel like rejection.
- C. Accept it and give him a gift of similar value next week — Owner: Gift reciprocity (social, not professional, norm). Matching the gift treats the relationship as a social exchange and blurs the boundary.
- **Pair to learn:** Child's small handmade gift → accept and explore (ACA A.10.f)

139 — C

Legal & Ethical Compliance · Task 6.20 · Chapter 9

- **C. Not view it unless the client consents — Owner: Client privacy on social media (ACA H.6.c).** H.6.c directs counselors to respect clients' privacy and not view their social media without consent.
- A. View it, since the profile is public — Owner: Public posting (not consent under H.6.c). A public profile is not the client's consent; H.6.c still applies.
- B. Follow it from her professional account — Owner: Personal virtual relationships with clients (ACA A.5.e). Following a client creates a virtual relationship and still involves viewing without consent.
- **Pair to learn:** Viewing a client's social media → Only with consent (H.6.c)

140 — A

Intake & Assessment · Task 2B.12 · Chapter 19

- **A. Is there clinically significant distress or impairment? — Owner: First: step 6 (boundary with no mental disorder).** The last step establishes the boundary with no mental disorder by asking whether distress or impairment is clinically significant.
- B. Could a substance or medication explain the symptoms? — Owner: First: step 2 (substance or medication cause). Ruling out a substance or medication cause is the second step.
- C. Are the symptoms being intentionally produced? — Owner: First: step 1 (rule out malingering and factitious disorder). Ruling out intentional symptom production is the first step, not the last.
- **Pair to learn:** Final step: clinically significant distress or impairment → Boundary with no mental disorder (First)

141 — B

Legal & Ethical Compliance · Task 6.1 · Chapter 17

- **B. Lock the door and stay out of sight with the client — Owner: Practice lockdown procedure (emergency plan).** A lockdown announcement activates the plan: secure the room, stay away from doors and windows, and remain with the client until the all-clear.
- A. Leave with the client for the parking lot right away — Owner: Evacuation procedure (wrong incident type). Evacuation fits a fire; during a lockdown, moving through halls or outdoors can expose both people to the threat.
- C. Step into the hallway to confirm that the threat is real — Owner: Verification delay (error pattern). The announcement, not personal verification, triggers the plan; checking exposes the counselor and leaves the client alone.

- **Pair to learn:** Lockdown announced → secure the room with the client, not evacuate

142 — C

Provision of Counseling Interventions · Task 4.11 · Chapter 2

- **C. Reaction formation — Owner: Freud / Anna Freud (defense mechanisms).** Expressing the opposite of what one feels is reaction formation.
- A. Rationalization — Owner: Freud / Anna Freud (defense mechanisms). Rationalization creates a logical-sounding excuse.
- B. Sublimation — Owner: Freud / Anna Freud (defense mechanisms). Sublimation channels impulses into acceptable outlets.
- **Pair to learn:** Expressing the opposite of what one feels → reaction formation

143 — A

Intake & Assessment · Task 2B.5 · Chapter 19

- **A. Insight — Owner: MSE: insight (understanding of one's own problem).** Insight is the client's understanding of his problem; denying a clear problem reflects poor insight.
- B. Thought process — Owner: MSE: thought process (how the client thinks). Thought process describes the organization and flow of thinking, which is not described here.
- C. Judgment — Owner: MSE: judgment (ability to make sound decisions). Judgment concerns decision-making; this statement is about whether he understands that a problem exists.
- **Pair to learn:** Understanding of one's own problem → Insight (not judgment)

144 — C

Professional Development & Counselor Self-Awareness · Task 1A.1 · Chapter 22

- **C. Collegial consultation — Owner: Caplan (mental health consultation — coordinate relationship).** Caplan described consultation as a nonhierarchical relationship in which the consultee remains free to accept or reject the consultant's advice.
- A. Clinical supervision — Owner: Bernard & Goodyear (clinical supervision). Supervision is hierarchical, evaluative, and ongoing; this colleague evaluates nothing.
- B. Referral for treatment — Owner: Referral (ACA A. 11.d transfer of services). Referral moves the client's care to another provider; here the counselor keeps the client.
- **Pair to learn:** No evaluative authority, free to accept or reject → consultation (Caplan)

145 — B

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **B. Publicity test — Owner: Stadler: publicity test.** The publicity test asks whether you would want your behavior reported in the press.
- A. Justice test — Owner: Stadler: justice test. The justice test asks whether you would treat others in the same situation the same way.
- C. Universality test — Owner: Stadler: universality test. Universality asks whether you would recommend the same action to another counselor in the same situation.
- **Pair to learn:** Would I want this in the news? → Publicity test (Stadler)

146 — C

Treatment Planning & Continuity of Care · Task 3B.9 · Chapter 21

- **C. In informed consent, revisiting it as goals near — Owner: Raise termination early (planned termination).** Termination is introduced early, starting with informed consent, and revisited as goals are approached.
- A. Only when the client brings up wanting to stop — Owner: Client-initiated termination. Waiting for the client to raise it leaves ending unplanned and often abrupt.
- B. In the final session, once the goals are met — Owner: Last-session announcement (punished). Raising termination for the first time in the last session is what the exam punishes.
- **Pair to learn:** Raise termination early → Starting with informed consent

147 — A

Legal & Ethical Compliance · Task 6.2 · Chapter 17

- **A. Review the contact policy and explore the pattern in session — Owner: Practice SOP (between-session contact policy).** The written policy sets consistent response limits; bringing the pattern into session turns it into clinical work.
- B. Reply to each message right away with detailed reassurance — Owner: Reassurance seeking (accommodation of anxiety). Prompt reassurance relieves worry briefly but reinforces the reassurance-seeking cycle and bypasses the policy.
- C. Give the client a personal cell number for faster replies — Owner: Personal device use outside practice safeguards. Personal numbers sidestep the policy and the secure portal, blurring boundaries and record-keeping.

- **Pair to learn:** Excess between-session contact → apply the written contact policy and work with it in session

148 — B

Intake & Assessment · Task 2B.3 · Chapter 11

- **B. Refer her for medically managed withdrawal — Owner: Sedative withdrawal is potentially life-threatening.** Sedative withdrawal, like alcohol withdrawal, can cause seizures and delirium, so it needs medical care and a supervised taper.
- A. Explain that withdrawal is unpleasant but safe — Owner: Opioid withdrawal profile. “Rarely life-threatening” describes opioid withdrawal in healthy adults, not sedative withdrawal.
- C. Affirm her plan and check in on Monday — Owner: MI affirmation (misapplied). Affirming motivation is useful, but supporting an unsupervised stop ignores a dangerous withdrawal.
- **Pair to learn:** Sedative withdrawal → potentially life-threatening; medically managed withdrawal

149 — C

Provision of Counseling Interventions · Task 4.14 · Chapter 4

- **C. “I have, and it was disorienting. What has it been like for you?” — Owner: Appropriate self-disclosure (brief, relevant, returns focus).** Brief, relevant, and immediately returns the focus to the client.
- A. “Yes, and honestly, I’m still struggling with it most days.” — Owner: Self-disclosure that meets the counselor’s needs. Sharing current struggle burdens the client with the counselor’s needs.
- B. “Yes. Let me tell you about my father’s illness and how we coped.” — Owner: Counselor-centered self-disclosure. A lengthy account shifts the session to the counselor’s experience.

- **Pair to learn:** Brief, relevant, back to the client → appropriate self-disclosure

150 — B

Professional Development & Counselor Self-Awareness · Task 1A.2 · Chapter 22

- **B. The discrimination model — Owner: Bernard (discrimination model).** Bernard's discrimination model crosses three supervisor roles with three focus areas: intervention, conceptualization, and personalization.
- A. The Integrated Developmental Model — Owner: Stoltenberg & Delworth (IDM). The IDM tracks supervisees through developmental levels using self/other awareness, motivation, and autonomy.
- C. The systems approach to supervision — Owner: Holloway (systems approach). Holloway's model places the supervisory relationship at the center, surrounded by contextual factors.
- **Pair to learn:** Teacher/counselor/consultant × three skill areas → Bernard (discrimination model)

151 — C

Provision of Counseling Interventions · Task 4.5 · Chapter 22

- **C. Universality — Owner: Yalom: universality.** Discovering that others share the same feeling, so one is not alone, is universality.
- A. Catharsis — Owner: Yalom: catharsis. Catharsis is emotional release, not the discovery of shared experience.
- B. Instillation of hope — Owner: Yalom: instillation of hope. Hope comes from seeing others improve, not from finding shared guilt.
- **Pair to learn:** Others feel it too → Universality (Yalom)

152 — C

Legal & Ethical Compliance · Task 6.5 · Chapter 17

- **C. Fidelity — Owner: Fidelity (Kitchener; ACA principles).** Honoring commitments and keeping promises is fidelity.
- A. Veracity — Owner: Veracity (ACA principles). Veracity is dealing truthfully with people; the core of this scenario is keeping a promise.
- B. Justice — Owner: Justice (Kitchener; ACA principles). Justice concerns fairness and equal access across people, not keeping a promise to one client.
- **Pair to learn:** Keeping a promise → Fidelity

153 — C

Legal & Ethical Compliance · Task 6.3 · Chapter 17

- **C. Send them only with the client's specific written authorization — Owner: Psychotherapy notes authorization (45 CFR 164.508(a)(2)).** Psychotherapy notes generally require a specific authorization, and a health plan may not condition payment or eligibility on the client signing it.
- A. Send them, since payment disclosures need no authorization — Owner: Treatment, payment, and operations permission (45 CFR 164.506). The TPO permission does not reach psychotherapy notes, which carry special protection.
- B. Send them once the plan limits its request to the minimum necessary — Owner: Minimum necessary standard (45 CFR 164.502(b)). Minimum necessary limits the scope of permitted disclosures; it does not replace the authorization psychotherapy notes require.
- **Pair to learn:** Insurer wants psychotherapy notes → specific client authorization (164.508(a)(2))

154 — C

Intake & Assessment · Task 2B.11 · Chapter 18

- **C. History — Owner: History (Campbell & Stanley).** An outside event occurring during the study that affects results is the history threat.
- A. Maturation — Owner: Maturation (Campbell & Stanley). Maturation refers to natural changes within participants over time, such as growing older or recovering on their own, not an outside event.
- B. Experimental mortality — Owner: Experimental mortality / attrition (Campbell & Stanley). Mortality is uneven dropout across groups; nothing here indicates participants left the study.
- **Pair to learn:** Outside event during the study → History

155 — B

Provision of Counseling Interventions · Task 4.11 · Chapter 5

- **B. Herbert Benson — Owner: Benson (relaxation response).** The relaxation response is Herbert Benson's concept.
- A. Joseph Wolpe — Owner: Wolpe (systematic desensitization). Wolpe pairs relaxation with an imagined hierarchy; the relaxation response is Benson's.
- C. Edmund Jacobson — Owner: Jacobson (progressive relaxation). Jacobson owns progressive muscle relaxation.
- **Pair to learn:** Relaxation response → Benson (not Jacobson's progressive relaxation)

156 — A

Professional Development & Counselor Self-Awareness · Task 1A.3 · Chapter 22

- **A. Retained certificates listing provider, date, and hours — Owner: CE recordkeeping for audit.**

Credentialing bodies audit renewals and expect completion records, kept for the period each body specifies.

- B. A list of course titles the counselor recalls attending — Owner: Undocumented self-report. Recall cannot verify completion or hours and does not satisfy an audit.
- C. The provider's online catalog showing the courses offered — Owner: Catalog listing (not proof of completion). A catalog shows a course existed, not that this counselor completed it.
- **Pair to learn:** CE audit → retained certificates of completion

157 — B

Intake & Assessment · Task 2B.3 · Chapter 11

- **B. José Szapocznik — Owner: Szapocznik (BSFT).** Szapocznik developed BSFT, a family-based approach for adolescent substance use.
- A. Howard Liddle — Owner: Liddle (MDFT). Liddle developed Multidimensional Family Therapy, a different family approach for adolescent substance use.
- C. Scott Henggeler — Owner: Henggeler (MST). Henggeler developed Multisystemic Therapy for serious antisocial behavior.
- **Pair to learn:** BSFT → Szapocznik (not Liddle's MDFT)

158 — C

Professional Development & Counselor Self-Awareness · Task 1A.6 · Chapter 22

- **C. Aspirational ethics — Owner: Aspirational ethics (highest standard).** Aspirational ethics means striving for the highest standards of practice and thinking about the spirit behind the Code, not only its minimum rules.
- A. Mandatory ethics — Owner: Mandatory ethics (minimum compliance). Mandatory ethics is the minimum level — complying with required standards

(ACA C.1) to stay out of trouble — which the counselor wants to move beyond.

- B. Principle ethics — Owner: Principle ethics (Meara, Schmidt, & Day, 1996). Principle ethics uses moral principles such as autonomy and beneficence to answer "What should I do?" in a dilemma; it is not defined by striving beyond the minimum.
- **Pair to learn:** Beyond the minimum, highest standard → aspirational ethics (minimum → mandatory)

159 — B

Legal & Ethical Compliance · Task 6.6 · Chapter 17

- **B. Follow board rules to secure and register a new supervisor — Owner: State board supervision requirements.** Board rules, which vary by state, govern changing supervisors, often requiring notice or approval before supervised practice continues.
- A. Keep seeing clients and log hours until someone new starts — Owner: Unsupervised associate practice (state licensure rules). Associates generally practice only under an approved supervision arrangement; hours without one may not count and may violate board rules.
- C. Arrange for a fellow associate to supervise her for now — Owner: Peer consultation (not board-approved supervision). Peer consultation has value, but a fellow associate lacks the credentials boards require of a supervisor.
- **Pair to learn:** Supervisor leaving → secure a board-approved replacement per board rules

160 — A

Provision of Counseling Interventions · Task 4.6 · Chapter 2

- **A. The Premack principle — Owner: David Premack.** Using a preferred activity (playing outside) to reinforce a less-preferred one (reading) is the Premack principle.

- B. A token economy — Owner: Ayllon & Azrin. A token economy uses tokens exchanged for rewards; no tokens are involved here.
- C. Negative reinforcement — Owner: Skinner (operant). Negative reinforcement removes something unpleasant; here a pleasant activity is offered.
- **Pair to learn:** Preferred activity reinforces a less-preferred one → Premack principle (not token economy)

161 — A

Indirect Client Care · Task 5.8 · Chapter 17

- **A. Dispose of them in a way that protects confidentiality — Owner: Records retention and disposal (ACA B.6.h).** After the required retention period, records are disposed of in a manner that protects client confidentiality.
- B. Place them in the office's regular paper recycling bin — Owner: Unsecured disposal (confidentiality breach). Ordinary recycling exposes client information; disposal must protect confidentiality.
- C. Give them to the records custodian named in her will — Owner: Professional will / records custodian (ACA C.2.h, B.6.i). A records custodian handles records if the counselor dies or becomes incapacitated; that isn't the situation here.
- **Pair to learn:** Past retention period → Confidential disposal (B.6.h)

162 — C

Professional Development & Counselor Self-Awareness · Task 1A.4 · Chapter 22

- **C. Client characteristics, culture, and preferences — Owner: APA Task Force on Evidence-Based Practice (2006); Sackett.** Evidence-based practice joins research, clinical expertise, and the client's characteristics, culture, and preferences.

- A. The counselor's preferred theoretical orientation — Owner: Theoretical allegiance (orientation loyalty). Loyalty to a theory is a known source of bias, not a component of evidence-based practice.
- B. The treatment with the largest published effect size — Owner: Effect-size ranking (research leg alone). Ranking by effect size is still research only; it leaves out the client.
- **Pair to learn:** Research + expertise + client → evidence-based practice (Sackett; APA 2006)

163 — B

Intake & Assessment · Task 2B.11 · Chapter 18

- **B. Stratified sampling — Owner: Stratified random sampling.** The population is divided into subgroups (strata) and then randomly sampled from each, often in proportion to size.
- A. Systematic sampling — Owner: Systematic sampling. Systematic sampling takes every nth person from a list; it doesn't divide the population into strata first.
- C. Cluster sampling — Owner: Cluster sampling. Cluster sampling randomly selects whole groups and studies everyone in them, rather than sampling individuals within every group.
- **Pair to learn:** Random selection within proportional subgroups → Stratified random sampling

164 — A

Professional Development & Counselor Self-Awareness · Task 1B.11 · Chapter 22

- **A. Self-awareness and impact on clients — Owner: 1B.11: power and privilege (MSJCC).** Awareness of the power the counselor's role and identity carry is part of self-awareness and ethical practice.
- B. Nonpossessive warmth toward the client — Owner: Truax & Carkhuff: nonpossessive warmth. Warmth is

caring without creating dependence, not awareness of power.

- C. Empathic attunement to the client — Owner: 4.14: empathic attunement. Attunement is sensing the client's experience, not examining the counselor's own power.
- **Pair to learn:** Awareness of the counselor's power and privilege → Self-awareness (1B.11)

165 — A

Legal & Ethical Compliance · Task 6.8 · Chapter 17

- **A. A possible fraud and payer contract violation — Owner: Routine copay waiver (insurance fraud risk).** Routinely waiving copays misstates the true fee to the payer and can violate payer contracts and fraud laws; documented, case-by-case hardship adjustments are different.
- B. A fee adjustment for financial hardship — Owner: ACA A.10.c (establishing fees). A.10.c lets counselors adjust fees, when legally permissible, for an individual client's hardship — not waive copays for everyone as marketing.
- C. Pro bono service to the public good — Owner: ACA C.6.e (contributing to the public good). Pro bono work is unpaid service offered freely; billing the insurer while waiving the client's share is not pro bono.
- **Pair to learn:** Waiving every client's copay → fraud risk, not a hardship adjustment

166 — B

Provision of Counseling Interventions · Task 4.8 · Chapter 9

- **B. Social model — Owner: Social model of disability.** The social model sees disability as created by environmental and social barriers.
- A. Biopsychosocial model — Owner: Biopsychosocial model (addiction). The biopsychosocial model (the mainstream addiction model) describes interacting

biological, psychological, and social factors; the client points specifically to environmental barriers.

- C. Medical model — Owner: Medical model of disability. The medical model treats disability as an individual deficit.
- **Pair to learn:** Disability from environmental barriers → social model (not medical)

167 — C

Treatment Planning & Continuity of Care · Task 3A.9 · Chapter 21

- **C. Call or meet up with one friend twice a week — Owner: Well-formed goal (de Shazer, SFBT).** It is small, concrete, behavioral, within her control, and stated as the presence of something.
- A. Never feel lonely on a weekend again — Owner: Absence-based, unachievable goal (fails SMART: Achievable). It is stated as an absence and is unrealistic, so it is neither well-formed nor achievable.
- B. Understand why she keeps pulling away from others — Owner: Insight goal (not behavioral). Understanding is an insight aim, not a concrete behavior she can do and observe.
- **Pair to learn:** Presence of a concrete behavior → Well-formed goal (de Shazer), applied rewrite

168 — B

Professional Development & Counselor Self-Awareness · Task 1A.5 · Chapter 22

- **B. With consent, appeal using outcome data showing medical necessity — Owner: ACA A.7.a–b; Advocacy Competencies (client/student level).** A.7.a supports advocacy to remove barriers and A.7.b requires client consent; a data-based appeal is client/student-level advocacy.
- A. Bill under a more severe diagnosis the insurer will approve — Owner: Diagnostic upcoding (insurance

fraud). Changing a diagnosis to win coverage is fraudulent and misrepresents the client's record.

- C. Accept the denial and end at the last authorized session — Owner: Passive acceptance (no advocacy). Ending treatment without appeal or other options leaves a barrier unchallenged when the client still needs care.
- **Pair to learn:** Insurer denial, data show need → consented, data-based appeal (ACA A.7)

169 — C

Legal & Ethical Compliance · Task 6.7 · Chapter 17

- **C. Have told the client in advance and offered a chance to pay — Owner: Nonpayment and collections (ACA A.10.d).** A.10.d requires informing clients in advance that collection agencies may be used and giving them an opportunity to pay first.
- A. Give the agency the client's diagnosis to support the claim — Owner: Minimal disclosure (ACA B.2.e). A collection agency doesn't need clinical information; disclosing a diagnosis goes far beyond what is essential.
- B. End services at once and then file with the agency — Owner: Abandonment (ACA A.12). Ending services abruptly without pretermination counseling or referrals risks abandonment, and it skips the required notice.
- **Pair to learn:** Collections → Advance notice and chance to pay (A.10.d)

170 — C

Legal & Ethical Compliance · Task 6.9 · Chapter 17

- **C. HHS OCR ended that enforcement discretion in 2023 — Owner: HHS OCR notice (discretion expired August 9, 2023).** OCR's telehealth enforcement discretion ended with the public health emergency and a transition period that closed in August 2023.

- A. The flexibility lasts as long as Medicare telehealth extensions — Owner: Medicare telehealth payment flexibilities (separate from HIPAA). Extensions of Medicare telehealth flexibilities concern coverage and payment, not HIPAA enforcement for platforms.
- B. Encryption alone makes a consumer app compliant — Owner: Encryption-only myth (a BAA is still required). Encryption is one safeguard; a vendor handling client information still needs a business associate agreement.
- **Pair to learn:** Pandemic telehealth leniency → ended 2023 (HHS OCR)

Score by Domain

Domain	Items	Your correct	%
Professional Development & Counselor Self-Awareness	25		
Intake & Assessment	35		
Treatment Planning & Continuity of Care	23		
Provision of Counseling Interventions	39		
Indirect Client Care	17		
Legal & Ethical Compliance	31		
Total	**170**		

Scaled NCE scores are not percentages, and no practice percentage maps onto a pass or fail. Use this table to find your two weakest domains and give them extra study time next week.

Item numbers by domain

- **Professional Development & Counselor Self-Awareness:** 6, 10, 18, 30, 42, 44, 54, 66, 75, 84, 93, 98, 102, 111, 120, 126, 132, 138, 144, 150, 156, 158, 162, 164, 168

- **Intake & Assessment:** 2, 7, 13, 14, 19, 23, 25, 32, 34, 35, 37, 38, 46, 47, 50, 53, 55, 56, 74, 76, 77, 82, 88, 91, 104, 122, 125, 131, 134, 140, 143, 148, 154, 157, 163
- **Treatment Planning & Continuity of Care:** 1, 12, 16, 20, 24, 28, 29, 36, 48, 60, 62, 64, 85, 86, 89, 92, 94, 97, 107, 110, 113, 146, 167
- **Provision of Counseling Interventions:** 4, 5, 11, 17, 22, 31, 43, 49, 52, 59, 65, 67, 70, 71, 79, 80, 83, 95, 100, 103, 106, 109, 112, 116, 118, 119, 121, 124, 127, 128, 130, 133, 136, 142, 149, 151, 155, 160, 166
- **Indirect Client Care:** 9, 21, 26, 33, 40, 45, 57, 61, 68, 69, 78, 87, 96, 105, 114, 115, 161
- **Legal & Ethical Compliance:** 3, 8, 15, 27, 39, 41, 51, 58, 63, 72, 73, 81, 90, 99, 101, 108, 117, 123, 129, 135, 137, 139, 141, 145, 147, 152, 153, 159, 165, 169, 170

WORKBOOK

Study Plan, Drills, and Trackers

Workbook — Study Plan, Drills, and Trackers

This workbook turns the book into a routine. Use it alongside the chapters and the audio companion.

Part 1 — The 8-Week Plan

Total time: about 80 hours (10 hours a week), allocated by scored items (Chapter 1). Adjust the weeks to your exam date; the proportions matter more than the calendar.

Domain	Scored items	Hours
Professional Development & Counselor Self-Awareness	21	12
Intake & Assessment	25	14
Treatment Planning & Continuity of Care	21	12
Provision of Counseling Interventions	28	16
Indirect Client Care	17	10
Legal & Ethical Compliance	28	16

Every Day

- **15 minutes** of ownership drills (Chapter 2 three-pass drill, or the flash pairs below)
- **One audio track** during a drive, walk, workout, or chore. Repeat a track until you can answer every item before the pause ends.
- **5 minutes** logging any misses in the error log (Part 4)

Week by Week

Week	Read	Audio	Practice
1	Ch 1, 2	2.1, 2.2	Ch 1–2 Quick Checks as a baseline; log every miss by domain
2	Ch 3, 4	2.3, 2.4, 3.1, 4.1	Ch 3–4 Quick Checks; three-pass drill daily
3	Ch 5 (with Frameworks and Tools), 6, 7	5.1, 6.1, 7.1	Quick Checks; review error log patterns
4	Ch 8, 9, 17 (with Practice Management and Compliance)	3.3, 8.1, 9.1, 17.1	Quick Checks; time yourself at 70 seconds per item
5	Ch 23, 19, 20	3.4, 19.1, 20.1	**Practice Exam 1** under timed conditions (3 hours, 45 minutes, break after item 85); review with the Ownership Map
6	Ch 21 (with Continuity of Care), 22 (with Self-Awareness), 18	18.1, 21.1, 22.1; replay 17.1	Quick Checks; drill every pair you missed on Exam 1
7	Ch 10, 11, 12, 13	3.1, 10.1, 11.1, 12.1, 13.1	**Practice Exam 2**; compare domain scores with Exam 1
8	Ch 14, 15, 16; review weakest domains	3.2, 14.1, 15.1, 16.1; replay Ch 2 tracks	**Practice Exam 3** at least 3 days before the test; final error-log review; rest the day before

The order puts the two 20% domains and Chapter 23 in the first five weeks, and finishes the 2027 sections by week 6. Chapters 10–16 come later because presenting concerns are now supplemental content guidance: they show up as the client in the stem, inside all six domains. Chapter 23 and the 2027 sections have no audio track; read their swap-pair drills out loud instead.

If you have less time: keep the daily audio and ownership drills, read the chapters for your weakest domains first, and take at least one full timed practice exam.

Part 2 — Flash Pairs: 83 Most-Swapped Owners

Cut these into cards or cover the right column. Say the answer out loud.

Theories (Chapter 2)

Term	Owner
Social interest; birth order	Adler
Collective unconscious; archetypes	Jung
Free association; defense mechanisms	Freud
Unconditional positive regard	Rogers
Empty chair; unfinished business	Perls (Gestalt)
Logotherapy; paradoxical intention	Frankl
Four givens: death, freedom, isolation, meaninglessness	Yalom
Reciprocal inhibition; SUDS	Wolpe
Shaping; schedules of reinforcement	Skinner
Modeling; self-efficacy	Bandura
Disputing irrational beliefs; A-B-C	Ellis (REBT)
Cognitive triad; automatic thoughts	Beck
WDEP; quality world	Glasser / Wubbolding
Stress inoculation	Meichenbaum
BASIC I.D.	Lazarus

Term	Owner
Parent, Adult, Child ego states	Berne (TA)
OARS; change talk	Miller & Rollnick (MI)
Precontemplation → maintenance	Prochaska & DiClemente
Distress tolerance; wise mind	Linehan (DBT)
Defusion; psychological flexibility	Hayes (ACT)
Miracle question; exceptions; scaling	de Shazer & Berg (SFBT)
Externalizing; unique outcomes	White & Epston (narrative)
Differentiation; triangles; genogram	Bowen
Boundaries; enmeshment; enactment	Minuchin
Directives; ordeal	Haley
Circular questioning	Milan group
Double bind	Bateson
Placater, blamer, super-reasonable	Satir
Four Horsemen	Gottman
Pursue–withdraw; softening	Sue Johnson (EFT)

Development, Career, Group, Testing, Research (Chapter 3)

Term	Owner
Industry vs. inferiority	Erikson
Conservation; object permanence	Piaget
Zone of proximal development	Vygotsky
Law and order; social contract	Kohlberg
Ethic of care	Gilligan
Foreclosure; moratorium	Marcia
Strange Situation	Ainsworth
Imaginary audience; personal fable	Elkind
Tasks of mourning	Worden
Alarm, resistance, exhaustion	Selye
Encounter; immersion–emersion	Cross

Term	Owner
Contact; disintegration; reintegration	Helms
Integration vs. assimilation (acculturation)	Berry
RIASEC	Holland
Life-Career Rainbow	Super
Fantasy, tentative, realistic	Ginzberg
Planned happenstance	Krumboltz
Circumscription and compromise	Gottfredson
Self-efficacy in career (SCCT)	Lent, Brown, & Hackett
Forming, storming, norming	Tuckman
Transition and working stages	Corey
Universality; instillation of hope	Yalom
Psychodrama; role reversal	Moreno
Fight–flight; pairing	Bion
Deviation IQ	Wechsler
g factor	Spearman
Fluid vs. crystallized	Cattell
Threats to internal validity	Campbell & Stanley
Grounded theory	Glaser & Strauss

Clinical, Ethics, and Beyond (Chapters 4–22)

Term	Owner / answer
SOLER	Egan
Five levels of empathy	Carkhuff
Goals, tasks, bond	Bordin
Seven-stage crisis model	Roberts
Interpersonal theory of suicide	Joiner
Safety Planning Intervention	Stanley & Brown
Duty to protect (1976)	Tarasoff II
Cycle of violence	Walker
Learned helplessness	Seligman
Rumination prolongs depression	Nolen-Hoeksema
Two-factor theory of fear	Mowrer

Term	Owner / answer
Safety, remembrance and mourning, reconnection	Herman
Window of tolerance	Siegel
Biosocial theory	Linehan
Ambiguous loss	Boss
Dual process model of grief	Stroebe & Schut
Socioemotional selectivity	Carstensen
Goals of misbehavior	Dreikurs
Coercion theory	Patterson
Minority stress	Meyer
PLISSIT	Annon
Transactional model of stress	Lazarus & Folkman
Six necessary and sufficient conditions	Rogers
Seven-step ethical decision model	Forester-Miller & Davis

Part 3 — The Four-Step Item Routine (Pocket Card)

1. **Read the last line first.** What's being asked?
2. **Name the owner.** Which theory, domain, or framework is this item living in?
3. **Predict, then look.**
4. **Eliminate by owner.** Who owns each wrong option?

For applied items: safety → relationship → assessment → code and law → scope → client autonomy.

For statistics items: how many groups, what kind of data, difference or relationship?

Part 4 — Error Log

Copy this table into a notebook or spreadsheet. Fill it in for every missed or guessed item.

#	Domain	Topic	My answer	Who/ what owns my answer	Correct answer	Who/ what owns the correct answer	Why I missed it (didn't know / confused pair / misread / rushed)
1							
2							
3							

Review weekly: count misses by domain and by reason. If "confused pair" dominates, spend more time on the swap-pair tables and audio. If "misread" dominates, slow down on step 1. If "rushed" dominates, practice pacing.

Part 5 — Practice Exam Score Worksheet

Domain	Items on practice exam	Correct	%	Target
Professional Development & Counselor Self-Awareness				
Intake & Assessment				
Treatment Planning & Continuity of Care				
Provision of Counseling Interventions				
Indirect Client Care				
Legal & Ethical Compliance				
Total	**170**			

Copy the item counts from the Score by Domain table at the end of each answer key. The practice exams follow the 2027 domain weights, each domain within about three percentage points. NBCC reports a scaled score from 100–500, and **a scaled score is not a percentage** (Chapter 1), so use these results to find weak domains,

not to predict a pass. Your two weakest domains get the next week's extra study time.

Part 6 — Pacing Checkpoints

Write these on your scratch materials at the start of the exam.

Item	Target time elapsed	Hard limit
43	0:50	0:57
85 (end of first half)	1:39	1:52
128	2:29	2:49
170	3:18	3:45

At item 85: review your flags in the first half and check for blanks **before** you submit it. Once you submit and the break begins, you can't go back. You get an optional 15-minute break at the halfway point.

Part 7 — Test-Day Checklist

The week before - [ ] Confirm the test center location, time, and ID requirements in your registration materials - [ ] Take your last timed practice exam no later than 3 days before - [ ] Review your error log, not new material

The day before - [ ] Light review only: one or two audio tracks - [ ] Prepare ID and directions - [ ] Sleep

Exam day - [ ] Eat, hydrate, arrive early - [ ] Write pacing checkpoints on scratch materials - [ ] Use the four-step routine on every item - [ ] Flag an item only when you can name a reason to revisit it - [ ] Answer every item. Strange items may be unscored; don't let them shake you - [ ] At item 85: review flags and blanks in the first half, then submit it. You can't return to it after the break begins - [ ] Take the optional break if you need it: water, stretch, breathe - [ ] Check that nothing in the second half is blank before submitting

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BACK MATTER

“Who Owns It?” Master Index

Every swap pair in the book, alphabetized by the term you’ll see in a question stem. Find the term, name the owner, and note the look-alike it gets confused with. The chapter column points to the full teaching. If the owner surprises you, that pair goes in your error log.

How to drill this index: cover the “Owner” column, read the term, and say the owner out loud. Then cover the “Term” column, read an owner, and name one term it owns. The “Not…” column is the distractor the exam will offer you instead.

382 pairs from Chapters 2–22. Chapter 4’s microskills drill uses sample counselor responses rather than terms, so it lives in Chapter 4 only. The swap pairs in Chapter 23 and in the four 2027 sections (Chapters 5, 17, 21, and 22) stay with those sections.

Numbers

Term	Owner	Not…	Ch
3 of 15 criteria, 12 months, violating rights	Conduct disorder	ODD	16
4 S’s of transition	Schlossberg	Super (career stages)	3

A

Term	Owner	Not…	Ch
Acceptance plus change in couples	IBCT (Christensen & Jacobson)	EFT	7
Accommodations without special education	Section 504	IDEA	12

Term	Owner	Not…	Ch
Accurate empathy, nonpossessive warmth, genuineness	Truax & Carkhuff	Rogers's six conditions	22
ACE Study	Felitti & Anda	van der Kolk	10
Adjacent top codes	Consistency	Differentiation	14
Adolescent family therapy for substance use	BSFT (Szapocznik), MDFT (Liddle)	Filial therapy	11
Adult romantic attachment styles	Hazan & Shaver	Ainsworth (infants)	13
Adults' level of care tool	LOCUS	ASAM	19
Alarm, resistance, exhaustion	Selye	Kübler-Ross	3
Anomic suicide	Durkheim	Shneidman	6
Antecedent, behavior, consequence	Functional behavior assessment	Conflict resolution	16
Approves programs	Accreditation (CACREP)	Certification	17
Archetypes; collective unconscious; persona	Jung	Adler or Freud	2
Associations, affect, ambivalence, autism	Bleuler	Kraepelin	16
At least 4 symptoms, 6 months, angry and defiant	ODD	Conduct disorder	16
Ataque de nervios, susto	Cultural concepts of distress	Panic disorder	15

Term	Owner	Not…	Ch
Attention, power, revenge, inadequacy	Dreikurs	Baumrind	13
Attorney-issued document requesting records	Subpoena	Court order	17
Authoritative vs. authoritarian	Baumrind	Thomas & Chess (temperament, not parenting)	3
Autocratic, democratic, laissez-faire	Lewin, Lippitt, & White	Yalom	3
Avoiding harm	Nonmaleficence	Beneficence	17
Awareness, knowledge, skills	Sue, Arredondo, & McDavis	MSJCC (Ratts et al.)	9

B

Term	Owner	Not…	Ch
BASIC I.D.	Lazarus	Beck	2
Bed only for sleep; get up if awake	Stimulus control (Bootzin)	Sleep restriction (Spielman)	16
Behavior, intervention, response, plan	BIRP	DAP	21
Being real, no façade	Genuineness	Empathy	22
Beliefs about worry	Wells (metacognitive)	Dugas	10
Binge + compensatory behavior	Bulimia nervosa	Binge-eating disorder	16
Biopsychosocial model	Engel	Rosenstock	16
Biosocial theory of dysregulation	Linehan	Gross	10

Term	Owner	Not…	Ch
Blind and hidden areas	Luft & Ingham (Johari Window)	Yalom	3
Blocking career beliefs	Career Beliefs Inventory (Krumboltz)	Career Thoughts Inventory	14
Boundaries; enmeshment; enactment; joining	Minuchin	Bowen or Haley	2
Building theory from data	Grounded theory (Glaser & Strauss)	Phenomenology	18

C

Term	Owner	Not…	Ch
Career adaptability; career stories	Savickas	Super (who used “career maturity”)	3
Career decision-making styles: rational, intuitive, dependent	Harren	Tiedeman	14
Career indecision	Career Decision Scale (Osipow)	Career Maturity Inventory (Crites)	14
CASVE cycle	Peterson, Sampson, & Reardon	Tiedeman & O’Hara	3
Catastrophic misinterpretation of body sensations	Clark	Borkovec	10
CBT-based, non-12-step mutual support	SMART Recovery	AA	9
Change in the rules themselves	Second-order change	First-order change	7

Term	Owner	Not…	Ch
Changes because they know they're observed	Hawthorne effect	Placebo effect	18
Changes when or for whom an effect holds	Moderator	Mediator	18
Chasing losses	Gambling disorder	Substance use disorder	11
Chronic difficulty deciding anything	Indecisiveness	Career indecision	14
Circular questioning; positive connotation; invariant prescription	Milan	MRI	2
Circumscription and compromise	Gottfredson	Ginzberg (who also used "compromise")	3
Client must perceive empathy and UPR	Rogers's sixth condition	Rogers's first condition	22
Client says "I feel down"	Mood	Affect	19
Client's collaborative follow-through with treatment	Adherence	Compliance (older term)	21
Client's own words in the note	S in SOAP	O	21
Client's right to choose	Autonomy	Beneficence	17
Clinician explains why criteria aren't met	Other specified	Unspecified	19
Coaching parents live through an earpiece	PCIT (Eyberg)	Filial therapy	5

Term	Owner	Not…	Ch
Cocoanut Grove fire; acute grief	Lindemann	Caplan	6
Coercion theory	Patterson	Baumrind	13
Coercive control	Stark	Walker	6
Cognitive triad; collaborative empiricism; Socratic questioning	Beck (CT)	Ellis	2
Cohesion and flexibility	Olson’s Circumplex Model	McMaster Model	7
Coined “ageism”	Robert Butler	Rowe & Kahn	12
Commenting on how the group interacts	Process observation	Summarizing content	8
Commitment, control, challenge	Kobasa (hardiness)	Lazarus	16
Communicating understanding	Empathic responding	Empathic attunement	22
Compare group means on two or more IVs	Factorial ANOVA	One-way ANOVA	18
Compare groups on two or more DVs	MANOVA	ANCOVA	18
Compare two group means	t-test	ANOVA	18
Compares interests with satisfied workers	Strong Interest Inventory	SDS	14
Conflict matters more than divorce	Amato	Wallerstein	13
Conformity → integrative awareness	Atkinson, Morten, & Sue	Cross	3

Term	Owner	Not…	Ch
Connecting two members' concerns	Linking	Universalizing (the leader names a shared theme for the whole group)	8
Conservation; object permanence	Piaget	Erikson or Vygotsky	3
Contact; disintegration; reintegration	Helms	Cross	3
Contemplation	Prochaska & DiClemente (stages of change)	Corey's group stages (contemplation is never a group stage)	3
Contempt predicts divorce	Gottman	Sue Johnson (EFT)	7
Control group tries harder	John Henry effect	Hawthorne effect	18
Correlates with similar measures	Convergent validity	Discriminant validity	3
Counselor can't evaluate own client for court	ACA E.13.c	ACA A.8	13
Counselor imposes own culture as reality	Cultural encapsulation (Wrenn)	Microaggression	9
Counselor observes tearful, restricted expression	Affect	Mood	19
Counselor-observed affect; test score	O in SOAP	S	21
Crash: fatigue, hypersomnia, hunger, low mood	Stimulant withdrawal	Cannabis withdrawal	11
Critical consciousness	Freire	Martín-Baró	15

Term	Owner	Not...	Ch
Crystallizing, specifying, implementing	Super's exploration stage	Ginzberg	14

D

Term	Owner	Not...	Ch
Danger Assessment	Campbell	Posner (C-SSRS, suicide)	6
Day program, home at night	Partial hospitalization	Residential	19
Demonstrably effective relationship elements	Norcross & Lambert	Wampold	22
Desire, Ability, Reasons, Need	Change talk (MI)	Stages of change	5
Desire, excitement, orgasm	Kaplan	Masters & Johnson	16
Deviation IQ	Wechsler	Terman (ratio IQ)	3
Differentiation; triangles; genogram; cutoff	Bowen	Minuchin	2
Directives; ordeal; prescribing the symptom	Haley (strategic)	Minuchin	2
Disability from environmental barriers	Social model	Medical model	9
Disputing irrational beliefs; "musts"	Ellis (REBT)	Beck: Beck *tests* automatic thoughts collaboratively; Ellis *disputes* beliefs directly	2
Distress about gender incongruence	Gender dysphoria	Being transgender	15

Term	Owner	Not…	Ch
Doing good	Beneficence	Nonmaleficence	17
Double bind	Bateson / MRI	Milan	2
Duty, dereliction, direct causation, damages	Malpractice	Ethical complaint	17
Duty to warn (1974)	Tarasoff I	Tarasoff II (duty to protect, 1976)	6

E

Term	Owner	Not…	Ch
Ego states (Parent, Adult, Child); strokes; games	Berne (TA)	Freud's id/ego/superego	2
Ego-dystonic obsessions	OCD	OCPD (ego-syntonic)	10
Emerging adulthood	Arnett	Levinson	12
Employment tests must be job-related	*Griggs v. Duke Power*	*Larry P. v. Riles*	20
Empty chair; unfinished business; "what" and "how"	Perls (Gestalt)	Rogers: Rogers is nondirective and reflective	2
Encounter; immersion–emersion	Cross	Helms	3
Eros, ludus, storge, pragma, mania, agape	Lee	Sternberg	13
Essence of lived experience	Phenomenology	Ethnography	18
Ethic of care	Gilligan	Kohlberg	3
Ethical duty not to disclose	Confidentiality	Privilege	17

Term	Owner	Not…	Ch
Evaluative, ongoing, shared responsibility	Supervision	Consultation	17
Excessive thoughts about real physical symptoms	Somatic symptom disorder	Illness anxiety disorder	16
Excitement, plateau, orgasm, resolution	Masters & Johnson	Kaplan	16
Exhaustion, depersonalization, reduced accomplishment	Maslach (burnout)	Figley (compassion fatigue)	14
Expected rejection, concealment, internalized stigma	Proximal minority stressors (Meyer)	Distal stressors	15
Experiencing, then reflecting on, what just happened in the room	Here-and-now (Yalom)	Catharsis	8
Explains how the IV affects the DV	Mediator	Moderator	18
Externalizing; unique outcomes; re-authoring	White & Epston (narrative)	SFBT	2
Extra protection for process notes	HIPAA psychotherapy notes	42 CFR Part 2	17
Extreme scorers drift toward the mean on retest	Statistical regression	Maturation	3
Extreme scorers move toward the mean	Statistical regression	Testing	18

F

Term	Owner	Not…	Ch
Face the obsession, block the compulsion	ERP	Systematic desensitization	5
Fairness and equal access	Justice	Autonomy	17
Faith develops through six stages	Fowler	Kohlberg	9
Fakes symptoms for money or to avoid prosecution	Malingering	Factitious disorder	19
Fakes symptoms to be seen as sick	Factitious disorder	Malingering	19
False positive	Type I	Type II	18
Family acceptance protects LGBTQ youth	Caitlin Ryan	Meyer	15
Family criticism and over-involvement predict relapse	Expressed emotion (Brown; Vaughn & Leff)	Double bind	16
Family's links to school, church, agencies	Ecomap (Hartman)	Genogram	7
Fantasy, immersion, awareness…	Papernow	Visher & Visher	13
Fantasy, tentative, realistic	Ginzberg et al.	Super	3
Fear acquired by classical, maintained by operant	Mowrer's two-factor theory	Wolpe	10

Term	Owner	Not…	Ch
Fear of confirming a stereotype lowers performance	Stereotype threat (Steele & Aronson)	Impostor phenomenon	15
Fear of death drives worldview defense	Terror management theory	Yalom	15
Fear of illness with few or no symptoms	Illness anxiety disorder	Somatic symptom disorder	16
Feedback that keeps things the same	Negative feedback (morphostasis)	Positive feedback	7
Feeling like a fraud	Impostor phenomenon (Clance & Imes)	Stereotype threat	15
Feeling sorry for the client	Sympathy	Empathy	22
Feeling with, from the client's frame	Empathy	Sympathy	22
Fight–flight; pairing	Bion	Tuckman	3
Five faces of oppression	Iris Marion Young	Freire	15
Fixed questions in fixed order	Structured interview (SCID-5)	Unstructured	19
Fluid vs. crystallized	Cattell (and Horn)	Gardner	3
Focus on meaningful relationships as time shortens	Socioemotional selectivity (Carstensen)	Continuity theory	12
Foreclosure; moratorium	Marcia	Erikson (Marcia built on Erikson's identity stage)	3

Term	Owner	Not…	Ch
Former client romantic relationship	Prohibited for 5 years (A.5.c)	2 years	17
Four givens (death, freedom, isolation, meaninglessness)	Yalom	Frankl, who focuses on meaning	2
Four Horsemen	Gottman	Sue Johnson (EFT)	2

G

Term	Owner	Not…	Ch
g factor	Spearman	Thurstone (primary mental abilities)	3
Gambling disorder severity: 6–7 criteria	Moderate	Mild	11
Gaming addiction: DSM condition for further study	Internet gaming disorder	Gambling disorder	11
Gender identity vs. sexual orientation	Distinct constructs	The same thing	15
General systems theory	von Bertalanffy	Wiener (cybernetics)	7
Good-enough mother; holding environment	Winnicott	Bowlby	2
Grief not socially recognized	Doka (disenfranchised grief)	Worden	6
Group equivalent of the alliance	Cohesiveness	Universality	8

H

Term	Owner	Not…	Ch
Hearing voices with no source	Hallucination	Illusion	19
Hero, scapegoat, lost child, mascot	Wegscheider-Cruse	Satir’s stances	9
Hierarchy of needs	Maslow	Rogers	2
High-effort coping and health	John Henryism (Sherman James)	Stereotype threat	15
High-risk period after hospital discharge	Prompt follow-up	Discharge without plan	21
Holding to the first diagnosis despite new data	Anchoring bias	Confirmation bias	19
Home- and community-based treatment for antisocial youth	MST (Henggeler)	FFT	16
Honeymoon, crisis, recovery, adaptation	Oberg’s culture shock	Kübler-Ross	15
Honeymoon, disenchantment, reorientation	Atchley’s retirement phases	Kübler-Ross	12
Hopelessness predicts suicide	Beck	Joiner	6

I

Term	Owner	Not…	Ch
“I don’t see color”	Microinvalidation	Microassault	9
“I’ve blown it, so I might as well keep going”	Abstinence violation effect (Marlatt)	Contemplation	5

Term	Owner	Not...	Ch
I-messages	Thomas Gordon	Ellis	5
Identify gaps before designing	Needs assessment	Outcome evaluation	18
Identity confusion → synthesis	Cass	Marcia (identity statuses, not sexual identity)	3
Imaginary audience; personal fable	Elkind	Piaget	3
Improve a program in progress	Formative evaluation	Summative evaluation	3
Improve a program while running	Formative	Summative	18
Incredible Years	Webster-Stratton	Sanders (Triple P)	13
Industry vs. inferiority	Erikson	Piaget (stages named by cognition, not crisis)	3
Inflated responsibility in OCD	Salkovskis	Rachman	10
Inner experience matches outward expression	Congruence	Positive regard	22
Insomnia: 3 nights a week for 3 months	Insomnia disorder	Hypersomnolence	16
Instant love myth; wicked stepmother	Visher & Visher	Papernow	13
Intact groups, no random assignment	Quasi-experiment	True experiment	18

Term	Owner	Not...	Ch
Integration vs. assimilation	Berry	Piaget's assimilation (a cognitive process)	3
Intense imagined exposure	Implosive therapy (Stampfl)	Flooding (real life)	5
Intentional, repeated, power imbalance	Olweus's definition of bullying	Conflict	16
Internal, stable, global attributions	Abramson, Seligman, & Teasdale	Lewinsohn	10
Intimacy, passion, commitment	Sternberg	Lee	13
Intimate terrorism vs. situational couple violence	Michael P. Johnson	Walker's cycle	13
Intolerance of uncertainty	Dugas	Wells	10
Intrinsic vs. extrinsic religiosity	Allport	Pargament	15
Inviting a quiet member in	Drawing out	Linking	8
Items hang together	Internal consistency	Test-retest	3

J

Term	Owner	Not...	Ch
Judge-signed order	Court order	Subpoena	17

K

Term	Owner	Not...	Ch
Keeping promises	Fidelity	Veracity	17

Term	Owner	Not…	Ch
Know yourself + know the work world + match	Parsons	Williamson (who systematized it)	3

L

Term	Owner	Not…	Ch
Law and order; social contract	Kohlberg	Gilligan	3
Learned helplessness	Seligman	Beck	10
Learning how one affects others	Interpersonal learning	Imitative behavior	8
Legal right in court; client holds it	Privileged communication	Confidentiality	17
Licensure for telehealth	Where the client is located	Where the counselor is located only	9
Life-Career Rainbow; self-concept; maintenance stage	Super	Holland or Ginzberg	3
Locus of control and locus of responsibility	Sue’s worldview model	Rotter’s locus of control alone	9
Loss, rejection, guilt, grief, identity, intimacy, mastery	Silverstein & Kaplan	Worden	13

M

Term	Owner	Not…	Ch
Mean 50, SD 10	T-score	Stanine or z	3

Term	Owner	Not…	Ch
Mean 50, SD 10	T-score	Normal curve equivalent (SD 21.06)	20
Meaning; logotherapy; paradoxical intention	Frankl	Haley's paradoxical *directives*	2
Members can join anytime	Open group	Closed group	8
Members test the leader and conflict appears	Transition (Corey) / storming (Tuckman)	Initial / forming	8
Micro-, meso-, exo-, macro-, chronosystem	Bronfenbrenner's ecological systems theory	Hays's ADDRESSING	12
Miracle question; exceptions; scaling	de Shazer & Berg (SFBT)	Narrative	2
Missing an intimate attachment	Emotional loneliness (Weiss)	Social loneliness	16
Modeling; self-efficacy	Bandura	Skinner	2
Modern hospice movement	Cicely Saunders	Kübler-Ross	12
Moral, religious, or spiritual problem	Z65.8 (focus of attention, not a disorder)	A mental disorder	15
Most children of divorce adjust well	Hetherington	Wallerstein	13
Motor and vocal tics, over 1 year, onset before 18	Tourette's disorder	Persistent tic disorder	12

Term	Owner	Not...	Ch
Mutual withdrawal of elders and society	Disengagement theory (Cumming & Henry)	Activity theory	12

N

Term	Owner	Not...	Ch
Needs and press	Murray (TAT)	Maslow	3
No logical connection between ideas	Loose associations	Flight of ideas	19
Nominal frequencies	Chi-square	Pearson r	3
Nominal frequencies	Chi-square	t-test	18
Nonverbal reasoning	Raven's	WAIS verbal index	20
NREM, early night, little memory	Sleep terrors	Nightmares	16

O

Term	Owner	Not...	Ch
OARS; change talk; rolling with resistance	Miller & Rollnick (MI)	Prochaska & DiClemente	2
Observer is part of the system	Second-order cybernetics (von Foerster)	First-order	7
OCEAN	NEO-PI (Costa & McCrae)	16PF	20
One type clearly highest	Differentiation	Congruence	14
Ongoing connection to the deceased is healthy	Continuing bonds (Klass et al.)	Disenfranchised grief	12

Term	Owner	Not…	Ch
Opposite on the hexagon	R–S, I–E, A–C	Adjacent types	14
Oscillating between loss and restoration	Dual process model (Stroebe & Schut)	Worden’s tasks	12
Outside event during the study	History	Maturation	18

P

Term	Owner	Not…	Ch
Parent–child relationship shapes career	Roe	Super	3
Parent/teacher behavior ratings	CBCL (Achenbach)	MMPI-A	20
Parents run play sessions at home	Filial therapy (Guerney)	Child-centered play therapy (Axline/Landreth)	5
Parents take charge of eating	Family-based treatment (Maudsley)	Structural family therapy	5
Participants drop out unevenly	Mortality/ attrition	Selection	3
Participants improve because they’re observed	Hawthorne effect	Placebo effect	3
Percentage scoring at or below	Percentile rank	Percentage correct	20
Permission, limited information, specific suggestions, intensive therapy	PLISSIT (Annon)	Sensate focus	16
Person, place, time	Orientation ×3	Insight	19
Person–environment fit	Congruence	Consistency	14

Term	Owner	Not…	Ch
Physically present, psychologically absent	Ambiguous loss (Boss)	Disenfranchised grief (Doka)	12
Pinpoint pupils, slow breathing	Opioid intoxication	Stimulant intoxication	11
Placater, blamer, super-reasonable, irrelevant; sculpting	Satir	Bowen	2
Planned happenstance; learning experiences	Krumboltz	Savickas	3
Positive vs. negative religious coping	Pargament	Allport	15
Posttraumatic growth	Tedeschi & Calhoun	Bonanno	10
Prealcoholic, prodromal, crucial, chronic	Jellinek	Prochaska	11
Precontemplation → maintenance	Prochaska & DiClemente	MI or group stages	2
Predicts later performance	Predictive validity	Concurrent validity	3
Predisposing, precipitating, perpetuating, protective	4 Ps case formulation	Biopsychosocial model	19
Preventive psychiatry; crisis lasts about 4–6 weeks	Caplan	Lindemann	6
Primary and secondary appraisal	Lazarus & Folkman	Selye	16
Privileged/ marginalized counselor and client quadrants	MSJCC (Ratts et al.)	Sue et al. (1992)	9

Term	Owner	Not…	Ch
Process model of emotion regulation	Gross	Linehan	10
Program evaluation: outcomes in dollars	Cost-benefit	Cost-effectiveness	18
Prolonged grief disorder in adults	12 months	6 months (children and adolescents)	12
Protagonist; auxiliary ego; role reversal	Moreno (psychodrama)	Perls (Gestalt also uses enactment, but these terms are Moreno's)	3
Protest → despair → detachment	Bowlby (with Robertson)	Kübler-Ross	12
Psychache	Shneidman	Joiner	6
Psychotic symptoms 1 day to under 1 month, full recovery	Brief psychotic disorder	Schizophreniform	16
Pursue–withdraw cycle; softening; attachment in couples	Sue Johnson (EFT)	Gottman	2

R

Term	Owner	Not…	Ch
Race-based traumatic stress	Robert Carter	Sue	15
Raising culture directly in session	Broaching (Day-Vines)	Cultural humility	9
Random assignment	True experiment	Quasi-experiment	18
Random assignment present	True experiment	Quasi-experiment	3

Term	Owner	Not…	Ch
Rapid jumps between related topics	Flight of ideas	Loose associations	19
Rapprochement	Mahler	Bowlby	3
Ratio IQ (MA/CA × 100)	Terman / Stanford-Binet	Wechsler	3
Reducing frequency before ending	Tapering	Abrupt termination	21
Referring a client because of the counselor's beliefs	Values-based referral (prohibited, ACA A.11.b)	Competence-based referral	9
Rejecting a true null	Type I	Type II	3
Relaxation + imagined hierarchy	Systematic desensitization (Wolpe)	Flooding	5
Relaxation response	Herbert Benson	Edmund Jacobson (progressive relaxation)	5
REM, later night, remembered	Nightmares	Sleep terrors	16
Replaced the DOT	O*NET	OOH	14
Replaced the GAF	WHODAS 2.0	Multiaxial Axis V	19
Respect for persons, beneficence, justice	Belmont Report	ACA principles	18
Responsive desire	Basson	Kaplan	16
Restriction without body-image concern	ARFID	Anorexia	16

Term	Owner	Not...	Ch
RIASEC; hexagon; congruence	Holland	Super	3
Rumination prolongs depression	Nolen-Hoeksema	Beck	10
Runny nose, yawning, goosebumps, diarrhea	Opioid withdrawal	Alcohol withdrawal	11

S

Term	Owner	Not...	Ch
Safety Planning Intervention	Stanley & Brown	Jobes (CAMS)	6
Safety, remembrance and mourning, reconnection	Herman	Kübler-Ross	10
Same start, different outcomes	Multifinality	Equifinality	7
Satisfaction, alternatives, investments	Rusbult	Gottman	13
Schizophrenia symptoms 1 to under 6 months	Schizophreniform	Schizophrenia	16
Second dip on returning home	W-curve; reverse culture shock	U-curve	15
Selection, optimization, compensation	Baltes & Baltes	Carstensen	12
Self-administered, self-scored RIASEC	SDS (Holland)	Strong Interest Inventory	14
Self-efficacy + outcome expectations in career	Lent, Brown, & Hackett (SCCT)	Krumboltz	3

Term	Owner	Not…	Ch
Self-insight, self-integration, anxiety management	Managing countertransference (Hayes & Gelso)	Burnout prevention	22
Sensing the client's experience internally	Empathic attunement	Empathic responding	22
Separation–individuation	Mahler	Erikson	2
Seven-stage crisis intervention model	Roberts	James & Gilliland	6
Seven-step ethical decision-making model	Forester-Miller & Davis	Kitchener	17
Severity based on adaptive functioning	Intellectual developmental disorder	ADHD	12
Severity rated by levels of support needed	Autism spectrum disorder	Intellectual disability	12
Shadow mistaken for a person	Illusion	Hallucination	19
Shaping; token economy; schedules of reinforcement	Skinner / operant	Wolpe	2
Shattered assumptions	Janoff-Bulman	Tedeschi & Calhoun	10
"She nags because he withdraws because she nags"	Circular causality	Linear causality	7
Six dimensions incl. roles and behavior control	McMaster Model (Epstein)	Circumplex	7

Term	Owner	Not…	Ch
Six necessary and sufficient conditions	Rogers (1957)	Truax & Carkhuff	22
Six R processes	Rando	Worden	12
Six stations of divorce	Bohannan	Kübler-Ross	13
Six-step crisis model: listening, then acting	James & Gilliland	Roberts	6
Sleeper effect	Wallerstein	Hetherington	13
Small, concrete, presence of something	Well-formed goal (de Shazer)	SMART	21
Smoothing over pain too fast	Rescuing / band-aiding	Supporting	8
Social interest; birth order; lifestyle	Adler	Freud: Freud is drives and psychosexual stages	2
Social, problem, interaction, goal-setting, task-setting	Haley’s first interview	Roberts’s crisis model	7
Speaker–listener technique	PREP (Markman et al.)	Gottman	7
Special education with an IEP	IDEA	Section 504	12
Speech wanders and never gets to the point	Tangential	Circumstantial	19
Speech wanders but gets to the point	Circumstantial	Tangential	19

Term	Owner	Not…	Ch
Staggered intervention across behaviors	Multiple baseline	ABAB	18
State permission to practice	Licensure	Certification	17
State vs. trait anxiety	Spielberger (STAI)	Beck (BAI)	10
STEP	Dinkmeyer & McKay	Gordon (PET)	13
Stopping harmful behavior to protect a member	Blocking	Cutting off (managing time or focus)	8
Stories about ambiguous pictures	TAT (Murray)	Rorschach	20
Storming; norming	Tuckman	Corey	3
Strange Situation	Ainsworth	Bowlby (theory) or Harlow (monkeys)	3
Strengths perspective	Saleebey	Maslow	21
Stress inoculation; self-instructional training	Meichenbaum	Beck	2
Structured review of one’s whole life	Life review (Butler)	Reminiscence therapy	5
Substances with no DSM withdrawal diagnosis	Hallucinogens, PCP, inhalants	Cannabis (which has one)	11

Term	Owner	Not...	Ch
SUD remission: 12+ months without criteria (except craving)	Sustained remission	Early remission	11
SUD remission: 3–12 months without criteria (except craving)	Early remission	Sustained remission	11
SUD severity: 2–3 criteria	Mild	Moderate	11
SUD severity: 4–5 criteria	Moderate	Severe	11
SUD severity: 6 or more criteria	Severe	Moderate	11
Sudden, fluctuating confusion with poor attention	Delirium	Dementia	12
Supervisor liable for supervisee	Vicarious liability	Direct liability	17
Susceptibility, severity, benefits, barriers, cues	Health belief model	Biopsychosocial model	16
Symptoms before age 12, two or more settings	ADHD	Specific learning disorder	12
Systematic desensitization; reciprocal inhibition; SUDS	Wolpe	Skinner: Skinner is operant (consequences)	2

T

Term	Owner	Not...	Ch
Tail to the right; mean > median	Positive skew	Negative skew	18

Term	Owner	Not…	Ch
Tasks of mourning	Worden	Kübler-Ross (stages of dying)	3
Teaching skills, prevention focus	Psychoeducational group	Counseling group	8
Team consensus on themes	CQR (Hill)	Grounded theory	18
Telling the truth	Veracity	Fidelity	17
Tension → battering → honeymoon	Walker	Duluth (Power and Control Wheel)	6
Termination: raise the ending early	Planned termination	Last-session announcement	21
Test identifies those who don't have the condition	Specificity	Sensitivity	20
Test identifies those who have the condition	Sensitivity	Specificity	20
Test in the primary language	*Diana v. State Board of Education*	*Hobson v. Hansen*	20
Test must cover what was taught	*Debra P. v. Turlington*	*Griggs*	20
Test of best possible performance	Maximal performance	Typical performance	20
Test of potential to learn	Aptitude	Achievement	20
Test of usual (typical) behavior	Typical performance	Maximal performance	20
Test of what's been learned	Achievement	Aptitude	20
Test too hard; everyone scores low	Floor effect	Ceiling effect	20

Term	Owner	Not…	Ch
Test with easy items, strict time	Speed test	Power test	20
Test with harder items, generous time	Power test	Speed test	20
Thwarted belongingness; perceived burdensomeness	Joiner	Durkheim	6
Time structure, social contact, purpose, status, activity	Jahoda	Maslach	14
Too little response-contingent reinforcement	Lewinsohn	Seligman	10
Training family to encourage a loved one into treatment	CRAFT (Meyers)	Johnson intervention	9
Transgender emergence	Lev	Cass	15
Transition stage; working stage	Corey	Tuckman	3
Traumagenic dynamics	Finkelhor & Browne	Walker	13
Treatment plan: broad outcome	Long-term goal	Short-term objective	21
Treatment plan: measurable step	Short-term objective	Long-term goal	21
Treatment progress stalls	Review plan, check alliance, reassess	Repeat the same intervention	21
Tremor, sweating, seizures, hallucinations	Alcohol or sedative withdrawal	Opioid withdrawal	11

Term	Owner	Not…	Ch
True score range around observed	SEM confidence band	Standard deviation	20
Two groups, compare means	t-test	ANOVA	3

U

Term	Owner	Not…	Ch
Unconditional positive regard; nondirective	Rogers	Gestalt	2
Universal (culture-general) approach	Etic	Emic	9
Universality; instillation of hope	Yalom	Lewin	3
Using spirituality to avoid pain	Spiritual bypassing (Welwood)	Spiritual emergency (Grof)	15

V

Term	Owner	Not…	Ch
Valuing the person without conditions	Unconditional positive regard	Approval of all behavior	22
Vicarious introspection	Kohut	Rogers	22

W

Term	Owner	Not…	Ch
WDEP; quality world; five needs	Glasser / Wubbolding	Ellis	2
Who owns the problem; I-messages	Gordon (PET)	Dreikurs	13

Term	Owner	Not…	Ch
Window of tolerance	Siegel	Porges	10
Work values	MIQ (Dawis & Lofquist)	Strong	14
Worry as avoidance of emotional arousal	Borkovec	Clark	10
Would I want this in the news?	Publicity test	Universality test	17

Z

Term	Owner	Not…	Ch
Zone of proximal development; scaffolding	Vygotsky	Piaget	3

Glossary

Definitions are short by design. The chapter number after each entry points to the full explanation.

A

Abandonment — Leaving clients without arranging continued care, including during vacations, illness, and after termination. It is prohibited (ACA A.12). Compare: Appropriate termination. *Ch 21*

Abstinence violation effect — The all-or-nothing reaction to one slip: "I've blown it, so I might as well keep going." Counselors reframe a lapse as a learning event (Alan Marlatt). Compare: Sustain talk. *Ch 5*

Accounting of disclosures — A client's HIPAA right to a list of certain disclosures of their information, generally within 60 days. It excludes treatment, payment, and operations disclosures, disclosures to the client, and authorized ones (45 CFR 164.528). Compare: Right of access. *Ch 17*

Acculturation strategies — Four ways of handling contact with a new culture: integration (keep heritage and join the host culture), assimilation (drop heritage), separation (keep heritage, avoid host), and marginalization (neither) (John Berry). *Ch 3*

Active referral — With consent, the counselor makes contact, books or helps book the appointment, sends records with a release, and confirms the client arrived. It closes the loop. Compare: Passive referral. *Ch 21*

Actualizing tendency — Rogers's term for a person's inherent drive toward growth; it belongs to a therapy, not a motivational theory (Carl Rogers). Compare: Self-actualization. *Ch 2*

Acute stress disorder — Trauma symptoms right after exposure, in the window from 3 days to 1 month after the trauma. Compare: Posttraumatic stress disorder. *Ch 10*

ADA Title III — Treats a counseling office as a public accommodation: qualified interpreters and other auxiliary aids for effective communication, at no charge to the client. The practice may not require a client to bring their own interpreter (28 CFR 36.301(c), 36.303). *Ch 17*

Adjustment disorder — Emotional or behavioral symptoms in response to an identifiable stressor that don't meet criteria for another disorder. Symptoms begin within 3 months and resolve within 6 months after the stressor ends. *Ch 10*

Advance Beneficiary Notice (ABN) — A notice given to Original Medicare beneficiaries before a service Medicare may not cover. It is not for self-pay clients generally. Compare: Good faith estimate. *Ch 17*

Advocacy Competencies — ACA's model crossing client involvement (collaborating with vs. acting on behalf of) with level (client/student, school/community, public arena) to give six domains (Lewis, Arnold, House, & Toporek; updated by Toporek & Daniels). Compare: MSJCC. *Ch 22*

Affect — The counselor's observation of the client's emotional expression in the moment, described by range, congruence, and lability. Think of it as the "weather." Compare: Mood. *Ch 19*

Aftercare plan — How the client keeps recovery going after formal treatment: continuing services, mutual-help or peer support, the relapse prevention plan, routines, supports, and how to return. Compare: Discharge plan. *Ch 21*

Akathisia — Inner restlessness and an urge to move on an antipsychotic, often mistaken for anxiety or worsening psychosis. The counselor documents it and reports it to the prescriber. Compare: Tardive dyskinesia. *Ch 23*

Alliance rupture — A strain in the counseling relationship, either a withdrawal rupture (going quiet, agreeing too easily) or a confrontation rupture (open complaints). Repair by naming and exploring it nondefensively (Safran & Muran). *Ch 4*

Ambiguous loss — A loss without clarity or closure: physically absent but psychologically present, or physically present but psychologically absent, as with a parent who has dementia (Pauline Boss). Compare: Disenfranchised grief. *Ch 12*

Angoff method — How NBCC sets the NCE's passing point: experts estimate how a minimally qualified candidate would perform on each item. Later forms are matched to that standard by statistical equating. *Ch 1*

ANOVA (one-way) — Tests whether three or more group means differ on one independent variable, using the F statistic (Ronald Fisher). Compare: t-test. *Ch 18*

Apparently irrelevant decisions — Small choices that look unrelated to use but move the client toward risk, like taking the route past the old bar; later renamed seemingly irrelevant decisions (Marlatt & Gordon). Compare: High-risk situation. *Ch 21*

Ask Suicide-Screening Questions (ASQ) — A brief screen for medical settings: four yes/no questions, then an acuity question if any answer is yes. It sorts patients for assessment; it is not a risk assessment (NIMH). Compare: C-SSRS. *Ch 5*

Assertive community treatment (ACT) — A multidisciplinary team that delivers services in the community, around the clock, to adults with serious mental illness and frequent hospital use (Stein & Test). Compare: Wraparound. *Ch 23*

Assimilation — A word with two owners: fitting new information into an existing schema (Piaget), or giving up one's heritage culture to join the host culture (Berry). The

stem's context decides. Compare: Acculturation strategies. *Ch 3*

Authorization (HIPAA) — The client's signed permission for uses and disclosures outside treatment, payment, and health care operations and other permitted purposes (45 CFR 164.508). It is not the privacy notice. Compare: Notice of Privacy Practices. *Ch 17*

Automation bias — Trusting output because a machine produced it, such as signing AI drafts unread. The counselor stays accountable for the clinical content (Parasuraman & Manzey). Compare: Algorithm aversion. *Ch 17*

Autonomy — The ethical principle of fostering the client's right to control the direction of their own life. Compare: Beneficence. *Ch 17*

B

BASIC I.D. — The seven modalities assessed in multimodal therapy: Behavior, Affect, Sensation, Imagery, Cognition, Interpersonal, and Drugs/biology (Arnold Lazarus). *Ch 2*

Beneficence — The ethical principle of working for the good of the individual and society by promoting mental health and well-being. Compare: Nonmaleficence. *Ch 17*

Biofeedback — Sensors measure a body process, such as muscle tension, skin temperature, skin conductance, or heart rate variability, and feed it back in real time so the client learns to change it. BCIA certification is voluntary and does not expand legal scope. Compare: Relaxation training. *Ch 5*

Biopsychosocial model — The view that illness involves biological, psychological, and social factors; the basis for the whole-person intake history (George Engel). Compare: Health belief model. *Ch 16*

Bipolar II disorder — At least one hypomanic episode and at least one major depressive episode, with never a full manic episode. Compare: Bipolar I disorder. *Ch 10*

BIRP note — A progress note: Behavior (the client's presentation), Intervention (what the counselor did), Response (how the client responded), and Plan. Compare: DAP note. *Ch 21*

Blocking — A group leader skill that stops harmful or unproductive behavior while protecting the member. Compare: Cutting off. *Ch 8*

Boundary crossing — A departure from the usual counselor role that may be harmless or even helpful, like a brief condolence card. It calls for careful, documented judgment. Compare: Boundary violation. *Ch 17*

Boxed warning — An FDA drug safety warning on the label for serious risks. All antidepressants carry one for increased suicidal thoughts and behaviors in children, adolescents, and young adults under 25. *Ch 23*

Breach notification — An impermissible use or disclosure of PHI is presumed a breach unless a documented risk assessment shows a low probability it was compromised. Clients get notice no later than 60 days after discovery; PHI encrypted to HHS standards is not "unsecured" (45 CFR 164.402, 164.404). *Ch 17*

Broaching — The counselor openly raising race, ethnicity, and culture in counseling rather than waiting for the client to bring them up (Norma Day-Vines). Compare: Cultural humility. *Ch 9*

Bulimia nervosa — Recurrent binge eating plus compensatory behaviors such as vomiting, fasting, or excessive exercise, both at least once a week for 3 months. Weight is often in the normal range. Compare: Binge-eating disorder. *Ch 16*

Burnout — Exhaustion and cynicism from chronic workplace stress, with three dimensions: emotional exhaustion, depersonalization, and reduced personal accomplishment (Christina Maslach). Compare: Compassion fatigue. *Ch 14*

Business associate agreement (BAA) — The contract a vendor that creates, receives, stores, or transmits PHI for the practice (telehealth platform, EHR, billing service, AI note-taker) needs before handling PHI. It does not replace client consent (45 CFR 164.502(e), 164.504(e)). *Ch 17*

C

CAMS — A framework in which counselor and client complete the Suicide Status Form side by side and target the client's suicide drivers (David Jobes). Compare: CBT-SP. *Ch 5*

CAQH ProView — The shared credentialing profile many commercial payers pull from; providers re-attest periodically. Compare: National Practitioner Data Bank. *Ch 17*

CBT-SP — Cognitive behavior therapy for suicide prevention, developed for adolescents after an attempt: chain analysis, early safety planning, skills, and a relapse-prevention task. Suicidal behavior is the direct target (Stanley, Brown, et al.). Compare: Safety plan. *Ch 5*

Certification — A credential from a professional body, such as the NCC from NBCC; voluntary at the national level. Compare: Licensure. *Ch 17*

Change talk — Client statements in favor of change: Desire, Ability, Reasons, Need (preparatory) and Commitment, Activation, Taking steps (mobilizing), remembered as DARN-CAT (Miller & Rollnick). Compare: Sustain talk. *Ch 5*

Chi-square — Tests whether observed frequencies in categories differ from expected frequencies, using nominal data (Karl Pearson). Compare: Pearson r. *Ch 3*

Circular causality — Behaviors cause and are caused by each other in loops: "She nags because he withdraws; he withdraws because she nags." Compare: Linear causality. *Ch 7*

Circumstantial thought — Speech that wanders through extra detail but eventually reaches the point. Compare: Tangential thought. *Ch 19*

Claims-made policy — Liability coverage for claims made and reported while the policy is active. When it ends, the counselor needs tail coverage. Compare: Occurrence policy. *Ch 17*

Clinically significant change — Reliable change plus movement from the clinical range into the functional range (Jacobson & Truax). Compare: Reliable change index. *Ch 21*

Clinical vs. actuarial prediction — Statistical formulas matched or outperformed unaided clinical judgment in nearly every comparison. The “broken leg” exception allows an override when the clinician holds decisive information the formula misses (Paul Meehl). Compare: Self-assessment bias. *Ch 22*

Cognitive dissonance — Discomfort from holding conflicting beliefs or acting against one’s beliefs; a general social psychology term (Leon Festinger). Compare: Abstinence violation effect. *Ch 21*

Cognitive Processing Therapy — A cognitive PTSD treatment, primarily for adults, that challenges stuck points and uses an impact statement (Patricia Resick). Compare: Prolonged Exposure. *Ch 5*

Cognitive reappraisal — Changing the meaning of a situation, which changes the emotion itself (James Gross). Compare: Expressive suppression. *Ch 22*

Cognitive triad — Beck’s description of depression as negative views of the self, the world, and the future (Aaron Beck). *Ch 2*

Collaborative care — Team-based depression care in primary care: a care manager, the primary care provider, and a consulting psychiatrist track symptoms with repeated measures and adjust treatment (Unützer et al.). Compare: Measurement-based care. *Ch 23*

Collaborative empiricism — Beck's stance in which client and counselor test thoughts together like scientists (Aaron Beck). Compare: Disputing. *Ch 2*

Community needs and strengths assessment — Asks what a community lacks and what it already has, before a program starts. Methods include key informants, community forums, surveys, social indicators, and service utilization. Compare: Program evaluation. *Ch 5*

Compassion fatigue — Emotional and physical exhaustion from caring for people who are suffering; includes secondary traumatic stress (Charles Figley). Compare: Burnout. *Ch 14*

Compassion satisfaction — The pleasure and meaning a helper draws from doing the work well; it can offset burnout and secondary traumatic stress (Beth Hudnall Stamm). Compare: Compassion fatigue. *Ch 22*

Confidential communications — A client's HIPAA right to ask to be contacted by other means or at other locations. Providers must accommodate reasonable requests without asking why (45 CFR 164.522(b)). Compare: Authorization (HIPAA). *Ch 17*

Confidentiality — The counselor's ethical, and often legal, duty not to disclose client information. Compare: Privileged communication. *Ch 17*

Confrontation — Gently pointing out a discrepancy (words vs. words, words vs. actions, words vs. nonverbals), best used once the alliance is established. Compare: Immediacy. *Ch 4*

Congruence (counselor) — The counselor's inner experience matches what the counselor outwardly communicates. It doesn't mean saying everything one feels (Carl Rogers). Compare: Genuineness. *Ch 22*

Congruence (Holland) — The fit between a person's personality type and the work environment's type (John Holland). Compare: Consistency (Holland). *Ch 14*

Consistency (Holland) — How close a person's top types are on the hexagon; adjacent types are consistent and opposite types are inconsistent (John Holland). Compare: Congruence (Holland). *Ch 14*

Consultation — A voluntary, nonhierarchical request for advice; the consulting counselor keeps full responsibility for the client. Compare: Supervision. *Ch 17*

Contemplation — A stage of change in which the client acknowledges a problem but feels ambivalent. It is never a group stage (Prochaska & DiClemente). Compare: Transition stage. *Ch 5*

Continuing bonds — Maintaining a connection to the deceased can be healthy, not a failure to "let go" (Klass, Silverman, & Nickman). Compare: Prolonged grief disorder. *Ch 12*

Counseling Compact — Lets a counselor practice in member states under a privilege to practice, based on an active, unencumbered home-state license. The counselor follows the laws of the remote state where the client is located; the NCC is not required. Compare: Telehealth licensure. *Ch 17*

Countertransference — The counselor's own emotional reactions to the client, often rooted in the counselor's history. It is managed through self-reflection and supervision or consultation. Compare: Transference. *Ch 5*

Court order — A disclosure order signed by a judge. Comply, but disclose only what is required and ask the court to limit disclosure if needed. Compare: Subpoena. *Ch 17*

Crisis — A person's response when an event overwhelms their usual coping. Crises are time-limited (often about 4 to 6 weeks; Gerald Caplan) and carry both danger and opportunity (James & Gilliland). *Ch 6*

Critical incident stress debriefing (CISD) — A structured single-session group meeting held within days of a traumatic event (Jeffrey Mitchell). A Cochrane review

found it did not prevent PTSD and may harm, and WHO advises against routine debriefing. Compare: Psychological first aid. *Ch 22*

C-SSRS — The Columbia-Suicide Severity Rating Scale grades ideation from a wish to be dead to active ideation with plan and intent, and asks separately about behavior (Posner et al.). Compare: Ask Suicide-Screening Questions (ASQ). *Ch 5*

Cultural Formulation Interview — A 16-question DSM-5-TR interview covering the cultural definition of the problem, perceived causes and supports, self-coping and past help-seeking, and current help-seeking. It can be used with any client. *Ch 19*

Cultural humility — An other-oriented, curious stance of ongoing self-reflection that recognizes the limits of one's cultural knowledge (Joshua Hook and colleagues). Compare: Broaching. *Ch 9*

Cutting off — A group leader skill that interrupts a member who goes on too long or off topic, to manage time or focus. Compare: Blocking. *Ch 8*

D

DAP note — A progress note: Data (client report and counselor observations combined), Assessment, and Plan. Compare: SOAP note. *Ch 21*

Defense mechanism — An unconscious strategy that protects against anxiety, such as repression, projection, or displacement (Sigmund Freud; later systematized by Anna Freud). *Ch 2*

De-identification — Removing identifiers so information is no longer PHI, by safe harbor or expert determination. Removing a last name is not de-identification (45 CFR 164.514(b)). Compare: Minimum necessary. *Ch 17*

Delirium — Sudden (hours to days), fluctuating confusion with markedly impaired attention, usually from a

medical cause. It is a medical emergency and often reversible. Compare: Major neurocognitive disorder. *Ch 12*

Delusion — A fixed false belief held with full conviction despite evidence. Compare: Obsession. *Ch 19*

Deviation IQ — A standard score with a mean of 100 and an SD of 15 (David Wechsler). Compare: Ratio IQ. *Ch 3*

Dialectical behavior therapy — A therapy that balances acceptance and change, teaching mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness skills (Marsha Linehan). *Ch 2*

Diana v. State Board of Education (1970) — Court case holding that children must be tested in their primary language, with nonverbal measures, before special education placement. Compare: Larry P. v. Riles. *Ch 20*

Differentiation (Holland) — How clearly one personality type stands out from the others; a flat profile is undifferentiated (John Holland). Compare: Differentiation of self. *Ch 14*

Differentiation of self — Bowen's central concept. Low differentiation lets anxiety pass across generations, so raising it is the goal of therapy (Murray Bowen). Compare: Enmeshment. *Ch 2*

Discharge plan — How the client leaves this service safely: started at admission, with a booked follow-up, a signed release, records sent, a medication bridge, and a crisis plan. Compare: Aftercare plan. *Ch 21*

Disenfranchised grief — Grief over a loss that isn't socially recognized, such as the death of an ex-partner, a pet, or a pregnancy loss (Kenneth Doka). Compare: Ambiguous loss. *Ch 12*

Disputing — Ellis's direct, often forceful challenge to irrational beliefs such as "musts" (Albert Ellis). Compare: Collaborative empiricism. *Ch 2*

Disruptive mood dysregulation disorder — Severe temper outbursts 3 or more times a week with irritable

mood between them, for at least 12 months; diagnosed ages 6 to 18, with onset before 10. Compare: Oppositional defiant disorder. *Ch 10*

Duty to protect — Tarasoff II (1976): take reasonable steps to protect the intended victim of a client's serious threat, such as warning, notifying police, or arranging hospitalization. Compare: Duty to warn. *Ch 6*

Duty to warn — Tarasoff I (1974): therapists must warn an identifiable victim of a client's serious threat. Compare: Duty to protect. *Ch 6*

E

Ecomap — A diagram of a family's connections to outside systems such as school, work, church, and agencies (Ann Hartman). Compare: Genogram. *Ch 7*

Effectiveness — Whether a treatment works in routine practice with typical, often comorbid clients. Compare: Efficacy. *Ch 22*

Effect size — How large a difference is, apart from statistical significance. For Cohen's d, about .2 is small, .5 medium, and .8 large. *Ch 18*

Efficacy — Whether a treatment works under ideal research conditions, in tightly controlled trials with screened samples. Compare: Effectiveness. *Ch 22*

Empathy — Understanding the client's experience from the client's frame of reference. Compare: Sympathy. *Ch 22*

Empirically supported therapy criteria — Levels for therapies tested in randomized trials with a manual and a defined population: efficacious and specific, efficacious, and possibly efficacious (Chambless & Hollon). Compare: Evidence-based practice. *Ch 22*

Empty chair — A signature Gestalt technique, alongside two-chair dialogue, tied to here-and-now awareness and

unfinished business (Fritz Perls). Compare: Psychodrama. *Ch 2*

Enactment — Having family members interact in session so the counselor can see the pattern live (Salvador Minuchin). *Ch 7*

Enmeshment — Diffuse boundaries between family members (Salvador Minuchin). Compare: Triangulation. *Ch 2*

Ethic of care — Moral reasoning grounded in care and relationships, not only justice; a critique of Kohlberg (Carol Gilligan). Compare: Moral development stages. *Ch 3*

Evidence-based practice — Integrating the best available research with clinical expertise and client characteristics, culture, and preferences (Sackett et al.; APA Presidential Task Force, 2006). Compare: Empirically supported therapy criteria. *Ch 22*

Exception questions — SFBT questions about times the problem was absent or weaker (de Shazer & Berg). Compare: Unique outcomes. *Ch 2*

Exposure and response prevention — Facing the obsession without doing the compulsion; the treatment with the strongest support for OCD. Compare: Systematic desensitization. *Ch 5*

Expressed emotion — Family criticism, hostility, and emotional over-involvement linked to higher relapse in schizophrenia; family psychoeducation lowers it (George Brown; Vaughn & Leff). Compare: Double bind. *Ch 16*

Externalizing — Talking about the problem as a separate entity acting on the client: “The person is not the problem; the problem is the problem” (White & Epston). Compare: Exception questions. *Ch 2*

External validity — Whether results generalize to other people, settings, and times; supported by random sampling. Compare: Internal validity. *Ch 18*

F

Factitious disorder — Intentionally falsifying symptoms without an obvious external reward, in order to take on the sick role; imposed on self or on another. Compare: Malingering. *Ch 19*

Family-based treatment — The first-line treatment for adolescent anorexia, in which parents temporarily take charge of refeeding; also called the Maudsley approach (Lock & Le Grange). *Ch 5*

FERPA — The federal law governing education records held by schools, including a school-employed counselor's records; these are excluded from HIPAA. Compare: HIPAA. *Ch 17*

Fidelity — The ethical principle of honoring commitments and keeping promises. Compare: Veracity. *Ch 17*

Filial therapy — Parents are trained to run child-centered play sessions at home (Bernard & Louise Guerney). Compare: PCIT. *Ch 5*

First-order change — Change within the system's rules, so the pattern stays, like grounding a teen for a week instead of a day (MRI). Compare: Second-order change. *Ch 7*

Five stages of dying — Denial, anger, bargaining, depression, and acceptance; described for terminally ill patients and not a required sequence (Elisabeth Kübler-Ross). Compare: Tasks of mourning. *Ch 12*

Flight of ideas — Rapid jumps between loosely connected topics, common in mania. Compare: Loose associations. *Ch 19*

Flooding — Immediate, intense, prolonged real-life exposure to a feared situation. Compare: Implosive therapy. *Ch 5*

Forester-Miller & Davis model — The practitioner's seven-step ethical decision-making model: identify the problem, apply the ACA Code, determine the nature of

the dilemma, generate actions, consider consequences and choose, evaluate, and implement. Compare: Tarvydas integrative model. *Ch 17*

Four Horsemen — Criticism, contempt, defensiveness, and stonewalling; contempt is the strongest predictor of divorce (John & Julie Gottman). Compare: Pursue–withdraw cycle. *Ch 7*

G

Gambling disorder — The only behavioral addiction in the DSM-5-TR addictions chapter: at least 4 of 9 criteria in 12 months, with "chasing" losses as the classic sign. Suicide risk is elevated. *Ch 11*

Gender dysphoria — Clinically significant distress related to incongruence between experienced and assigned gender, lasting at least 6 months. Being transgender is not itself a mental disorder. *Ch 15*

General Adaptation Syndrome — The stress response stages of alarm, resistance, and exhaustion; also distinguishes eustress from distress (Hans Selye). Compare: Transactional model of stress. *Ch 3*

Generalized anxiety disorder — Excessive, hard-to-control worry about many things plus at least 3 of 6 symptoms (1 in children), for at least 6 months. Compare: Panic disorder. *Ch 10*

Genogram — A diagram of three or more generations showing relationships, patterns, cutoffs, illnesses, and conflict (Bowen's concept; standardized by McGoldrick & Gerson). Compare: Ecomap. *Ch 7*

Genuineness — Being real and authentic, not hiding behind a professional role or façade. Compare: Congruence (counselor). *Ch 22*

Good faith estimate — A written estimate of expected charges for uninsured and self-pay clients under the No Surprises Act (45 CFR 149.610). Compare: Advance Beneficiary Notice (ABN). *Ch 17*

Griggs v. Duke Power Co. (1971) — Court case holding that employment tests must be job-related; tests that screen out protected groups without job relevance are discriminatory. *Ch 20*

Grounded theory — A qualitative tradition that develops a theory from the data (Glaser & Strauss). Compare: Phenomenology. *Ch 18*

Group cohesiveness — Belonging and acceptance; the group equivalent of the therapeutic alliance and a precondition for the other factors (Irvin Yalom). Compare: Universality. *Ch 8*

H

Hallucination — A sensory experience with no stimulus present; auditory hallucinations are most common in psychotic disorders. Compare: Illusion. *Ch 19*

Hawthorne effect — Participants change because they know they're being studied (Elton Mayo). Compare: Placebo effect. *Ch 18*

Here-and-now focus — Attention to present experience. It is a central focus of Gestalt therapy (Fritz Perls); in groups, members first experience reactions in the room, then reflect on what just happened (Irvin Yalom). Compare: Immediacy. *Ch 8*

Hierarchy of needs — Maslow's motivational theory: physiological, safety, love/belonging, esteem, then self-actualization. Unmet lower needs block growth (Abraham Maslow). Compare: Actualizing tendency. *Ch 2*

HIPAA — Federal law protecting the privacy and security of health information. Clients can access their records, and psychotherapy notes get extra protection. *Ch 17*

HIPAA contingency plan — The Security Rule plan for data backup, disaster recovery, and emergency-mode operation so electronic records survive (45 CFR 164.308(a)(7)). Compare: Professional will. *Ch 17*

Hypertensive crisis — An emergency from an MAOI taken with tyramine-rich foods or some decongestants: sudden severe headache, stiff neck, pounding heart, nausea. Compare: Serotonin syndrome. *Ch 23*

Hypomanic episode — A manic-like mood pattern lasting at least 4 consecutive days, without marked impairment, hospitalization, or psychotic features. Compare: Manic episode. *Ch 10*

I

Identified patient — The family member whose symptoms bring the family to counseling; the symptom often serves a function for the family. *Ch 7*

Identity statuses — Four statuses: diffusion (no exploration, no commitment), foreclosure (commitment without exploration), moratorium (exploration without commitment), and achievement (commitment after exploration) (James Marcia). *Ch 3*

Illusion — Misperceiving a real stimulus, like a coat on a chair that looks like a person. Compare: Hallucination. *Ch 19*

Immediacy — Addressing what is happening between counselor and client right now. Compare: Self-disclosure. *Ch 4*

Impairment — Physical, mental, or emotional problems that compromise a counselor's work. Counselors refrain from practicing when impaired and seek help (ACA C.2.g). *Ch 17*

Implosive therapy — Intense imagined exposure, often exaggerated (Thomas Stampfl). Compare: Flooding. *Ch 5*

Impostor phenomenon — High achievers feeling like frauds, often heightened for people underrepresented in their fields (Clance & Imes). Compare: Stereotype threat. *Ch 15*

Independent variable — What the researcher manipulates or compares, such as type of treatment. Compare: Dependent variable. *Ch 18*

Industry vs. inferiority — Erikson's school-age stage, when a child compares her work to classmates' and may feel incompetent (Erik Erikson). Compare: Concrete operational stage. *Ch 3*

Informed consent — An ongoing process, not a one-time form, covering services, risks, fees, confidentiality and its limits, and the right to refuse (ACA A.2). *Ch 17*

Intellectual developmental disorder — Deficits in both intellectual and adaptive functioning, beginning in the developmental period. Severity is based on adaptive functioning, not IQ alone. *Ch 12*

Internal Family Systems (IFS) — The mind is made of parts (exiles, managers, firefighters) led by an undamaged core Self; healing comes through unblending and unburdening (Richard Schwartz). Compare: Empty chair. *Ch 5*

Internal validity — Whether the independent variable really caused the change in the dependent variable; supported by random assignment. Compare: External validity. *Ch 18*

Interpersonal theory of suicide — Thwarted belongingness plus perceived burdensomeness create the desire for death, and acquired capability makes an attempt possible (Thomas Joiner). Compare: Psychache. *Ch 6*

Interpretation — Proposing a connection outside the client's current awareness, often to the past, offered tentatively. Compare: Reframe. *Ch 4*

Interpreter — A professional who renders spoken or signed language in real time. A qualified interpreter is trained, impartial, and bound by confidentiality; being bilingual is not enough. Compare: Translator. *Ch 23*

Intimate terrorism — Violence used as part of a pattern of coercive control, usually more frequent, severe, and escalating (Michael P. Johnson). Compare: Situational couple violence. *Ch 13*

J

Jaffee v. Redmond (1996) — U.S. Supreme Court case recognizing psychotherapist–patient privilege in federal courts, including for licensed clinical social workers. *Ch 17*

John Henry effect — The control group works harder to compete with the treatment group. Compare: John Henryism. *Ch 18*

John Henryism — High-effort coping with chronic stressors, including discrimination, linked to health problems in Black Americans with limited resources (Sherman James). Compare: John Henry effect. *Ch 15*

Joining — Connecting with every family member, accommodating the family's style, and earning trust before trying to change anything (Salvador Minuchin). *Ch 7*

Justice — The ethical principle of treating individuals equitably and fostering fairness and equality. Compare: Autonomy. *Ch 17*

K

Kruskal–Wallis test — The nonparametric alternative to one-way ANOVA. Compare: Mann–Whitney U. *Ch 18*

L

Lapse — A single slip, such as one drink; an event and a learning opportunity. A relapse is a return to the old pattern. Compare: Abstinence violation effect. *Ch 21*

Larry P. v. Riles (1979) — Court case holding that IQ tests could not be used to place Black children in California classes for "educable mentally retarded" students be-

cause of cultural bias. Compare: Diana v. State Board of Education. *Ch 20*

Learned helplessness — Repeated uncontrollable bad events teach a person that action is useless (Martin Seligman). Compare: Reformulated helplessness theory. *Ch 10*

Licensure — State permission to practice, such as the LPC; the strongest credential, protecting both title and practice. Compare: Certification. *Ch 17*

Life review — A structured review of one's whole life to resolve conflicts and find meaning, suited to late life (Robert Butler). Compare: Reminiscence therapy. *Ch 5*

Life-span, life-space stages — Growth (0–14), exploration (15–24), establishment (25–44), maintenance (45–64), and disengagement (65+), with recycling to earlier stages possible (Donald Super). Compare: Ginzberg's developmental periods. *Ch 14*

Linking — A group leader skill that connects one member's concern to another's, building universality and member-to-member interaction. Compare: Universalizing. *Ch 8*

Lithium toxicity — An emergency: coarse tremor, vomiting, diarrhea, confusion, unsteady gait, slurred speech. Dehydration, GI illness, low salt, and NSAIDs raise levels. *Ch 23*

Loose associations — Ideas with no logical connection, common in psychosis. Compare: Flight of ideas. *Ch 19*

M

Major depressive episode — At least 5 of 9 symptoms, including depressed mood or loss of interest, for at least 2 weeks, with significant distress or impairment. Compare: Persistent depressive disorder. *Ch 10*

Malingering — Intentionally producing or exaggerating symptoms for an external reward, such as money or

avoiding prosecution; a Z code, not a mental disorder. Compare: Factitious disorder. *Ch 19*

Malpractice — A legal claim that generally requires four Ds: duty, dereliction (breach of the standard of care), direct causation, and damages. *Ch 17*

Mandated reporting — Counselors must report suspected child abuse on reasonable suspicion, not proof, promptly and without investigating. Reporters acting in good faith are protected. *Ch 6*

Manic episode — Abnormally elevated, expansive, or irritable mood with increased energy for at least 1 week (any duration if hospitalized), with marked impairment. Compare: Hypomanic episode. *Ch 10*

Maslach Burnout Inventory — Measures burnout's three dimensions: emotional exhaustion, depersonalization, and reduced personal accomplishment (Maslach & Jackson). Compare: ProQOL. *Ch 22*

MBCT — Mindfulness-Based Cognitive Therapy: an 8-week group for relapse prevention in recurrent depression, built on decentering and the 3-minute breathing space (Segal, Williams, & Teasdale). Compare: MBSR. *Ch 5*

MBTI — Myers-Briggs Type Indicator: four dichotomies (E–I, S–N, T–F, J–P) yield 16 types. Used for self-understanding, not hiring (Myers & Briggs, from Jung). Compare: Strong Interest Inventory. *Ch 5*

Means restriction counseling — Helping a client and family limit access to firearms, stockpiled medications, and other lethal means during periods of suicide risk. *Ch 6*

Measurement-based care — Giving a standardized measure routinely, reviewing it with the client, and using the data to keep or change the plan (Lewis et al.). Compare: Routine outcome monitoring. *Ch 21*

Mediator — A variable that explains how or why the independent variable affects the dependent variable. Compare: Moderator. *Ch 18*

Mental status exam — A snapshot of current mental functioning, mostly from observation: appearance, behavior, speech, mood, affect, thought process and content, perception, cognition, insight, and judgment. *Ch 19*

Microinsult — Subtle rudeness that demeans a person's identity, like telling a Black client "You're so articulate" (Derald Wing Sue). Compare: Microinvalidation. *Ch 9*

Microinvalidation — A remark that negates a person's thoughts, feelings, or experiences, like "I don't see color" (Derald Wing Sue). Compare: Microinsult. *Ch 9*

Minimum necessary — Use, disclose, and request only the PHI the purpose needs. It does not apply to disclosures for treatment, to the client, or under an authorization (45 CFR 164.502(b), 164.514(d)). Compare: Psychotherapy notes. *Ch 17*

Minority stress model — Distal stressors (discrimination, harassment) and proximal stressors (expected rejection, concealment, internalized stigma) explain higher distress among sexual and gender minority people. The stress, not the identity, is the problem (Ilan Meyer). *Ch 15*

Miracle question — "Suppose you went to sleep tonight and the problem was solved. What would be different tomorrow?" (de Shazer & Berg). Compare: Exception questions. *Ch 2*

Moderator — A variable that changes the strength or direction of a relationship, showing for whom or when an effect holds. Compare: Mediator. *Ch 18*

Mood — The client's self-reported, sustained emotional state, often in their own words. Think of it as the "climate." Compare: Affect. *Ch 19*

Moral development stages — Preconventional (obedience and punishment; self-interest), conventional (interpersonal conformity; law and order), and postconventional (social contract; universal ethical principles), judged by justice reasoning (Lawrence Kohlberg). Compare: Ethic of care. *Ch 3*

Motivational interviewing — A method for resolving ambivalence, built on a spirit of partnership, acceptance, compassion, and evocation and the OARS skills (Miller & Rollnick). Compare: Transtheoretical model. *Ch 5*

MSJCC — The Multicultural and Social Justice Counseling Competencies: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions (Ratts et al.). Compare: Multicultural orientation. *Ch 22*

Multicultural orientation — A way of being with culture in session, built on cultural humility, cultural opportunities, and cultural comfort (Owen et al.). Compare: MSJCC. *Ch 5*

N

National Practitioner Data Bank (NPDB) — The federal data bank of malpractice payments and adverse licensure and privileging actions; payers and hospitals query it. Compare: CAQH ProView. *Ch 17*

Necessary and sufficient conditions — Rogers's six conditions for change: psychological contact, client incongruence, counselor congruence, unconditional positive regard, empathic understanding, and the client perceiving that empathy and regard (Carl Rogers). *Ch 22*

Negative feedback — Feedback that reduces deviation and keeps the system the same (morphostasis). "Negative" describes direction, not value. Compare: Positive feedback. *Ch 7*

Negative reinforcement — Removing something unpleasant to increase a behavior, like a seatbelt chime that stops when you buckle. Compare: Punishment. *Ch 2*

Nigrescence model — Black racial identity stages: pre-encounter, encounter, immersion–emersion, internalization, and internalization–commitment (William Cross). Compare: White racial identity model. *Ch 3*

Nonmaleficence — The ethical principle of avoiding actions that cause harm. Compare: Beneficence. *Ch 17*

Normal curve — A symmetrical distribution where mean, median, and mode are equal; about 68%, 95%, and 99.7% of scores fall within 1, 2, and 3 SDs. *Ch 18*

Norm-referenced — Scoring that compares a person to other people. Compare: Criterion-referenced. *Ch 3*

No-secrets policy — In couple or family work, information shared individually may be brought into joint sessions. Explain it at the start, before anyone shares a secret. *Ch 7*

No-suicide contract — A promise not to harm oneself. It has no evidence of preventing suicide and can give the counselor false reassurance. Compare: Safety plan. *Ch 6*

Notice of Privacy Practices (NPP) — The HIPAA notice given no later than the first service, with a good-faith effort to get written acknowledgment, and also posted in the office and on the website. Care is never withheld for a missing signature (45 CFR 164.520). Compare: Authorization (HIPAA). *Ch 17*

NPI — National Provider Identifier, issued through NPPES: Type 1 for an individual (one per person, for life), Type 2 for an organization. Compare: CAQH ProView. *Ch 17*

Null hypothesis — The prediction of no difference or relationship. Researchers reject or fail to reject it; they never prove it. *Ch 18*

O

OARS — Motivational interviewing's core skills: Open questions, Affirmations, Reflections, and Summaries (Miller & Rollnick). *Ch 5*

Obsession — An intrusive, unwanted thought, urge, or image that causes anxiety and is usually experienced as alien (ego-dystonic). Compare: Delusion. *Ch 10*

Occurrence policy — Liability coverage for incidents that occur during the policy period, whenever the claim is filed; no tail is needed. Compare: Claims-made policy. *Ch 17*

O*NET — The Department of Labor's occupational database, which replaced the Dictionary of Occupational Titles. Compare: Occupational Outlook Handbook. *Ch 14*

Open question — A question that invites elaboration, usually starting with "what" or "how"; the default choice. Compare: Closed question. *Ch 4*

Ordinal scale — Rank order, like class rank; Likert ratings are usually treated as ordinal. Compare: Interval scale. *Ch 3*

Other specified disorder — A diagnosis in which the clinician states why full criteria aren't met. Compare: Unspecified disorder. *Ch 19*

P

Panic disorder — Recurrent unexpected panic attacks followed by at least 1 month of worry about more attacks or changed behavior to avoid them. Compare: Generalized anxiety disorder. *Ch 10*

Paradoxical intention — Asking an individual to intend the feared symptom, like trying hard to stay awake for insomnia (Viktor Frankl). Compare: Prescribing the symptom. *Ch 2*

Paraphrase — Restating the content of one client statement in fresh words. Compare: Reflection of feeling. *Ch 4*

Parenting styles — Authoritative (warm and firm, best outcomes), authoritarian (firm, not warm), and permissive (warm, not firm); Maccoby & Martin added neglectful/uninvolved (Diana Baumrind). *Ch 3*

Part 2 (42 CFR Part 2) — Federal rules giving extra confidentiality protection to records from federally assisted substance use disorder programs; disclosure generally requires specific written consent. Compare: HIPAA. *Ch 9*

Passive referral — Handing over a name, list, or number and leaving the client to follow through. Compare: Active referral. *Ch 21*

PCIT — Parent–Child Interaction Therapy, which coaches parents in real time, often through an earpiece, with a behavioral focus (Sheila Eyberg). Compare: Filial therapy. *Ch 5*

PCOMS — Two brief scales given each session: the Outcome Rating Scale for how the client is doing and the Session Rating Scale for the session and alliance (Miller & Duncan). Compare: Routine outcome monitoring. *Ch 21*

Pearson r — Measures the relationship between two interval/ratio variables, from −1.00 to +1.00; the absolute value shows strength. Compare: Spearman rho. *Ch 18*

Peer recovery specialist — Shares lived experience of recovery and supports recovery goals, but does not diagnose, write the treatment plan, or prescribe (SAMHSA). *Ch 23*

Persistent depressive disorder — Depressed mood most days plus at least 2 other symptoms for at least 2 years in adults (1 year in children and adolescents). Compare: Major depressive episode. *Ch 10*

Planned happenstance — Treating unplanned events, like a layoff, as career opportunities (John Krumboltz). Compare: Career adaptability. *Ch 3*

Positive feedback — Feedback that amplifies deviation and pushes the system toward change (morphogenesis), like an argument that escalates. "Positive" describes direction, not value. Compare: Negative feedback. *Ch 7*

Positive skew — A distribution with its tail to the right and most scores low, as on a hard test; mean > median > mode. Compare: Negative skew. *Ch 18*

Posttraumatic stress disorder — After actual or threatened death, serious injury, or sexual violence: intrusion, avoidance, negative changes in thinking and mood, and arousal symptoms for more than 1 month. Compare: Acute stress disorder. *Ch 10*

Prescribing the symptom — A paradoxical directive used with a family to interrupt a stuck pattern (Jay Haley; MRI). Compare: Paradoxical intention. *Ch 2*

Principle ethics — Asks "What shall I do?" and solves a specific dilemma using obligations and principles (Meara, Schmidt, & Day). Compare: Virtue ethics. *Ch 22*

Privileged communication — A legal right that protects client communications from disclosure in court; the client holds it and can waive it. Compare: Confidentiality. *Ch 17*

Professional will — A written plan naming a colleague or records custodian to notify clients, protect records, and transfer care if the counselor dies or is incapacitated (ACA C.2.h). Compare: HIPAA contingency plan. *Ch 21*

Projection — A defense that attributes one's own unacceptable feelings to someone else (Sigmund Freud). Compare: Displacement. *Ch 2*

Prolonged Exposure — A PTSD treatment, primarily for adults, using imaginal and in vivo exposure for emotional processing (Edna Foa). Compare: Cognitive Processing Therapy. *Ch 5*

Prolonged grief disorder — Intense yearning for or preoccupation with the deceased, plus other symptoms, at least 12 months after the death for adults (6 months for

youth), beyond cultural norms. Compare: Continuing bonds. *Ch 12*

ProQOL — The Professional Quality of Life Scale, measuring compassion satisfaction, burnout, and secondary traumatic stress (Beth Hudnall Stamm). Compare: Maslach Burnout Inventory. *Ch 22*

Psychache — Intolerable psychological pain, the core of suicide in Shneidman's view (Edwin Shneidman). Compare: Interpersonal theory of suicide. *Ch 6*

Psychodrama — A group method with a protagonist, director, auxiliary egos, audience, and stage, using doubling, role reversal, and the mirror technique (J. L. Moreno). Compare: Empty chair. *Ch 3*

Psychological first aid — An evidence-informed early response after a disaster: contact, safety, stabilization, practical help, and connection to supports, without pushing survivors to retell the trauma. Compare: Psychological debriefing. *Ch 6*

Psychotherapy notes — The counselor's separately kept notes analyzing session content. They need a specific authorization for most disclosures, even to a health plan, and are excluded from the HIPAA right of access (45 CFR 164.508(a)(2)). Compare: Minimum necessary. *Ch 17*

Punishment — A consequence that decreases behavior: positive punishment adds something unpleasant, and negative punishment removes something pleasant. Compare: Negative reinforcement. *Ch 2*

p-value — The probability of results at least this extreme if the null hypothesis were true; the result is significant when p is below alpha. *Ch 18*

Q

Quasi-experiment — A design that manipulates the independent variable without random assignment, often using intact groups such as two classrooms. Compare: True experiment. *Ch 18*

R

Random assignment — Placing participants into groups by chance; it supports internal validity and cause-and-effect conclusions. Compare: Random sampling. *Ch 18*

Reaction formation — A defense that expresses the opposite of what one feels (Sigmund Freud). *Ch 2*

Reflection of feeling — Naming the emotion under the client's content. Compare: Paraphrase. *Ch 4*

Reflection of meaning — Naming the value or belief underneath, or why something matters to the client. Compare: Reflection of feeling. *Ch 4*

Reframe — Offering a new meaning for the same facts. Compare: Interpretation. *Ch 4*

Relapse prevention — A cognitive-behavioral model of relapse as a process: lifestyle imbalance, apparently irrelevant decisions, a high-risk situation, coping or not, and the abstinence violation effect (Marlatt & Gordon). *Ch 21*

Reliable change index — Tests whether a score change is larger than measurement error would produce (Jacobson & Truax). Compare: Clinically significant change. *Ch 21*

Repression — A defense that pushes threatening material out of awareness (Sigmund Freud). Compare: Denial. *Ch 2*

Resilient practitioner — Counseling is one-way caring, so resilience requires balancing other-care with self-care. The cycle of caring runs through empathic attachment, active involvement, felt separation, and re-creation (Thomas Skovholt). *Ch 22*

RIASEC — Six personality and work types: Realistic, Investigative, Artistic, Social, Enterprising, Conventional, arranged in a hexagon. Opposite pairs are R–S, I–E, and A–C (John Holland). *Ch 14*

Right of access — A client's HIPAA right to the designated record set, currently within 30 days with one 30-

day extension. Separately kept psychotherapy notes are excluded (45 CFR 164.524). Compare: Accounting of disclosures. *Ch 17*

Romantic relationships with former clients — Prohibited for 5 years after the last professional contact; after that, counselors must still show the relationship isn't exploitative (ACA A.5.c). *Ch 17*

Routine outcome monitoring — Measuring outcomes routinely; feedback on not-on-track clients improves their outcomes and reduces deterioration (Michael Lambert, OQ-45). Compare: Measurement-based care. *Ch 21*

S

Safety plan — A written plan made with the client: warning signs, internal coping, distracting people and places, people to ask for help, professionals and 988, and making the environment safe (Stanley & Brown). Compare: No-suicide contract. *Ch 6*

Schizophrenia — At least 2 of 5 psychotic symptoms with a decline in functioning; active symptoms for at least 1 month and continuous signs for at least 6 months. Compare: Schizophreniform disorder. *Ch 16*

Schizophreniform disorder — The same symptoms as schizophrenia, lasting at least 1 month but less than 6 months. Compare: Schizophrenia. *Ch 16*

Second-order change — Change to the system's rules themselves, like parents starting to agree on rules together. Compare: First-order change. *Ch 7*

Section 504 plan — Accommodations for a student whose disability substantially limits a major life activity, even without a need for special education. Compare: Individualized Education Program. *Ch 12*

Self-actualization — The top of Maslow's hierarchy of needs; a motivational theory, not a therapy (Abraham Maslow). Compare: Actualizing tendency. *Ch 2*

Self-Directed Search — A self-administered, self-scored, self-interpreted interest inventory that yields a three-letter Holland code (John Holland). Compare: Strong Interest Inventory. *Ch 5*

Self-disclosure — Briefly sharing something from the counselor's own life for the client's benefit, then returning focus to the client. Compare: Immediacy. *Ch 4*

Self-efficacy — Confidence in one's ability, built through mastery experiences, modeling, and encouragement. Low self-efficacy, not low ability, can keep capable clients from a field (Albert Bandura; applied to careers by Betz & Hackett). *Ch 14*

Sensitivity — A test's ability to correctly identify people who have the condition; screening tools prioritize it. Compare: Specificity. *Ch 20*

Sensorimotor Psychotherapy — Integrates body-based work with cognitive and emotional processing, tracking posture and movement and keeping arousal within the window of tolerance (Pat Ogden). Compare: Somatic Experiencing. *Ch 5*

Serotonin syndrome — An emergency, often from an MAOI combined with serotonergic drugs: agitation, confusion, sweating, fever, fast heartbeat, muscle twitching or rigidity, diarrhea. Compare: Hypertensive crisis. *Ch 23*

Situational couple violence — Violence arising from specific conflicts that escalate, without a broader pattern of control; the most common type in community samples (Michael P. Johnson). Compare: Intimate terrorism. *Ch 13*

SMART objectives — Treatment objectives that are Specific, Measurable, Achievable, Relevant, and Time-bound. Compare: Well-formed goals. *Ch 21*

SOAP note — A progress note: Subjective (the client's report, in their words), Objective (counselor observations and scores), Assessment, and Plan. Compare: BIRP note. *Ch 21*

Social interest — Adler's central concept (Gemeinschaftsgefühl). Adlerian counseling aims to increase it and to correct basic mistakes (Alfred Adler). *Ch 2*

SOLER — An attending posture: face the client Squarely, Open posture, Lean slightly in, Eye contact, Relaxed. Adapt it to the client's culture (Gerard Egan). *Ch 4*

Somatic Experiencing — A bottom-up trauma therapy using titration (small doses of activation) and pendulation (moving between activation and a resource) (Peter Levine). Compare: Sensorimotor Psychotherapy. *Ch 5*

Specificity — A test's ability to correctly identify people who don't have the condition (true negative rate). Compare: Sensitivity. *Ch 20*

Standard error of measurement — Builds a confidence band around one person's score: about 68% within ±1 SEM and 95% within ±2 SEM. Higher reliability means a smaller SEM. Compare: Standard deviation. *Ch 20*

Statistical regression — Participants selected for extreme scores drift toward the mean on retest. Compare: Maturation. *Ch 18*

Stereotype threat — Concern about confirming a negative stereotype about one's group lowers performance (Steele & Aronson). Compare: Impostor phenomenon. *Ch 15*

Strange Situation — A procedure identifying secure, anxious-avoidant, and anxious-resistant infant attachment; Main & Solomon later added disorganized (Mary Ainsworth). *Ch 3*

Strong Interest Inventory — Compares a person's interests with those of people satisfied in specific occupations; publisher-scored and professionally interpreted (E. K. Strong Jr.). Compare: Self-Directed Search. *Ch 5*

Subpoena — A request for records, often issued by an attorney. Don't ignore it or simply hand over records; con-

tact the client and assert privilege. Compare: Court order. *Ch 17*

Substance use disorder severity — Based on the number of 11 criteria met: mild 2–3, moderate 4–5, severe 6 or more. Early remission is 3 to under 12 months and sustained remission 12+ months with no criteria met except craving. Compare: Gambling disorder. *Ch 11*

Summary — A response that ties together several statements or themes. Compare: Paraphrase. *Ch 4*

Supervision — An ongoing, evaluative, hierarchical relationship in which the supervisor shares responsibility for client care. Compare: Consultation. *Ch 17*

Sustain talk — The client's arguments for staying the same, met with reflection rather than argument (Miller & Rollnick). Compare: Change talk. *Ch 5*

Sympathy — Feeling sorry for the client from one's own frame of reference. Compare: Empathy. *Ch 22*

Systematic desensitization — Relaxation paired with imagined, gradual steps up an anxiety hierarchy, based on reciprocal inhibition (Joseph Wolpe). Compare: Flooding. *Ch 5*

T

Tail coverage — An extended reporting endorsement that extends the time to report claims after a claims-made policy ends; the answer for a retiring counselor with a claims-made policy. Compare: Prior acts coverage. *Ch 17*

Tangential thought — Speech that wanders off and never returns to the point. Compare: Circumstantial thought. *Ch 19*

Tardive dyskinesia — Involuntary lip, tongue, or limb movements after long-term antipsychotic use; may be permanent. Screened with the AIMS. Compare: Acute dystonia. *Ch 23*

Tarvydas integrative model — An ethical decision-making model with four themes (reflection, balance, context, collaboration) and four stages. Stage III examines the counselor's own competing values and blind spots (Vilia Tarvydas). Compare: Forester-Miller & Davis model. *Ch 22*

Tasks of mourning — Accept the reality of the loss, process the pain, adjust to a world without the deceased, and find an enduring connection while embarking on a new life (J. William Worden). Compare: Five stages of dying. *Ch 12*

Telehealth licensure — Counselors generally must be licensed or otherwise authorized in the state where the client is physically located during the session. Under ACA H. 1.b, distance counselors may be subject to the laws of both the counselor's location and the client's place of residence. *Ch 9*

Termination criteria — Observable end points agreed with the client early, in informed consent and initial planning, and revisited at each review. *Ch 21*

Test-retest reliability — Stability over time, checked by giving the same test to the same people twice. Compare: Internal consistency. *Ch 3*

TF-CBT — Trauma-Focused CBT for children and adolescents with a nonoffending caregiver, following PRACTICE: skills first, then a trauma narrative shared with the caregiver (Cohen, Mannarino, & Deblinger). Compare: Cognitive Processing Therapy. *Ch 5*

Therapeutic factors — Yalom's 11 curative elements of group work, including instillation of hope, universality, altruism, group cohesiveness, catharsis, and interpersonal learning (Irvin Yalom). *Ch 8*

Tolerance — Needing more of a substance for the same effect, or getting less effect from the same amount. It doesn't count toward diagnosis with medications taken as prescribed. Compare: Withdrawal. *Ch 11*

TPO — Treatment, payment, and health care operations, for which HIPAA permits disclosure without authorization. Ethics and many state laws still expect client consent (45 CFR 164.506). Compare: Authorization (HIPAA). *Ch 17*

Transference — The client redirects feelings and expectations from important past relationships onto the counselor. Compare: Countertransference. *Ch 5*

Transition stage — The group stage of anxiety, resistance, conflict, and challenges to the leader (Tuckman's storming); the leader responds nondefensively (Gerald Corey). Compare: Contemplation. *Ch 8*

Translator — A professional who renders written text, such as consent forms and safety plan handouts, into another language. Compare: Interpreter. *Ch 23*

Transtheoretical model — The stages of change: precontemplation, contemplation, preparation, action, and maintenance. It describes readiness and is not a therapy (Prochaska & DiClemente). Compare: Motivational interviewing. *Ch 5*

Triangular theory of love — Love's three components are intimacy, passion, and commitment. All three make consummate love; intimacy plus commitment is companionate love (Robert Sternberg). *Ch 13*

Triangulation — Two people in conflict draw in a third to reduce tension, like a parent confiding in a child about the other parent (Murray Bowen). Compare: Enmeshment. *Ch 7*

True experiment — A design with random assignment, manipulation of the independent variable, and a control group; the strongest for cause and effect. Compare: Quasi-experiment. *Ch 18*

T-score — A standard score with a mean of 50 and an SD of 10. On the MMPI-3, 65 or above is generally clinically elevated. Compare: z-score. *Ch 20*

t-test — Compares two group means: independent for different groups, paired for the same people measured twice (William Gosset). Compare: ANOVA (one-way). *Ch 18*

Tuckman's stages — Forming, storming, norming, performing, and adjourning, with adjourning added in 1977 (Bruce Tuckman, with Mary Ann Jensen). Compare: Transition stage. *Ch 3*

Type I error — Rejecting a true null hypothesis: a false positive. Compare: Type II error. *Ch 18*

Type II error — Failing to reject a false null hypothesis: a false negative. Compare: Type I error. *Ch 18*

U

Unconditional positive regard — Valuing the client as a person without conditions; it doesn't mean approving of everything the client does (Carl Rogers). *Ch 22*

Unique outcomes — Narrative therapy's counterpart to the SFBT exception; the word, not the meaning, tells you the owner (White & Epston). Compare: Exception questions. *Ch 2*

Universality — Yalom's therapeutic factor of realizing "I'm not the only one" (Irvin Yalom). Compare: Group cohesiveness. *Ch 8*

Universalizing — A group leader skill that names a shared theme for the whole group. Compare: Linking. *Ch 8*

Unspecified disorder — A diagnosis in which the clinician chooses not to, or can't, specify why full criteria aren't met; common in emergency settings. Compare: Other specified disorder. *Ch 19*

V

Values-based referral — Referring a client solely because of the counselor's personal values, which is prohib-

ited (ACA A.11.b). Referral is appropriate for competence reasons, not values. *Ch 9*

Veracity — The ethical principle of dealing truthfully with individuals. Compare: Fidelity. *Ch 17*

Vicarious liability — Supervisors can be held legally responsible for supervisees' actions (respondeat superior). Compare: Direct liability. *Ch 17*

Vicarious trauma — Lasting changes in the helper's beliefs and worldview from repeated exposure to clients' trauma (McCann & Pearlman). Compare: Burnout. *Ch 14*

Virtue ethics — Asks "Who shall I be?" and focuses on the counselor's character, through virtues such as prudence, integrity, respectfulness, and benevolence (Meara, Schmidt, & Day). Compare: Principle ethics. *Ch 22*

W

Warm handoff — The strongest active referral: the counselor introduces the client to the new provider directly, in person or on a joint call. Compare: Passive referral. *Ch 21*

WDEP — Wants, Doing, Evaluation, Planning, as in "Is what you're doing getting you what you want?" (Robert Wubbolding, building on Glasser's reality therapy). Compare: Disputing. *Ch 2*

Well-formed goals — Goals that are small, concrete, behavioral, stated as the presence of something, important to the client, and within the client's control (Steve de Shazer). Compare: SMART objectives. *Ch 21*

WHODAS 2.0 — The disability assessment schedule the DSM-5-TR suggests for measuring functioning, in place of the dropped GAF. *Ch 19*

Window of tolerance — The zone of arousal where a person can function; trauma narrows it, causing swings into hyperarousal or hypoarousal (Daniel Siegel). *Ch 10*

Withdrawal — A substance-specific syndrome that appears when use stops or decreases. Compare: Tolerance. *Ch 11*

Working alliance — Agreement on goals, agreement on tasks, and the bond between client and counselor (Edward Bordin). *Ch 4*

Working stage — The group stage of cohesion, genuine self-disclosure, risk-taking, and direct feedback (Tuckman's norming/performing) (Gerald Corey). Compare: Transition stage. *Ch 8*

Wraparound — A family-driven planning process, not a level of care, that brings family, natural supports, and systems into one team writing one individualized, strengths-based plan (Bruns et al.; within systems of care, Stroul & Friedman). Compare: Assertive community treatment (ACT). *Ch 23*

Y

Yerkes–Dodson law — Performance improves with arousal up to a point, then declines (Yerkes & Dodson). *Ch 10*

Z

Z codes — Other conditions that may be a focus of clinical attention, such as uncomplicated bereavement or relationship distress; not mental disorders. *Ch 19*

Zone of proximal development — Learning that happens socially, just beyond current ability, supported by scaffolding (Lev Vygotsky). *Ch 3*

z-score — The basic standard score, with a mean of 0 and an SD of 1: z = (score − mean) ÷ SD. Compare: T-score. *Ch 20*

Audio Companion Track List

The audio companion is sold separately on PassReady Prep. Every track is call-and-response: you hear a term, the narrator pauses, you say the owner out loud, then the answer plays. Play tracks on repeat during drives, walks, and chores.

Track numbers match chapter numbers (Track 10.1 goes with Chapter 10). Chapter 1 covers exam format and strategy and has no track. Chapter 23 and the sections added for the 2027 specifications (in Chapters 5, 17, 21, and 22) have no track.

Track	Title	Chapter	Covers	Target length
2.1	Theory Ownership: Psychodynamic, Humanistic, and Existential	2. The Theory Ownership Matrix	Psychodynamic + Humanistic/ Existential	~12 min
2.2	Theory Ownership: Behavioral, Cognitive-Behavioral, and Change Models	2. The Theory Ownership Matrix	Behavioral + Cognitive-Behavioral + change models	~10 min
2.3	Theory Ownership: Postmodern and Family Systems	2. The Theory Ownership Matrix	Postmodern + Family Systems	~10 min
2.4	Swap Pairs Rapid Fire	2. The Theory Ownership Matrix	Swap Pairs Rapid Fire (all families)	~8 min
3.1	Ownership Beyond Theory: Development and Identity	3. Ownership Beyond Theory	Development + identity models	~5 min

Track	Title	Chapter	Covers	Target length
3.2	Ownership Beyond Theory: Career	3. Ownership Beyond Theory	Career theorists	~3 min
3.3	Ownership Beyond Theory: Group	3. Ownership Beyond Theory	Group stages and group owners	~2 min
3.4	Ownership Beyond Theory: Assessment and Research	3. Ownership Beyond Theory	Assessment and research rapid fire	~4 min
4.1	Name That Skill	4. The Alliance and the Microskills	Label the counselor's response	~10 min
5.1	Match It	5. Theory into Intervention	Problem, age, and stage → intervention	~10 min
6.1	Safety First	6. Crisis, Safety, and Risk	Crisis models, suicide assessment, duty to protect, reporting	~8 min
7.1	Think in Systems	7. Couples and Family Systems in Session	Concepts, tools, life cycle, couples	~7 min
8.1	Lead the Group	8. Group Work	Stages, skills, harmful behaviors, ethics	~7 min
9.1	Fit the Client	9. Delivery Formats and Special Populations	Culture, faith, addiction, telehealth, populations	~7 min
10.1	Time Frames and Owners	10. Mood, Anxiety, Obsessive, and Trauma Presentations	Mood, anxiety, OCD, trauma	~8 min
11.1	Count It and Spot It	11. Substance Use and Process Addictions	Criteria, severity, intoxication, withdrawal, gambling	~5 min

Track	Title	Chapter	Covers	Target length
12.1	Across the Life Span	12. Life Span and Development	Neurodevelopment, separation, aging, grief, end of life	~6 min
13.1	Families in Transition	13. Relationships and Family Life	Love, divorce, stepfamilies, adoption, parenting, violence	~6 min
14.1	Work It	14. Career and Occupation	Holland, Super, instruments, burnout, job loss	~4 min
15.1	Identity and Meaning	15. Culture, Identity, and Meaning	Culture shock, gender identity, minority stress, racism, faith, existential concerns	~4 min
16.1	Body and Mind	16. Body and Mind	Illness, behavior, bullying, sleep, eating, sex, stress, psychosis	~7 min
17.1	Code and Law	17. Ethics and Law	Principles, consent, confidentiality, boundaries, records, decision-making	~6 min
18.1	Pick the Test	18. Research, Statistics, and Program Evaluation	Designs, validity threats, statistics, evaluation	~5 min
19.1	Intake and MSE	19. Intake, Interview, Mental Status Exam, and Diagnosis	Interview, MSE terms, diagnosis, level of care	~5 min
20.1	Score It	20. Assessment and Testing	Test types, scores, SEM, instruments, ethics	~5 min
21.1	Plan It	21. Treatment Planning, Referral, and Termination	Goals, priorities, referral, notes, revision, termination	~4 min

Track	Title	Chapter	Covers	Target length
22.1	Be the Instrument	22. The Counselor as Instrument: Core Counseling Attributes	Rogers's conditions, empathy, congruence, regard, conflict, self-awareness	~4 min

27 tracks.

References

All sources cited in Chapters 1–23, in APA 7 style. Each chapter also ends with its own short reference list.

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